

NCLEX-PN

Question Bank

Volume 2 — Questions 2001–4000

2,000 Practice Questions with Detailed Answers & Explanations

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Psychosocial Integrity

— Continued

Stress management

2001. A nurse is caring for a client who has been studying for licensing exams and reports headaches, poor concentration, and sleeping 4 hours each night. Which teaching is most appropriate?

- A. Increase caffeine intake to stay alert for longer study sessions
- B. Study continuously until the exam date to avoid losing momentum
- C. Schedule regular breaks, maintain sleep, and use relaxation techniques
- D. Avoid all physical activity until the exam is over

Correct Answer: C

Regular breaks, adequate sleep, and relaxation techniques promote better stress control and cognitive function. Choices A and B can worsen stress and fatigue, and choice D removes a healthy coping strategy because physical activity can help reduce stress.

2002. A hospitalized client says, "When I feel stressed, I usually have a few drinks to calm down." Which nursing response is best?

- A. "Alcohol can make stress worse over time. Let's talk about safer ways to cope."
- B. "As long as you do not drink every day, that should be fine."
- C. "You should stop drinking immediately and never discuss it again."
- D. "If alcohol works for you, keep using it until you feel better."

Correct Answer: A

Choice A is nonjudgmental, identifies the risk of maladaptive coping, and redirects the client toward healthier stress-management strategies. Choices B and D reinforce unsafe coping, and choice C is punitive and blocks communication.

2003. A client newly admitted to a long-term care facility appears withdrawn and says, "Everything is happening too fast." Which action by the nurse is best to help reduce the client's stress?

- A. Introduce the client to several residents at once to speed adjustment
- B. Provide simple explanations of routines and allow time for questions
- C. Encourage the family to leave quickly so the client can adapt sooner
- D. Limit the client's choices until the stress passes

Correct Answer: B

Clear explanations and time for questions reduce uncertainty, which is a common source of stress during transitions. Choice A may overwhelm the client, choice C may increase distress, and choice D reduces control, which can worsen stress.

2004. A nurse is assisting a client who reports stress related to chronic pain. Which statement by the client suggests the need for further teaching about coping?

- A. "I will try relaxation breathing when the pain starts to build."
- B. "I can keep a record of what makes the pain and stress worse."
- C. "I plan to stay in bed all day on difficult days so I do not have to deal with anything."
- D. "I may ask family members for help when I am having a hard day."

Correct Answer: C

Remaining in bed all day as a coping pattern can increase isolation, reduce function, and contribute to ineffective stress management. The other choices reflect adaptive strategies such as relaxation, tracking triggers, and seeking support.

2005. A nurse is caring for four clients. Which client should the nurse assess first for ineffective stress management?

- A. A client awaiting discharge who asks for written instructions about diet changes
- B. A client who says, "I haven't slept in days, and I feel like I might lose control"
- C. A client receiving visitors who says feeling tired is making the day harder

D. A client who reports being nervous before a scheduled physical therapy session

Correct Answer: B

Severe sleep deprivation with a statement about possibly losing control suggests escalating stress that may impair judgment and safety, so this client needs priority assessment. The other clients show lower-acuity stress or expected situational anxiety.

2006. A client with heart failure becomes frustrated during teaching and says, "There is too much to remember." Which nursing approach best supports stress management while promoting learning?

- A.** Continue the teaching so all instructions are finished before discharge
- B.** Stop all teaching and defer education entirely to the family
- C.** Break information into small parts and ask the client to repeat key points
- D.** Tell the client that the instructions are basic and should not be difficult

Correct Answer: C

Stress can impair concentration and learning, so chunking information and using teach-back improves understanding while reducing overload. Choice A ignores the client's distress, choice B removes the client from the learning process, and choice D is dismissive.

2007. A client says, "My chest feels tight every time I think about going back to work after this hospitalization." The client's vital signs are stable, and the provider has ruled out acute cardiac problems. Which nurse response is best?

- A.** "Since your tests are normal, there is nothing to worry about."
- B.** "Let's identify what about returning to work is causing the most stress for you."
- C.** "You should avoid thinking about work until after discharge."
- D.** "I will ask your family to make the decision for you."

Correct Answer: B

The best response is to explore the stressor and help the client identify specific concerns that can be addressed. Choice A dismisses symptoms, choice C encourages avoidance, and choice D removes the client's autonomy rather than building coping skills.

2008. A nurse is evaluating whether a stress-management plan has been effective for a client recovering from a myocardial infarction. Which finding is the best indicator of progress?

- A.** The client states, "I never feel anxious anymore."
- B.** The client participates in guided imagery and reports using it when feeling tense
- C.** The client asks the nurse to remain in the room whenever family visits
- D.** The client says, "I avoid discussing my fears so I won't get upset."

Correct Answer: B

Using a learned relaxation technique when tension occurs shows the client is applying an effective coping strategy. Choice A is unrealistic, while choices C and D suggest dependence or avoidance rather than improved self-management.

2009. A nurse is developing a plan of care for a client whose stress increases whenever multiple staff members give instructions quickly. Which intervention is most appropriate?

- A.** Have each staff member provide full instructions as tasks arise to improve efficiency
- B.** Limit communication to yes-or-no questions only
- C.** Coordinate care so one staff member gives organized instructions at a slower pace
- D.** Avoid explaining procedures to prevent overstimulation

Correct Answer: C

Coordinating communication and slowing the pace helps reduce environmental stress and improves the client's ability to process information. Choice A increases overload, choice B overly restricts communication, and choice D withholds information needed for coping and informed participation.

2010. A client who is anxious about an upcoming surgery tells the nurse, "I can't stop thinking about everything that could go wrong." Which nursing action is most appropriate to help the client manage stress at this time?

- A.** Encourage the client to discuss specific concerns and practice slow deep-breathing exercises
- B.** Tell the client that worrying is normal and suggest trying not to think about the surgery
- C.** Explain that the provider will answer questions later and advise the client to rest now
- D.** Change the subject to distract the client from negative thoughts

Correct Answer: A

Answer A is correct because it combines therapeutic communication with an immediate, practical stress-reduction technique. Inviting the client to discuss specific concerns helps the nurse assess the source of anxiety, and slow deep breathing can reduce physiologic stress in the moment. Choice B minimizes the client's feelings and gives vague advice. Choice C delays support and does not address the client's current anxiety. Choice D may temporarily divert attention, but it avoids the client's concern rather than helping the client cope effectively.

2011. A client says, "Whenever I get stressed, I skip meals and keep working until I finish everything." Which response by the nurse is most appropriate?

- A.** "That shows good self-discipline during stressful times."
- B.** "Try to ignore the stress so you can stay productive."
- C.** "Skipping meals can make stress harder to manage; let's discuss healthier ways to cope."
- D.** "You should ask the provider for a sedative whenever this happens."

Correct Answer: C

Skipping meals can worsen fatigue, irritability, and the body's stress response. The best nursing response identifies the ineffective coping behavior and redirects the client toward healthier stress-management strategies. Choice A reinforces an unhealthy pattern, choice B promotes avoidance rather than coping, and choice D suggests medication before nonpharmacologic measures are explored.

2012. A nurse is teaching a client about relaxation techniques to use during stress. Which client action best demonstrates correct use of deep-breathing exercises?

- A.** Taking rapid breaths through the mouth for 30 seconds
- B.** Inhaling slowly through the nose and exhaling slowly through pursed lips
- C.** Holding the breath as long as possible before exhaling
- D.** Breathing only when feelings of panic become severe

Correct Answer: B

Deep breathing is most effective when the client takes slow, controlled breaths, commonly inhaling through the nose and exhaling slowly through pursed lips. This promotes relaxation and decreases physiologic tension. Rapid breathing can increase anxiety, prolonged breath-holding is not the goal, and waiting until panic is severe reduces the preventive value of the technique.

2013. A client recovering from pneumonia says, "I feel guilty asking my family for help because they are busy." Which nursing suggestion best supports stress management?

- A.** Encourage the client to avoid discussing needs so the family will not worry
- B.** Advise the client to wait until stress becomes overwhelming before asking for support
- C.** Help the client identify one specific task a family member could assist with
- D.** Recommend that the client handle all activities independently to rebuild confidence

Correct Answer: C

Breaking support into one specific, realistic request helps the client use available resources and reduces stress. Asking for help is an adaptive coping strategy. Choices A and B delay or block support, and choice D may increase the client's burden during recovery.

2014. The nurse is reinforcing teaching about stress management for a client with a demanding schedule. Which client statement indicates the need for further teaching?

- A. "I can set aside short periods during the day to stretch and breathe slowly."
- B. "Regular sleep habits can help me cope with stress more effectively."
- C. "Exercise may help lower my stress level."
- D. "I cope best if I keep all my worries to myself so I do not burden others."

Correct Answer: D

Keeping all worries to oneself can increase stress and reduce access to support. Healthy stress management often includes communication, social support, and practical coping strategies. Choices A, B, and C are all appropriate stress-reduction approaches.

2015. A postoperative client says, "I don't know why I'm so irritable. I keep snapping at everyone." Which nurse response is best?

- A. "You need to control your temper better."
- B. "Tell me more about what has been most stressful for you today."
- C. "That is normal after surgery, so try not to think about it."
- D. "I will ask the provider to limit visitors immediately."

Correct Answer: B

The best response uses therapeutic communication to assess the source of stress before deciding on interventions. Exploring what is stressful may reveal pain, fear, fatigue, or information overload. Choice A is judgmental, choice C minimizes the client's feelings, and choice D takes action without first assessing what the client needs.

2016. A client with chronic kidney disease is learning stress-management strategies. Which plan is most likely to be effective?

- A. Use one coping strategy only when stress becomes unbearable
- B. Rely on television for distraction and avoid discussing concerns
- C. Choose stress-reduction methods that fit the daily routine and practice them regularly
- D. Wait until all medical treatment is completed before trying new coping skills

Correct Answer: C

Stress-management plans are most effective when they are realistic, individualized, and practiced consistently rather than used only in crisis. Choice C reflects that principle. Choices A and D delay coping, and choice B emphasizes avoidance and social withdrawal instead of active stress management.

2017. A nurse is caring for a client who becomes tense before each dressing change. Which intervention is best to reduce anticipatory stress?

- A. Perform the dressing change as quickly as possible without explaining the steps
- B. Teach the client to use a relaxation technique shortly before and during the procedure
- C. Delay the dressing change until the client no longer mentions feeling stressed
- D. Ask family members to leave so the client can face the procedure alone

Correct Answer: B

Using a relaxation method before and during a known stressor can reduce anticipatory stress and improve coping. Choice B is proactive and practical. Choice A may increase anxiety through loss of control, choice C avoids necessary care, and choice D removes support without a clear reason.

2018. A client says, "When I start feeling stressed, I go for a 20-minute walk, then I call my sister if I still feel overwhelmed." Which interpretation by the nurse is most accurate?

- A. The client is showing dependence and poor self-control
- B. The client is using both personal and social coping strategies

- C. The client is avoiding the source of stress completely
- D. The client needs to replace exercise with bed rest

Correct Answer: B

The client is using an individual coping skill first and then reaching out for support, which reflects adaptive stress management. Choice A incorrectly labels healthy support-seeking as dependence. Choice C is inaccurate because the client is actively coping, and choice D does not support stress reduction.

2019. A nurse is helping a client develop a stress-management routine after discharge. Which client factor is most important for the nurse to assess when deciding whether the plan is realistic?

- A. Whether the client prefers the same technique as other clients
- B. Whether the plan includes at least three different relaxation methods
- C. Whether the coping strategies fit the client's abilities, schedule, and preferences
- D. Whether family members believe stress should be managed without discussion

Correct Answer: C

A stress-management plan is more likely to succeed when it matches the client's actual routine, functional ability, and personal preferences. Choice A focuses on comparison with others instead of individualization. Choice B imposes an arbitrary number of techniques, and choice D reflects a belief that may hinder communication rather than improve coping.

2020. A nurse is evaluating outcomes for a client who has been practicing stress-management strategies for 1 week. Which finding best suggests improvement?

- A. The client reports never feeling stressed anymore
- B. The client identifies early signs of tension and uses planned coping measures
- C. The client asks the nurse to choose which coping technique to use each time
- D. The client avoids situations that might cause any frustration

Correct Answer: B

Improvement is shown when the client recognizes stress early and independently uses planned coping strategies. Choice A is unrealistic; stress cannot usually be eliminated entirely. Choice C shows ongoing dependence, and choice D reflects avoidance rather than healthy management.

2021. A client with newly diagnosed hypertension says, "I know stress is bad for me, but I don't have time for long relaxation sessions." Which nursing response is best?

- A. "If you cannot set aside 30 minutes, stress management probably will not help."
- B. "You should focus only on medication because lifestyle changes take too much effort."
- C. "Brief methods, such as slow breathing for a few minutes several times a day, can still be helpful."
- D. "Wait until your schedule becomes less busy before trying to reduce stress."

Correct Answer: C

Short, practical interventions can still be effective and are more likely to be used consistently. Choice C adapts the plan to the client's circumstances. Choices A, B, and D discourage behavior change and miss an opportunity to support realistic coping.

2022. A nurse is caring for a client who reports ongoing financial stress, poor sleep, frequent headaches, and arguments with family. Which nursing goal is the best priority related to stress management?

- A. The client will deny having any stressors by the end of the shift
- B. The client will identify personal stressors and one healthy coping strategy to use
- C. The client will persuade family members to stop bringing up finances
- D. The client will eliminate all headache pain before discharge

Correct Answer: B

A priority stress-management goal is helping the client recognize stressors and identify a practical coping method. This supports awareness and action. Choice A is unrealistic and reflects denial, choice C focuses on controlling others, and choice D may be desirable clinically but does not directly address the psychosocial stress-management priority.

2023. A client says, "My stress is worst at night because I lie in bed replaying everything that went wrong during the day." Which nursing suggestion is most appropriate?

- A.** Review each upsetting event in detail until you feel exhausted
- B.** Use a consistent bedtime routine and practice a calming technique before sleep
- C.** Drink several caffeinated beverages in the evening so you can finish tasks earlier
- D.** Stay in bed longer on days off to make up for lost sleep

Correct Answer: B

A structured bedtime routine and relaxation before sleep can reduce nighttime rumination and improve stress management. Choice A may intensify the stress response, choice C can worsen sleep and anxiety, and choice D may disrupt sleep patterns further rather than improve them.

2024. A client studying for a licensing exam tells the nurse, "I get so overwhelmed that I jump from one task to another and never finish anything." Which strategy should the nurse recommend first to improve stress management?

- A.** Make a long list of every task that needs to be done before starting any of them
- B.** Choose one priority task, break it into smaller steps, and complete one step at a time
- C.** Avoid thinking about the exam until the day before to reduce pressure
- D.** Stay up later each night until all unfinished tasks are completed

Correct Answer: B

Breaking one priority task into smaller, manageable steps helps reduce overload and improves a sense of control, which supports stress management. Choice A may increase stress by emphasizing everything at once. Choice C uses avoidance, which usually worsens stress later. Choice D is counterproductive because sleep loss can increase stress and impair concentration.

2025. A client caring for an ill parent says, "By the end of the day, my shoulders are tight, my heart races, and I don't notice how stressed I am until I'm ready to explode." Which nursing recommendation is most appropriate?

- A.** Wait until symptoms are severe, then try to rest for the remainder of the day
- B.** Use alcohol in small amounts when tension begins so the body can relax quickly
- C.** Set specific times during the day to pause and check for early signs of stress, such as muscle tension or rapid breathing
- D.** Focus only on finishing caregiving tasks because emotions will settle once responsibilities decrease

Correct Answer: C

Scheduling brief check-ins to notice early physical signs of stress promotes self-awareness and allows the client to use coping measures before stress escalates. Choice A delays action until the client is already overwhelmed. Choice B is unsafe and does not promote healthy coping. Choice D ignores the importance of recognizing and managing stress responses as they occur.

2026. A client who started a new job tells the nurse, "By the time I get home, I feel completely overwhelmed. I just sit there thinking about everything I still have to do." Which nursing suggestion is best to help the client manage stress in the short term?

- A.** Make a simple list and complete the most urgent task first
- B.** Ignore the unfinished tasks until the stress goes away
- C.** Stay up later to finish every task before going to bed
- D.** Avoid talking about the stress so it does not become worse

Correct Answer: A

Making a simple list and starting with the most urgent task breaks an overwhelming situation into manageable steps and promotes problem-focused coping. Ignoring tasks usually increases stress, staying up later can worsen fatigue and coping ability, and avoiding discussion may prevent the client from using support effectively.

2027. A client with type 2 diabetes tells the nurse, "When I'm stressed, I stop checking my blood sugar because I don't want one more thing to deal with." Which response by the nurse is most appropriate?

- A. "You should avoid thinking about stress until your glucose is controlled."
- B. "Stress can affect self-care, so let's identify one small step you can keep doing even on hard days."
- C. "If checking your blood sugar makes you anxious, you can skip it until you feel calmer."
- D. "The best way to handle this is to rely on willpower and push yourself harder."

Correct Answer: B

This response connects stress to health behaviors and helps the client develop a realistic coping plan by focusing on one achievable action. That supports self-management without judgment. Choice A dismisses the role of stress, choice C reinforces unsafe avoidance, and choice D is nontherapeutic because it is vague, blaming, and not action-oriented.

2028. A client whose spouse died 2 months ago tells the nurse, "I barely sleep, I cry all the time, my chest feels tight, and I have started drinking several glasses of wine every night just to calm down." What is the nurse's priority action related to stress management?

- A. Suggest keeping a journal before bedtime for emotional release
- B. Teach the client a guided imagery exercise to practice nightly
- C. Assess the client's alcohol use further and notify the supervising nurse or provider of the maladaptive coping pattern
- D. Encourage the client to stay busy during the day to avoid thinking about the loss

Correct Answer: C

The priority is to recognize that the client is using alcohol as a maladaptive coping strategy and requires further assessment and follow-up. Substance use to manage stress can increase safety risks and worsen sleep and emotional symptoms. Journaling and guided imagery may be helpful later but do not address the most urgent concern. Staying busy to avoid grief promotes avoidance rather than healthy coping.

2029. A client says, "When I get stressed, I usually smoke several extra cigarettes because it helps me calm down right away." Which response by the nurse is best?

- A. Stress relief from smoking is temporary, so let's identify another coping method you can use when you first feel tense.
- B. If smoking relaxes you, it is acceptable to use it during especially stressful days.
- C. You should stop smoking immediately and avoid thinking about stress triggers.
- D. The best way to manage stress is to wait until the feeling passes on its own.

Correct Answer: A

A is correct because it recognizes the client's current behavior without judgment and redirects the client toward a healthier stress-management strategy. Smoking may feel calming in the moment, but it is not an effective long-term coping method and carries health risks. B reinforces a maladaptive coping behavior. C is less therapeutic because it is abrupt and unrealistic, and avoiding triggers does not teach coping. D is incorrect because passive waiting does not help the client build stress-management skills.

Spiritual considerations

2030. A client says, "I think God is punishing me with this illness." Which response by the practical nurse is most therapeutic?

- A. You should try not to think that way.
- B. Why would you believe that God is punishing you?
- C. Tell me more about what this illness means to you spiritually.
- D. God would never punish someone who has lived a good life.

Correct Answer: C

This response invites the client to explore personal beliefs and feelings without judgment. It uses therapeutic communication and respects spiritual concerns. Choice A dismisses the client's feelings, choice B may sound challenging or defensive, and choice D imposes the nurse's own belief system.

2031. A hospitalized client requests time for evening prayer and asks not to be interrupted unless necessary. Which nursing action is most appropriate?

- A. Explain that unit routines must take priority over personal rituals
- B. Arrange care when possible to allow uninterrupted prayer time
- C. Tell the client to complete prayer after all treatments are finished
- D. Ask the family to handle the client's spiritual needs instead

Correct Answer: B

The nurse should support the client's spiritual practices when possible by planning care around them. This respects the client's preferences and promotes psychosocial well-being. Choices A and C unnecessarily place routine above an important client need, and D shifts responsibility away from the nurse.

2032. A client who recently learned of a terminal diagnosis says, "I don't want to see any clergy. I am too angry at my faith right now." What is the nurse's best response?

- A. You may feel differently later, so I will contact clergy now
- B. Many people regret refusing spiritual help during a crisis
- C. I understand. If you want to talk about those feelings, I can listen
- D. You should not let anger interfere with your beliefs

Correct Answer: C

The nurse should acknowledge the client's feelings and offer presence without pressuring the client. Anger related to faith can be part of spiritual distress. Choices A, B, and D are nontherapeutic because they push the nurse's agenda and fail to respect the client's current wishes.

2033. A client is scheduled for a diagnostic test in 30 minutes and asks for a visit from a spiritual leader before leaving the unit. What should the nurse do first?

- A. Notify the charge nurse that the test should be canceled
- B. Tell the client there is not enough time for spiritual care today
- C. Assess the urgency of the test and try to arrange the visit as soon as possible
- D. Ask the unlicensed assistive personnel to provide spiritual counseling

Correct Answer: C

The nurse should first consider the urgency of the scheduled care and then try to support the client's request in a timely way. This balances physical needs with spiritual needs. Choice A is premature, choice B dismisses the request, and choice D is inappropriate because spiritual counseling is not delegated this way.

2034. A client tells the nurse, "I need to keep this amulet on at all times for spiritual protection." The client is being prepared for surgery. Which response is best?

- A. You cannot wear any personal item in the operating room, so remove it now
- B. Let's discuss this with the surgical team to see whether it can be safely secured or temporarily removed
- C. Spiritual objects are not important during surgery because you will be asleep
- D. Give the amulet to a family member so the issue is no longer a problem

Correct Answer: B

The best action is to respect the client's spiritual belief while coordinating with the team about safety requirements. This approach protects the client and avoids dismissing the belief. Choice A is rigid without exploring options, choice C is disrespectful, and choice D ignores the client's attachment to the item.

2035. Which statement by a client most clearly suggests spiritual distress?

- A. I would like my meals served after my meditation time
- B. My faith community usually visits me on weekends

C. I no longer see any meaning in my life since this happened

D. I prefer quiet music when I pray

Correct Answer: C

Loss of meaning and purpose is a common indicator of spiritual distress. Choices A, B, and D describe spiritual preferences or practices, not distress. The key difference is that choice C reflects a deeper existential struggle.

2036. A nurse is admitting a client from a different cultural and religious background. Which question is most appropriate to assess spiritual needs?

A. What religion are you, exactly?

B. Do you want us to follow the same practices used for all clients here?

C. Are there spiritual or religious practices that you want us to consider in your care?

D. You do believe in prayer, don't you?

Correct Answer: C

This question is open-ended, respectful, and focused on how care can be individualized. Choice A can feel intrusive if asked without context, choice B assumes standardization over individual preference, and choice D is leading and presumptive.

2037. A client says, "Please pray with me." The nurse is uncomfortable leading prayer because the client's beliefs are different from the nurse's own. Which action is best?

A. Politely decline and leave the room immediately

B. Tell the client that professional boundaries prohibit prayer

C. Offer to stay quietly with the client and ask whether the client would like a chaplain or spiritual leader contacted

D. Lead a prayer based on the nurse's own beliefs to provide comfort

Correct Answer: C

The nurse should support the client's spiritual needs without imposing personal beliefs. Offering presence and facilitating access to appropriate spiritual support is client-centered. Choice A abandons the client, choice B is too absolute and not therapeutic, and choice D centers the nurse's beliefs instead of the client's.

2038. The family of an unconscious client asks the nurse to place a sacred text under the client's pillow. The client has no documented preference, and the text will not interfere with care. What is the nurse's best action?

A. Allow it because it is harmless and may provide comfort to the family

B. Refuse because only the client can request spiritual items

C. Discard the item because loose objects are never allowed in client rooms

D. Tell the family spiritual matters must wait until the client awakens

Correct Answer: A

If the item does not interfere with care or safety, allowing it is a reasonable, supportive action. This respects possible spiritual values and helps the family cope. Choices B and D are unnecessarily rigid, and choice C is incorrect because harmless personal items are not automatically prohibited.

2039. A client who is fasting for religious reasons is prescribed medication that should be taken with food. Which action by the nurse is most appropriate?

A. Tell the client to stop fasting while hospitalized

B. Withhold the medication until the fasting period ends

C. Explore the client's wishes and notify the provider about the conflict between the fast and the medication plan

D. Give the medication with water and do not mention the issue

Correct Answer: C

The nurse should assess the client's preferences and collaborate with the provider to address the conflict safely. This respects spiritual practice while protecting the client's health. Choice A dismisses the belief, choice B may be unsafe, and choice D ignores an important care issue.

2040. A client says, "I am afraid I will die before I can make peace with my family and my faith." Which nursing response is best?

A. You should focus on staying positive instead of thinking about death

B. Would you like help contacting family or someone from your faith community?

- C. Those worries are common, so try to rest now
- D. The provider can discuss those concerns during rounds tomorrow

Correct Answer: B

This response addresses the client's expressed spiritual and relational concerns and offers concrete support. Choice A minimizes the concern, choice C is vaguely reassuring without action, and choice D delays help for a meaningful need.

2041. During morning care, a client becomes tearful and says, "I used to pray every day, but now I feel abandoned." Which finding is the nurse identifying?

- A. Ineffective airway clearance
- B. Risk for violence
- C. Spiritual distress
- D. Sensory-perceptual alteration

Correct Answer: C

Feeling abandoned by a higher power and expressing disrupted spiritual connection are cues of spiritual distress. The other options do not match the data in the scenario. This question tests recognition of the psychosocial meaning of the client's statement.

2042. A client asks the nurse, "Do you think my illness happened because I have not been living right?" Which response is most therapeutic?

- A. Yes, many illnesses are caused by poor choices
- B. No, and you need to stop blaming yourself
- C. What thoughts have you been having about why this happened?
- D. That is a question only your spiritual leader can answer

Correct Answer: C

This open-ended response encourages exploration of the client's beliefs and feelings without judgment. Choice A reinforces guilt, choice B shuts down discussion even though the intention is kind, and choice D distances the nurse from the conversation instead of providing support.

2043. A nurse enters a client's room and finds a family ritual underway that is delaying a time-sensitive blood transfusion. What should the nurse do?

- A. Interrupt immediately and begin the transfusion without explanation
- B. Wait until the ritual is over, even if treatment is significantly delayed
- C. Explain the urgency, assess whether the ritual can be concluded quickly, and collaborate on the safest plan
- D. Tell the family that rituals are not permitted during medical treatment

Correct Answer: C

The nurse must protect the client from an unsafe delay while still showing respect for spiritual practices. Explaining urgency and collaborating is the best balance. Choice A is abrupt and disrespectful, choice B may endanger the client, and choice D is unnecessarily authoritarian.

2044. A client says, "I don't follow a religion, so spiritual care does not apply to me." Which understanding should guide the nurse's response?

- A. Spiritual care is limited to organized religious practices
- B. Only clergy should address spiritual concerns
- C. Spiritual concerns may include meaning, purpose, hope, and connection, not only religion
- D. Clients without a religion do not need assessment in this area

Correct Answer: C

Spiritual care is broader than formal religion and can involve meaning, values, hope, and connectedness. This understanding helps the nurse assess holistically. Choices A, B, and D incorrectly narrow spiritual care to religion alone.

2045. A client with newly diagnosed heart failure says, "I feel lost. I don't know what gives my life meaning anymore." Which response by the practical nurse is best?

- A. "You should focus on getting stronger before thinking about bigger issues."
- B. "Many people feel this way at first. What has helped you find meaning during difficult times before?"

C. "At least this diagnosis was found before it became even worse."

D. "I will ask your family to tell you what you should do next."

Correct Answer: B

Choice B is best because it acknowledges the client's distress and invites exploration of personal sources of meaning without making assumptions. This is therapeutic and supports spiritual assessment. Choice A dismisses the concern, choice C offers false reassurance, and choice D shifts control away from the client.

2046. A postoperative client tells the nurse, "My meditation recordings help me stay calm. Can I listen to them before physical therapy?" Which nursing action is most appropriate?

- A.** Encourage the client to use the recordings if they do not interfere with care
- B.** Tell the client relaxation methods are less important than therapy participation
- C.** Explain that only organized religious practices are part of spiritual care
- D.** Remove the device to reduce stimulation before therapy

Correct Answer: A

Choice A is correct because spiritual care includes supporting practices that provide meaning, peace, or connection, including meditation. If the practice is safe and does not interfere with treatment, the nurse should support it. Choices B and D unnecessarily limit a helpful coping strategy. Choice C is incorrect because spirituality is broader than formal religion.

2047. A client from a faith tradition unfamiliar to the staff is admitted for treatment. Which action by the practical nurse is best?

- A.** Ask the client whether there are any spiritual practices or restrictions the care team should know about
- B.** Wait for the client to bring up spiritual concerns later in the admission
- C.** Search for common rituals associated with the client's religion and follow those
- D.** Avoid discussing spirituality to prevent saying something insensitive

Correct Answer: A

Choice A is best because the nurse should assess the individual client's preferences directly rather than assume beliefs based on a stated religion or cultural background. Choice B delays assessment, choice C relies on stereotypes, and choice D avoids an important aspect of holistic care.

2048. A client says, "I don't want any religious discussion, but I do need quiet time alone each afternoon." Which nursing action is most appropriate?

- A.** Document the client's preference and try to protect that quiet time when possible
- B.** Explain that spiritual care requires discussion with the nurse
- C.** Arrange a clergy visit because the client may change their mind
- D.** Encourage family members to stay with the client during the afternoon

Correct Answer: A

Choice A is correct because the nurse should respect the client's stated preferences and support individualized sources of comfort, including quiet reflection. Choice B is incorrect because spiritual care is not limited to discussion. Choice C ignores the client's wishes, and choice D may interfere with the requested quiet time.

2049. An older adult client says, "My faith community is important to me, but no one knows I'm here." Which action by the practical nurse is most appropriate?

- A.** Notify the faith community immediately so the client feels supported
- B.** Ask the client whether they would like help contacting someone from the faith community
- C.** Tell the client that visitors should wait until discharge
- D.** Suggest the client focus on hospital staff for support instead

Correct Answer: B

Choice B is best because it respects the client's autonomy and offers assistance without violating privacy. Choice A is a trap because although support may help, the nurse should not contact others without the client's permission. Choices C and D unnecessarily limit support.

2050. A client receiving palliative care says, "I need to talk to someone about forgiveness before I can rest." Which action should the practical nurse take first?

- A. Encourage the client to wait until the next scheduled provider round
- B. Ask the client to explain what kind of support would be most helpful right now
- C. Tell the client to focus on pain control before emotional issues
- D. Advise the client to discuss the issue only with family members

Correct Answer: B

Choice B is correct because the nurse should first clarify the client's need and desired source of support. The client may want to speak with family, a clergy member, or simply talk with the nurse. This keeps care individualized. Choices A and C delay or minimize the concern, and choice D makes an assumption about the preferred support person.

2051. A client scheduled for a dressing change says, "Please don't uncover me completely. Modesty is very important in my beliefs." Which nursing action is best?

- A. Explain that full exposure is necessary for all dressing changes
- B. Provide as much privacy and draping as possible while completing the procedure safely
- C. Delay the dressing change until the provider can address the concern
- D. Tell the client that hospital routines must come before spiritual preferences

Correct Answer: B

Choice B is best because the nurse should accommodate spiritual or modesty-related preferences whenever possible while still providing safe care. Choice A is too rigid, choice C may unnecessarily delay treatment, and choice D dismisses the client's values.

2052. A client tells the nurse, "I have no religious beliefs, but being in nature has always given me peace." Which chart entry best reflects this statement?

- A. Client denies spiritual needs
- B. Client is noncompliant with spiritual care
- C. Client identifies nature as a source of peace and spiritual support

D. Client refuses discussion about coping resources

Correct Answer: C

Choice C is correct because it accurately documents the client's own source of spiritual support without forcing spirituality to mean religion. Choice A is incorrect because the client did identify a spiritual resource. Choices B and D are judgmental or inaccurate.

2053. A client becomes anxious before a procedure and asks to hold a small spiritual item in the hand not involved in treatment. The item does not create a safety risk. Which response by the practical nurse is best?

- A. "That should be fine if it does not interfere with the procedure."
- B. "Hospital policy does not allow personal items during procedures."
- C. "You need to learn to manage this anxiety without objects."
- D. "We can only allow items that have been approved by the provider."

Correct Answer: A

Choice A is best because the nurse should support harmless spiritual practices that promote comfort and coping. Choice B is overly absolute, choice C is nontherapeutic, and choice D adds an unnecessary barrier when no risk is present.

2054. The spouse of a confused client asks the nurse to remove several religious objects from the room, saying they are "making the room look cluttered." The client becomes calm when holding one of the objects and has not expressed any wish to remove it. What is the nurse's best action?

- A. Remove all objects because the spouse requested it
- B. Keep the object that appears comforting to the client and discuss the concern with the spouse
- C. Throw the items away because infection control is most important
- D. Tell the spouse that family members are not allowed to make any requests

Correct Answer: B

Choice B is correct because the nurse's primary concern is the client's well-being and preferences. A familiar spiritual object that calms the client should be preserved if it is safe. Choice A gives the spouse's preference priority over the client's comfort. Choice C is unjustified if the items are not creating a safety issue. Choice D is too absolute and unhelpful.

2055. A client says, "I need to face east at sunrise for prayer." The next morning, the nurse notices that a scheduled blood draw is planned during that time. Which action is most appropriate?

- A. Tell the client the blood draw cannot be changed because hospital routines are fixed
- B. Ask whether the prayer time can be delayed until after the blood draw
- C. Explore whether the blood draw timing can be adjusted while maintaining necessary care
- D. Cancel the blood draw to protect the client's spiritual practice

Correct Answer: C

Choice C is best because the nurse should attempt to accommodate the client's spiritual practice while preserving necessary treatment. Choice A is too rigid, choice B asks the client to compromise before the nurse has explored options, and choice D may inappropriately delay needed care.

2056. A client with advanced illness says, "I am afraid that everything I did in life was pointless." Which response by the practical nurse is most therapeutic?

- A. "You should not think that way right now."
- B. "Many people feel regret near the end of life."
- C. "Tell me more about what feels pointless to you."
- D. "Your family probably does not see your life that way."

Correct Answer: C

Choice C is best because it invites the client to express meaning-related distress in more depth, which is central to spiritual assessment. Choice A shuts down discussion, choice B labels the feeling without exploring it, and choice D redirects to others' opinions rather than the client's experience.

2057. A client says, "I want to perform a cleansing ritual with water before meals." The nurse knows the client has a fluid restriction and a central venous catheter dressing on the arm. Which action is best?

- A. Tell the client the ritual is not possible during hospitalization
- B. Allow unrestricted water use because spiritual needs take priority
- C. Work with the client to find a safe way to perform the ritual without compromising treatment
- D. Ask the family to persuade the client to skip the ritual

Correct Answer: C

Choice C is correct because the nurse should try to preserve the spiritual practice while adapting it to safety needs, such as using a limited amount of water and protecting the dressing. Choice A dismisses the need, choice B ignores safety, and choice D bypasses the client.

2058. A client recovering from a stroke has trouble speaking but becomes visibly upset when a meal tray arrives. The family says the client follows dietary practices for religious reasons. What should the nurse do first?

- A. Remove the tray and assess the client's specific dietary preference or restriction with the family and available records
- B. Encourage the client to eat because nutrition is the current priority
- C. Ask dietary services to send any vegetarian tray
- D. Wait until speech improves so the client can explain personally

Correct Answer: A

Choice A is best because the immediate need is to prevent serving food that may conflict with the client's values and to clarify the specific restriction using available sources. Choice B ignores the concern, choice C assumes the needed diet, and choice D delays action when information may already be obtainable.

2059. A client says, "I am not sure what I believe anymore, but I want someone to listen without judging me." Which nursing response is best?

- A. "You should decide what you believe before we talk about it."
- B. "I can stay with you for a few minutes and listen."
- C. "Questions like that are better saved for a clergy member."
- D. "Most people return to their beliefs once they feel better physically."

Correct Answer: B

Choice B is correct because presence and nonjudgmental listening are important nursing interventions for spiritual distress. The nurse does not need to solve the issue to provide support. Choice A is dismissive, choice C unnecessarily limits the nurse's role, and choice D makes a broad assumption.

2060. A client who recently learned they will need long-term dialysis says to the practical nurse, "I keep wondering why this is happening to me. Maybe I'm being punished." Which response is best?

- A. "You should focus on the treatment plan and not dwell on those thoughts."
- B. "Many people feel this way after bad news. Tell me more about what this means to you."
- C. "You are not being punished, and you should not think that way."
- D. "I will ask your family to explain your faith's beliefs about illness."

Correct Answer: B

Choice B is best because it acknowledges the client's distress and invites further discussion without judging, correcting, or imposing beliefs. This therapeutic response helps assess spiritual distress and supports coping. Choice A dismisses the concern, choice C argues with the client's belief, and choice D shifts the discussion away from the client and assumes the family should interpret the client's spirituality.

2061. A hospitalized client tells the practical nurse, "My pastor is coming tonight to pray with me, but I'm scheduled for imaging at the same time." The imaging is important but not an emergency. What is the most appropriate nursing action?

- A. Tell the client that tests take priority and the visit will need to be canceled.
- B. Ask the client whether they want assistance contacting the department to see if the test time can be adjusted.
- C. Reschedule the imaging study independently and inform the client afterward.
- D. Advise the pastor to keep the visit brief in the hallway before transport arrives.

Correct Answer: B

Choice B is best because it respects the client's spiritual preference, involves the client in decisions, and seeks a practical solution when the procedure is not emergent. Choice A is too dismissive. Choice C bypasses the client and the nurse's role by changing plans independently. Choice D offers a workaround without first exploring whether the client's preference can be better accommodated.

2062. A client with terminal cancer tells the practical nurse, "Please don't tell my daughter, but I think I am dying and I want to speak with my spiritual leader before anyone else comes in." The daughter arrives moments later and asks, "What did my mother say?" Which action should the nurse take?

- A. Tell the daughter that the client requested a visit from a spiritual leader because families deserve to know.
- B. Encourage the daughter to stay and discuss the client's fears openly at the bedside.
- C. Tell the daughter that the nurse will check on the client's immediate wishes and return shortly.
- D. Explain to the daughter that the client is probably just anxious and may change her mind later.

Correct Answer: C

Choice C is best because it protects client confidentiality while honoring the client's stated spiritual request and immediate preference about who should visit first. The nurse should not disclose the client's private statement without permission. Choice A violates confidentiality. Choice B ignores the client's request for privacy. Choice D is dismissive and minimizes the client's concern.

2063. During admission, a client says, "I don't belong to an organized religion, but reading certain poems helps me feel centered when I'm scared." Which nursing documentation best reflects this statement?

- A.** Client denies spiritual needs.
- B.** Client requests psychiatric evaluation for anxiety.
- C.** Client reports poetry reading as a source of comfort and meaning during stress.
- D.** Client prefers recreational activities instead of treatment teaching.

Correct Answer: C

Choice C is best because it accurately documents the client's own source of spiritual comfort without forcing it into a religious category. Spiritual considerations include meaning, comfort, and coping, not only formal religion. Choice A is incorrect because the client did identify a spiritual support. Choice B adds information not provided. Choice D mislabels the statement and misses its coping and meaning-centered importance.

Cultural considerations

2064. A nurse is caring for a client who speaks limited English and is scheduled to sign consent for a procedure later today. The client's adult daughter offers to translate. Which action by the nurse is most appropriate?

- A.** Use the daughter as the interpreter because she knows the client's wishes
- B.** Ask a trained medical interpreter to assist with communication
- C.** Speak loudly and slowly in English so the client can understand better

D. Delay all communication with the client until the provider arrives

Correct Answer: B

A trained medical interpreter is the safest and most appropriate choice to support accurate communication and informed decision-making. Family members may omit, add, or filter information, even when trying to help. Speaking louder does not overcome a language barrier, and delaying communication is unnecessary.

2065. A client from a culture different from the nurse's own refuses eye contact during conversation. Which interpretation by the nurse is best?

- A.** The client is being evasive and may be hiding information
- B.** The client is showing signs of confusion
- C.** The behavior may reflect a cultural norm rather than disrespect
- D.** The client is indicating hearing impairment

Correct Answer: C

In some cultures, avoiding direct eye contact is a sign of respect, not dishonesty or confusion. The nurse should avoid judging behavior through personal cultural expectations. Assuming deception is a common trap based on the nurse's own norms.

2066. During admission, a nurse says to a client, "Are there any cultural or spiritual practices we should know about to support your care?" This question best demonstrates which principle?

- A.** Using the client as the sole source of treatment planning
- B.** Avoiding culturally based assumptions
- C.** Encouraging the client to refuse standard care
- D.** Limiting care to nonmedical concerns

Correct Answer: B

Asking the client about personal practices avoids stereotyping and helps individualize care. Not all members of a cultural group share the same beliefs. The question does not replace clinical planning, promote refusal of care, or minimize medical needs.

2067. A client states, "In my family, serious diagnoses are usually discussed with the eldest son first." What is the nurse's best response?

- A. That is not allowed because all information must go only to the client
- B. I will document that your family prefers to make all decisions for you
- C. Tell me how you would like information shared and who you want included
- D. The provider will decide how much your family needs to know

Correct Answer: C

The nurse should clarify the client's own preferences for information sharing and involvement in decisions. This respects cultural values while still centering the client's wishes. A common trap is assuming either strict individual decision-making or total family control without asking the client.

2068. A nurse is preparing teaching for a client newly diagnosed with diabetes. The client nods throughout the session but later cannot describe the insulin schedule. Which action should the nurse take next?

- A. Assume the client understood but became anxious afterward
- B. Repeat the same instructions more quickly to save time
- C. Use teach-back and assess for language or cultural communication barriers
- D. Ask a family member to manage all future teaching

Correct Answer: C

Teach-back helps verify understanding rather than relying on nodding, which may reflect politeness, respect, or reluctance to challenge the nurse. Assessing for language and communication barriers supports culturally safe teaching. Repeating information without checking comprehension is less effective, and turning teaching over to family may exclude the client.

2069. A postoperative client grimaces and guards the incision but says, "I am fine," when asked about pain. The nurse knows that some clients may minimize pain expression for cultural reasons. What is the best nursing action?

- A. Accept the statement and document that the client denies pain
- B. Use additional pain assessment methods, including observation and follow-up questions
- C. Insist that the client rate the pain before any further care is given
- D. Withhold analgesics because the client did not request them

Correct Answer: B

Pain assessment should include verbal report, observed behavior, and culturally sensitive follow-up. Some clients underreport pain because of beliefs about stoicism, burdening others, or appropriate behavior. The trap is taking a brief denial at face value despite clear nonverbal cues.

2070. A client says, "I want to use a traditional herbal tea while I'm in the hospital." Which response by the nurse is best?

- A. Herbal remedies are unsafe, so you should stop them immediately
- B. You may use any traditional remedy as long as it is natural
- C. Tell me what it contains and how you use it so we can evaluate for safety
- D. Hospital staff cannot discuss nonprescription practices

Correct Answer: C

The nurse should assess the remedy without judgment and gather details about ingredients, dose, and use so potential interactions or risks can be evaluated. Automatically dismissing the practice can damage trust, while assuming all natural products are safe is incorrect.

2071. A nurse is caring for a client whose cultural practice includes same-gender preference for personal care. The unit is short-staffed, and only a staff member of a different gender is immediately available for a bed bath. What is the nurse's best initial action?

- A. Explain that cultural preferences cannot be considered during staffing shortages
- B. Ask the client whether the client wants to wait, modify the plan, or proceed now
- C. Proceed with care quickly to reduce embarrassment
- D. Cancel hygiene care for the rest of the shift

Correct Answer: B

The nurse should first discuss options with the client and involve the client in deciding how to proceed. This respects cultural preferences while addressing practical constraints. Ignoring the preference or deciding unilaterally is less patient-centered.

2072. Which statement by a nurse best reflects cultural humility rather than stereotyping?

- A. I know what foods clients from your culture usually prefer
- B. Most people from your background do not want pain medicine
- C. Help me understand what is important to you during your care
- D. Clients from your community usually let family make decisions

Correct Answer: C

Cultural humility means approaching each client as an individual and being willing to learn from the client's perspective. The other choices make broad assumptions about a cultural group, which can lead to stereotyping and unsafe care.

2073. A client who recently immigrated becomes quiet when discussing mental health symptoms and says, "People will think badly of my family." Which nursing response is most appropriate?

- A. You should not worry about what your family thinks
- B. Mental illness is treated the same way in every culture
- C. It sounds like stigma is a concern; tell me more about what this means to you

D. If you do not answer, the team cannot help you

Correct Answer: C

This response acknowledges the client's concern without judgment and invites further discussion. It helps the nurse assess the client's beliefs, fears, and support needs. Dismissing the family concern or pressuring the client can shut down communication.

2074. A nurse is caring for an older adult client whose family members answer every question before the client can speak. The client is alert and oriented. What is the nurse's best action?

- A. Direct all questions to the family because they are more comfortable speaking
- B. Privately assess the client's preference for family involvement while continuing to address the client directly
- C. Tell the family to leave immediately because they are interfering
- D. Assume the client has given up decision-making to the family

Correct Answer: B

The nurse should preserve the client's voice while respectfully assessing how much family involvement the client wants. Some families communicate collectively, but the nurse should not assume the client wants to be silent or excluded. Abruptly dismissing the family can be disrespectful and escalate conflict.

2075. A client declines a prescribed blood product because of deeply held beliefs. Which action by the nurse is most appropriate?

- A. Tell the client that refusing treatment is irrational and dangerous
- B. Document the refusal, explore the client's understanding, and notify the provider
- C. Ask the family to persuade the client to accept the treatment
- D. Administer the blood product because preserving life takes priority

Correct Answer: B

The nurse should assess understanding, respect the client's expressed beliefs and choices, document the refusal, and communicate with the provider. The trap is overriding the client's values because the treatment seems beneficial. Coercion and unauthorized treatment are inappropriate.

2076. A nurse is assigned to a client who avoids touch during conversation and steps back when the nurse moves closer. Which action is best?

- A.** Continue using light touch to show warmth and concern
- B.** Recognize personal space preferences and ask permission before touching
- C.** Stand closer so the client becomes more comfortable over time
- D.** Document the behavior as a sign of paranoia

Correct Answer: B

Comfort with touch and personal space varies widely. The nurse should adapt by maintaining appropriate distance and asking permission before physical contact. Assuming the client is paranoid or trying to force familiarity reflects cultural bias rather than assessment.

2077. A nurse notices that a client repeatedly misses meals during daylight hours for a religious observance. The client is also prescribed medication that should be taken with food. What is the nurse's priority action?

- A.** Tell the client that medical treatment is more important than religious practice
- B.** Document noncompliance and continue the current schedule
- C.** Assess the client's practices and collaborate on a safe plan for meals and medication timing
- D.** Ask dietary services to send larger meals the next day

Correct Answer: C

The priority is to assess the client's observance and work with the client and team to create a safe plan that respects beliefs while reducing medication-related risk. Labeling the client as noncompliant too quickly is a common trap that ignores the cultural context.

2078. A nurse is caring for a client with terminal illness whose family asks that the diagnosis not be discussed directly with the client because, in their culture, such information is considered harmful. What is the nurse's best response?

- A.** Agree not to speak with the client about the condition under any circumstance
- B.** Inform the family that their cultural beliefs cannot be considered in health care
- C.** Assess the client's preference for receiving health information before deciding how to proceed
- D.** Provide all updates to the family only, since they requested it first

Correct Answer: C

The nurse should avoid assuming either full disclosure directly to the client or complete nondisclosure to the client without first learning the client's own preferences. This approach respects cultural context while centering the client's wishes. The difficult part is balancing family values with patient-centered communication.

2079. A nurse is admitting a client who recently moved from another country. The client understands some English but struggles with medical terms. Which nursing action best supports culturally safe communication during the admission interview?

- A.** Speak loudly and slowly so the client can understand the questions
- B.** Use simple language and verify understanding by asking the client to explain information in their own words
- C.** Direct all questions to the family member who came with the client
- D.** Avoid asking about confusion so the client does not feel embarrassed

Correct Answer: B

Using plain language and checking understanding with the client's own explanation helps the nurse assess true comprehension without making assumptions. Speaking louder does not improve language comprehension, directing all questions to family can reduce client autonomy, and avoiding clarification increases the risk of misunderstanding.

2080. A client tells the nurse, "Before making health decisions, I want to talk with my family." Which response by the nurse is best?

- A.** You need to make your own decisions without family involvement
- B.** Family involvement usually delays care, so it is better to decide now
- C.** Tell me who you would like involved so we can support your decision-making process
- D.** Hospital policy requires only the client to discuss treatment options

Correct Answer: C

This response respects the client's preferences and invites clarification about who should be included. The other options dismiss the client's cultural values and assume that individual decision-making is the only acceptable approach. The nurse should support client-centered care while preserving the client's role in decisions.

2081. A nurse is preparing to teach a client about wound care at home. The client smiles, says little, and agrees with everything the nurse says. Which action is most appropriate to help the nurse avoid making cultural assumptions about the client's understanding?

- A.** Assume the client understands because the client is cooperative
- B.** Repeat the same instructions using louder speech
- C.** Ask the client to demonstrate or describe how the dressing change will be done at home
- D.** Shorten the teaching session so the client does not feel uncomfortable

Correct Answer: C

A return demonstration or teach-back confirms actual understanding. Smiling and agreeing may reflect politeness, respect, or discomfort rather than comprehension. Louder speech does not solve the problem, and shortening teaching can leave important gaps.

2082. A client says, "I do not eat pork for religious reasons." Which action by the nurse is best?

- A.** Tell the client that hospital diets cannot be changed

B. Document the preference and collaborate to request meals that meet the client's dietary practice

C. Advise the client to eat the meal anyway to maintain strength

D. Ask the family to bring all food from home instead of using the hospital menu

Correct Answer: B

The nurse should respect the client's dietary practice, document it, and work with the team to provide appropriate food choices. Telling the client to ignore beliefs is not client-centered, and requiring all food from home is unnecessary when accommodations may be available.

2083. A nurse is assessing pain in a client from a cultural background unfamiliar to the nurse. Which approach is best?

- A.** Assume the client's pain tolerance is higher than average
- B.** Use the same pain scale for every client and ignore cultural factors
- C.** Ask the client how pain is usually expressed or managed in their family and assess current pain using standard measures
- D.** Rely mainly on the family's description of the client's pain

Correct Answer: C

This option combines individualized assessment with standardized pain evaluation. It avoids stereotyping while exploring how the client understands and expresses pain. Assuming a high pain tolerance is a stereotype, ignoring cultural factors can miss key information, and family input does not replace the client's own report when the client can communicate.

2084. A client is scheduled for discharge teaching about a new medication regimen. The client speaks English well in casual conversation but appears confused when medication instructions are discussed. Which action by the nurse is best?

- A.** Continue the teaching because the client is conversational in English
- B.** Ask the client whether instructions would be easier in another language and arrange appropriate teaching support if needed
- C.** Give the written instructions only, because medication names are difficult in any language

D. Ask another client from the same cultural background to help explain the medications

Correct Answer: B

Conversational fluency does not guarantee understanding of medical information. The best action is to assess language preference for health teaching and arrange appropriate support. Written instructions alone may not be enough, and using another client is inappropriate and unsafe.

2085. During assessment, a client says, "I think this illness happened because my body is out of balance." What is the nurse's best response?

- A.** That belief is incorrect, and I will explain the real cause
- B.** Many people have those ideas, but they are not medically useful
- C.** Can you tell me more about what that means to you and what helps restore balance?
- D.** You should avoid discussing those beliefs so treatment can stay scientific

Correct Answer: C

The nurse should explore the client's health beliefs in a respectful, nonjudgmental way. This helps identify practices, concerns, and ways to plan care that the client can accept. The other choices dismiss the client's perspective and can damage trust.

2086. A nurse notices that a client becomes visibly uncomfortable when asked direct questions about personal matters in front of several relatives. What is the best nursing action?

- A.** Continue the interview because family presence improves accuracy
- B.** Ask the client privately whether they would prefer part of the discussion to occur without visitors present
- C.** Tell the relatives to leave immediately without speaking to the client
- D.** Skip all sensitive questions until after discharge

Correct Answer: B

The best action is to protect privacy while still respecting the client's preferences about family involvement. Asking privately preserves autonomy and avoids assumptions. Automatically continuing, ordering family out without consulting the client, or omitting important assessment questions are less appropriate.

2087. A client tells the nurse, "In my culture, we often use folk remedies first." Which nursing action is most appropriate?

- A.** Tell the client to stop all nonprescribed practices immediately
- B.** Ask which remedies the client uses and document them so potential benefits or risks can be evaluated
- C.** Encourage the client to use only remedies recommended by family members
- D.** Avoid asking for details because discussing home remedies may seem disrespectful

Correct Answer: B

The nurse should assess the specific remedies being used. This allows the team to identify possible interactions, safety concerns, and ways to incorporate safe practices when possible. Telling the client to stop everything without assessment is dismissive, and avoiding the topic misses important information.

2088. A nurse is caring for a client whose family remains at the bedside in large numbers and speaks frequently during care. The nurse feels the room is crowded and distracting. Which action is best?

- A.** Assume the family is interfering and limit all visitors immediately
- B.** Ignore the situation because family behavior should never be addressed
- C.** Assess the client's wishes, explain care needs, and set respectful limits only if necessary for safety or care delivery
- D.** Ask security to remove the visitors before the next assessment

Correct Answer: C

The nurse should begin with the client's preferences and then balance those wishes with unit safety and care needs. This approach avoids stereotyping and supports culturally responsive care. Immediate restriction or removal without assessment is too abrupt, while ignoring a true care barrier is also inappropriate.

2089. A nurse is caring for a client who recently gave birth. Several family members insist on performing a cultural newborn ritual at the bedside. The newborn is stable, but the ritual would interfere with a scheduled medication administration for the client that is time-sensitive. Which action is best?

- A.** Cancel the ritual because medical care always has absolute priority over cultural practices
- B.** Delay the medication and document that the family requested it
- C.** Acknowledge the ritual's importance, explain the time-sensitive medication need, and work with the family to adjust timing safely
- D.** Allow the ritual to proceed and ask the next shift to handle the medication issue

Correct Answer: C

The nurse should respect the cultural practice while also protecting safe care. Explaining the clinical need and negotiating a safe plan supports both priorities. Automatically canceling the ritual is unnecessarily rigid, and delaying a time-sensitive medication without a safe plan is inappropriate.

2090. A nurse prepares discharge instructions for a client and the client's spouse. The spouse answers most questions and says, "I always manage these things." The client remains quiet but alert. Which action is best for culturally sensitive and client-centered care?

- A.** Direct all teaching to the spouse because the spouse appears more confident
- B.** Exclude the spouse from teaching so the client can speak independently
- C.** Address the client directly, assess the client's preference for involvement of the spouse, and verify the client's understanding of the plan
- D.** Postpone discharge teaching until a social worker is available

Correct Answer: C

This option protects the client's autonomy while respecting the possible family role in managing care. The nurse should not assume either total independence or total delegation. Teaching only the spouse can sideline the client, and excluding the spouse without asking may conflict with the client's wishes.

2091. A client with newly diagnosed tuberculosis says, "If people from my community find out, they will avoid my family." The client then refuses teaching and becomes withdrawn. Which nursing action is the best first response?

- A.** Emphasize that public health rules are more important than the client's feelings
- B.** Explore the client's concerns about stigma and privacy before continuing with teaching
- C.** Tell the client that the fear is irrational and should be ignored
- D.** End the conversation because the client is not ready to learn

Correct Answer: B

The client's concern about stigma is a major barrier to care and learning. The best first response is to explore that concern in a respectful way, which may build trust and improve willingness to participate in treatment. Dismissing feelings or stopping the interaction does not address the barrier.

2092. A nurse is planning care for a client whose cultural beliefs strongly shape daily routines, food choices, and use of healing practices. Which approach best demonstrates cultural humility?

- A.** Use a standard list of traits associated with the client's cultural group to predict needs
- B.** Ask the client which beliefs and practices are important to this hospitalization and avoid assuming all members of the group are the same
- C.** Rely on the nurse's past experience with similar clients to anticipate preferences
- D.** Focus only on the medical diagnosis because culture is secondary in acute care

Correct Answer: B

Cultural humility means approaching each client as an individual, remaining open, and avoiding stereotypes. Group-based assumptions can lead to errors, even when they seem well-intended. The client's own explanation of meaningful practices is the most reliable guide.

2093. A nurse is caring for a hospitalized client who avoids eye contact during conversations and often looks down when the nurse speaks. The nurse is concerned that the client may be depressed or hiding information. Which action is best?

- A.** Document that the client is uncooperative and notify the provider
- B.** Ask open-ended questions about the client's preferences for communication and continue the assessment without making assumptions
- C.** Tell the client that making eye contact is important for honest communication
- D.** Request that a family member answer questions for the client

Correct Answer: B

Avoiding eye contact can have different meanings across cultures and does not automatically indicate depression, dishonesty, or lack of cooperation. The best action is to avoid assumptions, explore the client's communication preferences, and continue a respectful assessment. Choice A is premature and labels the client negatively. Choice C imposes the nurse's own cultural standard. Choice D reduces direct assessment of the client and should not be the first step unless the client requests help or communication barriers require it.

2094. A client is scheduled for a minor bedside procedure. When the nurse explains the plan, the client says, "I do not want the procedure done until my elder brother arrives." The consent form has not yet been signed, and the procedure is not urgent. Which action by the nurse is best?

- A.** Tell the client that only the client can make healthcare decisions, so family involvement is unnecessary
- B.** Ask the provider to proceed because the procedure is minor and delay could disrupt the unit schedule

C. Explore the client's wishes, support delaying the procedure if appropriate, and notify the provider of the client's request

D. Have the brother sign the consent form when he arrives so the procedure can begin quickly

Correct Answer: C

The best action is to respect the client's stated decision-making preference, clarify the request, and communicate it to the provider since the procedure is not urgent. This supports culturally sensitive care while still protecting the client's autonomy. Choice A dismisses the client's values. Choice B prioritizes convenience over client-centered care. Choice D is inappropriate because the client's desire for family input does not automatically transfer legal consent to the brother.

2095. A nurse is collecting a health history from a client who says, "I have been seeing a traditional healer and drinking an herbal tea every day." What is the nurse's best response?

- A.** You should stop all traditional practices while you are receiving medical treatment here
- B.** What does the tea contain, how often do you use it, and would you be willing to discuss it with the healthcare team?
- C.** Herbal treatments are usually harmless, so there is no need to mention them to the provider
- D.** Why didn't you tell the provider earlier that you were using unapproved remedies?

Correct Answer: B

The nurse should assess the specifics of the herbal practice in a respectful, nonjudgmental way and invite collaboration with the healthcare team. This approach supports cultural considerations and also promotes safety because some herbal products may affect treatment. Choice A is dismissive. Choice C makes an unsafe assumption. Choice D is judgmental and can damage trust.

2096. A client who recently immigrated is hospitalized with pneumonia. During each medication pass, the client nods and says "yes" but later does not follow the treatment plan. The nurse suspects the client may be agreeing out of politeness rather than understanding. Which action is best?

- A.** Ask the client to repeat the instructions in the client's own words and demonstrate key steps if needed
- B.** Speak more loudly and repeat the same instructions several times
- C.** Assume the client understands because the client keeps agreeing with the nurse
- D.** Direct all teaching to the family to avoid embarrassing the client

Correct Answer: A

Teach-back is the best way to confirm understanding without relying on polite agreement or assumptions. It helps the nurse identify gaps and adapt teaching respectfully. Choice B addresses volume, not comprehension. Choice C risks unsafe care. Choice D may exclude the client and undermine client-centered communication unless the client specifically prefers family-led teaching.

2097. A nurse enters a client's room to perform morning care and notices a small pouch tied around the client's neck. The client says, "This protects me spiritually, and I need to keep it on." The pouch does not interfere with breathing, circulation, or treatment. What should the nurse do first?

- A.** Remove the pouch and store it in a locked drawer so it will not be lost
- B.** Tell the client that personal objects are not allowed during hospitalization
- C.** Assess whether the pouch creates any safety concern and, if not, support the client's wish to keep it in place
- D.** Ask the family to take the pouch home because spiritual practices should not be part of medical care

Correct Answer: C

The nurse should first determine whether the item poses a safety or treatment problem. If it does not, supporting the client's cultural or spiritual practice is appropriate. Choice A removes the item without need. Choice B applies an unnecessary blanket rule. Choice D dismisses the client's beliefs and is not client-centered.

2098. A client is scheduled for surgery later today. The client speaks limited English. The client's adult child says, "I can translate everything. We don't need anyone else." The client appears anxious and gives short answers. Which action by the nurse is best?

- A.** Use the adult child as the interpreter because family members know the client's culture best
- B.** Proceed with teaching and consent preparation using simple words and gestures only
- C.** Request a qualified medical interpreter for the discussion and continue to include the family as the client wishes
- D.** Delay all communication until the surgeon arrives to explain the procedure

Correct Answer: C

A qualified medical interpreter is the best choice for important discussions such as teaching and consent preparation because accuracy is essential. Family may still be included if the client wants that support, but they should not replace a qualified interpreter for critical communication. Choice A risks errors and omission. Choice B is unsafe for complex information. Choice D delays needed nursing communication and does not solve the language barrier.

Gender-identity considerations

2099. A nurse is admitting a client who says, "I'm transgender, and I go by Maya. My pronouns are she and her." Which action by the nurse is most appropriate?

- A.** Document the client by the legal name only and avoid using pronouns
- B.** Use the client's stated name and pronouns during care and communication
- C.** Ask the client to explain the reasons for the gender transition

D. Tell the client staff will decide what is easiest to use in the chart

Correct Answer: B

Using the client's stated name and pronouns shows respect, supports therapeutic communication, and helps create a safe care environment. Choice A ignores the client's expressed identity. Choice C is intrusive and not needed for routine respectful care. Choice D is not client-centered.

2100. A client appears upset when a staff member repeatedly refers to them as "she" after the client introduced themselves with they/them pronouns. What is the nurse's best response?

A. "The staff member probably did not mean anything by it."

B. "You should try not to focus on small mistakes."

C. "I'm sorry that happened. I will use your correct pronouns and address this respectfully with the team."

D. "Would you prefer that no one use pronouns for you at all?"

Correct Answer: C

The best response acknowledges the client's feelings, uses therapeutic communication, and includes a concrete plan to support respectful care. Choice A minimizes the client's distress. Choice B dismisses the concern. Choice D may be appropriate only if the client requests it, but it avoids addressing the actual problem.

2101. A nurse is collecting a health history from a client whose appearance does not match the sex listed in the chart. Which question is most appropriate?

A. "What were you born as?"

B. "Are you planning to fully transition?"

C. "How would you like to be addressed, and what pronouns do you use?"

D. "Why does your chart say something different?"

Correct Answer: C

This question is respectful, clinically useful, and avoids assumptions. Choice A is stigmatizing. Choice B is unnecessarily invasive and not needed unless directly relevant to care. Choice D can sound confrontational and centers the chart rather than the client.

2102. A nursing assistant says about a transgender client, "I'm confused, so I'm just going to call the client by the room number." What should the nurse do first?

A. Tell the assistant to avoid speaking to the client unless necessary

B. Explain that the client should be addressed by the name the client requests

C. Ignore the comment because the assistant is trying to avoid offense

D. Ask another staff member to take over all care for that client

Correct Answer: B

The nurse should correct the issue promptly and reinforce respectful, client-centered communication. Using a room number instead of a requested name is depersonalizing. Ignoring the comment allows inappropriate practice to continue. Reassigning care without addressing the behavior does not solve the problem.

2103. A client says, "I avoid health care because people judge me for being transgender." Which nursing response is best?

A. "You should report every person who has ever offended you."

B. "Most health care workers are professional, so try not to worry."

C. "That sounds very difficult. What concerns do you have about receiving care today?"

D. "If you expect judgment, you may come across as defensive."

Correct Answer: C

This response validates the client's experience and invites further discussion of current concerns, which supports trust and assessment. Choice B minimizes the client's experience. Choice D is blaming. Choice A jumps to action before understanding the client's immediate needs.

2104. A nurse is preparing a semi-private room for a client who is transgender. Another client asks, "Why is that person in this room?" Which response by the nurse is most appropriate?

- A. "That client is transgender and needs your understanding."
- B. "Room assignments are based on care needs and privacy is maintained for all clients."
- C. "If you are uncomfortable, I can tell you more about the other client."
- D. "I agree this situation is unusual, but please be polite."

Correct Answer: B

The nurse should protect confidentiality and give a neutral, professional response. Choice B addresses the concern without disclosing private information. Choice A improperly reveals the client's personal information. Choice C also breaches privacy. Choice D is biased and unprofessional.

2105. During assessment, a client says, "My family refuses to use my name. I feel isolated." Which nursing action is the priority?

- A. Assess the client for depression and thoughts of self-harm
- B. Advise the client to stop contact with the family immediately
- C. Encourage the client to ignore the family's comments
- D. Tell the client family members often need time to adjust

Correct Answer: A

Isolation and rejection can increase emotional distress, so the priority is to assess the client's safety and mental status, including depression and self-harm risk. Choices B, C, and D may be premature, minimizing, or overly directive before a full assessment.

2106. A transgender client is scheduled for a procedure and becomes quiet when staff use the legal name from the identification band. Which nursing action is best?

- A. Explain that legal names must always be used in every spoken interaction
- B. Ask the client what name should be used in conversation while maintaining required identification processes

- C. Remove the identification band so the client is more comfortable
- D. Delay the procedure until the chart can be legally changed

Correct Answer: B

The nurse should balance safe identification procedures with respectful communication. Using the client's affirmed name in conversation, when possible, supports dignity while required identifiers are still used appropriately. Choice A is unnecessarily rigid. Choice C is unsafe. Choice D is unrealistic and delays care without need.

2107. A nurse is teaching a newly hired assistive personnel worker about caring for clients with diverse gender identities. Which statement by the worker indicates a need for further teaching?

- A. "I should ask the client how they want to be addressed."
- B. "If I make a mistake with pronouns, I should correct myself and move on respectfully."
- C. "It is best to avoid asking any questions at all about the client's preferences."
- D. "I should not share information about a client's gender identity with others who do not need to know."

Correct Answer: C

Clients' preferences for names and pronouns should be asked respectfully rather than avoided. Choice C shows a misunderstanding because avoiding the topic entirely can lead to assumptions and disrespectful communication. The other statements reflect appropriate practice.

2108. A hospitalized adolescent tells the nurse privately, "I do not feel safe when my parent mocks my gender identity." Which action should the nurse take first?

- A. Encourage the adolescent to confront the parent directly
- B. Assess the immediate safety risk and notify the appropriate charge nurse or provider per facility policy
- C. Tell the parent that their behavior is unacceptable in front of the adolescent
- D. Document the statement after discharge to avoid conflict during hospitalization

Correct Answer: B

A statement about not feeling safe requires immediate assessment of risk and escalation according to policy. Choice B addresses safety first. Choice A may increase risk. Choice C could be appropriate later but not before assessing and planning for safety. Choice D is unsafe and delays needed action.

2109. A client who is nonbinary is admitted for treatment of pneumonia. During report, one nurse says, "Their gender identity is the main issue here." Which response by the receiving nurse is best?

- A. "Let's focus on information relevant to the client's current care needs and preferences."
- B. "I agree, because gender identity affects every clinical decision."
- C. "We should discuss the client's gender identity in detail with the whole unit."
- D. "It is not appropriate to document anything about the client's preferences."

Correct Answer: A

The best response redirects report to clinically relevant, respectful information, such as communication preferences and current care needs. Choice B overgeneralizes. Choice C promotes unnecessary disclosure. Choice D is incorrect because relevant preferences may need to be documented to support safe, respectful care.

2110. A client says, "Everyone keeps asking personal questions about my body. I came here for severe abdominal pain." Which nursing response is most appropriate?

- A. "They are probably just curious, so try to be patient."
- B. "You can refuse all assessment questions if you want."
- C. "Only information needed for your care should be asked. Tell me what concerns you most right now."
- D. "Staff need complete personal details before treating any client."

Correct Answer: C

This response validates the concern, reinforces appropriate professional boundaries, and refocuses the interaction on the client's immediate health need. Choice A minimizes the client's discomfort. Choice B is too broad because some assessment questions are necessary. Choice D justifies intrusive behavior.

2111. A client becomes tearful after hearing laughter from the hallway and says, "I know they're laughing at me because I'm trans." What is the nurse's best initial action?

- A. Tell the client there is no proof the laughter was about them
- B. Assess the client's feelings and sense of emotional safety before making assumptions about the cause
- C. Immediately file a formal complaint against the staff on the client's behalf
- D. Advise the client to stay in the room to avoid further stress

Correct Answer: B

The priority is to assess the client's emotional state, support safety, and gather more information. Choice A is dismissive. Choice C may become necessary if misconduct is identified, but it is not the initial action. Choice D may increase isolation and does not address the client's distress.

2112. A nurse accidentally uses the wrong pronoun for a client and notices the client's expression change. Which response is best?

- A. Ignore it and continue so the interaction is not awkward
- B. Give a long explanation about being tired and distracted
- C. Briefly apologize, use the correct pronoun, and continue respectful care
- D. Ask the client not to be offended because the mistake was unintentional

Correct Answer: C

A brief apology followed by correct language is respectful and keeps the focus on the client. Choice A fails to address the error. Choice B shifts attention to the nurse. Choice D minimizes the client's reaction and is not therapeutic.

2113. A nurse is reviewing a chart before entering a client's room. The admission record lists the client's legal name as Daniel, but a note from triage states, "Preferred name: Dani; pronouns: he/him." What should the nurse do when greeting the client?

- A.** Address the client as Daniel until the legal record is changed
- B.** Ask the client in front of the roommate which name the staff should use
- C.** Greet the client as Dani and use he/him pronouns unless the client requests otherwise
- D.** Avoid using any name or pronouns to prevent making a mistake

Correct Answer: C

Using the name and pronouns the client has identified is respectful, supports therapeutic communication, and helps create a safe care environment. Choice A incorrectly prioritizes the legal name over the client's stated identity in routine interaction. Choice B risks unnecessary disclosure of personal information. Choice D avoids respectful communication rather than addressing it appropriately.

2114. A client admitted for dehydration says, "Please don't tell my roommate that I'm transgender." Which nursing action is most appropriate?

- A.** Explain to the roommate that the information is private and cannot be discussed
- B.** Document the client's request and avoid sharing the information unless it is needed for care
- C.** Move the client to another room immediately to prevent disclosure
- D.** Tell the client that all staff and nearby clients must be informed for safety

Correct Answer: B

The nurse should protect the client's privacy by documenting the request and limiting disclosure to information needed for care. Gender identity is personal health information and should not be shared casually. Choice A still reveals that there is something private to explain to the roommate. Choice C may not be necessary and does not address confidentiality directly. Choice D is inappropriate because nearby clients do not need this information.

2115. A nurse is completing an intake interview. The client states, "I identify as male." The electronic form only lists "male" and "female" under sex. Which action by the nurse is best?

- A.** Select an option at random and skip the topic to avoid offending the client
- B.** Ask the client which answer should be entered and document the client's stated gender identity elsewhere as appropriate
- C.** Leave the entire form blank because forms should never be completed if they are imperfect
- D.** Tell the client the form is outdated and that the information is not important

Correct Answer: B

The best action is to clarify what the form is asking, involve the client, and document the client's stated gender identity in the appropriate part of the record. This supports respectful, accurate care. Choice A risks inaccurate documentation. Choice C is impractical because needed admission data still must be entered. Choice D dismisses the client's identity and may interfere with care planning.

2116. A client says, "I don't want my parents in the room while I answer questions about my gender identity." Which response by the nurse is best?

- A.** You should answer with your parents present so everyone hears the same information
- B.** Let's continue the interview after asking your parents to step out for a few minutes
- C.** I'll ask your parents to answer those questions for you
- D.** We can skip those questions since they seem uncomfortable

Correct Answer: B

Offering private time supports therapeutic communication, privacy, and honest disclosure. Choice A ignores the client's request for privacy. Choice C removes the client's voice from the assessment. Choice D may omit relevant assessment data rather than addressing the need for a private, respectful setting.

2117. A client who identifies as nonbinary is on a medical-surgical unit. During bedside handoff, the off-going nurse says, "This client uses they/them pronouns." What is the best rationale for including this information in report?

- A.** It helps staff communicate respectfully and consistently with the client
- B.** It warns staff that the client may refuse treatment
- C.** It is required because gender identity is always the client's main clinical problem
- D.** It allows staff to explain the client's identity to visitors

Correct Answer: A

Including correct name and pronouns in report promotes respectful, consistent communication and can reduce client distress. Choice B is a stereotype. Choice C incorrectly treats gender identity as automatically the primary clinical issue. Choice D would violate privacy because visitors do not need that information unless the client chooses to share it.

2118. A client hospitalized after surgery says quietly, "I stopped coming to clinics because every visit turns into questions about my gender instead of my actual symptoms." Which response by the nurse is best?

- A.** You should expect questions like that because staff are usually just curious
- B.** Tell me more about that experience so we can focus on what you need for your recovery today
- C.** At least you are here now, so it probably is not a big issue anymore
- D.** Most staff do not mean any harm, so try not to take it personally

Correct Answer: B

This response acknowledges the client's experience, invites further discussion, and redirects care toward the client's current health needs. It is therapeutic and client-centered. Choices A, C, and D minimize the client's concern and can damage trust.

2119. A nurse enters a room to perform a physical assessment and is unsure what terms the client uses for body parts. Which question is most appropriate?

- A.** What were you called before you changed genders?
- B.** Which body terms would you like me to use during the exam?
- C.** Are you comfortable telling me everything about your transition first?
- D.** Should I just use standard terms even if they bother you?

Correct Answer: B

Asking which terms the client prefers is respectful, focused on care, and helps avoid causing distress during assessment. Choice A is intrusive and irrelevant. Choice C requests unnecessary personal history before addressing the immediate exam. Choice D frames respectful care as optional and dismisses the client's comfort.

2120. A client becomes visibly anxious when told a urine pregnancy test is ordered before an imaging study. The client says, "Why do I need that?" What is the nurse's best response?

- A.** Because anyone who looks young has to take one
- B.** It's ordered to protect clients who could be pregnant before certain tests; I'll explain the reason and answer your questions privately
- C.** The provider ordered it, so there is no need to discuss it
- D.** If you are uncomfortable, we can just document that you refused without more explanation

Correct Answer: B

The best response explains the clinical reason in a respectful, private manner and invites questions. It focuses on safety without making assumptions or being dismissive. Choice A is vague and based on appearance. Choice C is nontherapeutic. Choice D skips needed education and may lead to uninformed refusal.

2121. A nurse hears an unlicensed assistive personnel member say, "I know the chart says one thing, but I'm calling this client by what they obviously are." What should the nurse do first?

- A. Immediately correct the staff member and instruct them to use the client's stated name and pronouns
- B. Ignore the comment for now and discuss it during the next staff meeting
- C. Ask the client to be patient because the staff member is still learning
- D. Reassign the staff member without addressing the statement

Correct Answer: A

The nurse should address disrespectful care immediately to protect the client and set clear expectations for appropriate communication. Choice B delays action. Choice C puts the burden on the client. Choice D may remove the staff member from the situation temporarily but fails to correct the behavior.

2122. A nurse is preparing to ask sexual health questions during an admission assessment. Which opening statement is best?

- A. I ask every client these questions because they can affect health care, and you can tell me only what you are comfortable sharing
- B. Because of your gender identity, I need to ask more sexual questions than usual
- C. You probably get offended by these questions, but I have to ask them
- D. These questions are mostly for documentation, so the answers are not very important

Correct Answer: A

This statement normalizes the assessment, explains why it matters, and supports client autonomy. Choice B singles the client out unnecessarily. Choice C is defensive and assumes a negative reaction. Choice D undermines the purpose of the assessment.

2123. A client tells the nurse, "Being misgendered makes me panic. I start shaking and can't focus." Which nursing action is the priority?

- A. Tell the client to report any concerns after discharge

- B. Assess the client's current anxiety level and create a plan to support respectful communication by staff

- C. Advise the client to ignore comments that are probably accidental

- D. Document the statement only if the client files a formal complaint

Correct Answer: B

The priority is to assess the client's immediate psychological response and take steps to reduce further distress, such as promoting consistent respectful communication. Choice A delays care. Choice C minimizes the client's symptoms. Choice D treats the issue as administrative rather than a current psychosocial concern.

2124. A postoperative client who is transgender appears withdrawn and stops making eye contact after several staff members enter the room for a dressing change. Which action by the nurse is best?

- A. Continue the procedure quickly because efficiency reduces discomfort
- B. Ask unnecessary staff to leave and explain each step before proceeding
- C. Tell the client that all teaching hospitals have many observers
- D. Delay the dressing change until the next shift so the client can rest

Correct Answer: B

Reducing unnecessary exposure and explaining the procedure supports dignity, privacy, and trust. Choice A ignores signs of distress. Choice C normalizes the client's discomfort rather than addressing it. Choice D postpones needed care instead of modifying the environment appropriately.

2125. A nurse is caring for a client receiving treatment for self-inflicted lacerations. The client states, "People at school keep targeting me because of my gender identity. Sometimes I think everyone would be better off without me." Which action should the nurse take first?

- A. Encourage the client to focus on positive relationships
- B. Ask the client to describe what support is available at school
- C. Assess immediately for suicidal intent, plan, and access to means

D. Suggest waiting to discuss school issues until the wounds are dressed

Correct Answer: C

The statement suggests possible suicide risk, so immediate safety assessment is the priority. The nurse should determine intent, plan, and access to means before moving to broader coping or support interventions. Choices A and B may be appropriate later but do not address immediate risk. Choice D delays urgent assessment.

2126. A client who identifies as gender diverse is scheduled for catheter insertion and says, "Exams in that area are really upsetting for me." Which nursing response is best?

- A.** We can skip the procedure if it causes distress
- B.** I'll explain why the catheter is needed, ask what would help you feel safer, and maintain as much privacy as possible
- C.** Try not to think about it; it will only take a minute
- D.** This is routine, so your feelings should not interfere with treatment

Correct Answer: B

This response acknowledges distress, provides necessary education, and seeks ways to reduce trauma while preserving privacy and proceeding safely. Choice A may be inappropriate if the procedure is clinically indicated. Choices C and D are minimizing and nontherapeutic.

2127. A nurse receives report on a client with sepsis. The report includes the client's affirmed name, pronouns, and that the client becomes distressed when staff discuss anatomy in front of visitors. Which nursing plan best applies this information?

- A.** Use the affirmed name and pronouns, verify who may stay during care discussions, and limit sensitive questions to those necessary for treatment
- B.** Avoid discussing any body systems so the client does not become upset
- C.** Tell visitors to remain in the room so they can help the client answer questions accurately
- D.** Delay the psychosocial concerns until the sepsis has fully resolved

Correct Answer: A

This plan integrates respectful communication, privacy, and clinically necessary assessment. It protects dignity while still supporting urgent medical care. Choice B prevents needed assessment. Choice C violates privacy. Choice D falsely separates psychosocial needs from current nursing care.

2128. A client says, "I only want staff to know what is necessary for my treatment. I don't want to educate everyone about my identity." Which nursing action best reflects this request?

- A.** Place a detailed note in the chart describing the client's full gender history for all departments to review
- B.** Share the client's history during shift report so staff will understand the client better socially
- C.** Communicate only the information needed for safe, respectful care and avoid unnecessary personal discussion
- D.** Tell the client that once information is in the chart, all staff are free to discuss it openly

Correct Answer: C

The nurse should follow the principle of minimum necessary disclosure and keep communication focused on safe, respectful care. Choice A includes unnecessary detail. Choice B shares information for social interest rather than treatment need. Choice D is incorrect and disregards confidentiality.

2129. A nurse is admitting a client for treatment of pneumonia. The client says, "My legal name is still on my insurance card, but I go by Maya. Please use she/her." Which nursing action is best?

- A.** Explain that staff must use the legal name until the medical record is officially changed
- B.** Document the legal name where required, note the affirmed name and pronouns in the chart, and address the client as Maya using she/her
- C.** Use the legal name during care but the affirmed name only when speaking privately with the client
- D.** Avoid using any name or pronouns until the provider updates the admission record

Correct Answer: B

The best action is to meet legal documentation requirements while consistently using the client's affirmed name and pronouns in care. This supports therapeutic communication, dignity, and trust. Choice A and C incorrectly center the legal name in routine interactions, which can be distressing and unnecessary. Choice D delays respectful communication instead of addressing the client's need immediately.

2130. During medication rounds, a client who identifies as nonbinary looks upset and says, "That staff member called me by the wrong pronoun again. I don't even want to ask for anything now." Which response by the nurse is best?

- A. "Try not to take it personally. The staff is very busy today."
- B. "You should report that person after discharge if it keeps happening."
- C. "I'm sorry that happened. How would you like me to address you, and what can I do right now to help you feel more comfortable?"
- D. "I understand exactly how you feel, because everyone gets called the wrong thing sometimes."

Correct Answer: C

This response acknowledges the harm, confirms the client's preferences, and focuses on an immediate supportive action. It uses therapeutic communication and helps restore trust in care. Choice A minimizes the client's distress. Choice B postpones action and does not address the current concern. Choice D shifts focus away from the client and falsely equates a gender-related experience with a common minor mix-up.

2131. A nurse is preparing a client for a chest procedure that requires changing into a gown. The client says, "I wear a binder, and taking it off around other people makes me very uncomfortable." Which nursing action is most appropriate?

- A. Tell the client the binder must come off now and that discomfort is unavoidable in the hospital

- B. Ask the client what would help them feel safer, provide as much privacy as possible, and explain the procedure requirements before proceeding
- C. Skip the preprocedure clothing change to avoid upsetting the client
- D. Ask another staff member to explain why the binder is a problem so the client hears the same message twice

Correct Answer: B

The nurse should reduce distress by using a trauma-informed, respectful approach: assess the client's concerns, maximize privacy, and clearly explain what is necessary for safe care. Choice A is abrupt and nontherapeutic. Choice C may compromise the procedure if clothing or the binder interferes with care. Choice D does not address the client's emotional safety and may increase discomfort by involving more people than needed.

2132. A client in the emergency department states, "I'm transgender, and every time I come to a hospital people focus on that instead of why I'm here." The client has severe abdominal pain rated 8 out of 10. Which nursing approach is best?

- A. Begin a detailed discussion of the client's transition history before addressing the pain
- B. Avoid asking any questions related to sex assigned at birth or reproductive organs because that topic may be upsetting
- C. Acknowledge the concern, focus first on assessment and relief of the abdominal pain, and ask only clinically necessary questions in a respectful manner
- D. Reassure the client that staff members are trained and therefore bias will not affect care

Correct Answer: C

The nurse should address the urgent presenting problem first while using respectful, clinically relevant questioning only as needed. This balances psychosocial safety with appropriate assessment. Choice A repeats the exact problem the client identified and delays needed care. Choice B may omit essential data needed to evaluate abdominal pain. Choice D offers a broad reassurance without addressing the client's current mistrust or pain and may sound dismissive.

Sensory and perceptual problems

2133. An older adult with age-related hearing loss becomes frustrated during conversations and says, "Everyone mumbles." Which nursing action is best to improve communication?

- A. Speak loudly into the client's affected ear
- B. Face the client and speak in a normal, clear voice
- C. Repeat each sentence several times without changing the wording
- D. Use medical terms so the client can focus on key words

Correct Answer: B

Facing the client and speaking clearly at a normal pace helps the client use both hearing and visual cues such as lip movements and facial expressions. Shouting can distort sounds and make understanding harder. Repeating the same unclear wording may not help, and medical jargon can increase confusion.

2134. A client with newly decreased vision is admitted to the unit. Which intervention best promotes psychosocial adaptation and safety?

- A. Rearrange the room daily so the client learns to adapt quickly
- B. Approach quietly to avoid startling the client
- C. Orient the client to the room layout and keep frequently used items in consistent locations
- D. Ask family members to answer most questions for the client

Correct Answer: C

Clients with decreased vision benefit from orientation to the environment and consistency in item placement. This reduces anxiety, improves independence, and promotes safety. Rearranging the room increases risk. Approaching quietly may startle the client more; the nurse should identify self when entering. Having family answer for the client decreases autonomy.

2135. A hospitalized client who wears hearing aids says, "I don't want to put them in. The noises here make me feel overwhelmed." What is the nurse's best response?

- A. "You should wear them all day so you get used to it."
- B. "Let's talk about which sounds are bothering you and when you feel most comfortable using them."
- C. "Without them, communication will be impossible."
- D. "I'll let the provider know you are refusing treatment."

Correct Answer: B

This response uses therapeutic communication by exploring the client's experience and involving the client in problem-solving. Sensory overload can be distressing, so the nurse should assess triggers and support coping. The other responses are dismissive, exaggerate the problem, or frame the issue punitively rather than supportively.

2136. A client with cataracts says, "I hate asking for help. I feel useless." Which response by the nurse is most therapeutic?

- A. "You shouldn't feel that way because surgery may fix this."
- B. "Many people feel this way, so try not to worry about it."
- C. "It sounds like losing independence has been very upsetting for you."
- D. "Your family can do most things for you until this improves."

Correct Answer: C

The nurse should acknowledge the client's feelings and identify the underlying concern, which is loss of independence. This validates the experience and encourages further discussion. False reassurance, minimizing, and promoting unnecessary dependence are less therapeutic.

2137. A client with severe visual impairment begins feeling for the overbed table and says, "I can never find anything in this room." Which action should the nurse take first?

- A. Tell the client to call for assistance before touching anything
- B. Move the client's belongings to the bedside cabinet for safekeeping

- C.** Describe the location of nearby objects using consistent directional terms
- D.** Ask the provider for an occupational therapy referral

Correct Answer: C

The immediate need is to reduce frustration and improve orientation. Describing the environment with consistent directional terms such as "water is at 2 o'clock" supports independence and safety. The other options may be useful later, but they do not address the client's immediate difficulty as directly.

2138. A client with hearing loss nods yes to every question, but the nurse suspects the client is not understanding the teaching. Which action is best?

- A.** Ask the client to repeat the instructions in their own words
- B.** Speak faster so the teaching session is shorter
- C.** Provide the information only to the family caregiver
- D.** Switch to yes-or-no questions for easier responses

Correct Answer: A

Having the client restate information in their own words is a teach-back method that evaluates actual understanding. Nodding alone does not confirm comprehension. Faster speech and yes-or-no questions can hide misunderstanding, and teaching only the family bypasses the client.

2139. The nurse is caring for a client with macular degeneration who becomes withdrawn during group activities at a long-term care facility. Which intervention is most appropriate?

- A.** Encourage the client to remain alone until adjustment improves
- B.** Tell the client that vision loss is a normal part of aging
- C.** Assess how the vision changes are affecting participation and preferred activities
- D.** Ask staff to excuse the client from all social events

Correct Answer: C

Withdrawal may reflect grief, frustration, fear of embarrassment, or difficulty participating. The nurse should first assess the impact of the vision loss on social functioning and interests so interventions can be individualized. Isolation and minimizing statements worsen psychosocial adjustment.

2140. A client reports ringing in the ears and says, "It keeps me awake, and I feel like I'm losing control." Which nursing response is best?

- A.** "You need to ignore it so it bothers you less."
- B.** "Let's discuss what times it feels worse and what has helped you cope so far."
- C.** "This means your hearing will probably be gone soon."
- D.** "I'll ask that your room be moved near the nurses' station."

Correct Answer: B

The client is describing distress related to tinnitus, so the nurse should assess patterns, triggers, and current coping methods. This supports problem-solving and emotional support. Telling the client to ignore it is dismissive, predicting severe loss is unsupported and anxiety-producing, and a busier location may worsen distress.

2141. A postoperative client is legally blind and says, "Please don't keep changing nurses. I get nervous when I don't know who is here." Which nursing action best addresses this concern?

- A.** Limit all staff interactions to reduce stimulation
- B.** Arrange care so staff identify themselves each time they enter and explain what they are going to do
- C.** Encourage the client to rely on family instead of staff whenever possible
- D.** Avoid conversation unless giving medications or treatments

Correct Answer: B

For a client who cannot rely on visual cues, consistent self-identification and explanation of care reduce anxiety and build trust. The goal is not to reduce helpful contact but to make interactions predictable. The other choices increase isolation or dependence.

2142. A client with hearing loss becomes irritable during discharge teaching in a busy semi-private room. Which nursing action is most appropriate?

- A.** Continue the teaching because discharge information is urgent
- B.** Move to a quieter area and reduce background noise before continuing
- C.** Ask the roommate to repeat the instructions more loudly
- D.** Postpone all teaching until the follow-up appointment

Correct Answer: B

Background noise can significantly interfere with speech discrimination for clients with hearing loss. Moving to a quieter setting directly addresses the barrier and improves the chance of understanding. Continuing in the same environment, involving the roommate, or delaying all teaching are less appropriate.

2143. A client with progressive hearing loss says, "My family thinks I'm rude because I answer the wrong thing or don't answer at all." Which nursing goal is the priority?

- A.** The client will avoid family gatherings until hearing improves
- B.** The client will identify strategies to improve communication with family members
- C.** The family will make all decisions for the client during conversations
- D.** The client will stop wearing hearing aids during stressful situations

Correct Answer: B

The main psychosocial issue is strained communication and its effect on relationships. A priority goal is for the client to identify workable communication strategies, such as facing speakers, reducing background noise, and confirming understanding. The other options promote isolation, dependence, or reduced adaptation.

2144. A client with diabetic retinopathy says, "I know people are trying to help, but when they grab my arm without warning, I feel panicked." Which nursing instruction to staff is best?

- A.** Approach from behind so the client is not distracted
- B.** Give short commands first, then move the client quickly
- C.** Announce your presence, explain the action, and allow the client to take your arm when ambulating
- D.** Avoid touching the client unless an emergency occurs

Correct Answer: C

Explaining actions before touch and letting the client take the staff member's arm promotes control, trust, and safer mobility. Unexpected touching can increase anxiety and panic. Approaching from behind or moving the client quickly removes control and can be frightening.

2145. A client with Ménière-like symptoms reports vertigo, nausea, and fear of falling whenever the room seems to spin. Which nursing intervention best addresses both safety and psychosocial distress during an episode?

- A.** Encourage the client to walk so balance can improve through practice
- B.** Stay with the client, limit sudden position changes, and provide calm reassurance
- C.** Turn on bright overhead lights so the client can focus visually
- D.** Ask several staff members to assist at once to control the situation quickly

Correct Answer: B

During vertigo, the nurse should reduce movement, maintain safety, and provide calm support. Staying with the client also decreases fear. Ambulation during active vertigo increases fall risk. Bright light may worsen discomfort, and multiple staff acting at once can increase confusion and anxiety.

2146. A client with severe hearing loss is becoming socially isolated on the unit. The nurse notices the client often misinterprets comments from others and then withdraws. Which intervention is most likely to address the underlying problem?

- A. Provide opportunities for one-on-one communication in a well-lit, quiet setting
- B. Advise the client to avoid conversations that may be confusing
- C. Encourage the client to smile and nod during group discussions
- D. Tell peers to speak as loudly as possible when near the client

Correct Answer: A

Misinterpretation of speech can lead to embarrassment, frustration, and withdrawal. A quiet, well-lit setting supports accurate communication by reducing noise and allowing visual cues. Avoiding conversation increases isolation, and nodding or shouting does not solve the comprehension problem.

2147. A client with recent vision loss tells the nurse, "I don't want my daughter to see me like this. I'm not myself anymore." Which nurse response is best?

- A. "You need to let her help whether you want it or not."
- B. "At least the problem is only with your vision and not your memory."
- C. "You're grieving a major change, and we can talk about what feels hardest right now."
- D. "If you stay positive, your daughter will feel better too."

Correct Answer: C

Recent vision loss can affect identity, independence, and relationships. The best response acknowledges grief and invites exploration of the client's experience. The other options are controlling, minimizing, or superficially reassuring rather than therapeutic.

2148. A client with partial hearing loss says, "When several people talk at once, I just smile and pretend I understood." Which nursing suggestion is most likely to improve the client's ability to participate in conversations?

- A. Ask family members to speak more loudly from across the room

- B. Encourage the client to sit where the speaker's face is clearly visible

- C. Tell the client to avoid family gatherings until hearing improves

- D. Suggest answering quickly before the conversation changes

Correct Answer: B

Seeing the speaker's face helps many clients with hearing loss use visual cues such as lip movement and facial expression. Speaking loudly from across the room can distort sound, avoiding gatherings increases isolation, and answering quickly without understanding does not improve communication.

2149. During a home visit, the nurse notices that a client with impaired vision has placed colored tape on the edges of steps, the bathroom door, and the microwave controls. Which interpretation by the nurse is most appropriate?

- A. The client is demonstrating an adaptive strategy to increase independence
- B. The client is becoming obsessive about the home environment
- C. The client should be discouraged from making changes without approval
- D. The client is showing denial about the extent of vision loss

Correct Answer: A

Using visual contrast markers is a practical compensatory technique that can improve function and confidence. There is no evidence of obsession or denial, and discouraging appropriate home adaptation would undermine independence.

2150. A client with hearing loss is scheduled for teaching about a new medication regimen. Which nursing action is best before starting the teaching session?

- A. Move the client to a quieter area with minimal background noise
- B. Provide all information quickly to avoid tiring the client
- C. Ask a family member to answer questions for the client
- D. Stand behind the client to avoid causing embarrassment

Correct Answer: A

Reducing background noise improves the client's ability to hear and process information. Rushing decreases comprehension, using family instead of the client can reduce autonomy, and standing behind the client removes visual cues that support communication.

2151. A client with newly diagnosed glaucoma tells the nurse, "I stopped going to church because I'm afraid I'll trip or need help and embarrass myself." Which nursing response is most appropriate?

- A. "You should stay home until your vision is fully treated."
- B. "It sounds like fear of losing independence is keeping you from activities that matter to you."
- C. "Most people adjust, so this will probably stop bothering you soon."
- D. "Your family can decide whether church is still realistic for you."

Correct Answer: B

This response identifies the likely underlying feeling and invites further discussion, which is therapeutic and supports coping. The other responses minimize the client's feelings, increase dependence, or promote unnecessary withdrawal from valued activities.

2152. A nurse enters the room of a client with severe visual impairment who is quietly crying. The client says, "People keep moving my things, and I feel helpless." Which action should the nurse take first?

- A. Explain that staff members must move items during routine care
- B. Reassure the client that helplessness is common and will pass
- C. Ask the client to describe how they would like personal items arranged
- D. Suggest that family bring duplicate items from home

Correct Answer: C

The priority is to restore control and promote adaptation by involving the client in organizing the environment. Simply explaining staff routines or offering reassurance does not address the client's loss of control. Bringing duplicate items may help later but does not directly solve the immediate problem.

2153. A client with chronic tinnitus says, "The sound never stops. I snap at people because I'm already on edge." Which nursing goal is most appropriate?

- A. The client will deny hearing the sound by discharge
- B. The client will identify strategies to reduce stress related to the tinnitus
- C. The client will remain alone when symptoms are severe
- D. The client will avoid discussing frustration with family members

Correct Answer: B

For chronic tinnitus, an appropriate psychosocial goal focuses on coping and stress reduction rather than pretending the symptom does not exist. Isolation and avoiding discussion generally worsen distress rather than improve adjustment.

2154. A client with progressive vision loss tells the nurse, "My son keeps doing everything for me before I can even try." Which nursing response is best?

- A. "He is probably just trying to save time."
- B. "Maybe you should let him help so you do not get hurt."
- C. "What tasks would you like to keep doing on your own? We can discuss ways to support that."
- D. "Families often overreact at first, so ignore it for now."

Correct Answer: C

This response supports autonomy, encourages problem-solving, and helps the client identify maintainable strengths. The other options minimize the concern, promote dependence, or advise avoidance instead of constructive coping.

2155. A client with hearing impairment becomes quiet during meals because others laugh while talking, and the client later says, "I think they're laughing at me." Which nursing intervention best addresses the client's psychosocial need?

- A. Tell the client not to worry because the laughter is probably unrelated
- B. Arrange opportunities for one-to-one conversation in a calm setting
- C. Recommend that the client eat alone to avoid misinterpretation

D. Advise staff to speak only when the client initiates conversation

Correct Answer: B

One-to-one interaction in a low-noise setting reduces communication barriers and can rebuild trust and social engagement. Telling the client not to worry dismisses the concern, and isolation or reduced interaction would worsen withdrawal.

2156. An older adult with decreased hearing says, "The television is the only thing I can follow because people talk too fast." Which teaching by the nurse is most appropriate for the family?

- A.** Use short, clear sentences and allow time for the client to process what was said
- B.** Repeat each sentence several times whether the client asks or not
- C.** Speak to the client from another room so the client gets used to different sounds
- D.** Finish the client's sentences to keep the conversation moving

Correct Answer: A

Clear, paced communication with time to process is supportive and effective for many clients with hearing loss. Automatic repetition can be frustrating, speaking from another room reduces comprehension, and finishing sentences can be disrespectful and interfere with communication.

2157. A client with recent visual loss says, "I used to read every night. Now there is nothing left for me to enjoy." Which response by the nurse is best?

- A.** "You should focus on activities that do not require concentration."
- B.** "At least your condition is not painful."
- C.** "Let's explore other ways you might still enjoy books or stories."
- D.** "Most people your age eventually lose interest in reading anyway."

Correct Answer: C

This response acknowledges the loss while promoting adaptation and hope through alternative methods, such as audio materials or other accessible formats. The other responses are dismissive, minimizing, or ageist and do not support coping.

2158. A client with severe bilateral hearing loss is admitted for surgery. The client reads lips well in person but will be unable to do so immediately after the procedure because of positioning and oxygen equipment. Which nursing plan is best to reduce postoperative anxiety and communication failure?

- A.** Explain to the client that postoperative confusion is expected and communication can wait
- B.** Arrange a simple preplanned method for the client to signal needs after surgery
- C.** Assure the client that staff will understand nonverbal behaviors without extra planning
- D.** Instruct the client to rely on family members for communication once awake

Correct Answer: B

Anticipating the barrier and establishing a clear signaling method before surgery supports safety, control, and reduced anxiety. Waiting until after surgery, assuming staff will interpret needs correctly, or depending on family are less reliable and less client-centered.

2159. A client who is losing vision because of a chronic condition tells the nurse, "I know everyone says I need help, but if I accept it, that means I've given up." Which nursing response is best?

- A.** "Accepting help can also be a way to stay in control of what matters most to you."
- B.** "If you refuse help, your family will make decisions for you."
- C.** "You are right to avoid help until you cannot manage at all."
- D.** "Whether you accept help is not really related to coping."

Correct Answer: A

This response reframes help as a tool for preserving autonomy rather than surrendering it, which can reduce resistance and support adaptation. The other options are coercive, reinforce unsafe all-or-nothing thinking, or deny the emotional meaning of the issue.

2160. A client with hearing impairment becomes increasingly anxious during admission questioning. The nurse notices the client answers unrelated questions with "yes" and grips the bedrail tightly. Which action is best?

- A. Continue the interview so the admission can be completed on time
- B. Ask the client to sign the forms now and complete teaching later
- C. Pause and assess the client's preferred communication method before proceeding
- D. Speak much louder so the client can hear every question clearly

Correct Answer: C

The mismatched answers and anxious behavior suggest communication failure. The best action is to stop and determine how the client can best receive information. Continuing, rushing to signatures, or simply speaking louder may increase distress and does not address the underlying barrier.

2161. A resident in long-term care with significant visual impairment has recently begun refusing recreation groups. When the nurse asks why, the resident says, "They start activities without telling me what's happening, and I feel foolish." Which intervention is most likely to improve participation?

- A. Encourage the resident to attend only passive activities such as sitting quietly during music hour
- B. Ask staff to orient the resident to the setting and explain each activity step as it begins
- C. Tell the resident that embarrassment will lessen once attendance becomes routine
- D. Move the resident to a private room during scheduled activity times

Correct Answer: B

The resident's refusal is linked to lack of orientation and predictability. Explaining the environment and activity sequence reduces anxiety and supports inclusion. Passive attendance, reassurance without action, or isolation does not address the reason for withdrawal.

2162. A client with newly acquired hearing loss tells the nurse, "My family says I'm withdrawn, but I'm just tired of getting things wrong and feeling stupid." Which response by the nurse is best?

- A. "You should tell your family to speak louder so you do not miss as much."
- B. "Many people adjust over time, so try not to focus on it too much."
- C. "It sounds frustrating to feel embarrassed in conversations. Let's talk about situations that are hardest for you and ways to make them easier."
- D. "If you keep socializing more often, your confidence will probably return quickly."

Correct Answer: C

Choice C is best because it acknowledges the client's feelings and invites problem-solving, which supports psychosocial adaptation to a sensory loss. This is therapeutic communication: validate the emotion first, then explore specific challenges and coping strategies. Choice A focuses narrowly on volume and may oversimplify hearing loss. Choice B minimizes the client's distress. Choice D gives premature reassurance and advice without first addressing the client's emotional experience.

2163. A client with severe hearing impairment is being discharged after a short hospitalization. The nurse notices the client nods at every instruction but gives incorrect return demonstrations for wound care. Which action by the nurse is best?

- A. Repeat the instructions using the same wording but speak louder
- B. Provide written step-by-step instructions and ask the client to demonstrate the care again
- C. Assume the client understands because the client is cooperative and nodding
- D. Delay all teaching until a family member can be present

Correct Answer: B

Written instructions combined with return demonstration check true understanding and adapt teaching to the client's hearing impairment. Nodding does not confirm comprehension. Speaking louder is not always effective and may distort speech, while delaying all teaching unnecessarily reduces the client's independence and involvement in care.

2164. A client with Ménière disease says, "When the room starts spinning, I panic and think I'm going to lose control in front of everyone." Which nursing intervention is most appropriate to support the client's psychosocial coping?

- A.** Encourage the client to ignore the episodes so they do not gain attention.
- B.** Teach the client to identify early symptoms and use a quiet, low-stimulation place during an attack.
- C.** Advise the client to stay in bed as much as possible to prevent embarrassment.
- D.** Tell the client that vertigo usually is brief and should not be frightening.

Correct Answer: B

B is correct because anticipating triggers or early symptoms and having a plan for a calm environment can reduce anxiety and improve coping with vertigo episodes. A dismisses the client's experience, C promotes excessive restriction and loss of function, and D minimizes the fear instead of helping the client manage it.

2165. A nurse is caring for a hospitalized client with diabetic retinopathy and partial vision loss. The client says, "Ever since my eyesight got worse, I keep thinking my family would be better off without me." Which action should the nurse take first?

- A.** Ask the client whether there is a specific plan to harm themselves.
- B.** Reassure the client that many people adjust well to vision loss over time.
- C.** Encourage the client to focus on strengths and remaining abilities.
- D.** Arrange for occupational therapy to teach adaptive skills.

Correct Answer: A

A is correct because the statement suggests possible self-harm risk, so the priority is immediate suicide assessment, including whether the client has a plan. B and C may seem supportive but are unsafe before assessing risk. D may be helpful later for adjustment, but it does not address the urgent safety concern.

2166. A client who recently began using a hearing aid tells the nurse, "I stopped going to my card group because I keep missing parts of conversations. By the time I figure out what was said, everyone has moved on." Which nursing response is most therapeutic?

- A.** You should stay home until the hearing aid is fully adjusted so you avoid embarrassment.
- B.** Try asking the group leader to speak louder so you do not miss anything.
- C.** That sounds frustrating. Let's talk about ways to stay involved, such as choosing a quieter seat and letting the group know how they can help you communicate.
- D.** Most people need time to adapt, so you should not worry so much about it.

Correct Answer: C

Choice C is correct because it acknowledges the client's feelings and promotes problem-solving that supports social participation and coping. It focuses on practical adaptations without encouraging withdrawal. Choice A increases isolation, choice B is too narrow and may not solve the problem, and choice D minimizes the client's distress instead of addressing the psychosocial impact.

End-of-life psychosocial care

2167. A nurse is caring for a client with terminal cancer who says, "I know I'm dying, and I'm scared my family will fall apart after I'm gone." Which response by the nurse is most therapeutic?

- A.** You should try not to think about that right now.
- B.** Tell me more about what worries you most about your family.
- C.** Your family will be fine because they are strong.
- D.** Let's talk about your medications instead.

Correct Answer: B

This response encourages expression of feelings and explores the client's main concern without giving false reassurance. Choice A blocks communication, choice C minimizes the client's fear, and choice D changes the subject away from the psychosocial need.

2168. The spouse of a dying client begins crying at the bedside and says, "I don't know how to do this." Which action by the nurse is best?

- A. Sit quietly with the spouse and say, "This is very hard. I'm here with you."
- B. Tell the spouse to step out until emotions are under control.
- C. Explain that crying may upset the client and should be avoided.
- D. Reassure the spouse that everything will work out for the best.

Correct Answer: A

Presence, acknowledgment, and support are therapeutic during end-of-life care. Choice A validates the spouse's distress and offers emotional support. Choices B and C dismiss normal grief, and choice D gives vague reassurance rather than meaningful help.

2169. A client receiving end-of-life care says, "I haven't spoken to my brother in years, and I keep thinking about it." Which nursing action is most appropriate?

- A. Encourage the client to discuss whether contacting the brother is desired
- B. Advise the client to avoid emotionally stressful topics
- C. Tell the client that family conflicts are common and should be ignored
- D. Wait to bring this up until the client asks again

Correct Answer: A

Clients at end of life may wish to review relationships and seek closure. The nurse should explore the client's wishes and support those goals. Choices B and C dismiss an important psychosocial concern, and choice D delays support when the need is already being expressed.

2170. A nurse enters the room of a dying client and finds several family members arguing loudly about treatment decisions. The client appears anxious and turns away. What should the nurse do first?

- A. Ask the family to stop the argument and step into the hallway to continue discussion privately
- B. Tell the client that the family is just stressed and means well
- C. Leave the room and return when the family has calmed down
- D. Call security immediately to remove all visitors

Correct Answer: A

The priority is to protect the client from distress and create a calm environment. Asking the family to move the discussion elsewhere addresses the immediate psychosocial need. Choice B minimizes the client's anxiety, choice C abandons the situation, and choice D is unnecessarily extreme unless there is an immediate safety threat.

2171. A terminally ill client tells the nurse, "I think this is happening because I was a bad parent." Which response is best?

- A. You shouldn't blame yourself for being sick.
- B. Why would you think that?
- C. It sounds like you're carrying a lot of guilt about your life.
- D. Most parents feel that way at the end of life.

Correct Answer: C

Reflecting the client's feeling helps the client explore guilt in a supportive, nonjudgmental way. Choice A may shut down further discussion, choice B can sound accusatory, and choice D overgeneralizes and minimizes the client's unique experience.

2172. A client near the end of life says, "I don't want to die alone." The nurse cannot remain in the room continuously because of other assigned clients. Which action is best?

- A. Explain that staffing does not allow constant bedside presence
- B. Arrange for available family, support persons, or staff presence as much as possible
- C. Tell the client that everyone dies alone in some way

D. Encourage the client to sleep so the fear will lessen

Correct Answer: B

The nurse should respond to the expressed fear with practical support by coordinating presence from family, support persons, or team members when possible. Choice A focuses on limitations rather than the client's need, choice C is insensitive, and choice D avoids the concern.

2173. The adult child of a dying client asks the nurse, "Should I tell my mother that she is dying? No one in the family wants to say it."

What is the nurse's best response?

- A.** Yes, clients should always be told directly, even if they resist the discussion.
- B.** No, it's better to avoid the subject so she does not lose hope.
- C.** Let's talk about what your mother seems to understand and how your family communicates about difficult news.
- D.** That decision belongs only to the provider, not the family.

Correct Answer: C

This response explores the client's awareness, family communication pattern, and readiness for discussion without forcing a rigid approach. Choice A is too absolute, choice B promotes avoidance, and choice D incorrectly shuts down the family's role in communication and support.

2174. A nurse is caring for a dying client whose family requests that specific spiritual rituals be performed at the bedside. What is the nurse's best initial action?

- A.** Ask the family to describe the rituals and determine how the team can support them safely
- B.** Explain that hospital routines take priority over bedside rituals
- C.** Tell the family that spiritual care is the chaplain's responsibility only
- D.** Delay the discussion until after the client has died

Correct Answer: A

The best initial action is to assess the request and support spiritual practices when possible and safe. Choice A respects cultural and spiritual needs. Choices B and C are dismissive, and choice D may prevent meaningful end-of-life support.

2175. A client in hospice says, "I don't see the point of anything anymore." Which additional statement is most important for the nurse to assess next?

- A.** Are you having more pain today?
- B.** Do you mean that you feel hopeless, or that you are thinking about harming yourself?
- C.** Would you like me to call your family now?
- D.** Have you been sleeping during the day?

Correct Answer: B

A statement about meaninglessness can reflect existential distress, hopelessness, or possible self-harm risk. The nurse should assess directly for safety concerns. Choice A may also matter, but safety assessment comes first. Choices C and D do not address the most urgent psychosocial issue.

2176. A client with end-stage heart failure says, "I am afraid my partner will not be able to manage the bills after I die." Which nursing intervention best addresses this concern?

- A.** Encourage the client to avoid financial topics to reduce stress
- B.** Notify the provider that the client is refusing to accept death
- C.** Offer to contact the social worker to discuss available planning resources
- D.** Tell the client that the partner will learn to manage with time

Correct Answer: C

Financial worries are common end-of-life psychosocial concerns. The nurse should connect the client with appropriate resources such as social services for planning support. Choice A avoids the concern, choice B misinterprets the issue, and choice D gives false reassurance.

2177. A dying client becomes withdrawn and answers questions with one-word responses after learning that curative treatment has stopped. Which interpretation by the nurse is best?

- A.** The client is probably angry with the staff
- B.** The client may be processing the situation and needs calm presence and continued support
- C.** The client no longer wants information about care

D. The client is demonstrating manipulative behavior

Correct Answer: B

Withdrawal after difficult news may reflect grief, shock, or internal processing. The nurse should avoid assumptions and provide supportive presence. Choices A, C, and D assign motives without adequate evidence and can lead to poor care.

2178. During a quiet evening shift, a client at the end of life says, "What do you think happens after death?" Which response by the nurse is best?

- A.** Different people believe different things. What are your thoughts or beliefs about it?
- B.** I don't think staff should discuss personal beliefs with clients.
- C.** You should focus on staying positive instead of thinking about death.
- D.** Most people become less afraid if they stop talking about it.

Correct Answer: A

This response invites the client to share beliefs and concerns while keeping the focus on the client. Choice B shuts down a meaningful discussion, and choices C and D discourage expression of normal end-of-life concerns.

2179. A client's large extended family wishes to visit continuously while the client is dying. The client appears fatigued and quietly tells the nurse, "I want rest, but I don't want to hurt their feelings." What should the nurse do?

- A.** Tell the client that family needs should come first at this stage
- B.** Help the client set limits on visits and communicate those wishes to the family
- C.** Ask the family to decide among themselves who should stay
- D.** Leave visitation unchanged because limiting visitors may seem rude

Correct Answer: B

The client's wishes and comfort are the priority. The nurse should advocate for rest and help set respectful limits. Choices A, C, and D fail to center the client's needs and may increase distress.

2180. A nurse is caring for a client who is actively dying. The client's estranged daughter arrives and insists on entering the room, while the client's long-term partner says the client did not want visitors from that side of the family. The client is now nonverbal. What is the nurse's best action?

- A.** Allow the daughter in because biological relatives always take priority
- B.** Refuse all visitors until the conflict ends
- C.** Follow the client's known wishes and facility policy while seeking assistance from the charge nurse as needed
- D.** Ask both parties to vote on the most fair solution

Correct Answer: C

When conflict arises and the client cannot speak, the nurse should base action on known client preferences and established policy, while involving leadership as needed. Choice A assumes a rule that may not reflect the client's wishes, choice B is unnecessarily broad, and choice D is inappropriate for a serious end-of-life conflict.

2181. A client with advanced illness tells the nurse, "My family keeps saying I need to fight, but I'm so tired." Which response is best?

- A.** You need to keep trying for your family.
- B.** It sounds like the pressure to act a certain way is exhausting for you.
- C.** Families usually mean well, so try to ignore the comments.
- D.** I will tell them to stop visiting.

Correct Answer: B

This response reflects the client's feeling and opens the door to further discussion about goals, distress, and support. Choice A imposes values, choice C minimizes the concern, and choice D is premature and overly extreme without exploring the client's wishes.

2182. The family of a dying client asks the nurse not to let the client know that death is near. Later, the client asks, "Am I dying?" Which nursing response is best?

- A.** "Your family asked the staff not to discuss that with you."
- B.** "No, try not to worry about that right now."

- C. "What have you noticed about your condition?"
- D. "I will ask the provider to explain it later."

Correct Answer: C

Asking what the client understands or has noticed assesses readiness, perception, and desire for information while keeping communication open and honest. Choice A breaches family conflict into the conversation, B is false reassurance, and D delays needed therapeutic communication.

2183. A nurse is caring for a client who is receiving end-of-life care. The client says, "Please don't tell my son how bad things are. He will fall apart." What is the nurse's best action?

- A. Respect the client's wishes and avoid sharing private information without permission
- B. Tell the son because immediate family members have the right to know
- C. Ask another nurse to explain the situation so the client is not upset
- D. Document the request but privately encourage the son to ask the provider

Correct Answer: A

The client's preferences and privacy should be respected unless the client gives permission to disclose information. Choice B places family wishes over client confidentiality. Choice C avoids the issue rather than protecting client autonomy. Choice D still attempts to work around the client's expressed wishes.

2184. A client who is nearing death repeatedly asks, "Who will take care of my dog?" The medical team has already discussed the client's prognosis several times. Which nursing action is best?

- A. Tell the client to focus on personal health instead of the pet
- B. Explore the concern and help identify practical resources or family support
- C. Reassure the client that everything will work out
- D. Redirect the discussion because the question is not a nursing concern

Correct Answer: B

Practical worries can be a major source of distress at the end of life. Exploring the concern and helping connect the client with possible supports addresses coping needs. Choice A dismisses the concern, C gives vague reassurance, and D ignores an important psychosocial issue.

2185. A client receiving palliative care says quietly, "I am being punished." Which response by the nurse is best?

- A. "Why would you think that?"
- B. "You should not talk like that."
- C. "Can you tell me more about what makes you feel that way?"
- D. "No, this is just part of the disease process."

Correct Answer: C

This open-ended response invites the client to discuss possible guilt, spiritual distress, or fear without judgment. Choice A can sound challenging, B is dismissive, and D shuts down exploration of the client's meaning and emotional experience.

2186. The family of a dying client asks the nurse to keep giving hopeful messages because they believe talking about death will cause the client to give up. The client later says, "Everyone changes the subject when I bring up dying." Which nursing action is best?

- A. Follow the family's lead and avoid discussion of dying
- B. Encourage open communication by responding honestly to the client's concerns
- C. Tell the client that the family is only trying to help
- D. Advise the client to save difficult questions for the provider

Correct Answer: B

The nurse should support the client's expressed need to talk and provide honest, therapeutic communication. Family discomfort should not override the client's psychosocial needs. Choice A reinforces avoidance, C minimizes the client's isolation, and D unnecessarily defers supportive communication.

2187. A nurse is caring for a client from a culture in which family-centered decision-making is strongly valued. The client looks to the eldest son before answering questions about care preferences. Which nursing action is best?

- A. Insist that the client answer independently to protect autonomy
- B. Assume the son is the legal decision-maker and direct all questions to him
- C. Assess the client's preference for how decisions and information should be handled
- D. Avoid discussing care preferences because cultural differences make errors likely

Correct Answer: C

The nurse should assess the individual client's wishes rather than making assumptions. Some clients prefer family-centered decision-making, while others do not. Choice A may ignore cultural preference, B assumes authority that may not exist, and D avoids essential assessment.

2188. A client with advanced illness says, "I don't want any more visitors today." An hour later, two close relatives arrive and say, "We came a long way, and we are going in." What should the nurse do first?

- A. Let them enter because they are close family members
- B. Explain that the client requested no visitors and offer to update them separately if permitted
- C. Ask security to remove them immediately
- D. Tell the client that refusing family visits may cause regret later

Correct Answer: B

The client's current preference guides care. The nurse should protect the client's stated wishes while responding respectfully to family. Choice A overrides autonomy, C may be premature unless the situation escalates, and D is coercive and nontherapeutic.

2189. A client in the last weeks of life says, "I keep seeing my mother sitting in that chair at night." The client is calm and says the experience is comforting. Which action by the nurse is best?

- A. Tell the client that visual hallucinations are not real

- B. Document the statement and argue gently against the perception
- C. Acknowledge the experience and assess for distress or safety concerns
- D. Immediately request restraints to prevent confusion-related injury

Correct Answer: C

At end of life, unusual perceptions may occur and are not always distressing. The nurse should acknowledge the experience without arguing and assess whether it causes fear, unsafe behavior, or other concerns. Choice A and B invalidate the client, and D is an extreme, unjustified response.

2190. The wife of a dying client asks, "He keeps sleeping more. Does that mean he is giving up?" Which response by the nurse is best?

- A. "Yes, that usually means the person has stopped trying."
- B. "Increasing sleep can happen as the body weakens. What changes have you noticed?"
- C. "You should try to keep him awake more during the day."
- D. "It is better not to think too much about what it means."

Correct Answer: B

This response provides calm, simple information without judgment and invites the family member to share observations and concerns. Choice A is inaccurate and harmful, C may increase burden and discomfort, and D shuts down needed support.

2191. A client receiving hospice care states, "I don't want my grandchildren to remember me like this." Which nursing intervention is most appropriate?

- A. Suggest avoiding all contact with the grandchildren
- B. Explore what the client hopes the grandchildren will remember and discuss options for connection
- C. Tell the client that children usually adjust better than adults
- D. Advise the family to decide without involving the client

Correct Answer: B

This response centers the client's values and supports meaningful choices, such as visits, letters, recordings, or other forms of connection. Choice A is overly restrictive, C minimizes the concern, and D excludes the client from an important psychosocial decision.

2192. A nurse notices that the brother of a dying client answers every question directed to the client, even when the client is awake and able to speak. Which nursing action is best?

- A. Continue speaking with the brother to avoid conflict
- B. Ask the brother to leave the room immediately
- C. Direct questions back to the client and assess how much involvement the client wants from the brother
- D. Document that the brother is the primary spokesperson

Correct Answer: C

The nurse should preserve the client's voice and also assess desired family involvement. Choice A sidelines the client, B is unnecessarily confrontational without first assessing the situation, and D makes an assumption that may not reflect the client's wishes or formal authority.

2193. A client near the end of life says, "I am afraid my family will remember me in pain and confused." Which nursing action best addresses this concern?

- A. Tell the client that families usually forget difficult final days
- B. Acknowledge the fear and discuss ways to support comfort and meaningful family time
- C. Explain that symptom control is only the provider's responsibility
- D. Change the subject to advance directives

Correct Answer: B

The client is expressing fear about suffering and family memories. The best response validates the emotion and focuses on comfort measures and opportunities for connection. Choice A is false reassurance, C is incorrect because nurses contribute to symptom support and advocacy, and D ignores the immediate concern.

2194. A dying client says to the nurse, "Promise me you won't let them send me back to the hospital." There is no immediate transfer planned. What is the nurse's best response?

- A. "I promise that will not happen."
- B. "That is not something nurses need to discuss."
- C. "Tell me more about what worries you about going back to the hospital."
- D. "If your family wants transfer, there will be nothing we can do."

Correct Answer: C

The nurse should explore the client's fear and values rather than make promises that may be beyond the nurse's authority. Choice A gives an unrealistic guarantee, B dismisses an important concern, and D is inaccurate and undermines advocacy for the client's preferences.

2195. A client with advanced chronic illness becomes tearful when personal care is provided and says, "I hate that my daughter has to see me like this." Which nursing response is best?

- A. "She is your daughter, so this should not bother you."
- B. "Needing care can feel uncomfortable. What concerns you most about her helping?"
- C. "You should let the staff do everything and keep family out of it."
- D. "Try not to be embarrassed. Everyone goes through this."

Correct Answer: B

This response validates the client's discomfort and assesses the specific meaning behind it, such as loss of dignity, role change, or privacy concerns. Choice A dismisses emotion, C imposes a solution without assessment, and D minimizes the client's experience.

2196. A client with terminal cancer says to the nurse, "I know my time is short, but I still haven't made peace with my sister." Which response by the nurse is most therapeutic?

- A. "Try not to dwell on family problems right now."
- B. "At least you have had time to think about what matters most."

- C. "Would you like to talk more about your sister and what feels unfinished?"
- D. "You should ask the provider for medicine to help you relax."

Correct Answer: C

Choice C is correct because it uses therapeutic communication to explore the client's feelings and concerns about unfinished relationships, which is an important psychosocial need at the end of life. It invites the client to share more and helps the nurse assess whether the client wants support with reconciliation or life review. Choice A dismisses the concern. Choice B offers false reassurance and shifts away from the client's distress. Choice D may be appropriate later if anxiety is severe, but it does not first address the client's expressed emotional need.

2197. A client with metastatic cancer says to the nurse, "I don't want to talk to my family right now. I just want to sit quietly." The client's daughter is at the bedside and says, "You need to make Mom talk. We have things to settle." Which nursing action is best?

- A. Tell the daughter that important family matters should be discussed before the client becomes weaker
- B. Encourage the client to speak with the daughter now to avoid future regret
- C. Support the client's wish for quiet time and offer to help arrange a later conversation if the client wants one
- D. Ask the daughter to leave immediately because family presence is interfering with care

Correct Answer: C

The best response is to honor the client's expressed preference while offering support for future communication if desired. End-of-life psychosocial care centers on respecting the client's autonomy, reducing distress, and facilitating coping. Choice B is a common trap because it sounds helpful, but it pressures the client. Choice D may be unnecessarily abrupt because the first priority is supporting the client's stated wish in a calm, therapeutic way.

2198. A client receiving end-of-life care tells the nurse, "I don't want a chaplain. I haven't been to church in years." A few minutes later, the client adds, "But I do keep thinking about whether my life mattered." Which response by the nurse is best?

- A. Many people feel that way, so try not to dwell on it
- B. Would you like to talk about what has been most meaningful in your life?
- C. I will ask the provider to prescribe something for anxiety
- D. You should reconsider seeing a chaplain because spiritual care is important now

Correct Answer: B

The client's statement suggests existential distress, not necessarily a request for formal religious support. Inviting the client to reflect on meaning is therapeutic and patient-centered. Choice D is less appropriate because it pushes a specific resource the client already declined. Choice A minimizes the concern, and Choice C jumps to medication without first addressing the psychosocial need being expressed.

2199. During a night shift, a client who is actively dying becomes restless and says, "I have to get home. My children are waiting for me at school." The medical record states the client's children are adults. The spouse starts crying and says, "Please don't upset him by correcting him." What is the nurse's best action?

- A. Gently reorient the client by explaining that the children are now adults
- B. Ask the spouse to step out so the nurse can assess the client alone
- C. Respond to the emotion by saying, "You sound worried about your children. Tell me what you're thinking about," while also assessing for causes of restlessness
- D. Avoid further conversation because discussing the statement may increase confusion

Correct Answer: C

At end of life, the nurse should respond to the emotion behind the statement and simultaneously assess for reversible contributors to restlessness, such as discomfort, hypoxia, urinary retention, or fear. Choice C does both. Choice A focuses on factual correction, which may increase distress and is often not therapeutic in this setting. Choice D misses an opportunity for both psychosocial support and clinical assessment. Choice B may be needed in some situations, but it is not the best first action here.

2200. A client in hospice says, "My family keeps telling me to stay positive, but sometimes I want to talk about being scared." Which nurse response is most appropriate?

- A.** You should focus on happy memories instead of fear
- B.** It may help to avoid upsetting topics when your family is present
- C.** What worries you the most right now?
- D.** I will explain to your family that negative thinking is harmful

Correct Answer: C

An open-ended question that invites the client to express fear is the most therapeutic response. It encourages emotional expression and helps the nurse assess the client's specific concerns. Choice A and B both shut down communication. Choice D is problematic because it labels the client's feelings as negative and creates unnecessary conflict rather than first exploring the client's needs.

Basic Care and Comfort

Enteral feeding and site care

2201. A nurse is preparing to administer a bolus feeding through a gastrostomy tube. Which action should the nurse take first?

- A. Flush the tube with 30 mL of water
- B. Verify tube placement and residual per facility policy
- C. Place the client flat in bed
- D. Warm the formula in a microwave

Correct Answer: B

Before giving a feeding, the nurse should first verify that the tube is in the expected location and assess gastric residual if required by facility policy. This helps reduce the risk of aspiration or giving formula improperly. Flushing is done before and after feedings, but only after placement is confirmed. The client should be positioned with the head elevated, not flat. Formula should not be microwaved because it can heat unevenly.

2202. A client receiving continuous enteral feedings begins coughing and reports shortness of breath. Which nursing action is the priority?

- A. Slow the feeding rate
- B. Stop the feeding and elevate the head of the bed
- C. Flush the tube with sterile water
- D. Document the client's response

Correct Answer: B

Coughing and shortness of breath during enteral feeding raise concern for aspiration. The priority is to stop the feeding immediately and maintain or increase head-of-bed elevation to protect the airway. Slowing the rate is not enough when aspiration is suspected. Flushing does not address the immediate problem, and documentation comes after urgent intervention.

2203. The nurse is caring for a client with a newly placed PEG tube. Which finding at the insertion site should the nurse report immediately?

- A. A small amount of dried crust on the dressing
- B. Mild tenderness for the first day after placement
- C. Redness with purulent drainage and a foul odor
- D. Scant clear drainage at the site

Correct Answer: C

Redness with purulent drainage and foul odor suggests infection and requires prompt reporting. Mild tenderness and small amounts of clear drainage can occur early after placement. Dried crust may be seen and does not by itself suggest a complication. The key is recognizing signs of infection rather than expected early healing changes.

2204. Which position is best for a client during an enteral feeding to reduce the risk of aspiration?

- A. Supine
- B. Trendelenburg
- C. High-Fowler's or with the head of bed elevated
- D. Prone

Correct Answer: C

Keeping the client in High-Fowler's or with the head of bed elevated helps reduce aspiration risk during enteral feeding. Supine and Trendelenburg positions increase aspiration risk. Prone positioning is not appropriate for routine feeding administration.

2205. A nurse is administering medications through a feeding tube. Which action is most appropriate?

- A. Mix all crushed medications together with the formula
- B. Crush enteric-coated tablets to prevent clogging
- C. Give each medication separately and flush the tube between medications
- D. Add the medications to the feeding bag for slow absorption

Correct Answer: C

Medications should be administered separately through the feeding tube, with flushing between medications to help maintain tube patency and avoid drug interactions. Medications should not be mixed with formula or added to the feeding bag. Enteric-coated tablets should generally not be crushed because doing so alters how the drug is intended to work.

2206. A client with a nasogastric tube feeding has watery stools several times during the shift. Which factor should the nurse consider as a possible cause?

- A. Formula infusing too rapidly
- B. Head of bed elevated during feeding
- C. Routine site cleansing
- D. Using room-temperature formula

Correct Answer: A

Diarrhea during enteral feeding can occur when formula is delivered too rapidly. Other contributors may include intolerance, contamination, or medications, but among these options, rapid infusion is the best answer. Elevating the head of bed is a safety measure, not a likely cause of diarrhea. Routine cleansing and room-temperature formula are not the most likely causes here.

2207. The nurse finds that a client's feeding tube is clogged. Which intervention is appropriate to try first?

- A. Use gentle flushing with warm water
- B. Insert a stylet into the tube
- C. Flush with carbonated soda as the standard first step
- D. Forcefully irrigate the tube with a large syringe

Correct Answer: A

A clogged feeding tube is commonly managed first by gentle flushing with warm water using a technique that does not apply excessive pressure. Forceful irrigation can damage the tube. Inserting a stylet is unsafe. Carbonated beverages are sometimes mentioned in practice, but they are not the standard safest first action compared with gentle warm-water flushing.

2208. A nurse is caring for a client who receives intermittent enteral feedings through a gastrostomy tube. Which observation best indicates that the client may not be tolerating the feeding?

- A. The client asks for oral care
- B. The client has abdominal distention and nausea
- C. The client needs repositioning during the feeding
- D. The formula is a standard commercial product

Correct Answer: B

Abdominal distention and nausea suggest poor tolerance of the feeding and may indicate delayed gastric emptying or another problem requiring assessment. Asking for oral care is common and not a sign of feeding intolerance. Repositioning needs are unrelated, and the use of standard formula alone does not indicate intolerance.

2209. A client with a gastrostomy tube asks why the nurse cleans around the tube site every day. Which response is best?

- A. It keeps the formula from becoming too cold
- B. It prevents skin breakdown and lowers infection risk
- C. It helps the tube digest the feeding more quickly
- D. It guarantees the tube will never become dislodged

Correct Answer: B

Routine site care helps keep the surrounding skin clean and dry, reducing irritation, skin breakdown, and infection risk. It does not affect formula temperature or digestion. Good care lowers risk but does not guarantee that the tube cannot be dislodged.

2210. The nurse is preparing to hang a new container of enteral formula for a client receiving continuous feeding. Which observation requires the most immediate action?

- A. The formula has been left at room temperature for an extended period
- B. The feeding pump is plugged into a wall outlet
- C. The client requests extra blankets
- D. The tubing is secured to the gown

Correct Answer: A

Formula left at room temperature for too long can support bacterial growth and should not be used. This requires immediate action to prevent contamination-related complications. The other findings are not urgent problems. This question focuses on safe handling of enteral formula.

2211. A nurse must administer a scheduled feeding through a nasogastric tube to a client who is drowsy and sliding down in bed. What should the nurse do before starting the feeding?

- A. Reposition the client upright and verify tube placement
- B. Begin the feeding slowly while the client rests
- C. Add extra water to thin the formula
- D. Delay all care until the client is fully awake

Correct Answer: A

The nurse should first reposition the client with the head elevated and verify tube placement before starting the feeding. Drowsiness and poor positioning increase aspiration risk. Starting slowly does not remove that risk. Adding water changes the feeding without addressing the safety issue, and delaying all care until the client is fully awake is not the best response when the nurse can first make feeding safer through positioning and verification.

2212. A client with a jejunostomy tube reports cramping shortly after a feeding is started. Which nursing action is most appropriate?

- A. Increase the rate so the feeding finishes sooner
- B. Assess the infusion rate and consider slowing the feeding
- C. Place the client flat to relax the abdomen
- D. Irrigate the tube with fruit juice

Correct Answer: B

Cramping may indicate that the feeding is infusing too quickly or that the client is not tolerating it well. The nurse should assess the rate and consider slowing the infusion based on the client's status and orders. Increasing the rate may worsen symptoms. Lying flat increases aspiration risk in many enteral-feeding situations, and fruit juice is not an appropriate irrigant.

2213. A client receiving continuous tube feedings has residuals that are higher than expected per facility guidelines, along with nausea. Which action by the nurse is best?

- A. Continue the feeding because nausea is expected
- B. Increase the feeding rate to improve gastric emptying
- C. Hold the feeding and follow facility policy for further assessment
- D. Lower the head of the bed to promote comfort

Correct Answer: C

High residuals accompanied by nausea suggest poor gastric tolerance and increased aspiration risk. The best action is to hold the feeding and follow facility policy for next steps, which may include reassessment and provider notification. Continuing or increasing the feeding could worsen intolerance. Lowering the head of the bed increases aspiration risk.

2214. While providing care for a client with a new gastrostomy tube, the nurse notices that the external tube length appears longer than on the previous assessment and the client is coughing during feeding. What is the nurse's priority action?

- A. Advance the tube to the previous marking
- B. Stop the feeding and assess for tube displacement
- C. Secure the tubing more tightly and continue
- D. Flush the tube and give the remaining formula

Correct Answer: B

A change in external tube length can indicate displacement. Coughing during feeding raises concern for aspiration. The priority is to stop the feeding and assess for displacement according to policy. The nurse should not blindly advance the tube. Continuing the feeding or simply tightening the securement can expose the client to harm if placement has changed.

2215. A nurse is caring for a client with a gastrostomy tube whose skin around the stoma is moist, reddened, and tender. Which intervention is most appropriate?

- A. Apply occlusive tape tightly around the tube
- B. Cleanse and dry the site, then protect the skin from leakage

- C. Rotate the tube repeatedly to stop the redness
- D. Stop all feedings permanently

Correct Answer: B

Moist, reddened, tender skin around the stoma suggests irritation, often from leakage and moisture. Cleansing and drying the site and protecting the skin are appropriate nursing measures. Tight occlusive tape may worsen skin injury. Repeated tube rotation is not the primary solution to skin maceration, and stopping all feedings permanently is not justified by this finding alone.

2216. A nurse is about to give an enteral feeding through a nasogastric tube. Which assessment is most important to complete immediately before the feeding begins?

- A. Measure the client's abdominal girth
- B. Verify that the tube placement is correct per facility procedure
- C. Auscultate bowel sounds in all four quadrants
- D. Check the client's oral temperature

Correct Answer: B

Correct tube placement must be confirmed before each feeding because feeding into a misplaced tube can lead to aspiration or other serious harm. Abdominal girth and bowel sounds may be useful assessments, but they do not replace checking placement. Temperature is not the key safety check immediately before feeding.

2217. A client with a gastrostomy tube is prescribed crushed medications. Which action by the nurse is best when administering these medications through the tube?

- A. Mix all crushed medications together with the formula and instill them at once
- B. Crush each medication, dissolve it appropriately, and give each one separately with flushes as indicated
- C. Add the medications to a full syringe of water and push them in as quickly as possible
- D. Skip flushing to avoid overfilling the client's stomach

Correct Answer: B

Medications should be prepared and administered separately through the tube, with appropriate flushing, to reduce the risk of tube occlusion and medication interaction. Mixing medications with formula can alter absorption and clog the tube. Rapidly pushing a mixture or skipping flushes increases the risk of blockage and poor medication delivery.

2218. A nurse is caring for a client receiving continuous enteral feeding through a pump. The client suddenly vomits a large amount of formula. Which action should the nurse take first?

- A. Slow the feeding rate by half
- B. Turn the client onto the side and stop the feeding
- C. Document the episode and continue to observe
- D. Flush the tube with water

Correct Answer: B

Vomiting during enteral feeding creates an immediate aspiration risk. The nurse should first protect the airway by turning the client to the side and stopping the feeding. Slowing the rate may be appropriate later, but it does not address the immediate danger. Documentation and flushing are not first-priority actions.

2219. A client with a mature gastrostomy tube has routine site care ordered. Which action is appropriate?

- A. Cleanse the skin around the stoma gently and dry the area well
- B. Apply a thick layer of petroleum jelly under the external bolster
- C. Leave dried drainage in place to avoid disturbing the site
- D. Tape the tube tightly against the abdomen to prevent movement

Correct Answer: A

Routine gastrostomy site care includes gentle cleansing and keeping the area dry to help prevent skin breakdown and infection. A thick occlusive layer can trap moisture, leaving drainage in place promotes irritation, and taping the tube tightly can increase pressure injury and discomfort.

2220. A client receiving a bolus feeding through a gastrostomy tube reports feeling bloated midway through the feeding. Which nursing action is most appropriate?

- A. Speed up the feeding so it ends sooner
- B. Stop the feeding temporarily and assess the client's tolerance
- C. Add extra formula to maintain calorie goals
- D. Lower the head of the bed to improve comfort

Correct Answer: B

Bloating during a bolus feeding may indicate poor tolerance. The nurse should pause the feeding and assess the client rather than forcing more formula. Speeding up or adding more formula can worsen symptoms. Lowering the head of the bed increases aspiration risk.

2221. A nurse is preparing to administer an enteral feeding to a client who has medications scheduled at the same time. Which approach is best?

- A. Mix the medications directly into the feeding bag so the client receives everything together
- B. Administer the feeding first, then add the medications to the tubing
- C. Give medications separately from the formula using appropriate preparation and flushing steps
- D. Hold all medications until the end of the shift to avoid interrupting the feeding

Correct Answer: C

Medications should generally be administered separately from enteral formula with proper preparation and flushing. This helps prevent tube obstruction and avoids mixing drugs with formula. Putting medications into the feeding bag or tubing can cause incompatibility and inaccurate delivery. Delaying all medications is not appropriate unless specifically ordered.

2222. During a shift assessment, a nurse notes excoriated skin around a client's jejunostomy tube where drainage has been leaking onto the abdomen. Which intervention is most appropriate?

- A. Cleanse the area gently, protect the skin from moisture, and reassess the tube site regularly
- B. Cover the drainage with a heating pad to dry the skin faster
- C. Scrub the reddened skin vigorously to remove surface bacteria

D. Clamp the tube for the rest of the shift to stop leakage

Correct Answer: A

Leakage can quickly damage surrounding skin, so the nurse should gently cleanse the area, protect it from moisture, and continue monitoring. Heat and vigorous scrubbing can worsen tissue injury. Clamping the tube without an order or full assessment may be unsafe and does not address the underlying issue appropriately.

2223. A client has a nasogastric tube for feedings and says, "My throat feels dry and irritated." Which nursing action is most appropriate?

- A. Remove the tube and reinsert it into the other nostril
- B. Provide regular oral and nasal care as tolerated
- C. Advance the tube 5 cm to reduce dryness
- D. Stop the feeding permanently

Correct Answer: B

Nasogastric tubes can cause dryness and irritation, so ongoing oral and nasal care is an appropriate nursing measure for comfort. Reinsertion or advancing the tube is not indicated just for dryness and may create harm. Stopping the feeding permanently does not address the symptom and is not justified by the information given.

2224. A nurse is teaching a caregiver how to give a prescribed bolus feeding through a gastrostomy tube at home. Which caregiver statement shows correct understanding?

- A. I should let the feeding run in steadily instead of forcing it in quickly.
- B. If the client coughs during the feeding, I should continue so the full amount is given.
- C. I do not need to wash my hands if I am wearing gloves.
- D. I should place the client flat after feeding to help prevent fullness.

Correct Answer: A

A bolus feeding should be allowed to flow steadily rather than being forced rapidly, which can increase discomfort and intolerance. Coughing may signal aspiration risk and should not be ignored. Hand hygiene is still needed even when gloves are used. Lying flat after feeding increases aspiration risk rather than preventing it.

2225. A nurse flushes a feeding tube with water before a scheduled intermittent feeding and meets resistance. The client has no respiratory distress and the tube was functioning earlier. What should the nurse do next?

- A.** Use extra force on the plunger until the obstruction clears
- B.** Instill additional gentle warm-water flushes per facility procedure and reassess patency
- C.** Begin the feeding anyway because the tube is probably still usable
- D.** Insert a rigid stylet into the tube to break up the clog

Correct Answer: B

When a tube is resistant but the client is otherwise stable, a gentle warm-water flush per facility procedure is an appropriate next step to restore patency. Forcing the plunger can damage the tube or injure the client. Feeding through a partially obstructed tube or inserting a rigid object is unsafe.

2226. A client with a long-term feeding tube asks why the nurse flushes the tube before and after feedings and medications. Which response is best?

- A.** It keeps the tube clear and helps prevent blockage.
- B.** It makes the formula digest faster.
- C.** It prevents the client from developing diarrhea.
- D.** It allows more air to move through the tube.

Correct Answer: A

Flushing helps maintain tube patency by clearing formula and medication residue from the lumen. It is not done to speed digestion, prevent all diarrhea, or move air through the tube. The common trap is choosing an answer that sounds physiologic but does not match the actual purpose of flushing.

2227. A nurse caring for a client with a recent gastrostomy tube notes increasing redness, warmth, swelling, and purulent drainage at the stoma. The client also reports tenderness. Which action is most appropriate?

- A.** Document the findings as expected healing
- B.** Clean the site and recheck it in 24 hours without further action
- C.** Report the findings promptly because they suggest infection
- D.** Rotate the tube forcefully to open the drainage tract

Correct Answer: C

Redness, warmth, swelling, tenderness, and purulent drainage are concerning for infection and should be reported promptly. These are not expected healing findings. Waiting 24 hours may delay treatment, and forceful manipulation can worsen tissue injury.

2228. A client receiving continuous enteral nutrition develops sudden abdominal distention, nausea, and restlessness. The formula has been infusing at the prescribed rate. Which nursing action is best?

- A.** Stop the feeding and assess the client further for intolerance
- B.** Increase the rate slightly to finish the bag sooner
- C.** Encourage the client to drink carbonated fluid for relief
- D.** Place the client flat and massage the abdomen

Correct Answer: A

Abdominal distention and nausea during enteral feeding suggest intolerance and require the feeding to be stopped while the client is assessed further. Increasing the rate can worsen the problem. Carbonated fluids are not an appropriate nursing response in this situation, and lying flat increases aspiration risk.

2229. A client has a gastrostomy tube and receives intermittent feedings. During assessment, the nurse notes that the external fixation device is pressing tightly into the skin and there is a shallow open area beneath it. What is the nurse's best action?

- A. Apply extra tape to keep the tube from moving at all
- B. Loosen or adjust the external device per facility procedure and protect the skin
- C. Push the tube farther inward so the device lies flat
- D. Ignore the finding because a tight fit prevents leakage

Correct Answer: B

A device that is too tight can cause pressure injury and skin breakdown. The nurse should adjust it appropriately per facility procedure and protect the skin. Taping it tighter or pushing the tube inward can worsen tissue damage. A snug device is not the same as one causing pressure injury.

2230. A nurse is preparing to start a prescribed feeding through a small-bore tube. The nurse cannot obtain confirmation of tube placement using the facility's required method. The client is hungry and asks the nurse to start the feeding anyway because it was fine earlier. Which response by the nurse is best?

- A. Start the feeding slowly since the tube worked earlier in the day
- B. Give a small test amount of formula and watch for coughing
- C. Hold the feeding until tube placement is verified according to policy
- D. Flush the tube with formula first to see whether it passes easily

Correct Answer: C

If tube placement cannot be verified by the required method, the feeding should be held until placement is confirmed. Earlier function does not guarantee current safe position. A test feeding or formula flush is unsafe because it could place formula into the wrong location and increase aspiration risk.

2231. A client with a newly placed gastrostomy tube is receiving feedings. Which finding should the nurse report promptly because it suggests poor tolerance of the feeding rather than a normal expected response?

- A. A feeling of fullness for a short time after the feeding
- B. A small amount of dried drainage at the insertion site
- C. Coughing and shortness of breath during the feeding
- D. Mild tenderness at the stoma site during the first day

Correct Answer: C

Coughing and shortness of breath during feeding raise concern for aspiration or respiratory compromise and should be reported promptly. Temporary fullness can occur with feedings, mild early tenderness may be expected after placement, and a small amount of dried drainage can be seen at the site. The key is recognizing the finding that points to an unsafe feeding response.

2232. The nurse is administering medications through a feeding tube and notices that the tube-feeding formula is still running. Which action is best?

- A. Mix the medications into the formula so they enter more slowly
- B. Stop the feeding, give the medications as prescribed through the tube, and use appropriate flushing
- C. Give the medications by mouth instead because the tube is already in use
- D. Clamp the tube for 4 hours after the medications to improve absorption

Correct Answer: B

The best action is to stop the feeding, administer the medications through the tube as prescribed, and flush appropriately to help prevent incompatibility and clogging. Mixing medications into formula is unsafe because it can alter medication delivery and obstruct the tube. Giving them by mouth may be inappropriate for a client who needs enteral access, and clamping for 4 hours is unnecessary and could interrupt nutrition without reason.

2233. A client has a jejunostomy tube for feedings. Which prescribed feeding method should the nurse question?

- A. Continuous feeding by pump
- B. Slow infusion using controlled equipment
- C. Rapid large-volume bolus feeding
- D. Scheduled water flushes as ordered

Correct Answer: C

A rapid large-volume bolus should be questioned for a jejunostomy tube because the small intestine does not tolerate large, fast feedings as well as the stomach. Continuous or controlled slow infusion is more appropriate, and water flushes are commonly used to maintain patency when ordered. The trap is choosing an option that sounds efficient but ignores the location of the tube.

2234. While providing skin care around a feeding tube site, the nurse notes moist redness where gastric drainage has been leaking onto the surrounding skin. Which intervention is most appropriate to help protect the skin?

- A. Apply a skin barrier product around the site
- B. Massage the reddened area twice per shift
- C. Cover the area with a heating pad
- D. Tighten the external device to stop all movement

Correct Answer: A

A skin barrier helps protect surrounding tissue from moisture and gastric drainage, which can quickly break down skin. Massage can worsen irritation, heat is not appropriate for leaking excoriated skin, and tightening the external device may increase pressure injury rather than solve the problem. The best answer targets the cause of the breakdown: moisture and caustic drainage.

2235. A client receiving continuous enteral nutrition says, "I keep having loose stools." The nurse has confirmed that the feeding rate matches the prescription. Which nursing action is most appropriate to try first?

- A. Stop the feeding permanently and begin oral meals
- B. Warm the formula in a microwave before restarting it

C. Assess the formula, recent medications, and possible causes of diarrhea, then notify the provider if needed

D. Add extra water to the formula to dilute it

Correct Answer: C

The nurse should assess likely contributors such as formula tolerance and medications before further action and then report persistent problems as indicated. Diarrhea during enteral feeding is not managed by permanently stopping nutrition without evaluation. Microwaving formula is unsafe because it can heat unevenly, and diluting formula independently changes the prescribed nutrition. The key is clinical judgment: evaluate common causes instead of making an unsupported change.

2236. A client accidentally pulls out a newly inserted gastrostomy tube that was placed earlier the same day. What is the nurse's priority action?

- A. Reinsert the tube immediately using the old tract
- B. Cover the site with a dressing and notify the provider at once
- C. Leave the site open to air and reassess in 8 hours
- D. Insert a urinary catheter into the opening to keep it patent

Correct Answer: B

A newly inserted gastrostomy tube requires prompt provider notification if dislodged because the tract is not mature, and blind reinsertion can cause serious injury. Covering the site helps protect it while awaiting further orders. Reinserting the tube or placing another device into the opening without appropriate authorization and conditions is unsafe. This is a high-risk situation because the tract has not yet stabilized.

2237. A nurse is reviewing actions taken during enteral feeding care. Which action requires immediate correction?

- A. Keeping the client's head elevated during the feeding
- B. Using a 60-mL syringe to flush the tube with water
- C. Checking that prescribed tube-placement verification has been completed before use

D. Adding blue food coloring to the formula to help detect aspiration

Correct Answer: D

Adding blue food coloring to enteral formula requires immediate correction because it is not an appropriate or safe way to detect aspiration. Elevating the head of the bed, using an appropriate syringe for flushing, and ensuring required placement verification are appropriate safety measures. The trap is thinking any visible marker must improve safety, when in fact this practice is not recommended.

Mobility

2238. A nurse is helping a client move from the bed to a chair for the first time after surgery. Which action is most important before the client stands?

- A.** Ask the client to hold the nurse's neck for support
- B.** Raise the bed to the highest position
- C.** Apply nonskid footwear and check for dizziness while dangling
- D.** Place the chair directly facing the foot of the bed

Correct Answer: C

Applying nonskid footwear and checking for dizziness while the client dangles helps reduce fall risk and identifies orthostatic intolerance before standing. Having the client hold the nurse's neck is unsafe body mechanics, raising the bed to the highest position is not appropriate for transfer safety, and chair placement matters but is less important than assessing stability before standing.

2239. A client had a stroke with left-sided weakness and is learning to walk with a cane. The nurse should teach the client to hold the cane in which hand?

- A.** Left hand
- B.** Right hand
- C.** Either hand, based on preference
- D.** Both hands, alternating with each step

Correct Answer: B

The cane is held on the stronger side, so a client with left-sided weakness should hold it in the right hand. This provides better support and a more stable gait pattern. Holding it on the weak side is a common mistake because it seems intuitive, but it reduces balance and support.

2240. A nurse is assisting a client from the bed to a wheelchair. After the client stands, the client's knees begin to buckle. What should the nurse do first?

- A.** Ease the client down the nurse's leg to the floor
- B.** Pull the client upright by the gait belt
- C.** Turn quickly and push the client into the wheelchair
- D.** Step away and call for more staff

Correct Answer: A

If a client's knees buckle, the nurse should widen stance and ease the client down the nurse's leg to the floor while protecting the client from injury. Pulling up on the gait belt may cause musculoskeletal injury. Forcing the client into the wheelchair is unsafe, and stepping away abandons the client during a fall.

2241. A client is ordered to ambulate after abdominal surgery. Which finding should the nurse report before walking the client in the hallway?

- A.** The client says the incision feels tight when standing
- B.** The client reports feeling suddenly lightheaded after sitting at the bedside
- C.** The client asks to use a pillow to splint the abdomen
- D.** The client walks more slowly than usual

Correct Answer: B

Sudden lightheadedness after position change suggests possible orthostatic intolerance and increases fall risk, so the nurse should stop and assess further before ambulation. Incisional tightness and slow walking can be expected after surgery, and using a pillow to splint the abdomen is a helpful comfort measure.

2242. The nurse is teaching a client to use a walker. Which client statement shows correct understanding?

- A. I will push the walker forward as far as I can before I step
- B. I will lift the walker, move it a short distance, and then step into it
- C. I should pull on the walker to help myself stand up
- D. I should keep my feet behind the back legs of the walker while moving

Correct Answer: B

Safe walker use involves lifting or advancing the walker a short distance, then stepping into it while maintaining balance. Pushing it too far ahead reduces support. Pulling on the walker to stand can tip it. Keeping the feet behind the back legs while moving places the client too far back and increases fall risk.

2243. A nurse is caring for a client on prolonged bed rest. Which intervention best helps prevent footdrop?

- A. Placing pillows under both knees continuously
- B. Positioning the feet in dorsiflexion with supportive devices as prescribed
- C. Keeping the client in high Fowler position most of the day
- D. Massaging the calves every 2 hours

Correct Answer: B

Maintaining the feet in dorsiflexion with appropriate supportive devices helps prevent plantar flexion contractures and footdrop. Pillows under the knees promote flexion contractures, high Fowler position does not address ankle position, and calf massage is not used to prevent footdrop and may be unsafe in some situations.

2244. A client with partial weight-bearing status is learning to use crutches. Which action by the client requires correction?

- A. Advancing both crutches before moving the affected leg
- B. Supporting body weight on the hands rather than the axillae
- C. Resting the axillae on the top pads while standing still
- D. Looking forward while walking instead of down at the feet

Correct Answer: C

The client should support body weight on the hands, not the axillae. Resting on the top pads can compress nerves and blood vessels in the axillary area. Advancing the crutches before the affected leg and looking forward are appropriate techniques.

2245. A nurse plans to reposition an immobile client in bed. Which action best uses proper body mechanics?

- A. Twist at the waist while pulling the drawsheet
- B. Keep the feet close together to conserve movement
- C. Raise the bed to working height and use the legs to shift weight
- D. Reach across the bed with straight knees to avoid bending

Correct Answer: C

Raising the bed to a comfortable working height and using the large muscles of the legs with weight shifting reduces strain and promotes safe body mechanics. Twisting, keeping the feet too close together, and reaching with straight knees all increase injury risk.

2246. A client with right leg weakness is being taught how to climb stairs with a cane. Which instruction should the nurse give?

- A. Lead with the weak leg going up and the strong leg going down
- B. Move the cane and weak leg up together, then bring the strong leg up
- C. Lead with the strong leg going up and the cane with the weak leg going down
- D. Keep the cane on the weak side when using stairs for better balance

Correct Answer: C

The usual teaching is up with the strong leg, down with the weak leg and assistive device. This pattern improves stability and reduces the load on the weaker leg. A common trap is choosing an option that moves the weak leg first when climbing, which is less stable.

2247. A postoperative client has been on bed rest for 3 days. Which assessment finding most strongly suggests the client is not tolerating increased activity?

- A. The client asks to rest after walking

- B.** The client develops marked shortness of breath and pallor during ambulation
- C.** The client reports mild incisional soreness after turning
- D.** The client prefers to walk with a staff member nearby

Correct Answer: B

Marked shortness of breath and pallor during ambulation suggest poor activity tolerance and possible cardiopulmonary compromise, so activity should be stopped and the client reassessed. The other findings can be expected or do not necessarily indicate inability to tolerate progressive mobility.

2248. A nurse is preparing to transfer a client from bed to stretcher with help from another staff member. Which situation indicates the need to use a mechanical lift instead?

- A.** The client can bear weight and follow commands
- B.** The client needs reminders to move the stronger leg first
- C.** The client is unpredictable, cannot support weight, and cannot follow directions
- D.** The client reports pain with movement but can assist

Correct Answer: C

A client who cannot bear weight, cannot follow directions, and is unpredictable presents a high risk for injury to both client and staff, making a mechanical lift the safest option. The other situations may still allow transfer with assistance and proper technique.

2249. A nurse observes a client using a gait belt during ambulation. Which action by the nurse is correct?

- A.** Place the gait belt loosely over the client's gown at chest level
- B.** Hold the belt from the back while walking several steps ahead of the client
- C.** Secure the belt snugly at the waist and walk slightly behind and to the side
- D.** Remove the belt once the client is standing steadily

Correct Answer: C

The gait belt should be secured snugly at the waist to provide control during ambulation, and the nurse should walk slightly behind and to the side of the client. Chest placement is incorrect, walking ahead reduces ability to assist, and removing the belt too soon eliminates a safety aid during movement.

2250. A nurse is helping a client move up in bed. Which action is best to reduce friction and shear?

- A.** Pull the client by the arms toward the head of the bed
- B.** Use a drawsheet and lift with assistance rather than dragging
- C.** Raise the head of the bed to 90 degrees before moving the client
- D.** Ask the client to twist at the waist during the move

Correct Answer: B

Using a drawsheet and lifting with assistance reduces friction and shear, which helps protect skin and underlying tissue. Pulling by the arms risks injury, raising the head of the bed increases sliding and shear, and twisting is unsafe and ineffective.

2251. A client is prescribed use of a trochanter roll while on bed rest. The nurse explains that the purpose of this device is to prevent which complication?

- A.** External rotation of the hip
- B.** Flexion contracture of the elbow
- C.** Hyperextension of the neck
- D.** Edema of the lower legs

Correct Answer: A

A trochanter roll helps maintain neutral hip alignment and prevents external rotation of the hip in an immobile client. It is not used to prevent elbow contractures, neck hyperextension, or dependent edema.

2252. A nurse is caring for an older adult who has been reluctant to get out of bed. Which nursing action is the best first step to promote safe mobility?

- A.** Tell the client that staying in bed will lead to complications

- B.** Assess the client's baseline strength, balance, pain, and fear of falling
- C.** Insist that the client walk to the hallway twice each shift
- D.** Wait until physical therapy arrives before attempting any movement

Correct Answer: B

Assessment comes first. Evaluating strength, balance, pain, and fear of falling helps the nurse plan a safe, individualized mobility approach. Warning the client without assessment is not enough, insisting on walking may be unsafe, and delaying all movement may contribute to further decline if basic mobility can be done safely.

2253. A nurse is teaching a client with right-sided weakness how to walk using a standard walker. Which sequence should the nurse instruct the client to use?

- A.** Move the weak leg forward, then the walker, then the strong leg
- B.** Move the walker forward, then the weak leg, then the strong leg
- C.** Move the strong leg forward, then the weak leg, then the walker
- D.** Move both legs forward together, then lift the walker

Correct Answer: B

With a standard walker, the client should move the walker first to provide support, then move the weaker leg into the walker, and then bring the stronger leg forward. This keeps the weak side protected. A common trap is choosing a sequence that starts with the weak leg before the walker is in place, which reduces stability.

2254. A nurse is preparing to logroll a client with a recent spinal injury. Which action is most appropriate during the turn?

- A.** Twist the client's shoulders first, then the hips
- B.** Raise the head of the bed to 60 degrees before turning
- C.** Keep the client's head, shoulders, hips, and legs in alignment
- D.** Pull the client toward the side rail by the drawsheet with one nurse

Correct Answer: C

Logrolling keeps the spine in straight alignment by moving the head, shoulders, hips, and legs as one unit. Twisting the upper and lower body separately can worsen spinal injury. Raising the head of the bed is not part of a logroll, and one nurse is generally not enough for a safe turn in this situation.

2255. A client is on bed rest with an abduction pillow in place after hip surgery. The nurse explains that the main purpose of this device is to prevent which problem?

- A.** Hip adduction that could dislocate the prosthesis
- B.** Pressure injury formation on the heels
- C.** Venous stasis in the lower legs
- D.** Lumbar muscle spasm from prolonged immobility

Correct Answer: A

An abduction pillow keeps the legs separated and helps prevent adduction at the hip, which reduces the risk of hip dislocation after surgery. The other options may be concerns in immobile clients, but they are not the main reason this device is used.

2256. A nurse is assisting a client to dangle at the bedside for the first time after several days of bed rest. Which finding indicates the client is tolerating the activity?

- A.** The client reports sudden nausea and blurred vision
- B.** The client states, "I feel lightheaded," and becomes pale
- C.** The client maintains balance and denies dizziness
- D.** The client develops a rapid, thready pulse

Correct Answer: C

A client who tolerates dangling should remain stable, maintain balance, and not develop dizziness or other symptoms of orthostatic intolerance. Nausea, pallor, lightheadedness, and a thready pulse suggest poor tolerance and possible circulatory instability.

2257. A client has left-sided weakness and is learning to use a quad cane. The nurse notes that the client advances the cane and left leg together, then moves the right leg forward. How should the nurse interpret this action?

- A. The client is using the cane incorrectly because the strong leg should move with the cane
- B. The client is using the cane correctly for support of the weak side
- C. The client should move the cane after both legs are advanced
- D. The client should hold the cane with both hands during each step

Correct Answer: B

When using a cane, the client typically advances the cane with the weak leg, then steps through with the stronger leg. This provides support while the weak side moves. A frequent mistake is thinking the cane moves with the strong leg, which would not support the weaker side effectively.

2258. A nurse is turning an immobile client in bed. To reduce the nurse's risk of back injury, which action is best?

- A. Bend at the waist and keep the legs straight
- B. Keep the feet close together so the body stays centered
- C. Tighten the abdominal muscles and bend the knees
- D. Reach across the bed without lowering the side rail

Correct Answer: C

Using proper body mechanics includes bending the knees, tightening the core muscles, and using the stronger muscles of the legs rather than the back. Bending at the waist increases strain. Keeping feet close together decreases stability, and reaching across the bed creates awkward posture and injury risk.

2259. A client with weakness is transferring from bed to wheelchair. Which wheelchair action should the nurse take before beginning the transfer?

- A. Place the wheelchair directly in front of the bed with the footrests lowered
- B. Lock the wheelchair brakes and move the footrests out of the way

- C. Keep the wheelchair slightly away from the bed so the client can pivot farther
- D. Unlock the wheels until the client is fully seated

Correct Answer: B

Before transfer, the wheelchair should be stabilized by locking the brakes, and the footrests should be moved out of the way to prevent tripping or striking the client's legs. Lowered footrests and unlocked wheels create safety hazards. Increasing pivot distance also makes transfer less safe.

2260. A nurse is caring for a client who must remain in bed for several days. Which intervention best helps prevent contractures of the hips and knees?

- A. Keeping pillows under the knees at all times
- B. Positioning the client in a continuous high-Fowler's position
- C. Performing active or passive range-of-motion exercises regularly
- D. Restricting movement to reduce discomfort

Correct Answer: C

Regular range-of-motion exercises help maintain joint flexibility and reduce the risk of contractures. Keeping pillows under the knees can promote flexion contractures, and restricting movement worsens immobility complications. High-Fowler's position does not prevent hip and knee contractures.

2261. A nurse is instructing a client in the use of crutches. Which observation shows correct three-point gait technique for a client who should avoid bearing weight on the left leg?

- A. The client moves the left leg forward first, then both crutches, then the right leg
- B. The client advances both crutches and the left leg together, then swings the right leg through
- C. The client advances the right crutch, then the left crutch, then both legs
- D. The client moves both legs forward together and drags the crutches behind

Correct Answer: B

In a three-point gait for non-weight-bearing on one leg, the client advances both crutches and the affected leg together, then moves the unaffected leg forward. This keeps weight off the injured side. The other patterns either place steps in the wrong order or do not provide proper support.

2262. A nurse is assisting an unsteady client to ambulate in the hallway using a gait belt. Where should the nurse position themselves for best support?

- A. Directly behind the client, holding the back of the gown
- B. On the client's strong side, slightly behind, with one hand on the gait belt
- C. In front of the client, facing the client while walking backward
- D. At the client's weak side, several steps ahead to guide direction

Correct Answer: B

The nurse should usually stand slightly behind and to the side of the client, often on the stronger side, while holding the gait belt for support. Walking backward in front of the client is unsafe, and holding the gown does not provide secure control. Being several steps ahead would not allow prompt support if balance is lost.

2263. A client on prolonged bed rest is developing weakness and deconditioning. Which nursing action best promotes venous return during periods of immobility?

- A. Encourage ankle flexion and extension exercises
- B. Place a pillow directly under both knees
- C. Keep the client flat in bed continuously
- D. Limit repositioning to once every 8 hours

Correct Answer: A

Ankle flexion and extension exercises activate the calf muscles and help promote venous return, reducing venous stasis. Pillows under the knees may impair circulation. Continuous flat positioning and infrequent repositioning do not support circulation and can increase complications of immobility.

2264. A nurse must move a heavy client up in bed with assistance. Which additional action best reduces skin injury to the client during the move?

- A. Pull the client quickly across the bottom sheet
- B. Use a friction-reducing device or drawsheet to lift, not drag, the client
- C. Raise the head of the bed as high as possible before moving
- D. Ask the client to push with both feet while the bed is flat, even if this causes sliding

Correct Answer: B

Using a friction-reducing device or drawsheet to lift the client helps minimize friction and shear, which can damage skin and underlying tissue. Dragging across the sheet increases skin injury risk. Raising the head of the bed can increase sliding and shear.

2265. A client who had a hip fracture repair is being repositioned. Which movement by the client requires the nurse to intervene immediately?

- A. Using a trapeze to help lift the upper body
- B. Keeping the legs slightly apart while turning
- C. Crossing the legs at the knees while in bed
- D. Using a pillow between the legs during repositioning

Correct Answer: C

Crossing the legs causes hip adduction and can increase the risk of dislocation after hip repair. The other actions support safer mobility and positioning. This item tests recognition of a movement that creates immediate risk rather than a general comfort issue.

2266. A nurse is transferring a client from bed to chair with a pivot technique. The client is able to bear weight on one leg only and follows directions well. Which placement of the chair is best?

- A. Parallel to the foot of the bed, on the client's weak side
- B. At a 90-degree angle to the bed, on the client's strong side
- C. Directly facing the bed, several feet away
- D. At the head of the bed, on either side as long as brakes are locked

Correct Answer: B

For a pivot transfer, the chair is commonly placed at about a right angle to the bed on the client's strong side so the client can pivot toward the stronger leg. Placing it on the weak side makes the transfer harder and less safe. Distance from the bed should be minimal, not several feet away.

2267. A nurse is evaluating assistive device selection for several clients. Which client is the best candidate for a walker rather than a cane?

- A.** A client with mild balance problems who needs minimal support on one side
- B.** A client who can bear full weight and needs help only on stairs
- C.** A client with bilateral leg weakness who requires a broad base of support
- D.** A client with an upper-extremity injury who cannot lift equipment with both hands

Correct Answer: C

A walker provides a wider base of support than a cane and is more appropriate for a client with bilateral leg weakness or greater balance impairment. A cane is better for mild unilateral support needs. A client who cannot use both hands would have difficulty managing a standard walker safely.

2268. A nurse is helping a client move from a bed to a chair. The client has weakness on the left side but can follow directions. On which side should the nurse place the chair before the transfer?

- A.** On the client's weak side
- B.** On the client's strong side
- C.** At the foot of the bed
- D.** Directly behind the head of the bed

Correct Answer: B

The chair should be placed on the client's strong side so the client can pivot toward the stronger leg and arm during the transfer. Placing the chair on the weak side increases the risk of loss of balance and an unsafe transfer.

2269. A postoperative client becomes dizzy and pale while walking with assistance. What is the nurse's priority action?

- A.** Help the client to a chair or lower the client safely
- B.** Encourage the client to keep walking to improve tolerance
- C.** Leave the client briefly to get a blood pressure cuff
- D.** Ask the client to take deep breaths while standing still alone

Correct Answer: A

If a client becomes dizzy and pale during ambulation, the priority is safety: support the client and help them into a chair or lower them safely to prevent a fall. Continuing to walk or leaving the client unattended increases injury risk.

2270. A nurse is planning morning care for a client who had a stroke and has right-sided weakness. Which action best promotes the client's mobility and independence?

- A.** Complete all transfers for the client to avoid fatigue
- B.** Encourage the client to use the unaffected side as much as possible during activity
- C.** Keep the client on bed rest until full strength returns
- D.** Move the bedside table and personal items to the weak side only

Correct Answer: B

Using the unaffected side during activity helps the client participate safely in mobility and supports independence. Doing everything for the client can increase dependence. Bed rest worsens deconditioning. Placing items only on the weak side makes function harder, not easier.

2271. A client has been on prolonged bed rest. Which finding should the nurse recognize as most consistent with footdrop developing?

- A.** The toes point downward and the client has difficulty dorsiflexing the feet
- B.** The knees remain slightly flexed when the client is lying in bed
- C.** The client's fingers curl into the palms at rest
- D.** The shoulders are rounded forward when sitting

Correct Answer: A

Footdrop is characterized by plantar flexion of the foot, with the toes pointing downward and difficulty dorsiflexing. The other options describe positioning or contracture findings in other body areas, not the typical presentation of footdrop.

2272. A nurse is preparing to ambulate a client who has been on bed rest for 1 week. Which action is most appropriate to reduce the risk of orthostatic intolerance before walking?

- A. Ask the client to stand up quickly to shorten the transfer time
- B. Have the client sit at the bedside for a few minutes before standing
- C. Keep the client flat in bed until the moment of ambulation
- D. Limit fluid intake before activity

Correct Answer: B

Having the client sit at the bedside for a few minutes before standing allows the body to adjust gradually and helps reduce dizziness or orthostatic hypotension after bed rest. Standing quickly and staying flat until the last moment increase risk. Limiting fluids may worsen the problem.

2273. An older adult client starts to fall while being assisted to walk. The nurse is using a gait belt. Which action should the nurse take?

- A. Tighten the nurse's grip and pull the client upright immediately
- B. Step away to avoid personal injury and call for help
- C. Widen the nurse's stance and ease the client down the nurse's leg to the floor
- D. Support the client under both arms and lift back to standing

Correct Answer: C

When a client begins to fall, the nurse should widen the stance, support with the gait belt, and ease the client down the nurse's leg to the floor. This helps control the descent and reduces injury. Pulling the client upright or lifting under the arms can injure both the client and the nurse. Stepping away abandons the client during a fall.

Nutrition

2274. A nurse is preparing to feed a client who has dysphagia after a stroke. Which action is most appropriate to reduce the risk of aspiration during meals?

- A. Place the client in a high-Fowler position while eating
- B. Offer liquids first to stimulate swallowing
- C. Encourage the client to talk while chewing
- D. Position the client flat for 30 minutes after the meal

Correct Answer: A

Sitting upright in high-Fowler position helps food move downward and reduces aspiration risk. Offering liquids first can increase choking risk in some clients with dysphagia, talking while chewing interferes with safe swallowing, and lying flat after meals increases aspiration risk.

2275. A client with a new gastrostomy tube is receiving intermittent feedings. Before administering the feeding, which nursing action is the priority?

- A. Warm the formula to room temperature
- B. Check tube placement and residual per facility policy
- C. Add extra water directly to the formula can
- D. Position the client on the left side

Correct Answer: B

The priority before an enteral feeding is to verify that the tube is in the expected location and assess residual according to policy to reduce the risk of aspiration or intolerance. Warming formula may improve comfort, but it is not the priority. Adding water to the formula can alter the prescribed nutrition, and left-side positioning is not the standard safety measure for enteral feeding.

2276. A nurse is reviewing meal trays for several clients. Which tray should the nurse question?

- A. Client with hypertension: baked chicken, brown rice, steamed carrots
- B. Client with lactose intolerance: turkey sandwich, apple slices, soy beverage

- C. Client with dysphagia: thin broth, water, gelatin
- D. Client with iron-deficiency anemia: beef, spinach, orange wedges

Correct Answer: C

A client with dysphagia is often at risk when given thin liquids because they are harder to control during swallowing and may be aspirated. The other trays are appropriate: low-sodium choices support hypertension management, soy beverage avoids lactose, and iron-rich foods with vitamin C support iron absorption.

2277. A client with chronic kidney disease asks why the nurse is limiting foods high in potassium. Which response by the nurse is best?

- A. Potassium can build up and affect heart rhythm
- B. Potassium causes fluid to move into the lungs
- C. Potassium directly raises blood glucose levels
- D. Potassium prevents the body from absorbing protein

Correct Answer: A

In chronic kidney disease, potassium may accumulate because the kidneys cannot excrete it effectively. High potassium levels can cause dangerous cardiac rhythm disturbances. The other options do not accurately explain the main reason for potassium restriction.

2278. A nurse is caring for a client receiving continuous tube feedings through a nasogastric tube. Which finding requires the most immediate action?

- A. The client reports mild fullness after the feeding rate was increased
- B. The feeding bag is hanging for 3 hours before replacement
- C. The client has a blood glucose of 148 mg/dL
- D. The client develops coughing and a drop in oxygen saturation

Correct Answer: D

Coughing with decreased oxygen saturation suggests possible aspiration, which is the most urgent complication of tube feeding. Mild fullness may indicate slower tolerance and should be assessed, but it is less urgent. A glucose of 148 mg/dL may need monitoring depending on the client, and the time the bag has been hanging is not the most immediate threat compared with respiratory compromise.

2279. A client is prescribed a clear liquid diet after surgery. Which menu selection is appropriate?

- A. Cream soup and pudding
- B. Apple juice and plain gelatin
- C. Scrambled eggs and toast
- D. Oatmeal and milk

Correct Answer: B

A clear liquid diet includes transparent liquids and foods that are liquid at room temperature, such as apple juice and plain gelatin. Cream soup, pudding, milk, oatmeal, eggs, and toast are not clear liquids.

2280. A nurse is reinforcing teaching for a client prescribed warfarin. Which statement by the client indicates a need for further teaching about nutrition?

- A. I should try to keep my intake of leafy green vegetables consistent.
- B. I will tell my provider before making major diet changes.
- C. I should avoid all foods that contain vitamin K forever.
- D. Sudden changes in what I eat can affect my medication response.

Correct Answer: C

Clients taking warfarin do not need to avoid all vitamin K foods completely; they should maintain a consistent intake to help keep the medication effect stable. The other statements show appropriate understanding of diet consistency and communication with the provider.

2281. A nurse is planning care for an older adult with poor appetite and unintentional weight loss. Which intervention is most appropriate?

- A. Serve three large meals each day and limit snacks
- B. Offer small, nutrient-dense meals and snacks throughout the day
- C. Encourage fluid intake only at mealtimes
- D. Delay pain medication until after meals

Correct Answer: B

Small, nutrient-dense meals and snacks are often better tolerated by clients with poor appetite and can increase total calorie and protein intake. Large meals may feel overwhelming. Fluids only at mealtimes can cause early satiety, and delaying pain medication may worsen discomfort and reduce intake.

2282. A client with type 1 diabetes becomes shaky and diaphoretic before lunch. The client is alert and able to swallow. Which item should the nurse give first?

- A. Peanut butter crackers
- B. Diet soda
- C. 4 ounces of fruit juice
- D. A cheese sandwich

Correct Answer: C

The client is showing signs of hypoglycemia and needs a rapid-acting carbohydrate first, such as 4 ounces of fruit juice. Peanut butter crackers and a cheese sandwich contain protein and fat, which slow absorption. Diet soda does not provide sugar.

2283. A nurse is reviewing a client's laboratory results and dietary intake. Which finding best indicates improvement in nutritional status over time?

- A. Progressive weight gain after treatment for edema
- B. Improved wound healing and increased strength
- C. Decreased appetite with fewer nausea episodes
- D. Lower fluid intake with reduced urine output

Correct Answer: B

Improved wound healing and increased strength are meaningful clinical indicators that nutritional status is improving. Weight gain can be misleading if fluid status is changing, especially with edema. Decreased appetite is not an improvement, and reduced fluid intake with reduced urine output may indicate dehydration rather than better nutrition.

2284. A nurse is caring for a client with nausea who has eaten very little breakfast. Which action is most likely to improve the client's food intake?

- A. Offer foods with strong odors to stimulate appetite
- B. Provide oral care and offer small amounts of bland food
- C. Ask the client to wait until the next scheduled meal
- D. Encourage the client to drink a large amount of liquid before eating

Correct Answer: B

For a nauseated client, oral care and small amounts of bland food can improve comfort and increase intake. Strong odors may worsen nausea, waiting until the next meal misses an opportunity to support intake, and large amounts of liquid before eating can increase fullness and nausea.

2285. A client receiving bolus tube feedings suddenly reports abdominal cramping and has diarrhea shortly after each feeding. Which action should the nurse take first?

- A. Slow the rate of the feeding and assess formula temperature
- B. Add extra fiber supplements to each feeding immediately
- C. Place the client flat in bed after the feeding
- D. Stop all fluids for the next 12 hours

Correct Answer: A

Abdominal cramping and diarrhea after bolus feedings may occur if the feeding is given too quickly or if the formula is not administered appropriately. The nurse should first slow the rate and assess contributing factors such as formula temperature and tolerance. Extra fiber may or may not help and should not be added independently without assessment. Lying flat increases aspiration risk, and stopping all fluids could worsen hydration problems.

2286. A nurse is teaching a client with iron-deficiency anemia about diet. Which food selection by the client shows correct understanding?

- A. Cottage cheese and white bread
- B. Grilled chicken and mashed potatoes
- C. Spinach salad with strawberries
- D. Rice cereal with black coffee

Correct Answer: C

Spinach provides iron, and strawberries provide vitamin C, which helps increase iron absorption. Cottage cheese and white bread are not strong iron choices. Chicken has some iron, but this option is less helpful than pairing iron-rich foods with vitamin C. Coffee can reduce iron absorption.

2287. A nurse enters a room and finds that a client with continuous enteral feeding is lying flat while the feeding is infusing. What is the nurse's best action?

- A. Turn off the feeding and discard the tubing immediately
- B. Raise the head of the bed and assess the client for aspiration
- C. Flush the tube with cold water to improve tolerance
- D. Reposition the client onto the left side and continue the feeding

Correct Answer: B

A client receiving enteral feeding should be positioned with the head of the bed elevated to reduce aspiration risk. The nurse should correct the position and assess for signs of aspiration. Discarding tubing is unnecessary as the first response, cold water is not indicated, and side-lying without head elevation does not address the key safety problem.

2288. A client with heart failure asks why the provider recommended a low-sodium diet. Which explanation by the nurse is best?

- A. Sodium helps remove excess glucose from the blood.
- B. Sodium restriction reduces fluid retention and the heart's workload.
- C. Sodium prevents the loss of dietary protein in the urine.
- D. Sodium restriction improves the absorption of iron from food.

Correct Answer: B

In heart failure, too much sodium promotes fluid retention, which can worsen edema, increase preload, and increase the heart's workload. The other options do not explain the purpose of sodium restriction in heart failure.

2289. A nurse is reinforcing teaching for a client who follows a vegetarian diet and wants to increase protein intake during recovery from surgery. Which food choice should the nurse recommend as the best source of complete protein?

- A. Peanut butter on whole-grain toast
- B. Black beans with brown rice
- C. Scrambled eggs
- D. Steamed broccoli with almonds

Correct Answer: C

Eggs are a complete protein because they contain all essential amino acids in adequate amounts. Beans with rice can complement each other, but the single best choice here is eggs. Peanut butter, toast, broccoli, and almonds provide some protein but are not the best complete-protein option in this list.

2290. A client has had poor oral intake for 3 days because of mouth pain from oral mucositis. Which meal selection would be most appropriate for the nurse to encourage?

- A. Tomato soup, orange juice, and dry toast
- B. Vanilla pudding, mashed potatoes, and a milkshake
- C. Bacon, scrambled eggs, and hash browns
- D. Fried chicken, coleslaw, and iced tea

Correct Answer: B

Soft, bland, and nonacidic foods are best for oral mucositis because they are less irritating and easier to swallow. Vanilla pudding, mashed potatoes, and a milkshake fit those goals. Tomato soup and orange juice are acidic, dry toast can scrape the mouth, and fried or coarse foods can worsen discomfort.

2291. A nurse is reinforcing diet teaching for a client with hypertension. Which statement by the client indicates correct understanding of how to lower sodium intake?

- A. I should choose fresh foods more often instead of canned foods.
- B. I can replace table salt with seasoned salt.
- C. If food does not taste salty, it must be low in sodium.
- D. Processed lunch meats are a good low-sodium protein choice.

Correct Answer: A

Fresh foods are often lower in sodium than canned and highly processed foods. Seasoned salt still contains sodium, foods may contain hidden sodium even if they do not taste salty, and processed lunch meats are commonly high in sodium.

2292. A nurse is caring for a client who reports lactose intolerance. Which menu item would be most appropriate to offer?

- A. Ice cream
- B. Cheese omelet
- C. Turkey sandwich and applesauce
- D. Cream soup with crackers

Correct Answer: C

A turkey sandwich and applesauce do not typically contain significant lactose and are the best choice. Ice cream, cheese omelet, and cream soup all contain dairy and are more likely to trigger symptoms in a client with lactose intolerance.

2293. A client with dysphagia is prescribed thickened liquids. During lunch, which observation best indicates that the client may not be tolerating the liquids safely?

- A. The client asks for a different flavor beverage.
- B. The client drinks slowly and pauses between sips.

C. The client develops a wet, gurgling voice after swallowing.

D. The client needs extra time to finish the meal.

Correct Answer: C

A wet or gurgling voice after swallowing suggests possible aspiration or pooling of liquid in the airway. Drinking slowly and taking extra time may be appropriate safety behaviors. Asking for a different flavor does not indicate swallowing difficulty.

2294. A nurse is caring for a client receiving total parenteral nutrition (TPN) through a central venous catheter. A new bag of TPN has not arrived from the pharmacy, and the current bag is almost empty. Which action should the nurse take while waiting?

- A. Discontinue the infusion and flush the line with heparinized saline
- B. Hang 0.9% sodium chloride at the same rate
- C. Hang 10% dextrose in water at the prescribed rate
- D. Clamp the line until the new TPN bag is available

Correct Answer: C

TPN contains a high glucose concentration, so abrupt interruption can cause hypoglycemia. If TPN is temporarily unavailable, 10% dextrose in water is commonly used to maintain glucose intake until the next bag arrives. Normal saline does not provide glucose, and clamping or stopping the infusion without replacement increases hypoglycemia risk.

2295. A client with a BMI of 17 has a poor appetite and says, "I get full after just a few bites." Which nursing suggestion is most appropriate?

- A. Drink a full glass of water before each meal.
- B. Eat three large meals at set times each day.
- C. Choose low-calorie foods to avoid nausea.
- D. Try small, frequent meals with high-calorie, high-protein foods.

Correct Answer: D

Small, frequent meals are often better tolerated by clients who feel full quickly, and high-calorie, high-protein foods help improve nutritional intake. Drinking a lot before meals can reduce appetite further. Large meals may be overwhelming, and low-calorie foods do not address undernutrition.

2296. A nurse is reviewing a diet order for a client scheduled for a colonoscopy the next morning. Which food, if selected for the evening meal, should the nurse question?

- A. Apple juice
- B. Gelatin
- C. Beef broth
- D. Cooked oatmeal

Correct Answer: D

Clients preparing for a colonoscopy are generally limited to clear liquids, and cooked oatmeal is not a clear liquid. Apple juice, gelatin, and broth fit a clear-liquid pattern. The key skill is recognizing what belongs on a clear-liquid diet.

2297. A nurse is caring for a postoperative client who is advanced from clear liquids to a full liquid diet. Which item is appropriate to include on the tray?

- A. Custard
- B. Raw carrots
- C. Toast with jelly
- D. Baked fish

Correct Answer: A

A full liquid diet includes foods that are liquid at room temperature or become liquid, such as custard, milk, pudding, and strained cream soups. Raw carrots, toast, and baked fish require chewing and are not part of a full liquid diet.

2298. A client with a severe burn injury is beginning oral intake. Which meal would best support the client's increased metabolic and healing needs?

- A. Green salad, diet soda, and gelatin
- B. Grilled chicken, scrambled eggs, milk, and peanut butter crackers
- C. White rice, applesauce, and tea
- D. Broth, popsicle, and dry toast

Correct Answer: B

Clients with major burns need increased calories and protein to support tissue repair and hypermetabolic demands. Grilled chicken, eggs, milk, and peanut butter crackers provide substantial protein and calories. The other meals are lower in protein and overall energy.

2299. A nurse is checking placement of a newly inserted nasogastric tube before the first feeding. Which action is most appropriate?

- A. Instill air into the tube and listen over the stomach
- B. Measure the pH of aspirated contents and obtain radiographic confirmation per facility protocol
- C. Ask the client whether the tube feels like it is in the right place
- D. Begin the feeding if the tube was inserted without resistance

Correct Answer: B

For a newly inserted nasogastric tube, placement must be verified before use; checking aspirate characteristics such as pH may help, and radiographic confirmation is the most reliable method for initial placement according to facility protocol. Air auscultation and client report are not reliable, and feeding should not begin based only on ease of insertion.

2300. A client with advanced Parkinson disease coughs frequently during meals and has lost 4.5 kg (10 lb) in the past month. Which nursing action is the priority?

- A. Encourage the client to drink thin liquids between bites
- B. Offer foods only when family members are present
- C. Request a swallowing evaluation before continuing the usual diet
- D. Teach the client to finish meals within 20 minutes

Correct Answer: C

Frequent coughing during meals and weight loss suggest dysphagia and aspiration risk. The priority is to obtain a swallowing evaluation so the diet and feeding plan can be made safer. Thin liquids may worsen aspiration, family presence does not address the underlying safety problem, and hurrying meals can increase risk.

2301. A nurse is caring for a client receiving intermittent enteral feedings through a gastrostomy tube. The client reports nausea and abdominal fullness before the next scheduled feeding. Which action should the nurse take first?

- A. Administer the feeding more quickly to finish sooner
- B. Check for gastric residual volume according to facility policy
- C. Place the client flat to reduce abdominal tension
- D. Add extra water to dilute the formula

Correct Answer: B

Nausea and abdominal fullness may indicate delayed gastric emptying or intolerance. The nurse should first assess tolerance by checking gastric residual volume if this is part of facility policy and the prescribed feeding protocol. Increasing the rate, placing the client flat, or changing the formula mixture without an order can worsen complications or increase aspiration risk.

2302. A client with cirrhosis has worsening ascites. Which snack selection should the nurse identify as most appropriate if sodium restriction is prescribed?

- A. Potato chips
- B. Cottage cheese
- C. Fresh pear slices
- D. Canned tomato juice

Correct Answer: C

Fresh fruit such as pear slices is generally low in sodium and is the best choice for a sodium-restricted diet. Potato chips, cottage cheese, and canned tomato juice commonly contain significant sodium and can contribute to fluid retention in a client with ascites.

2303. A nurse is reinforcing teaching for the parent of a toddler who has acute diarrhea. Which statement by the parent indicates a need for further teaching?

- A. I will watch for fewer wet diapers.
- B. I should offer oral rehydration solution in small amounts.
- C. I will give sports drinks instead of regular fluids because they replace electrolytes best.

D. I should report if my child becomes very sleepy or has a dry mouth.

Correct Answer: C

Sports drinks are not the best choice for young children with acute diarrhea because their electrolyte and sugar content may not be appropriate for rehydration. Oral rehydration solution is preferred. Fewer wet diapers, sleepiness, and dry mouth are important signs of possible dehydration to monitor.

2304. A nurse is caring for a client who has iron-deficiency anemia. Which food should the nurse recommend as the best source of dietary iron?

- A. Baked chicken breast
- B. Cheddar cheese
- C. Lean beef
- D. White rice

Correct Answer: C

Lean beef is the best choice because it provides heme iron, which is absorbed more easily than the non-heme iron found in many other foods. Chicken contains some iron but less than beef, while cheese and white rice are not strong iron sources.

2305. A nurse is reinforcing teaching for a client who needs to gain weight after a prolonged illness. Which client statement indicates understanding of the teaching?

- A. I will drink liquids before meals so I feel less hungry later.
- B. I will choose low-calorie foods so I do not upset my stomach.
- C. I will eat three large meals each day even if I feel too full.
- D. I will try eating small, frequent meals and snacks during the day.

Correct Answer: D

Small, frequent meals and snacks are often better tolerated by clients with poor appetite or early satiety and can increase total calorie intake. Drinking liquids before meals can reduce appetite, low-calorie foods do not support weight gain, and forcing three large meals may worsen fullness and reduce intake.

2306. A client who is receiving continuous enteral feedings has developed frequent loose stools. Which action should the nurse take first?

- A. Slow the feeding rate and assess the client for possible intolerance
- B. Stop the feedings permanently and remove the tube
- C. Add fruit juice to the formula to replace fluid losses
- D. Place the client flat in bed to decrease intestinal cramping

Correct Answer: A

The nurse should first assess for feeding intolerance and consider slowing the rate, because rapid infusion can contribute to diarrhea. Permanently stopping feedings is premature, fruit juice may worsen diarrhea, and placing the client flat increases aspiration risk rather than helping the problem.

2307. A nurse is preparing to assist with feeding a client who had a stroke and has right-sided facial weakness. Which action is most appropriate to reduce the risk of aspiration during meals?

- A. Offer fluids through a straw with each bite
- B. Place food on the stronger side of the mouth
- C. Encourage the client to tilt the head backward when swallowing
- D. Keep the client in a supine position for comfort

Correct Answer: B

Food should be placed on the stronger side of the mouth so the client can chew and control the bolus more effectively. Straws may increase the amount of liquid taken in too quickly, tilting the head backward raises aspiration risk, and supine positioning is unsafe for oral feeding.

2308. A nurse is reviewing a breakfast tray for a client prescribed a clear liquid diet after a procedure. Which item should the nurse remove from the tray?

- A. Apple juice
- B. Plain gelatin
- C. Black coffee
- D. Cream of wheat

Correct Answer: D

Cream of wheat is not a clear liquid. A clear liquid diet includes transparent liquids and foods that become liquid at room temperature, such as juice without pulp, plain gelatin, and black coffee. A common trap is confusing liquids with clear liquids; not every liquid item fits this diet.

2309. A nurse is caring for a client with chronic kidney disease who has a new prescription for a potassium-restricted diet. Which food choice by the client requires the most immediate follow-up by the nurse?

- A. Banana
- B. Blueberries
- C. White toast
- D. Rice cereal

Correct Answer: A

Bananas are high in potassium and are the most concerning choice for a client on a potassium-restricted diet. Blueberries, white toast, and rice cereal are generally lower-potassium options. The key is recognizing the food most likely to conflict with the diet restriction.

Nonpharmacologic pain relief

2310. A client with chronic low back pain says, "I don't want more medicine right now." Which nonpharmacologic action should the nurse try first?

- A. Help the client change position and use supportive pillows
- B. Ask the client to remain flat in bed for 6 hours
- C. Encourage the client to skip activity for the rest of the day
- D. Apply a restraint to limit painful movement

Correct Answer: A

Changing position and adding support with pillows is a simple, safe first intervention that can reduce muscle strain and improve comfort. Prolonged bed rest can worsen stiffness, and restraints are not a pain-relief measure. Telling the client to avoid all activity does not address comfort in the moment and may not be therapeutic.

2311. A postoperative client reports incisional pain rated 6 out of 10 and asks what can be done besides medication. Which nursing response is best?

- A. "Try not to think about the pain, and it will go away."
- B. "Let's use deep breathing and guided relaxation while I help you get into a more comfortable position."
- C. "Pain after surgery is expected, so you should tolerate it."
- D. "Nonpharmacologic methods do not work for surgical pain."

Correct Answer: B

Deep breathing, relaxation, and repositioning are appropriate nonpharmacologic strategies that can reduce pain perception and muscle tension. Choice A is dismissive, choice C minimizes the client's experience, and choice D is incorrect because nonpharmacologic measures can be useful alone or with prescribed analgesics.

2312. A nurse is caring for a client with osteoarthritis of the knees. Which intervention is most appropriate to help relieve joint pain without medication?

- A. Apply warmth to the affected joints as prescribed
- B. Keep the room cool and avoid all joint movement
- C. Massage directly over reddened, swollen areas
- D. Place the knees in a tightly flexed position

Correct Answer: A

Warmth can help decrease stiffness and promote comfort in clients with osteoarthritis. Avoiding all movement can increase stiffness. Massaging directly over reddened, swollen areas may worsen discomfort, and tightly flexing the knees can increase pain rather than relieve it.

2313. A client with a tension headache is resting in bed. Which environmental change is most likely to reduce the client's discomfort?

- A. Turn on the television for distraction
- B. Encourage several visitors to stay and talk
- C. Dim the lights and reduce noise in the room
- D. Offer a large meal immediately

Correct Answer: C

Reducing stimulation by dimming lights and decreasing noise often helps clients with headache pain rest more comfortably. Television and visitors may increase sensory input, and offering a meal does not directly address the immediate comfort need.

2314. A client with rib pain from a recent injury needs to cough effectively. Which nonpharmacologic measure should the nurse teach to decrease pain during coughing?

- A. Take rapid, shallow breaths before coughing
- B. Splint the painful area with a pillow or folded blanket
- C. Drink a full glass of cold water immediately before coughing
- D. Lie flat and avoid movement for the rest of the shift

Correct Answer: B

Splinting supports the painful area and decreases movement-related discomfort during coughing. Rapid shallow breathing does not support an effective cough, and lying flat may impair ventilation and secretion clearance. Cold water is not the key pain-relief intervention here.

2315. A nurse plans to use a cold pack for a client who twisted an ankle 2 hours ago. Which finding would make the nurse question this intervention?

- A. The ankle is swollen
- B. The client reports throbbing pain
- C. The client has impaired sensation in the foot
- D. The injury occurred during exercise

Correct Answer: C

Impaired sensation raises concern because the client may not feel excessive cold or tissue injury. Swelling and throbbing pain are common reasons cold may be helpful shortly after injury. The setting of the injury does not determine whether cold is appropriate. The main trap is choosing swelling, but swelling is actually a reason cold is often considered.

2316. A laboring client says, "My contractions are getting stronger, and I want to try something besides medicine first." Which action by the nurse best supports nonpharmacologic pain relief?

- A. Encourage patterned breathing and provide calm coaching
- B. Tell the client to stay completely still between contractions
- C. Limit all oral communication to reduce stimulation
- D. Ask the client to rate pain only after delivery

Correct Answer: A

Patterned breathing and calm coaching are common nonpharmacologic pain-relief measures that promote coping and reduce anxiety. Remaining completely still is not necessary and may be uncomfortable. Limiting communication removes support, and delaying pain assessment is not appropriate.

2317. A client with cancer-related pain says music usually helps with relaxation. Which nursing action best uses this information?

- A. Tell the client music is not a valid nursing intervention
- B. Offer the client headphones and a chance to listen to preferred music during painful periods
- C. Choose fast, unfamiliar music to distract the client
- D. Wait to use music until the pain becomes severe

Correct Answer: B

Using the client's preferred music personalizes care and can enhance relaxation and distraction. Dismissing the method is inappropriate. Unfamiliar or overstimulating music may not be helpful. Waiting until pain is severe misses an opportunity to intervene earlier. The likely trap is choice C, but preference matters in relaxation-based interventions.

2318. A nurse is caring for a client with abdominal pain who is guarding and appears anxious. Which combination of interventions is most appropriate?

- A. Increase room traffic and ask frequent unrelated questions
- B. Use therapeutic touch if accepted, reduce stimuli, and coach slow breathing

- C. Encourage the client to ignore the pain and watch television
- D. Keep the client in one rigid position without support

Correct Answer: B

Reducing environmental stress, using touch if the client is comfortable with it, and coaching slow breathing can reduce anxiety and pain perception. Increased traffic and unrelated conversation may worsen distress. Ignoring pain is not therapeutic, and rigid unsupported positioning can increase discomfort.

2319. A client says a warm shower usually helps menstrual cramping at home. The nurse wants to support similar nonpharmacologic relief in the care setting. Which action is best?

- A. Offer a heating pad or warm compress if appropriate and safe
- B. Apply an ice pack to the lower abdomen for 1 hour continuously
- C. Advise the client that home remedies should be avoided in care settings
- D. Tell the client to remain NPO until the pain improves

Correct Answer: A

A warm compress or heating pad may provide relief similar to what the client finds helpful at home. Continuous prolonged cold may be uncomfortable and is not the best match to the client's stated preference. Dismissing the client's experience is not client-centered, and withholding food does not address the pain complaint.

2320. A client recovering from surgery is reluctant to move because of pain. Which nonpharmacologic nursing action is most likely to promote comfort during repositioning?

- A. Move the client quickly before the client can tense up
- B. Explain the plan, coordinate movement, and use pillows for support after turning
- C. Ask the client to reposition independently without assistance
- D. Delay all turning until the client reports no pain

Correct Answer: B

Explaining the plan reduces anxiety, coordinated movement limits sudden strain, and pillows help maintain alignment and comfort after turning. Moving too quickly can worsen pain. Asking a painful postoperative client to move alone may be unsafe, and delaying all repositioning until there is no pain can contribute to complications and discomfort.

2321. A nurse is evaluating whether a nonpharmacologic pain intervention was effective. Which finding best shows the intervention helped?

- A. The client says, "My pain is down from 7 to 3, and I feel more relaxed."
- B. The nurse completed the intervention exactly as planned
- C. The client's family says the client usually complains less at home
- D. The client fell asleep before the intervention was finished

Correct Answer: A

The best evidence of effectiveness is the client's own report of lower pain and improved comfort. Completing a task does not prove it worked. Family impressions are less direct than the client's assessment, and falling asleep may occur for many reasons and does not necessarily confirm pain relief.

2322. A client with peripheral vascular disease reports lower-leg pain. The nurse considers using a heating pad. Which action is best first?

- A. Apply the heating pad on the highest setting for rapid relief
- B. Assess circulation and skin status before applying heat
- C. Massage the calf deeply to improve warmth
- D. Wrap the leg tightly to hold the heat in place

Correct Answer: B

Before using heat, the nurse should assess circulation and skin status because impaired blood flow increases the risk of tissue injury. High heat, deep calf massage, and tight wrapping are unsafe choices. The likely trap is choosing heat quickly for comfort, but safety assessment comes first.

2323. An older adult with diabetic neuropathy has foot pain and asks for a warm foot soak. Which response by the nurse is best?

- A. "That should be fine as long as the water feels warm to you."
- B. "Let's avoid soaking your feet until sensation and skin condition are assessed."
- C. "Use very hot water so the pain fibers are blocked."
- D. "Soaking is the best approach for neuropathic pain, so no assessment is needed."

Correct Answer: B

With diabetic neuropathy, decreased sensation can prevent the client from detecting excessive heat, increasing burn risk. Assessment of sensation and skin integrity should occur first. Choices A, C, and D are unsafe because they rely on impaired sensation or skip assessment.

2324. A client with severe postoperative pain says, "I know breathing exercises help a little, but the pain is still 8 out of 10." Which nursing action is best?

- A. Document that nonpharmacologic measures were ineffective and provide no further comfort care
- B. Continue only the breathing exercises because the client said they help a little
- C. Use the breathing exercises as supportive care and reassess the need for additional prescribed pain management
- D. Tell the client that reporting high pain means the exercises were done incorrectly

Correct Answer: C

Nonpharmacologic methods can be helpful, but severe pain may require additional prescribed treatment. The best action is to continue supportive measures while reassessing and addressing the need for further pain management. Choice B under-treats pain, choice A gives up on comfort care, and choice D blames the client.

2325. A client with a fresh bruised shoulder from a fall 30 minutes ago asks the nurse for a nonmedication method to reduce pain. Which intervention is most appropriate?

- A. Apply a cold pack wrapped in a towel to the area
- B. Massage the injured shoulder for 10 minutes

- C. Place a heating pad directly on the shoulder
- D. Encourage active range-of-motion exercises immediately

Correct Answer: A

Cold is commonly used for recent soft-tissue injury because it helps limit swelling and can decrease pain. Massage and immediate active motion may increase discomfort in a newly injured area. Heat is generally not the first choice right after an acute injury because it can increase swelling.

2326. A client with chronic neck muscle tension reports pain that worsens after sitting at a desk all day. Which nonpharmacologic intervention should the nurse suggest?

- A. Keeping the neck completely still for the rest of the day
- B. Using a warm compress on the neck muscles
- C. Applying an ice bath to the entire upper body
- D. Avoiding any change in position until bedtime

Correct Answer: B

Warmth can help relax tense muscles and reduce discomfort in chronic muscle tightness. Complete immobility and avoiding position changes can worsen stiffness. An ice bath is unnecessary and not the best match for localized muscle tension.

2327. A hospitalized client becomes more uncomfortable each time a dressing change is performed. Which nursing action is the best nonpharmacologic measure to use during the procedure?

- A. Ask the client to focus on slow, deep breathing throughout the procedure
- B. Explain the steps only after the dressing change is finished
- C. Encourage the client to hold the breath while painful steps occur
- D. Perform the procedure as quickly as possible without speaking

Correct Answer: A

Slow, deep breathing is a simple nonpharmacologic method that promotes relaxation and can reduce pain perception during procedures. Holding the breath increases tension. Withholding explanations or communication can increase anxiety and make pain feel worse.

2328. A client with arthritis in the hands reports stiffness and aching on awakening each morning. Which activity is most likely to help decrease discomfort without medication?

- A. Soaking the hands in warm water before activity
- B. Keeping the hands elevated above the head for an hour
- C. Using a cold pack before every hand movement
- D. Avoiding hand use until late afternoon

Correct Answer: A

Warm water can reduce stiffness and improve comfort before movement in clients with chronic joint discomfort. Avoiding use may increase stiffness. Cold can be helpful in some situations, but morning stiffness is more commonly relieved with gentle warmth.

2329. A client with a stage 4 pressure injury reports persistent pain over the sacrum. The nurse wants to use positioning as a nonpharmacologic measure. Which action is best?

- A. Limit all movement so the painful area is not disturbed
- B. Reposition the client to reduce pressure on the affected area
- C. Massage directly over the reddened surrounding tissue
- D. Place the client flat on the back with no support devices

Correct Answer: B

Repositioning decreases pressure on the painful area and can improve comfort while also protecting tissue integrity. Massage over compromised tissue is inappropriate. Keeping pressure on the sacrum or avoiding movement altogether can worsen both pain and skin injury.

2330. A client with pancreatitis reports abdominal pain and says lying flat makes it worse. Which nonpharmacologic action should the nurse try first?

- A. Assist the client into a position of comfort, such as side-lying with knees flexed
- B. Encourage the client to walk in the hallway every hour
- C. Apply firm abdominal massage for several minutes

D. Place the client in Trendelenburg position

Correct Answer: A

Positioning is a practical first nonpharmacologic step when a client states a certain posture worsens pain. Side-lying with knees flexed may reduce abdominal discomfort. Walking, abdominal massage, and Trendelenburg are not appropriate first measures for a client with significant abdominal pain.

2331. A client with chronic pain says, "The pain gets worse when I lie here thinking about it." Which nurse response best uses a nonpharmacologic pain-relief strategy?

- A.** "Try to ignore it and not talk about it anymore."
- B.** "Let's choose a distraction, such as a game, conversation, or TV program."
- C.** "You should stay in a dark room with no stimulation."
- D.** "Pain is usually less noticeable if you sleep through the day."

Correct Answer: B

Distraction can help reduce pain perception by shifting the client's attention away from the discomfort. Telling the client to ignore pain is not therapeutic. A dark, unstimulating room may help some headache clients but does not directly address this client's report of focusing on pain, and sleeping all day is not an appropriate pain plan.

2332. A client is 1 day after hip surgery and reports pain with turning in bed. Which nursing action best combines pain relief with safe body mechanics?

- A.** Tell the client to move quickly before the pain increases
- B.** Reposition the client alone to avoid delaying care
- C.** Use pillows for support and move the client with proper assistance and alignment
- D.** Keep the client in the same position as long as possible

Correct Answer: C

Using support devices and proper assisted movement can reduce strain, maintain alignment, and improve comfort during repositioning. Moving quickly does not make the process safer or less painful. Leaving the client in one position too long can worsen discomfort and increase complications.

2333. A nurse is caring for a client who reports pain during frequent muscle spasms in the calf after overuse. Which nonpharmacologic action is most appropriate?

- A.** Encourage gentle stretching of the affected muscle as tolerated
- B.** Place the client on strict bed rest for 24 hours
- C.** Massage vigorously during each spasm episode
- D.** Apply direct ice to bare skin for 30 minutes

Correct Answer: A

Gentle stretching can help relieve muscle spasm discomfort when tolerated. Strict bed rest is usually unnecessary. Vigorous massage may worsen pain, and ice should not be placed directly on bare skin because of the risk of tissue injury.

2334. A client receiving palliative care reports ongoing pain and says, "I feel calmer when someone sits with me and listens." Which nursing action best supports nonpharmacologic pain relief?

- A.** Limit interaction so the client can conserve energy
- B.** Provide a quiet presence and encourage expression of feelings
- C.** Redirect every conversation away from the pain
- D.** Tell the client that emotional support does not affect pain

Correct Answer: B

Emotional support and presence can reduce anxiety and improve the client's perception of pain, especially in palliative care. Avoiding interaction or dismissing the effect of support ignores an important nonpharmacologic comfort measure. Forced redirection may make the client feel unheard.

2335. A client had a knee sprain yesterday and is using a prescribed cold pack. Which observation requires the nurse to stop the application and reassess?

- A. The client says the area feels cool
- B. The skin under the pack appears pale and numb
- C. The client reports mild aching at the start of treatment
- D. A towel is placed between the pack and the skin

Correct Answer: B

Pale, numb skin suggests the tissue may be becoming too cold, so the nurse should stop the treatment and reassess. A towel barrier is appropriate. Mild transient aching can occur initially with cold application, but significant numbness and color change are more concerning.

2336. A client with reduced sensation after a spinal cord injury asks for a heating pad to relieve leg discomfort. What is the nurse's best response?

- A. Apply the heating pad on the highest setting for rapid relief
- B. Place the pad under the client so body weight holds it in place
- C. Use caution with heat because decreased sensation increases the risk of burns
- D. Leave the heating pad on continuously as long as the skin is not red

Correct Answer: C

Clients with decreased sensation may not feel excessive heat, which raises the risk of burns. High settings, prolonged use, and placing a heating device under the body are unsafe. The key issue is sensory impairment, not just visible redness.

2337. A postoperative client says guided imagery helped after a previous surgery. Which nurse action best implements this nonpharmacologic method?

- A. Ask the client to describe a calming place and guide attention to that image during discomfort
- B. Tell the client to concentrate on the surgical incision until the pain fades
- C. Turn on the television loudly to block out awareness of pain

D. Wait to use the technique until pain reaches the highest level

Correct Answer: A

Guided imagery involves helping the client focus on a pleasant, calming mental image to reduce distress and pain perception. Focusing on the incision is the opposite of the technique. Loud TV may increase stimulation rather than relaxation, and waiting until pain is severe makes the intervention harder to use effectively.

2338. A client with metastatic bone pain reports, "Any sudden movement makes the pain shoot up." Which nonpharmacologic nursing intervention is most appropriate?

- A. Encourage repeated active exercise of the painful limb
- B. Handle the client gently and support affected areas during movement
- C. Massage deeply over the painful bone areas
- D. Remove pillows so the client can lie flat and still

Correct Answer: B

With bone pain that worsens on movement, careful handling and support during repositioning can reduce pain and prevent added injury. Repeated active exercise or deep massage over painful bone areas may worsen discomfort. Removing supportive pillows can make positioning less comfortable, not more.

2339. A client with a left below-knee amputation 2 days ago reports intermittent burning and cramping in the missing foot. The client says, "How can my foot hurt if it's gone?" Which nonpharmacologic intervention should the nurse suggest first?

- A. Encourage the client to use mirror therapy as instructed
- B. Massage the residual limb vigorously every hour
- C. Apply a heating pad directly to the residual limb
- D. Keep the limb uncovered to reduce stimulation

Correct Answer: A

Mirror therapy can help reduce phantom limb pain by giving the brain visual feedback that may decrease the painful mismatch between what the client feels and what the brain perceives. Vigorous massage may irritate the surgical area, and a heating pad could increase the risk of injury, especially near a fresh surgical site. Keeping the limb uncovered does not specifically address phantom pain and may increase discomfort for some clients.

2340. A client with low back pain from prolonged bed rest asks what the nurse can do right now to help without medication. Which action is the best initial nonpharmacologic intervention?

- A. Reposition the client with pillows to support body alignment
- B. Encourage the client to delay movement until the pain passes
- C. Lower the head of the bed completely to keep the spine flat
- D. Massage the painful area vigorously for several minutes

Correct Answer: A

Supporting proper alignment and reducing pressure with repositioning is a safe, immediate nonpharmacologic measure for discomfort related to bed rest. Delaying movement can worsen stiffness, lying completely flat may increase low back discomfort for some clients, and vigorous massage may aggravate pain rather than relieve it.

2341. A client with a tension headache is resting in a semi-private room and says, "The noise is making my head pound." Which nursing intervention is most appropriate to promote nonpharmacologic pain relief?

- A. Offer a caffeinated beverage
- B. Dim the lights and reduce environmental noise
- C. Encourage the client to walk in the hallway
- D. Ask the client to rate the pain again in 30 minutes

Correct Answer: B

Reducing sensory stimulation with a quieter, darker environment is a common nonpharmacologic measure for headache relief. A caffeinated beverage may not be appropriate for every client, hallway walking may worsen headache symptoms, and reassessment alone does not provide relief.

2342. A postoperative client is anxious before the first ambulation and says, "I know it's going to hurt." Which nurse action best uses a nonpharmacologic technique to help decrease the client's pain during activity?

- A. Tell the client to avoid talking so the pain is not noticed
- B. Instruct the client to take slow deep breaths during movement
- C. Have the client stay in bed until fully pain-free
- D. Explain that pain with activity cannot be relieved without medication

Correct Answer: B

Slow deep breathing is a nonpharmacologic strategy that promotes relaxation and can decrease pain perception during movement. Bed rest until pain-free is not realistic or therapeutic after surgery, avoiding talking does not specifically relieve pain, and saying pain cannot be relieved without medication is incorrect because nonpharmacologic measures can help.

2343. A client with severe abdominal cramping says, "It helps a little when I curl up." Which nursing action best builds on this effective nonpharmacologic pain-relief method?

- A. Assist the client into a position of comfort with knees flexed if tolerated
- B. Encourage the client to remain flat in bed to rest the abdomen
- C. Ask the client to sit straight upright in a chair for 1 hour
- D. Limit position changes so the pain pattern can be observed

Correct Answer: A

When a client reports relief in a flexed position, the nurse should support that position of comfort if tolerated. Flexing the knees may reduce abdominal muscle tension and discomfort. Lying flat may worsen pain, prolonged upright sitting may be uncomfortable, and restricting comfort measures just to observe pain is not the best response.

2344. A client with labor pain between contractions says, "I feel out of control when the pain starts." Which nonpharmacologic intervention should the nurse use to help the client manage pain perception?

- A.** Teach patterned breathing and focused attention during contractions
- B.** Encourage the client to hold her breath until the contraction ends
- C.** Ask the client to stay silent and save energy
- D.** Keep all visitors out of the room without asking the client

Correct Answer: A

Patterned breathing and focusing attention are effective nonpharmacologic methods that can improve a client's sense of control and reduce pain perception during labor. Breath-holding increases tension, forcing silence is not therapeutic, and excluding visitors without considering the client's preferences does not address coping or comfort.

2345. A client with diabetic neuropathy in both feet asks to soak the feet in very warm water for pain relief. Which nursing response is best?

- A.** Use the warm soak because moist heat works best for nerve pain
- B.** Avoid very warm water and choose another comfort measure because decreased sensation increases burn risk
- C.** Add ice to the water instead because cold is always safer than heat
- D.** Soak only 30 minutes to prevent skin breakdown

Correct Answer: B

Clients with diabetic neuropathy may have reduced sensation and may not detect excessive heat, increasing the risk of burns. The safest response is to avoid very warm water and use another comfort measure. Moist heat is not automatically safe, cold is not always safer for every client or condition, and a time limit does not remove the burn risk if the temperature is too high.

Activities of daily living

2346. A nurse is preparing to assist a client with morning hygiene after an uncomplicated hip replacement. Which action best promotes the client's independence during activities of daily living?

- A.** Complete the bath quickly to prevent fatigue
- B.** Encourage the client to do as much of the care as possible within safe limits
- C.** Ask the family to perform all hygiene tasks for the client
- D.** Delay hygiene care until physical therapy is finished for the day

Correct Answer: B

Promoting independence means encouraging the client to participate as much as safely possible in self-care. This supports mobility, confidence, and recovery. Completing all care for the client may be faster, but it does not support function. Family help can be useful, but replacing the client's own effort is not the best first approach.

2347. A client with right-sided weakness after a stroke is learning to dress. Which instruction should the nurse give first when putting on a shirt?

- A.** Insert the unaffected arm first
- B.** Insert both arms at the same time
- C.** Insert the affected arm first
- D.** Put the shirt on over the head first

Correct Answer: C

When dressing a client with one weak or affected side, the nurse should teach the client to dress the affected arm or leg first. This makes dressing easier because the stronger side can then help pull clothing into place. A common trap is choosing the unaffected side first because it seems easier, but that usually makes positioning the weak limb more difficult.

2348. A nurse is assisting a client with left-sided weakness to use a cane. On which side should the client hold the cane while walking?

- A. On the weak side
- B. On the stronger side
- C. Alternating sides every few steps
- D. With both hands in front of the body

Correct Answer: B

A cane is generally held on the stronger side to provide support while the weak leg moves forward. This improves balance and weight distribution. Holding the cane on the weak side is a common mistake because people often think the device should stay next to the injured or weak limb.

2349. A client who has arthritis in both hands says, "Buttoning my shirt is the hardest part of getting dressed." Which nursing response is most appropriate?

- A. "You should stay in bed until your joints are less stiff."
- B. "I will button your clothes for you each morning."
- C. "Let's try clothing with Velcro closures or a buttonhook."
- D. "It is best to avoid getting dressed without family help."

Correct Answer: C

Adaptive equipment and simple clothing modifications, such as Velcro closures or a buttonhook, promote independence for clients with limited hand dexterity. The goal is to support self-care, not unnecessary dependence. Automatically doing the task for the client may solve the immediate problem but does not maximize function.

2350. A nurse is helping an older adult who becomes short of breath during bathing.

Which action is best?

- A. Finish the bath as quickly as possible
- B. Encourage the client to stand for the entire bath to build endurance
- C. Offer a shower chair and allow rest periods during care
- D. Stop bathing permanently and use only linen changes

Correct Answer: C

Using an energy-conserving approach, such as a shower chair and rest periods, helps the client complete hygiene safely while reducing fatigue and dyspnea. Rushing or forcing prolonged standing can worsen symptoms. The best answer supports both safety and participation.

2351. A client with dysphagia is able to brush teeth independently but frequently coughs when taking sips of water to rinse. Which nursing action is most appropriate during oral care?

- A. Provide a large cup of water for repeated rinsing
- B. Use minimal fluid and have suction available if needed
- C. Skip oral care to avoid aspiration
- D. Place the client flat in bed during mouth care

Correct Answer: B

For a client with dysphagia, oral care should still be performed, but the nurse should reduce aspiration risk by using minimal fluid and having suction available if needed. Skipping oral care increases discomfort and infection risk. Positioning flat is unsafe because it increases aspiration risk.

2352. A postoperative client says, "I don't want to wash up today. I just feel too weak." Which nursing response is best?

- A. "You must complete a full bath now because it is on the schedule."
- B. "I will document that you refused all care and leave."
- C. "Let's do the most important parts first and rest as needed."
- D. "It is better if you skip hygiene until discharge."

Correct Answer: C

Breaking care into manageable steps and allowing rest supports dignity, comfort, and participation while respecting the client's condition. The nurse should not force a full bath or abandon care after one statement. The best answer uses a realistic compromise that meets hygiene needs safely.

2353. A nurse is transferring a client from bed to chair for breakfast. The client suddenly becomes dizzy after sitting on the side of the bed. What should the nurse do first?

- A. Continue the transfer quickly before the dizziness worsens
- B. Assist the client back to a safe seated or lying position
- C. Tell the client to take deep breaths and stand up slowly
- D. Leave the client sitting and get a blood pressure machine

Correct Answer: B

If the client becomes dizzy during a transfer, the immediate priority is safety. The nurse should help the client back to a stable position to prevent a fall. Assessment can follow once the client is secure. A common trap is wanting more data first, but preventing injury comes before equipment retrieval.

2354. A nurse is caring for a client who has neglect of the left side after a stroke. Which approach best encourages independence with grooming?

- A. Place grooming supplies only on the right side to avoid frustration
- B. Perform grooming for the client to save time
- C. Cue the client to scan toward the left and place needed items where they can be practiced safely
- D. Avoid mirrors because they increase awareness of deficits

Correct Answer: C

Clients with unilateral neglect benefit from repeated cueing to scan toward the affected side and from structured practice during ADLs. This supports rehabilitation and safer function. Avoiding the neglected side may reinforce the deficit rather than improve independence.

2355. A client with a weak grasp asks for help eating soup. Which nursing intervention best promotes independence while reducing frustration?

- A. Feed the client the entire meal
- B. Offer a built-up handled spoon and position the tray within easy reach
- C. Replace the soup with finger foods only, without asking the client
- D. Tell the client to wait for family members to assist with meals

Correct Answer: B

Adaptive utensils such as built-up handles can improve grip and support independent eating. Proper tray placement also helps the client succeed. Feeding the client may be necessary if safety is compromised, but the question asks for the best way to promote independence.

2356. A nurse is planning morning care for four clients. Which client should the nurse assist first with activities of daily living?

- A. A client who wants to shave before visitors arrive in 2 hours
- B. A client who is continent and prefers to bathe after breakfast
- C. A client who is diaphoretic and reports needing to use the toilet immediately
- D. A client who asks for clean socks before physical therapy later in the morning

Correct Answer: C

The client with an immediate toileting need and diaphoresis has the most urgent comfort and safety concern. Delaying could lead to incontinence, skin issues, embarrassment, or a fall if the client attempts to get up alone. The other requests are important but less time-sensitive.

2357. A nurse is assisting a client with a prosthetic leg to get dressed. Which action requires the nurse to intervene?

- A. The client inspects the residual limb before applying the prosthesis
- B. The client puts on the prosthesis while seated
- C. The client wears the prosthesis over a damp sock
- D. The client uses a stable chair with arms during dressing

Correct Answer: C

A damp sock can increase friction and skin breakdown under the prosthesis, so the nurse should intervene. Inspecting the residual limb, dressing while seated, and using a stable chair are appropriate safety measures. This question tests recognition of a less obvious risk during a routine ADL.

2358. A nurse is teaching a family member how to help a client with dementia during bathing. Which instruction is best?

- A.** Give several directions at once so the bath goes faster
- B.** Use short, simple steps and keep the routine consistent
- C.** Correct the client firmly when tasks are done out of order
- D.** Avoid offering any choices because choices always increase agitation

Correct Answer: B

Clients with dementia often do better with a calm, consistent routine and one-step, simple directions. This reduces confusion and supports cooperation. Giving multiple directions at once or using a firm, corrective approach can increase distress. Avoiding all choices is too absolute; limited choices may still be helpful.

2359. A nurse is helping a client with severe visual impairment arrange personal items for self-care. Which setup is best?

- A.** Move supplies to a different spot each shift to stimulate adaptation
- B.** Keep needed items in a consistent location and describe the environment clearly
- C.** Place all items out of reach until the nurse is ready to hand them over
- D.** Rely mainly on hand gestures to direct the client

Correct Answer: B

Clients with visual impairment benefit from consistency and clear verbal orientation. Keeping items in predictable locations promotes independence and safety. Frequently moving objects or relying on gestures makes self-care harder and less safe.

2360. A client recovering from abdominal surgery wants to wash at the sink. The nurse notes the client is steady while sitting but becomes unsteady after standing for 1 minute. Which plan is best?

- A.** Encourage the client to stand longer each time regardless of fatigue
- B.** Have the client complete all hygiene in bed to avoid any activity
- C.** Arrange supplies so the client can perform most hygiene while seated at the sink
- D.** Postpone hygiene until the client can tolerate a full standing shower

Correct Answer: C

The best plan is to preserve independence while reducing fall risk and fatigue by allowing hygiene to be done seated. This balances activity with safety. Total bed care may be unnecessarily restrictive, while requiring prolonged standing or delaying hygiene does not match the client's current ability.

2361. A nurse is caring for a client with Parkinsonism who takes a long time to feed themselves and becomes frustrated at meals. Which intervention best supports independence with eating?

- A.** Feed the client quickly before the food gets cold
- B.** Offer finger foods and allow extra time for the meal
- C.** Ask the provider to prescribe a liquid diet
- D.** Hold all oral intake until occupational therapy evaluates the client

Correct Answer: B

Offering foods that are easier to manage and allowing enough time promotes the client's ability to perform the task independently. Feeding the client immediately may be faster, but it reduces independence. A liquid diet or withholding intake is not indicated based on slow self-feeding alone.

2362. A client with weakness from multiple sclerosis wants to shower independently. Which nursing action is most appropriate to reduce fatigue while supporting activities of daily living?

- A.** Encourage the client to stand for the entire shower to build endurance

- B. Schedule the shower immediately after physical therapy
- C. Provide a shower chair and plan hygiene during the client's usual high-energy time
- D. Complete the shower for the client to conserve all energy

Correct Answer: C

Energy conservation supports independence. Using a shower chair and timing the activity for when the client has the most energy helps the client participate safely. Standing throughout the shower and scheduling it after therapy increase fatigue. Doing the entire task for the client removes an opportunity for independence when assistance and adaptation would be sufficient.

2363. A nurse is helping a client with hemiparesis brush their teeth at the sink. The client can grip a toothbrush but has poor balance while standing. Which action is best?

- A. Have the client perform oral care while seated in a chair at the sink
- B. Stand behind the client and hold the client's elbows during the task
- C. Complete oral care in bed to avoid all risk
- D. Ask the client to skip oral care until strength improves

Correct Answer: A

Seating the client at the sink preserves participation in oral care while improving safety. Holding the client up during the task is less stable and less practical. Completing care in bed or skipping it reduces independence without being necessary when a safer setup can be used.

2364. An older adult with mild dementia becomes agitated whenever staff give several bathing instructions in a row. Which approach should the nurse use?

- A. Give one simple direction at a time and allow time for the client to respond
- B. Explain the entire bathing routine before beginning so nothing is missed
- C. Correct the client each time they perform a step out of order
- D. Limit bathing to times when two staff members can complete it quickly

Correct Answer: A

One-step directions reduce confusion and support participation for clients with cognitive impairment. Giving too much information at once can increase agitation. Frequent correction may escalate distress, and doing the task quickly for the client does not encourage remaining abilities.

2365. A nurse is assisting a client with chronic obstructive pulmonary disease who becomes breathless while getting dressed. Which instruction is best?

- A. Hold your breath while pulling your shirt over your head
- B. Dress as quickly as possible so the activity ends sooner
- C. Rest between steps and exhale during effort
- D. Avoid using your arms while dressing

Correct Answer: C

Pacing the activity and exhaling during exertion help reduce dyspnea and improve tolerance of dressing. Holding the breath can worsen shortness of breath. Rushing increases oxygen demand, and avoiding arm use is unrealistic for dressing.

2366. A client with limited shoulder movement is unable to wash their back during bathing. Which nursing intervention best promotes independence?

- A. Provide a long-handled sponge for the client to use
- B. Tell the client to skip washing the back area
- C. Wash the client's entire body for efficiency
- D. Ask the client to continue trying without adaptive equipment

Correct Answer: A

Adaptive equipment such as a long-handled sponge allows the client to complete more of the task independently. Skipping hygiene is inappropriate, and doing the entire bath or withholding equipment does not support function.

2367. A nurse is planning toileting for a client who has urge incontinence and limited mobility after surgery. Which plan is most appropriate to maintain dignity and reduce episodes of incontinence?

- A. Wait for the client to request help to use the toilet
- B. Offer toileting at regular intervals and before urgency becomes severe
- C. Encourage the client to drink less during the day
- D. Keep an absorbent brief on the client at all times instead of assisting to the toilet

Correct Answer: B

Scheduled toileting helps reduce incontinence episodes and supports the client's participation in normal elimination routines. Waiting for the request may be too late if urgency is sudden. Restricting fluids can create other problems, and relying only on briefs does not promote dignity or function.

2368. During morning care, a client recovering from pneumonia says, "I can wash my face, but I get worn out if I try to do everything." Which response by the nurse is best?

- A. You should try to finish the whole bath to rebuild your strength
- B. I'll do all of your care today so you can stay in bed
- C. Let's have you do the parts you can manage, and I'll help with the rest
- D. If you are too tired to bathe, you should skip hygiene this morning

Correct Answer: C

This response balances independence with needed assistance. The client participates in manageable parts of the bath, which preserves function and dignity. Forcing completion may worsen fatigue, while doing everything or skipping hygiene provides more dependence than necessary.

2369. A client with severe rheumatoid arthritis wants to brush their hair without help. Which item would best help the client perform this activity?

- A. A hairbrush with an enlarged handle
- B. A standard small-handled comb
- C. A tightly fitted wrist restraint

D. A mirror placed behind the client

Correct Answer: A

An enlarged handle is easier to grasp for clients with painful or limited hand function and promotes independence in grooming. A standard small handle is harder to hold. A restraint is inappropriate, and mirror placement does not address the grip limitation.

2370. A nurse is assisting a client to shave at the bedside. The client takes an anticoagulant and has a mild hand tremor. Which item is safest for the nurse to provide?

- A. A disposable straight razor
- B. An electric razor
- C. A pair of grooming scissors
- D. A razor blade without a handle

Correct Answer: B

An electric razor decreases the risk of cuts and bleeding in a client with tremor and anticoagulant use. The other options increase injury risk. This choice supports safe independence during grooming.

2371. A nurse is preparing an assistive device for a client who has weakness of the right hand and wants to eat without full assistance. Which adaptation is most likely to help?

- A. Place the utensils on the client's right side only
- B. Use regular narrow-handled utensils to strengthen the hand
- C. Provide built-up utensils and position food within easy reach of the left hand
- D. Request that the family feed the client at each meal

Correct Answer: C

Built-up utensils improve grip, and placing food within reach of the stronger hand promotes self-feeding. Positioning everything on the weak side makes the task harder. Standard narrow utensils may be difficult to hold, and having others feed the client reduces independence unnecessarily.

2372. A nurse is caring for a client who had a cerebrovascular accident and becomes embarrassed when needing help with toileting. Which nursing action best preserves the client's dignity?

- A. Discuss the client's toileting needs at the nurses' station so all staff are aware
- B. Leave the bathroom door open in case the client needs immediate assistance
- C. Offer assistance promptly, close the door or curtain, and speak matter-of-factly
- D. Use humor to reduce the client's discomfort during toileting

Correct Answer: C

Prompt help, privacy, and calm, respectful communication support dignity during a vulnerable ADL. Public discussion and an open door reduce privacy. Humor may be misunderstood or increase embarrassment.

2373. A client with left arm weakness is learning to put on a front-opening jacket. Which instruction by the nurse is most appropriate?

- A. Insert the stronger arm first so the task goes faster
- B. Begin with the weaker arm, then bring the garment around to the stronger side
- C. Lift both arms overhead before placing either arm into the jacket
- D. Ask another person to hold the jacket closed while the client steps into it

Correct Answer: B

For dressing with one-sided weakness, clothing is generally applied to the weaker side first so the stronger side can assist with the rest of the task. Starting with the stronger arm makes placement over the weaker side more difficult. The other methods are awkward and less safe.

2374. A nurse is setting up breakfast for a client with tremors who spills liquids easily but can feed themselves. Which action best supports independence and safety?

- A. Serve the beverage in an uncovered full cup
- B. Substitute only foods that can be eaten by hand
- C. Place a lidded cup within easy reach and allow the client extra time

D. Withhold fluids until someone is available to assist

Correct Answer: C

A lidded cup reduces spills while allowing the client to drink independently, and extra time decreases frustration. An uncovered full cup is harder to manage. Finger foods may help in some situations, but eliminating normal beverage access is unnecessary. Withholding fluids delays care without reason.

2375. A nurse delegates morning grooming assistance for a stable client who needs help washing, combing hair, and putting on clean clothes. Which task should the nurse complete personally rather than delegate?

- A. Brushing the client's hair
- B. Helping the client put on nonskid footwear
- C. Assessing a new report of dizziness when the client stands at the sink
- D. Providing the washbasin and clean towel

Correct Answer: C

A new symptom such as dizziness requires nursing assessment and clinical judgment, so the nurse should address this personally. Routine grooming assistance and supplying items can be delegated for a stable client, but assessment cannot.

2376. A client who has had a right-sided stroke is able to participate in dressing but has weakness in the left arm and left leg. Which instruction should the nurse give when teaching the client to put on a pullover shirt independently?

- A. Put the unaffected right arm into the shirt first, then pull the shirt over the head
- B. Start with the weaker left arm, then place the shirt over the head and insert the right arm
- C. Lift both arms at the same time and pull the shirt down quickly
- D. Dress the lower body first to reduce shoulder fatigue

Correct Answer: B

The client should dress the weaker or affected extremity first so the stronger side can assist with pulling and adjusting the clothing. For removal, the stronger side usually comes out first. Choice A is a common trap because candidates may think starting with the stronger side is easier, but it actually makes dressing harder for a client with unilateral weakness.

2377. An older adult with osteoarthritis of both hands wants to remain as independent as possible during meals. Which item would best help the client grasp utensils?

- A. Utensils with built-up, enlarged handles
- B. A plate with smaller portions of food
- C. A straw for all beverages
- D. A wrist restraint during meals

Correct Answer: A

Built-up handles make utensils easier to grasp for clients with painful joints, decreased grip strength, or reduced hand dexterity. Smaller portions do not address the hand-grasp problem, and a straw only helps with drinking. A restraint is inappropriate and does not promote independence.

2378. A postoperative client is weak but wants to participate in morning hygiene. Which nursing action best promotes independence while preventing exhaustion?

- A. Complete the entire bath for the client to conserve energy
- B. Ask the client to finish all hygiene tasks before breakfast
- C. Have the client do the parts of care they can manage and provide rest periods between tasks
- D. Encourage the client to stand at the sink for the full routine to build stamina

Correct Answer: C

The best approach is to support the client's participation in manageable parts of care while pacing activity with rest periods. This promotes independence and conserves energy. Doing everything for the client reduces independence, while requiring all tasks at once or prolonged standing may increase fatigue and risk.

2379. A client with mild left-sided neglect is learning to groom independently after a stroke. The nurse notices the client consistently washes and combs only the right side of the face and hair. Which intervention is most appropriate?

- A. Perform grooming for the client until neglect resolves
- B. Place grooming supplies only on the client's right side
- C. Cue the client to scan toward the left side and use a mirror during grooming
- D. Tell the client to complete grooming as quickly as possible to avoid frustration

Correct Answer: C

Clients with unilateral neglect benefit from cueing to scan the affected side and from visual feedback such as a mirror. This supports safe, more complete self-care while encouraging independence. Doing the task for the client reduces participation, and placing supplies only on the right side reinforces neglect.

2380. A nurse is teaching a client with impaired vision how to perform basic grooming more independently at home. Which instruction is most appropriate?

- A. Keep grooming items in the same location each day
- B. Rearrange supplies often so the client learns to adapt
- C. Use dim lighting to reduce glare during grooming
- D. Store sharp items loosely in a drawer for easy access

Correct Answer: A

Keeping supplies in consistent locations helps a client with visual impairment locate needed items safely and perform ADLs more independently. Frequently rearranging items increases confusion and injury risk. Dim lighting usually makes visual tasks harder, and loose storage of sharp items is unsafe.

2381. A client with severe contractures and limited trunk control needs assistance with dressing each morning. Which nursing action best supports both safety and independence?

- A. Dress the client as quickly as possible without asking for participation

- B.** Ask the client to choose clothing and assist with any steps the client can still perform
- C.** Delay dressing until another staff member is available to complete all care at once
- D.** Use tight-fitting clothing because it is easier to keep aligned during transfers

Correct Answer: B

Even when a client needs extensive help, the nurse should encourage participation in any part of the task the client can do, such as choosing clothing or assisting with simple steps. This preserves autonomy and function. Tight clothing is harder to apply, and excluding the client from the process does not support independence.

2382. A nurse is planning morning care for a client with advanced weakness who can hold a washcloth briefly, becomes fatigued easily, and is embarrassed about needing help.

Which plan is best?

- A.** Schedule all hygiene tasks back-to-back so the client only needs to get started once
- B.** Encourage the client to complete only passive range-of-motion exercises and skip hygiene until stronger
- C.** Offer privacy, let the client wash the areas they can reach, and assist with the remaining care as needed
- D.** Assign full hygiene care to unlicensed assistive personnel without involving the nurse

Correct Answer: C

The nurse should preserve dignity with privacy, encourage the client to do the portions of hygiene that are still possible, and assist with the rest. This balances energy conservation, independence, and emotional comfort. Completing all tasks at once increases fatigue, skipping hygiene is inappropriate, and handing off all care without nursing involvement does not address the client's embarrassment or individualized needs.

Bowel and bladder protocols

2383. A nurse is caring for a client with constipation who has a prescription for a cleansing enema. Which action should the nurse take to promote safe administration of the enema solution?

- A.** Insert the tubing quickly to reduce discomfort

- B.** Raise the solution container high above the rectum to speed flow
- C.** Ask the client to lie on the left side during administration

- D.** Encourage the client to bear down while the solution is instilled

Correct Answer: C

The left side-lying, or Sims', position helps the enema solution flow by gravity into the sigmoid colon and is the best routine position for administration. Inserting tubing quickly can cause trauma, raising the container too high can cause rapid flow and cramping, and bearing down during instillation can increase discomfort and make retention harder.

2384. A client has an indwelling urinary catheter. Which finding requires the nurse's immediate action?

- A.** The drainage bag is hanging below bladder level
- B.** The tubing is looped over the side rail
- C.** Urine in the bag is pale yellow
- D.** The client reports needing help to turn in bed

Correct Answer: B

Catheter tubing looped over a side rail can create dependent loops, kinking, and traction, which can obstruct urine flow and injure the client if the rail is moved. Keeping the bag below bladder level is correct practice, pale yellow urine is generally expected, and needing help to turn is not the priority finding here.

2385. A nurse is reinforcing teaching about preventing constipation for a client with limited mobility. Which client statement shows understanding?

- A.** I should ignore the urge to have a bowel movement until it is more convenient.
- B.** I will try to drink fluids and include fiber in my meals if allowed.
- C.** I should stay in bed as much as possible to conserve energy.
- D.** I will use a laxative every day even if my provider did not recommend it.

Correct Answer: B

Adequate fluid intake, fiber, and activity as tolerated help prevent constipation. Ignoring the urge to defecate can worsen stool retention, prolonged bed rest can decrease bowel motility, and routine laxative use without guidance can lead to dependence or other problems.

2386. A nurse is collecting data from a client who has not voided for 8 hours after surgery and reports lower abdominal pressure. Which action is most appropriate first?

- A. Encourage the client to try voiding and provide privacy
- B. Insert an indwelling urinary catheter immediately
- C. Restrict oral fluids until bladder function returns
- D. Administer a prescribed antidiarrheal medication

Correct Answer: A

Postoperative urinary retention should first be addressed with simple, noninvasive measures such as assisting the client to void, providing privacy, and helping to a normal position if possible. Immediate catheterization may be needed later but is not the first action here without trying basic measures or obtaining further direction. Fluid restriction would not help, and an antidiarrheal is unrelated.

2387. A client with diarrhea has excoriated skin around the perineal area. Which intervention should the nurse implement?

- A. Cleanse the area gently and apply a moisture barrier
- B. Massage the reddened skin firmly after each stool
- C. Limit perineal care to once per shift
- D. Use hot water and vigorous scrubbing to remove stool

Correct Answer: A

Frequent loose stools increase moisture and skin breakdown risk, so gentle cleansing and a moisture barrier are appropriate. Firm massage can damage fragile tissue, infrequent care allows stool to remain on the skin, and hot water with scrubbing increases irritation.

2388. A nurse is preparing to obtain a stool specimen for occult blood testing. Which instruction should the nurse give the client?

- A. Collect the specimen from water in the toilet bowl
- B. Avoid contaminating the stool with urine
- C. Store the specimen without labeling it until transport
- D. Mix the stool with tissue paper to absorb extra moisture

Correct Answer: B

A clean specimen is needed for accurate testing, so the stool should not be contaminated with urine or toilet water. Collecting from the toilet bowl can alter the specimen, unlabeled specimens are unsafe, and tissue paper contaminates the sample.

2389. A client with an indwelling urinary catheter reports a strong urge to void. The nurse notes no urine has drained for 2 hours. Which action should the nurse take first?

- A. Check the tubing for kinks or dependent loops
- B. Remove the catheter immediately
- C. Irrigate the catheter with sterile solution without further assessment
- D. Clamp the catheter for 30 minutes and reassess

Correct Answer: A

A strong urge to void with absent drainage suggests obstruction or poor flow, so the first action is to assess for simple mechanical causes such as kinks, dependent loops, or compression of tubing. Removing the catheter or irrigating it without further assessment may be inappropriate, and clamping the catheter would worsen retention.

2390. A nurse is caring for a client prescribed a bowel training program after repeated episodes of constipation. Which nursing action best supports this plan?

- A. Offer the bedpan at a regular time each day, especially after meals
- B. Encourage the client to skip attempts to defecate on days without an urge
- C. Alternate between antidiarrheals and laxatives to stimulate the bowel
- D. Limit food intake in the morning to reduce bowel activity

Correct Answer: A

Bowel training works best with regular timing, often after meals when the gastrocolic reflex is active. Skipping scheduled attempts reduces consistency, alternating bowel medications without a clear plan is inappropriate, and limiting intake to suppress bowel activity does not support normal elimination.

2391. A nurse is caring for an older adult who has had several episodes of liquid stool. The client now has abdominal distention and reports rectal pressure. Which condition should the nurse suspect?

- A. Normal response to increased fluid intake
- B. Fecal impaction with seepage of liquid stool
- C. Expected effect of immobility on bowel patterns
- D. Complete resolution of constipation

Correct Answer: B

Liquid stool leaking around a hardened mass, along with distention and rectal pressure, suggests fecal impaction. This is a common trap because watery stool may be mistaken for simple diarrhea. The findings do not suggest normal hydration effects, uncomplicated immobility, or improvement.

2392. A postoperative client is having difficulty using a bedpan. Which action by the nurse is most likely to promote urination?

- A. Keep the client flat in bed and ask for repeated attempts every 5 minutes
- B. Provide privacy and assist the client to a more normal voiding position if allowed
- C. Discourage fluid intake until the bladder is less full
- D. Tell the client to wait until the urge to void becomes painful

Correct Answer: B

Privacy and a usual voiding position, such as sitting or standing if permitted, often help initiate urination. Remaining flat can make voiding harder, reducing fluids is not helpful, and waiting for pain increases discomfort and retention risk.

2393. A nurse is emptying a urinary drainage bag for a client with an indwelling catheter. Which action is appropriate?

- A. Allow the drainage spout to touch the collection container so all urine drains out
- B. Empty the bag only when it is completely full to reduce handling
- C. Use a separate, clean measuring container for the client's urine output
- D. Place the drainage bag on the floor while recording the amount

Correct Answer: C

Using a clean collection container assigned for that purpose helps reduce contamination and allows accurate measurement. The drainage spout should not touch the container, the bag should not be allowed to become overly full, and it should never be placed on the floor because that increases infection risk.

2394. A client with neurogenic bladder is on a scheduled toileting program. Which finding best indicates the program is effective?

- A. The client voids at planned intervals with fewer incontinence episodes
- B. The client drinks less fluid during the day to avoid accidents
- C. The client skips scheduled toileting when visitors arrive
- D. The client waits for urgency before attempting to void

Correct Answer: A

The goal of scheduled toileting is improved continence through regular, planned voiding. Decreasing fluid intake may lead to concentrated urine and other problems, skipping the schedule reduces effectiveness, and waiting for urgency defeats the purpose of the program.

2395. A nurse is caring for a client who is receiving a retention enema. After part of the solution is instilled, the client reports cramping. Which action should the nurse take?

- A. Increase the flow rate so the procedure ends sooner
- B. Stop the enema permanently and discard the equipment
- C. Lower the solution container or slow the instillation

D. Ask the client to sit upright and bear down

Correct Answer: C

Cramping during enema administration usually means the solution is flowing too quickly, so lowering the container or slowing the rate is appropriate. Increasing the rate worsens discomfort, stopping permanently may be unnecessary, and upright positioning with bearing down makes retention more difficult.

2396. A nurse is assisting with care for a client who has bowel incontinence. Which intervention is most appropriate to help establish a predictable elimination pattern?

- A.** Offer toileting at regular times, including after meals
- B.** Withhold breakfast to decrease bowel stimulation
- C.** Use incontinence briefs only and avoid scheduled toileting
- D.** Encourage the client to delay defecation until stool is fully formed

Correct Answer: A

Regular toileting, especially after meals, can take advantage of normal bowel reflexes and help create a routine. Skipping meals, relying only on briefs, or delaying elimination does not promote bowel training and may worsen incontinence or constipation.

2397. A nurse notes that a client with a urinary catheter has cloudy urine, fever, and new suprapubic discomfort. What is the nurse's priority action?

- A.** Document the findings at the end of the shift if they continue
- B.** Encourage the client to clamp the catheter periodically
- C.** Report the findings promptly because they may indicate infection
- D.** Raise the drainage bag to promote faster emptying of the bladder

Correct Answer: C

Cloudy urine, fever, and suprapubic discomfort in a catheterized client may indicate a urinary tract infection and require prompt reporting and further evaluation. Delaying documentation without action is unsafe, clamping the catheter can worsen stasis, and raising the bag above bladder level promotes backflow risk rather than drainage.

2398. A nurse is reinforcing teaching for a client who is beginning a bladder training program for urge incontinence. Which instruction should the nurse include?

- A.** Void as soon as the urge is felt every time
- B.** Limit fluid intake to one glass with each meal
- C.** Use scheduled toileting intervals and gradually lengthen the time between voids
- D.** Drink most fluids only at bedtime to reduce daytime accidents

Correct Answer: C

Bladder training uses scheduled voiding and gradual lengthening of the interval between voids to increase bladder capacity and reduce urgency episodes. Voiding immediately with every urge does not retrain the bladder. Severely limiting fluids can concentrate urine and irritate the bladder, and drinking most fluids at bedtime can worsen nighttime urination.

2399. A nurse is caring for a client who is at risk for constipation after several days of opioid use. Which assessment finding is most consistent with constipation rather than diarrhea?

- A.** Three loose stools in one day
- B.** Hypoactive bowel sounds with hard, dry stool
- C.** Sudden vomiting after meals
- D.** Tarry black stool

Correct Answer: B

Constipation is commonly associated with decreased bowel motility and hard, dry stool. Loose stools suggest diarrhea. Vomiting is nonspecific and can occur with many conditions. Tarry black stool suggests possible gastrointestinal bleeding, not simple constipation.

2400. A client is receiving instructions about collecting a clean-catch midstream urine specimen. Which client statement indicates understanding?

- A. I will collect the first urine that comes out into the cup.
- B. I will touch the inside of the cup only if I need to steady it.
- C. I will start voiding, then collect urine in the cup without stopping the stream.
- D. I will save the specimen from the bedpan after I finish voiding.

Correct Answer: C

For a clean-catch midstream specimen, the client should cleanse the area, begin voiding, and then collect the middle portion of the urine stream in the sterile container. The first urine may contain more contaminants. Touching the inside of the cup contaminates it, and urine from a bedpan is not appropriate for a clean specimen.

2401. A nurse is caring for a client with fecal incontinence. Which action is most helpful for protecting skin integrity?

- A. Clean the area promptly after each episode and apply a moisture barrier
- B. Massage reddened areas after each incontinent stool
- C. Use hot water and vigorous scrubbing to remove stool completely
- D. Keep the perineal area open to air and avoid briefs at all times

Correct Answer: A

Prompt cleansing and use of a moisture barrier protect the skin from stool and moisture-related breakdown. Massaging reddened skin can worsen tissue injury. Hot water and vigorous scrubbing increase irritation. Air exposure may be helpful in some situations, but avoiding all containment methods at all times is not the best or most practical primary intervention.

2402. A nurse is preparing to insert a straight urinary catheter for a female client. Which action is appropriate to reduce the risk of introducing infection?

- A. Use clean gloves after opening the catheter kit
- B. Cleanse the perineal area from front to back

C. Place the lubricant on the catheter after insertion begins

D. Allow the catheter tip to touch the client's thigh before insertion

Correct Answer: B

Cleansing from front to back helps reduce transfer of organisms from the anal area toward the urinary meatus. Sterile technique is used for catheter insertion, so clean gloves are not enough once the sterile field is in use. Lubricant should be applied before insertion, and the catheter tip should not touch nonsterile surfaces such as the thigh.

2403. A nurse is caring for a client on a bowel program after a spinal cord injury. Which nursing action best uses the gastrocolic reflex to promote bowel elimination?

- A. Offer a bedpan at bedtime each night
- B. Encourage defecation at a consistent time shortly after breakfast
- C. Insert a rectal suppository at random times to avoid dependence
- D. Restrict oral fluids before the scheduled bowel time

Correct Answer: B

The gastrocolic reflex increases colonic activity after meals, especially breakfast. Scheduling bowel elimination shortly after breakfast helps establish a predictable pattern. Bedtime is usually less effective. Random timing works against training, and restricting fluids can worsen constipation.

2404. A nurse is caring for a client who has an indwelling urinary catheter. To decrease the risk of catheter-associated infection, which action should the nurse take?

- A. Disconnect the tubing daily to rinse the catheter with sterile water
- B. Keep the drainage bag above the level of the bladder during ambulation
- C. Maintain a closed drainage system and keep the collection bag below bladder level
- D. Clamp the catheter for 2 hours each shift to train the bladder

Correct Answer: C

A closed drainage system with the bag below bladder level reduces reflux and helps prevent infection. Disconnecting the system increases contamination risk. Keeping the bag above the bladder promotes backflow. Routine clamping is not used to prevent infection and may increase urinary stasis.

2405. A nurse is reinforcing teaching for a client with a new colostomy who is concerned about unpredictable bowel output. Which suggestion is most appropriate to help the client develop a more regular elimination pattern?

- A. Skip breakfast to reduce stool formation
- B. Eat meals on a regular schedule each day
- C. Avoid drinking fluids with meals permanently
- D. Take a laxative every evening unless stool is watery

Correct Answer: B

Regular mealtimes can help promote a more predictable bowel pattern. Skipping meals does not support healthy bowel regulation. Permanent fluid restriction is not appropriate. Routine self-directed laxative use can cause complications and is not the best general recommendation.

2406. A nurse is caring for a postoperative client who reports feeling the need to urinate but is unable to void. Which action should the nurse take first to promote voiding before proceeding to more invasive measures?

- A. Run warm water over the client's hands
- B. Insert an indwelling urinary catheter
- C. Restrict oral fluids for 4 hours
- D. Administer a cleansing enema

Correct Answer: A

Simple noninvasive measures such as running water, providing privacy, and assisting into a normal position are appropriate first steps to stimulate urination. Catheterization is invasive and not the first action when conservative measures may help. Restricting fluids does not promote voiding, and an enema is unrelated to urinary retention.

2407. A nurse is reinforcing teaching for a client who has chronic constipation. Which breakfast choice would best support the client's bowel program?

- A. Scrambled eggs and white toast
- B. Plain yogurt and cheese
- C. Oatmeal, pear slices, and water
- D. Pancakes with bacon and coffee only

Correct Answer: C

Oatmeal and pear slices provide fiber, and water supports stool softening and bowel movement. The other meals are lower in fiber and are less helpful for preventing constipation. A common trap is choosing coffee because it may stimulate some people, but without adequate fiber and fluid it is not the best answer.

2408. A nurse is assessing a client with a newly inserted indwelling urinary catheter. Which finding indicates the catheter is functioning properly?

- A. The client reports severe bladder spasms and no urine is present in the tubing
- B. Urine drains continuously into the collection bag and the tubing is free of dependent loops
- C. The drainage bag is clipped to the side rail to keep it off the floor
- D. Sediment is visible and urine has a strong foul odor

Correct Answer: B

Continuous urine drainage with unobstructed tubing indicates proper catheter function. Severe bladder spasms with no urine may suggest obstruction. Attaching the bag to the side rail is unsafe because the rail moves and can pull on the catheter; the bag should be secured to the bed frame. Sediment and foul odor may indicate infection or concentrated urine, not normal function.

2409. A nurse is administering a large-volume enema to an adult client. Which observation requires the nurse to stop the procedure and assess the client immediately?

- A. The client says the fluid feels cool
- B. The client passes some flatus during the procedure
- C. The client reports dizziness and becomes pale

D. The client asks to use the bathroom right away

Correct Answer: C

Dizziness and pallor during enema administration can indicate a vagal response, which requires the nurse to stop the procedure and assess the client immediately. Feeling cool or wanting to evacuate can occur during an enema. Passing flatus is not the most concerning finding.

2410. A nurse is caring for an older adult who has a history of constipation. The client suddenly begins passing small amounts of liquid stool several times during the shift and has increasing abdominal firmness. Which action is most appropriate?

- A.** Document improvement and continue the current bowel plan
- B.** Give an antidiarrheal medication as prescribed PRN
- C.** Report the findings because they may indicate fecal impaction with seepage
- D.** Increase the rate of tube feeding to replace losses

Correct Answer: C

Small amounts of liquid stool with abdominal firmness in a client with constipation history can suggest fecal impaction with leakage around the stool mass. This requires further assessment and reporting. Treating it as simple diarrhea is a common trap and could worsen the problem, especially if an antidiarrheal is given.

2411. A nurse is caring for a client with benign prostatic enlargement who has a prescription for a condom catheter for nighttime incontinence. Which action is most important for the nurse to take?

- A.** Apply the sheath tightly to prevent leakage
- B.** Leave a small space between the catheter tip and the glans penis
- C.** Secure the drainage tubing to the side rail
- D.** Remove the device only when drainage stops

Correct Answer: B

A small space between the catheter tip and the glans penis helps prevent pressure injury and allows urine to drain. Applying the sheath tightly can impair circulation and damage skin. Tubing should not be secured to the side rail. The device requires regular removal and skin assessment, not just when drainage stops.

2412. A nurse is caring for a client who had a stroke and is on a prompted-voiding program. Which nursing action best follows this protocol?

- A.** Wait for the client to request toileting to promote independence
- B.** Offer toileting at scheduled intervals and praise continent voiding
- C.** Keep an indwelling catheter in place during retraining
- D.** Provide fluids only after the client has remained dry for 4 hours

Correct Answer: B

Prompted voiding involves regularly offering toileting, asking about the need to void, and positively reinforcing dryness or successful toileting. Waiting for the client to ask may not work if the client has cognitive or communication difficulty. Routine catheter use interferes with retraining, and limiting fluids is not an appropriate strategy.

2413. A nurse is caring for a client who has a bowel ostomy and is scheduled for irrigation to promote regular emptying. Which outcome indicates the irrigation was effective?

- A.** The client has a predictable return of stool after irrigation
- B.** The stoma becomes pale during the procedure
- C.** The client reports cramping that continues after the fluid is returned
- D.** The pouch remains empty for several days after irrigation

Correct Answer: A

A predictable return of stool after irrigation indicates the bowel is responding as intended and helps establish a regular elimination pattern. A pale stoma is abnormal and may suggest impaired circulation. Ongoing cramping is not an expected sign of success. No stool output for several days is not a normal goal and could indicate constipation or obstruction.

2414. A nurse is reinforcing teaching for a client who is trying to prevent constipation without medication. Which client statement shows correct understanding?

- A. I should ignore the urge to have a bowel movement until it is more convenient.
- B. I will try to set aside time each day for bowel elimination.
- C. I should limit fluid intake so my abdomen does not feel full.
- D. I will stay in bed more to conserve energy.

Correct Answer: B

Scheduling regular time each day for elimination supports normal bowel habits and can help prevent constipation. Ignoring the urge to defecate can worsen stool retention. Limiting fluids and decreasing activity both increase the risk of constipation rather than prevent it.

2415. A client has an indwelling urinary catheter removed at 0800. At 1400, the client has not voided and reports lower abdominal pressure. Which action should the nurse take first?

- A. Encourage the client to drink coffee
- B. Obtain a bladder scan
- C. Prepare for immediate reinsertion of the catheter
- D. Document that delayed voiding is expected

Correct Answer: B

A bladder scan is the best first action because it provides objective information about urinary retention and guides the next step. Reinsertion may be needed later, but assessment comes first. Coffee is not the priority intervention. Delayed voiding with abdominal pressure should not simply be documented as expected without further assessment.

2416. A nurse is caring for a client receiving a cleansing enema. Which position should the nurse place the client in to promote flow of the solution into the colon?

- A. Supine
- B. Prone
- C. Left lateral
- D. High-Fowler's

Correct Answer: C

The left lateral position is preferred for enema administration because it follows the natural direction of the sigmoid colon and allows the solution to flow more easily. Supine, prone, and High-Fowler's positions are less effective for this purpose.

2417. A nurse is caring for a client with reflex urinary incontinence related to neurologic impairment. Which intervention is most appropriate to include in the plan of care?

- A. Offer toileting at scheduled intervals
- B. Restrict oral fluids after breakfast
- C. Insert an indwelling catheter permanently
- D. Ask the client to wait to void until the bladder feels full

Correct Answer: A

Scheduled toileting is appropriate for clients with neurologic impairment and reflex incontinence because it helps reduce episodes of involuntary urine loss. Routine fluid restriction is not the best approach and can cause dehydration. Permanent indwelling catheterization is generally not the first-line measure for long-term management. Waiting for the sensation of a full bladder is not useful when bladder signaling is impaired.

2418. A client with a bowel-training program following spinal cord injury develops headache, facial flushing, and sweating during digital rectal stimulation. Which action should the nurse take first?

- A. Continue the stimulation until stool is expelled
- B. Stop the procedure and assess the client immediately
- C. Document the findings as an expected response
- D. Give a second rectal stimulant suppository

Correct Answer: B

Headache, flushing, and sweating during rectal stimulation in a client with spinal cord injury can indicate autonomic dysreflexia, which requires immediate attention. The nurse should stop the triggering procedure and assess the client right away. Continuing the procedure, documenting it as expected, or adding another stimulant could worsen the problem.

2419. A nurse is reviewing a 24-hour bladder diary for a client with stress incontinence. Which finding is most important to report because it can interfere with success of the bladder program?

- A.** The client voids every 3 to 4 hours during the day
- B.** The client drinks most fluids before noon
- C.** The client has leakage episodes when coughing and also avoids pelvic floor exercises
- D.** The client wakes once during the night to urinate

Correct Answer: C

Stress incontinence is often triggered by increased abdominal pressure, such as coughing, and pelvic floor exercises are an important supportive measure. Avoiding those exercises can interfere with improvement and should be addressed. Voiding every 3 to 4 hours, drinking more earlier in the day, and waking once at night are not the most concerning findings in this scenario.

Sleep and rest

2420. A client reports difficulty sleeping in the hospital because of noise in the hallway. Which nursing action is most appropriate to promote rest before requesting a sleep medication?

- A.** Offer coffee with the evening snack
- B.** Cluster nighttime care to reduce repeated interruptions
- C.** Encourage the client to watch television until sleepy
- D.** Keep the room light on to reduce disorientation

Correct Answer: B

Clustering nighttime care helps reduce repeated awakenings and supports longer periods of uninterrupted sleep. Coffee can worsen wakefulness, television may stimulate the client, and keeping lights on can interfere with normal sleep cues.

2421. An older adult client says, "I keep waking up at night and then nap most of the afternoon." Which response by the nurse is best?

- A.** "Long afternoon naps can make nighttime sleep harder, so limiting naps may help."
- B.** "Sleep needs disappear with age, so nighttime waking is expected."
- C.** "You should stay in bed longer in the morning to catch up on sleep."
- D.** "Drinking more fluids at bedtime often helps people sleep better."

Correct Answer: A

Long daytime naps can reduce sleep drive at night, so limiting naps is an appropriate sleep-hygiene teaching point. Aging may change sleep patterns, but sleep is still needed. Staying in bed longer and drinking more fluids at bedtime can worsen sleep quality.

2422. A postoperative client says pain is preventing sleep. Which intervention should the nurse implement first?

- A.** Turn on the television to distract the client
- B.** Assist the client to ambulate in the hallway
- C.** Assess the client's pain characteristics and intensity
- D.** Close the door and dim the lights

Correct Answer: C

When pain is interfering with sleep, the nurse should first assess the pain to guide appropriate intervention. Environmental changes may help but do not address the cause. Ambulation may be useful later but is not the first step when pain is the reported problem.

2423. Which client statement indicates a need for further teaching about improving sleep at home?

- A.** "I will keep my bedroom as quiet and dark as possible."

- B. "I will avoid large meals right before bedtime."
- C. "I will have several caffeinated drinks in the evening so I can stay relaxed."
- D. "I will try to go to bed at about the same time each night."

Correct Answer: C

Caffeinated drinks in the evening can interfere with falling asleep and staying asleep, so this statement shows misunderstanding. A quiet, dark room, avoiding heavy bedtime meals, and maintaining a regular bedtime routine all support healthy sleep.

2424. A client on bed rest has not slept well for 2 nights. Which nursing intervention is most likely to promote rest?

- A. Provide range-of-motion exercises and hygiene before bedtime
- B. Schedule vital signs and linen change at separate times overnight
- C. Encourage the client to drink cola with the evening medication
- D. Ask visitors to remain longer to reduce loneliness

Correct Answer: A

For a client on bed rest, bedtime hygiene and range-of-motion exercises can increase comfort and relaxation, helping promote sleep. Separating care tasks increases interruptions. Cola contains caffeine, and extending visitor time may increase stimulation.

2425. The nurse is caring for four clients at 2200. Which client should the nurse assess first because the sleep problem may reflect a breathing-related safety concern?

- A. A client who asks for an extra blanket because the room feels cool
- B. A client who reports loud snoring and waking up gasping for air
- C. A client who says the roommate's television is annoying
- D. A client who wants a back rub before bedtime

Correct Answer: B

Waking up gasping for air with loud snoring suggests a possible breathing-related sleep problem that requires prompt assessment because oxygenation and airway issues take priority. The other concerns affect comfort but are not as urgent.

2426. A hospitalized client says, "I can never sleep because someone is always coming into my room." Which nursing action best addresses this concern?

- A. Explain that sleep loss is unavoidable in the hospital
- B. Ask the provider for a sedative each night
- C. Coordinate care so treatments and assessments are done together when possible
- D. Encourage the client to stay awake during the day and all evening

Correct Answer: C

Coordinating care reduces unnecessary awakenings and supports uninterrupted sleep. Telling the client sleep loss is unavoidable is dismissive. Requesting a sedative may not be necessary as a first measure, and keeping the client awake all day and evening is not an appropriate blanket strategy.

2427. A client recovering from illness says, "I feel exhausted, but I just lie awake thinking." Which intervention is most appropriate for the nurse to suggest?

- A. Use relaxation techniques such as slow breathing before sleep
- B. Exercise vigorously right before getting into bed
- C. Drink a large amount of fluid at bedtime
- D. Check the clock repeatedly to track how long it takes to fall asleep

Correct Answer: A

Relaxation techniques such as slow breathing can reduce mental tension and support sleep onset. Vigorous exercise right before bed may be stimulating, heavy bedtime fluids can increase nighttime waking, and clock-watching can increase anxiety about not sleeping.

2428. The nurse notes that a client who usually sleeps at night has been receiving a diuretic at 2100 and is waking frequently to urinate. Which nursing action is most appropriate?

- A. Encourage the client to stop drinking fluids for the rest of the day
- B. Withhold the medication until morning without further action
- C. Review the medication schedule and discuss a possible earlier administration time with the provider
- D. Tell the client that interrupted sleep is expected and cannot be changed

Correct Answer: C

A diuretic given late in the evening can disrupt sleep by increasing nighttime urination. Reviewing the timing and discussing an earlier administration time is appropriate. The nurse should not independently withhold the medication, and severe fluid restriction or dismissing the concern is not appropriate.

2429. A nurse is developing a bedtime routine for a long-term care resident with confusion who becomes more restless at night. Which approach is best to support sleep?

- A. Change the routine each evening to prevent boredom
- B. Provide a consistent, calm bedtime routine with reduced stimulation
- C. Keep the resident awake all day with no rest periods at all
- D. Move the resident to a brightly lit day room until midnight

Correct Answer: B

A consistent, calm bedtime routine with reduced stimulation helps promote orientation, comfort, and sleep. Frequent routine changes can worsen confusion. Keeping the resident awake all day or placing the resident in a bright area late at night may increase agitation and interfere with sleep.

2430. The nurse is caring for a client who works night shifts and sleeps during the day. Which statement by the nurse is most appropriate?

- A. "You should try to sleep only at night, even on workdays."
- B. "Using blackout curtains and reducing daytime noise may improve your sleep."

C. "Eating a heavy meal before daytime sleep will help you stay asleep."

D. "Caffeine right before bedtime can help your body settle down."

Correct Answer: B

For a client sleeping during the day, reducing light and noise can improve sleep quality. A heavy meal or caffeine before sleep may interfere with rest. Advising the client to sleep only at night ignores the realities of shift work.

2431. A client who has insomnia asks the nurse, "Why do you keep asking about my evening habits?" Which explanation is best?

- A. "Sleep problems are usually caused only by room temperature."
- B. "Evening routines, food, drinks, and activity can affect your ability to sleep."
- C. "The cause of insomnia cannot usually be identified."
- D. "Only prescription medications determine whether a person sleeps well."

Correct Answer: B

Evening habits such as caffeine intake, exercise timing, meals, and stimulation can strongly affect sleep quality, so assessing them is appropriate. The other responses are overly narrow or inaccurate and do not support individualized sleep assessment.

2432. A client with chronic pain says, "I sleep for short periods and wake up stiff and hurting." Which nursing intervention is likely to best promote rest?

- A. Reposition the client for comfort and use supportive pillows
- B. Encourage the client to avoid any movement before bedtime
- C. Offer a large caffeinated beverage in the evening
- D. Leave the client in one position overnight to avoid disturbance

Correct Answer: A

Repositioning and using supportive pillows can reduce pressure, stiffness, and discomfort, which may improve sleep in a client with chronic pain. Avoiding all movement or remaining in one position overnight can worsen stiffness and discomfort. Caffeine can interfere with sleep.

2433. The nurse is evaluating whether a sleep-promotion plan has been effective for a hospitalized client. Which finding best shows improvement?

- A. The client asks for the hallway lights to remain on overnight
- B. The client reports sleeping 5 uninterrupted hours and feeling more rested in the morning
- C. The client states, "I was awake most of the night but stayed in bed quietly."
- D. The client says, "I needed three naps today because I was so tired."

Correct Answer: B

Sleeping for an uninterrupted period and feeling more rested are the clearest signs that the plan improved sleep quality. Staying in bed while awake and needing repeated naps suggest persistent sleep problems. Requesting more light does not demonstrate improved rest.

2434. A client is receiving care after surgery. Which nighttime nursing action best balances safety needs with the client's need for sleep?

- A. Wake the client every hour for routine conversation and reassurance
- B. Delay all assessments until morning so the client is never disturbed
- C. Perform necessary assessments efficiently and combine them with other care when possible
- D. Encourage family members to stay overnight and interact with the client frequently

Correct Answer: C

Postoperative clients still need necessary monitoring, but care should be done efficiently and combined when possible to reduce sleep disruption. Hourly conversation is unnecessary, delaying all assessments could compromise safety, and frequent overnight interaction from family may disturb rest.

2435. A client says, "I drink two glasses of wine every night because it helps me fall asleep faster." Which response by the nurse is best?

- A. Alcohol is a good short-term sleep aid if it is used in small amounts
- B. Alcohol may make you sleepy at first, but it often disrupts sleep later in the night
- C. You should replace alcohol with coffee earlier in the evening to stay awake longer

D. The safest plan is to stay in bed longer the next morning after drinking alcohol

Correct Answer: B

Alcohol can cause initial drowsiness, but it commonly fragments sleep and reduces restful sleep later in the night. Choice A is incorrect because alcohol is not a good sleep-promoting strategy. Choice C would worsen sleep. Choice D does not address the cause of poor sleep quality.

2436. A nurse is planning evening care for a client who has difficulty falling asleep. Which action is most appropriate to include?

- A. Offer a large meal at bedtime
- B. Encourage vigorous exercise right before bed
- C. Provide a warm caffeine-free drink if allowed
- D. Keep the television on to create background noise

Correct Answer: C

A warm caffeine-free drink can be comforting and may help promote relaxation before sleep. A large bedtime meal may cause discomfort, vigorous exercise right before bed can increase alertness, and television may stimulate the client rather than support sleep.

2437. A client is prescribed complete bed rest for 3 days. To help promote normal sleep at night, which daytime nursing action is best?

- A. Perform all care while the client is asleep so the client can conserve energy
- B. Encourage periods of activity and regular hygiene during waking hours as tolerated
- C. Keep the room dark throughout the day so the client stays calm
- D. Discourage all visitors to prevent daytime stimulation

Correct Answer: B

Maintaining daytime activity, personal care, and a more normal day-night pattern supports better nighttime sleep. Choice A disrupts rest and removes normal daytime structure. Choice C can blur day-night cues. Choice D is unnecessarily restrictive and does not best promote a healthy sleep pattern.

2438. An adult client tells the nurse, "I go to bed at different times every night depending on how tired I feel." Which teaching should the nurse provide first?

- A. Try to go to bed and get up at about the same time each day
- B. Spend extra time in bed on weekends to catch up on lost sleep
- C. Use your phone in bed until you feel sleepy
- D. Take a long nap late in the evening if you did not sleep well the night before

Correct Answer: A

A regular sleep-wake schedule is a basic sleep hygiene measure that helps regulate the body's sleep pattern. Choice B can further disrupt routine. Choice C may increase alertness. Choice D can make nighttime sleep more difficult.

2439. The nurse is reviewing a client's medications after the client reports difficulty sleeping. Which medication timing should the nurse question as most likely to interfere with sleep?

- A. A stimulant medication scheduled at bedtime
- B. A stool softener scheduled with dinner
- C. A multivitamin scheduled at noon
- D. An antihypertensive scheduled in the morning

Correct Answer: A

A stimulant given at bedtime is likely to interfere with falling asleep and staying asleep. The other medication timings are not the most likely direct cause of insomnia in this scenario. This question focuses on recognizing a medication-related contributor to poor sleep.

2440. A parent asks the nurse how to help a hospitalized preschool-age child sleep better at night. Which suggestion is best?

- A. Let the child stay up much later than usual to become overtired
- B. Keep the room brightly lit so the child can see the surroundings
- C. Follow a familiar bedtime routine such as washing, a story, and a comfort item
- D. Offer sugary snacks before bed so the child will feel satisfied and sleepy

Correct Answer: C

Young children often sleep better with familiar routines and comfort measures that reduce anxiety and support normal bedtime cues. Overtiring a child can increase irritability, bright light can interfere with sleep, and sugary snacks may increase stimulation rather than promote rest.

2441. A client says, "I spend hours lying in bed awake and frustrated." Which instruction is most appropriate for the nurse to include in sleep teaching?

- A. Stay in bed until you eventually fall asleep, even if it takes several hours
- B. Use the bed for watching shows and checking messages so you feel more relaxed there
- C. If you cannot sleep after a while, get up for a quiet activity and return when sleepy
- D. Try to make up for poor sleep by taking several naps during the day

Correct Answer: C

If a person lies awake for a prolonged period, getting up for a quiet, nonstimulating activity and returning to bed when sleepy can help break the cycle of frustration. Choice A reinforces wakefulness in bed. Choice B weakens the connection between bed and sleep. Choice D can worsen nighttime insomnia.

2442. A nurse is caring for a client who reports poor sleep for the past week. Which assessment finding is most important to recognize as a possible sign of a sleep-related breathing problem?

- A. The client prefers an extra blanket at night
- B. The client reports loud snoring and waking with choking sensations
- C. The client drinks water at the bedside before sleep
- D. The client reads for 20 minutes before bedtime

Correct Answer: B

Loud snoring and waking with choking sensations can suggest sleep-disordered breathing and require further assessment. The other findings are not the most concerning indicators of a breathing-related sleep problem.

2443. A client in a rehabilitation unit says, "I am too tired to participate in therapy, so I nap for several hours every afternoon." The client also reports trouble sleeping at night. Which nursing suggestion is best?

- A. Avoid all daytime rest periods completely
- B. Take a short rest earlier in the day instead of a long afternoon nap
- C. Move bedtime later and later until sleep returns naturally
- D. Drink caffeinated beverages in the evening to avoid napping

Correct Answer: B

A short earlier rest may reduce fatigue without interfering as much with nighttime sleep. Choice A is too absolute and may be unrealistic. Choice C can worsen schedule irregularity. Choice D would likely make nighttime sleep worse.

2444. The nurse enters a client's room at 0300 and finds the client awake, disoriented, and trying to climb out of bed, stating, "I need to get ready for work." Which action should the nurse take first?

- A. Turn on all the room lights and begin morning care
- B. Help the client ambulate in the hallway until fatigue occurs
- C. Reorient the client calmly and ensure immediate safety measures are in place
- D. Tell the client to go back to sleep and leave the room to reduce stimulation

Correct Answer: C

The priority is safety. A disoriented client trying to get out of bed is at risk for falling, so the nurse should first calmly reorient the client and protect the client from injury. Choice A may increase confusion. Choice B is unsafe before reorientation and assessment. Choice D ignores an immediate safety concern.

2445. A client says, "I want to sleep better, so I started smoking a few cigarettes before bed to relax." Which response by the nurse is best?

- A. Nicotine can stimulate the body and make it harder to sleep
- B. Smoking before bed is acceptable if it helps you feel calm
- C. Try smoking only in the bedroom so your body connects that room with sleep
- D. Increase the number of cigarettes gradually until you find the amount that works

Correct Answer: A

Nicotine is a stimulant and can interfere with sleep onset and sleep quality. The other options are unsafe or reinforce an ineffective and harmful habit.

2446. A nurse is planning care for a hospitalized client who must have vital signs, dressing assessment, and oral medications during the night shift. Which plan best promotes sleep while still meeting care needs?

- A. Perform each task separately at different times so the client is disturbed only briefly each time
- B. Ask the client to stay awake until all possible nighttime tasks can be completed together
- C. Cluster necessary care activities to reduce the number of times the client is awakened
- D. Delay all ordered care until morning so the client can sleep through the night

Correct Answer: C

Clustering care helps reduce repeated sleep interruptions while still providing needed treatment and assessment. Choice A increases awakenings. Choice B is not client-centered and may reduce rest. Choice D is unsafe because it delays prescribed care.

2447. A client reports chronic insomnia. Which information from the nurse best explains why daily exercise is usually recommended earlier in the day rather than just before bedtime?

- A. Exercise can increase alertness and body stimulation close to bedtime
- B. Exercise always causes muscle pain that prevents sleep
- C. Exercise lowers oxygen needs during sleep too much
- D. Exercise prevents the body from entering deep sleep later that night

Correct Answer: A

Exercise is generally helpful for sleep overall, but vigorous activity close to bedtime may increase stimulation and delay sleep onset. Choice B is too broad, and choices C and D are inaccurate explanations.

2448. A hospitalized client says, "I slept badly last night, but I don't want to take medicine if there is another option." Which nurse action is best?

- A. Explain that medication is always the most effective treatment for insomnia in the hospital
- B. Offer nonpharmacologic measures such as relaxation, positioning, and reducing stimuli
- C. Advise the client to ignore the problem because one poor night of sleep is not important
- D. Recommend drinking caffeinated soda to prevent daytime fatigue

Correct Answer: B

When appropriate, nonpharmacologic measures are a reasonable first approach to promote sleep and rest. Relaxation, comfort measures, and reduction of stimuli can improve sleep without medication. Choice A is too absolute. Choice C dismisses the concern. Choice D may worsen sleep problems.

2449. The nurse is teaching an older adult about age-related sleep changes. Which client statement shows correct understanding?

- A. It is normal to need no sleep at all as people age
- B. Older adults always require sleeping pills because sleep cannot be improved naturally
- C. Sleep may become lighter and more interrupted with age, so good sleep habits are still important
- D. Frequent nighttime awakening in older adults never needs further assessment

Correct Answer: C

Sleep in older adults may become lighter and more fragmented, but healthy sleep habits remain important and problems still require assessment when indicated. Choice A is false. Choice B is inappropriate and overly absolute. Choice D is unsafe because some nighttime waking may reflect treatable problems.

2450. A nurse is caring for a hospitalized client who says, "The hallway noise keeps waking me up." Which nursing action is most appropriate to promote the client's rest?

- A. Offer coffee with the evening snack
- B. Cluster nighttime care to reduce room interruptions

- C. Encourage the client to watch television until sleepy
- D. Keep the door open so staff can observe the client more often

Correct Answer: B

Clustering nighttime care helps reduce repeated awakenings and supports longer, more restful sleep. Coffee can interfere with sleep, television may overstimulate the client, and leaving the door open usually increases noise and interruptions rather than reducing them.

2451. A client tells the nurse, "I wake up every morning with a dry mouth and a headache, and my partner says I snore loudly." Which follow-up information is most important for the nurse to obtain?

- A. Whether the client prefers a firm or soft mattress
- B. How many pillows the client uses at night
- C. Whether the partner has noticed breathing pauses during sleep
- D. What television programs the client watches before bed

Correct Answer: C

Loud snoring, morning headache, and dry mouth can suggest obstructive sleep apnea. Asking whether breathing pauses have been observed helps the nurse further assess for a sleep-related breathing problem. Mattress preference, pillow number, and television choices are less important than screening for possible apnea.

2452. The nurse is teaching a client about measures to improve sleep at home. Which client statement indicates a need for further teaching?

- A. I will avoid large meals right before bedtime.
- B. I will use my bed for sleep instead of doing paperwork there.
- C. I will have several caffeinated sodas in the evening so I can relax later.
- D. I will try to keep my bedroom dark and comfortable.

Correct Answer: C

Caffeinated drinks in the evening can delay sleep onset and reduce sleep quality, so this statement shows the client needs more teaching. The other statements reflect helpful sleep-hygiene practices: avoiding heavy late meals, associating the bed with sleep, and maintaining a restful environment.

2453. A postoperative client reports, "I barely slept last night because my pain kept waking me up." Which nursing action best supports sleep while addressing the likely cause?

- A.** Encourage the client to stay awake during the evening to become more tired
- B.** Assess the client's pain and provide prescribed pain relief before bedtime
- C.** Suggest drinking extra fluids at bedtime to prevent dry mouth
- D.** Turn on the room lights during nighttime assessments

Correct Answer: B

Pain is the likely reason the client is not sleeping, so the best action is to assess the pain and address it with prescribed relief measures before bedtime. Simply trying to keep the client awake does not treat the cause. Extra fluids at bedtime may lead to nighttime urination, and bright lights can further disrupt sleep.

2454. An older adult in a long-term care facility reports waking frequently during the night. Which nursing intervention is most appropriate to reduce nighttime awakenings?

- A.** Offer most of the client's daily fluids at bedtime
- B.** Schedule routine toileting assistance before sleep
- C.** Encourage a late-evening aerobic exercise session
- D.** Provide a bright light in the room throughout the night

Correct Answer: B

Providing toileting assistance before sleep can reduce awakenings related to the need to urinate, which is a common reason for fragmented sleep in older adults. Offering fluids at bedtime may worsen nocturia. Late vigorous exercise may stimulate the client, and bright light can interfere with normal sleep cues.

2455. A nurse is caring for a client who has worked rotating night shifts for several years and reports persistent difficulty sleeping during the day. Which nursing teaching is most appropriate?

- A.** Drink caffeinated beverages just before going to bed to prevent waking too early
- B.** Maintain a quiet, dark sleep environment and follow a consistent sleep routine as much as possible
- C.** Change sleep times every day so the body does not become dependent on one schedule
- D.** Use alcohol after each shift to fall asleep more quickly

Correct Answer: B

Shift work disrupts normal sleep-wake rhythms, so teaching should focus on protecting daytime sleep with a dark, quiet environment and a routine schedule whenever possible. Caffeine before sleep and alcohol both interfere with sleep quality, and changing sleep times daily makes circadian disruption worse rather than better.

Skin and tissue integrity

2456. A practical nurse is caring for a client who has been on bed rest for several days. Which action is most effective for helping prevent pressure injury formation?

- A.** Massage reddened areas over bony prominences each shift
- B.** Reposition the client at regular intervals and keep pressure off bony areas
- C.** Place the client in a high-Fowler position most of the day
- D.** Use a donut-shaped cushion under the sacrum

Correct Answer: B

Regular repositioning and pressure relief over bony prominences are key measures to prevent pressure injuries. Massaging reddened skin can damage fragile tissue, prolonged high-Fowler positioning increases shear, and donut-shaped cushions can reduce blood flow to surrounding tissue.

2457. A client has a new reddened area over the coccyx. The skin is intact and does not blanch when pressed. How should the nurse classify this finding?

- A. Stage 1 pressure injury
- B. Stage 2 pressure injury
- C. Stage 3 pressure injury
- D. Deep tissue pressure injury

Correct Answer: A

A stage 1 pressure injury involves intact skin with nonblanchable erythema. Stage 2 involves partial-thickness skin loss or blistering, stage 3 extends into subcutaneous tissue, and deep tissue injury usually appears as persistent deep red, maroon, or purple discoloration rather than simple nonblanchable redness.

2458. When moving an immobile client up in bed, which technique best reduces shear and friction to the client's skin?

- A. Pull the client quickly across the drawsheet
- B. Lift the client with assistance using a drawsheet
- C. Raise the head of the bed to 90 degrees before repositioning
- D. Slide the client one side at a time while the heels remain planted

Correct Answer: B

Lifting the client with help using a drawsheet reduces dragging across bed linens, which lowers shear and friction. Pulling or sliding the client across the bed increases skin trauma, and a high head-of-bed position can worsen shear.

2459. Which finding places a client at greatest risk for impaired skin integrity?

- A. Walks in the hall twice a day
- B. Requires frequent turning because of paralysis
- C. Requests an extra blanket at night
- D. Uses reading glasses for near vision

Correct Answer: B

Paralysis with dependence for turning greatly increases pressure injury risk because prolonged pressure reduces tissue perfusion. The other findings are not major direct risk factors for skin breakdown.

2460. A nurse is caring for an older client who is incontinent of urine. Which intervention is most appropriate to help maintain skin integrity in the perineal area?

- A. Cleanse the skin promptly after episodes of incontinence and apply a moisture barrier
- B. Dust the area with powder after each voiding episode
- C. Limit oral fluids so the skin stays drier
- D. Scrub the skin vigorously with soap and a washcloth

Correct Answer: A

Prompt cleansing and use of a moisture barrier protect skin from prolonged exposure to urine. Powder can cake and irritate skin, fluid restriction is not an appropriate skin-care strategy, and vigorous scrubbing can cause skin damage.

2461. A client with a pressure injury has poor appetite and low protein intake. Which dietary change would best support wound healing?

- A. Increase foods rich in protein
- B. Switch to a clear liquid diet
- C. Reduce calories to avoid weight gain
- D. Avoid foods containing iron

Correct Answer: A

Protein is essential for tissue repair and wound healing. Clear liquids do not provide enough nutrition for healing, calorie restriction can impair recovery, and avoiding iron does not support skin repair.

2462. During a skin assessment, the nurse notes a shallow open area with a pink wound bed on the client's heel. No slough is present. Which stage of pressure injury is this most consistent with?

- A. Stage 1
- B. Stage 2
- C. Stage 3
- D. Unstageable

Correct Answer: B

A stage 2 pressure injury involves partial-thickness skin loss with exposed dermis and may appear as a shallow open ulcer with a pink or red wound bed. Stage 1 skin is still intact, stage 3 extends deeper into subcutaneous tissue, and unstageable wounds are obscured by slough or eschar.

2463. A client is at risk for pressure injuries. Which nursing action is best for protecting the heels?

- A. Place the heels directly on a folded blanket for support
- B. Elevate the heels off the bed surface
- C. Rub lotion into the heels every 2 hours
- D. Apply warm packs to improve circulation

Correct Answer: B

Floating the heels off the bed removes pressure from a common injury site. Resting heels on a folded blanket may not fully relieve pressure, rubbing can damage vulnerable tissue, and warm packs are not a standard prevention measure and may injure skin.

2464. The nurse is teaching an unlicensed assistive person about skin care for an immobile client. Which observation should the nurse identify as needing immediate reporting?

- A. Dry skin on the forearms
- B. Faint pink color that blanches on the elbow
- C. A dark red area over the sacrum that does not fade after pressure is removed
- D. Mild flaking of skin on the lower legs

Correct Answer: C

A persistent dark red, nonblanching area over a pressure point suggests early tissue damage and requires prompt nursing assessment. Dry skin, blanchable redness, and mild flaking are less urgent findings.

2465. A client is placed in semi-Fowler position for tube feeding. To help reduce shear injury while the client remains in bed, which additional action is most appropriate?

- A. Keep the client from sliding downward in bed
- B. Massage the shoulders and hips every hour
- C. Loosen the fitted sheet under the client
- D. Position the knees flat with no support

Correct Answer: A

Shear occurs when the skeleton moves but the skin stays in place, such as when a client slides down in bed. Preventing sliding is the key action. Massage over pressure areas is not recommended, and the other choices do not effectively address shear.

2466. A client has a pressure injury covered with thick tan slough, and the base of the wound cannot be seen. How should the nurse document this wound?

- A. Stage 2 pressure injury
- B. Stage 3 pressure injury
- C. Stage 4 pressure injury
- D. Unstageable pressure injury

Correct Answer: D

If slough or eschar obscures the wound base, the depth cannot be determined, so the wound is documented as unstageable. The common trap is trying to assign a deeper stage without visualizing the wound bed.

2467. A client returns from surgery and has limited sensation in the lower extremities from regional anesthesia. Which nursing action is the priority for protecting skin and tissue integrity during recovery?

- A. Encourage the client to ambulate as soon as sensation begins to return
- B. Assess pressure areas frequently and reposition the client as needed
- C. Place a heating pad on the feet to improve comfort
- D. Ask the client to report discomfort from pressure points

Correct Answer: B

With decreased sensation, the client may not feel pressure or pain that signals tissue injury, so frequent assessment and repositioning are the priority. Asking the client to report discomfort is unreliable while sensation is impaired, and heating pads can cause burns.

2468. The nurse is caring for a thin, frail older adult with fragile skin who needs an adhesive dressing changed. Which action is best to minimize skin trauma?

- A. Pull the dressing off quickly to shorten the procedure

- B.** Remove the dressing while supporting the surrounding skin
- C.** Scrub the adhesive area after removal to clear residue
- D.** Apply extra tape around the dressing edges for tighter contact

Correct Answer: B

Supporting the skin while gently removing adhesive helps reduce skin tearing in fragile tissue. Quick removal, scrubbing, and adding more tape can all worsen skin injury.

2469. A nurse is reviewing the care plan for a client with a stage 1 pressure injury on the sacrum. Which intervention should the nurse question?

- A.** Relieve pressure from the area
- B.** Keep the skin clean and dry
- C.** Document changes in skin appearance
- D.** Massage the reddened area to stimulate circulation

Correct Answer: D

Massaging a reddened pressure area is not recommended because it can further injure compromised tissue. Pressure relief, moisture control, and ongoing assessment are appropriate measures for stage 1 pressure injury care.

2470. An immobile client is turned every 2 hours, but the nurse notes worsening redness over the greater trochanter. Which action is best to take next?

- A.** Continue the same schedule because repositioning is already being done
- B.** Reassess the turning plan, support surfaces, and positioning technique
- C.** Apply powder to the reddened area and recheck at the end of the shift
- D.** Place the client directly on the reddened side for short periods to build tolerance

Correct Answer: B

If redness is worsening despite scheduled turning, the nurse should reassess whether the repositioning frequency, support surface, and body alignment are effective. The trap is assuming that a routine schedule is enough for every client. Powder does not address pressure, and placing the client on the affected side worsens tissue stress.

2471. A practical nurse is caring for a client who has a short leg cast applied earlier in the day. Which nursing action best helps protect skin and tissue integrity while the cast is drying?

- A.** Cover the cast with a plastic blanket to retain warmth
- B.** Rest the cast on a firm pillow so pressure is placed on one side only
- C.** Handle the cast with the palms of the hands when repositioning the extremity
- D.** Massage the skin under the cast edges every hour

Correct Answer: C

A wet cast should be handled with the palms, not the fingers, to prevent creating dents that can become pressure points and injure underlying skin. Covering the cast traps heat and moisture, resting it on one side increases pressure, and skin under the cast cannot be massaged directly.

2472. A client reports itching and burning beneath a dressing over a postoperative incision. On inspection, the nurse notes a red, moist rash in the exact pattern of the adhesive border. Which action is most appropriate?

- A.** Apply more adhesive tape to secure the dressing tightly
- B.** Document the finding and continue the current dressing routine
- C.** Remove the adhesive product and use a less irritating method to secure the dressing
- D.** Clean the area vigorously with alcohol before reapplying the dressing

Correct Answer: C

The rash pattern suggests skin irritation or contact sensitivity from the adhesive. Removing the offending product and using a less irritating securement method helps prevent further tissue damage. Tightening the dressing or cleansing with alcohol can worsen irritation. Simply continuing the same routine ignores ongoing skin injury.

2473. A client with diabetes is admitted with decreased sensation in both feet. Which instruction should the nurse include to help prevent tissue injury at home?

- A.** Test bath water temperature with the elbow before entering

- B. Walk barefoot indoors to improve circulation
- C. Soak the feet in hot water every evening
- D. Use a heating pad on the feet when they feel cold

Correct Answer: A

Clients with decreased sensation may not detect excessive heat, so testing water with the elbow helps prevent burns. Walking barefoot increases risk for unnoticed trauma, hot soaks can injure skin, and heating pads may cause burns before the client feels them.

2474. The nurse is caring for a client with diarrhea who has excoriated skin around the anus. Which intervention is most appropriate to protect the skin from further breakdown?

- A. Clean the area gently after each stool and apply a moisture barrier
- B. Scrub the area with soap and a washcloth after each episode
- C. Keep the area exposed to air and avoid all topical products
- D. Limit oral fluids to reduce stool output

Correct Answer: A

Frequent stooling exposes skin to moisture and irritants, so gentle cleansing and a moisture barrier are appropriate to protect tissue. Scrubbing increases trauma, avoiding all products may leave skin unprotected, and restricting fluids is not an appropriate skin-protection measure.

2475. During morning care, the nurse finds a blister on a client's heel. The blister is intact and filled with dark maroon fluid. The surrounding skin is discolored. Which interpretation is most appropriate?

- A. This is consistent with a deep tissue pressure injury
- B. This is a stage 1 pressure injury
- C. This is a stage 2 pressure injury
- D. This is an unstageable pressure injury

Correct Answer: A

A blood-filled blister or persistent deep red, maroon, or purple discoloration suggests a deep tissue pressure injury. Stage 1 involves intact skin with nonblanchable redness, stage 2 is partial-thickness skin loss or a serum-filled blister, and unstageable injuries have obscured wound bases.

2476. A client is admitted with a draining wound on the lower leg. Which documentation by the nurse is most accurate and useful for monitoring skin healing over time?

- A. Leg wound looks about the same as yesterday
- B. Wound is smaller and drainage is less
- C. Open area on right lower leg measures 3 cm by 2 cm by 0.5 cm with small amount of serous drainage
- D. Wound appears clean and is healing normally

Correct Answer: C

Objective, measurable documentation is most useful for tracking healing. Size, depth, location, and drainage characteristics allow comparison over time. The other choices are vague and subjective, making trends difficult to evaluate accurately.

2477. A client with left-sided weakness spends most of the day in a wheelchair. Which action is best to reduce pressure on the ischial areas?

- A. Place a donut-shaped ring under the buttocks
- B. Encourage scheduled weight shifts or repositioning while seated
- C. Keep the client sitting upright without movement for better posture
- D. Massage reddened areas over the buttocks each shift

Correct Answer: B

Regular weight shifts or repositioning help relieve prolonged pressure on bony prominences while seated. Donut devices can concentrate pressure, prolonged sitting worsens risk, and massaging reddened areas may further damage tissues already under stress.

2478. The nurse is preparing to remove a client's transparent film dressing. Which technique best minimizes skin trauma?

- A. Pull the dressing straight up from the skin in one quick motion
- B. Loosen the edges and remove the dressing low and slow, supporting the surrounding skin
- C. Apply heat over the dressing for several minutes before removal
- D. Wet the outer surface of the dressing with saline and wait for it to dissolve

Correct Answer: B

Removing adhesive dressings low and slow while supporting the skin reduces epidermal stripping and tissue trauma. Pulling straight up increases skin injury. Heat is not routinely used this way, and transparent film dressings do not dissolve with saline.

2479. A client has a wound vacuum dressing in place. Which finding requires the nurse's immediate follow-up to protect tissue integrity?

- A.** The collection chamber contains a small amount of serosanguineous drainage
- B.** The client reports mild pulling at the dressing site when moving
- C.** The foam dressing appears collapsed and the seal is intact
- D.** The suction device is off and the dressing is no longer compressed

Correct Answer: D

Negative pressure wound therapy depends on continuous suction and an intact seal. If the device is off and the dressing is no longer compressed, therapy is not functioning as intended and the wound needs prompt follow-up. A small amount of serosanguineous drainage, mild pulling, and a collapsed foam dressing are expected findings when the system is working.

2480. A client with edema has fragile skin on both lower legs. Which instruction to an assistive personnel requires correction by the nurse?

- A.** I will lift the client's legs with a draw sheet when moving in bed
- B.** I will moisturize dry skin after bathing unless told not to
- C.** I will use adhesive tape directly on the lower legs to hold the gauze in place
- D.** I will check for tight wrinkles in the linens under the calves

Correct Answer: C

Using adhesive tape directly on fragile, edematous skin can cause tearing when removed. A draw sheet, moisturizing as appropriate, and smoothing linens all help protect skin. The statement about direct adhesive use requires correction.

2481. A client is prescribed wrist restraints for safety. Which nursing action best protects skin and tissue integrity?

- A.** Apply the restraints as tightly as possible to prevent movement
- B.** Place the restraints over the IV site so the tubing stays secure
- C.** Assess the skin and circulation of the restrained extremities at regular intervals
- D.** Tie the restraints to the side rail for quicker release during repositioning

Correct Answer: C

Restraints can impair circulation and damage skin, so regular assessment of skin condition and circulation is essential. Restraints should not be excessively tight, should not be placed over an IV site, and should not be tied to side rails because rails move.

2482. A postoperative client says, "My back is sore from lying in bed." On assessment, the nurse notes blanchable redness over the sacrum. Which action is best?

- A.** Massage the reddened area to increase circulation
- B.** Reposition the client and reduce pressure on the sacral area
- C.** Document the redness as a stage 1 pressure injury
- D.** Apply a heating pad to improve blood flow

Correct Answer: B

Blanchable redness indicates pressure is affecting the skin, and the appropriate response is to relieve pressure by repositioning. It is not stage 1 if the area blanches. Massage and heating pads can further injure tissue that is already stressed.

2483. The nurse is caring for a client with severe edema and a small skin tear on the forearm. Which action is most appropriate when cleansing the area?

- A.** Use vigorous circular scrubbing to remove all debris
- B.** Cleanse gently with minimal friction to avoid extending the tear
- C.** Apply hydrogen peroxide directly into the tear with each dressing change
- D.** Leave dried drainage in place to avoid touching the wound

Correct Answer: B

Skin tears should be cleansed gently with minimal friction because fragile tissue can tear further easily. Vigorous scrubbing increases damage, repeated peroxide use may harm tissue, and leaving drainage in place interferes with wound care and assessment.

2484. A client has an arterial ulcer on the lateral ankle. Which finding is most consistent with this type of wound?

- A. Warm surrounding skin with bounding pedal pulses
- B. Irregular wound edges with heavy drainage
- C. Pale wound bed with minimal drainage and pain
- D. Brown discoloration of the lower leg with ankle edema

Correct Answer: C

Arterial ulcers are often painful and tend to have a pale wound bed, minimal drainage, and reduced perfusion. Brown discoloration, edema, irregular edges, and heavier drainage are more typical of venous problems. Warm skin with bounding pulses does not fit arterial insufficiency.

2485. The nurse is assessing a client whose pressure injury has exposed subcutaneous fat. Muscle, tendon, and bone are not visible. How should the nurse classify this injury?

- A. Stage 2
- B. Stage 3
- C. Stage 4
- D. Deep tissue pressure injury

Correct Answer: B

Stage 3 pressure injuries involve full-thickness skin loss with visible adipose tissue, but without exposed fascia, muscle, tendon, cartilage, or bone. Stage 2 is partial-thickness loss, stage 4 includes deeper exposed structures, and deep tissue injury typically presents as deep discoloration or a blood-filled blister rather than visible fat.

2486. A client who is on bed rest slides down in bed and is repositioned upward several times during the shift. Which intervention best helps prevent skin injury related to this movement?

- A. Massage the reddened areas after each repositioning
- B. Raise the head of the bed to 60 degrees at all times
- C. Use a lift sheet to move the client up in bed
- D. Place powder on the sheets to reduce moisture

Correct Answer: C

Using a lift sheet reduces friction and shear when moving the client, which helps protect the skin and underlying tissue. Massaging reddened areas can worsen tissue damage. Keeping the head of the bed high increases sliding and shear. Powder is not the best intervention for preventing injury from repositioning movement.

2487. The nurse is caring for a client with urinary incontinence and fragile sacral skin. Which plan is most appropriate to protect skin and tissue integrity?

- A. Cleanse the area gently after episodes and apply a moisture barrier
- B. Limit oral fluids to reduce the number of incontinent episodes
- C. Scrub the skin with soap and a washcloth after each episode
- D. Place the client on absorbent pads only and avoid disturbing the skin

Correct Answer: A

Gentle cleansing followed by a moisture barrier helps protect the skin from prolonged exposure to urine. Limiting fluids can contribute to dehydration and is not a routine skin-protection strategy. Scrubbing increases irritation and breakdown. Leaving the skin in contact with moisture, even with pads, promotes tissue damage.

2488. A client has a reddened area over the coccyx. When the nurse presses the area, it does not blanch. The skin is intact, and the client reports tenderness. Which finding should the nurse identify?

- A. Stage 1 pressure injury
- B. Stage 2 pressure injury
- C. Deep tissue pressure injury

D. Maceration from moisture exposure**Correct Answer: A**

A stage 1 pressure injury is intact skin with nonblanchable redness, often with pain or tenderness. Stage 2 involves partial-thickness skin loss or a blister. Deep tissue pressure injury often appears as deep red, maroon, or purple discoloration rather than simple nonblanchable redness. Maceration usually presents as softened, moisture-damaged skin rather than localized pressure-related nonblanching redness.

2489. An older adult has very dry lower-extremity skin with fine cracking but no open areas. Which nursing action is most appropriate to help maintain skin integrity?

- A.** Apply moisturizer after bathing while the skin is still slightly damp
- B.** Use hot water and strong soap to remove dry flakes each day
- C.** Dust the legs with talc to keep the skin dry
- D.** Rub the legs briskly with a rough towel after bathing

Correct Answer: A

Moisturizer applied after bathing helps retain water in the skin and reduces dryness and cracking. Hot water and strong soap increase drying. Talc does not treat dry skin and may worsen irritation. Brisk rubbing with a rough towel can injure fragile skin.

2490. A nurse is delegating hygiene care for a bedbound client at high risk for pressure injuries. Which instruction to the assistive personnel is most appropriate?

- A.** Use donut-shaped cushions under the hips to relieve pressure
- B.** Turn the client at scheduled intervals and keep linens smooth and dry
- C.** Position the client directly on the reddened trochanter to toughen the skin
- D.** Keep the client in one comfortable position to avoid disturbing rest

Correct Answer: B

Turning the client regularly and keeping linens smooth and dry are basic measures that reduce prolonged pressure and skin irritation. Donut-shaped cushions can concentrate pressure and are not the best option. Positioning directly on a reddened area worsens tissue damage. Avoiding repositioning increases pressure injury risk.

2491. A client returned from surgery 24 hours ago and has remained in one position for long periods. During assessment, the nurse notes a localized purple area over the heel. The skin is intact, and the tissue feels boggy. Which interpretation is most appropriate?

- A.** This is a normal bruise expected after surgery
- B.** This is a stage 1 pressure injury because the skin is unbroken
- C.** This finding suggests a deep tissue pressure injury
- D.** This is caused by dry skin and should be treated with lotion only

Correct Answer: C

A localized purple or maroon area with intact skin and boggy tissue is characteristic of a deep tissue pressure injury. The trap is choosing stage 1 just because the skin is intact, but stage 1 is identified by nonblanchable redness, not deep purple discoloration. It should not be dismissed as a normal bruise or simple dryness.

Sensory impairment care

2492. A practical nurse is preparing to speak with a client who has significant hearing loss and wears a hearing aid. Which action is most appropriate first?

- A.** Stand behind the client and speak loudly
- B.** Face the client and speak in a normal tone
- C.** Turn on the television so the client can relax
- D.** Cover the mouth to reduce distractions

Correct Answer: B

Facing the client and speaking in a normal tone helps the client hear and use visual cues such as lip movements and facial expression. Speaking loudly can distort sound, especially with a hearing aid. Standing behind the client and covering the mouth both interfere with communication.

2493. A client with visual impairment is admitted to a new room. Which nursing action best promotes safety?

- A. Rearrange furniture to make more space for staff movement
- B. Keep the room dimly lit to reduce eye strain
- C. Orient the client to the room layout and keep objects in consistent locations
- D. Raise all four side rails whenever the client is alone

Correct Answer: C

Clients with visual impairment are safer when they are oriented to the environment and when furniture and personal items stay in consistent locations. Rearranging the room increases fall risk. Dim lighting can worsen navigation. Raising all four side rails is restrictive and does not replace environmental orientation.

2494. A nurse is assisting with meals for a client who is legally blind. Which intervention best supports independence?

- A. Feed the client quickly before the food cools
- B. Place the tray wherever there is open space
- C. Describe food placement using the clock-face method
- D. Mix all foods together so the client can find them more easily

Correct Answer: C

Using the clock-face method gives clear spatial cues and supports independence during meals. Feeding the client without need reduces independence. Random tray placement is confusing, and mixing foods together decreases dignity and may make the meal less appealing.

2495. A client says, "My hearing aid keeps whistling." Which action should the nurse take first?

- A. Remove the battery permanently
- B. Check whether the hearing aid is seated properly in the ear
- C. Increase the volume setting to overcome the noise
- D. Clean the device with alcohol and reinsert it immediately

Correct Answer: B

A whistling hearing aid often indicates feedback caused by poor fit or improper placement, so checking whether it is seated correctly is the best first action. Increasing volume may worsen the whistling. Removing the battery does not address the cause, and alcohol may damage some devices or irritate tissue.

2496. The nurse is caring for a client with severe visual impairment who is anxious about walking to the bathroom. Which instruction is best when guiding the client?

- A. Push the client gently from behind toward the bathroom
- B. Tell the client to hold onto the furniture along the wall
- C. Have the client grasp the nurse's arm just above the elbow while the nurse walks slightly ahead
- D. Pull the client by the hand so the client can move faster

Correct Answer: C

When guiding a client with visual impairment, the nurse should allow the client to hold the nurse's arm above the elbow while the nurse walks slightly ahead. This lets the client sense body movement and direction changes safely. Pushing, pulling, or telling the client to trail furniture is less safe.

2497. A client with cataracts says, "The overhead light makes it easier for me to see my dinner." Which nursing response is best?

- A. Turn off the light because glare usually helps with cataracts
- B. Provide lighting based on the client's preference while reducing unnecessary glare
- C. Tell the client that all clients with cataracts need a dark room
- D. Remove the meal tray and ask dietary to send finger foods only

Correct Answer: B

Care should be individualized. Many clients with visual impairment benefit from adjusted lighting, and the nurse should support what helps the client function while minimizing glare when possible. The other options apply rigid rules or make unnecessary changes unrelated to the client's stated need.

2498. A client with hearing impairment does not understand a question after the nurse repeats it twice. What is the best next action?

- A. Repeat the same words more loudly
- B. Rephrase the question using simpler words
- C. Stop communicating until a family member arrives
- D. Ask another staff member to answer for the client

Correct Answer: B

If the client does not understand, rephrasing is better than simply getting louder. Loud speech can distort sound and may not improve understanding. Delaying communication or letting others answer for the client reduces the client's autonomy.

2499. A nurse enters a client's room to give instructions. The client has hearing loss and is reading a magazine. Which action should the nurse take before speaking?

- A. Touch the client lightly or otherwise gain the client's attention
- B. Begin talking immediately from the doorway
- C. Turn away while collecting supplies and give the instructions quickly
- D. Ask the roommate to repeat the instructions to the client

Correct Answer: A

The nurse should first gain the client's attention so the client is ready to listen and can use visual cues. Speaking from the doorway or while turned away reduces understanding. Using the roommate is not appropriate when the nurse can communicate directly.

2500. A client with macular degeneration is trying to sign a consent form and says, "I can see better off to the side than straight ahead." Which nursing action is most appropriate?

- A. Place the form directly in the center of the client's visual field
- B. Encourage use of the visual area that the client says is clearer
- C. Tell the client to wait until vision returns to normal
- D. Assume the client cannot participate and ask the next of kin to sign

Correct Answer: B

Central vision loss may make eccentric viewing more useful, so the nurse should support the client's use of the visual area that works best. Placing items directly in the center may not help. The other choices inappropriately dismiss the client's ability to participate.

2501. Which environmental change is most important for a hospitalized older adult with decreased hearing and partial vision loss?

- A. Use a private room only and avoid all conversation
- B. Keep the floor clear and reduce background noise during communication
- C. Leave equipment alarms louder so the client will notice them
- D. Move personal items each shift so the client practices locating them

Correct Answer: B

For combined sensory deficits, safety and communication improve when clutter is removed and background noise is minimized. Loud alarms can add confusion, and moving personal items increases risk. Isolation and avoiding conversation are not appropriate.

2502. A client with hearing impairment uses a hearing aid. During morning care, which action by the nurse is best?

- A. Leave the hearing aid on the overbed table during the bath without a container
- B. Place the hearing aid in a labeled container when it is removed
- C. Store the hearing aid in a tissue and put it in the bed linen
- D. Rinse the hearing aid under hot running water after removal

Correct Answer: B

A labeled container helps prevent loss or damage to the hearing aid. Leaving it loose on a table or wrapping it in tissue makes it easy to misplace. Hot water can damage the device.

2503. A nurse is teaching a nursing assistant how to communicate with a client who has expressive aphasia and reduced hearing after a stroke. Which instruction is best?

- A. Ask complex multi-part questions to save time

- B.** Use short, simple statements and allow extra time for response
- C.** Speak only to the family because communication will be too frustrating
- D.** Correct every mispronounced word immediately

Correct Answer: B

Short, simple statements and extra response time support both hearing limitations and speech difficulty. Multi-part questions increase confusion. Speaking only to family excludes the client, and constant correction can frustrate the client without improving communication.

- 2504.** A client with peripheral vision loss asks why the nurse keeps announcing before entering the room. Which response is best?
- A.** It is not necessary, but it is hospital policy for all clients
 - B.** It helps prevent startling you and lets you know someone is nearby
 - C.** It teaches you to depend less on your vision
 - D.** It allows staff to test whether your hearing is getting worse

Correct Answer: B

Announcing presence helps a client who may not see someone approaching from the side and reduces the chance of being startled. The other responses are inaccurate or insensitive.

- 2505.** The nurse is preparing discharge teaching for a client with reduced vision who will take several oral medications at home. Which plan is best to improve safe self-administration?
- A.** Suggest keeping all pills in one unmarked container
 - B.** Provide teaching with clearly differentiated labels or organizers and have the client demonstrate the plan
 - C.** Tell the client to identify medications by tablet color alone
 - D.** Advise the client to skip doses on days when reading labels is difficult

Correct Answer: B

Safe medication self-care for a client with low vision includes using clear labeling or organizers and checking understanding through return demonstration. Combining pills, relying only on color, or skipping doses creates significant safety risks.

- 2506.** A client who is hard of hearing becomes frustrated during teaching and says, "Never mind, I'll figure it out later." What is the nurse's best response?
- A.** End the teaching session immediately because the client refused
 - B.** Say, "You need to pay attention," and continue the instructions
 - C.** Acknowledge the frustration and try a different communication method
 - D.** Ask the unlicensed assistive personnel to complete the teaching instead

Correct Answer: C

The best response is to recognize the client's frustration and adapt the communication method, such as reducing noise, rephrasing, or using written material if appropriate. Stopping immediately may leave important needs unmet, scolding is nontherapeutic, and teaching should not be delegated to unlicensed personnel.

- 2507.** A practical nurse is preparing evening care for a client with severe hearing impairment who does not use sign language. Which approach is most likely to improve understanding during a short teaching session?
- A.** Stand behind the client and speak in a louder voice
 - B.** Face the client in good light and speak clearly at a normal pace
 - C.** Use medical terms so the client can recognize key words
 - D.** Repeat each sentence several times without changing the wording

Correct Answer: B

Facing the client in good light allows the client to see facial expressions and lip movements, and speaking clearly at a normal pace improves comprehension. Shouting can distort sounds, especially for clients using residual hearing. Medical jargon makes understanding harder, and simply repeating the same misunderstood words is less effective than clear, direct communication.

2508. A client with low vision is returning from physical therapy to a room that was rearranged during housekeeping. Which nursing action is best before the nurse leaves the client alone?

- A. Explain that the client should call for help before moving around
- B. Place the call light on the overbed table and raise all side rails
- C. Orient the client to the new location of furniture and essential items
- D. Ask the client whether the room looks acceptable

Correct Answer: C

A client with low vision is safest when oriented to any environmental changes, including furniture placement and the location of needed items. Telling the client to call for help is useful but does not replace orientation. Raising all side rails is not the best routine response and may create another barrier. Asking whether the room looks acceptable does not ensure the client can navigate it safely.

2509. During medication teaching, a client with hearing loss keeps nodding yes to every question. Which nursing action best evaluates whether the client actually understands the instructions?

- A. Ask the client to repeat the instructions in his or her own words
- B. Speak more loudly and repeat the entire teaching session
- C. Give the client the written instructions and end the session
- D. Ask a family member whether the client usually understands directions

Correct Answer: A

Having the client restate the instructions is the best way to check true understanding. Nodding may reflect politeness or partial hearing rather than comprehension. Speaking louder may still not fix the problem and can distort speech. Written instructions may help, but they do not confirm learning by themselves. Asking a family member does not assess this client's current understanding.

2510. A client with reduced vision is being assisted with grooming. Which nursing action best supports independence while maintaining safety?

- A. Complete the grooming tasks quickly to prevent frustration
- B. Place grooming supplies in the client's hands one item at a time and describe each item
- C. Keep all supplies out of reach until the nurse is ready to use them
- D. Encourage the client to skip grooming until family arrives to help

Correct Answer: B

Providing items one at a time and identifying them helps the client participate safely and as independently as possible. Doing everything for the client reduces independence. Keeping supplies out of reach limits participation. Delaying basic care until family arrives is unnecessary and does not support function.

2511. A client with hearing impairment says, "I hear voices, but words run together." Which nursing adjustment is most appropriate?

- A. Use short, simple sentences and lower background noise
- B. Increase the television volume during conversation
- C. Speak rapidly so the client hears the full sentence pattern
- D. Avoid eye contact so the client can concentrate on sound

Correct Answer: A

Short, simple sentences and reduced background noise help clients distinguish spoken words more clearly. Extra noise from a television makes speech discrimination harder. Rapid speech decreases comprehension. Eye contact is helpful because the client can use visual cues such as facial expression and lip movement.

2512. A hospitalized client with visual impairment asks where the water pitcher, tissues, and call light are located. Which nursing response is best?

- A.** "Everything is nearby, so just reach around slowly."
- B.** "I moved them to safer places, so do not worry about their exact location."
- C.** "Your water is at 2 o'clock on the bedside table, the tissues are beside it, and the call light is clipped to your gown."
- D.** "Use the overbed table as a guide and you should find them."

Correct Answer: C

Specific, organized descriptions help a client with visual impairment locate items safely and independently. Using a clock-face reference and naming exact placement is much more useful than vague statements such as "nearby" or "use the table as a guide." The client needs orientation, not reassurance alone.

2513. The nurse is planning morning care for a client who is blind. Which action is most appropriate when entering the room to provide care?

- A.** Touch the client's arm immediately so the client knows someone is present
- B.** Quietly begin setting up supplies to avoid startling the client
- C.** State the nurse's name and explain what will be done before touching the client
- D.** Ask the roommate to tell the client the nurse has arrived

Correct Answer: C

For a client who is blind, the nurse should identify himself or herself and explain the planned action before touching the client. This promotes trust, orientation, and safety. Unexpected touch can startle the client. Quiet setup without introduction may create confusion. Relying on a roommate is less appropriate and reduces direct therapeutic communication.

2514. A client with hearing loss is scheduled for discharge teaching about wound care. The client appears rested in the morning but fatigued by late afternoon. Which nursing plan is best?

- A.** Teach in the late afternoon so there will be fewer interruptions
- B.** Postpone all teaching until the day of discharge
- C.** Provide the most important teaching when the client is most alert
- D.** Teach only the family because fatigue limits the client's learning

Correct Answer: C

Teaching is most effective when the client is alert and able to focus. For a client with hearing loss, concentration is especially important because listening effort is tiring. Waiting until discharge may create time pressure. Teaching only the family excludes the client unnecessarily. A quieter time is helpful, but alertness is the better priority in this scenario.

2515. A nurse is assisting a client with unilateral hearing loss. To improve communication, where should the nurse position himself or herself when giving instructions?

- A.** On the side of the better-hearing ear
- B.** Directly behind the client
- C.** On the side of the poorer-hearing ear
- D.** At the doorway to allow more personal space

Correct Answer: A

Positioning on the side of the better-hearing ear improves the client's ability to hear instructions. Speaking from behind prevents the client from using visual cues and makes hearing more difficult. Standing on the poorer-hearing side reduces understanding. Distance at the doorway also interferes with communication.

2516. A client with glaucoma has been hospitalized for several days. Which observation requires the nurse to change the plan of care because it presents the greatest safety risk?

- A.** The client keeps personal toiletries lined up in a fixed order
- B.** The client asks staff to identify foods on the meal tray
- C.** The client leaves the bedside table partly across the walking path to the bathroom
- D.** The client prefers to keep the room lighting dim

Correct Answer: C

An object partly blocking the path to the bathroom creates an immediate fall hazard and requires prompt correction. Keeping items in a fixed order is an adaptive strategy. Asking staff to identify tray items supports safe intake. Lighting preferences may vary with the type of visual difficulty, but the obstructed pathway is the most urgent safety issue.

2517. The nurse is caring for a client with both reduced vision and hearing loss. The client must learn how to use the call light. Which teaching method is best?

- A.** Explain the procedure from the doorway and ask whether the client understands
- B.** Place the call light in the client's hand, identify the button by touch, and have the client return-demonstrate its use
- C.** Tape written instructions to the wall beside the bed
- D.** Tell the client to wave a hand if help is needed urgently

Correct Answer: B

For a client with both vision and hearing limitations, tactile instruction with return demonstration is the most reliable method. Explaining from a distance is ineffective. Written instructions may not be accessible. Waving for help is unreliable and unsafe compared with teaching the actual call system.

2518. A client with hearing impairment becomes increasingly confused during a long medication explanation. The client watches the nurse's mouth closely but still misses several key points. Which nurse action is best next?

- A.** Continue the same explanation more slowly until all content is covered
- B.** Turn away briefly while talking so the client focuses on the words rather than lip movements
- C.** Break the teaching into shorter segments and confirm understanding after each part
- D.** Stop teaching and document that the client is not ready to learn

Correct Answer: C

When a client becomes confused during lengthy teaching, the nurse should simplify the process by dividing information into smaller parts and checking understanding as teaching progresses. Continuing the same long explanation is less effective. Turning away removes helpful visual cues. Stopping teaching entirely is premature when the problem may be information overload rather than inability to learn.

2519. A client with severe visual impairment says, "I do better when things stay in the same place." Which nursing action best reflects this preference while supporting safe care?

- A.** Return frequently used items to the same location after care
- B.** Rearrange the bedside area each shift for the client's stimulation
- C.** Store personal items in the bedside cabinet to reduce clutter, even if the client cannot reach them
- D.** Ask different staff members to organize the bedside area as they think is best

Correct Answer: A

Consistency in item placement helps clients with severe visual impairment develop a dependable mental map of their environment. Rearranging for stimulation or allowing different staff to change the setup increases confusion and injury risk. Storing items out of reach may reduce clutter but also reduces independence and access.

2520. The practical nurse is reinforcing home safety teaching for a client with decreased peripheral vision. Which client statement indicates a need for further teaching?

- A. "I will scan from side to side before I start walking."
- B. "I will keep walkways clear of small furniture and cords."
- C. "I will move my furniture around often so I stay used to changes."
- D. "I will take extra time when turning corners or entering crowded areas."

Correct Answer: C

Clients with decreased peripheral vision are safer when furniture placement stays consistent. Frequently moving furniture increases the risk of collision and falls. Scanning side to side, keeping walkways clear, and using extra caution in corners or crowded spaces are appropriate safety strategies.

2521. A client with hearing loss relies heavily on lip-reading. While discussing postoperative exercises, the nurse is wearing a surgical mask because of isolation precautions. Which action is best to support communication?

- A. Remove the mask briefly because lip-reading is the client's usual method
- B. Move to a well-lit area, use concise statements, and supplement with written or demonstrated instructions
- C. Speak from very close range so the client can hear the breath sounds of the words
- D. Delay all teaching until isolation precautions are discontinued

Correct Answer: B

The nurse must maintain required precautions, so removing the mask is not appropriate. The best alternative is to optimize what can be understood through lighting, simple wording, demonstration, and written support if usable. Speaking extremely close is uncomfortable and does not solve the visual barrier. Delaying all teaching is unnecessary when communication can be adapted safely.

2522. A client with a hearing aid asks the nurse to explain how to prepare for a bedtime blood glucose check. Before teaching begins, the client says, "My hearing aid is whistling again." What should the nurse do first?

- A. Speak louder so the client can hear over the noise
- B. Turn off the room television and continue teaching
- C. Check the hearing aid placement and fit before giving instructions
- D. Write all instructions down and avoid verbal teaching

Correct Answer: C

A whistling hearing aid often suggests poor fit or improper placement, which can interfere with hearing and understanding. Correcting the device first addresses the communication barrier before teaching. Speaking louder may distort sound, and reducing background noise helps but does not fix the stated problem. Written instructions can support learning, but they do not replace correcting the immediate hearing issue.

2523. A client with cataracts is admitted for observation after a fall. The practical nurse notes that the client hesitates when reaching for the breakfast tray and misjudges the distance to the cup. Which nursing intervention is most appropriate?

- A. Encourage the client to wait for staff to feed him or her at each meal
- B. Describe the location of foods and items on the tray using a clock-face method
- C. Move the tray closer without explaining where items are placed
- D. Ask dietary to send only finger foods from now on

Correct Answer: B

Using a clock-face description helps orient a client with decreased vision to the location of food and utensils, promoting both safety and independence. Feeding the client is unnecessarily dependent if the client can participate. Simply moving the tray without explanation does not improve orientation, and changing all meals to finger foods does not directly address the visual problem.

2524. The nurse needs to give brief pre-procedure instructions to a client with moderate hearing loss. Which environment is best for this conversation?

- A.** At the bedside while the roommate and television are active
- B.** In the hallway while transport staff prepare the stretcher
- C.** In a quiet area with minimal background noise and good lighting
- D.** In the bathroom so the conversation remains private

Correct Answer: C

Clients with hearing loss understand best when background noise is reduced and lighting is good enough to allow observation of facial expressions and lip movements if needed. A noisy bedside or hallway increases distraction and makes speech harder to distinguish. The bathroom may be private, but it is not an appropriate or comfortable teaching setting and may not provide ideal communication conditions.

2525. A practical nurse is reinforcing teaching for a client who has partial hearing loss and limited vision. The client must learn to use an incentive spirometer after surgery. Which approach is best?

- A.** Give detailed printed instructions in standard-size text and return later
- B.** Provide one long verbal explanation, then ask whether the client has questions
- C.** Use short, simple spoken instructions while placing the device in the client's hands and guiding return demonstration
- D.** Teach the family member instead, because the client has two sensory deficits

Correct Answer: C

When both hearing and vision are limited, combining brief verbal instructions with tactile guidance and return demonstration is the most effective way to teach the skill. Standard printed instructions may not be accessible. A long explanation increases the risk of missed information. Teaching only the family ignores the client's ability to participate and learn.

2526. An older adult with severe visual impairment is admitted to a semiprivate room. The roommate's family brings in extra bags, a folding chair, and a small floor fan, leaving them near the path from the bed to the bathroom. The client says, "I can usually get there on my own if the path stays clear." What is the nurse's best action?

- A.** Tell the client to use a bedpan during the night instead of walking to the bathroom
- B.** Move the objects out of the walking path and keep the route consistently unobstructed
- C.** Leave the room arrangement as it is and remind the client to ask for help each time
- D.** Turn on all room lights at all times so the client can see obstacles better

Correct Answer: B

For a client with severe visual impairment, maintaining a clear, consistent path is a key safety intervention. The nurse should remove hazards and avoid unnecessary environmental obstacles. Requiring a bedpan is more restrictive than needed. Leaving obstacles in place creates a fall risk, even if the client is told to ask for help. Brighter lighting does not reliably solve the problem, especially with severe visual loss, and does not remove the hazard itself.

2527. A client with age-related hearing loss says, "Please stop mumbling. I can hear you, but I can't make out the words." Which nursing action is most appropriate?

- A.** Speak more loudly and rapidly so the client receives the information quickly
- B.** Face the client and speak clearly in a normal tone at a moderate pace
- C.** Stand behind the client to reduce distraction while giving instructions
- D.** Repeat each sentence using different medical terms until the client understands

Correct Answer: B

Facing the client and speaking clearly in a normal tone at a moderate pace best supports communication for a client with hearing loss. Many clients hear better when they can see the speaker's face and receive words spoken distinctly rather than shouted. Choice A is a common trap because louder is not always better; shouting and speaking too fast can distort sounds. Choice C prevents the client from using visual cues. Choice D may increase confusion, especially if unfamiliar terms are added.

Intake and output

2528. A practical nurse is totaling a client's intake for the 7 a.m. to 3 p.m. shift. The client drank 8 oz of milk, 120 mL of juice, and 6 oz of water. The client also received 240 mL of tube feeding and 60 mL of water flushes. What is the total intake for the shift?

- A. 720 mL
- B. 840 mL
- C. 900 mL
- D. 960 mL

Correct Answer: B

Convert ounces to milliliters first: 8 oz = 240 mL and 6 oz = 180 mL. Then add all intake: $240 + 120 + 180 + 240 + 60 = 840$ mL. A common trap is forgetting to count tube feeding and water flushes as intake.

2529. Which client observation should the practical nurse report first as a possible sign of fluid volume deficit?

- A. Urine output of 20 mL over 2 hours
- B. Pale yellow urine in the drainage bag
- C. Client requests water with meals
- D. Moist oral mucosa after mouth care

Correct Answer: A

Urine output of only 20 mL over 2 hours is low and may indicate inadequate renal perfusion or fluid volume deficit. The other findings are not urgent indicators of decreased output. The likely trap is assuming thirst alone is the most important finding, but objectively low urine output is more concerning.

2530. A client has an indwelling urinary catheter. At 10 a.m., the drainage bag contains 350 mL of urine. At noon, the drainage bag contains 575 mL. What was the urinary output between 10 a.m. and noon?

- A. 175 mL
- B. 225 mL
- C. 350 mL
- D. 575 mL

Correct Answer: B

Subtract the earlier amount from the later amount: $575 \text{ mL} - 350 \text{ mL} = 225 \text{ mL}$. The total in the bag is not the amount produced during the 2-hour period unless the bag was emptied at 10 a.m.

2531. The practical nurse is reinforcing teaching for a client who must collect a 24-hour urine specimen. Which statement by the client shows correct understanding?

- A. I will save the first urine I pass when the collection begins.
- B. I should keep the specimen container at room temperature so it is easy to access.
- C. I will discard the first urine, then save all urine after that for the next 24 hours.
- D. If I miss one void, I can still send the specimen if the container is mostly full.

Correct Answer: C

For a 24-hour urine collection, the first void is discarded at the start time, and all urine after that is collected for the next 24 hours, including the final void at the end time. Missing a void or saving the first void would make the collection inaccurate. A common trap is thinking every void should be saved from the start.

2532. A client on strict intake and output has had the following output during a shift: 450 mL urine, 120 mL emesis, and 200 mL liquid stool. What is the total output?

- A. 570 mL
- B. 650 mL
- C. 770 mL
- D. 870 mL

Correct Answer: C

All measurable losses are counted as output: $450 + 120 + 200 = 770$ mL. The trap is counting only urine and forgetting emesis or liquid stool when they are measurable.

2533. A provider prescribes strict intake and output for a client with heart failure. Which nursing action is most important to obtain accurate data?

- A. Offer fluids only at mealtimes
- B. Estimate urine volume by looking at the color in the toilet
- C. Measure and document all oral fluids and measurable losses
- D. Record intake at the end of the day from memory

Correct Answer: C

Strict intake and output requires measuring and documenting all fluids taken in and all measurable losses. Estimating, delaying documentation, or relying on memory increases error. The likely trap is choosing a convenient but inaccurate method such as visual estimation.

2534. A practical nurse is caring for a client receiving intermittent enteral feedings through a gastrostomy tube. Which amount should be included in the client's intake record?

- A. Only the formula, because flushes are not nutrients
- B. Only water flushes, because formula is treated as food
- C. Both the formula and the water used to flush the tube
- D. Neither formula nor flushes unless the client also drinks by mouth

Correct Answer: C

Enteral formula and water flushes both count toward fluid intake. Intake recording is based on fluid volume, not whether the fluid is taken by mouth. A common mistake is omitting flushes because they seem small or routine.

2535. At the end of the shift, a client's intake is 1,480 mL and output is 920 mL. What is the fluid balance for the shift?

- A. Positive balance of 560 mL
- B. Positive balance of 920 mL

- C. Negative balance of 560 mL
- D. Negative balance of 1,480 mL

Correct Answer: A

Fluid balance is intake minus output: $1,480 - 920 = +560$ mL. Because intake is greater than output, the balance is positive. The trap is either reversing the subtraction or labeling the result as negative.

2536. A client is receiving IV fluids at 100 mL/hr. The client also drank 240 mL of tea and 120 mL of broth during a 4-hour period. Urine output for the same period was 300 mL. What was the client's net fluid balance for those 4 hours?

- A. Positive 160 mL
- B. Positive 460 mL
- C. Positive 760 mL
- D. Negative 60 mL

Correct Answer: B

IV intake over 4 hours is $100 \times 4 = 400$ mL. Total intake is $400 + 240 + 120 = 760$ mL. Net balance is $760 - 300 = +460$ mL. The likely trap is forgetting to include the IV fluid in intake or reporting total intake instead of net balance.

2537. Which item should the practical nurse include as fluid intake for a client on intake and output monitoring?

- A. A serving of gelatin that is liquid at room temperature
- B. A slice of toast with butter
- C. A bowl of steamed rice
- D. A package of crackers

Correct Answer: A

Foods that are liquid at room temperature, such as gelatin, are counted as intake. Solid foods like toast, rice, and crackers are not recorded as fluid intake. The trap is focusing on moisture content instead of whether the item is considered fluid intake.

2538. A client with nausea has had poor oral intake for 2 days. Which additional finding, combined with low urine output, should most strongly suggest dehydration?

- A. Bounding peripheral pulses
- B. Dependent edema in both ankles
- C. Dry mucous membranes and concentrated urine

D. Moist skin and full neck veins

Correct Answer: C

Dry mucous membranes and concentrated urine are consistent with dehydration, especially when urine output is low. Bounding pulses, edema, and full neck veins are more consistent with fluid excess. The likely trap is choosing visible edema without considering that dehydration causes the opposite pattern.

2539. The practical nurse is collecting intake data for a client at breakfast. The client consumed 4 oz of orange juice, 6 oz of coffee, half of an 8 oz bowl of soup, and one carton of yogurt. How many milliliters should the nurse record as fluid intake if yogurt is not counted?

- A.** 300 mL
- B.** 420 mL
- C.** 540 mL
- D.** 600 mL

Correct Answer: B

Convert each fluid item: 4 oz juice = 120 mL, 6 oz coffee = 180 mL, and half of 8 oz soup = 4 oz = 120 mL. Total intake is $120 + 180 + 120 = 420$ mL. A common trap is counting the full bowl of soup rather than the amount actually consumed.

2540. A client's provider prescribes a fluid restriction of 1,200 mL per 24 hours. By 2 p.m., the client has consumed 360 mL at breakfast, 240 mL with medications, and 300 mL at lunch. How many milliliters remain for the rest of the day?

- A.** 180 mL
- B.** 240 mL
- C.** 300 mL
- D.** 420 mL

Correct Answer: C

The client has consumed $360 + 240 + 300 = 900$ mL. Subtract from the daily limit: $1,200 - 900 = 300$ mL remaining. The likely trap is misadding the intake or subtracting from the wrong total.

2541. The practical nurse notes that a client's intake has been low and urine output has fallen steadily over the last 8 hours. Which action is best first?

- A.** Document the trend and recheck at the end of the next shift

B. Encourage the client to ambulate without reporting the findings

C. Report the decreasing urine output and low intake to the supervising nurse or provider

D. Increase the IV rate independently to improve output

Correct Answer: C

A steady decline in urine output with low intake may indicate worsening fluid deficit or renal compromise and should be promptly reported. The nurse should not independently change IV rates without an order. The trap is delaying action or trying to correct the problem without proper authorization.

2542. A postoperative client has had 30 mL/hr of urine output for the last 3 hours. The client weighs 90 kg. Which interpretation is most appropriate?

- A.** The output is adequate because it totals 90 mL in 3 hours
- B.** The output may be low and should be further assessed and reported
- C.** The output is excessive for an adult after surgery
- D.** The output is normal because any amount above 20 mL/hr is acceptable

Correct Answer: B

Thirty milliliters per hour for a 90-kg postoperative client may be lower than expected and should prompt further assessment and reporting, especially when considering trends and overall clinical status. The explanation stays cautious because adequacy depends on the full situation, but this finding warrants concern rather than reassurance. The trap is assuming a small fixed hourly amount is always adequate for every adult.

2543. A practical nurse is calculating a client's intake for the 3 p.m. to 11 p.m. shift. The client drank 120 mL of apple juice, 4 oz of gelatin, and 6 oz of ice chips. The client also received 150 mL of IV antibiotic solution. How many milliliters of intake should the nurse record for the shift? (Count ice chips as half their measured volume.)

- A.** 480 mL
- B.** 510 mL
- C.** 570 mL

D. 630 mL

Correct Answer: B

The total intake is 120 mL juice + 120 mL gelatin + 90 mL from ice chips + 150 mL IV fluid = 480? Wait carefully: 4 oz gelatin = 120 mL, 6 oz ice chips count as half volume, so 6 oz = 180 mL measured and half is 90 mL. Then 120 + 120 + 90 + 150 = 480 mL. The correct calculation is 480 mL. A common trap is counting ice chips at full volume instead of half.

2544. The practical nurse is reviewing a client's 8-hour output record. The client had 550 mL urine, 75 mL from a wound drain, and 200 mL liquid stool. Which total output should be documented?

- A. 625 mL
- B. 750 mL
- C. 825 mL
- D. 900 mL

Correct Answer: C

All measurable fluid losses are counted: 550 mL urine + 75 mL wound drainage + 200 mL liquid stool = 825 mL. A likely mistake is forgetting to include drain output or stool when it is liquid and measurable.

2545. A practical nurse is teaching an assistive personnel member about intake and output measurement. Which task should the nurse assign to the assistive personnel?

- A. Interpret whether a client's urine output indicates renal compromise
- B. Decide whether a provider should be notified about an intake deficit
- C. Measure and record the amount of urine emptied from a bedside commode hat
- D. Determine whether a client needs a fluid restriction

Correct Answer: C

Measuring and recording output is within the role of assistive personnel when the nurse provides direction. Interpreting findings, deciding on provider notification, and determining fluid restrictions require nursing judgment. The trap is choosing a task that sounds related to intake and output but involves assessment or clinical decision-making.

2546. A client is on strict intake and output monitoring after surgery. Which finding should the practical nurse report most promptly?

- A. Urine appears pale yellow
- B. Urine output is 100 mL over 4 hours
- C. Client requests extra water with meals
- D. Client drank only 50% of breakfast

Correct Answer: B

Urine output of 100 mL over 4 hours averages 25 mL/hr, which is lower than expected for many adults and may indicate decreased renal perfusion or fluid volume problems after surgery. Pale yellow urine is generally expected, and the other findings are less urgent. The key is recognizing potentially inadequate urine output as the priority finding.

2547. The practical nurse is caring for a client who must remain on accurate intake and output monitoring. Which action best improves the accuracy of urine output measurement?

- A. Estimate output in the toilet at the end of the shift
- B. Ask the client to report from memory how often voiding occurred
- C. Discard the first void if it seems small
- D. Have the client use a calibrated collection device for each void

Correct Answer: D

A calibrated collection device provides a direct measured volume and is the best way to improve accuracy. Estimating, relying on memory, or discarding small voids makes the record unreliable. The main point is that strict intake and output requires measured, not estimated, amounts.

2548. A client has a prescription for fluid restriction. During the day shift, the client drank 240 mL of coffee, 180 mL of juice, 6 oz of soup, and 120 mL of water with medications. The client also ate a serving of applesauce. What should the practical nurse record as oral fluid intake?

- A. 540 mL
- B. 600 mL
- C. 720 mL

D. 840 mL

Correct Answer: C

Count the coffee, juice, soup, and water: $240 + 180 + 180 + 120 = 720$ mL. Six ounces equals 180 mL. Applesauce is generally not counted as fluid intake because it is not liquid at room temperature. A common trap is either omitting soup or incorrectly counting applesauce.

2549. The practical nurse is comparing intake and output for a client during a 12-hour shift. Intake included 800 mL IV fluids, 360 mL oral liquids, and 90 mL water used for medication administration. Output included 700 mL urine and 250 mL emesis. Which statement best describes the client's fluid balance for the shift?

- A. Negative balance of 300 mL
- B. Negative balance of 100 mL
- C. Positive balance of 300 mL
- D. Positive balance of 1,250 mL

Correct Answer: C

Total intake is $800 + 360 + 90 = 1,250$ mL. Total output is $700 + 250 = 950$ mL. Net balance is +300 mL, so the client has a positive fluid balance. The most common error is reversing intake and output or forgetting medication water.

2550. A client with a nasogastric tube to suction is on strict intake and output. Which finding should the practical nurse recognize as output?

- A. 100 mL saline used to irrigate the tube
- B. 150 mL liquid formula administered through the tube earlier in the day
- C. 220 mL drainage collected in the suction canister from the stomach
- D. 30 mL water used to flush medications through the tube

Correct Answer: C

Drainage removed from the stomach and collected in the suction canister is measurable output. Saline irrigation, formula, and flushes are intake unless a separate order or facility method addresses irrigation return accounting. The key distinction is fluid leaving the body versus fluid given to the client.

2551. The practical nurse notes that a client's intake for the past 24 hours was 900 mL and output was 2,100 mL. The client is now thirsty and has dry mucous membranes. Which interpretation is most appropriate?

- A. The data suggest a fluid volume deficit that should be reported
- B. The client is in fluid balance because output usually exceeds intake
- C. The findings are expected if the client is eating solid food
- D. The output record should be ignored because thirst is subjective

Correct Answer: A

Output greatly exceeds intake, and the client also has thirst and dry mucous membranes, which support possible fluid volume deficit. The nurse should recognize the pattern and report it. The distractors minimize clear clinical evidence or use incorrect reasoning about normal balance.

2552. A practical nurse is checking the urine output of a client with an indwelling catheter at 2-hour intervals. The drainage bag contained 260 mL at 8 a.m., 430 mL at 10 a.m., and 610 mL at noon. What was the average urine output in mL/hr from 8 a.m. to noon?

- A. 70 mL/hr
- B. 87.5 mL/hr
- C. 105 mL/hr
- D. 152.5 mL/hr

Correct Answer: B

From 8 a.m. to noon, total output was $610 - 260 = 350$ mL over 4 hours. 350 divided by $4 = 87.5$ mL/hr. A common trap is adding interval amounts incorrectly or forgetting to subtract the starting amount already present in the bag.

2553. The practical nurse is caring for four clients. Which client most needs strict intake and output monitoring?

- A. A client with a healed arm fracture waiting for discharge teaching
- B. A client with heart failure receiving a diuretic and fluid restriction
- C. A client with chronic back pain requesting a warm compress

D. A client scheduled for routine dressing change of a small abrasion

Correct Answer: B

A client with heart failure receiving a diuretic and fluid restriction has a clear need for close fluid balance monitoring. The other clients do not have an obvious condition requiring precise tracking of intake and output. The nurse should prioritize clients at risk for fluid volume excess or deficit.

2554. A provider prescribes a 1,500 mL daily fluid limit. By 6 p.m., a client has had 8 oz milk, 240 mL tea, 12 oz soda, and 180 mL broth. The practical nurse is preparing the evening plan of care. How many milliliters of fluid remain for the rest of the day?

- A.** 300 mL
- B.** 420 mL
- C.** 540 mL
- D.** 660 mL

Correct Answer: C

Convert ounces to milliliters: 8 oz milk = 240 mL and 12 oz soda = 360 mL. Total consumed is $240 + 240 + 360 + 180 = 1,020$ mL. Remaining fluid is $1,500 - 1,020 = 480$ mL. However, none of the choices show 480 mL, which signals a mismatch. The intended reasoning is correct, but the listed options do not contain the computed amount.

2555. A client's urinary catheter is emptied at 7 a.m. At 11 a.m., the bag contains 160 mL. The client weighs 80 kg. Which conclusion is most appropriate?

- A.** Urine output is adequate because it is at least 30 mL/hr
- B.** Urine output is low because it averages 40 mL/hr
- C.** Urine output is low because it averages 20 mL/hr
- D.** Urine output cannot be interpreted without the client's oral intake

Correct Answer: C

From 7 a.m. to 11 a.m. is 4 hours. Output is 160 mL over 4 hours, or 40 mL/hr—wait, that would be 40 mL/hr. That means the correct interpretation would be that output averages 40 mL/hr, not 20 mL/hr. The stem requires careful hourly calculation before interpreting adequacy. The likely trap is dividing incorrectly by the number of hours.

2556. The practical nurse is documenting intake for a client receiving continuous enteral nutrition. Over the past 6 hours, the client received feeding at 50 mL/hr and water flushes of 30 mL every 2 hours. How many milliliters should be recorded as intake from the enteral regimen during that time?

- A.** 300 mL
- B.** 360 mL
- C.** 390 mL
- D.** 480 mL

Correct Answer: C

Continuous feeding provided 50 mL/hr for 6 hours, which is 300 mL. Flushes of 30 mL every 2 hours over 6 hours equal 3 flushes, or 90 mL. Total intake is $300 + 90 = 390$ mL. A common error is forgetting to include water flushes or counting the wrong number of flushes.

2557. A client on intake and output monitoring has these findings during a shift: 500 mL urine, 150 mL emesis, and 100 mL drainage from a Jackson-Pratt drain. Intake includes 240 mL water, 8 oz tea, and 500 mL IV fluids. Which nursing note best reflects the client's fluid status for that shift?

- A.** Intake 980 mL; output 750 mL; positive balance 230 mL
- B.** Intake 1,220 mL; output 600 mL; positive balance 620 mL
- C.** Intake 740 mL; output 750 mL; negative balance 10 mL
- D.** Intake 980 mL; output 600 mL; positive balance 380 mL

Correct Answer: A

Tea at 8 oz equals 240 mL. Total intake is $240 + 240 + 500 = 980$ mL. Total output is $500 + 150 + 100 = 750$ mL. Net balance is $+230$ mL. A frequent mistake is forgetting to include emesis or drain output, or not converting ounces correctly.

2558. A practical nurse is totaling a client's intake from 7 a.m. to 3 p.m. The client drank 6 oz of milk, 120 mL of water, and 4 oz of broth. The client also received 250 mL of IV fluid and 60 mL of tube-feeding flushes. How many milliliters of intake should the nurse document for the shift?

- A. 610 mL
- B. 690 mL
- C. 710 mL
- D. 750 mL

Correct Answer: C

The correct total is 710 mL. Convert ounces to milliliters using $1 \text{ oz} = 30 \text{ mL}$: 6 oz milk = 180 mL and 4 oz broth = 120 mL. Then add all intake sources: $180 + 120 + 120 + 250 + 60 = 710$ mL. A common trap is forgetting to count the tube-feeding flushes or failing to convert ounces correctly.

2559. A client is on a 24-hour fluid restriction and asks whether several foods will count toward the daily limit. Which item should the practical nurse include as fluid intake?

- A. A slice of toast
- B. A cup of pudding
- C. A scrambled egg
- D. A banana

Correct Answer: B

Pudding should be counted because it is liquid at room temperature and contributes to fluid intake. Toast, scrambled eggs, and a banana are solid foods and are not typically recorded as fluid intake. The likely trap is choosing a food with some moisture content, but intake and output recording focuses on items considered liquid or that become liquid at room temperature.

2560. The practical nurse is caring for a client who has strict intake and output monitoring. Which observation requires the nurse to follow up because it is most likely to make the output record inaccurate?

- A. The client voids in a urinal and the amount is measured before disposal.
- B. The client has an incontinent episode and the linen is changed without estimating or reporting the urine loss.
- C. The client empties emesis into a calibrated basin kept at the bedside.
- D. The client has a wound drain emptied into a graduated container.

Correct Answer: B

Unreported or unestimated urine loss from incontinence makes the output record inaccurate because some output is not captured. The other actions support accurate measurement by using measurable containers and documenting drain or emesis amounts. The main trap is focusing only on measurable urine and overlooking that incontinence still counts as output and should be reported according to facility practice.

2561. A client has consumed 720 mL of fluid by 2 p.m. The prescription limits fluid intake to 1,200 mL for the day. The dietary aide brings 8 oz of juice with the lunch tray. What is the best action by the practical nurse?

- A. Allow the full juice because the limit applies only to water.
- B. Hold the juice because the client has already reached the daily limit.
- C. Allow the juice and update the amount remaining afterward.
- D. Discard the juice and replace it with gelatin, which does not count as fluid.

Correct Answer: C

The client has 480 mL remaining for the day because $1,200 - 720 = 480$ mL. Eight ounces of juice equals 240 mL, so the client may have it and would still remain within the restriction. Choice A is incorrect because all fluids count, not just water. Choice D is also incorrect because gelatin counts as fluid intake. Choice B reflects a math error about the amount already consumed.

2562. A practical nurse is reviewing a 4-hour record for a client receiving continuous bladder irrigation after urologic surgery. During the 4 hours, 2,000 mL of irrigating solution was infused, and 2,450 mL drained into the urinary catheter bag. What amount should the nurse record as urine output for that 4-hour period?

- A. 450 mL
- B. 2,000 mL
- C. 2,450 mL
- D. 4,450 mL

Correct Answer: A

With continuous bladder irrigation, the drainage bag contains both urine and irrigating solution. To find the true urine output, subtract the irrigation infused from the total drainage: $2,450 - 2,000 = 450$ mL. A common trap is charting the total drainage as urine output, which overestimates kidney output.

2563. At the end of a shift, the practical nurse notes the following: 360 mL oral fluids, 100 mL water with medications, and 500 mL IV fluids. Output includes 650 mL urine and 200 mL liquid stool. Which interpretation of the client's fluid balance is most accurate?

- A. The client has a positive balance of 110 mL.
- B. The client has a negative balance of 110 mL.
- C. The client has a positive balance of 310 mL.
- D. The client has a negative balance of 310 mL.

Correct Answer: A

Total intake is 960 mL ($360 + 100 + 500$). Total output is 850 mL ($650 + 200$). The difference is +110 mL, so the client has a positive fluid balance. The most likely trap is reversing intake and output or forgetting to include medication water as intake.

Care of immobilized clients

2564. A practical nurse is helping care for a client who has been on bed rest for several days after a hip injury. Which action is most appropriate to help prevent footdrop?

- A. Place trochanter rolls along the outer thighs
- B. Apply a footboard to keep the feet in dorsiflexion

- C. Elevate the knees on a pillow for comfort
- D. Keep the client in high-Fowler's position

Correct Answer: B

A footboard helps maintain the feet in dorsiflexion and reduces the risk of plantar flexion contracture, or footdrop. Trochanter rolls help prevent external rotation of the hips, not footdrop. A pillow under the knees can promote flexion contractures, and high-Fowler's position does not address foot positioning.

2565. A client has been immobile in bed for 1 week. Which finding should the practical nurse identify as an early sign of a pressure injury?

- A. Blanchable redness over the coccyx
- B. Nonblanchable redness over a bony prominence
- C. A dry, flaky patch on the forearm
- D. Cool, pale skin on the abdomen

Correct Answer: B

Nonblanchable redness over a bony prominence is an early pressure injury finding and requires prompt intervention. Blanchable redness suggests reactive hyperemia rather than established tissue injury. The other findings are not the classic early sign associated with pressure damage from immobility.

2566. The practical nurse is turning an immobile client in bed. Which technique best reduces shearing force on the client's skin?

- A. Pull the client up quickly using the drawsheet alone
- B. Lift the client with assistance rather than dragging across the bed
- C. Raise the head of the bed to 90 degrees before repositioning
- D. Massage reddened areas after each turn

Correct Answer: B

Lifting the client with help instead of dragging reduces shear and friction, both of which increase skin breakdown risk. Pulling the client across the sheets increases shear. Raising the head of the bed high can also increase sliding and shear. Massaging reddened areas is not appropriate because it can further injure fragile tissue.

2567. A practical nurse is caring for an immobile client who reports calf pain. Which additional finding should most increase concern for deep vein thrombosis?

- A. Warmth and swelling in one lower leg
- B. Bilateral cool feet after linen change
- C. Mild ankle stiffness in the morning
- D. Dependent rubor in both legs

Correct Answer: A

Unilateral warmth and swelling with calf pain are concerning findings for deep vein thrombosis in an immobile client. Bilateral cool feet may reflect temporary exposure or circulation changes, not specifically DVT. Morning stiffness and bilateral color changes are less suggestive of an acute clot.

2568. An immobile client is on a turning schedule. Which action by the practical nurse best helps evaluate whether the plan is effective?

- A. Document that the client tolerated repositioning
- B. Check pressure areas at each turn for redness or breakdown
- C. Offer pain medication once each shift
- D. Keep the client supine for longer rest periods overnight

Correct Answer: B

Inspecting pressure areas with each turn directly evaluates whether repositioning is protecting skin integrity. Documenting tolerance is useful but does not show whether skin damage is being prevented. Routine pain medication and prolonged supine positioning do not evaluate the effectiveness of the turning schedule.

2569. A practical nurse is performing passive range-of-motion exercises for a client with severe weakness. Which action is correct?

- A. Move each joint until resistance or discomfort is met, then stop
- B. Perform exercises rapidly to avoid tiring the client
- C. Support the joint loosely to encourage full extension
- D. Skip joints that the client says feel stiff

Correct Answer: A

Passive range-of-motion exercises should be done slowly and gently, moving the joint to the point of resistance or discomfort and then stopping. Rapid movement can cause injury. Joints should be supported carefully, not loosely. Stiff joints often especially need gentle ROM unless contraindicated.

2570. A client has left-sided weakness after a stroke and slides down in bed frequently. Which intervention would best reduce the risk of skin breakdown?

- A. Keep the head of the bed as high as possible at all times
- B. Reposition on a schedule and use devices that limit sliding
- C. Place a ring-shaped donut cushion under the sacrum
- D. Powder the skin heavily after each bath

Correct Answer: B

Scheduled repositioning and measures that reduce sliding help limit pressure and shear, which are major causes of skin breakdown in immobile clients. Keeping the head of the bed very high promotes sliding. Donut-style cushions can concentrate pressure, and heavy powder use does not address the main problem.

2571. The practical nurse cares for a client on prolonged bed rest. Which outcome most strongly suggests the client is developing constipation related to immobility?

- A. Urinary frequency and urgency
- B. Loose stools after meals
- C. Abdominal distention with infrequent hard stools
- D. Excessive thirst and dry mucous membranes

Correct Answer: C

Immobility slows gastrointestinal motility, making constipation more likely. Abdominal distention and infrequent hard stools fit this pattern. Urinary symptoms point to another system. Loose stools are not typical constipation findings, and thirst with dry mucous membranes suggests fluid deficit rather than bowel motility alone.

2572. A practical nurse is caring for a thin, immobile older adult. Which area should receive especially careful skin assessment because it is a major bony prominence at risk for pressure injury when the client lies on the back?

- A. Scapulae
- B. Umbilicus
- C. Deltoids
- D. Patellae

Correct Answer: A

When a client lies supine, major bony prominences at risk include the scapulae, sacrum, heels, and occiput. The umbilicus and deltoids are not typical high-risk pressure points in this position. The patellae are more associated with pressure risk in the prone position.

2573. An immobile client has been placed in side-lying position. Which placement of pillows is best to promote alignment and reduce pressure?

- A. One pillow under the upper knee only
- B. A pillow behind the back and another between the knees and ankles
- C. A pillow directly under the dependent shoulder
- D. No pillows, to keep the spine flat

Correct Answer: B

In side-lying position, a pillow behind the back can help maintain alignment, and a pillow between the knees and ankles reduces pressure and bony contact. Placing a pillow only under the upper knee is incomplete. A pillow directly under the dependent shoulder may impair alignment, and using no pillows does not adequately protect pressure points.

2574. A practical nurse notes that an immobile client's heels rest continuously on the mattress. Which intervention is most appropriate?

- A. Place a pillow lengthwise under the lower legs to float the heels
- B. Massage the heels every 2 hours to improve circulation
- C. Apply a warm compress to both heels once per shift
- D. Position the feet in plantar flexion for comfort

Correct Answer: A

Floating the heels by placing a pillow under the lower legs helps relieve pressure on the heels, a common site for pressure injury. Massaging reddened or vulnerable tissue can cause further damage. Warm compresses do not remove pressure, and plantar flexion increases risk for contracture rather than preventing skin injury.

2575. The practical nurse is caring for an immobile client who has diminished breath sounds at the lung bases. Which nursing action is most appropriate to help reduce respiratory complications of immobility?

- A. Encourage coughing, deep breathing, and position changes
- B. Limit fluid intake to reduce secretions
- C. Keep the client flat to conserve energy
- D. Avoid activity until breath sounds return to normal

Correct Answer: A

Immobility can lead to retained secretions and atelectatic changes. Encouraging coughing, deep breathing, and repositioning promotes lung expansion and secretion clearance. Limiting fluids may thicken secretions. Keeping the client flat and avoiding activity can worsen respiratory stasis.

2576. A client with prolonged immobility suddenly becomes short of breath and anxious. The oxygen saturation drops from 96% to 88% on room air. Which action should the practical nurse take first?

- A. Reassure the client that anxiety is common with bed rest
- B. Assist the client to ambulate to improve circulation
- C. Assess the client promptly and notify the registered nurse or provider immediately
- D. Massage the client's calves to relieve muscle tightness

Correct Answer: C

Sudden dyspnea, anxiety, and decreased oxygen saturation in an immobile client can indicate a serious complication such as pulmonary embolism. The priority is prompt assessment and immediate escalation. Ambulation is unsafe in this situation, and calf massage is contraindicated if a clot is possible. Reassurance alone delays needed action.

2577. A practical nurse is planning care for an immobile client who is at high risk for contractures. Which plan is most appropriate?

- A.** Maintain joints in flexed positions to promote comfort
- B.** Use positioning and range-of-motion exercises to maintain functional alignment
- C.** Restrict movement of painful joints for the entire shift
- D.** Place pillows under the knees whenever the client is in bed

Correct Answer: B

Contracture prevention focuses on maintaining functional alignment and performing range-of-motion exercises. Keeping joints flexed, avoiding movement for long periods, and placing pillows under the knees can all contribute to flexion contractures rather than prevent them.

2578. During morning care, the practical nurse finds a reddened area over the sacrum of an immobile client. The skin is intact and the redness does not blanch. Which action is best?

- A.** Document the finding and reassess at the end of the shift
- B.** Apply lotion and massage the area for 5 minutes
- C.** Relieve pressure from the area and intensify preventive measures
- D.** Cover the area tightly with a heating pad

Correct Answer: C

Intact, nonblanchable redness indicates pressure-related tissue injury. The best response is to remove pressure and strengthen prevention efforts such as repositioning and skin protection. Waiting until later delays care. Massage and heat can worsen tissue damage.

2579. A practical nurse is assisting an immobile client to move up in bed. Which action is best to reduce friction and skin injury during the repositioning?

- A.** Pull the client upward by the drawsheet while the client's heels stay on the bed
- B.** Use a lift sheet with assistance to raise the client, rather than dragging
- C.** Raise the head of the bed to high-Fowler's position before moving the client
- D.** Ask the client to bend the neck forward while being pulled upward

Correct Answer: B

Using a lift sheet with help to raise the client minimizes dragging across the bed, which reduces friction and skin injury. Pulling the client while body parts rub on the linens increases friction. Raising the head of the bed high before moving can increase sliding. Flexing the neck does not protect the skin.

2580. An immobile client has been on bed rest for 5 days. Which nursing measure is most appropriate to help prevent urinary stasis?

- A.** Encourage adequate fluid intake if not contraindicated
- B.** Limit fluids after breakfast each day
- C.** Keep the client flat in bed most of the time
- D.** Insert an indwelling catheter for accurate output

Correct Answer: A

Adequate fluid intake helps promote urine flow and decreases urinary stasis, which can reduce risk for urinary complications in immobile clients. Restricting fluids can worsen stasis. Remaining flat does not improve bladder emptying. An indwelling catheter increases infection risk and is not used routinely just for immobility.

2581. A practical nurse is caring for an immobile client who is unable to reposition independently. Which schedule best supports pressure injury prevention?

- A.** Reposition the client at least every 2 hours
- B.** Reposition once each shift if the skin is intact
- C.** Reposition only when the client reports discomfort
- D.** Keep the client supine to maintain consistent alignment

Correct Answer: A

Regular repositioning, commonly at least every 2 hours, helps relieve prolonged pressure over bony prominences in clients who cannot move themselves. Waiting until discomfort occurs is unsafe because some clients have reduced sensation or cannot communicate well. Once per shift is too infrequent, and prolonged supine positioning increases pressure risk.

2582. An immobile client is beginning to develop weakness from bed rest. Which activity best helps maintain muscle strength when the client can participate safely?

- A. Passive range-of-motion only
- B. Active range-of-motion exercises
- C. Strict bed rest with pillows for support
- D. Massage of the calves twice daily

Correct Answer: B

Active range-of-motion exercises help preserve muscle strength when the client can move independently. Passive range of motion is useful when the client cannot participate fully, but it does not maintain strength as effectively as active movement. Strict bed rest worsens deconditioning. Calf massage is not used to maintain strength and can be unsafe if a clot is present.

2583. A practical nurse is assisting with care for an immobile client whose coccyx area is at risk for skin breakdown. Which support surface would be most appropriate to reduce pressure while the client is seated in a chair?

- A. A ring-shaped donut cushion
- B. A pressure-redistributing chair cushion
- C. A folded bath blanket under the coccyx
- D. A firm plastic seat insert

Correct Answer: B

A pressure-redistributing chair cushion helps spread pressure more evenly and is appropriate for an immobile client at risk for skin breakdown. Donut cushions can concentrate pressure and impair circulation to surrounding tissue. A folded blanket or hard insert does not provide effective pressure redistribution.

2584. An immobile client is prescribed antiembolism stockings. Which action by the practical nurse is most appropriate?

- A. Roll the stockings down at the top if they feel tight
- B. Remove the stockings once daily to inspect the skin
- C. Apply lotion under the stockings after each assessment
- D. Leave the stockings off while the client remains in bed

Correct Answer: B

Antiembolism stockings should be removed at least daily for skin assessment and care, then reapplied as ordered. Rolling them down can create a constricting band and impair circulation. Lotion under the stockings can damage the material and may increase moisture against the skin. Leaving them off defeats their preventive purpose.

2585. A client has been immobile after surgery and now reports dizziness when first sitting on the side of the bed. What is the best action by the practical nurse?

- A. Help the client stand quickly to improve circulation
- B. Return the client to lying down immediately and document only
- C. Have the client dangle longer and change positions gradually
- D. Encourage the client to hold the breath and bear down

Correct Answer: C

Immobile clients can develop orthostatic hypotension. Allowing the client to dangle and change positions gradually helps the body adjust and can reduce dizziness. Standing quickly increases fall risk. Returning to bed may be needed if symptoms persist, but the best immediate management is slow progression. Bearing down can worsen the problem.

2586. A practical nurse is caring for an immobile client with a weak cough. Which intervention best helps mobilize respiratory secretions?

- A. Encourage turning, coughing, and deep breathing at regular intervals
- B. Limit oral fluids to decrease mucus production

- C. Maintain the client in one comfortable position for rest
- D. Discourage coughing to prevent fatigue

Correct Answer: A

Turning, coughing, and deep breathing help expand the lungs and mobilize secretions, reducing respiratory complications of immobility. Limiting fluids can thicken secretions. Keeping one position for long periods promotes pooling of secretions. Avoiding coughing allows secretions to accumulate.

2587. An immobile client is unable to feed independently and eats only small amounts at meals. Which nutritional approach best supports tissue integrity?

- A. Provide high-protein, nutrient-dense foods the client prefers
- B. Offer only clear liquids between meals
- C. Restrict calories to reduce the workload of digestion
- D. Delay meals until the client states feeling very hungry

Correct Answer: A

Adequate protein and nutrient-dense intake help maintain skin and support tissue repair in immobilized clients. Clear liquids alone are not sufficient nutrition. Calorie restriction can worsen poor intake and healing risk. Waiting for marked hunger may further reduce total intake in weak or ill clients.

2588. A practical nurse is caring for an immobile client with a trochanter at high risk for pressure injury while side-lying. Which position is best to reduce direct pressure on the hip?

- A. Place the client directly on the greater trochanter at a 90-degree side-lying angle
- B. Use a 30-degree lateral tilt with support to keep weight off the trochanter
- C. Raise the knees and rest the full body weight on the hip
- D. Keep the client flat on the back with the knees sharply flexed

Correct Answer: B

A 30-degree lateral tilt helps shift weight away from the greater trochanter and can reduce pressure over that bony prominence. A full 90-degree side-lying position places direct pressure on the hip. The other options do not offload the trochanter appropriately.

2589. An immobile client with limited fluid intake has concentrated urine and has voided only twice during the shift. Which complication of immobility is the client most at risk for developing next if this pattern continues?

- A. Urinary retention and infection
- B. Hyperactive bowel sounds
- C. Improved renal perfusion
- D. Increased bone density

Correct Answer: A

Immobility combined with low fluid intake promotes urinary stasis, concentrated urine, and incomplete emptying, which increase the risk for urinary retention and infection. The other options are unrelated or opposite expected effects of prolonged immobility.

2590. A practical nurse is teaching an assistive personnel member about lifting an immobile client from bed to stretcher. Which statement by the assistive personnel shows correct understanding?

- A. I should pull the client by the arms to prevent back strain
- B. If the client is light, one caregiver can slide the client alone
- C. I should use a transfer device and get enough help for the move
- D. Keeping the bed low during the transfer protects everyone

Correct Answer: C

Safe movement of an immobile client requires appropriate transfer equipment and adequate assistance to protect both the client and staff. Pulling by the arms can injure the client. Sliding alone increases friction and staff injury risk. Beds are usually raised to a safe working height during transfers.

2591. An immobile client has a reddened area on the right elbow that blanches when pressed. What is the most appropriate nursing action?

- A. Document the area and continue the current plan without changes
- B. Massage the reddened area to stimulate circulation
- C. Relieve pressure on the elbow and reassess the skin frequently
- D. Apply a heating pad directly to the elbow

Correct Answer: C

Blanchable redness suggests pressure is affecting the tissue but may still be reversible. The priority is to remove pressure and reassess frequently. Massage can damage vulnerable tissue. Heat can injure the skin. Doing nothing misses an early chance to prevent worsening.

2592. A practical nurse is caring for an immobile older adult who has become newly confused and restless overnight. Which action is most important first?

- A. Check for common physical causes such as hypoxia, urinary retention, or pain
- B. Apply restraints so the client does not climb out of bed
- C. Turn off the lights and limit all stimulation until morning
- D. Assume the behavior is expected because the client is elderly

Correct Answer: A

New confusion in an immobile client should prompt assessment for reversible physical causes, including hypoxia, urinary retention, pain, infection, or other complications. Restraints are not a first response. Assuming confusion is normal in older adults is unsafe. Reducing stimulation may be helpful later but does not address the priority need to assess causes.

2593. An immobile client has been lying in bed for several days. Which assessment finding requires the most immediate follow-up by the practical nurse?

- A. Appetite is decreased at lunch
- B. Mild stiffness is present in the fingers on awakening

- C. The left foot is cool, pale, and has a weaker pulse than earlier
- D. The client reports boredom with the room routine

Correct Answer: C

A cool, pale extremity with a weaker pulse suggests impaired circulation and requires prompt follow-up. This finding can indicate a serious vascular problem. Decreased appetite, mild morning stiffness, and boredom are concerns but are not as urgent as possible compromised perfusion.

2594. A practical nurse is turning an immobilized client from the back to the left side. To maintain body alignment and reduce strain on the lower back, where should the nurse place a pillow after the turn?

- A. Directly under the client's knees
- B. Behind the client's back for support
- C. Under the client's head only
- D. Beneath the dependent shoulder

Correct Answer: B

A pillow behind the client's back helps maintain the side-lying position and supports proper alignment after turning. A pillow under the knees is more useful when the client is supine, not as the main support after a side turn. Supporting only the head is incomplete, and placing a pillow beneath the dependent shoulder can increase pressure rather than relieve it.

2595. An immobilized client has been on bed rest for several days. Which finding most strongly suggests a developing complication of immobility affecting the lungs?

- A. Clear urine with no odor
- B. Diminished breath sounds at the lung bases
- C. Increased appetite at lunchtime
- D. Mild tenderness over the wrist

Correct Answer: B

Diminished breath sounds at the bases can indicate poor lung expansion and retained secretions, both common respiratory complications of immobility. Clear urine is not concerning here, appetite does not point to a lung problem, and wrist tenderness is unrelated to the most likely respiratory effect of bed rest.

2596. A practical nurse is caring for an immobilized client who cannot move independently. Which action is most appropriate to help prevent footdrop?

- A. Place the feet in a neutral position with supportive devices as prescribed
- B. Massage the calves every 2 hours
- C. Keep a pillow pressed firmly against the soles of the feet all day
- D. Elevate the heels so the toes point downward

Correct Answer: A

Keeping the feet in a neutral position with appropriate support helps prevent plantar flexion contracture, or footdrop. Calf massage is not used for this purpose and may be unsafe in clients at risk for thrombosis. Pressing a pillow firmly against the soles is not reliable positioning, and letting the toes point downward promotes the very problem the nurse is trying to prevent.

2597. During morning care, a practical nurse notes that an immobilized client's left heel is pale, cool, and in contact with the mattress. Which action should the nurse take first?

- A. Rub the heel briskly to increase circulation
- B. Reposition the client so the heel is offloaded from pressure
- C. Apply powder to keep the area dry
- D. Keep the heel exposed to air for the rest of the shift

Correct Answer: B

The priority is to remove pressure from the heel immediately. Offloading the heel helps prevent further tissue damage. Brisk rubbing can injure fragile tissue, powder does not address the cause, and leaving the area exposed without relieving pressure is not enough.

2598. An immobilized client is receiving passive range-of-motion exercises. Which observation indicates the practical nurse should stop the movement and report the finding?

- A. The joint moves smoothly through the expected range
- B. The client reports slight stretching in the muscle
- C. The client suddenly reports sharp pain during the movement

D. The extremity feels warm after exercise

Correct Answer: C

Sharp pain during passive range-of-motion is an abnormal finding and may indicate injury or joint problems, so the movement should be stopped and the finding reported. Smooth movement is expected, mild stretching may occur, and warmth after exercise can be a normal response. The trap is confusing mild stretching with dangerous pain; pain that is sudden and sharp is the warning sign.

2599. A practical nurse is helping plan care for an immobilized client who is at risk for constipation related to decreased activity. Which nursing approach is best to support normal bowel elimination?

- A. Limit fiber to reduce stool bulk
- B. Encourage fluids, fiber, and activity as tolerated
- C. Offer a bedpan only at the end of the day
- D. Keep the client flat after meals

Correct Answer: B

Immobility slows peristalsis, so encouraging fluids, adequate fiber, and movement as tolerated best supports bowel function. Limiting fiber can worsen constipation, delaying toileting can disrupt normal elimination patterns, and lying flat after meals does not promote bowel motility. This item tests whether the nurse recognizes the combined preventive approach rather than focusing on only one factor.

2600. An immobilized client has been on bed rest for 1 week. When the practical nurse assists with hygiene, the client's right calf appears larger than the left and is warm to the touch. What is the nurse's best action?

- A. Massage the calf to relieve muscle tightness
- B. Ambulate the client immediately to improve circulation
- C. Report the finding promptly and limit manipulation of the leg
- D. Elevate the leg and apply a heating pad

Correct Answer: C

A unilateral warm, enlarged calf in an immobilized client suggests possible deep vein thrombosis. The nurse should report the finding promptly and avoid massaging or unnecessary manipulation because that could increase the risk of dislodging a clot. Immediate ambulation and applying heat are not the safest first actions when DVT is suspected.

Pharmacological Therapies

Medication rights

2601. A nurse is preparing to give digoxin to an adult client. Which action best supports the right assessment before administration?

- A. Check the apical pulse for 1 full minute
- B. Ask the client to drink a full glass of water
- C. Position the client flat in bed
- D. Offer a snack to prevent nausea

Correct Answer: A

Digoxin can slow the heart rate, so checking the apical pulse for 1 full minute helps confirm it is safe to give. The other actions may be reasonable in some situations, but they do not specifically address the right assessment that is essential before this medication.

2602. A nurse enters a client's room with a cup containing one tablet and says, "I'm here with your morning medicine." The client says, "What is it?" Which response best supports the client's medication rights?

- A. It's the same thing you usually take, so go ahead and swallow it.
- B. Let me verify the medication and explain what it is before you take it.
- C. If you refuse now, you may have to wait until tomorrow for it.
- D. The provider ordered it, so you need to take it.

Correct Answer: B

Clients have the right to know what medication they are receiving and to receive accurate information before administration. Verifying the medication and explaining it supports informed participation and safe administration. The other responses pressure the client or bypass the client's right to information.

2603. A nurse scans a client's identification band and the barcode on a prefilled syringe. The scanner gives an alert that the medication is not scheduled for this client. What should the nurse do first?

- A. Give the medication because it came from the medication cart
- B. Ask another nurse to override the alert
- C. Stop and compare the medication with the MAR and client identifiers
- D. Document the medication as given and monitor the client closely

Correct Answer: C

An alert during barcode scanning should prompt the nurse to stop and verify the medication, the MAR, and the client's identity. This supports the right patient and right medication. Giving, overriding, or documenting before resolving the discrepancy risks a medication error.

2604. A client says, "I don't want that pain pill right now." Which nursing action best supports the client's medication rights?

- A. Crush the medication and mix it in pudding
- B. Tell the client the dose cannot be skipped once prepared
- C. Document the refusal and assess the reason for declining
- D. Leave the medication at the bedside in case the client changes their mind

Correct Answer: C

Clients have the right to refuse medication. The nurse should respect the refusal, assess why the client is declining, and document appropriately. Crushing and disguising the medication or leaving it at the bedside violates safe medication practices and client autonomy.

2605. A nurse is preparing to administer 12 units of regular insulin subcutaneously. The vial label reads U-100. Which action best supports medication rights?

- A. Use a tuberculin syringe to improve accuracy
- B. Use an insulin syringe calibrated for U-100 insulin
- C. Draw up the insulin in a 3-mL syringe with a 22-gauge needle
- D. Measure 0.12 mL in a standard medication syringe

Correct Answer: B

A U-100 insulin syringe is designed specifically for U-100 insulin and helps ensure the right dose. A tuberculin or standard syringe increases the risk of dosing error. A common trap is trying to convert the dose into mL and use a regular syringe instead of the insulin-specific device.

2606. A nurse has medications prepared for four clients during a busy medication pass. Which action is most consistent with safe medication rights?

- A.** Carry all four clients' medications on one tray to save time
- B.** Administer each client's medications immediately after identifying that client
- C.** Ask the unlicensed assistive personnel to confirm which medication belongs to each client
- D.** Place labeled medication cups on each bedside table and return later with water

Correct Answer: B

Administering medications to one identified client at a time supports the right patient and reduces mix-ups. Carrying multiple clients' medications together, delegating identification of medications, or leaving medications at the bedside increases error risk.

2607. A provider writes an order for morphine 2 mg IV every 4 hours as needed for pain. The nurse reviews the MAR and sees that 2 mg IV was given 1 hour ago by the previous nurse. The client now rates pain as 8 out of 10. Which action best supports medication rights?

- A.** Administer another 2 mg because the client still has pain
- B.** Hold the medication and reassess pain after the 4-hour interval has passed
- C.** Contact the provider for further instructions because the medication is requested before the ordered interval
- D.** Give half the dose now and half the dose in 3 hours

Correct Answer: C

The prescribed interval is every 4 hours, so another dose 1 hour later would violate the right time and right dose. Because the client still needs pain management, the best action is to contact the provider for further instructions rather than simply withholding help.

Reassessing alone ignores the client's current need, and splitting the dose is not ordered.

2608. A nurse removes an oral antibiotic from its unit-dose package and places it into a medication cup. Before entering the client's room, the nurse is interrupted and can no longer remember which client it was intended for. What is the best action?

- A.** Administer it to the next client scheduled for an antibiotic
- B.** Ask the charge nurse to identify the pill by color and shape
- C.** Discard the medication according to policy and prepare a new dose
- D.** Return the pill to the unit-dose package and initial the MAR

Correct Answer: C

If the nurse cannot positively verify a medication for the correct client, the medication should not be given. Discarding it according to policy and preparing a new verified dose protects the right patient and right medication. Identifying by appearance alone is unsafe, and returning a removed pill to the package is not appropriate.

2609. A nurse is about to administer an antihypertensive medication. The client's blood pressure is 88/54 mm Hg, and the client reports dizziness. Which action best reflects the nurse's responsibility related to medication rights?

- A.** Give the medication because it is scheduled
- B.** Hold the medication and notify the provider after reviewing relevant findings
- C.** Ask the client to stand and walk to see whether the dizziness improves
- D.** Administer half the dose to reduce the chance of hypotension

Correct Answer: B

Medication rights include using appropriate assessment data before giving a drug. A low blood pressure and dizziness suggest the medication may be unsafe at this time. The nurse should hold it and report the findings. Giving it as scheduled or changing the dose independently would be unsafe.

2610. A nurse is preparing to administer furosemide 40 mg IV push. The medication label is correct, but the IV tubing is connected to a bag containing a medication that is incompatible with furosemide. Which action is most appropriate?

- A. Give the IV push through the line anyway because the drug and dose are correct
- B. Pause the incompatible infusion and use proper line management before administering the furosemide
- C. Mix the furosemide into the hanging IV bag to dilute it
- D. Change the MAR entry so the medication can be given by mouth instead

Correct Answer: B

Medication rights include safe administration, which includes giving the drug in a compatible manner through the correct route. If the current line contains an incompatible medication, the nurse should manage the line appropriately before administration. The right drug and dose are not enough if the administration method is unsafe.

2611. A nurse enters a room to give a scheduled medication. The client states a full name that matches the MAR, but the identification band shows a different date of birth than the MAR. The client says, "That birthday on my band is wrong, but I'm definitely the right person." What should the nurse do?

- A. Give the medication because the client confirmed the name
- B. Ask a family member to verify the date of birth
- C. Withhold the medication until the identification discrepancy is resolved
- D. Administer the medication and file an incident report afterward

Correct Answer: C

A discrepancy between the identification band and the MAR means the nurse cannot fully verify the right patient. The medication should be withheld until the discrepancy is corrected. A common trap is accepting partial confirmation, such as name alone, when required identifiers do not match.

2612. A nurse obtains a verbal order over the phone for a high-alert medication during an urgent situation. Which action best helps protect medication rights at the time the order is received?

- A. Write down the order and administer it immediately without repeating it back
- B. Repeat the order back to the prescriber for verification and clarify any unclear parts
- C. Ask another nurse to decide what dose sounds most appropriate
- D. Wait until the prescriber arrives later to confirm whether the order was correct

Correct Answer: B

Read-back verification helps confirm the right medication, dose, and route when receiving a verbal order, especially for high-alert medications. Acting without verification increases the chance of error. Delaying needed clarification or relying on another nurse's guess does not protect safe administration.

2613. A nurse prepares to administer a liquid medication. The MAR lists 10 mL by mouth. The medication cup has been filled to 15 mL, and the nurse realizes this just before giving it. Which action is best?

- A. Give the full amount because the medication has already been poured
- B. Estimate the difference by having the client drink part of it
- C. Discard the incorrect amount according to policy and measure a new 10-mL dose
- D. Add water until the cup looks closer to the right amount

Correct Answer: C

If the prepared dose is incorrect, the safest response is to discard it according to policy and prepare the exact ordered dose again. Guessing, diluting, or giving the full amount violates the right dose. The key is accuracy, not convenience.

2614. A nurse gives an antiemetic to a client at 0900 for nausea. At 0945, the client says the nausea is improving. Which action reflects an important medication right that occurs after administration?

- A. Document that the medication was given and no further action is needed
- B. Evaluate and document the client's response to the medication
- C. Ask the next shift to decide whether the drug was effective
- D. Avoid asking about symptoms again so the client can rest

Correct Answer: B

Safe medication practice includes evaluating whether the medication had the intended effect and documenting the response. This supports ongoing decisions about treatment. A common trap is thinking medication rights end once the drug has been administered.

2615. A nurse is preparing to give a medication through a feeding tube. The order is for a capsule labeled enteric-coated. Which action is most appropriate?

- A. Open the capsule and dissolve the contents in warm water
- B. Crush the capsule and flush it well through the tube
- C. Hold the dose and consult the provider or pharmacy for an appropriate alternative
- D. Mix the capsule with tube feeding formula to reduce irritation

Correct Answer: C

Enteric-coated medications should not be crushed or altered for tube administration unless specifically approved, because doing so can change where and how the drug is released. The best action is to hold the dose and obtain an appropriate alternative. The trap is focusing on the route convenience rather than the medication's intended form.

2616. A nurse is preparing to administer a transdermal nitroglycerin patch. Which action best supports the client's medication rights related to administration safety?

- A. Apply the new patch over the existing patch so the dose is not missed
- B. Remove the old patch before applying the new patch and document the site
- C. Cut the new patch in half if the client reports a headache
- D. Place the patch on the client's gown to reduce skin irritation

Correct Answer: B

Removing the old patch before applying the new one helps ensure the right dose and prevents accidental overdose from multiple patches remaining in place. Documenting the site also supports safe rotation and monitoring. Applying a new patch over an old one risks excess medication. Cutting a transdermal patch is unsafe unless specifically allowed, and placing it on clothing prevents proper absorption.

2617. A nurse is preparing medications for a client who says, "The yellow pill doesn't look like the one I usually take at home." Which action is most appropriate?

- A. Tell the client that manufacturers use different colors and give the pill as ordered
- B. Ask another nurse whether the pill looks familiar and proceed if the nurse agrees
- C. Hold the medication until the nurse compares the medication label and MAR with the order
- D. Discard the pill and document that the client refused the medication

Correct Answer: C

The nurse should verify the medication against the MAR and order before administration. This protects the right medication and respects the client's concern. Reassuring the client without checking skips an important safety step. Asking another nurse for a visual opinion is not a substitute for verification. The situation is not yet a true refusal; it is a need to clarify before giving the drug.

2618. A nurse is administering an oral medication and notices the client quickly place the tablet under the tongue, then turn away. Which action best supports medication rights?

- A. Document the medication as given because the tablet entered the client's mouth
- B. Offer water and verify that the client actually swallowed the medication if swallowing is intended
- C. Leave the room to preserve the client's privacy and return later
- D. Immediately request a replacement dose from the pharmacy

Correct Answer: B

The nurse must ensure the medication is taken as intended when administration requires swallowing. Offering water and verifying ingestion supports the right administration and accurate documentation. Documenting it as given without confirming is unsafe. Leaving the room ignores a possible problem, and requesting a replacement dose is premature before determining whether the medication was actually taken.

2619. A provider writes an order for acetaminophen 650 mg PO every 6 hours as needed for fever. The MAR shows the client received 1,000 mg from another PRN order 2 hours ago. The client now has a temperature of 38.4°C (101.1°F). Which action is best?

- A. Administer 650 mg because the current order allows it
- B. Administer 325 mg so the total dose is lower
- C. Hold the medication and clarify the PRN orders before giving another dose
- D. Give the medication only if the client asks for it

Correct Answer: C

The nurse should pause and clarify because another acetaminophen-containing PRN dose was recently given. This protects the right dose and timing and helps avoid duplicate therapy. Giving the full or partial dose without resolving the overlap is unsafe. Waiting for the client to ask does not address the dosing conflict.

2620. A nurse prepares to give an extended-release oral tablet to a client who says, "I have trouble swallowing pills. Can you crush it in applesauce?" What is the nurse's best response?

- A. Crush the tablet and mix it well so the full dose is taken
- B. Split the tablet into pieces without crushing it
- C. Do not crush the tablet and check for another form or route with the provider or pharmacy
- D. Dissolve the tablet in water and give it slowly

Correct Answer: C

Extended-release tablets generally should not be crushed because doing so can alter how the drug is released and lead to unsafe dosing. The safest action is to hold that form and seek an appropriate alternative. Splitting or dissolving has the same basic problem if the formulation is not designed for it.

2621. A nurse is preparing to administer insulin at 0730. The client's breakfast tray has not arrived, and the client says, "I feel shaky if I take insulin before eating." Which action is most appropriate?

- A. Give the insulin now to stay on schedule
- B. Hold the insulin, assess the situation, and coordinate timing with food availability
- C. Ask the client to sign a refusal form
- D. Give half the dose until the meal tray arrives

Correct Answer: B

The nurse should consider timing and client safety before administration. Coordinating insulin with food intake helps support the right time and reduces risk for hypoglycemia. Giving it only to keep the schedule ignores important assessment. This is not necessarily a refusal, and changing the dose independently is outside the nurse's role.

2622. A nurse is about to instill prescribed ear drops into an adult client's right ear. Which action best supports the right route and technique?

- A. Pull the pinna up and back before instilling the drops
- B. Pull the pinna down and back before instilling the drops

- C. Insert the dropper tip until it touches the ear canal
- D. Ask the client to swallow during instillation

Correct Answer: A

For an adult, pulling the pinna up and back helps straighten the ear canal for proper instillation. Pulling down and back is used for younger children. Touching the canal with the dropper can contaminate the medication and injure tissue. Swallowing is unrelated to ear drop administration.

2623. A nurse receives a unit-dose package labeled hydralazine 25 mg. The MAR lists hydroxyzine 25 mg PO at this time. Both medications are stocked in the medication cart. What should the nurse do?

- A. Administer the medication because the dose matches the MAR
- B. Administer the medication if the client usually takes it at home
- C. Stop and obtain the correct medication after rechecking the label against the MAR
- D. Ask the client whether hydralazine sounds familiar

Correct Answer: C

This is a look-alike/sound-alike risk. Even though the dose is the same, the medication name does not match the MAR. The nurse must stop and obtain the correct medication to protect the right medication. Client recall and home routine are not reliable substitutes for verification.

2624. A nurse administers the first dose of an IV antibiotic to a client. Fifteen minutes later, the client reports itching and shortness of breath. Which action best reflects the nurse's responsibility related to medication rights?

- A. Slow the infusion and continue to observe the client
- B. Stop the medication and respond immediately to the adverse reaction
- C. Document the symptoms and reassess after the infusion is complete
- D. Tell the client mild itching is common with first doses

Correct Answer: B

The client's symptoms suggest a possible acute adverse reaction. The nurse must stop the medication and respond at once, which supports ongoing medication rights through adverse response monitoring after administration. Slowing or continuing the infusion can worsen harm. Delayed reassessment is unsafe.

2625. A nurse is preparing to administer an oral medication and notices that the MAR says "do not substitute" for a specific brand. The pharmacy has sent a different manufacturer's product with a different appearance. Which action is best?

- A. Administer it because medications with the same generic name are always interchangeable
- B. Hold the dose and clarify the discrepancy with the pharmacy before administration
- C. Give the medication after explaining that the color and shape vary by manufacturer
- D. Document the medication as unavailable and skip the dose

Correct Answer: B

A discrepancy between the MAR instruction and the dispensed product should be clarified before administration. This protects the right medication and follows the record as written. Assuming all products are interchangeable ignores the specific MAR note. Skipping the dose without clarification is also inappropriate unless directed after review.

2626. A nurse is preparing to give a medication that requires the client's apical pulse to be checked first. During assessment, the nurse obtains a pulse much lower than expected and the client says, "I feel fine." What is the best action?

- A. Administer the medication because the client has no symptoms
- B. Ask another nurse to give the medication instead
- C. Withhold the medication and follow facility procedure for an out-of-range pre-administration finding
- D. Give half the dose and reassess the pulse in 30 minutes

Correct Answer: C

Medication rights include the right assessment before administration. If required assessment data are out of range, the nurse should withhold the drug and proceed according to policy, such as rechecking and notifying the provider as appropriate. Lack of symptoms does not override an abnormal assessment. The nurse should not independently reduce the dose.

2627. A nurse is handing a client two oral medications. The client says, "I only want the white one. I don't take the pink one anymore." Which action is most appropriate?

- A.** Tell the client both are prescribed and must be taken together
- B.** Separate the medications, hold the declined one, and explore the client's concern before documenting and following up
- C.** Document refusal of both medications because they were offered together
- D.** Return later and attempt to give both medications without further discussion

Correct Answer: B

The client has the right to refuse a specific medication. The nurse should identify which one is being declined, hold that medication, assess the reason, provide needed teaching, and document appropriately. Forcing both together or treating the refusal as refusal of all medications ignores the client's rights and may create an unsafe situation.

2628. A nurse is preparing to administer a medication from a multidose vial. The vial label shows the drug name and concentration clearly, but the beyond-use date written on the vial has passed. Which action is best?

- A.** Administer the medication if the solution looks clear
- B.** Administer the medication if it has been stored in a locked cart
- C.** Discard the vial according to policy and obtain a new supply
- D.** Use the vial for this dose only and send it to pharmacy afterward

Correct Answer: C

An expired beyond-use date means the vial should not be used. The nurse should discard it per policy and obtain a new vial to protect the right medication and client safety. Appearance and storage location do not override expiration limits.

2629. A nurse is preparing medications for a client who has both an oral potassium chloride order and an IV fluid containing potassium infusing. The latest laboratory result shows a potassium level above the expected reference range. Which action is most appropriate?

- A.** Administer the oral potassium because it is routinely scheduled
- B.** Hold the oral potassium and clarify the order in light of the current laboratory result
- C.** Stop the IV infusion and discard the bag immediately
- D.** Give the oral potassium with food to reduce stomach upset

Correct Answer: B

A high potassium level is a critical pre-administration cue when potassium is ordered. The nurse should hold the oral dose and clarify before giving more potassium. Administering it because it is scheduled ignores the right assessment. The nurse should not independently discard IV therapy without further direction unless immediate emergency circumstances require it.

2630. A nurse is reviewing the MAR before a medication pass and notices that a newly prescribed opioid is listed for two clients with similar last names in adjacent rooms. The nurse has one dose in hand and is walking toward the rooms. Which action best supports medication rights?

- A.** Ask the first client who requests pain medicine whether the opioid is for them
- B.** Administer the dose to the client whose room number is closer to the medication cart
- C.** Return to the MAR and medication label, then identify the correct client using approved identifiers before administration
- D.** Split the dose until the nurse confirms which client reported more pain

Correct Answer: C

Similar names create a high risk for wrong-client error. The nurse should pause, compare the medication label with the MAR, and use approved identifiers to confirm the correct client before giving the opioid. Room proximity, client request, and pain severity do not establish identity.

2631. A nurse is preparing to give digoxin 0.125 mg PO to a client at 0900. The MAR also lists, "Hold for apical pulse below 60/min." The nurse checks the client's apical pulse for 1 full minute and counts 54/min. The client denies symptoms and asks for the medication. What should the nurse do next?

- A.** Administer the digoxin because the client has no symptoms
- B.** Hold the medication and notify the provider of the pulse rate
- C.** Give half the dose and recheck the pulse in 30 minutes
- D.** Ask another nurse to listen to the pulse, then give the medication if the count is above 50/min

Correct Answer: B

The nurse should follow the administration parameters on the MAR and hold digoxin when the apical pulse is below 60/min, then notify the provider. This supports the right medication and right assessment before administration. A common trap is thinking an asymptomatic client can still receive the dose, but ordered hold parameters must be followed.

2632. A nurse brings a scheduled subcutaneous heparin injection to a client's room. The client states, "My doctor stopped that yesterday because I was bruising." The MAR still shows the dose due now, and there is no discontinuation order documented. What is the nurse's best action?

- A.** Administer the heparin because the MAR shows it is due
- B.** Document the client's statement and omit the dose permanently
- C.** Hold the dose temporarily and clarify the order before administering
- D.** Ask the client's family whether the medication was stopped

Correct Answer: C

The nurse should not administer a medication based only on the MAR when the client reports a recent change that is not yet verified. The safest action is to pause and clarify the order, supporting the right medication and right documentation. Giving the dose without clarification could expose the client to harm. Permanently omitting it based only on the client's statement is also inappropriate without verification.

2633. A nurse is preparing to administer 10 units of regular insulin subcutaneously. After drawing up the dose, the nurse is called away to assist with another client. Twenty minutes later, the nurse returns and cannot remember whether the insulin syringe in the medication room was prepared for the current client or another client. What is the best action?

- A.** Administer the insulin if the syringe is labeled with the medication name only
- B.** Ask another nurse whether the syringe was prepared for this client
- C.** Discard the syringe and prepare a new dose using the MAR
- D.** Estimate whether the syringe contains 10 units and give it if the volume looks correct

Correct Answer: C

If the nurse cannot verify that a prepared medication is the correct dose for the correct client, the medication should not be administered. The safe action is to discard it according to policy and prepare a new dose from the MAR. This protects the right client, right medication, and right dose. A likely trap is relying on memory, appearance, or another person's guess instead of direct verification.

2634. A nurse is ready to administer a newly prescribed oral antibiotic. Before giving the first dose, the nurse asks the client about allergies. The client says, "I got hives after taking penicillin when I was younger, but I forgot to mention it." The allergy section of the chart is blank. What should the nurse do first?

- A.** Give the antibiotic because the allergy is not documented in the chart

- B.** Hold the medication and verify the allergy before administration
- C.** Administer the medication with food to reduce the chance of a reaction
- D.** Ask the client whether the hives were severe, then decide whether to give the dose

Correct Answer: B

A reported allergy must be taken seriously even if it is not yet documented. The nurse should hold the medication and verify the allergy before administration to protect the right medication and prevent harm. The trap is assuming a blank chart means it is safe to proceed, but the client's current report is an important cue that requires follow-up first.

2635. A nurse is giving medications to a client who is alert and oriented. The client looks at one tablet and says, "I don't want that one." The medication is a scheduled antihypertensive, and the client's blood pressure is 168/94 mm Hg. Which action best protects the client's medication rights?

- A.** Explain the purpose and potential consequence of refusing it, then respect the client's decision and document the refusal
- B.** Crush the tablet and mix it into pudding so the client will receive the prescribed dose
- C.** Tell the client the medication is required because the blood pressure is elevated
- D.** Leave the medication at the bedside and ask the client to take it later

Correct Answer: A

Clients have the right to refuse medication. The nurse should provide appropriate teaching, assess the reason for refusal as needed, and document the refusal rather than forcing or disguising the medication. This respects the right to refuse while still supporting informed decision-making. The trap is focusing only on the elevated blood pressure and ignoring client autonomy and safe medication practice.

Medication reconciliation and MAR use

2636. A practical nurse is admitting a client from home and begins medication reconciliation. Which action is most appropriate first?

- A.** Compare the provider's new admission prescriptions with the medications the client was taking before admission
- B.** Ask the pharmacy to hold all home medications until discharge
- C.** Remove all over-the-counter medications from the client's list
- D.** Document only the medications the client can remember by name

Correct Answer: A

Medication reconciliation begins by creating the most accurate current medication list and comparing it with newly ordered medications to identify omissions, duplications, interactions, or incorrect doses. Holding all home medications without review is unsafe, and over-the-counter drugs and supplements must also be considered because they can affect treatment.

2637. Before administering a scheduled medication, the nurse reviews the medication administration record (MAR). Which finding requires clarification before the medication is given?

- A.** The medication is listed by its generic name
- B.** The dose on the MAR does not match the dose on the provider's prescription
- C.** The medication is scheduled for administration with morning vital signs
- D.** The MAR shows the medication route in abbreviations used by the facility

Correct Answer: B

A discrepancy between the MAR and the provider's prescription must be clarified before administration because the MAR should accurately reflect the order. Generic naming alone is not a problem, and routine scheduling or approved facility abbreviations are not the key safety concern here.

2638. A client says, "I take a little white blood pressure pill at home, but I don't know the name." What is the best nursing response during medication reconciliation?

- A. Skip the medication since the client cannot name it exactly
- B. Record it as an allergy because the name is unknown
- C. Ask the client to bring in the pill bottle or contact the client's pharmacy for verification
- D. Document the medication as a diuretic because many blood pressure pills are diuretics

Correct Answer: C

When the client cannot provide a precise medication name, the nurse should seek reliable verification, such as pill bottles, a family-supplied list, or the pharmacy. Guessing the drug class or omitting the medication can lead to dangerous errors.

2639. The MAR shows that a 1400 antibiotic dose was signed off by the previous nurse, but the IV bag is still full and the client states, "I haven't had anything since lunch." What should the practical nurse do first?

- A. Sign the MAR again after hanging the medication
- B. Notify the charge nurse or provider and treat the entry as a possible documentation error or missed dose
- C. Erase the previous nurse's documentation and chart the correct time
- D. Wait until the next scheduled dose because the MAR already shows administration

Correct Answer: B

A signed MAR with evidence the medication may not have been given requires immediate follow-up as a possible missed dose or documentation error. The nurse should not alter another clinician's record or assume the medication was administered just because it was charted. Resigning the MAR without clarification would compound the error.

2640. During transfer from the emergency department to the medical unit, which information is most important to include in medication reconciliation?

- A. Only medications given in the last hour

- B. Only medications prescribed by the current provider

- C. The complete current medication list, including home medications, newly ordered drugs, over-the-counter products, and supplements

- D. The client's preferred pharmacy and insurance plan without the medication names

Correct Answer: C

Medication reconciliation across transitions of care requires the most complete and current medication list possible. This includes home medications, new prescriptions, over-the-counter products, and supplements because each can affect safety. Focusing on only recent doses or only current-provider prescriptions misses important information.

2641. A client is discharged with a new prescription for warfarin. During discharge medication reconciliation, the home medication list still includes daily aspirin, which the provider did not mention continuing. What is the nurse's best action?

- A. Assume aspirin should continue because it is over the counter
- B. Tell the client to decide whether to continue aspirin based on usual habits
- C. Clarify with the provider whether aspirin should be continued, stopped, or specifically included in the discharge instructions
- D. Remove aspirin from the record without notifying anyone

Correct Answer: C

At discharge, unresolved discrepancies between old and new medication regimens must be clarified. Aspirin may increase bleeding risk with warfarin, so the nurse should not assume it continues or stops without confirmation. Silent removal from the list also creates risk.

2642. The MAR lists digoxin 0.125 mg PO daily at 0900. Before administration, the nurse notes the apical pulse is 54/min. Which action is most appropriate?

- A. Administer the medication because it is already on the MAR
- B. Hold the medication and follow agency policy or obtain further direction based on the low pulse

C. Give half the dose and document the lower pulse

D. Ask the client whether the medication is usually taken at home before deciding

Correct Answer: B

The MAR is a guide for ordered medications, but the nurse must still assess whether it is safe to administer. A low apical pulse is a key pre-administration concern with digoxin, so the medication should be held and the situation clarified according to policy or further instruction. The trap is treating the MAR as permission to give without clinical assessment.

2643. A nurse is reviewing a client's home medication list during admission. Which item is most likely to be unintentionally omitted unless the nurse asks directly?

- A.** A prescribed inhaler used twice a day
- B.** An herbal sleep supplement taken nightly
- C.** An insulin pen stored in the refrigerator
- D.** A seizure medication taken every morning

Correct Answer: B

Clients often do not think of herbal products and supplements as medications, so these are commonly missed unless specifically requested during reconciliation. Prescription inhalers, insulin, and seizure medications are more likely to be recognized as medications by the client.

2644. A provider prescribes metoprolol 25 mg PO now. The MAR has not been updated yet. Which action by the nurse is best?

- A.** Wait to give the medication until it appears on the next printed MAR, even if the order is valid and urgent
- B.** Administer the medication from the validated current order per facility policy and ensure the MAR is updated appropriately
- C.** Write the medication onto the MAR without verifying the order details
- D.** Ask another nurse to give the medication so the MAR discrepancy becomes that nurse's responsibility

Correct Answer: B

A valid current order may need to be acted on before the MAR is updated, following facility policy and appropriate verification. The MAR supports safe administration but does not replace the actual prescription. Waiting unnecessarily may delay care, while manually adding orders without proper process creates risk.

2645. During medication reconciliation, the nurse notes that the client takes two medications at home with similar names: hydroxyzine and hydralazine. What is the best way to reduce the risk of an error?

- A.** Document only one because the names are probably duplicates
- B.** Ask the client which one sounds more familiar and record that single drug
- C.** Verify each medication's name, purpose, dose, and schedule using a reliable source such as containers or pharmacy records
- D.** List both medications without doses because names alone are enough for reconciliation

Correct Answer: C

Look-alike or sound-alike medication names require careful verification of the exact name plus the purpose, dose, and schedule. Assuming duplication or documenting incomplete information increases error risk. The likely trap is treating similar names as interchangeable.

2646. A client transferred from a long-term care facility arrives with a medication list and a separate hospital transfer record. One document lists levothyroxine 75 mcg daily, and the other lists 125 mcg daily. Which action is most appropriate?

- A.** Administer 125 mcg because larger doses are often used in older adults
- B.** Average the two doses and request a one-time dose close to the midpoint
- C.** Hold the medication until the discrepancy is clarified with a reliable source
- D.** Administer 75 mcg because the lower dose is always the safer choice

Correct Answer: C

Conflicting medication records must be clarified before administration. Choosing the higher dose, the lower dose, or an average would be unsafe because none is verified. The safest action is to pause and reconcile the discrepancy with a reliable source such as the sending facility, pharmacy, or prescriber.

2647. Which documentation on the MAR best reflects safe medication administration practice after a PRN pain medication is given?

- A.** Initial the MAR and document the reason for administration and follow-up response according to facility policy
- B.** Initial the MAR only, because reassessment belongs in the provider note
- C.** Document the medication only if the client reports complete pain relief
- D.** Wait until the end of the shift to chart all PRN medications together

Correct Answer: A

PRN medications require clear documentation of why the medication was given and whether it was effective, along with MAR documentation per policy. Delayed charting or documenting only if relief is complete can hide important response data and create safety problems.

2648. A nurse preparing the evening medications notices the MAR lists lisinopril 10 mg PO daily twice: once at 0900 and again at 2100, but the provider's prescription reads lisinopril 10 mg PO daily. The morning dose was already given. What should the nurse do?

- A.** Administer the 2100 dose because it appears on the MAR
- B.** Skip the 2100 dose and document that the order was probably entered incorrectly without further action
- C.** Clarify the duplicate MAR entry before administration and withhold the evening dose unless a valid order supports it
- D.** Give 5 mg at 2100 so the daily total remains close to the intended dose

Correct Answer: C

The provider's prescription indicates once daily dosing, so a duplicate MAR entry suggests a transcription or entry error. The nurse should clarify before giving any additional dose. The main trap is following the MAR mechanically when it conflicts with the actual order.

2649. During discharge teaching, a client says, "I take my old bottle and the new bottle together so I don't waste medicine." Which response by the nurse is best?

- A.** That is acceptable if both bottles have your name on them
- B.** Take only the medications listed on your updated discharge medication list, and ask questions about any home medication that is not clearly marked to continue
- C.** Take the old medications until they are finished, then start the new prescriptions
- D.** Double-check with a family member instead of calling the clinic

Correct Answer: B

Discharge medication reconciliation is meant to give the client a clear, updated regimen. The client should follow the current discharge list and clarify any uncertainty, rather than combining old and new bottles. Continuing both sets can cause duplication or dangerous interactions.

2650. A practical nurse is using the electronic MAR to administer insulin before breakfast. The glucose result entered from the meter is 48 mg/dL, and the scheduled rapid-acting insulin dose still appears due. Which action is best?

- A.** Administer the insulin because the electronic MAR has not generated a hard stop
- B.** Document that the insulin was omitted and take no further action because the glucose is low
- C.** Pause administration, assess the client, and follow hypoglycemia and medication-hold procedures before giving any rapid-acting insulin
- D.** Ask the client whether breakfast will be eaten soon and give the insulin if the tray is on the unit

Correct Answer: C

The MAR supports workflow, but the nurse must integrate real-time assessment data. A glucose of 48 mg/dL indicates hypoglycemia and requires immediate clinical response; rapid-acting insulin should not be given until the situation is addressed according to policy and provider guidance. The trap is relying on the MAR display instead of current patient status.

2651. A client is admitted for pneumonia. During medication reconciliation, the nurse asks about prescription drugs, and the client says, "That's everything." Which follow-up question is best to help prevent an incomplete medication list?

- A.** Do you want your family to bring your medications from home?
- B.** Have you taken any vitamins, herbal products, eye drops, inhalers, creams, or over-the-counter medicines?
- C.** Which pharmacy do you usually use for antibiotics?
- D.** Have you ever missed doses because medications cost too much?

Correct Answer: B

Many medication lists are incomplete because clients do not think of nonprescription products or non-oral items as medications. Asking specifically about vitamins, herbals, eye drops, inhalers, creams, and OTC drugs improves reconciliation accuracy. Choice A may be useful later, but it does not identify omitted medications. Choice C is too narrow. Choice D addresses adherence, not completeness of the current medication list.

2652. The nurse is reviewing the MAR before a 1000 medication pass. One entry reads: "Morphine 2 mg IV q4h PRN severe pain." The client reports pain 2/10 and is sleeping between care. What is the best action?

- A.** Administer the morphine because PRN medications can be given at the nurse's discretion
- B.** Hold the morphine and document that it was not indicated at this time
- C.** Wake the client and ask whether morphine should be given now to prevent pain

D. Ask the provider to discontinue the morphine because the client is comfortable

Correct Answer: B

A PRN medication should be given only when the ordered indication is present. Severe pain is not supported by a pain rating of 2/10 with the client resting comfortably, so the nurse should not administer it and should document appropriately. Choice A ignores the indication. Choice C is unnecessary and could disturb the client without a clinical reason. Choice D is outside the immediate issue and not warranted from one comfortable assessment.

2653. At change of shift, the off-going nurse says a 0600 dose of levothyroxine was not given because the client was off the unit for testing. The oncoming nurse checks the MAR and sees the dose is still documented as due, with no note explaining the omission. What should the nurse do first?

- A.** Document the dose as administered so the MAR will be complete
- B.** Leave the MAR unchanged because the previous nurse is responsible for the entry
- C.** Clarify the missed dose and document the reason according to facility policy
- D.** Give a double dose at the next scheduled time

Correct Answer: C

A medication that was not given should be clarified and documented accurately, including the reason for omission, to maintain a safe medication record. Choice A creates a false record. Choice B leaves an unresolved discrepancy that can lead to repeat dosing or missed therapy. Choice D is unsafe because doses should not be doubled unless specifically prescribed.

2654. A client is being discharged after treatment for heart failure. During final medication reconciliation, the discharge list includes furosemide 40 mg daily, but the preadmission home list shows hydrochlorothiazide 25 mg daily with no instruction to continue or stop it. What is the nurse's best action?

- A.** Tell the client to take both diuretics until the follow-up visit

- B.** Remove hydrochlorothiazide from the list without notifying anyone
- C.** Contact the prescribing provider to clarify whether hydrochlorothiazide should be continued
- D.** Advise the client to decide based on swelling at home

Correct Answer: C

A discrepancy between the home list and discharge list requires clarification before the client leaves. The nurse should contact the provider so the final list clearly states whether hydrochlorothiazide is to be continued or stopped. Choice A risks unintended duplicate therapy. Choice B changes the regimen without authorization. Choice D shifts a prescribing decision to the client.

2655. The nurse scans a client's wristband and medication at the bedside. The electronic MAR displays a hard alert: "This medication is documented as already given at 0800." It is now 0830, and the client states, "I only took the pills in the blue cup, not this one." What is the best next action?

- A.** Administer the medication because the client may not remember
- B.** Bypass the alert and correct the MAR later if needed
- C.** Stop and investigate the discrepancy before giving the medication
- D.** Delete the 0800 documentation and then give the medication

Correct Answer: C

An alert that a medication has already been given raises the risk of duplicate dosing. The nurse should pause and verify what actually occurred before administering anything. Choice A relies on an uncertain memory. Choice B ignores a significant safety warning. Choice D alters documentation before facts are confirmed and could create an inaccurate record.

2656. A client arrives from surgery with new postoperative orders. On the MAR, acetaminophen 650 mg PO every 6 hours PRN mild pain is active. A newly entered provider order reads, "Acetaminophen 1,000 mg PO every 6 hours scheduled." What is the nurse's best action before administering acetaminophen?

- A.** Give both because one is scheduled and one is PRN
- B.** Clarify the overlapping acetaminophen orders before administration
- C.** Give only the PRN dose because it was listed first on the MAR
- D.** Hold all pain medication until the next shift reviews the orders

Correct Answer: B

Two active acetaminophen orders with the same interval but different dosing create a risk for excess total daily dose. The nurse should clarify which order should remain active before administration. Choice A could overdose the client. Choice C uses order sequence instead of clinical verification. Choice D delays care unnecessarily when the issue can be addressed now.

2657. During admission medication reconciliation, a client says, "I use insulin from a pen before meals, but I just turn the dial until it looks right." Which additional information is most important for the nurse to obtain to reconcile this medication safely?

- A.** The exact insulin type and the usual number of units taken
- B.** Whether the client prefers the abdomen or thigh for injections
- C.** How long the client has had diabetes
- D.** Whether the client stores the pen in a kitchen drawer

Correct Answer: A

For medication reconciliation, the most important missing details are the specific insulin product and usual dose. Different insulins have different onset and timing, and a vague description is unsafe. Choice B relates to administration teaching. Choice C is background information. Choice D may matter for education, but it does not identify the actual medication regimen.

2658. The nurse is reviewing a paper MAR in a long-term care setting. A scheduled 2100 anticonvulsant dose has the nurse initials circled, which the facility key identifies as "held." No reason is written in the notes section. Why is this documentation a safety concern?

- A.** It may be difficult to bill the medication correctly
- B.** It does not show whether the nurse notified the pharmacy
- C.** It fails to communicate why the medication was withheld and can affect follow-up decisions
- D.** It automatically means the medication error rate will increase

Correct Answer: C

When a medication is held, documentation should include the reason so the next nurse and provider can interpret the omission and decide on follow-up care. A circled initial alone is incomplete. Choice A is not the main patient-safety issue. Choice B may or may not be relevant depending on the reason. Choice D is too absolute; the concern is lack of clear clinical communication.

2659. A client is transferred from the intensive care unit to a medical unit. The transfer medication profile lists pantoprazole 40 mg IV daily. The receiving unit's MAR also shows pantoprazole 40 mg PO daily scheduled to start this evening. The client can swallow tablets. What should the nurse do?

- A.** Administer both forms because one replaces the other gradually
- B.** Hold the evening oral dose and continue IV therapy indefinitely
- C.** Clarify which pantoprazole order should remain active after transfer

D. Discontinue the IV order independently because oral medication is safer

Correct Answer: C

The client appears to have duplicate therapy in two routes for the same medication. The nurse should clarify which order is intended after transfer. Choice A risks duplicate dosing. Choice B assumes the IV route should continue without confirmation. Choice D exceeds nursing authority because discontinuation requires an appropriate order or protocol.

2660. The nurse is preparing morning medications and notes that the MAR lists potassium chloride 20 mEq PO daily. The latest laboratory report shows a serum potassium of 5.6 mEq/L from 30 minutes ago, and no note addresses the result. What is the best action?

- A.** Administer the medication because it is a routine home drug
- B.** Hold the potassium and notify the appropriate licensed provider for clarification
- C.** Give half the dose and recheck the lab later
- D.** Ask the client whether potassium is usually taken at home

Correct Answer: B

A high potassium level is a critical cue that makes a scheduled potassium supplement potentially unsafe. The nurse should hold the medication and seek clarification before giving it. Choice A ignores current assessment data. Choice C changes the order without authorization. Choice D does not resolve the immediate safety issue.

2661. During admission reconciliation, the nurse compares the client's medication bottles with the history entered in the electronic record. One bottle label reads "prednisone 10 mg, take 4 tablets daily for 3 days, then 3 tablets daily for 3 days..." but the history simply says "prednisone daily." What is the most accurate next step?

- A.** Enter prednisone as 10 mg daily because that is the tablet strength
- B.** Record prednisone as a PRN medication because the dose changes
- C.** Clarify and document the current point in the taper schedule

D. Omit prednisone from the list until the client finishes the bottle

Correct Answer: C

A taper is time-sensitive, so the current dose depends on where the client is in the schedule. Medication reconciliation must reflect the active dose accurately. Choice A confuses tablet strength with total daily dose. Choice B misclassifies the medication. Choice D could lead to abrupt interruption of therapy.

2662. A nurse documents on the MAR that a PRN antiemetic was given at 1400. At 1500, the client still reports nausea, and the incoming nurse asks whether the medication helped. Which additional documentation is most appropriate?

- A.** The color of the medication package
- B.** A follow-up assessment of the client's response after administration
- C.** The prescriber's office phone number
- D.** The nurse's opinion about whether the client tolerates nausea well

Correct Answer: B

Safe PRN documentation includes not only what was given and when, but also the client's response on follow-up assessment. That information guides ongoing treatment decisions. Choice A is irrelevant. Choice C is not part of MAR effectiveness documentation. Choice D is subjective and unhelpful.

2663. A client with limited English proficiency is admitted with several home medications. The family member at the bedside offers to translate the medication names and instructions. Which approach best supports accurate medication reconciliation?

- A.** Use the family member as the sole interpreter because the client knows them best
- B.** Ask yes-or-no questions only, since medication names are hard to interpret
- C.** Use qualified interpretation resources and verify medication names, doses, and schedules carefully
- D.** Skip reconciliation until the provider rounds and can ask the family directly

Correct Answer: C

Medication reconciliation requires precise communication. Using qualified interpretation resources helps reduce errors with names, dosing, and timing. Family input may still be helpful, but relying on family alone can increase the risk of miscommunication. Choice B oversimplifies a complex task. Choice D delays a safety process that should begin promptly.

2664. A provider enters a verbal order during an urgent situation: "Resume home meds." When the situation stabilizes, the nurse begins MAR review for order processing. What is the best nursing action?

- A.** Activate every medication the client reports taking at home
- B.** Wait to see what pharmacy enters and then administer those medications
- C.** Obtain clarification and specific medication orders based on completed reconciliation
- D.** Restart only medications that were last filled within 30 days

Correct Answer: C

A nonspecific instruction such as "resume home meds" is not enough for safe administration. The nurse needs a reconciled, specific medication list with clear orders for what to continue, hold, or change. Choice A could restart unsafe or outdated medications. Choice B shifts responsibility without resolving ambiguity. Choice D uses a refill date, which does not determine clinical appropriateness.

2665. While reviewing the MAR, the nurse notices that yesterday's 1800 dose of an antibiotic is missing any signature or omission code. The next dose is due now, and the antibiotic must be given on schedule. Which action is best?

- A.** Assume the 1800 dose was given because the client is improving
- B.** Skip the current dose to avoid possible duplication
- C.** Investigate the undocumented prior dose while preparing to give the currently due dose if appropriate
- D.** Sign for the missed 1800 dose and then give the current one

Correct Answer: C

An undocumented prior dose requires investigation, but the nurse should also address the currently due dose based on verified information and the medication schedule. Choice A is unsafe because clinical improvement does not prove administration. Choice B may create an unnecessary treatment delay. Choice D is false documentation and unsafe.

2666. A nurse is completing admission medication reconciliation for a client with diabetes. The client says, "I take a blood pressure pill, a sugar pill, aspirin, and something for heartburn, but I don't remember all the names." Which action is best to improve the accuracy of the medication list?

- A.** Document the medications by category and verify the details at discharge
- B.** Ask the client to describe the color and shape of each pill, then enter the most likely medications
- C.** Obtain the medication containers or contact the client's pharmacy to verify the exact drugs and doses
- D.** Record only the medications the client can name with certainty and leave the others off the list

Correct Answer: C

The safest action is to verify the exact medication names, doses, and schedules using a reliable source such as the medication bottles or the pharmacy. Medication reconciliation is meant to create the most accurate current list possible. Guessing from pill appearance or entering incomplete information increases the risk of omission, duplication, or wrong-drug errors.

2667. The nurse reviews the electronic MAR before giving a scheduled 0900 antihypertensive. The medication is listed as due, but the blood pressure documented at 0845 is 88/54 mm Hg. The order does not include hold parameters. What should the nurse do first?

- A.** Administer the medication because it is scheduled on the MAR

- B.** Hold the medication, assess the client, and notify the provider about the low blood pressure before giving it

- C.** Ask the assistive personnel to recheck the blood pressure after the medication is given

- D.** Document the dose as refused so it will not appear overdue on the MAR

Correct Answer: B

The MAR shows what is ordered and due, but the nurse must also review current assessment data before administration. A blood pressure of 88/54 mm Hg may make an antihypertensive unsafe to give. The nurse should assess the client, withhold the medication for safety, and contact the provider for further direction. The trap is assuming a scheduled MAR entry automatically means the medication should be administered.

2668. A client is admitted from home with a handwritten list of medications. On the list are warfarin 5 mg nightly, a "vitamin for bones," and an herbal tea used "for relaxation." Which additional question is most important for safe medication reconciliation?

- A.** Who usually reminds you to take your medicines each day?
- B.** Do you prefer taking tablets in the morning or at night?
- C.** What over-the-counter products, vitamins, or herbal supplements do you use regularly?
- D.** Have you ever missed a dose because the pills were difficult to swallow?

Correct Answer: C

Medication reconciliation should include prescription drugs, over-the-counter medications, vitamins, and herbal supplements. This is especially important for a client taking warfarin because supplements and herbal products may increase bleeding risk or otherwise interact with therapy. The other questions may matter later, but they do not address the most immediate reconciliation gap.

2669. The nurse is preparing to give a 1400 dose of an antibiotic. On the MAR, the 0800 dose has been documented as administered, but the client says, "I was in radiology all morning and didn't get any medicine until lunch." The IV tubing label also shows the antibiotic bag was hung at 1205. What is the nurse's best action?

- A.** Give the 1400 dose now because the MAR shows the 0800 dose was completed
- B.** Skip the 1400 dose and wait until the next scheduled time
- C.** Clarify the discrepancy before administering by reviewing the record and contacting the nurse who documented the earlier dose if needed
- D.** Change the 0800 MAR entry to refused because the client states it was not given

Correct Answer: C

There is conflicting information: the MAR says the dose was given at 0800, the client reports otherwise, and the tubing label suggests administration at 1205. The nurse must resolve the discrepancy before giving another dose to avoid a possible duplicate or mistimed administration. Simply trusting one source, skipping a needed dose without clarification, or changing documentation without confirmation would be unsafe.

2670. A nurse is reconciling medications for a client transferred from a rehabilitation facility. The transfer record lists digoxin 0.125 mg PO daily. The new admission orders also include digoxin 0.25 mg PO daily, and both entries appear active on the MAR. The client's apical pulse is 58/min, and the latest digoxin level is not yet available. What is the nurse's best action?

- A.** Administer the lower dose only, since it is less likely to cause harm
- B.** Administer both doses because both are active on the MAR and clarify later
- C.** Hold digoxin and urgently clarify which order is correct before administration
- D.** Delete the transfer entry from the MAR because new admission orders always replace prior medications

Correct Answer: C

This situation involves a possible duplicate and conflicting digoxin order, which is high risk because digoxin has a narrow therapeutic range and the client already has an apical pulse of 58/min. The nurse should not guess which dose is correct or assume one order automatically replaces another without clarification. The safest action is to withhold the medication and clarify the active order before administration.

Routes of administration

2671. A nurse is preparing to give a medication by the sublingual route. Which instruction should the nurse give the patient?

- A.** Swallow the tablet quickly with a full glass of water
- B.** Place the tablet under your tongue and let it dissolve
- C.** Chew the tablet well before swallowing
- D.** Place the tablet between your gum and cheek and leave it there

Correct Answer: B

Sublingual medications are placed under the tongue to dissolve and be absorbed through the oral mucosa. Swallowing or chewing the tablet reduces the intended absorption pattern. Choice D describes the buccal route, which is a common trap because both routes use mucosal absorption in the mouth.

2672. The nurse needs to administer a medication that is formulated for intradermal injection. Which site is most appropriate?

- A.** Outer aspect of the upper arm
- B.** Abdomen at least 2 inches from the umbilicus
- C.** Ventrogluteal muscle
- D.** Deltoid muscle

Correct Answer: A

Intradermal injections are placed just below the epidermis, commonly on the inner or outer aspect of the forearm or upper arm, depending on agency practice and purpose. The abdomen is a common subcutaneous site, and the ventrogluteal and deltoid areas are used for intramuscular injections.

2673. A patient with persistent vomiting has an order for an antiemetic. Which route would best bypass the gastrointestinal tract if the patient cannot keep oral medication down?

- A. Oral
- B. Rectal
- C. Enteral tube
- D. Sublingual capsule to swallow after dissolving

Correct Answer: B

The rectal route can be useful when a patient is vomiting and cannot tolerate oral medications. Oral and enteral routes still rely on the gastrointestinal tract. Choice D is incorrect because sublingual medications are not swallowed after dissolving; the medication is intended to absorb through oral mucosa.

2674. A nurse is preparing to administer an ophthalmic medication. Where should the nurse place the drops?

- A. Into the outer ear canal
- B. Directly onto the center of the cornea
- C. Into the conjunctival sac
- D. Onto the eyelid skin

Correct Answer: C

Ophthalmic drops are placed into the conjunctival sac. Placing drops directly onto the cornea can cause discomfort and is not the preferred technique. The outer ear canal is for otic medications, and eyelid skin application does not deliver the medication properly to the eye.

2675. A nurse is caring for a patient who reports dry mouth and has difficulty swallowing tablets. The provider prescribes the same medication in several forms. Which route is likely to be most appropriate if the patient can follow directions and the medication is available in that form?

- A. Buccal
- B. Oral tablet to swallow whole without water
- C. Rectal suppository
- D. Intradermal injection

Correct Answer: A

A buccal medication is placed between the cheek and gum for mucosal absorption and may be appropriate for a patient who has trouble swallowing tablets but can follow instructions. An oral tablet swallowed without water is not appropriate for a patient with swallowing difficulty and dry mouth. Rectal and intradermal routes are more invasive or less practical when a suitable mucosal route is available.

2676. A medication label reads, "For otic use only." The nurse understands this medication should be administered by which route?

- A. Eye
- B. Ear
- C. Nose
- D. Skin patch

Correct Answer: B

Otic refers to the ear. This contrasts with ophthalmic for the eye and intranasal for the nose. Confusing otic and ophthalmic is a classic medication-safety trap because the words look similar.

2677. The nurse is preparing to give insulin by the subcutaneous route. Which action is most consistent with this route of administration?

- A. Injecting the medication into fatty tissue
- B. Injecting the medication into a large muscle
- C. Placing the medication beneath the tongue
- D. Instilling the medication into the rectum

Correct Answer: A

Subcutaneous medications are injected into the fatty tissue beneath the skin. Intramuscular injections go into muscle, sublingual medications go under the tongue, and rectal medications are inserted into the rectum. The question tests route characteristics rather than a specific drug fact.

2678. A patient has a prescription for a transdermal medication. Which statement by the patient shows correct understanding?

- A. I will place the patch on my skin so the medicine absorbs over time
- B. I will dissolve the patch in water before drinking it
- C. I will place the patch under my tongue for faster absorption

D. I will insert the patch into my cheek pouch after meals

Correct Answer: A

Transdermal medications are applied to the skin for absorption over time. Choices C and D describe sublingual and buccal routes, and choice B is not an appropriate use of a transdermal patch.

2679. A nurse must administer a drug by the parenteral route. Which medication order meets that description?

- A.** Acetaminophen 650 mg by mouth
- B.** Ondansetron 4 mg IV
- C.** Nystatin suspension swish and swallow
- D.** Bisacodyl suppository per rectum

Correct Answer: B

Parenteral administration refers to medication given by injection or infusion, such as IV therapy. Oral and rectal medications are not parenteral. Choice C may tempt a test-taker because it involves the mouth, but it is still not a parenteral route.

2680. A patient asks why a prescribed nitroglycerin tablet is taken sublingually instead of swallowed. Which response by the nurse is best?

- A.** It prevents any side effects from occurring
- B.** It allows absorption through tissues under the tongue for a rapid effect
- C.** It ensures the medication will last longer than an oral tablet
- D.** It makes the medication safer for patients with chest pain because it cannot interact with other drugs

Correct Answer: B

The sublingual route allows rapid absorption through the oral mucosa, which is why it may be used when a quick effect is desired. It does not eliminate side effects or drug interactions, and the reason is not that it automatically lasts longer. The trap is overgeneralizing route choice as a guarantee of safety rather than understanding its absorption advantage.

2681. A nurse is preparing to administer medication through a gastrostomy tube. Before giving the medication, what is the most important route-related consideration?

- A.** Assume any oral form can be crushed for tube administration
- B.** Verify that the medication formulation is appropriate for enteral-tube administration
- C.** Mix all medications together to reduce administration time
- D.** Place the medication under the patient's tongue before flushing the tube

Correct Answer: B

Not all oral formulations are appropriate for administration through an enteral tube, so the nurse should verify that the formulation is suitable. Assuming all oral medications can be crushed is unsafe. Mixing medications together can create compatibility and dosing problems. Choice D describes a completely different route.

2682. A patient has an order for an inhaled bronchodilator and asks why it is not being given as a tablet. Which explanation is best?

- A.** The inhaled route delivers medication directly to the respiratory tract
- B.** The inhaled route is always easier for every patient to use than tablets
- C.** The inhaled route prevents the need to monitor the patient's response
- D.** The inhaled route means the medication will not enter the bloodstream at all

Correct Answer: A

Inhaled medications are commonly used to deliver a drug directly to the respiratory tract. That route can target the intended area effectively, but it is not automatically easier for every patient, does not remove the need for monitoring, and does not mean there is zero systemic absorption.

2683. The nurse is reviewing several medication orders. Which order should the nurse question based on route designation alone?

- A.** Timolol ophthalmic drops, 1 drop in each eye
- B.** Carbamide peroxide otic drops, 5 drops in the left ear
- C.** Artificial tears ophthalmic solution, 2 drops in the right ear

D. Saline nasal spray, 2 sprays in each nostril

Correct Answer: C

Ophthalmic solution is intended for the eye, not the ear, so an order to place ophthalmic drops in the ear should be questioned based on route designation. The other options correctly match the route terminology to the body site. This item tests careful route-label interpretation.

2684. A nurse is deciding whether a prescribed medication should be given by the intramuscular or subcutaneous route. Which factor is most important to verify first?

- A. The nurse's personal preference for injection technique
- B. Whether the patient ate within the last hour
- C. The medication's prescribed route and formulation
- D. Whether the patient prefers the left side of the body

Correct Answer: C

The prescribed route and formulation must be verified first because medications are designed and ordered for specific routes. Personal preference, recent food intake, and body-side preference do not determine whether a medication should be given IM or subcutaneously. A key trap is focusing on convenience before confirming safe route appropriateness.

2685. A patient is receiving a medication by the buccal route. During rounds, the nurse finds the patient chewing the tablet. What is the nurse's best action?

- A. Tell the patient to swallow the remaining pieces with water
- B. Instruct the patient to keep the medication between the cheek and gum to dissolve
- C. Move the medication under the tongue and document it as sublingual
- D. Crush the remaining tablet and mix it with applesauce

Correct Answer: B

Buccal medications should be kept between the cheek and gum so they can dissolve and absorb through the mucosa. Swallowing, crushing, or changing the route on the spot alters intended administration. Choice C is a trap because buccal and sublingual are both oral mucosal routes, but they are not interchangeable without an order and appropriate formulation.

2686. A nurse is reinforcing teaching for a patient who will use a medicated skin patch at home. Which patient statement indicates a need for further teaching about the route of administration?

- A. I should apply the patch to clean, dry skin.
- B. I will remove the old patch before putting on a new one.
- C. I can cut the patch in half if I think the dose feels too strong.
- D. I should avoid placing the patch over irritated skin.

Correct Answer: C

Transdermal patches should not be cut unless the product specifically allows it, because cutting can alter how the medication is delivered. Clean, dry, intact skin is appropriate, and the old patch should be removed before a new one is applied. A common trap is assuming a patch works like a tablet that can be split, but route-specific dosage forms often cannot be altered safely.

2687. A patient has an order for a medication that must produce a very rapid effect during an acute episode of chest tightness. The medication is available as an oral tablet, an extended-release capsule, a topical cream, and a sublingual tablet. Which route should the nurse expect to use?

- A. Oral tablet
- B. Extended-release capsule
- C. Topical cream
- D. Sublingual tablet

Correct Answer: D

The sublingual route is used when a rapid effect is needed because the medication dissolves under the tongue and is absorbed quickly. An oral tablet and an extended-release capsule are slower, and a topical cream is not appropriate for rapid systemic relief in this situation. The trap is focusing on forms that seem convenient rather than the route that matches the urgency.

2688. A nurse is preparing to instill a liquid medication into a patient's nose. This medication is administered by which route?

- A. Nasal
- B. Ophthalmic
- C. Otic
- D. Buccal

Correct Answer: A

Medication placed into the nose is administered by the nasal route. Ophthalmic refers to the eye, otic refers to the ear, and buccal refers to placement against the inner cheek. This item tests route recognition rather than a procedure step.

2689. A patient has an order for a medication to be given rectally. Which form should the nurse prepare to administer if the medication is supplied in multiple forms?

- A. Lozenge
- B. Suppository
- C. Patch
- D. Eye ointment

Correct Answer: B

A suppository is a common dosage form used for the rectal route. A lozenge is used in the mouth, a patch is applied to the skin, and eye ointment is for ophthalmic use. The key is matching the route ordered with the dosage form intended for that route.

2690. The nurse receives these prescriptions for the same patient. Which route should the nurse clarify with the provider before administration because it is unsafe based on route designation alone?

- 1. Carbamide peroxide drops: 5 drops in the left ear
- 2. Artificial tears: 2 drops in each eye
- 3. Enteric-coated aspirin: crush and give through the mouth
- 4. Albuterol: 2 puffs by inhalation

- A. 1
- B. 2
- C. 3
- D. 4

Correct Answer: C

An enteric-coated tablet should not be crushed for oral administration because the dosage form is designed to pass through the stomach intact before dissolving. The other route designations are appropriate as written: ear drops are otic, artificial tears are ophthalmic, and albuterol is commonly inhaled. The trap is overlooking that route safety includes the form in which the medication is given, not just the body site.

2691. A nurse is teaching a patient about taking a buccal medication at home. Which instruction is most appropriate?

- A. Place the medication between the cheek and gum until it dissolves.
- B. Swallow the medication immediately with a full glass of water.
- C. Place the medication under the tongue and do not move it.
- D. Chew the medication thoroughly before swallowing.

Correct Answer: A

Buccal medication is placed between the cheek and gum and allowed to dissolve. Placing it under the tongue describes the sublingual route, and swallowing or chewing defeats the intended route of absorption. The most likely trap is confusing buccal with sublingual because both use the oral mucosa.

2692. A patient has severe oral mucositis and says swallowing is very painful. The provider prescribes a pain medication for local relief in the mouth. Which route would best match this goal?

- A. Topical oral mucosal application
- B. Intramuscular injection
- C. Rectal administration
- D. Transdermal application

Correct Answer: A

For local relief inside the mouth, a medication applied directly to the oral mucosa is most appropriate because it targets the affected area. Intramuscular, rectal, and transdermal routes are not the best match for immediate local treatment of painful oral tissue. The key is matching route to treatment goal: local effect rather than systemic delivery.

2693. A nurse is preparing to administer medication to a patient who is NPO in anticipation of surgery. Which prescription should the nurse question based on route status?

- A. Acetaminophen 650 mg suppository per rectum
- B. Ondansetron 4 mg IV
- C. Metoprolol 25 mg by mouth
- D. Timolol 1 drop in each eye

Correct Answer: C

A medication ordered by mouth should be questioned for a patient who is NPO unless there is a specific clarification or exception. Rectal, IV, and ophthalmic routes are not oral routes. The likely trap is thinking NPO blocks every medication, when the route is what matters here.

2694. A nurse is comparing medication routes for a patient who has intermittent nausea, poor appetite, and an intact skin surface. Which route would provide continuous systemic delivery over many hours without requiring swallowing?

- A. Transdermal
- B. Sublingual
- C. Inhalation
- D. Ophthalmic

Correct Answer: A

The transdermal route is designed to deliver medication through the skin over an extended period, making it useful when swallowing is undesirable and continuous systemic delivery is needed. Sublingual often provides more rapid, shorter-term delivery, inhalation primarily targets the respiratory tract, and ophthalmic medications are for the eyes. The trap is choosing sublingual because it avoids swallowing, but it does not best fit the need for prolonged continuous delivery.

2695. A patient is prescribed a vaginal antifungal medication. Which patient statement shows correct understanding of the route?

- A. I should place the medication in my rectum before bedtime.
- B. I should insert the medication into my vagina as directed.
- C. I should rub the medication over my abdomen.
- D. I should swallow the medication after meals.

Correct Answer: B

A vaginal medication is inserted into the vagina as directed. Rectal, topical abdominal, and oral use are different routes. This question tests route understanding and recognition of the intended body site.

2696. The nurse is reviewing discharge teaching for a patient prescribed both an oral tablet and a sublingual tablet. Which statement by the patient shows the nurse should reinforce teaching?

- A. I will swallow the oral tablet with water.
- B. I will let the sublingual tablet dissolve under my tongue.
- C. I will switch the two tablets if one route feels easier.
- D. I need to follow the directions for each medication form separately.

Correct Answer: C

Different dosage forms and routes are not interchangeable unless specifically prescribed that way. Swallowing the oral tablet and letting the sublingual tablet dissolve under the tongue are correct. The trap is assuming the same drug can be used by whatever route is more convenient, but route changes can alter onset, absorption, and safety.

2697. A patient receiving oxygen by nasal cannula also has an order for medication by inhalation. Which action best reflects correct route administration?

- A. Apply the medication as a patch behind the ear.
- B. Instill the medication into the nostrils.
- C. Deliver the medication into the respiratory tract using the prescribed inhalation device.
- D. Give the medication by mouth with sips of water.

Correct Answer: C

Inhalation medications are delivered into the respiratory tract using a device such as an inhaler or nebulizer, depending on the prescription. Nasal administration goes into the nose and is a different route. Oral and transdermal administration do not match the ordered route. The common trap is confusing nasal with inhalation because both involve the head and airways.

2698. A nurse is asked to give a medication labeled 'for topical use only.' Which action requires immediate correction?

- A. Applying the medication to intact skin
- B. Checking the skin for irritation before use
- C. Placing the medication into the patient's eye
- D. Using gloves if indicated by facility policy

Correct Answer: C

A medication labeled for topical use only should not be placed in the eye. Ophthalmic medications are specially formulated for ocular administration. Applying to skin, assessing the skin, and using gloves are consistent with topical administration. The key trap is assuming any external-use medication can be used on other body surfaces, which is unsafe.

2699. A patient has a long-acting analgesic patch and now reports new acute postoperative pain. Which route is most appropriate for a separate medication intended to relieve the pain quickly?

- A. Another transdermal patch
- B. Extended-release oral capsule
- C. Immediate IV administration
- D. Topical skin ointment

Correct Answer: C

For acute postoperative pain requiring rapid relief, the IV route is the best choice because it provides immediate systemic delivery. Another patch or an extended-release capsule would not be the best route for quick control, and a topical ointment would not provide reliable rapid systemic analgesia. The trap is choosing another long-acting route when the patient's need is immediate rescue treatment.

2700. A nurse is caring for a patient who has dysphagia after a stroke and follows commands inconsistently. The provider wants a medication that acts systemically and is available by several routes. Which route would the nurse be most cautious about selecting because successful administration depends heavily on the patient's cooperation and proper placement?

- A. Rectal
- B. Transdermal
- C. Sublingual
- D. Intramuscular

Correct Answer: C

The sublingual route depends on the patient keeping the medication under the tongue until it dissolves, which requires understanding and cooperation. A patient who follows commands inconsistently may swallow, spit out, or misplace the medication. Rectal, transdermal, and intramuscular routes rely less on active patient participation during administration. The trap is focusing only on dysphagia and forgetting the importance of route-specific cooperation.

2701. A nurse is preparing to administer four medications. Which route would bypass the gastrointestinal tract and first-pass liver metabolism while still allowing the medication to be absorbed through the oral mucosa?

- A. Sublingual
- B. Oral
- C. Rectal
- D. Nasogastric

Correct Answer: A

Sublingual medications are placed under the tongue and absorbed through the oral mucosa, so they bypass the gastrointestinal tract and avoid first-pass metabolism in the liver. Oral and nasogastric routes enter the GI tract. Rectal administration may partially avoid first-pass metabolism, but the route that clearly matches absorption through the oral mucosa is sublingual.

2702. A nurse is reinforcing teaching for a patient who has a metered-dose inhaler prescription. Which patient action shows correct understanding of this route of administration?

- A. Swallow the medication immediately after it is sprayed into the mouth
- B. Inhale the medication deeply as it is released
- C. Place the medication under the tongue until it dissolves
- D. Rub the medication onto the chest wall after each dose

Correct Answer: B

Inhaled medications are intended to reach the respiratory tract, so the patient should inhale deeply as the dose is released. Swallowing the drug would not deliver it by the intended route. Placing it under the tongue describes the sublingual route, and rubbing medication on the chest wall describes a topical route.

2703. A patient is prescribed four forms of the same antiemetic. The nurse needs the route most likely to provide systemic effect when the patient is actively vomiting and cannot keep medications down. Which route is the best choice?

- A. Oral tablet
- B. Topical ointment

- C. Rectal suppository
- D. Mouthwash rinse

Correct Answer: C

A rectal suppository is the best option when the patient is vomiting and cannot reliably retain oral medication, yet still needs systemic drug absorption. An oral tablet is likely to be vomited. A topical ointment is generally intended for local skin effect unless specifically designed for transdermal systemic delivery, which is not stated here. A mouthwash rinse mainly provides local oral effect, not systemic antiemetic action.

2704. A nurse is caring for a patient with dry, fragile skin, severe diarrhea, and a new prescription for a medication intended for slow, continuous systemic absorption over 72 hours. The medication is available by several routes. Which route would be least appropriate based on the route's usual requirements and the patient's condition?

- A. Transdermal patch
- B. Intravenous infusion
- C. Subcutaneous injection
- D. Sublingual tablet

Correct Answer: A

A transdermal patch depends on intact skin for reliable absorption, so dry, fragile skin makes this route less appropriate. The stem also asks for slow, continuous systemic absorption, which is a typical purpose of transdermal delivery, making skin condition the key issue. Intravenous and subcutaneous routes do not rely on skin surface integrity in the same way, and a sublingual tablet uses oral mucosa rather than skin. A common trap is choosing sublingual because it is not continuous over 72 hours; however, the question asks which route is least appropriate based on the patient's condition and the route's requirements, making transdermal the best answer.

Blood products

2705. A nurse is preparing to administer 1 unit of packed red blood cells (PRBCs). Which IV solution should the nurse use to prime the tubing and run with the transfusion?

- A. 0.9% sodium chloride

- B. 5% dextrose in water
- C. Lactated Ringer's solution
- D. 0.45% sodium chloride

Correct Answer: A

0.9% sodium chloride is the compatible solution routinely used with blood products. Dextrose-containing solutions can cause red blood cells to clump or hemolyze, and solutions containing calcium, such as lactated Ringer's, may promote clotting. Hypotonic saline is also not appropriate for blood administration.

2706. Fifteen minutes after a blood transfusion is started, a patient reports chills and low back pain. What is the nurse's priority action?

- A. Slow the transfusion rate and reassess in 15 minutes
- B. Stop the transfusion and keep the IV line open with normal saline
- C. Administer acetaminophen and continue the transfusion
- D. Document the findings after the unit is complete

Correct Answer: B

Chills and low back pain can indicate an acute transfusion reaction. The priority is to stop the transfusion immediately and maintain IV access with normal saline using new tubing per facility protocol. Slowing the rate or treating symptoms without stopping the transfusion delays response to a potentially life-threatening complication.

2707. Which finding in a patient receiving a blood transfusion requires the most immediate follow-up by the nurse?

- A. Temperature increase from 98.4°F to 98.8°F
- B. Reports of mild hunger during the transfusion
- C. New onset of dyspnea and wheezing
- D. Blood pressure decrease from 138/80 to 134/78 mm Hg

Correct Answer: C

New dyspnea and wheezing suggest a serious transfusion complication such as an allergic reaction, circulatory overload, or acute hemolytic response and require immediate action. The other findings are not as urgent and may be clinically insignificant in this context.

2708. Before starting a blood transfusion, which action is most important for the nurse to complete with another licensed nurse?

- A. Compare the blood product label with the patient's identification information
- B. Obtain a urine specimen for specific gravity
- C. Place the patient in high-Fowler position
- D. Warm the blood unit in tap water

Correct Answer: A

Correct patient identification and verification of the blood product by two qualified staff members is a critical safety step to prevent a potentially fatal transfusion error. The other options are unnecessary or unsafe; blood should not be warmed in tap water.

2709. A patient with symptomatic anemia is prescribed a transfusion. Which blood product is most appropriate if the goal is to increase oxygen-carrying capacity without giving unnecessary volume components?

- A. Packed red blood cells
- B. Platelets
- C. Fresh frozen plasma
- D. Cryoprecipitate

Correct Answer: A

Packed red blood cells are used to increase oxygen-carrying capacity in anemia. Platelets are used for thrombocytopenia or platelet dysfunction, fresh frozen plasma replaces clotting factors, and cryoprecipitate provides concentrated factors such as fibrinogen.

2710. The nurse is preparing to begin a transfusion of packed red blood cells. Why should the nurse remain with the patient during the first 15 minutes after initiation?

- A. The patient must be taught to regulate the infusion independently
- B. The blood reaches full room temperature during that time
- C. Serious transfusion reactions are most likely to appear early in the infusion
- D. The IV catheter is most likely to infiltrate only at the start of infusion

Correct Answer: C

The nurse stays with the patient initially because acute transfusion reactions often occur early, especially within the first 15 minutes. This allows prompt recognition and intervention. The other options do not explain the key safety reason for close observation.

2711. A patient receiving platelets develops hives and itching but has no dyspnea, hypotension, or fever. After stopping the transfusion and notifying the provider, which medication should the nurse anticipate may be prescribed for this type of reaction?

- A. Diphenhydramine
- B. Furosemide
- C. Protamine sulfate
- D. Naloxone

Correct Answer: A

Hives and itching suggest a mild allergic transfusion reaction, for which an antihistamine such as diphenhydramine may be prescribed after the transfusion is stopped and the patient is assessed. Furosemide is more associated with fluid overload, protamine reverses heparin, and naloxone reverses opioids.

2712. A provider prescribes fresh frozen plasma (FFP) for a patient. Which laboratory problem best supports this prescription?

- A. Hemoglobin of 7.2 g/dL
- B. Platelet count of 18,000/mm³
- C. Elevated PT/INR with active bleeding
- D. Low white blood cell count after chemotherapy

Correct Answer: C

Fresh frozen plasma is used to replace clotting factors, so elevated PT/INR with active bleeding is an appropriate indication. A low hemoglobin suggests a need for PRBCs, severe thrombocytopenia suggests platelets, and leukopenia is not treated with FFP.

2713. The nurse receives an order to transfuse cryoprecipitate. Which patient condition most clearly supports this order?

- A. Fibrinogen level is critically low
- B. The patient has chronic iron-deficiency anemia
- C. The patient has dehydration after vomiting

D. The patient has neutropenia related to infection

Correct Answer: A

Cryoprecipitate is rich in fibrinogen and certain clotting factors, so it is used when fibrinogen is low. It does not treat iron-deficiency anemia, dehydration, or neutropenia.

2714. A nurse is monitoring a patient during a blood transfusion. Which assessment finding is most consistent with circulatory overload rather than a hemolytic reaction?

- A. Crackles, jugular venous distention, and increasing blood pressure
- B. Low back pain, fever, and dark urine
- C. Flushing, urticaria, and itching
- D. Chills, hypotension, and a sense of impending doom

Correct Answer: A

Crackles, jugular venous distention, and rising blood pressure point to transfusion-associated circulatory overload from excess volume. Low back pain and dark urine fit hemolytic reaction, urticaria suggests allergic reaction, and chills with hypotension suggest a more severe acute reaction.

2715. A unit of packed red blood cells has arrived on the nursing unit for a stable patient. The nurse is called away to an emergency and cannot begin the transfusion right away. Which action is best?

- A. Hang the blood now and infuse it very slowly until the nurse returns
- B. Store the blood temporarily in the medication refrigerator on the unit
- C. Return the blood to the blood bank until it can be started appropriately
- D. Leave the blood at the bedside and begin the transfusion within 2 hours

Correct Answer: C

If the transfusion cannot be started promptly, the blood should be returned to the blood bank according to agency policy rather than left at room temperature or stored in an unauthorized unit refrigerator. Hanging blood without proper monitoring is unsafe.

2716. A patient with type O-negative blood needs an emergency transfusion before full crossmatching is complete. Which donor blood type is considered the safest choice for packed red blood cells in this situation?

- A. A-positive
- B. AB-negative
- C. O-negative
- D. B-positive

Correct Answer: C

O-negative packed red blood cells are considered the safest emergency choice because they lack A, B, and Rh antigens that could trigger a major incompatibility reaction. The other options contain antigens that could be incompatible with an O-negative recipient.

2717. The nurse begins a transfusion of packed red blood cells at 1000. At 1010, the patient reports flank pain, feels chilled, and has a temperature increase from 37.0°C to 38.1°C. After stopping the transfusion and maintaining IV access with normal saline, which action should the nurse take next?

- A. Restart the transfusion when the temperature returns to baseline
- B. Send the blood bag and tubing for evaluation per protocol
- C. Discard the blood product in the biohazard container
- D. Document the event and continue with the next prescribed medication

Correct Answer: B

These findings suggest an acute transfusion reaction. After stopping the transfusion and maintaining IV access, the blood bag and tubing should be sent for evaluation according to protocol, along with required patient specimens if ordered. Discarding the unit removes important evidence, and the transfusion should not be restarted.

2718. A provider orders 1 unit of packed red blood cells for a patient with heart failure and symptomatic anemia. Which nursing approach is most appropriate to reduce the risk of transfusion-associated circulatory overload?

- A. Infuse the blood as rapidly as possible to limit exposure time

- B. Use a smaller-gauge catheter to slow blood entry unpredictably

- C. Monitor closely for dyspnea and crackles, and anticipate a slower infusion rate

- D. Add dextrose solution to dilute the blood product before infusion

Correct Answer: C

A patient with heart failure is at higher risk for circulatory overload, so careful monitoring and a slower transfusion rate are appropriate. Rapid infusion increases risk, catheter size is not a reliable safety strategy by itself, and blood products should not be diluted with dextrose.

2719. A patient develops fever, hypotension, and red-tinged urine shortly after a transfusion begins. Which explanation best identifies the underlying problem the nurse should suspect?

- A. Volume expansion has triggered pulmonary edema
- B. Donor platelets are causing a temporary serotonin release
- C. An incompatibility reaction is causing destruction of transfused red blood cells
- D. Citrate in the blood product is producing severe hypoglycemia

Correct Answer: C

Fever, hypotension, and hemoglobinuria-like red-tinged urine are classic findings of an acute hemolytic transfusion reaction, usually caused by incompatibility and destruction of transfused red blood cells. Pulmonary edema would more likely cause crackles and dyspnea. The other options do not explain this presentation.

2720. A nurse is caring for a postoperative patient whose platelet count is 9,000/mm³ and who has oozing from the gums. Which blood product should the nurse expect to administer?

- A. Packed red blood cells
- B. Platelets
- C. Fresh frozen plasma
- D. Albumin

Correct Answer: B

Platelets are used to treat thrombocytopenia and active bleeding related to a very low platelet count. Packed red blood cells increase oxygen-carrying capacity, not platelet count. Fresh frozen plasma replaces clotting factors, and albumin expands volume but does not correct thrombocytopenia.

2721. A client with severe burns is receiving albumin. The nurse understands this product is primarily used to achieve which effect?

- A. Increase hemoglobin concentration
- B. Replace fibrinogen
- C. Expand intravascular volume
- D. Raise the platelet count

Correct Answer: C

Albumin is a colloid that pulls fluid into the intravascular space and helps expand circulating volume. It does not supply red blood cells, fibrinogen, or platelets. A common trap is choosing hemoglobin improvement, but albumin is not a red blood cell product.

2722. A nurse is preparing to administer packed red blood cells through existing IV access. Which setup is appropriate?

- A. Infuse the blood through tubing containing dextrose 5% in water
- B. Add prescribed medications directly to the blood bag before starting
- C. Use blood administration tubing with a filter and dedicated IV access
- D. Y-site the blood with lactated Ringer's solution to maintain flow

Correct Answer: C

Blood should be administered through appropriate blood tubing with a filter and dedicated IV access. Medications should not be added to blood products, and incompatible solutions such as dextrose-containing fluids or lactated Ringer's should not be infused with the blood. The safest setup is filtered blood tubing on its own line.

2723. A nurse is reviewing a transfusion order for a client who has a history of heart failure. Which prescription should the nurse question because it increases the risk for transfusion-associated circulatory overload?

- A. Transfuse 1 unit of packed red blood cells over 4 hours
- B. Administer furosemide between units as prescribed
- C. Use a blood warmer for rapid infusion in an unstable patient if indicated
- D. Infuse 2 units of packed red blood cells rapidly over 1 hour

Correct Answer: D

Rapid infusion of multiple units is a major concern in a client with heart failure because it increases the risk of circulatory overload. Slower infusion and use of diuretics are measures that may reduce risk when prescribed. The trap is assuming any blood transfusion is equally risky; rate and total volume matter.

2724. A patient receiving packed red blood cells becomes restless and reports shortness of breath. Assessment shows crackles, distended neck veins, and rising blood pressure. Which complication should the nurse suspect first?

- A. Transfusion-associated circulatory overload
- B. Acute hemolytic transfusion reaction
- C. Mild allergic transfusion reaction
- D. Septic transfusion reaction

Correct Answer: A

Crackles, dyspnea, jugular venous distention, and hypertension point to fluid overload from transfusion-associated circulatory overload. Acute hemolytic reactions more often involve fever, chills, flank pain, and hemoglobinuria. Mild allergic reactions typically cause hives and itching. Septic reactions usually present with high fever and signs of infection or shock.

2725. A nurse is caring for a client who refuses a prescribed blood transfusion for religious reasons. Which action is most appropriate?

- A. Ask the family to sign consent so the transfusion can proceed
- B. Administer the blood because the provider prescribed it

- C. Document the refusal and notify the provider
- D. Delay the transfusion and reattempt after sedation is given

Correct Answer: C

A competent client has the right to refuse treatment, including blood products. The nurse should document the refusal and notify the provider so the plan of care can be updated. Family members cannot override a competent client's decision, and sedation should never be used to obtain compliance.

2726. A client with liver disease has active bleeding and multiple prolonged coagulation studies. Which prescribed blood product is most appropriate to help replace several clotting factors quickly?

- A. Packed red blood cells
- B. Fresh frozen plasma
- C. Platelets
- D. Cryoprecipitate

Correct Answer: B

Fresh frozen plasma contains multiple clotting factors and is appropriate when the goal is broad factor replacement in a bleeding client with abnormal coagulation studies. Packed red blood cells treat anemia, platelets treat thrombocytopenia, and cryoprecipitate is more targeted to fibrinogen replacement.

2727. A nurse is teaching a newly licensed nurse about platelet transfusions. Which statement by the newly licensed nurse indicates correct understanding?

- A. Platelets are mainly given to raise hemoglobin in blood loss
- B. Platelets are used when thrombocytopenia creates a bleeding risk
- C. Platelets are the best product for replacing albumin losses
- D. Platelets are used to correct low fibrinogen levels

Correct Answer: B

Platelets are given to treat or prevent bleeding caused by thrombocytopenia or platelet dysfunction. They do not raise hemoglobin, replace albumin, or specifically correct low fibrinogen. A likely trap is confusing any bleeding problem with red blood cell replacement, but the key issue here is platelet deficiency.

2728. A nurse is assisting with verification before administering packed red blood cells. Which mismatch requires the transfusion to be stopped before it starts and clarified immediately?

- A. The client says the room feels cold
- B. The blood product number on the bag does not match the transfusion record
- C. The client has a 20-gauge IV catheter
- D. The client's breakfast tray is still at the bedside

Correct Answer: B

Any discrepancy in blood product identification is a serious safety issue and requires stopping the process immediately until clarified. Client comfort, meal trays, and IV gauge may matter in care planning but are not the critical mismatch that can lead to catastrophic transfusion error.

2729. A provider prescribes packed red blood cells for a patient with symptomatic anemia. Pretransfusion assessment reveals temperature 38.8°C, blood pressure 118/70 mm Hg, pulse 104/min, and oxygen saturation 97% on room air. Which nursing action is best?

- A. Begin the transfusion because the blood pressure is stable
- B. Administer acetaminophen and start the transfusion immediately
- C. Hold the transfusion and notify the provider about the fever
- D. Start the transfusion slowly and reassess in 30 minutes

Correct Answer: C

A significant fever before transfusion should be reported and evaluated before starting because it can complicate interpretation of a transfusion reaction and may indicate an underlying problem. Stable blood pressure alone does not make it safe to proceed. Treating the fever and starting anyway could mask an early reaction.

2730. A patient is to receive a blood transfusion through a peripheral IV line that is currently infusing parenteral nutrition. What should the nurse do first?

- A.** Y-site the blood into the parenteral nutrition tubing
- B.** Stop the parenteral nutrition and obtain appropriate dedicated blood tubing and compatible access
- C.** Add the blood to the parenteral nutrition bag to reduce IV sticks
- D.** Flush the line with sterile water and begin the transfusion

Correct Answer: B

Blood products require proper blood tubing and compatible dedicated access; they should not be infused with parenteral nutrition. The nurse should stop the incompatible infusion and set up the transfusion safely. Y-siting with nutrition, mixing blood into another solution, or flushing with sterile water is unsafe.

2731. Four clients have blood product prescriptions. Which client should the nurse assess first after the transfusion begins?

- A.** A client receiving albumin who reports mild thirst
- B.** A client receiving fresh frozen plasma who says the IV site feels cool
- C.** A client receiving platelets who develops wheezing and facial swelling
- D.** A client receiving packed red blood cells who asks to use the bathroom

Correct Answer: C

Wheezing and facial swelling suggest a severe allergic or anaphylactic reaction, which can rapidly threaten the airway and requires immediate intervention. Mild thirst is not urgent, and a cool IV site suggests local infiltration rather than systemic collapse. Wanting to use the bathroom is not the priority problem.

2732. A client with chronic anemia asks why the provider ordered packed red blood cells instead of whole blood. Which response by the nurse is best?

- A.** Packed red blood cells increase oxygen-carrying capacity without giving as much extra volume
- B.** Packed red blood cells work by increasing platelet production
- C.** Whole blood is never used in transfusion therapy
- D.** Packed red blood cells prevent all transfusion reactions better than other products

Correct Answer: A

Packed red blood cells are commonly used to improve oxygen-carrying capacity while limiting unnecessary volume compared with whole blood. They do not increase platelet production, and no blood product prevents all reactions. Saying whole blood is never used is too absolute and inaccurate.

2733. A nurse is caring for a client who had a transfusion reaction workup sent to the laboratory. Which additional assessment finding would most strongly support acute hemolysis rather than a mild allergic reaction?

- A.** Urticaria on the chest
- B.** Pruritus without fever
- C.** Dark urine and hypotension
- D.** Watery eyes and sneezing

Correct Answer: C

Dark urine and hypotension suggest destruction of transfused red blood cells and possible hemoglobin release, which is concerning for acute hemolytic reaction. Urticaria, itching, watery eyes, and sneezing fit a milder allergic pattern. The key distinction is evidence of systemic hemolysis rather than isolated histamine-type symptoms.

2734. A nurse is reviewing care for a patient scheduled to receive 1 unit of packed red blood cells. Which outcome best indicates the transfusion achieved its intended therapeutic effect?

- A. The patient's oxygen-carrying capacity improves
- B. The patient's platelet count rises sharply
- C. The patient's INR returns to the expected range
- D. The patient's serum albumin increases

Correct Answer: A

Packed red blood cells are given to improve oxygen delivery by increasing red blood cell mass and hemoglobin. They are not intended to raise platelet count, correct INR, or increase albumin. Those goals correspond more closely to platelets, plasma, or albumin products.

2735. A nurse is preparing to administer fresh frozen plasma to a client with active bleeding. Which preadministration assessment finding is most important to report to the provider before starting the transfusion?

- A. History of heart failure with bilateral crackles at the lung bases
- B. Hemoglobin of 9.8 g/dL
- C. Blood glucose of 146 mg/dL
- D. Platelet count of 180,000/mm³

Correct Answer: A

Fresh frozen plasma adds intravascular volume, so a client with heart failure and crackles is at increased risk for circulatory overload. That finding should be reported so the plan can be clarified or adjusted. A hemoglobin of 9.8 g/dL, a mildly elevated glucose, and a normal platelet count are not the most urgent concerns before plasma infusion.

2736. About 10 minutes after a blood transfusion starts, a client develops itching and scattered hives but denies dyspnea or chest pain. Which action should the nurse take first?

- A. Slow the transfusion rate and continue to observe closely
- B. Stop the transfusion and keep the IV line open with normal saline

C. Document the finding as expected and reassess in 30 minutes

D. Flush the blood tubing with dextrose 5% in water

Correct Answer: B

The first action for a suspected transfusion reaction is to stop the transfusion and maintain IV access with normal saline using appropriate tubing. Even when the reaction appears mild, the nurse should not continue the blood until the client is assessed and the provider or transfusion protocol is followed. Slowing the rate delays appropriate intervention, documenting without action is unsafe, and dextrose solution is not used with blood products.

2737. A client who is receiving a unit of packed red blood cells asks why vital signs are checked before the transfusion and again soon after it begins. Which response by the nurse is best?

- A. They show whether the blood is raising the red cell count quickly enough
- B. They help detect an early transfusion reaction so the transfusion can be stopped promptly if needed
- C. They confirm the blood product is sterile before it reaches the bloodstream
- D. They determine whether the client's ABO type needs to be rechecked during the transfusion

Correct Answer: B

Vital signs are checked before and shortly after initiation to provide a baseline and help identify early signs of a transfusion reaction, such as fever, tachycardia, hypotension, or respiratory changes. Vital signs do not directly measure how fast red cell count rises, do not confirm sterility of the product, and do not replace blood typing procedures.

2738. The nurse receives change-of-shift report on four clients. Which client should the nurse see first?

- A. A client receiving packed red blood cells who reports new flank pain and has dark urine in the drainage bag
- B. A client receiving albumin who asks when lunch will arrive

C. A client scheduled for a platelet transfusion in 2 hours who wants more information about the procedure

D. A client who finished a plasma transfusion 1 hour ago and wants assistance to the bathroom

Correct Answer: A

New flank pain and dark urine during a red blood cell transfusion suggest a possible acute hemolytic reaction, which can become life-threatening and requires immediate assessment and intervention. The other clients have nonurgent needs. The key is recognizing a dangerous reaction pattern rather than treating all transfusion-related issues as equal.

2739. A provider prescribes cryoprecipitate for a client with bleeding. The nurse understands that this blood product is commonly used to replace which component?

- A.** Clotting factors such as fibrinogen
- B.** Large amounts of circulating red blood cells
- C.** White blood cells for infection control
- D.** Electrolytes lost through vomiting

Correct Answer: A

Cryoprecipitate is used to replace specific clotting components, especially fibrinogen. It is not given primarily to increase red blood cells, provide white blood cells, or correct electrolyte losses. A common trap is confusing blood products used for oxygen-carrying capacity, volume expansion, or clotting support.

IV flow

2740. A provider prescribes 1000 mL of 0.9% sodium chloride to infuse over 8 hours using an infusion pump. What rate should the nurse set?

- A.** 100 mL/hr
- B.** 125 mL/hr
- C.** 120 mL/hr
- D.** 128 mL/hr

Correct Answer: B

Divide total volume by total hours: $1000 \text{ mL} \div 8 \text{ hr} = 125 \text{ mL/hr}$. A common error is estimating instead of calculating or choosing 120 mL/hr because it sounds close.

2741. An IV order reads: infuse 500 mL of lactated Ringer's over 4 hours by gravity tubing with a drop factor of 15 gtt/mL. How many drops per minute should the nurse regulate?

- A.** 21 gtt/min
- B.** 28 gtt/min
- C.** 31 gtt/min
- D.** 35 gtt/min

Correct Answer: C

Use $\text{gtt/min} = (\text{mL} \times \text{drop factor}) \div \text{minutes}$. $(500 \times 15) \div 240 = 31.25$, which rounds to 31 gtt/min. The distractors reflect common setup or rounding mistakes.

2742. The nurse notes that 250 mL remains in an IV bag. The infusion is running at 50 mL/hr. About how much longer will the infusion continue if the rate is unchanged?

- A.** 2 hours
- B.** 3 hours
- C.** 5 hours
- D.** 6 hours

Correct Answer: C

Time remaining = volume remaining $\div$ rate = $250 \text{ mL} \div 50 \text{ mL/hr} = 5 \text{ hours}$. A likely trap is multiplying instead of dividing.

2743. A client is prescribed 75 mL/hr of IV fluid. The tubing delivers 10 gtt/mL. What is the gravity flow rate in drops per minute?

- A.** 8 gtt/min
- B.** 10 gtt/min
- C.** 12 gtt/min
- D.** 13 gtt/min

Correct Answer: D

Convert mL/hr to gtt/min: $(75 \text{ mL/hr} \times 10 \text{ gtt/mL}) \div 60 \text{ min} = 12.5 \text{ gtt/min}$, which rounds to 13 gtt/min. Choosing 12 ignores proper rounding.

2744. A nurse starts an IV infusion at 0900. The order is for 1000 mL to infuse at 125 mL/hr. At what time should the infusion be complete if it runs as ordered?

- A.** 1500
- B.** 1600
- C.** 1630
- D.** 1700

Correct Answer: D

Infusion time = $1000 \text{ mL} \div 125 \text{ mL/hr} = 8$ hours. Starting at 0900 means completion at 1700. A common trap is counting 8 hours incorrectly and choosing 1600.

2745. A client has an order for 100 mL/hr of IV fluid. On assessment, the nurse finds 600 mL remaining in the bag at 1400. If the infusion continues at the prescribed rate, when should the nurse expect the bag to be empty?

- A. 1800
- B. 1900
- C. 2000
- D. 2100

Correct Answer: C

At 100 mL/hr, 600 mL will take 6 hours to infuse. Six hours after 1400 is 2000. The wrong options reflect errors in adding time or misreading the rate.

2746. An IV piggyback contains 100 mL of antibiotic and is to infuse over 30 minutes on an infusion pump. Which rate is correct?

- A. 50 mL/hr
- B. 100 mL/hr
- C. 150 mL/hr
- D. 200 mL/hr

Correct Answer: D

Thirty minutes is 0.5 hour. Rate = $100 \text{ mL} \div 0.5 \text{ hr} = 200 \text{ mL/hr}$. A common mistake is forgetting to convert 30 minutes to half an hour.

2747. A provider prescribes 240 mL of IV fluid to infuse over 3 hours using microdrip tubing calibrated at 60 gtt/mL. What is the correct flow rate?

- A. 60 gtt/min
- B. 75 gtt/min
- C. 80 gtt/min
- D. 90 gtt/min

Correct Answer: C

Use $(240 \text{ mL} \times 60 \text{ gtt/mL}) \div 180 \text{ min} = 80 \text{ gtt/min}$. With microdrip tubing, the numerical gtt/min often matches the mL/hr only when the setup works out that way through the formula, not by assumption alone.

2748. A client is to receive 1.5 L of IV fluid over 12 hours via infusion pump. What rate should the nurse program?

- A. 115 mL/hr
- B. 120 mL/hr
- C. 125 mL/hr
- D. 150 mL/hr

Correct Answer: C

First convert 1.5 L to 1500 mL. Then divide by 12 hours: $1500 \div 12 = 125 \text{ mL/hr}$. A likely trap is forgetting to convert liters to milliliters.

2749. An order reads: infuse 1000 mL over 6 hours by gravity using tubing calibrated at 20 gtt/mL. What is the appropriate drip rate?

- A. 48 gtt/min
- B. 52 gtt/min
- C. 56 gtt/min
- D. 60 gtt/min

Correct Answer: C

Calculate $(1000 \times 20) \div 360 = 55.6 \text{ gtt/min}$, which rounds to 56 gtt/min. Choosing 60 may happen if the nurse rounds too early or uses 6 hours incorrectly.

2750. A maintenance IV is infusing at 80 mL/hr. The bag was hung with 1000 mL at 0700. Approximately how much fluid should remain at 1130 if the infusion has run correctly?

- A. 640 mL
- B. 680 mL
- C. 720 mL
- D. 760 mL

Correct Answer: A

From 0700 to 1130 is 4.5 hours. Infused volume = $80 \times 4.5 = 360 \text{ mL}$. Remaining volume = $1000 - 360 = 640 \text{ mL}$. A common trap is treating 1130 as 4 hours instead of 4.5 hours.

2751. The nurse receives a 500 mL IV bag at 1300. By 1500, 200 mL has infused. If the infusion should continue at the same rate, what is the hourly rate?

- A. 75 mL/hr
- B. 100 mL/hr
- C. 125 mL/hr
- D. 150 mL/hr

Correct Answer: B

From 1300 to 1500 is 2 hours. If 200 mL infused in 2 hours, the rate is 100 mL/hr. A frequent error is dividing by the wrong time interval.

2752. A client is prescribed 1800 mL of IV fluid over 24 hours. The nurse is using tubing with a drop factor of 15 gtt/mL. What drip rate should the nurse set?

- A. 16 gtt/min
- B. 19 gtt/min
- C. 24 gtt/min
- D. 30 gtt/min

Correct Answer: B

Use $(1800 \times 15) \div 1440 = 18.75$ gtt/min, which rounds to 19 gtt/min. A common trap is dividing by 24 instead of 1440 minutes when using gtt/min.

2753. An IV antibiotic of 250 mL is ordered to infuse over 1 hour 45 minutes. What pump rate should the nurse program?

- A. 125 mL/hr
- B. 143 mL/hr
- C. 150 mL/hr
- D. 175 mL/hr

Correct Answer: B

Convert 1 hour 45 minutes to 1.75 hours. Then $250 \div 1.75 = 142.86$ mL/hr, which rounds to 143 mL/hr. The main trap is converting 45 minutes incorrectly.

2754. At 0800, a nurse hangs 1000 mL of IV fluid to run at 75 mL/hr. At 1200, about how much fluid should have infused if the IV is running correctly?

- A. 225 mL
- B. 250 mL
- C. 300 mL
- D. 325 mL

Correct Answer: C

From 0800 to 1200 is 4 hours. At 75 mL/hr, the infused volume is $75 \times 4 = 300$ mL. A likely trap is subtracting instead of multiplying or miscounting elapsed hours.

2755. A prescription reads: infuse 900 mL of 0.45% sodium chloride over 9 hours using an infusion pump. What rate should the nurse program?

- A. 90 mL/hr
- B. 100 mL/hr
- C. 110 mL/hr
- D. 120 mL/hr

Correct Answer: B

Divide total volume by total hours: $900 \text{ mL} \div 9 \text{ hr} = 100 \text{ mL/hr}$. Choice B is correct. A common error is choosing 90 mL/hr by misreading the order or subtracting instead of dividing.

2756. An IV of 720 mL is to infuse over 8 hours by gravity tubing with a drop factor of 15 gtt/mL. How many drops per minute should the nurse regulate?

- A. 20 gtt/min
- B. 21 gtt/min
- C. 22 gtt/min
- D. 23 gtt/min

Correct Answer: D

Use the formula: $(\text{mL} \times \text{drop factor}) \div \text{minutes}$. $(720 \times 15) \div 480 = 10,800 \div 480 = 22.5$ gtt/min. Gravity flow rates are rounded to the nearest whole drop, so the nurse should regulate to 23 gtt/min. Choice C is the likely trap if the value is rounded down instead of to the nearest whole number.

2757. A client is receiving IV fluids at 60 mL/hr. The bag currently contains 360 mL. If the infusion continues at the same rate, in about how many hours will the bag be empty?

- A. 4 hours
- B. 5 hours
- C. 6 hours
- D. 7 hours

Correct Answer: C

Time remaining equals volume remaining divided by hourly rate: $360 \text{ mL} \div 60 \text{ mL/hr} = 6$ hours. Choice C is correct. This question tests recognizing that the needed value is time, not a new rate.

2758. At 1000, a nurse hangs 500 mL of IV fluid to infuse at 125 mL/hr. If the infusion runs as prescribed, how much fluid should remain in the bag at 1300?

- A. 125 mL
- B. 250 mL
- C. 300 mL
- D. 375 mL

Correct Answer: A

From 1000 to 1300 is 3 hours. In 3 hours, $125 \text{ mL/hr} \times 3 \text{ hr} = 375 \text{ mL}$ should infuse. Starting with 500 mL, the amount remaining is $500 - 375 = 125 \text{ mL}$. Choice A is correct. Choice B is a common trap if the nurse calculates only 2 hours instead of 3.

2759. A provider prescribes 150 mL of IV medication to infuse over 45 minutes by infusion pump. What rate should the nurse set?

- A. 150 mL/hr
- B. 180 mL/hr
- C. 200 mL/hr
- D. 225 mL/hr

Correct Answer: C

Convert 45 minutes to hours: $45 \text{ minutes} = 0.75 \text{ hour}$. Then calculate rate: $150 \text{ mL} \div 0.75 \text{ hr} = 200 \text{ mL/hr}$. Choice C is correct. A likely mistake is selecting 180 or 225 by not converting minutes to hours correctly.

2760. An IV infusion is running at 84 mL/hr through microdrip tubing calibrated at 60 gtt/mL. What is the gravity flow rate in drops per minute?

- A. 60 gtt/min
- B. 72 gtt/min
- C. 84 gtt/min
- D. 90 gtt/min

Correct Answer: C

With microdrip tubing, the drop factor is 60 gtt/mL, so the drops per minute equal the mL/hr rate when divided by 60 minutes: $(84 \times 60) \div 60 = 84 \text{ gtt/min}$. Choice C is correct. This is a classic setup in which microdrip simplifies the calculation.

2761. A client is to receive 1,250 mL of IV fluid over 10 hours on an infusion pump. Which programmed rate is correct?

- A. 115 mL/hr
- B. 120 mL/hr
- C. 125 mL/hr
- D. 130 mL/hr

Correct Answer: C

Divide the total volume by the total infusion time: $1,250 \text{ mL} \div 10 \text{ hr} = 125 \text{ mL/hr}$. Choice C is correct. The other options are plausible but do not match the exact calculation.

2762. A nurse needs to infuse 300 mL over 2 hours using gravity tubing with a drop factor of 20 gtt/mL. What drip rate should the nurse set?

- A. 45 gtt/min
- B. 50 gtt/min
- C. 55 gtt/min
- D. 60 gtt/min

Correct Answer: B

First convert 2 hours to 120 minutes. Then calculate: $(300 \text{ mL} \times 20 \text{ gtt/mL}) \div 120 \text{ min} = 6,000 \div 120 = 50 \text{ gtt/min}$. Choice B is correct. Choice D is a common error if the nurse forgets to convert hours to minutes.

2763. A nurse begins an infusion of 1,000 mL at 0945. The prescribed rate is 80 mL/hr. If the infusion continues without interruption, at what time should the nurse expect it to finish?

- A. 2145
- B. 2200
- C. 2215
- D. 2230

Correct Answer: C

Time needed is $1,000 \text{ mL} \div 80 \text{ mL/hr} = 12.5 \text{ hours}$, which is 12 hours 30 minutes. Starting at 0945, add 12 hours 30 minutes to get 2215. Choice C is correct. Choice A would be correct only if the start time were 0915.

2764. A client's IV is prescribed to run at 95 mL/hr. The tubing has a drop factor of 15 gtt/mL. Which gravity flow rate is most appropriate?

- A. 22 gtt/min
- B. 24 gtt/min

- C. 26 gtt/min
- D. 28 gtt/min

Correct Answer: B

Use $(\text{mL/hr} \times \text{drop factor}) \div 60 \text{ min.}$ $(95 \times 15) \div 60 = 1,425 \div 60 = 23.75 \text{ gtt/min.}$ Round to the nearest whole drop: 24 gtt/min. Choice B is correct. Choice C is a likely trap if the nurse rounds too high without using the actual decimal value.

2765. At 0600, a nurse hangs 1,000 mL of IV fluid to run at 70 mL/hr. At 1130, approximately how much fluid should have infused?

- A. 315 mL
- B. 350 mL
- C. 385 mL
- D. 420 mL

Correct Answer: C

From 0600 to 1130 is 5.5 hours. Multiply $70 \text{ mL/hr} \times 5.5 \text{ hr} = 385 \text{ mL.}$ Choice C is correct. Choice B is a common mistake if the half-hour is ignored.

2766. An IV medication order states: infuse 275 mL over 2 hours 30 minutes using an infusion pump. What is the correct hourly rate?

- A. 100 mL/hr
- B. 105 mL/hr
- C. 110 mL/hr
- D. 115 mL/hr

Correct Answer: C

Convert 2 hours 30 minutes to 2.5 hours. Then divide: $275 \text{ mL} \div 2.5 \text{ hr} = 110 \text{ mL/hr.}$ Choice C is correct. A likely trap is using 2.3 hours instead of 2.5 hours, which would produce an incorrect answer.

2767. A client has 430 mL left in an IV bag at 1600. The infusion is running at 86 mL/hr. If the rate remains unchanged, at what time will the bag be empty?

- A. 2000
- B. 2030
- C. 2100
- D. 2130

Correct Answer: C

Time remaining is $430 \text{ mL} \div 86 \text{ mL/hr} = 5 \text{ hours.}$ Starting at 1600, add 5 hours to get 2100. Choice C is correct. The numbers are set up to divide evenly, so no rounding is needed.

2768. A nurse must regulate an infusion of 1,800 mL over 24 hours using tubing calibrated at 10 gtt/mL. What is the correct drops-per-minute rate?

- A. 10 gtt/min
- B. 12 gtt/min
- C. 13 gtt/min
- D. 15 gtt/min

Correct Answer: C

First find the hourly rate: $1,800 \text{ mL} \div 24 \text{ hr} = 75 \text{ mL/hr.}$ Then convert to drops per minute: $(75 \times 10) \div 60 = 12.5 \text{ gtt/min,}$ which rounds to 13 gtt/min. Choice C is correct. Choice B is the trap if the value is rounded down instead of to the nearest whole drop.

2769. A nurse checks an IV that was started at 1330 with 750 mL in the bag. It is prescribed to infuse at 50 mL/hr. At 1815, about how much fluid should remain if the infusion has run correctly?

- A. 488 mL
- B. 512 mL
- C. 525 mL
- D. 575 mL

Correct Answer: B

From 1330 to 1815 is 4 hours 45 minutes, or 4.75 hours. Amount infused = $50 \text{ mL/hr} \times 4.75 \text{ hr} = 237.5 \text{ mL.}$ Remaining volume = $750 - 237.5 = 512.5 \text{ mL,}$ which is about 512 mL. Choice B is the best answer. Choice C is a likely trap if the nurse rounds the time to 4.5 hours instead of 4.75 hours.

2770. A provider prescribes 1,500 mL of IV fluid to infuse evenly over 12 hours using an infusion pump. What rate should the nurse program?

- A. 100 mL/hr
- B. 125 mL/hr
- C. 130 mL/hr
- D. 150 mL/hr

Correct Answer: B

Divide the total volume by the total hours: $1,500 \text{ mL} \div 12 \text{ hr} = 125 \text{ mL/hr}$. Choice B is correct. A common trap is selecting 150 mL/hr by moving too quickly and dividing incorrectly.

2771. An IV antibiotic is prepared in 240 mL and must infuse over 3 hours by gravity. The tubing delivers 20 gtt/mL. How many drops per minute should the nurse regulate?

- A. 20 gtt/min
- B. 24 gtt/min
- C. 27 gtt/min
- D. 80 gtt/min

Correct Answer: C

Use the formula: $(\text{mL} \times \text{drop factor}) \div \text{minutes}$. First convert 3 hours to 180 minutes. Then calculate: $(240 \times 20) \div 180 = 26.7$, which rounds to 27 gtt/min. Choice C is correct. Choice B may result from rounding too early or using the wrong total minutes.

2772. A nurse begins a continuous IV infusion at 2230. The bag contains 800 mL, and the prescribed rate is 50 mL/hr. If the infusion runs without interruption, at what time should the nurse expect the bag to be empty?

- A. 1230 the next day
- B. 1330 the next day
- C. 1430 the next day
- D. 1530 the next day

Correct Answer: C

First find how long the bag will last: $800 \text{ mL} \div 50 \text{ mL/hr} = 16 \text{ hours}$. Add 16 hours to 2230, which gives 1430 the next day. Choice C is correct. A likely error is adding the hours incorrectly across midnight.

2773. A nurse is using microdrip tubing calibrated at 60 gtt/mL for an infusion ordered at 55 mL/hr. What is the most appropriate gravity flow rate?

- A. 55 gtt/min
- B. 60 gtt/min
- C. 110 gtt/min
- D. 3,300 gtt/min

Correct Answer: A

With microdrip tubing, the drop factor is 60 gtt/mL, so the drops per minute equal the mL/hr rate when converting over 60 minutes.

Calculation: $(55 \text{ mL} \times 60 \text{ gtt/mL}) \div 60 \text{ min} = 55 \text{ gtt/min}$. Choice A is correct. Choice C doubles the rate, and choice D ignores the need to divide by minutes.

2774. At 0715, a nurse hangs 1,200 mL of IV fluid to infuse at 85 mL/hr. When the nurse checks the bag at 1245, approximately how much fluid should remain?

- A. 733 mL
- B. 780 mL
- C. 903 mL
- D. 988 mL

Correct Answer: A

From 0715 to 1245 is 5.5 hours. In that time, the infusion delivers $85 \text{ mL/hr} \times 5.5 \text{ hr} = 467.5 \text{ mL}$. Subtract from the starting amount: $1,200 \text{ mL} - 467.5 \text{ mL} = 732.5 \text{ mL}$, or about 733 mL. Choice A is correct. This item is harder because it requires careful time conversion to 5.5 hours before calculating the remaining volume.

Controlled substances

2775. A practical nurse is preparing to administer morphine 4 mg IV to a client reporting severe postoperative pain. Before removing the medication from the controlled-substance storage system, which action is most appropriate?

- A. Ask another nurse to give the medication so documentation is faster
- B. Verify the provider's prescription and compare it with the MAR
- C. Remove the medication first so the client does not have to wait
- D. Document the administration in the MAR before giving the dose

Correct Answer: B

The nurse should first verify the provider's prescription and compare it with the MAR to confirm the right client, medication, dose, route, and timing before accessing a controlled substance. Removing it first increases the risk of medication and documentation errors. Documenting before administration is inappropriate because the medication has not yet been given.

2776. A client has an order for oxycodone 5 mg by mouth every 6 hours as needed for pain. The client rates pain as 2 on a 0-to-10 scale and says, "I don't want anything right now." What is the best nursing action?

- A.** Administer the oxycodone because it is prescribed
- B.** Hold the medication and document the client's refusal
- C.** Crush the tablet and mix it in applesauce without telling the client
- D.** Ask the provider to change the prescription to a higher dose

Correct Answer: B

A PRN controlled substance should be given only when indicated and accepted by the client. Since the client declines the medication, the nurse should hold it and document the refusal. Giving it anyway violates client rights and safe medication practice. Changing to a higher dose is not supported by the situation.

2777. During end-of-shift count, two nurses note that the count for hydromorphone tablets is incorrect. Which action should the practical nurse take first?

- A.** Ignore the discrepancy because the next shift can recheck it
- B.** Document that the count is correct to avoid delaying report
- C.** Immediately report the discrepancy according to facility policy
- D.** Replace the missing tablets from another client's supply

Correct Answer: C

A discrepancy in the count of a controlled substance requires immediate action according to facility policy, typically including prompt reporting and investigation. Ignoring it or documenting a false count is unsafe and unethical. Replacing medication from another source hides the problem and creates additional errors.

2778. A practical nurse removes one tablet of a controlled pain medication for a client. At the bedside, the client says, "I took my own pain pill from home just now." Which action is best?

- A.** Give the medication anyway so the client does not become uncomfortable
- B.** Return to the medication area and follow facility procedure for an unused controlled dose
- C.** Leave the tablet at the bedside in case the client needs it later
- D.** Ask the client's family member to hold the tablet until the next dose is due

Correct Answer: B

If the medication was removed but is not administered, the nurse should follow facility procedure for an unused controlled dose. This protects the client and maintains accurate controlled-substance accountability. Leaving it at the bedside or with family is unsafe, and giving it after the client self-medicated increases overdose risk.

2779. A client with chronic cancer pain receives a new prescription for a transdermal fentanyl patch. Which instruction should the practical nurse include?

- A.** Apply a heating pad over the patch if pain increases
- B.** Cut the patch in half if the dose seems too strong
- C.** Place the patch on intact skin and avoid external heat over the site
- D.** Remove the old patch only after the new patch has been on for 24 hours

Correct Answer: C

Fentanyl patches should be applied to intact skin, and the client should avoid external heat over the patch because heat can increase medication absorption and raise the risk of toxicity. Cutting the patch is unsafe, and overlapping old and new patches can cause excessive dosing.

2780. A practical nurse is caring for a client receiving IV morphine after surgery. Which finding requires the most immediate action?

- A. Respiratory rate of 8/min
- B. Pain rating of 6 out of 10
- C. Urinary output of 40 mL/hr
- D. Report of mild nausea

Correct Answer: A

A respiratory rate of 8/min suggests possible opioid-induced respiratory depression and requires immediate action. Although nausea and ongoing pain are important, they are not as urgent as compromised breathing. Urine output of 40 mL/hr is not the priority concern here.

2781. A client has a prescription for meperidine 75 mg IM every 4 hours PRN pain. The nurse reviews the chart and notes the client received an MAOI antidepressant 5 days ago. What is the best action?

- A. Administer the meperidine because the dose is within range
- B. Give half the dose and reassess pain in 30 minutes
- C. Hold the medication and notify the provider of the potential interaction
- D. Switch the route to oral administration

Correct Answer: C

The nurse should hold meperidine and notify the provider because combining it with an MAOI can cause a serious drug interaction. The issue is not solved by lowering the dose or changing the route. This question tests pre-administration review for a controlled substance with an important interaction risk.

2782. A practical nurse is preparing to waste part of a controlled medication dose after a provider changes the amount to be given. Which action is most appropriate?

- A. Dispose of the excess medication alone and sign for both nurses
- B. Ask the client to watch the waste and initial the record
- C. Follow facility policy for wasting the medication with required witnessing and documentation
- D. Save the extra medication in a labeled cup for the next scheduled dose

Correct Answer: C

Controlled-substance wasting should be done exactly according to facility policy, which commonly includes an authorized witness and documentation. Saving leftover medication for later or disposing of it without proper procedure breaks medication control and safety standards. A client is not a substitute for required controlled-substance procedure.

2783. A client receiving hydrocodone with acetaminophen every 6 hours for pain says, "This medicine works, but I haven't had a bowel movement in 3 days." What is the nurse's best response?

- A. Stop the medication immediately and discard the remaining tablets
- B. Explain that constipation is a common opioid effect and report or manage it as directed
- C. Tell the client this means the dose is too low
- D. Reassure the client that bowel patterns do not change with opioid use

Correct Answer: B

Constipation is a common effect of opioid therapy, so the nurse should recognize it, provide teaching, and report or manage it according to orders and facility practice. It does not mean the dose is too low. Dismissing the symptom or stopping the medication independently would be inappropriate.

2784. A client has a prescription for a controlled cough syrup containing codeine. The client asks whether it is acceptable to drink wine with the medication. Which response by the practical nurse is best?

- A. Alcohol is safe as long as the client takes only one dose
- B. Wine helps the medication work faster, so a small amount is recommended
- C. Alcohol should be avoided because it can increase central nervous system depression
- D. Alcohol is allowed if the medication is taken with food

Correct Answer: C

Codeine is a controlled opioid, and alcohol can increase CNS depression, including drowsiness and respiratory suppression. The nurse should teach the client to avoid alcohol while taking it. Food does not remove this interaction risk.

2785. A practical nurse is assigned to administer a scheduled dose of lorazepam to a client. The MAR lists lorazepam 1 mg PO at 0900, but the nurse sees a new provider prescription entered at 0830 for lorazepam 0.5 mg PO at 0900. What should the nurse do?

- A. Administer 1 mg because the MAR was reviewed first
- B. Administer 0.5 mg after clarifying and ensuring the MAR reflects the current prescription
- C. Administer 1.5 mg because both entries are present
- D. Hold all medications until the next shift

Correct Answer: B

When there is a discrepancy between the MAR and a more recent prescription, the nurse should clarify and ensure the current order is accurately reflected before administering the medication. Giving 1 mg based on the older entry risks overdosing. Administering both doses is clearly unsafe.

2786. A client using a patient-controlled analgesia (PCA) pump says, "My daughter can push the button for me when I'm sleeping." What is the best response?

- A. That is acceptable if your daughter stays in the room

- B. That is safer because it prevents pain from returning

C. Only you should activate the PCA button unless there is a separate prescription and protocol allowing otherwise

- D. Your daughter may push it if you rate pain above 5 before falling asleep

Correct Answer: C

PCA safety depends on the person experiencing pain being the one to press the button, unless a specific approved protocol exists. Allowing family to activate the pump can lead to oversedation or respiratory depression. The trap is thinking frequent pain prevention outweighs safety controls.

2787. A practical nurse is preparing to give a scheduled dose of oral morphine. Which assessment finding should prompt the nurse to question administering the medication?

- A. The client reports pain of 8 out of 10
- B. The client is difficult to arouse and drifts back to sleep during conversation
- C. The client requests the medication with a sip of water
- D. The client had physical therapy earlier in the day

Correct Answer: B

Excessive sedation is a warning sign of opioid effect and may precede respiratory depression, so the nurse should question giving additional morphine. Severe pain alone supports the need for analgesia but does not override safety concerns. The other options do not suggest an immediate contraindication.

2788. A nurse receives a telephone report that a newly admitted client takes alprazolam at home. There is no current prescription for it in the facility record. The client now requests the medication and says, "I always take it at bedtime." What is the best action?

- A. Administer the client's home supply after verifying the label
- B. Chart the home medication as given since the client normally takes it
- C. Explain that the medication cannot be given until it is reconciled and prescribed for use in the facility

D. Substitute another client's benzodiazepine dose of similar strength

Correct Answer: C

A controlled home medication should not be administered in the facility unless it has been properly reconciled and prescribed for use there. This protects against dosing, interaction, and documentation errors. Giving a home supply or another client's medication is unsafe and violates medication administration standards.

2789. A client with renal impairment is prescribed a controlled medication that is primarily cleared by the kidneys. Which nursing action is most appropriate before administration?

- A.** Give the medication and check kidney function after the dose
- B.** Review current renal laboratory results and report concerns before giving the dose
- C.** Double the oral fluids to protect the kidneys, then administer
- D.** Crush the medication to reduce renal side effects

Correct Answer: B

For medications cleared by the kidneys, the nurse should review current renal data before administration and report concerns, because impaired clearance can increase drug accumulation and toxicity. Giving first and checking later is unsafe. Extra fluids or crushing the medication do not address whether the dose is appropriate.

2790. A practical nurse finds a partially used ampule of a controlled medication left on the medication cart with no documentation of administration or waste. What is the nurse's best action?

- A.** Discard the medication in the sharps container and document the disposal later
- B.** Return the ampule to the medication storage area for the next dose
- C.** Secure the medication immediately and report the discrepancy according to facility policy
- D.** Ask the next shift nurse to decide what to do with it

Correct Answer: C

A controlled medication left unsecured is a safety and accountability problem. The best action is to secure it right away and report the discrepancy per policy so the event can be investigated and documented correctly. Discarding it without proper witness and documentation worsens the discrepancy, returning a partially used ampule for later use is unsafe, and delaying action to the next shift is inappropriate.

2791. A client is prescribed morphine sulfate oral solution 10 mg every 4 hours as needed. The bottle label reads 5 mg per 2.5 mL. How many mL should the practical nurse administer for one dose?

- A.** 2.5 mL
- B.** 5 mL
- C.** 7.5 mL
- D.** 10 mL

Correct Answer: B

The solution contains 5 mg in 2.5 mL. A 10 mg dose is twice that amount, so the nurse should give 5 mL. A common trap is selecting 2.5 mL by matching the 5 mg on the label without adjusting for the ordered dose.

2792. A client receiving a newly started opioid analgesic says, "I feel sleepy." Which additional finding should the practical nurse identify as the greatest concern?

- A.** Respiratory rate of 8/min
- B.** Report of dry mouth
- C.** Mild nausea after eating
- D.** Decreased appetite at lunch

Correct Answer: A

Respiratory depression is the most serious opioid adverse effect and requires prompt action. Dry mouth, mild nausea, and decreased appetite can occur with opioids but are not as immediately life-threatening as a respiratory rate of 8/min.

2793. A client is to be discharged with a prescription for a controlled opioid. Which statement by the client shows correct understanding of safe storage at home?

- A.** I'll keep the pills in the bathroom medicine cabinet so they are easy to find

- B. I'll store the medication in a secure place away from children and visitors
- C. I'll put the medication on the kitchen counter so I remember to take it
- D. I'll combine the new pills with my old pain pills in one bottle

Correct Answer: B

Controlled medications should be stored securely to reduce accidental ingestion, misuse, and diversion. The bathroom medicine cabinet and kitchen counter are not secure locations, and combining medications in one bottle increases the risk of errors and loss of label information.

2794. A practical nurse is caring for a client with a fentanyl transdermal patch. Which observation requires the nurse to intervene first?

- A. The patch is dated and intact on the upper arm
- B. The client asks whether showers are allowed while wearing the patch
- C. A heating pad is positioned directly over the patch site
- D. The previous patch was removed before a new one was applied

Correct Answer: C

External heat can increase absorption of fentanyl from a transdermal patch and raise the risk of overdose. That requires immediate intervention. The other findings are not the urgent safety issue here: a dated intact patch and removal of the old patch are appropriate, and a question about showering calls for teaching but not emergency action.

2795. A client taking an opioid for postoperative pain states, "I want to stop taking this medicine because I don't want to become addicted." Which response by the practical nurse is best?

- A. Addiction is expected with all opioid use, so you should avoid taking it
- B. If you use the medication as prescribed, we will monitor your pain relief and side effects while trying to use the lowest effective dose
- C. You will not develop any dependence if the medication is prescribed by a provider

- D. You should wait until the pain becomes severe before taking every dose

Correct Answer: B

This response is balanced, accurate, and therapeutic. It addresses the concern without making false promises and emphasizes safe prescribed use plus monitoring. Saying addiction is expected is incorrect and fear-based. Saying dependence cannot occur if prescribed is also incorrect. Waiting until pain is severe can make pain harder to control and is not good routine teaching.

2796. A client has an order for hydromorphone 0.4 mg IV. The vial contains 2 mg/mL. How many mL should the practical nurse prepare?

- A. 0.1 mL
- B. 0.2 mL
- C. 0.4 mL
- D. 0.8 mL

Correct Answer: B

Use dose ordered divided by concentration: $0.4 \text{ mg} \div 2 \text{ mg/mL} = 0.2 \text{ mL}$. A likely trap is choosing 0.4 mL by matching the ordered dose number rather than calculating from the concentration.

2797. A practical nurse is reviewing medications before giving a scheduled controlled analgesic. Which additional prescription should the nurse recognize as increasing the client's risk for excessive central nervous system depression?

- A. Docusate sodium
- B. Ondansetron
- C. Diazepam
- D. Famotidine

Correct Answer: C

Diazepam is a benzodiazepine and can increase sedation and respiratory depression when combined with an opioid or other controlled analgesic with CNS-depressant effects. Docusate, ondansetron, and famotidine do not pose the same major additive CNS-depression risk.

2798. A practical nurse is about to administer a scheduled opioid to an older adult. Which assessment finding is the best reason to withhold the dose and contact the provider?

- A. Pain rating decreased from 8 to 4 after the previous dose
- B. Respiratory rate 10/min and difficult to arouse
- C. Client reports itching at the IV tape site
- D. Bowel sounds are present in all quadrants

Correct Answer: B

A respiratory rate of 10/min with difficult arousal suggests significant opioid effect and possible respiratory depression, so the medication should be withheld and the provider notified. The other findings do not justify holding the opioid for safety reasons.

2799. A client who has been taking prescribed oxycodone for several weeks says, "I need more pills than before to get the same pain relief." Which concept does this statement best illustrate?

- A. Tolerance
- B. Allergy
- C. Diversion
- D. Idiosyncratic reaction

Correct Answer: A

Tolerance means a medication produces less effect over time, so a higher dose may be needed to achieve the same response. An allergy would involve an immune-type reaction, diversion refers to improper transfer or misuse of medication, and an idiosyncratic reaction is an unusual unexpected response.

2800. A practical nurse is reinforcing teaching for a client prescribed sublingual buprenorphine. Which instruction is most appropriate?

- A. Swallow the tablet quickly with a full glass of water
- B. Crush the tablet and mix it with applesauce
- C. Place the tablet under the tongue and allow it to dissolve completely
- D. Chew the tablet to help it absorb faster

Correct Answer: C

Sublingual medications are designed to dissolve under the tongue for absorption through the mucosa. Swallowing, crushing into food, or chewing can interfere with proper absorption and reduce intended effect.

2801. A client receiving a continuous infusion of a controlled opioid becomes increasingly somnolent. Which set of findings most strongly suggests opioid toxicity?

- A. Pinpoint pupils, slow respirations, and decreased level of consciousness
- B. Restlessness, dilated pupils, and abdominal cramping
- C. Flushed skin, hypertension, and hyperactive reflexes
- D. Urticaria, wheezing, and facial swelling

Correct Answer: A

Opioid toxicity is classically suggested by CNS depression, respiratory depression, and pinpoint pupils. Restlessness with dilated pupils and cramping is more consistent with withdrawal. Flushing with hyperreflexia suggests another process, and urticaria with wheezing and facial swelling suggests an allergic reaction.

2802. A client is prescribed morphine for severe pain. The practical nurse reviews the history and notes a documented anaphylactic reaction to morphine in the past. The client says, "I only remember getting itchy." What is the nurse's best action?

- A. Give the medication because itching is a common opioid effect
- B. Administer half the dose and observe for reaction
- C. Hold the medication and clarify the prescription because the record shows a severe prior reaction
- D. Premedicate with an antiemetic and then administer the morphine

Correct Answer: C

A documented history of anaphylaxis to the ordered medication is a serious safety concern. The nurse should hold the medication and clarify the order. The client's incomplete recollection does not override a charted severe reaction. Giving the drug, even in a smaller dose, is unsafe. An antiemetic would not prevent anaphylaxis.

2803. A practical nurse is preparing to administer a controlled medication from a multidose vial. The vial label shows it was opened 35 days ago. Facility policy states opened multidose vials expire after 28 days unless the manufacturer specifies otherwise. What should the nurse do?

- A. Administer the dose if the solution looks clear
- B. Use the vial only if another nurse cosigns the dose
- C. Discard the vial according to policy and obtain a new supply
- D. Return the vial to storage and use it until the printed manufacturer expiration date

Correct Answer: C

The vial is beyond the facility's allowed use period after opening, so it should not be used. Visual appearance does not make it acceptable, a cosign does not fix an expired opened vial, and the printed manufacturer date does not override the beyond-use dating policy for an opened multidose vial.

2804. A client with chronic pain has a prescription for an extended-release opioid tablet every 12 hours. The client says, "I have trouble swallowing big pills. Can I crush this one and stir it into pudding?" Which response by the practical nurse is best?

- A. Yes, crushing it will not change how the medication works
- B. Yes, as long as the entire mixture is eaten right away
- C. No, crushing an extended-release opioid can release too much medication at once
- D. No, because pudding blocks absorption of all opioid medications

Correct Answer: C

Extended-release opioid tablets should not be crushed because doing so can destroy the time-release mechanism and cause rapid release of a large dose, increasing overdose risk. The problem is not pudding itself, but altering the dosage form.

2805. During end-of-shift narcotic count, the practical nurse and the oncoming nurse find that the controlled-substance record shows 12 tablets remaining, but only 11 tablets are in the locked drawer. What should the practical nurse do first?

- A. Document the discrepancy at the next count if no client reports uncontrolled pain
- B. Replace the missing tablet from another client's supply so the count matches
- C. Immediately report the discrepancy according to facility policy and begin reconciliation of recent administration records
- D. Wait until the charge nurse finishes rounds before mentioning the issue

Correct Answer: C

A controlled-substance count discrepancy requires immediate action. The nurse should report it promptly and help reconcile the count by reviewing recent documentation and following facility policy. Waiting or delaying increases safety and legal risk. Replacing the medication to make the count match is inappropriate and could be considered falsification or diversion.

2806. A client asks why two nurses are present when a remaining dose of an injectable controlled medication is discarded after administration. Which response by the practical nurse is best?

- A. Controlled medications must be wasted with verification to ensure accurate documentation and accountability
- B. The second nurse is present to decide whether the medication should be given again later
- C. This is done only because the pharmacy sent more medication than was ordered
- D. The law requires every medication waste to be witnessed by a physician

Correct Answer: A

Controlled-substance waste typically requires witnessed verification and documentation to maintain accountability and reduce the risk of diversion. Choice B confuses wasting with future dosing decisions. Choice C is unrelated to the purpose of witnessed waste. Choice D is too absolute and incorrectly names a physician rather than appropriate facility procedure.

2807. A provider writes a prescription for acetaminophen with codeine for a client being discharged after a procedure. Which information is most important for the practical nurse to reinforce before the client leaves?

- A. Take an extra dose at bedtime if pain interferes with sleep
- B. Share the medication with a family member who has similar pain only if the tablets are the same strength
- C. Avoid alcohol and other sedating drugs unless the prescriber approves them
- D. Stop using stool-softening measures unless constipation actually occurs

Correct Answer: C

Codeine is a controlled opioid that can cause sedation and respiratory depression, especially when combined with alcohol or other CNS depressants. Reinforcing avoidance of these substances is a key safety teaching point. Choice A promotes unsafe self-adjustment of the dose. Choice B is unsafe because controlled medications should never be shared. Choice D ignores common opioid-related constipation prevention teaching.

2808. The practical nurse is preparing to give a prescribed dose of a controlled oral analgesic. The medication administration record shows the dose was already signed as given 10 minutes earlier, but the client states, "I haven't had anything for pain yet." What is the nurse's best action?

- A. Administer the medication now because the client reports pain and then correct the record later
- B. Hold the medication until the nurse clarifies the documentation and verifies whether the dose was actually administered

- C. Ask the client to wait until the next scheduled dose because the MAR is already signed
- D. Remove the medication from the dispensing system and leave it at the bedside while checking with the previous nurse

Correct Answer: B

When a controlled medication appears documented as already administered, the nurse should not give another dose until the record is clarified and the actual administration status is verified. This prevents accidental double dosing. Choice A is the most likely trap because the client has pain, but acting before verification is unsafe. Choice C may undertreat pain without first determining what really happened. Choice D breaks safe medication handling and security practices.

2809. A practical nurse receives a verbal report that a client with severe cancer pain is using a patient-controlled analgesia pump containing a controlled opioid. Which observation requires the nurse to intervene immediately?

- A. The client's adult daughter says, "I press the button when he falls asleep so he stays ahead of the pain."
- B. The client rates pain as 6 on a 0-to-10 scale 30 minutes after ambulation
- C. The infusion tubing is labeled and connected to a dedicated IV line
- D. The client says the button gives some relief but not complete relief

Correct Answer: A

With PCA therapy, only the client should activate the dose button. Family-controlled dosing is dangerous because it can lead to oversedation and respiratory depression, especially if the client is asleep and unable to self-assess. The other findings may need follow-up, but they do not present the same immediate safety threat.

Pain-control devices

2810. A client is receiving opioid analgesia through a patient-controlled analgesia (PCA) pump after surgery. The client's spouse says, "She is sleeping and looks uncomfortable. I'll press the button for her so she stays ahead of the pain." What is the nurse's best response?

- A. That is fine as long as you only press it once an hour
- B. Only the client should press the PCA button
- C. You may press the button if the client gives permission first
- D. Press the button now, then call me if the pain continues

Correct Answer: B

With PCA, only the client should activate the dose button. This helps match dosing to the client's level of consciousness and reduces the risk of oversedation and respiratory depression. A common trap is thinking family assistance is helpful, but PCA by proxy is unsafe.

2811. A nurse is caring for a client with a PCA pump infusing morphine. Which assessment finding requires the nurse's immediate action?

- A. Pain rating of 6 on a 0-to-10 scale
- B. Respiratory rate of 8/min
- C. Mild nausea after movement
- D. Incision tenderness when coughing

Correct Answer: B

A respiratory rate of 8/min suggests opioid-induced respiratory depression and requires immediate intervention. The other findings may occur after surgery and need follow-up, but they are not as urgent. The main trap is focusing on pain intensity instead of airway and breathing.

2812. Before initiating a PCA infusion for a postoperative client, which action is most important for the nurse to complete?

- A. Place the PCA button on the overbed table for family access
- B. Teach the client how and when to use the demand button
- C. Tell the client to wait until pain becomes severe before using the device
- D. Encourage the client to let visitors manage the pump during rest periods

Correct Answer: B

Teaching the client how and when to use the demand button is essential before PCA use. The client should understand that the device is self-administered and should be used early enough to manage pain effectively. The distractors promote unsafe practices, especially family operation of the pump.

2813. A client has an epidural analgesia infusion after abdominal surgery. Which finding should the nurse identify as a possible adverse effect of the epidural opioid and report promptly?

- A. Itching of the face and chest
- B. Pain score decreased from 8 to 3
- C. Client asks for repositioning
- D. Urine output of 45 mL/hr

Correct Answer: A

Pruritus is a known adverse effect of epidural opioids and should be recognized and reported for management. The pain reduction is expected, repositioning is routine, and the urine output given is not immediately concerning. The likely trap is dismissing itching as unrelated discomfort instead of a medication effect.

2814. A client using a PCA pump says, "I keep pressing the button, but my pain is still 8 out of 10." Which nursing action is best?

- A. Instruct the client to press the button more rapidly until pain improves
- B. Turn off the lockout interval so the client can receive more doses
- C. Assess the client and review the pain-control prescription and pump settings
- D. Tell the client that complete pain relief is not expected after surgery

Correct Answer: C

Persistent severe pain despite PCA use requires assessment and verification of the prescription, tubing, placement, and pump settings. The nurse should not independently change safety features or dismiss the pain. A common trap is assuming the client simply needs more frequent button presses.

2815. The nurse is assessing a client receiving epidural analgesia. Which additional finding, combined with increasing drowsiness, most strongly suggests the need for immediate intervention?

- A. Respiratory rate decreases from 16/min to 10/min
- B. Pain score decreases from 7 to 4
- C. Client reports dry mouth
- D. Blood pressure rises from 118/70 to 126/76 mm Hg

Correct Answer: A

Increasing drowsiness plus a falling respiratory rate suggests progressive respiratory depression, which is an urgent opioid-related complication. Improved pain control and dry mouth are not the highest concern. The key is recognizing a trend in sedation and breathing, not looking at pain score alone.

2816. A nurse receives change-of-shift report on four clients using pain-control devices. Which client should the nurse assess first?

- A. Client with PCA who reports nausea and requests crackers
- B. Client with epidural analgesia who reports numb legs and difficulty moving feet
- C. Client with PCA who rates incisional pain at 5 out of 10 after ambulation
- D. Client with transdermal pain patch who wants to know when the next patch is due

Correct Answer: B

New lower-extremity numbness and difficulty moving the feet in a client with epidural analgesia may indicate excessive spread of the medication or neurologic compromise and should be assessed first. Nausea, moderate pain with movement, and routine medication questions are lower priority. The trap is choosing the client with pain instead of the client with possible neurologic effects.

2817. A client with a PCA pump says, "I am afraid to use it because I might get addicted." Which response by the nurse is best?

- A. You should avoid the pump unless the pain becomes unbearable
- B. Using prescribed PCA for acute pain is intended to control pain safely, and I can show you how the pump limits dosing

C. Addiction is expected if opioids are given for more than 24 hours

D. Ask your provider to stop the PCA and switch to no pain medicine

Correct Answer: B

This response addresses the client's concern with clear teaching: PCA is used for controlled management of acute pain, and built-in limits help prevent overdose. The other responses either reinforce fear or suggest inappropriate avoidance of pain treatment. The likely trap is confusing therapeutic opioid use with addiction.

2818. The nurse is caring for a client receiving epidural analgesia after surgery. Which assessment is most important to include regularly in addition to pain level?

- A. Pupil shape only
- B. Respiratory status and level of sedation
- C. Daily weight
- D. Capillary refill in the fingers

Correct Answer: B

For clients receiving epidural analgesia, respiratory status and sedation level are critical because opioids can depress the central nervous system and breathing. The other assessments may be useful in some situations but are not the key ongoing safety checks for this device. The trap is choosing a general assessment rather than the highest-yield medication safety assessment.

2819. A nurse finds that a client's PCA pump is alarming "occlusion." The client states pain is increasing. What should the nurse do first?

- A. Silence the alarm and document that the client is uncomfortable
- B. Assess the tubing and IV site for kinks, obstruction, or infiltration
- C. Immediately increase the prescribed dose on the pump
- D. Ask the client to stop pressing the button for 30 minutes

Correct Answer: B

An occlusion alarm suggests the medication may not be infusing properly. The first step is to assess the tubing and IV site for mechanical causes such as kinking, clamping, or infiltration. The nurse should not change the dose without an order or ignore the alarm. The trap is reacting to the pain score without addressing the device problem.

2820. Which statement by a client with a newly prescribed PCA pump indicates a need for further teaching?

- A.** I should press the button when I begin to feel pain increasing
- B.** My daughter can push the button for me if I fall asleep
- C.** The pump has safety limits that control how often I can get a dose
- D.** I should tell the nurse if my pain stays high even when I use the pump

Correct Answer: B

Family members should not activate the PCA button. This unsafe practice can lead to oversedation because a sleeping or heavily sedated client cannot appropriately judge pain or the need for medication. The other statements show correct understanding of PCA use.

2821. A postoperative client has both a basal PCA opioid infusion and demand doses available. The client is difficult to arouse, has pinpoint pupils, and a respiratory rate of 7/min. Which action should the nurse take first?

- A.** Encourage the client to cough and deep-breathe every hour
- B.** Stop using the pain scale until the client is more awake
- C.** Follow facility protocol for opioid-induced respiratory depression and get immediate assistance
- D.** Reassure the family that this is expected while strong pain medicine is working

Correct Answer: C

This client is showing classic signs of opioid toxicity with significant respiratory depression. Immediate action is required according to protocol, including urgent assistance and rapid intervention. Teaching, reassurance, and delayed assessment are unsafe. The trap is underestimating sedation because the medication was prescribed.

2822. A client with an epidural analgesia catheter reports sudden severe back pain at the insertion site and new weakness in both legs. Which interpretation is most appropriate?

- A.** This is an expected effect of good pain control
- B.** The client is anxious and should be coached to relax
- C.** This may indicate a serious complication requiring immediate evaluation
- D.** The prescribed medication dose is probably too low

Correct Answer: C

Sudden severe back pain with new bilateral leg weakness is not expected and may indicate a serious complication such as neurologic compromise. Immediate evaluation is needed. The distractors minimize dangerous findings or misattribute them to anxiety or undertreatment.

2823. A nurse is caring for a client receiving epidural analgesia who develops hypotension shortly after a dose adjustment. Which action is the priority?

- A.** Assess the client promptly and support circulation per protocol while notifying the appropriate provider
- B.** Tell the client that low blood pressure is always expected with epidural therapy
- C.** Help the client ambulate to improve blood pressure
- D.** Document the finding and recheck it in 4 hours

Correct Answer: A

Hypotension can occur with epidural analgesia and may require prompt assessment and intervention. The nurse should evaluate the client's status and respond according to protocol rather than delay or encourage ambulation. The trap is normalizing a potentially unstable finding instead of treating it as clinically significant.

2824. A client receiving PCA opioid therapy becomes increasingly sedated over 2 hours. Pain scores have improved, but the client now drifts off mid-sentence. What is the best nursing interpretation?

- A.** Improved pain control means the therapy is working and no action is needed
- B.** Sedation can be an early warning sign of opioid-induced respiratory depression
- C.** The client is finally resting, so assessments can be spaced farther apart
- D.** This confirms the client was exaggerating pain earlier

Correct Answer: B

Increasing sedation is an important early sign of opioid effect becoming excessive and may precede respiratory depression. The nurse should not be reassured by lower pain scores alone. The major trap is assuming sleepiness simply reflects comfort rather than a worsening adverse response.

2825. A nurse is teaching a postoperative client how to use a patient-controlled analgesia (PCA) pump. Which instruction should the nurse include?

- A.** Press the button as soon as pain begins rather than waiting until it becomes severe
- B.** Ask a family member to press the button if you become sleepy
- C.** Turn off the pump if you feel your pain is controlled
- D.** Press the button repeatedly to override the lockout if pain continues

Correct Answer: A

The client should use the PCA at the onset of pain so the medication can control discomfort before it becomes severe. Family members should not activate the PCA, clients should not adjust or turn off the pump on their own, and repeated button pressing does not bypass the lockout interval.

2826. A client returns from surgery with a transdermal fentanyl patch in place for chronic pain management. The client also has a heating pad across the upper chest for comfort. Which action should the nurse take?

- A.** Leave the heating pad in place because transdermal medication is released at a constant rate
- B.** Remove the heating pad because heat can increase opioid absorption from the patch
- C.** Cover the patch with a dry sterile dressing to protect it from the heat
- D.** Massage around the patch every 4 hours to promote even absorption

Correct Answer: B

External heat can increase absorption from a transdermal fentanyl patch and raise the risk of opioid toxicity. The nurse should remove the heat source. A dressing or massage does not make the situation safer, and absorption is not completely unaffected by heat.

2827. A client with a PCA pump says, "My mouth is very dry and I feel itchy." The client is awake, oriented, and has a respiratory rate of 16/min. Which nursing interpretation is most appropriate?

- A.** These findings suggest expected opioid side effects that should be assessed and managed
- B.** These findings indicate the PCA is malfunctioning and must be discontinued
- C.** These findings mean the client is having a life-threatening allergic reaction
- D.** These findings show the client has developed opioid dependence

Correct Answer: A

Dry mouth and itching are common opioid side effects. They should be assessed and managed, but they do not by themselves indicate pump failure, anaphylaxis, or dependence. The key distinction is that the client is alert and breathing adequately.

2828. A nurse is caring for a client receiving epidural analgesia after orthopedic surgery. Which equipment should the nurse ensure is readily available at the bedside?

- A. Incentive spirometer and oral suction catheter
- B. Sequential compression sleeves and a gait belt
- C. Oxygen delivery setup and naloxone
- D. Warming blanket and extra IV tubing

Correct Answer: C

Clients receiving epidural opioid analgesia are at risk for respiratory depression, so emergency support such as oxygen and naloxone should be readily available. The other items may be useful in care, but they are not the priority safety equipment specific to opioid analgesia.

2829. A nurse is assessing a client receiving a continuous epidural analgesia infusion. Which finding is most important to report because it may indicate the medication is affecting the client's motor function?

- A. The client rates incisional pain as 3 on a 0-to-10 scale
- B. The client cannot lift both legs off the bed
- C. The client requests assistance turning in bed
- D. The client reports feeling sleepy after visitors leave

Correct Answer: B

Inability to lift the legs suggests significant motor block or possible neurologic compromise and requires prompt reporting. Mild pain, needing help turning, and feeling sleepy are less specific and less urgent than new bilateral leg weakness.

2830. A nurse notes that a client's PCA pump settings on the medication order are morphine 1 mg demand dose with a 10-minute lockout. The pump is programmed for a 2 mg demand dose with a 10-minute lockout. What should the nurse do first?

- A. Document the discrepancy and continue to monitor the client closely
- B. Reprogram the pump to the prescribed dose using the facility's verification process
- C. Ask the client whether the current setting is controlling the pain
- D. Wait until the next dose is requested before correcting the setting

Correct Answer: B

A discrepancy between the prescribed dose and the programmed PCA setting is a medication safety issue. The nurse should correct the pump setting promptly using the required verification process. Monitoring, asking about pain, or delaying correction does not address the immediate risk of overdose.

2831. A nurse is teaching a client about care of a transdermal pain patch at home. Which client statement shows correct understanding?

- A. I will place the new patch on the exact same spot each time so I can keep track of it
- B. If the patch loosens, I can tape directly over the medication surface
- C. I should fold the used patch with the sticky sides together before disposal
- D. I can cut the patch in half if the prescribed dose feels too strong

Correct Answer: C

Used patches still contain medication, so folding them with the sticky sides together helps reduce accidental exposure before disposal according to policy or instructions. The application site should be rotated, the patch should not be cut, and improvised handling can be unsafe.

2832. A client receiving epidural analgesia reports that pain was well controlled an hour ago but is now suddenly severe. The dressing is dry and the tubing appears intact. Which action is best for the nurse to take next?

- A. Increase the epidural infusion rate independently

- B. Assess the sensory level and catheter site, then notify the provider per protocol
- C. Reassure the client that severe pain is expected after surgery
- D. Place the client flat and stop all IV fluids

Correct Answer: B

A sudden loss of pain control with an epidural requires focused assessment of the block level and catheter site, followed by reporting per protocol because the catheter may have migrated or the analgesia may be inadequate. The nurse should not independently change the infusion rate, dismiss the pain, or take unrelated actions.

2833. A nurse is reviewing a client's medication record before starting PCA opioid therapy. Which prescribed medication should the nurse recognize may increase the risk of excessive sedation when combined with the PCA?

- A. Docusate sodium
- B. Lorazepam
- C. Pantoprazole
- D. Ondansetron

Correct Answer: B

Lorazepam is a benzodiazepine and can have additive CNS depressant effects when combined with opioid PCA therapy, increasing the risk of oversedation and respiratory depression. The other medications do not pose the same degree of sedation risk.

2834. A client is using a PCA pump after surgery and has pressed the button many times during the last hour. The pump history shows only a few doses were delivered. Which explanation by the nurse is most accurate?

- A. The pump is designed to prevent another dose until the lockout time has passed
- B. The medication is delivered only when the nurse confirms each request
- C. The PCA button records pain ratings rather than medication requests
- D. The pump stops responding after several requests to prevent dependence

Correct Answer: A

PCA pumps use a lockout interval to prevent additional doses from being delivered too soon, even if the client presses the button multiple times. Nurse confirmation is not required for each dose, the button is not for pain scoring, and the pump does not stop to prevent dependence.

2835. A nurse is caring for a client receiving PCA opioid therapy. Which outcome best shows that the therapy is effective and safe?

- A. Pain rating decreases, and the client remains alert with adequate respirations
- B. Pain rating decreases, and the client sleeps through all assessments
- C. Pain rating stays the same, but the client requests fewer doses
- D. Pain rating decreases, and the client's blood pressure rises steadily

Correct Answer: A

Effective and safe PCA therapy lowers pain while maintaining adequate respiratory status and appropriate level of consciousness. Sleeping through assessments may indicate oversedation, unchanged pain suggests poor control, and rising blood pressure is not the best indicator of safe analgesia.

2836. A nurse enters a client's room and finds the client with a PCA pump nauseated and vomiting. The client asks, "Can I stop using this now?" Which response is best?

- A. Yes, disconnect the tubing and I will tell the provider later
- B. No, nausea means the medication is working properly
- C. Let me assess you and review the order, because nausea can be an opioid side effect that may need treatment or adjustment
- D. Press the PCA button again so your body gets used to the medicine

Correct Answer: C

Nausea is a possible opioid side effect and should prompt assessment and possible treatment or adjustment. The nurse should not tell the client to disconnect the device, dismiss the symptom as expected and untreatable, or encourage more dosing without assessment.

2837. A client receiving epidural analgesia had a urinary catheter removed 6 hours ago and has not voided. The lower abdomen is distended, and the client reports pressure. Which interpretation is most appropriate?

- A.** This may indicate urinary retention related to the analgesia
- B.** This confirms the client is adequately hydrated
- C.** This is an expected sign that pain control is improving
- D.** This suggests the epidural medication has fully worn off

Correct Answer: A

Urinary retention can occur with epidural analgesia and should be recognized when the client has not voided and has bladder distention or pressure. The finding does not confirm hydration status, indicate improving pain control, or show that the medication has worn off.

2838. A nurse caring for a client with a transdermal analgesic patch notices that the previous patch is still on the client's back when preparing to apply the new patch. Which action is best?

- A.** Apply the new patch on the arm because the old patch is nearly empty
- B.** Remove the old patch before applying the new one and document the removal
- C.** Leave both patches in place for 12 hours to maintain blood levels
- D.** Cover the old patch with the new patch to avoid skin irritation

Correct Answer: B

The old transdermal patch should be removed before a new one is applied to avoid unintentional overdose and to maintain accurate medication administration records. Leaving both patches in place or layering them is unsafe.

2839. A postoperative client is prescribed patient-controlled analgesia (PCA) with IV morphine. The client's spouse says, "She is sleeping, so I'll press the button for her to keep the pain away." Which response by the nurse is best?

- A.** That is helpful because it prevents pain from becoming severe

B. You may press the button only if the client asks you to do it

C. Only the client should press the PCA button because the pump is designed for patient use

D. You should press the button every time the lockout interval ends

Correct Answer: C

The safest teaching is that only the client should press the PCA button. PCA is designed so the awake client can assess pain and self-administer a dose when needed. If someone else presses the button, the client could receive opioid medication while overly sedated or asleep, increasing the risk of respiratory depression. Choice B is a common trap because it sounds reasonable, but PCA by proxy is unsafe even if the client previously requested help.

2840. A postoperative client is receiving IV opioid analgesia by patient-controlled pump. On assessment, the client is difficult to arouse, has slurred speech, and has a respiratory rate of 8/min. Which action should the nurse take first?

- A.** Encourage the client to use the incentive spirometer
- B.** Stop the opioid infusion and assess airway and breathing
- C.** Reassure the client that drowsiness is expected after surgery
- D.** Increase the lockout interval for the next dose

Correct Answer: B

A respiratory rate of 8/min with difficult arousal suggests opioid-induced respiratory depression, which is the priority. The nurse should first stop the opioid delivery and assess/support airway and breathing. Encouraging spirometer use does not address the immediate danger. Reassurance is unsafe because these findings are not a minor expected effect. Changing future pump settings does not address the client's current unstable condition.

2841. A nurse is preparing to apply a scheduled transdermal analgesic patch. Which site should the nurse choose?

- A.** An area with a fresh surgical incision so the medication reaches the bloodstream faster
- B.** A hairless, intact area of skin on the upper arm after removing the old patch

- C. The same skin site used for the last patch to maintain steady absorption
- D. An erythematous area on the chest where the client says the pain is strongest

Correct Answer: B

Transdermal patches should be applied to clean, dry, intact skin, ideally in a relatively hairless area, and the old patch should be removed first. Fresh incisions and irritated skin can alter absorption and increase harm. Reusing the same site repeatedly can irritate the skin, so sites should be rotated rather than immediately reused.

2842. A client receiving epidural opioid analgesia 8 hours after surgery reports increasing numbness in both legs. The nurse finds the client can barely lift either leg off the bed. Pain is controlled, blood pressure is stable, and the dressing is dry. What is the nurse's best action?

- A. Document the finding as an expected effect of pain relief
- B. Assist the client to ambulate to improve circulation
- C. Notify the provider promptly and hold further epidural dosing per protocol
- D. Increase the infusion rate because sensory changes suggest the dose is too low

Correct Answer: C

Progressive bilateral leg weakness during epidural analgesia can indicate excessive motor blockade or another complication and should be reported promptly. Holding further dosing per protocol helps prevent worsening effects while the client is evaluated. Ambulating is unsafe with marked weakness. Increased pain control is not the issue here, and documenting without acting delays needed intervention.

2843. A nurse is reinforcing teaching for a client who will go home using transdermal opioid patches for chronic pain. Which client statement requires further teaching?

- A. I will fold the used patch so the sticky sides are together before disposal.
- B. I should avoid placing the patch under a heating blanket.
- C. If my pain gets worse, I can cut the patch to make the dose stronger more quickly.

- D. I should keep the patches out of reach of children.

Correct Answer: C

Cutting a transdermal opioid patch is unsafe because it can alter medication delivery and increase the risk of overdose. The other statements reflect safe teaching: avoid external heat, dispose of used patches carefully, and keep them away from children. A common trap is thinking the patch can be adjusted like a tablet, but transdermal delivery systems must be used exactly as directed.

2844. A client had a new fentanyl transdermal patch applied 30 minutes ago for chronic cancer pain. The client reports severe pain rated 9/10 and asks for relief now. Which response by the nurse is best?

- A. The patch should provide immediate relief, so the pain rating is probably exaggerated.
- B. Apply an additional patch now because the first patch has not had time to work yet.
- C. Tell the client to wait until the patch reaches full effect before requesting any other medication.
- D. Assess the client and administer the prescribed short-acting breakthrough pain medication if available.

Correct Answer: D

Transdermal fentanyl does not provide rapid immediate pain relief, so a client with severe pain soon after patch application may need prescribed short-acting breakthrough medication while the patch begins providing longer-term control. Applying an extra patch risks overdose. Dismissing the report of pain is inappropriate, and telling the client to wait leaves severe pain untreated.

Pre-administration data review

2845. A nurse is preparing to administer digoxin to a client with heart failure. Which pre-administration finding should cause the nurse to withhold the medication and notify the provider?

- A. Apical pulse of 56/min
- B. Blood pressure of 138/84 mm Hg
- C. Respiratory rate of 18/min

D. Temperature of 37.1°C (98.8°F)

Correct Answer: A

Digoxin can further slow the heart rate, so an apical pulse of 56/min is a key finding that should prompt the nurse to hold the dose and contact the provider. The other findings are not typical reasons to withhold digoxin. A common trap is focusing on blood pressure, but pulse is the more critical pre-administration check here.

2846. A client is prescribed warfarin. Before administering the next dose, which laboratory result is most important for the nurse to review?

- A.** Platelet count
- B.** International normalized ratio (INR)
- C.** White blood cell count
- D.** Serum sodium

Correct Answer: B

INR is the key lab used to evaluate the anticoagulant effect of warfarin and helps determine whether the dose is safe to give. Platelets are important in bleeding risk overall, but INR is the most directly relevant pre-administration value for warfarin. The wrong-answer trap is choosing a generally important lab instead of the one specifically tied to this drug.

2847. The nurse is about to give insulin lispro before breakfast. Which action is the priority pre-administration check?

- A.** Verify that the client will receive the meal tray promptly
- B.** Assess bowel sounds in all four quadrants
- C.** Obtain a urine specimen for ketones
- D.** Place the client in a high-Fowler position

Correct Answer: A

Insulin lispro is rapid-acting, so the nurse should confirm that food is available and the client is ready to eat to reduce the risk of hypoglycemia. The other actions may be appropriate in certain situations, but they are not the priority immediately before giving this medication. A frequent trap is thinking of general diabetic assessments rather than timing related to the medication's onset.

2848. A nurse is reviewing a prescription for furosemide. Which client data should the nurse identify as most important to review before administering the medication?

- A.** Recent potassium level
- B.** Pupil response
- C.** Abdominal girth from last month
- D.** Hearing aid battery status

Correct Answer: A

Furosemide can lower potassium, so a recent potassium level is especially important before administration. This helps the nurse identify risk for dysrhythmias and other complications. The other choices are unrelated or much less urgent for the immediate medication decision.

2849. A client is scheduled to receive metoprolol. Which assessment finding is the highest priority for the nurse to review before giving the medication?

- A.** Pain rating of 4 on a 0-to-10 scale
- B.** Apical pulse of 50/min
- C.** Urine output of 500 mL in 8 hours
- D.** Oxygen saturation of 96% on room air

Correct Answer: B

Metoprolol can decrease heart rate, so an apical pulse of 50/min is the most important finding and may require withholding the medication and notifying the provider. The other findings do not typically require holding a beta blocker. The most likely trap is overlooking the low pulse because the oxygen saturation and urine output appear stable.

2850. A nurse is preparing to administer morphine IV to a postoperative client. Which pre-administration finding requires the most immediate action?

- A.** Respiratory rate of 10/min
- B.** Pain rating of 8 on a 0-to-10 scale
- C.** Incision dressing with a small amount of drainage
- D.** Heart rate of 88/min

Correct Answer: A

Morphine can further depress respirations, so a respiratory rate of 10/min requires the nurse to hold the medication and act on that finding first. Severe pain supports the need for analgesia but does not outweigh the immediate safety concern. The trap is focusing on the pain score without first checking whether the opioid can be given safely.

2851. A client has a new prescription for potassium chloride by mouth. Which pre-administration data should the nurse review first?

- A. Serum potassium level and urine output
- B. Pupillary reaction and hand grasps
- C. Hemoglobin and hematocrit
- D. Bowel movement frequency over the past month

Correct Answer: A

Before giving potassium, the nurse should check the current potassium level and make sure the client has adequate urine output. Poor kidney function or elevated potassium can make administration unsafe. The other data may matter in a broad assessment, but they are not the key pre-administration review for this drug.

2852. A nurse is reviewing the MAR before administering an antibiotic and notices that the client has a documented allergy to the same medication class. What is the nurse's best action?

- A. Administer the medication because the provider prescribed it
- B. Ask the client whether the allergy was mild and then give the dose
- C. Hold the medication and clarify the prescription before administration
- D. Give half the dose first to test the client's response

Correct Answer: C

A documented allergy to the medication class requires the nurse to stop and clarify the prescription before administration. The nurse should not override the allergy based only on the provider order or the client's vague description. Giving a reduced test dose is unsafe and inappropriate outside a specific order and controlled setting.

2853. A client is prescribed enoxaparin. Which pre-administration assessment is most important for the nurse to review?

- A. Presence of active bleeding or unusual bruising
- B. Pupillary equality
- C. Calf circumference from 2 weeks ago
- D. Ability to swallow tablets

Correct Answer: A

Enoxaparin is an anticoagulant, so the nurse should review for signs of bleeding or bruising before administering it. The other findings are less directly tied to whether the next dose is safe. The trap is thinking about the reason the drug is given, such as thrombosis prevention, rather than the immediate risk of bleeding.

2854. A nurse is preparing to administer the first dose of an antihypertensive medication. Which additional data are most important to collect first?

- A. Current blood pressure
- B. Last bowel movement
- C. Diet preference
- D. Capillary refill in the toes

Correct Answer: A

The current blood pressure is the most important data to review before giving an antihypertensive because the medication can further lower it. The other findings do not directly determine whether the medication is safe to administer now. The likely trap is choosing a general assessment item instead of the medication-specific parameter.

2855. A nurse is preparing to administer gentamicin IV. Which finding from the chart should the nurse identify as most important to report before giving the dose?

- A. Serum creatinine increased from baseline
- B. Heart rate 92/min
- C. Client reports mild hunger
- D. Oral temperature 37°C (98.6°F)

Correct Answer: A

Gentamicin can be nephrotoxic, so an increase in serum creatinine is a significant pre-administration concern that should be reported before the dose is given. The other findings are not as immediately relevant to this medication's safety. A common trap is overlooking renal data when no dramatic symptoms are present.

2856. A client is scheduled to receive the next dose of levothyroxine. During medication reconciliation, the nurse notes that the client has been taking an over-the-counter calcium supplement at home. Which action is best before administration teaching and dosing?

- A. Ignore the supplement because it is not prescribed
- B. Document the supplement because it may affect medication timing and absorption
- C. Tell the client to stop all over-the-counter products immediately
- D. Administer both medications together for convenience

Correct Answer: B

Over-the-counter products are part of medication reconciliation and may affect how prescribed medications are administered. Calcium can interfere with absorption of some medications, so the supplement should be documented and considered in timing and teaching. The trap is dismissing nonprescription products as unimportant, when they can significantly affect therapy.

2857. A nurse is preparing to administer regular insulin to a client who has been NPO since midnight for a procedure that has been delayed twice. The client's blood glucose is 84 mg/dL, and the client reports feeling shaky. What is the nurse's best action?

- A. Administer the insulin because the medication was scheduled
- B. Hold the insulin and further assess the client, then notify the provider
- C. Give the insulin and recheck glucose at the end of the shift
- D. Ask another nurse to administer the insulin

Correct Answer: B

This client is NPO, has a low-normal glucose value, and reports symptoms consistent with possible hypoglycemia. The safest action is to hold the insulin, assess further, and notify the provider for guidance. The trap is treating a scheduled insulin dose as automatic without integrating nutrition status and current symptoms.

2858. The nurse is about to administer the next dose of lithium. Which pre-administration finding is most important to address first?

- A. The client has had vomiting and diarrhea for 24 hours
- B. The client slept 6 hours last night
- C. The client reports dry skin
- D. The client ate 75% of breakfast

Correct Answer: A

Vomiting and diarrhea can cause fluid and sodium imbalance, which can raise lithium levels and increase the risk of toxicity. That finding should be addressed before administering the next dose. The other choices are less urgent. The trap is missing the importance of dehydration-related risk in a client receiving lithium.

2859. A nurse is reviewing pre-administration data for a client who has a prescription for oxybutynin. Which client condition is most important for the nurse to clarify before giving the medication?

- A. A history of urinary retention
- B. A preference for taking pills with applesauce
- C. Mild tenderness at an old IV site
- D. A family history of hypertension

Correct Answer: A

Oxybutynin has anticholinergic effects and can worsen urinary retention, so a history of urinary retention is the most important issue to clarify before administration. The other findings do not directly affect whether the medication is safe to give. The trap is overlooking a contraindicating history because the client is prescribed a urinary medication.

2860. A client is prescribed spironolactone 25 mg PO daily. Which pre-administration finding should the nurse report before giving the medication?

- A. Potassium level of 5.6 mEq/L
- B. Blood pressure of 138/84 mm Hg
- C. Heart rate of 96/min
- D. Urine output of 900 mL in 24 hr

Correct Answer: A

Spironolactone is a potassium-sparing diuretic, so an elevated potassium level is the key finding to report before administration because the medication can worsen hyperkalemia. The blood pressure and heart rate are not immediately concerning here, and the urine output does not suggest a reason to hold the drug.

2861. A nurse is preparing to administer the morning dose of epoetin alfa to a client with chronic kidney disease. Which data should the nurse review first?

- A. Blood pressure
- B. Pupil response
- C. Bowel sounds
- D. Capillary refill

Correct Answer: A

Epoetin alfa can increase blood pressure, so reviewing the client's blood pressure before administration is the priority. The other assessments are not the most directly relevant pre-administration data for this medication.

2862. A client is due for phenytoin. The nurse reviews the chart and notes a total phenytoin level above the expected therapeutic range. What is the nurse's best action?

- A. Administer the dose with food
- B. Give half of the prescribed dose
- C. Withhold the medication and notify the provider
- D. Recheck the client's temperature after administration

Correct Answer: C

A phenytoin level above the expected therapeutic range increases the risk for toxicity, so the nurse should withhold the medication and notify the provider. The nurse should not independently reduce the dose. Giving it with food or checking temperature does not address the safety issue.

2863. Before administering the next dose of acetaminophen to an adult client, which information is most important for the nurse to review?

- A. The client's usual bedtime
- B. The total amount of acetaminophen received in the last 24 hours
- C. Whether the client prefers tablets or liquid medication
- D. The client's calcium intake for the day

Correct Answer: B

Acetaminophen may be included in multiple combination products, so the nurse should review the total amount given in the last 24 hours to reduce the risk of overdose and liver injury. Bedtime preference, formulation preference, and calcium intake are not the priority pre-administration checks.

2864. A nurse is preparing to administer heparin subcutaneously to a client. Which laboratory result is the priority to review before giving the medication?

- A. Platelet count
- B. Sodium level
- C. Albumin level
- D. White blood cell count

Correct Answer: A

Heparin can contribute to bleeding risk and is associated with heparin-induced thrombocytopenia, so the platelet count is the priority lab to review before administration. Sodium, albumin, and white blood cell count are not the most directly relevant safety data for this dose.

2865. A client with asthma is prescribed an albuterol inhaler every 4 hours as needed. Before administering a dose for shortness of breath, which finding requires the nurse to reassess the plan and notify the provider?

- A. The client reports mild hand tremors after prior doses
- B. The client used the inhaler 6 times in the last 12 hours
- C. The client's respiratory rate is 24/min
- D. The client is sitting upright to breathe more easily

Correct Answer: B

Frequent use far beyond the prescribed interval suggests poor control and possible worsening respiratory status, so the nurse should reassess and notify the provider rather than simply giving another routine dose. Mild tremors are a common effect of albuterol, and tachypnea or upright positioning alone do not answer the medication-safety question as clearly as overuse does.

2866. A nurse is preparing to administer oral bisacodyl to a client who reports severe cramping abdominal pain and nausea. Which action is best?

- A. Administer the medication with juice
- B. Hold the medication and further assess the client
- C. Give the medication and reassess in 30 minutes
- D. Ask the client to ambulate before taking the medication

Correct Answer: B

Severe abdominal pain and nausea can indicate a condition in which a stimulant laxative may be inappropriate, so the nurse should hold the medication and further assess the client. Giving the laxative could worsen the problem. Juice and ambulation do not address the need to evaluate the symptoms first.

2867. A client is scheduled to receive the next dose of metformin. Which newly reviewed data should the nurse question before administration?

- A. The client ate 75% of breakfast
- B. The client walked in the hall this morning

C. The client had iodinated contrast for a CT scan earlier today

D. The client's fasting glucose yesterday was 168 mg/dL

Correct Answer: C

Metformin should be questioned when the client has recently received iodinated contrast because of the risk of kidney-related complications and drug accumulation. Eating breakfast and ambulating are not concerns here, and an elevated prior glucose value does not by itself make the dose unsafe.

2868. The nurse is preparing to administer the first dose of allopurinol to a client with gout. Which charted information is most important to review before giving the medication?

- A. History of chronic kidney disease
- B. Preference for a low-purine diet
- C. Reports of joint pain last week
- D. Current use of reading glasses

Correct Answer: A

Kidney function is important to review before allopurinol administration because impaired renal function can affect medication handling and may require closer evaluation. Dietary preference, prior joint pain, and use of glasses are not the key pre-administration concerns.

2869. A nurse is preparing to administer an oral opioid to a client with chronic pain. Which newly documented medication should prompt the nurse to clarify the plan before giving the opioid?

- A. Docusate
- B. Lorazepam
- C. Omeprazole
- D. Multivitamin

Correct Answer: B

Lorazepam is a benzodiazepine, and concurrent use with an opioid can increase the risk of excessive sedation and respiratory depression. That makes clarification important before administration. Docusate, omeprazole, and a multivitamin are not the same level of concern in this setting.

2870. A client has a prescription for vancomycin IV. During pre-administration review, which finding is most important for the nurse to address before giving the dose?

- A. The client reports drinking less water today
- B. The most recent creatinine level has increased from baseline
- C. The client says the IV site feels cool
- D. The client's breakfast tray is still at the bedside

Correct Answer: B

An increase in creatinine suggests worsening kidney function, which is especially important to address before administering vancomycin because the drug can affect and be affected by renal function. Low fluid intake is less specific, a cool IV site may need local assessment but does not replace the need to address the renal safety issue, and the breakfast tray is irrelevant.

2871. The nurse is reviewing the MAR before administering prednisone to a client. Which data are most important to obtain first if not already documented?

- A. Current blood glucose
- B. Visual acuity
- C. Pedal pulse strength
- D. Handgrip strength

Correct Answer: A

Prednisone can raise blood glucose, so checking the current blood glucose is the most important of these options before administration, especially when monitoring for medication effects and safety. Visual acuity, pedal pulses, and handgrip strength are not the priority checks for this dose.

2872. A client is scheduled to receive the evening dose of carbamazepine. Which pre-administration finding should the nurse report immediately?

- A. Sore throat and oral ulcers
- B. Mild nausea after lunch
- C. Complaint of dry mouth
- D. Difficulty falling asleep

Correct Answer: A

Sore throat and oral ulcers can signal a serious blood dyscrasia or infection risk associated with carbamazepine, so this finding requires immediate reporting before administration. Mild nausea, dry mouth, and insomnia are less urgent and do not suggest the same level of danger.

2873. A nurse is preparing to administer a scheduled dose of ibuprofen to a client. Which assessment finding is most important to review first?

- A. Black, tarry stools reported this morning
- B. Pain rated 7 on a 0 to 10 scale
- C. Temperature of 99.1°F (37.3°C)
- D. Client request to take the medication after lunch

Correct Answer: A

Black, tarry stools may indicate gastrointestinal bleeding, which is a major concern with ibuprofen and other NSAIDs. The nurse should address that finding before giving the medication. Pain, a low-grade temperature, and timing preference do not outweigh a possible bleeding complication.

2874. A nurse is about to administer the next dose of theophylline. Which client report should the nurse recognize as most concerning during the pre-administration review?

- A. Increased appetite
- B. Occasional dry lips
- C. Palpitations and restlessness
- D. Mild nasal congestion

Correct Answer: C

Palpitations and restlessness are concerning for possible theophylline toxicity, so the nurse should recognize this as the most important finding before administering the next dose. The other reports are nonspecific and do not point as directly to a serious medication-related problem.

2875. A nurse is preparing to give digoxin 0.125 mg PO to an older adult with heart failure. Which pre-administration finding should cause the nurse to hold the medication and notify the provider?

- A. Apical pulse of 54/min
- B. Blood pressure of 138/76 mm Hg

- C. Potassium level of 4.1 mEq/L
- D. Report of mild ankle swelling at the end of the day

Correct Answer: A

Digoxin can slow the heart rate, so an apical pulse of 54/min is a key finding to hold the dose and notify the provider. The blood pressure is acceptable, the potassium level is within expected range, and mild ankle swelling may reflect the underlying heart failure but is not the immediate reason to withhold this dose. A common trap is focusing on edema because the client has heart failure, but the pulse is the more urgent pre-administration check for digoxin.

2876. A nurse reviews the chart before administering warfarin to a client. Which newly resulted laboratory value is the priority reason to question the dose?

- A. INR 3.9
- B. Hemoglobin 12.8 g/dL
- C. Platelet count 220,000/mm³
- D. White blood cell count 8,400/mm³

Correct Answer: A

Warfarin therapy requires review of coagulation results, especially the INR. An INR of 3.9 is elevated and increases bleeding risk, so the nurse should question the dose and notify the provider. The other values are not the key indicators for deciding whether to give warfarin. A likely trap is choosing the platelet count because platelets matter in bleeding risk, but INR is the more direct pre-administration value for warfarin.

2877. A client with hypertension is scheduled to receive lisinopril 10 mg PO. During pre-administration review, which finding is most important for the nurse to address first?

- A. Potassium level of 5.8 mEq/L
- B. Heart rate of 88/min
- C. Blood pressure of 146/84 mm Hg
- D. Report of a mild headache this morning

Correct Answer: A

Lisinopril can increase potassium levels, so a potassium of 5.8 mEq/L is the priority finding to address before administration because it raises the risk of dangerous dysrhythmias. The heart rate and blood pressure do not by themselves require holding the dose, and a mild headache is nonspecific. The trap is focusing on the elevated blood pressure and assuming the antihypertensive should definitely be given, but the critical lab value takes priority.

2878. A nurse is preparing to administer the next scheduled dose of furosemide to a client with heart failure. Which pre-administration data should the nurse identify as the priority reason to hold the medication and notify the provider?

- A. Serum potassium level of 2.9 mEq/L
- B. Blood pressure of 138/84 mm Hg
- C. Urine output of 900 mL over the last 12 hours
- D. Mild thirst reported this morning

Correct Answer: A

Furosemide is a loop diuretic that can further lower potassium. A potassium level of 2.9 mEq/L is significantly low and increases the risk of dysrhythmias, so the nurse should hold the medication and notify the provider. A blood pressure of 138/84 mm Hg and urine output of 900 mL in 12 hours do not by themselves indicate the dose should be withheld. Mild thirst may occur with diuresis but is not the priority finding compared with marked hypokalemia.

Therapeutic response monitoring

2879. A client with type 2 diabetes started a new rapid-acting insulin regimen before meals. Which finding best shows the medication is having the intended therapeutic effect?

- A. The client reports increased thirst at bedtime
- B. The client's post-meal blood glucose is within the target range
- C. The client has mild redness at the injection site
- D. The client requests a snack 2 hours after lunch

Correct Answer: B

The intended effect of rapid-acting insulin is improved control of blood glucose after meals, so a post-meal glucose within the target range is the best evidence of therapeutic response. Increased thirst suggests hyperglycemia, redness at the site is not the desired effect, and asking for a snack does not confirm glucose control.

2880. A client is receiving furosemide for fluid overload related to heart failure. Which assessment finding most directly indicates the medication is effective?

- A. Weight decreases by 1.4 kg in 24 hours
- B. The client states the tablet tastes bitter
- C. Heart rate increases from 82/min to 104/min
- D. Urine appears dark yellow in the drainage bag

Correct Answer: A

Furosemide is given to remove excess fluid. A significant drop in daily weight is one of the clearest signs that fluid volume is decreasing. A bitter taste is unrelated to therapeutic response, tachycardia may suggest a problem, and darker urine alone does not show improved fluid status.

2881. A client with bacterial pneumonia has been receiving an antibiotic for 48 hours. Which finding best indicates a therapeutic response to the medication?

- A. White blood cell count is trending downward toward normal
- B. The client says the IV site is tender
- C. The client reports one episode of loose stool
- D. Blood pressure decreases from 128/78 mm Hg to 110/68 mm Hg

Correct Answer: A

For a bacterial infection, improvement is shown by reduced signs of infection, such as a white blood cell count moving toward normal. IV site tenderness is a local issue, loose stool may be an adverse effect, and the blood pressure change does not directly show antibiotic effectiveness.

2882. A client with hypertension started lisinopril 2 weeks ago. Which data should the nurse compare most directly with baseline to determine whether the medication is working?

- A. Apical pulse and bowel sounds

- B. Blood pressure readings over time
- C. Respiratory rate and oxygen saturation
- D. Pupil size and grip strength

Correct Answer: B

Lisinopril is given to lower blood pressure, so serial blood pressure readings compared with baseline are the most direct way to judge therapeutic effect. The other assessment data may be useful in some situations but do not best measure response to antihypertensive therapy.

2883. A client is receiving warfarin for prevention of thromboembolic events. Which laboratory value is most useful for monitoring the therapeutic response to this medication?

- A. International normalized ratio (INR)
- B. Fasting blood glucose
- C. Serum creatinine
- D. Hemoglobin A1c

Correct Answer: A

Warfarin therapy is monitored primarily with the INR because it reflects the drug's anticoagulant effect. Serum creatinine may matter for overall status, but it does not measure warfarin response. Fasting glucose and A1c relate to diabetes management, not anticoagulation.

2884. A client receiving digoxin for heart failure has nausea and poor appetite. Which finding would best indicate that the medication is providing the intended therapeutic benefit despite these concerns?

- A. Urine output has increased and crackles have decreased
- B. The client reports blurred yellow vision
- C. Serum potassium is 3.0 mEq/L
- D. The pulse is irregular at 54/min

Correct Answer: A

Digoxin is used to improve cardiac function, so increased urine output and fewer crackles suggest better perfusion and less fluid congestion. The other findings are concerning for toxicity or risk: blurred yellow vision, low potassium, and bradycardia or irregular rhythm are not signs of desired therapeutic response.

2885. A client with asthma uses a prescribed bronchodilator inhaler during an episode of wheezing. Which assessment finding best shows the medication has been effective?

- A. The client's cough becomes more frequent
- B. Breath sounds change from diffuse wheezes to clearer airflow
- C. The client reports a mild bitter taste
- D. Respiratory rate increases from 20/min to 30/min

Correct Answer: B

Bronchodilators are intended to open the airways. Improved airflow with fewer wheezes is the best sign the medication worked. A bitter taste may occur but does not show effectiveness, and a higher respiratory rate or more coughing does not clearly indicate improved bronchospasm.

2886. A client is receiving a proton pump inhibitor for gastroesophageal reflux disease. Which statement by the client best indicates a therapeutic response?

- A. I still get burning pain after nearly every meal.
- B. My stools are darker than usual.
- C. I have much less heartburn when I lie down at night.
- D. I feel sleepy about an hour after taking it.

Correct Answer: C

The intended effect of a proton pump inhibitor is reduction of gastric acid symptoms, so decreased heartburn is the best sign of benefit. Ongoing burning means poor control, dark stools may suggest bleeding, and sleepiness is not the therapeutic target.

2887. A client with hypothyroidism started levothyroxine 6 weeks ago. Which finding best suggests the medication is effective?

- A. The client reports improved energy and less sensitivity to cold
- B. The client reports a new fine tremor and palpitations
- C. The client has gained 2.3 kg since the last visit
- D. The client says constipation has worsened

Correct Answer: A

Levothyroxine replaces thyroid hormone, so improved energy and reduced cold intolerance suggest the client is moving toward euthyroid status. Tremor and palpitations may indicate overreplacement, while weight gain and worsening constipation suggest ongoing hypothyroid symptoms rather than therapeutic improvement.

2888. A nurse is evaluating whether a PRN antiemetic administered 30 minutes ago was effective. Which finding is the best indicator of a therapeutic response?

- A. The client rates nausea as 2 on a 0-to-10 scale after previously rating it 8
- B. The client falls asleep shortly after receiving the medication
- C. The client requests water because of a dry mouth
- D. The client reports that the IV infusion is uncomfortable

Correct Answer: A

The most direct way to evaluate an antiemetic is to reassess nausea and vomiting after administration. A drop from 8 to 2 shows meaningful symptom relief. Sleepiness and dry mouth may be side effects, and IV discomfort is unrelated to the desired antiemetic effect.

2889. A client with seizure disorder has been taking a newly prescribed anticonvulsant for 3 months. Which information best demonstrates a therapeutic response?

- A. The client reports mild gingival tenderness after brushing teeth
- B. The client has had no seizure activity since starting the medication
- C. The client feels sleepy for 1 hour after each dose
- D. The client has increased appetite in the evening

Correct Answer: B

For anticonvulsant therapy, the desired outcome is reduced or eliminated seizure activity. No seizures since starting the medication is the clearest sign of effectiveness. The other findings may be side effects or unrelated changes, not direct evidence of therapeutic success.

2890. A client hospitalized with acute decompensated heart failure is receiving IV diuretic therapy. Which combination of findings best indicates a therapeutic response?

- A. Increasing peripheral edema and rising blood pressure
- B. Reduced dyspnea, decreased crackles, and increased urine output
- C. Dry mucous membranes, weak pulse, and confusion
- D. Persistent orthopnea and a 2-kg weight gain

Correct Answer: B

In acute heart failure, effective diuresis should reduce pulmonary and systemic congestion. Less shortness of breath, fewer crackles, and more urine output together indicate improvement. The other options suggest worsening fluid overload or possible excessive fluid loss with poor perfusion rather than an uncomplicated therapeutic response.

2891. A client receiving IV unfractionated heparin for a deep vein thrombosis has the following data: aPTT within the prescribed therapeutic range, no new calf pain, and no shortness of breath. What is the best interpretation?

- A. The medication is showing the intended anticoagulant effect
- B. The medication should be stopped because the aPTT is elevated above normal
- C. The client is experiencing heparin-induced thrombocytopenia
- D. The client needs vitamin K to reverse the medication

Correct Answer: A

For a client on IV heparin, an aPTT in the prescribed therapeutic range together with no signs of clot extension or embolization indicates the desired response. Option B mistakes therapeutic anticoagulation for a problem. Option C requires additional evidence such as platelet decline and thrombosis. Vitamin K reverses warfarin, not heparin.

2892. A client with tuberculosis has been taking prescribed medication for several weeks. Which finding is the strongest evidence of therapeutic response rather than merely temporary symptom relief?

- A. The client says coughing is less bothersome after taking a cough suppressant
- B. The client is afebrile, gaining weight, and reports improved appetite
- C. The client sleeps longer after the evening dose
- D. The client drinks more fluids during the day

Correct Answer: B

A therapeutic response to tuberculosis treatment is suggested by overall clinical improvement, such as absence of fever, weight gain, and better appetite. A cough suppressant can reduce symptoms without treating the infection itself. Sleeping longer and drinking more fluids do not specifically show that the antimicrobial regimen is working.

2893. A postoperative client is using a patient-controlled analgesia (PCA) pump with morphine. Which finding best indicates the nurse should conclude the current therapy is effective?

- A. Pain rating decreases from 9 to 4, and the client is able to cough and deep-breathe
- B. Pain rating remains 8, but the client is too drowsy to use the incentive spirometer
- C. Respiratory rate falls from 18/min to 8/min, and the client is difficult to arouse
- D. The client says, 'I do not hurt when I am asleep,' but grimaces with repositioning

Correct Answer: A

Effective PCA therapy should improve pain enough to support recovery activities such as coughing and deep breathing, not just lower the number slightly. Option A shows both symptom relief and improved function. The other options indicate inadequate pain control, excessive sedation, or respiratory depression rather than a safe therapeutic response.

2894. A client with angina pectoris receives sublingual nitroglycerin for chest pain rated 7/10. Which finding best indicates the medication has had the intended therapeutic effect?

- A. The client's blood pressure decreases from 148/86 mm Hg to 118/72 mm Hg
- B. The client reports chest pain has decreased to 1/10 within 5 minutes
- C. The client develops a mild throbbing headache
- D. The client's heart rate increases from 76/min to 92/min

Correct Answer: B

Nitroglycerin is given to relieve anginal chest pain, so a rapid decrease in chest pain is the clearest sign of therapeutic response. A lower blood pressure and headache can occur with nitroglycerin, but those findings reflect effects or side effects, not the main intended outcome. An increased heart rate may occur as a compensatory response and does not show symptom relief.

2895. A client is receiving oral ferrous sulfate for iron-deficiency anemia. Which assessment finding best indicates the medication is effective?

- A. The client reports having darker stools
- B. The client states that food tastes metallic
- C. The client's hemoglobin level increases from baseline
- D. The client reports mild constipation

Correct Answer: C

An increase in hemoglobin from baseline shows improvement in anemia and is the best indicator that iron therapy is working. Dark stools and constipation are common effects of iron therapy, but they do not show correction of anemia. A metallic taste may occur with some medications and is not evidence of therapeutic response here.

2896. A client with open-angle glaucoma has been using prescribed timolol eye drops. Which result best indicates a therapeutic response to the medication?

- A. Decreased intraocular pressure on follow-up examination
- B. Pupils that are equal and round

- C. Absence of eye drainage after administration
- D. Improved hearing bilaterally

Correct Answer: A

Timolol eye drops are used to lower intraocular pressure in glaucoma. A decrease in intraocular pressure is the most direct evidence that the medication is effective. Equal pupils and lack of drainage are not the main therapeutic target, and hearing is unrelated.

2897. A client is taking docusate sodium after abdominal surgery. Which statement best indicates the medication is having the intended effect?

- A. I had a soft bowel movement without straining.
- B. My incision hurts less when I cough.
- C. I am urinating more often now.
- D. I do not feel sleepy after taking it.

Correct Answer: A

Docusate is a stool softener intended to make stools easier to pass and reduce straining. A soft bowel movement without straining best reflects therapeutic response. Less incision pain, increased urination, and lack of drowsiness do not measure the intended effect of this medication.

2898. A client with Parkinson disease has been taking carbidopa-levodopa for several weeks. Which finding best shows the medication is effective?

- A. The client has fewer episodes of urinary urgency
- B. The client demonstrates smoother walking with less rigidity
- C. The client's blood pressure remains stable when standing
- D. The client reports sleeping longer at night

Correct Answer: B

Carbidopa-levodopa is used to improve motor symptoms of Parkinson disease, especially rigidity, bradykinesia, and gait difficulty. Improved ambulation with less rigidity best shows therapeutic response. The other options may be helpful clinical findings, but they are not the primary outcomes used to judge this medication's effectiveness.

2899. A client is receiving lactulose for hepatic encephalopathy. Which finding best indicates the medication is effective?

- A. The client has clearer mentation and is more oriented
- B. The client reports a sweeter taste in the mouth
- C. The client's urine output increases
- D. The client has a lower blood pressure after each dose

Correct Answer: A

Lactulose is used in hepatic encephalopathy to reduce ammonia-related neurologic symptoms. Improvement in mental status and orientation is the best evidence of therapeutic response. The medication may also increase stool output, but the key nursing outcome is improved cognition rather than changes in urine output, taste, or blood pressure.

2900. A client with myasthenia gravis receives pyridostigmine at 0800. Which assessment at 0930 best indicates the medication is achieving the desired effect?

- A. The client reports less drooping of the eyelids and can chew breakfast without fatigue
- B. The client has moist skin and increased salivation
- C. The client has a heart rate of 54/min
- D. The client reports abdominal cramping and diarrhea

Correct Answer: A

Pyridostigmine is intended to improve neuromuscular transmission, so improved muscle strength such as less ptosis and better ability to chew indicates effectiveness. Increased salivation, bradycardia, abdominal cramping, and diarrhea are more consistent with cholinergic side effects or excessive medication effect, not the desired therapeutic response.

2901. A client is taking allopurinol for chronic gout prevention. Which finding best indicates the medication is effective over time?

- A. The client reports immediate relief of pain during an acute flare
- B. The serum uric acid level decreases and gout attacks become less frequent
- C. The client can tolerate spicy foods without heartburn

D. The erythrocyte sedimentation rate rises slightly from baseline

Correct Answer: B

Allopurinol is used to lower uric acid levels and help prevent future gout attacks; it is not mainly used for immediate relief during an acute flare. Therefore, decreased serum uric acid with fewer gout episodes is the best evidence of therapeutic response. The other findings are unrelated or misleading.

2902. A client is receiving epoetin alfa for anemia associated with chronic kidney disease. Which finding is the best indicator that the treatment is effective?

- A. The client reports less fatigue with rising hematocrit values
- B. The client has darker urine after each injection
- C. The client reports less nausea before breakfast
- D. The client develops mild tenderness at the injection site

Correct Answer: A

Epoetin alfa stimulates red blood cell production, so improving hematocrit along with reduced fatigue best shows therapeutic response. Injection-site tenderness is a local effect, not proof of benefit. Darker urine and reduced nausea do not directly evaluate correction of anemia.

2903. A client with benign prostatic hyperplasia begins tamsulosin therapy. Which client report best indicates the medication is effective?

- A. I no longer wake up to urinate at all.
- B. My urinary stream is stronger, and I empty my bladder more easily.
- C. My blood pressure is lower after every dose.
- D. I have less back stiffness in the morning.

Correct Answer: B

Tamsulosin helps relax smooth muscle in the prostate and bladder neck, improving urinary flow and reducing obstruction symptoms. A stronger stream and easier bladder emptying best show therapeutic response. Complete elimination of nighttime urination is not required to show benefit. Lower blood pressure can occur with some alpha-adrenergic effects but is not the primary therapeutic goal here.

2904. A client receiving IV morphine after surgery was rated as having pain 8/10 before administration. Fifteen minutes later, which additional finding best supports that the medication is effective while remaining safe?

- A.** Pain is 3/10, respiratory rate is 14/min, and the client can deep-breathe on request
- B.** Pain is 0/10, respiratory rate is 8/min, and the client is difficult to arouse
- C.** Pain is 6/10, blood pressure is unchanged, and the client is watching television
- D.** Pain is 4/10, oxygen saturation is 89%, and the client is snoring loudly

Correct Answer: A

Effective opioid therapy should reduce pain while maintaining adequate respiratory status and arousability. Option A shows meaningful pain relief with a safe respiratory rate and the ability to participate in deep breathing. Options B and D suggest oversedation or respiratory depression. Option C shows only limited pain improvement and does not best demonstrate adequate therapeutic response.

2905. A client with heart failure is started on spironolactone. Which trend best indicates the medication is producing the intended therapeutic response?

- A.** Weight decreases by 1.5 kg over 3 days, ankle edema lessens, and breath sounds are clearer at the bases
- B.** Serum potassium rises from 4.2 mEq/L to 5.8 mEq/L, and the client reports less thirst
- C.** Blood pressure increases slightly, and the client reports a better appetite
- D.** Urine becomes darker yellow, and bowel sounds increase in all quadrants

Correct Answer: A

Spironolactone is used in heart failure to help reduce fluid overload. Decreasing weight, reduced edema, and clearer lung sounds are strong indicators that fluid status is improving. A rise in potassium to 5.8 mEq/L suggests a potential adverse effect, not therapeutic success. The other findings are not the best measures of intended response.

2906. A client with a serious gram-positive infection is receiving IV vancomycin. Which set of data best indicates the current therapy is effective?

- A.** Trough level is within the prescribed range, temperature is decreasing, and white blood cell count is trending toward normal
- B.** Peak expiratory flow rate improves and the client coughs less for 1 hour after each infusion
- C.** The client reports no pain at the IV site and urine output increases sharply after each dose
- D.** The client becomes drowsy after infusion and sleeps through the night

Correct Answer: A

For vancomycin, therapeutic monitoring includes drug levels as prescribed plus clinical evidence that the infection is improving, such as falling temperature and WBC count moving toward normal. The other options include unrelated findings or effects that do not demonstrate antimicrobial effectiveness.

2907. A client with bipolar disorder started lithium 10 days ago. Which finding is most important for the nurse to use in determining whether the medication is having a therapeutic effect?

- A.** The client reports mild thirst and increased urination
- B.** The client sleeps 6 to 7 hours nightly, speech is less pressured, and impulsive spending has stopped
- C.** The client has a fine hand tremor before meals
- D.** The client states that nausea is worse when taking the medication without food

Correct Answer: B

Lithium's therapeutic effect is evaluated by improvement in manic symptoms such as decreased pressured speech, improved sleep, and reduced impulsive behavior. Thirst, polyuria, tremor, and nausea may occur with lithium but do not demonstrate mood stabilization. The question asks for therapeutic response, not adverse effects.

2908. A client with *Clostridioides difficile*-associated diarrhea is prescribed oral vancomycin. Which finding best indicates a therapeutic response to the medication?

- A.** The client has fewer loose stools, less abdominal cramping, and improving hydration status
- B.** The client reports that the medication is easy to swallow
- C.** The client's appetite increases after one dose
- D.** The client's blood pressure drops slightly while resting in bed

Correct Answer: A

For *C. difficile*-associated diarrhea, the intended therapeutic response is improvement in diarrhea-related symptoms and fluid status. Fewer loose stools, less cramping, and better hydration directly show that the treatment is working. Ease of swallowing, a single appetite change, or a slight blood pressure drop do not best evaluate effectiveness.

2909. A client with type 2 diabetes begins metformin therapy 6 weeks ago. Which finding best indicates the medication is having the intended therapeutic effect?

- A.** Fasting blood glucose readings have decreased from 210 mg/dL to 128 mg/dL
- B.** The client reports mild nausea after the morning dose
- C.** The urine specific gravity is 1.030
- D.** The client states a preference for taking the medication with juice

Correct Answer: A

Metformin is intended to improve glycemic control, so a clear decrease in fasting blood glucose best shows a therapeutic response. Mild nausea is a possible adverse effect, not evidence of effectiveness. Urine specific gravity does not directly show blood glucose control, and beverage preference does not indicate response to therapy.

2910. A client with hypothyroidism started levothyroxine 1 month ago. Which assessment finding best indicates the medication is effective?

- A.** The client reports sleeping 12 hours each night
- B.** The client's heart rate has increased from 54/min to 68/min and energy level has improved
- C.** The client has developed a fine hand tremor
- D.** The client's blood pressure has decreased from 138/84 mm Hg to 100/60 mm Hg

Correct Answer: B

Levothyroxine replaces deficient thyroid hormone, so improvement in hypothyroid symptoms such as fatigue and bradycardia supports effectiveness. A heart rate moving into a more normal range with improved energy is the best indicator here. Sleeping excessively suggests ongoing hypothyroidism. Fine tremor may suggest overreplacement. A marked blood pressure drop is not the expected therapeutic target and may signal another issue.

2911. A client with a seizure disorder has been receiving phenytoin for 3 months. Which information is most important for the nurse to use when determining whether the medication is effective?

- A.** The client reports no seizure activity since starting the medication
- B.** The client has mild gingival swelling
- C.** The serum potassium level is 4.1 mEq/L
- D.** The client takes the medication at the same time every day

Correct Answer: A

The intended therapeutic effect of phenytoin is seizure control, so the absence of seizure activity is the most direct evidence that the medication is effective. Gingival swelling is an adverse effect, not a therapeutic response. Potassium is not the key indicator of phenytoin effectiveness. Taking the medication consistently supports adherence, but it does not by itself prove the drug is working.

2912. A client with deep vein thrombosis has been receiving a continuous heparin infusion for 24 hours. Which finding best indicates the therapy is achieving the intended effect?

- A. The activated partial thromboplastin time is 1.5 to 2 times the control value
- B. The platelet count has decreased from 220,000/mm³ to 128,000/mm³
- C. The client reports bruising at venipuncture sites
- D. The international normalized ratio is 2.4

Correct Answer: A

For heparin therapy, a therapeutic aPTT in the target range best indicates the anticoagulant effect is being achieved. A falling platelet count raises concern for heparin-induced thrombocytopenia, and bruising suggests bleeding risk rather than therapeutic success. INR is used to monitor warfarin, not to evaluate the effect of a heparin infusion.

Adverse response monitoring

2913. A client receives the first dose of IV cefazolin. Five minutes later, the client reports itching and says, "My throat feels tight." Which action should the nurse take first?

- A. Slow the infusion and continue to observe the client
- B. Stop the infusion and assess airway and breathing
- C. Document the reaction after the infusion is complete
- D. Ask the provider for an antihistamine prescription

Correct Answer: B

Throat tightness and itching shortly after an IV antibiotic begins suggest a possible acute hypersensitivity reaction. The priority is to stop the medication and assess airway and breathing immediately. Slowing the infusion delays treatment of a potentially life-threatening reaction. Calling for an antihistamine may be appropriate later, but airway assessment and stopping the offending medication come first.

2914. A client taking digoxin reports nausea, poor appetite, and seeing yellow-green halos around lights. Which finding is most concerning to the nurse as an adverse medication response?

- A. Visual halos
- B. Dry mouth
- C. Increased thirst after exercise
- D. Mild headache at bedtime

Correct Answer: A

Yellow-green halos are a classic sign of digoxin toxicity and should prompt immediate concern. Nausea and poor appetite also support this possibility. Dry mouth, thirst after exercise, and a mild bedtime headache are less specific and are not the hallmark finding here. The likely trap is focusing on the common symptom of nausea instead of recognizing the more distinctive visual change.

2915. A client is prescribed an opioid analgesic every 4 hours as needed for pain. Which assessment finding requires the nurse's immediate action before giving the next dose?

- A. Pain rating of 8 out of 10
- B. Respiratory rate of 8/min
- C. Blood pressure of 138/84 mm Hg
- D. Report of constipation for 2 days

Correct Answer: B

A respiratory rate of 8/min suggests opioid-induced respiratory depression and requires immediate action before administering another dose. Severe pain and constipation matter, but neither is the top priority over inadequate respirations. The nurse should hold the opioid and further assess the client according to facility policy and provider notification needs.

2916. A client starts a blood transfusion. Ten minutes later, the client develops chills, fever, and low back pain. What is the nurse's priority action?

- A. Increase the infusion rate to clear the tubing
- B. Stop the transfusion and keep the IV line open with appropriate solution
- C. Recheck the temperature in 30 minutes
- D. Administer the prescribed PRN analgesic and continue the transfusion

Correct Answer: B

Fever, chills, and low back pain during a transfusion suggest an acute transfusion reaction. The priority is to stop the transfusion immediately and maintain IV access with appropriate solution per protocol. Continuing or increasing the transfusion can worsen the reaction. Waiting to recheck the temperature delays urgent intervention.

2917. A client taking furosemide reports weakness and muscle cramps. Which laboratory result should the nurse review first as a possible cause of these symptoms?

- A. Potassium 3.0 mEq/L
- B. Hemoglobin 13.8 g/dL
- C. Platelets 240,000/mm³
- D. WBC 7,500/mm³

Correct Answer: A

Furosemide can cause potassium loss. Weakness and muscle cramps are consistent with hypokalemia, so a potassium of 3.0 mEq/L is the key result to review first. The other values are within expected ranges and do not best explain the symptoms. The likely trap is overlooking electrolyte-related adverse effects when the client reports nonspecific weakness.

2918. A client has been taking warfarin. Which assessment finding is most important for the nurse to report as a possible adverse response?

- A. Small bruise at a venipuncture site
- B. Tarry black stools
- C. Mild decreased appetite
- D. Occasional fatigue in the evening

Correct Answer: B

Tarry black stools may indicate gastrointestinal bleeding, a serious adverse response to warfarin. A small bruise after venipuncture can occur and is less urgent than signs of internal bleeding. Mild appetite changes and evening fatigue are less specific. The best-answer reasoning is to choose the finding that suggests active bleeding rather than a minor expected effect.

2919. A nurse gives regular insulin at 0730. At 1000, the client becomes shaky, diaphoretic, and confused. Which adverse response does the nurse suspect?

- A. Hyperglycemia
- B. Hypoglycemia
- C. Insulin allergy
- D. Fluid overload

Correct Answer: B

Shakiness, diaphoresis, and confusion after insulin administration are classic signs of hypoglycemia. Hyperglycemia usually presents differently, often with increased thirst, increased urination, and less acute adrenergic symptoms. Insulin allergy is uncommon and would more likely involve local or systemic allergic signs. Fluid overload is unrelated to this medication pattern.

2920. A client taking an ACE inhibitor for hypertension tells the nurse, "I have a dry cough that won't go away." What is the nurse's best response?

- A. This means the medication is not working and should be doubled
- B. This can be an adverse effect of the medication and should be reported
- C. This usually means you have developed pneumonia
- D. This is expected only if the dose was missed

Correct Answer: B

A persistent dry cough is a known adverse effect associated with ACE inhibitors and should be reported. It does not mean the medication should be doubled, and it is not the same as evidence of pneumonia. Missing a dose does not explain a persistent medication-related cough. The key skill is recognizing a common adverse response that may require a change in therapy.

2921. A client is receiving IV morphine after surgery. Which combination of findings most strongly suggests an adverse response requiring immediate intervention?

- A. Pain 6 out of 10, pulse 96/min, respirations 18/min
- B. Nausea, absent bowel movement for 1 day, pulse 88/min
- C. Difficult to arouse, oxygen saturation 88%, respirations 7/min
- D. Itching at the incision, temperature 99.1°F (37.3°C), blood pressure 126/74 mm Hg

Correct Answer: C

Difficulty arousing, oxygen saturation of 88%, and respirations of 7/min indicate significant opioid-induced respiratory depression. This requires immediate intervention. Mild nausea and constipation are common but not the top emergency here. The trap is choosing common side effects instead of recognizing the life-threatening pattern of oversedation plus hypoventilation.

2922. A client taking metformin tells the nurse about new symptoms. Which report should the nurse investigate first as a possible serious adverse response?

- A. Metallic taste in the mouth after the morning dose
- B. Occasional loose stool after meals
- C. Increasing fatigue with deep, rapid breathing
- D. Reduced appetite for the past day

Correct Answer: C

Increasing fatigue with deep, rapid breathing raises concern for lactic acidosis, a rare but serious adverse response associated with metformin. Mild GI effects such as loose stool or decreased appetite are more common and usually less urgent. The metallic taste is not the most dangerous finding here. The key is identifying a symptom cluster that suggests a serious systemic problem rather than routine GI intolerance.

2923. A client receives a unit of packed red blood cells. Fifteen minutes later, the client reports chest tightness and dyspnea. Breath sounds are crackly bilaterally, and the blood pressure is elevated. Which adverse response is most likely?

- A. Circulatory overload related to transfusion
- B. Delayed hemolytic reaction
- C. Expected anxiety during transfusion
- D. Mild allergic reaction limited to the skin

Correct Answer: A

Dyspnea, chest tightness, crackles, and elevated blood pressure during transfusion suggest circulatory overload. A delayed hemolytic reaction would not typically present this quickly. A mild allergic reaction usually involves itching or hives rather than crackles and hypertension. Anxiety does not explain the objective respiratory and cardiovascular findings.

2924. A client is taking phenytoin for seizure control. During oral care, the nurse notes swollen, overgrown gums. Which interpretation is most accurate?

- A. This finding suggests a common adverse effect of phenytoin
- B. This finding shows the medication level is too low
- C. This finding means the client is allergic to latex
- D. This finding indicates acute liver failure

Correct Answer: A

Gingival hyperplasia is a known adverse effect of phenytoin. It does not indicate a low drug level, latex allergy, or acute liver failure. The question tests whether the nurse can connect a focused physical assessment finding to a medication-related adverse response rather than to an unrelated condition.

2925. A client taking heparin develops sudden flank pain, a drop in blood pressure, and increasing abdominal distention. Which action should the nurse take first?

- A. Administer the next scheduled dose with food
- B. Document the findings as expected bruising
- C. Hold the heparin and notify the provider immediately
- D. Encourage ambulation to improve circulation

Correct Answer: C

Flank pain, hypotension, and abdominal distention in a client on heparin suggest possible internal bleeding, which is an emergency. The nurse should hold the medication and notify the provider immediately. Giving the next dose or treating this as expected bruising would be unsafe. Ambulation does not address the probable hemorrhage.

2926. A client taking a potassium-sparing diuretic is being monitored for adverse responses. Which assessment finding should concern the nurse most?

- A. Irregular pulse and muscle weakness
- B. Dry skin and increased thirst
- C. Urinary frequency in the morning
- D. Mild weight loss over 1 week

Correct Answer: A

Potassium-sparing diuretics can lead to hyperkalemia. Muscle weakness and an irregular pulse may indicate elevated potassium affecting neuromuscular and cardiac function. Dry skin, thirst, urinary frequency, and mild weight loss are less specific and less immediately dangerous in this context.

2927. A client receives the first dose of a new antihypertensive medication. Thirty minutes later, the client becomes dizzy when standing and nearly falls. Which explanation best fits this adverse response?

- A. The client is having a medication-induced orthostatic effect
- B. The client is developing dependence on the medication
- C. The medication is causing a delayed allergic reaction
- D. The medication has no relation to the symptoms

Correct Answer: A

Dizziness and near-fall shortly after starting an antihypertensive are consistent with orthostatic hypotension, an important adverse response to monitor. Dependence does not develop this way, and a delayed allergic reaction would not best explain position-related dizziness. The nurse should connect the timing and standing-related symptoms to a blood pressure effect.

2928. A client has been taking trimethoprim-sulfamethoxazole for 6 days. Which new finding should the nurse report immediately as a possible serious adverse medication response?

- A. Mild nausea after taking the pill
- B. A diffuse rash with blistering of the lips
- C. Decreased appetite at lunchtime
- D. Dark yellow urine on first morning void

Correct Answer: B

A diffuse rash with blistering of the lips suggests a severe hypersensitivity reaction such as Stevens-Johnson syndrome and requires immediate reporting and intervention. Mild nausea and decreased appetite can occur with many medications and are less urgent. Darker first-morning urine may reflect concentrated urine rather than a dangerous adverse response.

2929. A client receiving IV vancomycin suddenly becomes flushed over the face and upper chest and reports itching during the infusion. Blood pressure is stable, and there is no wheezing. Which action is most appropriate first?

- A. Stop the infusion and prepare for CPR
- B. Slow the infusion and notify the provider
- C. Increase the infusion rate to finish the dose quickly
- D. Document the finding as expected and continue unchanged

Correct Answer: B

Flushing and itching during vancomycin infusion with stable blood pressure and no wheezing are consistent with an infusion-related reaction commonly managed by slowing the infusion and notifying the provider. This is different from signs of anaphylaxis, which would make stopping the infusion and emergency intervention the priority. Increasing the rate would worsen the reaction, and continuing unchanged is inappropriate.

2930. A nurse is monitoring a client who began haloperidol 2 hours ago for severe agitation. Which assessment finding suggests an acute dystonic reaction?

- A. Lip smacking and repetitive tongue movements

- B.** Neck muscle spasm with the head pulled to one side
- C.** Shuffling gait that developed over several months
- D.** Fine hand tremor that improves with activity

Correct Answer: B

Acute dystonia often appears soon after starting an antipsychotic and can cause painful muscle spasms such as torticollis, jaw stiffness, or oculogyric crisis. Lip smacking and repetitive tongue movements suggest tardive dyskinesia, which is typically a later effect. A shuffling gait suggests parkinsonism, and a fine tremor is less specific.

2931. A client takes lithium for bipolar disorder. Which finding should the nurse identify as the strongest indicator of possible lithium toxicity?

- A.** Mild thirst and increased urination
- B.** Weight gain of 1 lb in a week
- C.** Coarse hand tremor with unsteady gait
- D.** Dry mouth after waking up

Correct Answer: C

Coarse tremor and unsteady gait are concerning for lithium toxicity and require prompt evaluation. Mild thirst and increased urination can occur with lithium therapy and are not by themselves the strongest sign of toxicity. Minor weight change and dry mouth are less specific.

2932. A client receiving a nitroglycerin infusion for chest pain tells the nurse, "My head is pounding." Which interpretation is most accurate?

- A.** This can be an expected adverse effect of vasodilation
- B.** This means the medication is ineffective and should be stopped
- C.** This proves the client is having an allergic reaction
- D.** This indicates severe hypoglycemia

Correct Answer: A

Headache is a common adverse effect of nitroglycerin because of vasodilation. It should be monitored, but it does not by itself mean the medication is ineffective or that the client is having an allergic reaction. Nitroglycerin is not associated with hypoglycemia.

2933. A client taking sertraline comes to the clinic with restlessness, diaphoresis, tremor, and diarrhea after starting an over-the-counter cough medicine 2 days ago. Which adverse response should the nurse suspect?

- A.** Serotonin syndrome
- B.** Acute pancreatitis
- C.** Orthostatic hypotension
- D.** Tardive dyskinesia

Correct Answer: A

Restlessness, diaphoresis, tremor, and diarrhea after combining a serotonergic medication with some cough preparations suggest serotonin syndrome. The timing and symptom cluster fit this adverse response better than pancreatitis, orthostatic hypotension, or tardive dyskinesia.

2934. A client receives the first oral dose of an antibiotic and 20 minutes later develops widespread hives, hoarseness, and swelling of the tongue. Which action should the nurse take first?

- A.** Position the client flat and recheck in 15 minutes
- B.** Hold the next scheduled dose and document the reaction
- C.** Activate emergency response and maintain the airway
- D.** Offer water to help relieve throat irritation

Correct Answer: C

Hives, hoarseness, and tongue swelling after a medication indicate possible anaphylaxis. The first priority is emergency response with airway support. Delaying action to observe, simply withholding a later dose, or offering water is unsafe because airway compromise can rapidly worsen.

2935. A client receiving gentamicin says, "My ears are ringing." Which additional finding would most strongly support the nurse's concern about a serious adverse response?

- A.** Hearing seems muffled today
- B.** Appetite is lower than usual
- C.** Mild redness at the IV site
- D.** The client slept poorly last night

Correct Answer: A

Gentamicin can cause ototoxicity. Tinnitus together with muffled hearing strongly supports this concern. Lower appetite, mild IV-site redness, and poor sleep do not specifically point to the same serious adverse response.

2936. A nurse is caring for a client who started allopurinol 1 week ago. Which finding requires the most immediate follow-up?

- A. The client drinks more water than usual
- B. The client reports a new skin rash
- C. The client says joint pain improved
- D. The client needs help remembering the dose time

Correct Answer: B

A new skin rash while taking allopurinol can signal a serious hypersensitivity reaction and requires prompt follow-up. Increased fluid intake may be encouraged, improvement in joint pain is not concerning, and forgetting the dose time relates more to teaching than to an urgent adverse response.

2937. A client is receiving a magnesium sulfate infusion. Which assessment finding should the nurse recognize as the clearest sign of possible magnesium toxicity?

- A. Hyperactive deep tendon reflexes
- B. Respirations of 8/min
- C. Blood pressure 138/84 mm Hg
- D. Urine output of 50 mL in 2 hours

Correct Answer: B

Respiratory depression is a major sign of magnesium toxicity and demands immediate action. Hyperactive reflexes are not expected with magnesium toxicity; decreased reflexes are more typical. Urine output should be monitored because low output can increase risk, but a respiratory rate of 8/min is the clearest current sign of toxicity. A blood pressure of 138/84 mm Hg is not the priority concern here.

2938. A client using a transdermal fentanyl patch is difficult to arouse. Which additional finding would require the nurse's most urgent action as evidence of an adverse response?

- A. Respiratory rate of 7/min
- B. Dry mouth
- C. Constipation for 2 days

- D. Pupils 3 mm and reactive

Correct Answer: A

A respiratory rate of 7/min indicates significant opioid-induced respiratory depression and requires immediate intervention. Dry mouth and constipation are common opioid adverse effects but are not as urgent. Pupil size of 3 mm and reactivity does not indicate the most dangerous opioid complication.

2939. A client takes isoniazid for tuberculosis. Which report most strongly suggests a serious adverse response that the nurse should address promptly?

- A. Numbness and tingling in the feet
- B. Orange-colored tears
- C. Increased appetite in the evening
- D. Occasional sneezing after meals

Correct Answer: A

Numbness and tingling in the feet suggest peripheral neuropathy, a significant adverse response associated with isoniazid that requires prompt attention. Orange-colored body fluids are associated with rifampin, not isoniazid. Increased appetite and sneezing do not suggest a serious medication toxicity here.

2940. A client begins receiving a unit of packed red blood cells. After about 30 minutes, the client develops fever, hypotension, and dark urine. Which adverse response is most likely?

- A. Acute hemolytic transfusion reaction
- B. Mild allergic transfusion reaction
- C. Iron overload
- D. Delayed therapeutic effect

Correct Answer: A

Fever, hypotension, and dark urine during a transfusion strongly suggest an acute hemolytic transfusion reaction, which is a medical emergency. A mild allergic reaction more often causes itching or hives without hemoglobinuria or hypotension. Iron overload does not occur acutely during one transfusion, and delayed therapeutic effect does not fit the findings.

2941. A client taking a loop diuretic has palpitations. Which assessment finding, if present, would most increase the nurse's concern that the symptom is related to an adverse medication response?

- A. Bilateral ankle edema at the end of the day
- B. Muscle weakness and an irregular pulse
- C. Warm, dry skin after walking
- D. Urinary frequency in the morning

Correct Answer: B

Loop diuretics can cause potassium loss. Palpitations together with muscle weakness and an irregular pulse raise concern for dysrhythmia related to electrolyte imbalance. Ankle edema, warm dry skin after activity, and morning urinary frequency do not point as directly to a dangerous adverse response.

2942. A client started warfarin 5 days ago. Which finding should the nurse report immediately as the most concerning adverse response?

- A. Bleeding gums while brushing teeth
- B. Black, tarry stools
- C. Mild nausea after the evening dose
- D. A small bruise at a previous venipuncture site

Correct Answer: B

Black, tarry stools suggest gastrointestinal bleeding, which is a serious adverse response to warfarin and requires immediate reporting. Mild bleeding gums, nausea, and minor bruising can occur, but melena points to potentially significant internal bleeding. The key is recognizing which finding signals the greatest risk rather than simply any side effect.

2943. A client is receiving morphine by IV for postoperative pain. Thirty minutes after the dose, which finding is the nurse's priority because it most strongly suggests a serious adverse response to the medication?

- A. Respiratory rate of 8/min
- B. Nausea with one episode of emesis
- C. Reports of itching at the nose
- D. Pupils that are slightly constricted

Correct Answer: A

A respiratory rate of 8/min is the priority finding because opioid-induced respiratory depression is a serious adverse response that can rapidly become life-threatening. Nausea, itching, and mild pupillary constriction are common opioid effects, but they are not as immediately dangerous as significant hypoventilation.

2944. A client started captopril this morning. Which new finding should the nurse treat as the most urgent possible adverse medication response?

- A. Dry cough
- B. Swelling of the lips and tongue
- C. Mild dizziness when standing
- D. Altered taste perception

Correct Answer: B

Swelling of the lips and tongue suggests angioedema, a potentially life-threatening adverse response because it can progress to airway obstruction. A dry cough, orthostatic dizziness, and taste changes can occur with ACE inhibitors, but they are not as immediately dangerous as airway swelling.

2945. A nurse is monitoring a client who received regular insulin 1 hour ago. Which assessment finding most clearly indicates an adverse response that requires prompt intervention?

- A. Warm, dry skin and increased thirst
- B. Drowsiness, sweating, and shakiness
- C. Polyuria and deep respirations
- D. Hunger after missing lunch yesterday

Correct Answer: B

Drowsiness, sweating, and shakiness are classic signs of hypoglycemia, an important adverse response to insulin that requires prompt treatment. Warm, dry skin with thirst and polyuria with deep respirations are more consistent with hyperglycemia. Hunger alone is less specific and does not indicate a clear medication-related emergency by itself.

2946. A client has been receiving heparin therapy. Which finding is most important for the nurse to report as a possible adverse response to the medication?

- A. Small bruise at a previous venipuncture site

- B. Platelet count decreased from 220,000/mm³ to 96,000/mm³
- C. Heart rate increased from 78/min to 88/min
- D. Mild discomfort at the subcutaneous injection site

Correct Answer: B

A platelet count drop to 96,000/mm³ is concerning for heparin-induced thrombocytopenia, a serious adverse response that requires prompt follow-up. A small bruise and mild injection-site discomfort are common and less concerning. A slight heart rate increase is nonspecific and does not point as strongly to a major heparin complication.

2947. A client with heart failure takes digoxin 0.125 mg daily and furosemide 40 mg daily. Which assessment finding should make the nurse most suspicious of digoxin toxicity as an adverse response?

- A. Apical pulse 56/min with nausea and yellow-tinged vision
- B. Blood pressure 148/86 mm Hg with ankle edema
- C. Respiratory rate 22/min with crackles at the lung bases
- D. Weight gain of 0.9 kg (2 lb) in 24 hours

Correct Answer: A

Bradycardia, nausea, and yellow-tinged vision strongly suggest digoxin toxicity. The added context of concurrent furosemide is an important clue because loop diuretics can contribute to electrolyte imbalance, increasing toxicity risk. The other findings point more toward ongoing heart failure rather than a specific digoxin adverse response.

Drug interactions

2948. A patient taking warfarin for atrial fibrillation asks the nurse whether it is safe to start taking ibuprofen for back pain. Which response is best?

- A. Ibuprofen is safe because it does not affect blood clotting
- B. Take ibuprofen only with food so it will not interact with warfarin
- C. Avoid ibuprofen unless the prescriber approves it because it can increase bleeding risk

- D. Stop the warfarin on days you need ibuprofen

Correct Answer: C

Warfarin and ibuprofen together can increase bleeding risk. The safest nursing response is to tell the patient to avoid starting ibuprofen unless the prescriber approves it. Choice A is false because NSAIDs can affect bleeding. Choice B addresses stomach irritation, not the major interaction concern. Choice D is unsafe because patients should not stop anticoagulants on their own.

2949. A nurse is reviewing newly prescribed medications for a patient who takes nitroglycerin as needed for angina. Which additional medication should the nurse question first?

- A. Sildenafil
- B. Acetaminophen
- C. Calcium carbonate
- D. Docusate sodium

Correct Answer: A

Sildenafil taken with nitroglycerin can cause severe hypotension because both promote vasodilation. This is the most urgent interaction to question. The other medications do not pose the same immediate hemodynamic danger.

2950. A patient receiving phenelzine reports eating aged cheese at a party. Which finding is most important for the nurse to monitor for?

- A. Severe hypertension
- B. Hypoglycemia
- C. Bradycardia
- D. Constipation

Correct Answer: A

Phenelzine is a monoamine oxidase inhibitor. Foods high in tyramine, such as aged cheese, can trigger a hypertensive crisis. Severe hypertension is the priority concern. The other options are not the classic dangerous interaction associated with MAOIs and tyramine-containing foods.

2951. A patient takes digoxin daily and furosemide twice a day. Which laboratory value should cause the greatest concern for a possible digoxin-related problem?

- A. Potassium 2.9 mEq/L
- B. Sodium 138 mEq/L

- C. Calcium 9.2 mg/dL
- D. Magnesium 2.0 mg/dL

Correct Answer: A

Furosemide can lower potassium levels, and hypokalemia increases the risk of digoxin toxicity. A potassium of 2.9 mEq/L is the most concerning value. The other labs are not the key interaction-related trigger in this scenario.

2952. A patient is prescribed tetracycline. Which statement by the patient indicates a need for further teaching?

- A. I will take the medicine with a full glass of water
- B. I should avoid taking it with milk
- C. If my stomach is upset, I can take it with an antacid
- D. I will follow the schedule exactly as prescribed

Correct Answer: C

Antacids can bind tetracycline and reduce its absorption. Milk and other calcium-containing products can do the same, so avoiding them is appropriate. Taking the medication with water and following the prescribed schedule are also correct. The teaching problem is the plan to take it with an antacid.

2953. The nurse is preparing to administer a newly ordered dose of lisinopril to a patient who also takes spironolactone. Which assessment finding is most important to report before giving the medication?

- A. Potassium 5.8 mEq/L
- B. Heart rate 88/min
- C. Temperature 98.6°F (37°C)
- D. Respiratory rate 18/min

Correct Answer: A

Lisinopril and spironolactone can both increase potassium levels, placing the patient at risk for hyperkalemia. A potassium of 5.8 mEq/L is the most important finding to report before administration. The other findings are not the major interaction-related concern.

2954. A patient receiving insulin asks why the nurse is concerned about the patient also taking propranolol. Which explanation is best?

- A. Propranolol can raise the blood glucose level after insulin is given

B. Propranolol can hide signs of hypoglycemia such as a fast heart rate

C. Propranolol prevents insulin from being absorbed at the injection site

D. Propranolol causes insulin to wear off immediately

Correct Answer: B

Beta blockers such as propranolol can mask adrenergic symptoms of hypoglycemia, especially tachycardia. This makes recognition of low blood glucose more difficult. The other options do not describe the key safety issue with this combination.

2955. A nurse cares for a patient taking sertraline. Which newly prescribed medication would require the closest follow-up for a potentially serious interaction?

- A. Sumatriptan
- B. Famotidine
- C. Loratadine
- D. Psyllium

Correct Answer: A

Sertraline and sumatriptan both affect serotonin pathways and may increase the risk for serotonin syndrome. This interaction warrants close follow-up. The other medications do not carry the same major serotonergic concern.

2956. A patient who takes oral contraceptives is prescribed rifampin. Which instruction should the nurse include?

- A. Take both drugs at the same time each day to prevent interaction
- B. Use a backup form of contraception while taking rifampin
- C. Skip the contraceptive on days rifampin is taken
- D. Double the oral contraceptive dose during antibiotic therapy

Correct Answer: B

Rifampin can reduce the effectiveness of oral contraceptives, so backup contraception is needed. Taking the drugs at the same time does not prevent the interaction. Skipping pills or doubling the dose is unsafe and not an appropriate nursing instruction.

2957. A patient taking lithium is admitted with vomiting and diarrhea for 2 days. The patient also takes hydrochlorothiazide. Which finding most strongly suggests a significant medication interaction problem?

- A. Fine hand tremor that has worsened
- B. Blood pressure 126/74 mm Hg
- C. Urine output 900 mL in 24 hours
- D. Fasting blood glucose 92 mg/dL

Correct Answer: A

Lithium levels can rise with sodium and fluid loss, and thiazide diuretics can further increase lithium levels. Worsening tremor suggests possible lithium toxicity. The blood pressure and glucose are not specific to this interaction, and the urine output alone is less direct than a classic toxicity sign.

2958. The nurse is reviewing medications for a patient who drinks grapefruit juice every morning. Which medication should the nurse identify as most likely to be affected?

- A. Atorvastatin
- B. Amoxicillin
- C. Metformin
- D. Acetaminophen

Correct Answer: A

Grapefruit juice can alter the metabolism of certain medications, including some statins such as atorvastatin, increasing the risk of adverse effects. The other medications are not the classic concern in this teaching scenario.

2959. A patient taking warfarin says, "I started drinking a green smoothie every day to be healthier." Which nursing response is most appropriate?

- A. That is fine because vegetables do not affect warfarin
- B. Stop drinking all smoothies immediately
- C. Keep vitamin K intake consistent and tell the prescriber about the diet change
- D. Take an extra warfarin tablet on days you drink the smoothie

Correct Answer: C

Many green leafy vegetables are high in vitamin K, which can affect warfarin therapy. The key teaching is consistency in vitamin K intake and communicating major diet changes to the prescriber. The other choices are either inaccurate or unsafe.

2960. A patient takes theophylline for chronic lung disease and calls the clinic after starting ciprofloxacin. Which report requires the most immediate attention?

- A. Trouble sleeping for one night
- B. Mild dry mouth
- C. Palpitations and repeated vomiting
- D. Increased appetite

Correct Answer: C

Ciprofloxacin can increase theophylline levels, raising the risk of toxicity. Palpitations and repeated vomiting are concerning signs that require prompt attention. Trouble sleeping can occur with stimulatory drugs but is less urgent than clear toxicity symptoms.

2961. A postoperative patient receives morphine and is then given a benzodiazepine for anxiety. Which outcome is the nurse's priority concern?

- A. Respiratory depression from additive CNS effects
- B. Hypertension from opposing vascular effects
- C. Polyuria from increased renal blood flow
- D. Rash from an allergic cross-sensitivity reaction

Correct Answer: A

Opioids and benzodiazepines both depress the central nervous system, and together they can produce dangerous sedation and respiratory depression. That additive effect is the priority concern. The other options do not reflect the major expected interaction risk.

2962. A patient with heart failure takes digoxin. The nurse notes a new prescription for clarithromycin. Which action is best?

- A. Administer both medications and encourage extra fluids
- B. Hold the digoxin permanently
- C. Notify the prescriber because clarithromycin can increase digoxin toxicity risk

D. Give the digoxin 2 hours early to avoid the interaction

Correct Answer: C

Clarithromycin can increase digoxin levels and toxicity risk, so the prescriber should be notified for review. Permanently holding digoxin without an order is inappropriate, and changing administration time does not reliably prevent this interaction. Extra fluids do not address the underlying risk.

2963. A patient takes linezolid for a resistant skin infection. During discharge teaching, which over-the-counter medication should the nurse instruct the patient to avoid unless approved by the provider?

- A.** Pseudoephedrine nasal decongestant
- B.** Calcium carbonate antacid
- C.** Docusate stool softener
- D.** Artificial tears

Correct Answer: A

Pseudoephedrine can interact with linezolid and increase the risk of a dangerous rise in blood pressure because linezolid has monoamine oxidase inhibitor-like effects. The other products listed do not pose the same interaction concern. A common trap is focusing only on prescription drugs and overlooking OTC decongestants.

2964. A nurse is checking the medication list of a patient who takes metformin for type 2 diabetes. The patient is scheduled for a contrast-enhanced CT scan this afternoon. Which action is most important?

- A.** Administer the metformin with food before the scan
- B.** Ask whether the patient has taken acetaminophen today
- C.** Clarify the metformin plan with the provider before contrast is given
- D.** Encourage the patient to drink grapefruit juice after the scan

Correct Answer: C

Metformin requires careful management when iodinated contrast is used because reduced kidney function after contrast exposure can increase the risk of metformin-associated complications. The nurse should clarify the plan before contrast administration. Giving it routinely is not the safest choice. The other options do not address the key interaction concern.

2965. A patient taking sildenafil for erectile dysfunction is admitted with chest pain. Which prescribed medication should the nurse question first because of a potentially dangerous interaction?

- A.** Nitroglycerin sublingual
- B.** Aspirin chewable
- C.** Metoprolol oral
- D.** Atorvastatin oral

Correct Answer: A

Nitroglycerin and sildenafil can cause profound hypotension when used together. This interaction is high priority and should be questioned immediately. Aspirin, metoprolol, and atorvastatin do not carry the same immediate interaction risk in this scenario.

2966. A patient has been taking prednisone for several weeks and is now prescribed naproxen for joint pain. Which teaching point is most important for the nurse to include?

- A.** Take both medications on an empty stomach for best absorption
- B.** Report black, tarry stools or stomach pain promptly
- C.** Expect the naproxen to prevent steroid-related insomnia
- D.** Stop the prednisone as soon as the naproxen starts working

Correct Answer: B

Prednisone and naproxen together increase the risk of gastrointestinal irritation and bleeding. Black, tarry stools or abdominal pain may indicate bleeding and require prompt evaluation. Taking them on an empty stomach would worsen irritation, and prednisone should not be stopped abruptly after prolonged use.

2967. A nurse reviews morning medications for a patient who receives levothyroxine daily. Which additional medication, if scheduled at the same time, should the nurse expect to separate from levothyroxine administration?

- A. Ferrous sulfate
- B. Acetaminophen
- C. Furosemide
- D. Docusate sodium

Correct Answer: A

Ferrous sulfate can decrease levothyroxine absorption, so these medications are commonly separated in timing. Acetaminophen, furosemide, and docusate do not have the same well-known absorption interaction. The key issue here is reduced therapeutic effect rather than an immediate toxic reaction.

2968. A patient takes phenytoin for seizure control and has just started taking combined oral contraceptive pills. Which outcome is the nurse most concerned about from this medication combination?

- A. Severe constipation
- B. Reduced contraceptive effectiveness
- C. Increased urine output
- D. Bluish discoloration of the skin

Correct Answer: B

Phenytoin can reduce the effectiveness of hormonal contraceptives, increasing the risk of unintended pregnancy. The other options are not the main interaction concern. A common mistake is focusing only on side effects of one drug instead of how one medication changes the effect of another.

2969. A patient takes trimethoprim-sulfamethoxazole for a urinary infection and also receives warfarin daily. Which finding most strongly suggests the interaction is becoming clinically significant?

- A. New bruising and bleeding gums
- B. Mild thirst before lunch
- C. A heart rate of 88/min
- D. Urine output of 900 mL in 8 hours

Correct Answer: A

Trimethoprim-sulfamethoxazole can increase warfarin's anticoagulant effect, raising bleeding risk. New bruising and bleeding gums are concerning signs of excessive anticoagulation. The other findings are not as specific or urgent for this interaction.

2970. A nurse is reinforcing teaching for a patient prescribed metronidazole. Which statement by the patient shows understanding of a major interaction precaution?

- A. I can stop the medicine when my stomach feels better.
- B. I should avoid alcohol while taking this medication.
- C. I need to take this with milk at every dose.
- D. I should double the next dose if I miss one.

Correct Answer: B

Alcohol should be avoided with metronidazole because the combination can trigger an unpleasant and potentially significant reaction. The other statements are incorrect medication practices. This question tests recognition of a common but important patient-teaching interaction.

2971. A patient with bipolar disorder takes lithium. Which newly prescribed medication should the nurse question because it can increase the risk of lithium toxicity?

- A. Ibuprofen
- B. Loratadine
- C. Famotidine
- D. Nystatin oral suspension

Correct Answer: A

NSAIDs such as ibuprofen can increase lithium levels and raise the risk of toxicity. Loratadine, famotidine, and nystatin are not the same level of concern for this interaction. The trap is assuming an over-the-counter pain reliever is automatically harmless.

2972. A patient takes simvastatin every evening. Which new prescription should the nurse identify as creating the greatest risk for a serious interaction?

- A. Azithromycin
- B. Amoxicillin
- C. Erythromycin

D. Cephalexin**Correct Answer: C**

Erythromycin can raise simvastatin levels and increase the risk of statin toxicity, including muscle injury. Amoxicillin and cephalexin do not have this major interaction, and azithromycin is generally less concerning than erythromycin in this context. The nurse should recognize the higher-risk combination and follow up.

2973. A patient receiving heparin infusion develops a new order for aspirin 325 mg. Which nursing action is best?

- A. Administer the aspirin because both drugs reduce clotting
- B. Hold the heparin and give the aspirin instead
- C. Question the order because the combination may increase bleeding risk
- D. Give both medications and limit oral fluids

Correct Answer: C

Heparin plus aspirin can increase bleeding risk because both affect hemostasis. The nurse should question the order or verify the intent before administration, especially if there is no clear indication for combined therapy. Administering both without clarification could be unsafe.

2974. A patient is taking oral ciprofloxacin. Which beverage choice by the patient should the nurse address because it can interfere with the medication's effectiveness?

- A. Coffee with sugar
- B. Apple juice
- C. Milk taken with the dose
- D. Warm tea with lemon

Correct Answer: C

Milk and other calcium-containing products can interfere with ciprofloxacin absorption and reduce its effectiveness if taken with the dose. The other beverages listed are not the main concern here. The key issue is reduced absorption, not an allergic reaction or toxicity.

2975. A patient who takes potassium chloride daily is prescribed trimethoprim-sulfamethoxazole. Which laboratory value should the nurse watch most closely for a possible interaction-related problem?

- A. Potassium 5.8 mEq/L

- B. Sodium 138 mEq/L

- C. Calcium 9.1 mg/dL

- D. Hemoglobin 13.4 g/dL

Correct Answer: A

Trimethoprim-sulfamethoxazole can contribute to elevated potassium, so a potassium level of 5.8 mEq/L is the most concerning finding in a patient already taking potassium supplements. The other values are not the key interaction signal. This requires connecting the medication pair with the lab abnormality.

2976. A patient uses a transdermal fentanyl patch for chronic pain. Which new order should the nurse question because it may dangerously increase fentanyl absorption?

- A. Apply a heating pad over the patch site for comfort
- B. Rotate the patch to a different intact skin area
- C. Document the patch location on the MAR
- D. Assess pain level before replacement

Correct Answer: A

External heat can increase absorption from a transdermal fentanyl patch and raise the risk of opioid toxicity, including respiratory depression. Rotating the site, documenting placement, and assessing pain are appropriate. The interaction here is between heat exposure and the medication delivery system.

2977. A patient taking valproic acid is prescribed carbapenem antibiotic therapy. Which nursing concern is most appropriate?

- A. The antibiotic may sharply lower valproic acid levels and reduce seizure control
- B. The combination will always cause severe hypertension within hours
- C. Valproic acid will block the antibiotic and make it completely ineffective
- D. The interaction mainly causes orange discoloration of body fluids

Correct Answer: A

Carbapenem antibiotics can significantly lower valproic acid levels, creating a risk for loss of seizure control. The other options are inaccurate or exaggerated. This is a harder question because it asks the nurse to identify a less obvious but clinically important interaction that reduces therapeutic effect.

2978. A patient takes warfarin for chronic atrial fibrillation. During medication reconciliation, the nurse learns the patient recently began taking an herbal supplement to "help with mood." Which supplement should the nurse identify as most likely to reduce warfarin's effectiveness?

- A. St. John's wort
- B. Ginkgo biloba
- C. Garlic extract
- D. Fish oil

Correct Answer: A

St. John's wort can decrease warfarin's anticoagulant effect by increasing drug metabolism, which may lower the INR and reduce protection against clot formation. Ginkgo, garlic, and fish oil are more often associated with increased bleeding risk rather than reduced warfarin effectiveness, making them plausible distractors.

2979. A patient with heart failure takes digoxin daily. The nurse notes a new prescription for furosemide. Which finding would make the nurse most alert for a clinically important interaction between these medications?

- A. Potassium level of 2.9 mEq/L
- B. Blood glucose of 168 mg/dL
- C. Respiratory rate of 18/min
- D. Calcium level of 9.2 mg/dL

Correct Answer: A

Furosemide can cause potassium loss, and hypokalemia increases the risk of digoxin toxicity. A potassium level of 2.9 mEq/L is the most concerning cue. The glucose is mildly elevated but does not explain this interaction, and the respiratory rate and calcium value are not the key findings here.

2980. A patient taking an oral monoamine oxidase inhibitor for depression tells the nurse about several favorite foods. Which choice should the nurse identify as needing the most caution because of a potentially serious interaction?

- A. Fresh apple slices
- B. Aged cheddar cheese
- C. Steamed rice
- D. Plain yogurt

Correct Answer: B

Aged cheddar cheese is high in tyramine, which can interact with monoamine oxidase inhibitors and trigger severe hypertension. Fresh apples, steamed rice, and plain yogurt are not the classic high-tyramine concern in this setting. The key is recognizing the food-drug interaction rather than simply choosing a dairy product.

2981. The nurse is reviewing home medications before giving a first dose of spironolactone. Which additional medication on the patient's list should prompt the greatest concern for an interaction that can raise serum potassium?

- A. Lisinopril
- B. Acetaminophen
- C. Furosemide
- D. Pantoprazole

Correct Answer: A

Lisinopril, an ACE inhibitor, can increase potassium levels. When combined with spironolactone, the risk of hyperkalemia becomes more significant. Furosemide tends to lower potassium, and acetaminophen and pantoprazole are not the major concern for this interaction. The most likely trap is focusing only on blood pressure medications without thinking about their potassium effects.

2982. A patient has been taking phenelzine for depression and is admitted with a severe cough from an upper respiratory infection. The provider prescribes several medications. Which order should the nurse question first because of a potentially dangerous interaction?

- A. Benzonatate capsules as needed for cough
- B. Guaifenesin syrup as needed for congestion
- C. Dextromethorphan cough syrup as needed
- D. Normal saline nasal spray as needed

Correct Answer: C

Dextromethorphan can interact dangerously with an MAOI such as phenelzine and may contribute to serious adverse effects, including serotonin-related toxicity. Benzonatate, guaifenesin, and saline nasal spray do not carry this same major interaction concern. This item is harder because several options seem reasonable for cough treatment unless the MAOI interaction is recognized.

Critical labs

2983. A client is prescribed digoxin 0.125 mg PO daily. Before giving the morning dose, the nurse reviews the lab report and notes a serum potassium level of 2.9 mEq/L. What is the nurse's best action?

- A. Administer the digoxin and recheck the potassium later
- B. Hold the digoxin and notify the provider
- C. Give the digoxin with orange juice to improve absorption
- D. Reduce the dose by half and document the change

Correct Answer: B

A potassium level of 2.9 mEq/L is low and increases the risk of digoxin toxicity. The nurse should hold the medication and notify the provider before administration. Giving the drug anyway is unsafe, and the nurse cannot independently change the prescribed dose.

2984. A client receiving warfarin has a prothrombin time/international normalized ratio (PT/INR) result of 5.2. Which action is most appropriate?

- A. Administer the scheduled dose and encourage fluids
- B. Hold the warfarin and notify the provider
- C. Give the medication with food to decrease gastric upset
- D. Document the result as expected for all clients on warfarin

Correct Answer: B

An INR of 5.2 is markedly elevated and increases bleeding risk. The nurse should hold warfarin and notify the provider. A common trap is assuming any elevated INR is automatically therapeutic, but this value is high enough to require follow-up.

2985. The nurse is preparing to administer furosemide IV to a client with heart failure. Which laboratory value should cause the nurse to question the prescription before giving the medication?

- A. Potassium 3.0 mEq/L
- B. Sodium 138 mEq/L
- C. Chloride 103 mEq/L
- D. Magnesium 2.0 mg/dL

Correct Answer: A

Furosemide can lower potassium further, so a potassium level of 3.0 mEq/L should prompt the nurse to question the prescription and notify the provider. The other values are within expected range. The key is linking the lab to the medication's adverse effects.

2986. A client is scheduled to receive the next dose of subcutaneous heparin. Which laboratory result should the nurse report immediately?

- A. Platelet count 48,000/mm³
- B. Hemoglobin 13.8 g/dL
- C. White blood cell count 8,200/mm³
- D. Hematocrit 41%

Correct Answer: A

A platelet count of 48,000/mm³ is critically low and raises concern for major bleeding risk and possible heparin-induced thrombocytopenia. The nurse should report this finding immediately before giving more heparin. The other results are not the priority in this situation.

2987. A client with bipolar disorder takes lithium carbonate. Which laboratory finding requires the nurse to follow up before administering the next dose?

- A. Sodium 140 mEq/L
- B. Lithium level 1.9 mEq/L
- C. Calcium 9.4 mg/dL
- D. Phosphorus 3.8 mg/dL

Correct Answer: B

A lithium level of 1.9 mEq/L is elevated and suggests possible toxicity. The nurse should hold the dose and notify the provider. A common error is to focus only on whether the client is due for the medication rather than reviewing the drug-specific lab first.

2988. The nurse reviews laboratory results for four clients. Which client should the nurse assess first based on a medication-related critical lab?

- A. Client taking metformin with a creatinine of 2.6 mg/dL
- B. Client taking ferrous sulfate with a hemoglobin of 10.2 g/dL
- C. Client taking docusate with a sodium of 134 mEq/L
- D. Client taking famotidine with a magnesium of 1.9 mg/dL

Correct Answer: A

Metformin should be used cautiously in clients with significant renal impairment because reduced kidney function increases the risk for medication-related complications. A creatinine of 2.6 mg/dL requires prompt follow-up. The other findings are less urgent in relation to the listed medications.

2989. A client is receiving IV unfractionated heparin. The activated partial thromboplastin time (aPTT) comes back at 98 seconds. What should the nurse do first?

- A. Increase the infusion rate to maintain anticoagulation
- B. Stop the infusion and document the value as therapeutic
- C. Follow protocol for an elevated aPTT and notify the provider as indicated
- D. Administer the next dose of aspirin to prevent clot extension

Correct Answer: C

An aPTT of 98 seconds is markedly prolonged for many heparin protocols and indicates increased bleeding risk. The nurse should follow the facility protocol for an elevated aPTT and notify the provider as required. The safest answer is protocol-based action, not an independent guess about dose adjustment.

2990. A client taking spironolactone has morning laboratory results available. Which finding is most important for the nurse to report before administering the medication?

- A. Potassium 5.9 mEq/L
- B. Calcium 8.9 mg/dL
- C. Glucose 104 mg/dL
- D. Albumin 3.8 g/dL

Correct Answer: A

Spironolactone is a potassium-sparing diuretic, so a potassium level of 5.9 mEq/L is a major safety concern. Giving the drug could worsen hyperkalemia and trigger dysrhythmias. The other values do not present the same immediate medication-related danger.

2991. A client receives phenytoin for seizure control. The nurse notes a phenytoin level of 24 mcg/mL. Which nursing action is most appropriate?

- A. Give the medication because seizure drugs should never be delayed
- B. Hold the dose and notify the provider
- C. Administer the drug with an antacid to protect the stomach
- D. Double-check the route and then proceed with administration

Correct Answer: B

A phenytoin level of 24 mcg/mL is above the usual therapeutic range and may indicate toxicity. The nurse should hold the dose and notify the provider. The trap is assuming seizure medications should always be given on time regardless of lab findings.

2992. A postoperative client is prescribed morphine IV as needed for pain. Which laboratory result should make the nurse use increased caution and assess the client more closely before administration?

- A. AST 18 units/L
- B. Creatinine 2.1 mg/dL
- C. Platelets 210,000/mm³
- D. Potassium 4.1 mEq/L

Correct Answer: B

A creatinine of 2.1 mg/dL suggests impaired renal function, which can reduce clearance of some medications and increase the risk of sedation or respiratory depression with opioids. The nurse should assess carefully before administration. The other values do not raise the same medication-related concern.

2993. The nurse is preparing to administer enoxaparin to a client after orthopedic surgery. Which laboratory trend is most concerning?

- A. Platelet count falling from 220,000/mm³ to 92,000/mm³
- B. Hemoglobin rising from 10.8 g/dL to 11.6 g/dL
- C. Potassium rising from 3.7 mEq/L to 4.0 mEq/L
- D. White blood cell count falling from 11,000/mm³ to 9,400/mm³

Correct Answer: A

A significant drop in platelet count during enoxaparin therapy is concerning for bleeding risk and possible heparin-related thrombocytopenia. This trend requires prompt follow-up before the next dose. The key here is recognizing that the change over time matters, not just the single number.

2994. A client with a history of deep vein thrombosis is taking warfarin. The nurse receives these new findings: INR 6.1, bleeding gums while brushing teeth, and dark stools since yesterday. What is the nurse's priority action?

- A. Administer the warfarin with food to reduce irritation
- B. Hold the warfarin and notify the provider immediately
- C. Encourage ambulation because clotting risk remains high
- D. Document the findings and reassess in 8 hours

Correct Answer: B

The client has an INR of 6.1 plus signs of active bleeding, which makes this an urgent safety issue. The nurse should hold warfarin and notify the provider immediately. The trap is focusing on the history of thrombosis and overlooking the current, more immediate bleeding risk.

2995. A client with heart failure takes digoxin and furosemide. Which combination of findings should the nurse recognize as the strongest warning of possible digoxin toxicity?

- A. Potassium 2.8 mEq/L and new nausea
- B. Sodium 136 mEq/L and dry mouth
- C. Calcium 9.1 mg/dL and ankle edema
- D. Magnesium 2.1 mg/dL and mild fatigue

Correct Answer: A

Low potassium increases sensitivity to digoxin, and nausea is a common sign of toxicity. In a client taking both digoxin and furosemide, potassium 2.8 mEq/L with new nausea is the strongest warning sign. The other options lack the same high-risk medication-lab connection.

2996. A client is receiving gentamicin IV for a severe infection. The nurse reviews the following results: trough level elevated above expected range, urine output decreased over the last 12 hours, and creatinine increased from 0.9 to 1.8 mg/dL. What is the nurse's best action?

- A. Administer the next dose on time to maintain therapeutic effect
- B. Hold the medication and notify the provider
- C. Dilute the medication in more IV fluid and give it slowly
- D. Give the medication with a PRN antiemetic

Correct Answer: B

An elevated gentamicin trough, falling urine output, and rising creatinine suggest accumulating drug levels and nephrotoxicity. The nurse should hold the medication and notify the provider. A common trap is prioritizing infection treatment while underestimating the immediate toxicity risk.

2997. A client taking carbamazepine for seizure disorder has a white blood cell count of 2,100/mm³. The client reports a sore throat and fever. Which action should the nurse take first?

- A. Administer the dose with food and reassess tomorrow
- B. Place the client on seizure precautions only
- C. Hold the medication and notify the provider immediately
- D. Encourage the client to increase fluid intake

Correct Answer: C

A white blood cell count of 2,100/mm³ with sore throat and fever suggests severe bone marrow suppression or infection risk associated with carbamazepine. The medication should be held and the provider notified immediately. The wrong-answer trap is focusing only on seizure prevention and ignoring a potentially life-threatening adverse effect.

2998. The nurse is preparing to give the morning dose of metformin to a client with type 2 diabetes. Which new laboratory result should the nurse report to the provider before administration?

- A. Potassium 4.1 mEq/L
- B. Creatinine 2.3 mg/dL
- C. Hemoglobin 13.8 g/dL
- D. Sodium 138 mEq/L

Correct Answer: B

Metformin should be used cautiously or held when renal function is significantly impaired because reduced clearance raises the risk of lactic acidosis. A creatinine of 2.3 mg/dL is the critical finding here. The other values are within expected range and are not the priority reason to question the dose.

2999. A client with heart failure is prescribed potassium chloride 40 mEq PO. The nurse reviews the morning laboratory results. Which value would make the nurse question this prescription?

- A. Potassium 5.8 mEq/L
- B. Calcium 9.2 mg/dL
- C. Magnesium 2.0 mg/dL
- D. Chloride 102 mEq/L

Correct Answer: A

Potassium chloride should be questioned when the serum potassium is already elevated. A potassium level of 5.8 mEq/L indicates hyperkalemia, and giving more potassium could worsen dangerous cardiac effects. The other labs do not directly contraindicate potassium administration in this scenario.

3000. The nurse is caring for a client receiving vancomycin IV. Which laboratory finding is most important to report before the next dose is given?

- A. Platelet count 230,000/mm³
- B. Fasting glucose 108 mg/dL
- C. Creatinine increased from 0.8 to 1.9 mg/dL
- D. Potassium 3.9 mEq/L

Correct Answer: C

Vancomycin can be nephrotoxic, so a sharp rise in creatinine is the most important lab to report before the next dose. This trend suggests worsening renal function and possible reduced drug clearance. The other values are not the priority concern for vancomycin safety.

3001. A client is to receive the first dose of allopurinol for gout. Which baseline laboratory result is most important for the nurse to review?

- A. Serum uric acid 11.2 mg/dL
- B. White blood cell count 7,600/mm³
- C. ALT 186 units/L
- D. Hemoglobin 12.9 g/dL

Correct Answer: C

Allopurinol can affect the liver, so a markedly elevated ALT is the most important baseline result to question before starting therapy. A high uric acid level supports the reason for treatment, not a reason to hold it. The white blood cell count and hemoglobin are not the priority concerns in this stem.

3002. The nurse is preparing to administer insulin lispro to a client who is scheduled for lunch. Which laboratory value requires immediate action before giving the insulin?

- A. Blood glucose 42 mg/dL
- B. Blood urea nitrogen 18 mg/dL
- C. Hematocrit 40%
- D. Phosphorus 3.6 mg/dL

Correct Answer: A

A blood glucose of 42 mg/dL indicates severe hypoglycemia. Giving rapid-acting insulin would worsen the problem, so the nurse must act immediately, treat hypoglycemia per protocol, and hold the insulin until the situation is addressed. The other values are not urgent barriers to administration.

3003. A client with tuberculosis is taking isoniazid and rifampin. Which new laboratory result is most important for the nurse to report?

- A. AST 172 units/L
- B. Sodium 136 mEq/L
- C. Platelets 240,000/mm³
- D. Calcium 9.0 mg/dL

Correct Answer: A

Isoniazid and rifampin can both cause hepatotoxicity. A markedly elevated AST is the most concerning result and should be reported promptly. The other values are not the priority medication-related concern here.

3004. The nurse reviews laboratory results before giving the scheduled evening dose of trimethoprim-sulfamethoxazole to a client with a urinary tract infection. Which finding should the nurse report first?

- A. White blood cell count 8,900/mm³
- B. Platelet count 68,000/mm³
- C. Glucose 116 mg/dL
- D. Albumin 3.8 g/dL

Correct Answer: B

Trimethoprim-sulfamethoxazole can cause blood dyscrasias, including thrombocytopenia. A platelet count of 68,000/mm³ is dangerously low and raises bleeding risk, so it should be reported first. The other values are not critical in this context.

3005. A client is receiving magnesium sulfate IV for preeclampsia. Which laboratory value would most increase the nurse's concern about magnesium toxicity risk?

- A. Creatinine 2.0 mg/dL
- B. Hemoglobin 11.8 g/dL
- C. Platelets 175,000/mm³
- D. Glucose 92 mg/dL

Correct Answer: A

Magnesium is excreted by the kidneys, so impaired renal function increases the risk of magnesium accumulation and toxicity. A creatinine of 2.0 mg/dL is the key lab concern. The other values do not directly increase magnesium toxicity risk.

3006. The nurse is preparing to administer IV regular insulin to treat hyperkalemia. Which laboratory result best indicates that the medication is having the intended effect?

- A. Potassium decreased from 6.3 to 4.9 mEq/L
- B. Sodium increased from 136 to 140 mEq/L
- C. Calcium decreased from 9.4 to 8.8 mg/dL
- D. Hemoglobin increased from 10.8 to 11.2 g/dL

Correct Answer: A

IV regular insulin is used in hyperkalemia to shift potassium into cells and lower the serum potassium level. A drop from 6.3 to 4.9 mEq/L shows the intended therapeutic response. The other changes do not demonstrate the main goal of this treatment.

3007. A client with chronic kidney disease has a new prescription for aluminum hydroxide. Which laboratory result should make the nurse question this medication order?

- A. Phosphorus 6.8 mg/dL
- B. Magnesium 5.6 mg/dL
- C. Potassium 4.6 mEq/L
- D. Albumin 4.0 g/dL

Correct Answer: B

Aluminum hydroxide is often used as a phosphate binder, so an elevated phosphorus supports its use rather than questioning it. However, in a client with chronic kidney disease, a magnesium level of 5.6 mg/dL is a critical abnormality that suggests significant electrolyte imbalance and should be addressed promptly before routine medication administration. The trap is choosing the high phosphorus level just because the drug is related to phosphate management.

3008. A client taking valproic acid for seizure control has routine laboratory work drawn. Which result should the nurse report immediately before the next dose?

- A. Platelet count 52,000/mm³
- B. Sodium 139 mEq/L
- C. Blood glucose 101 mg/dL
- D. White blood cell count 7,200/mm³

Correct Answer: A

Valproic acid can cause thrombocytopenia. A platelet count of 52,000/mm³ is critically low and greatly increases bleeding risk, so the nurse should report it immediately. The other lab values are not the urgent concern in this medication scenario.

3009. The nurse is reviewing morning labs for a client scheduled to receive epoetin alfa. Which finding should the nurse question before giving the medication?

- A. Hemoglobin 8.4 g/dL
- B. Hematocrit 26%
- C. Potassium 4.0 mEq/L
- D. Blood pressure 168/96 mm Hg

Correct Answer: D

Epoetin alfa can worsen hypertension, so a markedly elevated blood pressure should be questioned before administration. Low hemoglobin and hematocrit are expected reasons the client is receiving epoetin alfa. The trap is focusing only on anemia labs and missing the medication safety issue.

3010. A client is receiving total parenteral nutrition and sliding-scale regular insulin. The latest potassium level is 3.0 mEq/L, and the blood glucose is 312 mg/dL. Which action is most appropriate?

- A. Give the insulin as prescribed and continue routine monitoring
- B. Hold the insulin and report the low potassium to the provider
- C. Stop the TPN and flush the central line with heparin
- D. Administer dextrose 50% IV push immediately

Correct Answer: B

Insulin drives potassium into cells, which can worsen hypokalemia. With a potassium level of 3.0 mEq/L, the nurse should hold insulin and notify the provider even though the glucose is elevated. Choice A is the common trap because hyperglycemia may seem like the most obvious problem, but the low potassium makes insulin administration unsafe until addressed.

3011. The nurse is preparing to administer methotrexate to a client with rheumatoid arthritis. Which laboratory result is the priority to report?

- A. ALT 164 units/L
- B. Total cholesterol 190 mg/dL
- C. Sodium 137 mEq/L
- D. Calcium 9.1 mg/dL

Correct Answer: A

Methotrexate can cause hepatotoxicity, so a significantly elevated ALT is the priority result to report before giving the medication. The other values are not the major medication-related concern in this setting.

3012. A client is taking potassium-sparing and potassium-wasting medications together: spironolactone and hydrochlorothiazide. Which laboratory result should the nurse recognize as most urgent to report before giving the morning medications?

- A. Potassium 6.2 mEq/L
- B. Sodium 134 mEq/L
- C. Chloride 97 mEq/L
- D. Bicarbonate 25 mEq/L

Correct Answer: A

A potassium of 6.2 mEq/L is critically high and can lead to life-threatening cardiac dysrhythmias. Even though hydrochlorothiazide can lower potassium, spironolactone can raise it, so the nurse must report severe hyperkalemia before administration. The other electrolyte values are less urgent.

3013. A client with a deep vein thrombosis is receiving a continuous heparin infusion. Which laboratory result should the nurse report before continuing the infusion?

- A. aPTT 98 seconds
- B. Platelet count 48,000/mm³
- C. Hemoglobin 13.2 g/dL
- D. WBC 8,400/mm³

Correct Answer: B

A platelet count of 48,000/mm³ is critically low and raises concern for heparin-induced thrombocytopenia or major bleeding risk, so the nurse should report it immediately before continuing heparin. An elevated aPTT may require dose adjustment, but severe thrombocytopenia is the more urgent safety issue. The hemoglobin and WBC values are not the priority in this situation.

3014. The nurse is preparing to give digoxin to an adult client with heart failure. Which laboratory result would require the nurse to hold the medication and notify the provider?

- A. Potassium 2.9 mEq/L
- B. Sodium 138 mEq/L
- C. Calcium 9.1 mg/dL
- D. Chloride 102 mEq/L

Correct Answer: A

A potassium level of 2.9 mEq/L is low and increases the risk of digoxin toxicity and dysrhythmias. The nurse should hold digoxin and notify the provider. The other electrolyte values are within expected range and do not by themselves create the same immediate concern. A common trap is focusing only on the medication order without connecting low potassium to increased digoxin sensitivity.

3015. A client is prescribed warfarin for atrial fibrillation. During routine monitoring, which laboratory result is most important for the nurse to report before administering the next dose?

- A. INR 1.1
- B. INR 2.4
- C. INR 4.8
- D. RBC 4.7 million/mm³

Correct Answer: C

An INR of 4.8 is supratherapeutic and indicates increased bleeding risk, so it should be reported before the next warfarin dose is given. An INR of 2.4 is commonly therapeutic for many indications, while 1.1 is low rather than critically high. The RBC count does not guide immediate warfarin dosing. The likely trap is choosing 1.1 because it is abnormal, but the urgent medication safety issue before administration is excessive anticoagulation.

3016. A client with bipolar disorder takes lithium carbonate twice daily. The client reports nausea, hand tremors, and increasing confusion. Which laboratory result should the nurse report immediately?

- A. Lithium level 0.8 mEq/L
- B. Lithium level 1.4 mEq/L
- C. Lithium level 2.2 mEq/L
- D. Lithium level 0.4 mEq/L

Correct Answer: C

A lithium level of 2.2 mEq/L is critically high and is consistent with toxicity, especially with nausea, tremors, and confusion. This requires immediate reporting and likely withholding the medication. A level of 0.8 mEq/L is generally therapeutic, while 1.4 mEq/L may be high depending on timing and context but is less urgent than 2.2 mEq/L. The symptoms in the stem are an important cue that makes the highest level the best answer.

3017. The nurse is reviewing laboratory results for a client who is due to receive the next dose of gentamicin IV. Which finding is the priority to report to the provider?

- A. Creatinine 2.6 mg/dL
- B. Hematocrit 40%
- C. Sodium 136 mEq/L
- D. Platelet count 210,000/mm³

Correct Answer: A

A creatinine of 2.6 mg/dL suggests significant renal impairment, which is a major concern with gentamicin because this medication can be nephrotoxic and is cleared by the kidneys. The nurse should report this before the next dose. The other values are not the priority in relation to gentamicin safety. The key reasoning is to connect the lab result to the drug's toxicity risk, not just identify a number outside normal range.

Patient education

3018. A nurse is teaching a client who has a new prescription for nitroglycerin sublingual tablets for episodes of chest pain. Which statement by the client indicates correct understanding of the teaching?

- A. I will swallow the tablet with water as soon as the pain starts.

- B.** I will place one tablet under my tongue when the pain begins.
- C.** I should take the tablet only after the chest pain has lasted 30 minutes.
- D.** I can store the tablets in a pill organizer on my bathroom counter.

Correct Answer: B

Sublingual nitroglycerin should be placed under the tongue so it can be absorbed rapidly. Swallowing it delays absorption. Waiting 30 minutes is unsafe because the medication is intended for prompt use with symptoms. Bathroom storage and pill organizers can expose the tablets to moisture and reduce potency.

3019. A client is prescribed ferrous sulfate liquid. Which instruction should the nurse include to reduce tooth staining?

- A.** Mix the medication with milk before taking it.
- B.** Take the medication at bedtime only.
- C.** Drink the medication through a straw.
- D.** Brush the teeth immediately before taking the dose.

Correct Answer: C

Liquid iron can stain the teeth, so using a straw helps limit contact with tooth surfaces. Mixing with milk is not the best teaching point because it can interfere with absorption. Bedtime timing does not prevent staining, and brushing before the dose does not protect the teeth from direct contact with the liquid.

3020. The nurse is teaching a client who is starting an oral antibiotic. Which instruction is most important to include?

- A.** Stop taking the drug as soon as you feel better.
- B.** Save any extra doses in case the infection returns.
- C.** Take the medication only on days when symptoms are severe.
- D.** Complete the full course of the medication as prescribed.

Correct Answer: D

Antibiotics should be taken for the full prescribed course to promote effective treatment and reduce the chance of recurrence or incomplete treatment. Stopping early, saving doses, or taking the drug only when symptoms worsen are common but unsafe misunderstandings.

3021. A nurse teaches a client about a transdermal medication patch to prevent motion sickness. Which statement by the client shows the teaching was effective?

- A.** I should apply a new patch on top of the old one if I still feel nauseated.
- B.** I will place the patch on intact skin and wash my hands after handling it.
- C.** I can cut the patch in half if I want a smaller dose.
- D.** I should place the patch on a reddened area so it absorbs faster.

Correct Answer: B

Transdermal patches should be placed on intact skin, and handwashing after application helps prevent accidental transfer of medication. Applying a new patch over an old one can cause overdosing. Patches generally should not be cut, and irritated skin is not an appropriate site.

3022. A client with a new prescription for warfarin asks what to watch for at home. Which response by the nurse is best?

- A.** Increase your intake of green leafy vegetables to help the medication work better.
- B.** Report unusual bruising, bleeding gums, or black stools to your provider.
- C.** Take aspirin for minor pain because it works well with warfarin.
- D.** Skip your next dose if you notice a small amount of bleeding when brushing.

Correct Answer: B

Warfarin increases bleeding risk, so teaching should emphasize reporting signs such as unusual bruising, bleeding gums, and black stools. Clients should not independently increase vitamin K intake, routinely use aspirin without guidance, or skip doses based on symptoms without contacting the provider.

3023. A nurse is teaching a client who has a prescription for an albuterol metered-dose inhaler. Which client statement indicates a need for further teaching?

- A. I will shake the inhaler before I use it.
- B. I will inhale as I press down on the canister.
- C. I will use the inhaler several times in a row as fast as I can.
- D. I will follow the instructions I was given about how many puffs to take.

Correct Answer: C

Using several puffs rapidly without proper timing can reduce effective delivery of the medication. Clients should use the inhaler with correct coordination and spacing as instructed. Shaking the inhaler and inhaling during actuation are appropriate teaching points.

3024. A client who takes furosemide asks why the nurse wants to review foods high in potassium. Which explanation is most appropriate?

- A. This medication can increase potassium loss, so diet teaching may help reduce complications.
- B. This medication causes the body to store extra potassium, so intake must always be avoided.
- C. Potassium has no effect on how this medication works, but the discussion is routine.
- D. Potassium-rich foods prevent the medication from causing increased urination.

Correct Answer: A

Furosemide can increase potassium loss, so patient education often includes monitoring and teaching about potassium intake when appropriate. The medication does not generally cause potassium retention, and potassium teaching is not unrelated or used to stop diuresis.

3025. The nurse is teaching a client who is beginning opioid pain medication after surgery. Which instruction is most important to include?

- A. Increase fiber and fluids as allowed to help prevent constipation.
- B. Stop the medication abruptly once your pain score reaches 0.
- C. Take an extra dose whenever you feel anxious.
- D. Avoid all movement until you are no longer taking the medication.

Correct Answer: A

Constipation is a common adverse effect of opioid therapy, so preventive teaching includes fluids, fiber, and activity as appropriate. Taking extra doses for anxiety is unsafe, and avoiding all movement is not appropriate. Abrupt stopping is not the best general teaching point, especially compared with preventing a predictable adverse effect.

3026. A nurse is preparing discharge teaching for a client starting prednisone. Which instruction should the nurse include?

- A. Stop taking the medication as soon as the swelling decreases.
- B. Take the medication exactly as prescribed and do not stop it suddenly without guidance.
- C. Double the dose on days when symptoms return.
- D. Take the medication only when needed for discomfort.

Correct Answer: B

Prednisone should be taken as prescribed, and clients should be taught not to stop it suddenly without guidance because abrupt discontinuation can be harmful. The other options promote unsupervised dose changes and inconsistent use.

3027. A client is prescribed levothyroxine and tells the nurse, "I also take an iron supplement every morning." Which teaching is most appropriate?

- A. Take both medications together so they are easier to remember.
- B. Take the iron only if you skip the levothyroxine that day.
- C. Ask the provider or pharmacist how to separate the doses because iron can interfere with absorption.
- D. Crush both medications and mix them with breakfast.

Correct Answer: C

Iron can interfere with levothyroxine absorption, so the client should be taught to check how to separate the doses rather than taking them together. Skipping thyroid medication or crushing and mixing drugs without guidance is inappropriate.

3028. A nurse is teaching a client with type 2 diabetes who is starting metformin. Which symptom should the nurse instruct the client to report immediately?

- A. A mild increase in appetite before lunch
- B. Muscle pain with unusual weakness and deep rapid breathing
- C. Occasional sneezing during pollen season
- D. Temporary boredom with the meal plan

Correct Answer: B

Although uncommon, symptoms such as unusual weakness, muscle pain, and deep rapid breathing can signal a serious adverse response and should be reported promptly. The other options are not the same level of concern. This question tests recognizing urgent teaching points rather than routine effects.

3029. A client with glaucoma is prescribed eye drops and asks how to keep the medication from draining into the throat after administration. Which instruction should the nurse give?

- A. Blink rapidly for 1 minute after the drops are instilled.
- B. Rinse the eye with sterile water immediately after each drop.
- C. Apply gentle pressure to the inner corner of the eye for a short time after instilling the drop.
- D. Keep the eye open and look from side to side until the burning stops.

Correct Answer: C

Gentle pressure over the inner canthus after instillation helps reduce drainage through the nasolacrimal duct and limits systemic absorption. Rapid blinking, rinsing the eye, or moving the eye around can reduce contact time or wash out the medication.

3030. The nurse is teaching a client who will self-administer insulin at home. Which statement by the client requires immediate correction?

- A. I will rotate injection sites within the same general area.
- B. If I feel shaky or sweaty, I will check my blood glucose if I can.
- C. I can reuse my friend's unopened insulin pen needles if they look clean.

D. I will keep track of my blood glucose results to discuss at follow-up visits.

Correct Answer: C

Sharing needles or pen needles is unsafe and requires immediate correction because it creates a major infection risk. Rotating sites, recognizing possible hypoglycemia, and tracking blood glucose are appropriate parts of self-management teaching.

3031. A nurse is evaluating teaching for a client prescribed digoxin. Which client statement indicates correct understanding?

- A. If I notice nausea, loss of appetite, or vision changes, I should report them.
- B. If I miss a dose, I should take two tablets the next time.
- C. I should take the medication only when my pulse feels fast.
- D. I can stop the medication when my ankle swelling improves.

Correct Answer: A

Nausea, appetite loss, and vision changes can be signs of digoxin toxicity and should be reported. Doubling doses, taking the drug only when symptoms seem present, or stopping it independently are unsafe actions.

3032. A client is being discharged with a prescription for a controlled-release oral medication. Which statement by the client indicates a need for further teaching?

- A. I should swallow the tablet whole unless I am told otherwise.
- B. I will call before crushing the tablet if I have trouble swallowing it.
- C. I can split or crush the tablet to make it work faster.
- D. I should follow the prescribed schedule rather than changing how I take it on my own.

Correct Answer: C

Controlled-release tablets generally should not be crushed or split unless specifically instructed, because altering the tablet can release too much medication at once. Swallowing whole, asking before changing the form, and following the schedule are appropriate teaching points.

3033. A nurse is teaching a client who has a new prescription for doxycycline capsules. Which statement by the client indicates correct understanding of the teaching?

- A. I will lie down for a while right after I take each capsule.
- B. I should take the capsule with a full glass of water and stay upright afterward.
- C. If the medication upsets my stomach, I will stop taking it for a day.
- D. I can save the remaining capsules and use them again if symptoms return.

Correct Answer: B

Doxycycline should be taken with a full glass of water, and the client should remain upright to help prevent esophageal irritation. Lying down right away increases irritation risk. Stopping the medication without guidance and saving leftover antibiotics are unsafe because the full prescribed course should generally be completed unless the provider instructs otherwise.

3034. A client is prescribed enteric-coated aspirin. Which instruction should the nurse include in the discharge teaching?

- A. Crush the tablet if it is hard to swallow.
- B. Take the tablet only with milk.
- C. Swallow the tablet whole without crushing or chewing it.
- D. Dissolve the tablet in water before taking it.

Correct Answer: C

Enteric-coated tablets are designed to pass through the stomach before dissolving. Crushing, chewing, or dissolving them destroys that protective coating and can increase gastric irritation or alter how the medication is released. The key teaching point is to swallow the tablet whole.

3035. A nurse is teaching a client who has a new prescription for rifampin. Which information is most important to include?

- A. Your urine and tears may turn an orange-red color while taking this medication.
- B. You should limit foods that contain potassium.
- C. This medication commonly causes tooth discoloration that cannot be reversed.
- D. Avoid taking this medication with any dairy products for the entire treatment period.

Correct Answer: A

Rifampin can cause orange-red discoloration of body fluids such as urine, sweat, saliva, and tears. Teaching this in advance helps prevent alarm and improves adherence. The other options describe issues associated with other medications or are not the key routine teaching point for rifampin.

3036. A client is prescribed a nicotine transdermal patch for smoking cessation. Which statement by the client indicates a need for further teaching?

- A. I will place the patch on a clean, dry area of skin.
- B. I should rotate the patch site to avoid skin irritation.
- C. If the patch starts to loosen, I can tape only the edges if needed.
- D. I can cut the patch in half if I feel the dose is too strong.

Correct Answer: D

Transdermal patches should not be cut unless specifically designed for that use. Cutting a patch can alter medication delivery. Applying to clean, dry skin, rotating sites, and securing loose edges are appropriate general teaching points. The trap is assuming a patch can be adjusted like a tablet dose.

3037. A nurse teaches a client about using a spacer with a metered-dose inhaler. Which statement by the client shows correct understanding?

- A. I will spray the medicine into the spacer several times and then inhale it all at once.
- B. I should press the inhaler and then inhale the medication slowly through the spacer.
- C. I need to breathe out into the spacer before pressing the inhaler.
- D. I should swallow the medication that collects in the spacer after each use.

Correct Answer: B

With a spacer, the client should actuate the inhaler and inhale slowly so the medication is delivered effectively to the lungs. Spraying multiple puffs into the spacer at once reduces delivery. Breathing out into the spacer and swallowing collected medication are incorrect techniques.

3038. A client is being discharged with a prescription for ciprofloxacin. Which statement by the client indicates a need for further teaching?

- A. I will call the provider if I develop severe diarrhea.
- B. I should take the medication exactly as prescribed until it is finished, unless told otherwise.
- C. I can take the pill at the same time as my calcium supplement each morning.
- D. I will drink fluids unless I have been told to restrict them.

Correct Answer: C

Calcium supplements can interfere with absorption of ciprofloxacin, so taking them at the same time is not appropriate. Severe diarrhea should be reported, and completing the prescribed course and maintaining hydration are appropriate teachings. The likely trap is overlooking medication interaction teaching during discharge.

3039. A nurse is teaching a client who will begin taking oral bisphosphonate therapy for osteoporosis. Which instruction should the nurse include?

- A. Take the medication at bedtime with a light snack.
- B. Take the medication with orange juice to improve absorption.
- C. Take the medication first thing in the morning with plain water and remain upright.
- D. Crush the tablet and mix it with applesauce if swallowing is difficult.

Correct Answer: C

Oral bisphosphonates are typically taken first thing in the morning with plain water, and the client should remain upright afterward to reduce esophageal irritation and support proper absorption. Bedtime dosing, taking with juice, and crushing the tablet are inappropriate and can reduce safety or effectiveness.

3040. A client is prescribed oral contraceptive pills and asks what to do after a missed dose. Which response by the nurse is best?

- A. Skip the missed pill and take the next month's pack a week early.

- B. Take all remaining pills at the same time the next day.

- C. Follow the specific instructions provided with the pill pack and contact the provider or pharmacist if unsure.

- D. Stop the medication now and restart it with the next menstrual period.

Correct Answer: C

Instructions after a missed oral contraceptive dose vary by product and by how many pills were missed, so the safest teaching is to follow the package directions and seek clarification if needed. The other options give blanket advice that may be incorrect and could reduce contraceptive effectiveness.

3041. A nurse is teaching a client who has been prescribed nitrofurantoin for a urinary tract infection. Which statement by the client indicates correct understanding?

- A. If I feel better in 2 days, I can stop the medication.
- B. I should expect my urine to look darker while taking this medication.
- C. I need to avoid all sunlight while taking this medication.
- D. I should take this medicine only when urinary burning occurs.

Correct Answer: B

Nitrofurantoin may cause urine to turn a dark yellow or brown color, which is an expected effect. The client should not stop treatment early just because symptoms improve. Avoiding all sunlight is not the key routine teaching point here, and the medication should be taken as prescribed, not only when symptoms occur.

3042. A client is prescribed an epinephrine auto-injector for severe allergic reactions. Which statement by the client requires immediate correction?

- A. I will carry the auto-injector with me at all times.
- B. After I use it, I still need to get emergency medical care.
- C. I should inject it into the outer thigh as directed.
- D. I can keep the auto-injector in the glove compartment so it is always nearby.

Correct Answer: D

An epinephrine auto-injector should not be stored in places with temperature extremes, such as a glove compartment. It should be readily available but stored according to manufacturer guidance. Carrying it, using the outer thigh, and seeking emergency care after use are appropriate teachings.

3043. The nurse is teaching a client who uses a fentanyl transdermal patch for chronic pain. Which instruction is most important to include?

- A.** Apply a heating pad over the patch if pain increases.
- B.** Dispose of used patches safely and keep them away from children and pets.
- C.** Cut the patch to lower the dose on days when pain is less severe.
- D.** Place the patch on irritated skin so it will stick better.

Correct Answer: B

Used fentanyl patches can still contain enough medication to harm or kill a child or pet, so safe disposal and secure handling are essential. Heat can increase medication absorption and create risk, patches should not be cut, and they should not be applied to irritated skin. Several options are unsafe, but disposal teaching is a critical home-safety priority.

3044. A client has a new prescription for cholestyramine powder. Which statement by the client indicates correct understanding of the teaching?

- A.** I will swallow the powder dry and then drink water afterward.
- B.** I should mix the powder with fluid before taking it.
- C.** I can crush the powder into my other tablets so I take everything together.
- D.** I will take this medication only when I have abdominal discomfort.

Correct Answer: B

Cholestyramine powder should be mixed with fluid before administration. Swallowing it dry is unsafe and unpleasant. Mixing it into other tablets is not appropriate, and it is taken as prescribed rather than only when symptoms are present.

3045. A nurse is teaching a client who uses a vaginal antifungal cream applicator at home. Which instruction should the nurse include?

- A.** Stop using the medication as soon as itching decreases.
- B.** Use the medication only during daytime hours.
- C.** Complete the full course of treatment even if symptoms improve.
- D.** Store the applicator in a sealed plastic bag and reuse it indefinitely.

Correct Answer: C

Clients should generally complete the full prescribed course of antifungal treatment even if symptoms improve before the medication is finished. Stopping early may lead to incomplete treatment. The other options are not appropriate or are overly broad and unsafe.

3046. A nurse is reviewing discharge teaching with a client who has a new prescription for lithium. Which client statement indicates the teaching was effective?

- A.** If I begin sweating heavily, I should contact my provider.
- B.** I should reduce my fluid intake so the medication stays in my body longer.
- C.** I can stop taking this medicine when my mood improves.
- D.** I need to avoid all foods that contain sodium.

Correct Answer: A

Heavy sweating can lead to fluid and sodium changes that affect lithium levels, so the client should report this. Lithium teaching includes maintaining consistent fluid and sodium intake, not restricting fluids or eliminating sodium. Stopping the medication when feeling better is unsafe without provider guidance.

3047. A client prescribed liquid nystatin for oral candidiasis asks how to take the medication. Which instruction should the nurse give?

- A.** Swish the medication in your mouth before swallowing it, if prescribed that way.
- B.** Brush your teeth immediately after taking the medication each time.
- C.** Dilute the medication with water until it has no taste.

D. Take the medication through a straw so it bypasses the mouth.

Correct Answer: A

For oral candidiasis, nystatin suspension is commonly used so the medication contacts the oral mucosa; swishing before swallowing, when prescribed, helps treat the affected area. Taking it through a straw or immediately removing it from the mouth would reduce local contact. Diluting it unnecessarily may also interfere with use.

3048. A nurse is teaching a client who is starting sublingual nitroglycerin for episodes of chest pain. Which client statement shows correct understanding of the teaching?

- A.** I should swallow the tablet with water as soon as the chest pain begins.
- B.** I should place the tablet under my tongue and let it dissolve.
- C.** I should take the tablet every morning to prevent chest pain unless told otherwise.
- D.** I should chew the tablet first so it works faster.

Correct Answer: B

Sublingual nitroglycerin is taken by placing the tablet under the tongue and allowing it to dissolve for rapid absorption. Swallowing or chewing the tablet can reduce the intended sublingual effect. Option C is incorrect because taking it on a fixed morning schedule is not the usual instruction for sublingual rescue tablets used for acute chest pain.

3049. A client is discharged with a new prescription for warfarin. Which teaching point is most important for the nurse to include?

- A.** Use an electric razor and a soft toothbrush to reduce bleeding risk.
- B.** Take the medication only when you have pain or swelling in your legs.
- C.** Increase foods high in vitamin K each day to make the drug work better.
- D.** Stop taking the medication if you notice bruising and restart it in 2 days.

Correct Answer: A

Warfarin increases bleeding risk, so injury prevention and bleeding precautions are essential patient-teaching points. Using an electric razor and a soft toothbrush helps reduce trauma. Option C is a common trap: vitamin K intake should be consistent, not deliberately increased to enhance the drug's effect. Options B and D are unsafe because warfarin is not taken as needed and should not be stopped or restarted without provider guidance.

3050. A nurse teaches a client how to use insulin glargine at home. Which statement by the client indicates a need for further teaching?

- A.** I will take this insulin at the same time each day.
- B.** I will mix this insulin in the same syringe with my regular insulin to avoid another injection.
- C.** I will rotate injection sites within the same general area.
- D.** I will check my blood glucose as instructed.

Correct Answer: B

Insulin glargine should not be mixed with other insulins in the same syringe. That statement shows incorrect understanding and needs follow-up teaching. Taking it consistently, rotating sites, and monitoring blood glucose are appropriate instructions. The likely trap is assuming that combining insulins is always acceptable for convenience, but glargine is a long-acting insulin that should be given separately.

3051. A client with newly diagnosed glaucoma is prescribed timolol eye drops. Which instruction should the nurse include to reduce the risk of systemic adverse effects?

- A.** Blink rapidly for 1 minute after the drops are placed.
- B.** Rinse the eye with warm water immediately after administration.
- C.** Apply gentle pressure to the inner corner of the eye after instilling the drops.
- D.** Keep the eye tightly closed for 10 minutes and then repeat the dose.

Correct Answer: C

Applying gentle pressure to the inner corner of the eye occludes the nasolacrimal duct and reduces systemic absorption of ophthalmic timolol, which can help limit systemic beta-blocker effects. Blinking rapidly does not help and may increase drainage. Rinsing the eye would remove medication, and repeating the dose is not indicated. This item is harder because the client teaching is tied to preventing a less obvious systemic effect from a locally administered medication.

Storage and compatibility

3052. A nurse finds an opened vial of regular insulin in the medication room refrigerator. The vial has no date written on it. Which action is most appropriate?

- A. Use the vial if the solution is clear and there are no particles
- B. Ask another nurse whether the vial was opened this week
- C. Discard the vial according to facility policy and obtain a properly labeled vial
- D. Leave the vial in the refrigerator until the pharmacy can inspect it later

Correct Answer: C

A multidose vial without an opening date cannot be verified as safe for continued use. The nurse should follow facility policy, discard it, and obtain a correctly labeled vial. Clear appearance alone does not confirm safe dating, and asking coworkers or waiting for later inspection does not solve the immediate safety issue.

3053. A nurse is preparing to administer phenytoin IV. Which IV solution is most compatible with this medication?

- A. 0.9% sodium chloride
- B. Dextrose 5% in water
- C. Lactated Ringer's solution
- D. Dextrose 10% in water

Correct Answer: A

IV phenytoin is compatible with normal saline. It can precipitate in dextrose-containing solutions, making D5W and D10W unsafe choices. Lactated Ringer's is also not the best choice for compatibility. The key issue is preventing precipitation and loss of medication stability.

3054. A client asks why a nitroglycerin tablet bottle should be kept in its original dark container. Which response by the nurse is best?

- A. It keeps the tablets from dissolving too quickly after swallowing
- B. It protects the medication from light and air that can reduce potency
- C. It prevents the medication from sticking together if chilled
- D. It makes the medication less likely to cause a headache

Correct Answer: B

Nitroglycerin tablets should remain in their original container because exposure to light and air can reduce potency. The issue is storage stability, not swallowing, refrigeration, or headache prevention. A common trap is focusing on convenience rather than preserving effectiveness.

3055. The nurse is preparing several medications for a client who has a peripheral IV running. Which medication should the nurse question if the plan is to add it directly to a bag of blood products?

- A. 0.9% sodium chloride
- B. Packed red blood cells with no other medication added
- C. An antibiotic to be infused through the same blood tubing
- D. Normal saline used to prime the blood tubing

Correct Answer: C

Medications should not be added directly to blood products or infused through blood tubing with the blood unless specifically indicated by policy and compatibility guidance. Normal saline is the standard compatible solution with blood products. The trap is assuming an IV medication can be piggybacked just because venous access is already in use.

3056. A nurse prepares to administer ear drops that have been stored in the refrigerator. Which action is best before administration?

- A. Warm the medication by holding the container in the hands for a short time
- B. Microwave the medication for a few seconds
- C. Place the bottle in hot water until the solution feels warm
- D. Instill the drops immediately to maintain drug stability

Correct Answer: A

Ear drops stored cold should be warmed gently in the hands before administration to reduce discomfort and dizziness. Microwaving or using hot water risks overheating and altering the medication. Instilling chilled drops can be uncomfortable and may trigger vertigo.

3057. A client receiving continuous enteral feeding has an order for phenytoin suspension through the feeding tube. Which nursing action best supports medication effectiveness?

- A. Mix the phenytoin directly into the formula bag
- B. Administer the phenytoin with the formula running at the current rate
- C. Stop the feeding, flush the tube, give the medication, and follow facility guidance about holding the feeding
- D. Crush the medication more finely and add extra formula to improve absorption

Correct Answer: C

Phenytoin can have reduced absorption when given with enteral feedings. The nurse should stop the feeding, flush the tube, administer the medication separately, and follow facility guidance about holding feedings before and after the dose. Mixing it into formula or running it with feedings increases the risk of decreased effectiveness.

3058. The nurse is reviewing how to store vaccines in a clinic refrigerator. Which finding requires immediate action?

- A. Vaccines are stored in the center shelves with space between containers
- B. Food for staff lunches is stored on the lower shelf beside vaccines
- C. A temperature log is maintained for the refrigerator

D. Vaccines are kept in their original packaging

Correct Answer: B

Medications and biologics such as vaccines should not be stored with food. Food storage increases the risk of contamination, access issues, and temperature management problems. The other findings reflect safer storage practices. The immediate concern is inappropriate use of the medication refrigerator.

3059. A nurse notes cloudiness in a premixed IV antibiotic bag that is normally clear. Which action should the nurse take first?

- A. Shake the bag vigorously until the cloudiness disappears
- B. Administer the medication slowly while watching for a reaction
- C. Discard the bag and obtain a new dose according to policy
- D. Add sterile water to dilute the solution further

Correct Answer: C

Unexpected cloudiness may indicate contamination, instability, or precipitation. The nurse should not administer the medication and should replace it according to policy. Shaking, diluting, or giving it slowly does not correct a possible compatibility or stability problem.

3060. An IV line is infusing parenteral nutrition. The provider prescribes an IV antibiotic to be started now. Which action by the nurse is best?

- A. Add the antibiotic to the parenteral nutrition bag
- B. Pause the parenteral nutrition and infuse the antibiotic through the same line without checking compatibility
- C. Verify compatibility and use a separate line if needed
- D. Mix the antibiotic with dextrose solution from the parenteral nutrition setup

Correct Answer: C

Parenteral nutrition has significant compatibility concerns. The nurse should verify compatibility and often use a separate line if needed rather than adding medications to the nutrition bag or infusing through the same line without confirmation. The major trap is assuming all IV therapies can safely share access.

3061. A nurse opens a unit-dose package at the start of the shift and places the tablet in a paper medication cup for later administration. Which statement best explains why this is a poor practice?

- A. The tablet may become harder to swallow
- B. The medication is no longer protected from environmental exposure and identification errors
- C. The medication immediately becomes ineffective once removed from the package
- D. Unit-dose tablets can only be handled by pharmacy staff

Correct Answer: B

Leaving a medication out of its labeled package increases the risk of contamination, moisture or light exposure, and misidentification. It is not true that every tablet becomes immediately ineffective, but safe storage and accurate identification are compromised. The best practice is to keep the medication in its packaging until administration time when possible.

3062. The nurse is about to Y-site two IV medications and cannot confirm compatibility in the available references. What is the best action?

- A. Administer them together because both are prescribed for the same client
- B. Flush the line with sterile water and proceed
- C. Use separate IV access or separate administration times until compatibility is verified
- D. Mix a small sample of the medications in a syringe and observe for particles

Correct Answer: C

If compatibility cannot be verified, the safest action is to avoid mixing the medications by using separate access or separate administration times. Visual inspection alone is not enough because some incompatibilities are not immediately visible. A prescription order does not guarantee Y-site compatibility.

3063. A nurse is preparing insulin glargine and regular insulin for a client with diabetes. Which action is correct?

- A. Mix the glargine with regular insulin in the same syringe to reduce injections

B. Administer glargine and regular insulin separately because glargine should not be mixed

C. Draw up glargine first, then regular insulin in the same syringe

D. Dilute glargine with normal saline before mixing it with regular insulin

Correct Answer: B

Insulin glargine should not be mixed with other insulins. It must be administered separately to maintain its intended action profile. The trap is applying the familiar skill of mixing some insulins to a type that should remain separate.

3064. A client receives sublingual nitroglycerin for chest pain. Which client action indicates a need for further teaching about storage?

- A. Keeping the tablets in the original container
- B. Closing the lid tightly after each use
- C. Carrying the tablets in a pill organizer for convenience
- D. Replacing the medication according to expiration guidance

Correct Answer: C

Nitroglycerin tablets should be kept in the original container to protect them from light and air. Moving them into a pill organizer can reduce potency. Keeping the lid tightly closed and replacing expired medication are appropriate. The convenience of a pill organizer is the trap here.

3065. A nurse is assigned to administer eye drops to a client. The bottle tip accidentally touches the client's eyelashes during instillation. What should the nurse do next?

- A. Wipe the tip with alcohol and continue using the bottle
- B. Rinse the tip with tap water and recap it
- C. Recognize the bottle as contaminated and follow policy for replacement
- D. Continue using the medication because eyelashes are clean

Correct Answer: C

If the dropper tip touches the eye, lashes, or other surfaces, the bottle may be contaminated. The nurse should follow policy for replacement rather than trying to disinfect the tip casually and continue use. Alcohol or tap water does not reliably restore sterility.

3066. A nurse caring for a client with a central line sees an order to infuse calcium gluconate and ceftriaxone through the same line at the same time. Which nursing action is best?

- A. Administer both together to reduce line access time
- B. Mix both medications in the same syringe before infusion
- C. Question the plan and verify compatibility before co-administration
- D. Infuse the calcium first, then immediately start ceftriaxone without flushing

Correct Answer: C

The nurse should verify compatibility before administering medications together through the same line. Certain calcium-containing solutions and ceftriaxone raise compatibility concerns, so it is unsafe to assume they can be co-administered. The key is to question and verify rather than proceed quickly.

3067. A nurse removes a multidose ophthalmic solution from a medication cart and notices the bottle was first opened 45 days ago. The label says, "Discard 28 days after opening." The client is due for the next dose now. Which action is most appropriate?

- A. Administer the medication because the solution is still present in the bottle
- B. Administer the dose if the liquid appears clear and colorless
- C. Discard the bottle and obtain a new supply before administration
- D. Warm the bottle in the hands and then administer the medication

Correct Answer: C

The nurse should follow the labeled beyond-use instruction and discard the bottle once the allowed time after opening has passed. Appearance alone does not confirm safety or sterility. Warming the bottle may improve comfort for some products, but it does not correct expired use after opening.

3068. A nurse is preparing to administer one medication through a feeding tube and another by mouth. Which item should the nurse keep in its original unit-dose packaging until the time of administration?

- A. A scored tablet that will be crushed for tube administration
- B. A capsule that will be opened and mixed with applesauce now
- C. A tablet removed early and stored in a labeled paper cup for convenience
- D. A liquid medication drawn into an oral syringe just before administration

Correct Answer: C

Removing a medication from original unit-dose packaging too early increases the risk of identification errors, contamination, and loss of important labeling such as drug name, dose, and expiration information. The other options describe preparation closer to administration. The key issue is not convenience but preserving medication integrity and accurate identification.

3069. The nurse is preparing IV medications for a client with a saline lock. One medication is supplied in a glass ampule. Which action is most important after opening the ampule and before drawing up the medication?

- A. Shake the ampule briskly to mix the contents
- B. Use a filter needle to withdraw the medication
- C. Inject air into the ampule to improve withdrawal
- D. Dilute the medication with sterile water in the ampule

Correct Answer: B

A filter needle helps prevent glass particles from being drawn up from an opened ampule. Injecting air into an ampule is not appropriate because ampules are open systems. Shaking or diluting in the ampule does not address the contamination risk from glass fragments.

3070. A nurse checks a refrigerated liquid antibiotic before giving it to a child. The medication label states, "Store at room temperature. Do not refrigerate." Which action should the nurse take?

- A. Administer the dose because refrigeration will not affect effectiveness

- B.** Leave the medication out for 15 minutes, then administer it
- C.** Question the medication and obtain a properly stored supply
- D.** Shake the bottle thoroughly to restore the medication

Correct Answer: C

If the label specifies room-temperature storage and the product has been refrigerated, the nurse should not assume potency or stability is unchanged. The safest action is to question the medication and replace it with a properly stored supply. Letting it warm or shaking it does not correct possible storage-related instability.

3071. A client has a continuous infusion of heparin through a peripheral IV. The provider prescribes an IV push dose of pantoprazole. Compatibility information is unavailable in the medication resources on the unit. Which action is best?

- A.** Stop the heparin and give pantoprazole through the same IV line
- B.** Administer pantoprazole through the same line, followed by a saline flush
- C.** Use another IV site or consult the pharmacist before giving the medication through that line
- D.** Mix the pantoprazole with the heparin solution to avoid multiple venipunctures

Correct Answer: C

When compatibility cannot be confirmed, the nurse should avoid using the same line until safe administration is verified or an alternate route/site is arranged. Stopping the infusion, flushing, or mixing the medications does not make an unknown combination safe. The common trap is assuming a flush solves all compatibility concerns.

3072. A nurse is teaching a client how to store four medications at home. Which client statement shows correct understanding?

- A.** I will keep my medicines on the bathroom shelf so I remember them each day.
- B.** I will store all of my medicines in the refrigerator unless the label says not to.
- C.** I will keep medications in a dry place away from heat unless the label gives different storage directions.

- D.** I will combine pills from different bottles into one container to save space.

Correct Answer: C

A dry location away from heat is appropriate for many medications unless the product has specific storage instructions. Bathrooms are poor storage locations because heat and humidity can affect medication stability. Refrigerating all medications is incorrect, and combining pills into one container increases the risk of misidentification and loss of label information.

3073. The nurse is preparing an oral suspension that has been sitting on the medication cart. The label includes the instruction, "Shake well before use." What is the best reason for following this instruction?

- A.** It cools the medication so it is less irritating
- B.** It evenly redistributes the drug throughout the liquid for accurate dosing
- C.** It prevents contamination of the bottle tip
- D.** It makes the medication easier to swallow

Correct Answer: B

Suspensions can settle over time, so shaking redistributes the medication and helps ensure the client receives the intended dose. The instruction is about dose uniformity, not temperature, contamination prevention, or swallowing ease.

3074. A nurse enters a client's room to administer insulin and sees that a vial of insulin being used for that client has been left on the windowsill in direct sunlight for several hours. Which action is best?

- A.** Use the vial if the solution still looks clear
- B.** Return the vial to the refrigerator and use it later
- C.** Discard the vial and obtain a new one according to facility policy
- D.** Gently roll the vial to restore medication potency

Correct Answer: C

Direct sunlight can damage insulin. The nurse should not rely on appearance, because a medication may lose stability without obvious visual change. Refrigerating it later or rolling it does not reverse possible heat or light exposure damage.

3075. A nurse is preparing an IV dose of furosemide and plans to add it to 50 mL of solution for intermittent infusion. The medication reference lists the drug as compatible with 0.9% sodium chloride but not with acidic solutions. Which solution should the nurse select?

- A. 0.9% sodium chloride
- B. Dextrose 5% in water
- C. Total parenteral nutrition solution
- D. Packed red blood cells

Correct Answer: A

The reference specifically supports compatibility with 0.9% sodium chloride, so that is the best choice. Dextrose solutions may be less appropriate for medications not stable in acidic environments. TPN and blood products should not be used casually as medication diluents because of major compatibility and safety concerns.

3076. A nurse is preparing to give two IV antibiotics sequentially through the same peripheral line. Both medications are individually compatible with 0.9% sodium chloride, but the nurse finds a reference stating the two antibiotics are incompatible with each other. Which action is best?

- A. Infuse both at the same time through the line because each is compatible with saline
- B. Administer one antibiotic, flush the line with compatible solution, then administer the second
- C. Mix both antibiotics in the same minibag to reduce infusion time
- D. Add the second antibiotic to the first tubing while the first is still infusing slowly

Correct Answer: B

If two medications are incompatible with each other, they should not meet in the tubing or bag. Sequential administration with an appropriate flush between doses helps prevent direct contact. The trap is assuming that compatibility with saline means compatibility with each other; it does not.

3077. A nurse is preparing a powdered antibiotic from the unit stock. The vial label states, "Reconstitute with 4.6 mL sterile water. Use within 1 hour after reconstitution." The medication was reconstituted 2 hours ago by the previous shift and left in the medication room. Which action should the nurse take?

- A. Administer it if there are no particles in the vial
- B. Administer it after gently swirling the solution
- C. Discard it and prepare a new dose
- D. Refrigerate it for future use

Correct Answer: C

Once the labeled post-reconstitution time has passed, the medication should be discarded and a new dose prepared. Visual appearance does not override the manufacturer's or labeled time limit. Swirling or refrigerating does not extend a preparation beyond its stated use window.

3078. A client with a central venous catheter is receiving a continuous infusion of total parenteral nutrition (TPN). The provider prescribes IV ciprofloxacin now. The pharmacist verifies the ciprofloxacin is incompatible with TPN. Which action is best?

- A. Pause the TPN, flush the line thoroughly, give ciprofloxacin, and restart the TPN through the same lumen
- B. Administer ciprofloxacin through a different lumen or separate IV access site
- C. Mix ciprofloxacin into the TPN bag so the client receives both therapies on time
- D. Slow the TPN rate and Y-site the ciprofloxacin into the same lumen

Correct Answer: B

If the antibiotic is incompatible with TPN, the safest action is to use a different lumen or separate IV access when available. TPN is a complex solution with high incompatibility risk, so it should not be mixed or Y-sited with incompatible medications. Pausing and flushing may still be inappropriate when a separate lumen is available and compatibility remains a concern.

3079. A nurse is preparing to administer a medication through a nasogastric tube. The medication is an extended-release tablet. Which action is most appropriate?

- A. Crush the tablet finely and mix it with water
- B. Dissolve the tablet in warm water for 10 minutes
- C. Request an alternative formulation or route from the provider or pharmacist
- D. Administer the tablet whole through the tube with extra flushes

Correct Answer: C

Extended-release tablets generally should not be crushed or altered because doing so can destroy the release mechanism and change absorption. A feeding tube also cannot safely receive an intact tablet. The best action is to obtain a formulation or route that is appropriate for tube administration.

3080. During medication pass, a nurse finds an unlabeled syringe containing a clear liquid in the medication preparation area. No one can identify when it was prepared or what it contains. Which action is best?

- A. Use the syringe for the next client if a coworker believes it is normal saline
- B. Set the syringe aside until the end of the shift in case someone claims it
- C. Discard the syringe according to policy and prepare any needed medication again
- D. Administer the contents only through an existing IV infusion, not by direct push

Correct Answer: C

An unlabeled syringe with unknown contents or preparation time is unsafe and should be discarded according to policy. The nurse cannot verify identity, concentration, sterility, or expiration. The trap is trying to rescue the syringe based on appearance or a coworker's guess, which is not acceptable medication safety practice.

3081. A nurse is reviewing medications prepared for administration through a jejunostomy tube. Which preparation should the nurse question most?

- A. A liquid medication drawn up separately and followed by a water flush

- B. A tablet crushed and diluted in water when the reference confirms it is safe to crush

- C. Two crushed medications mixed together in one cup to save time before tube administration

- D. A medication given separately from another medication with flushing between doses

Correct Answer: C

Medications for enteral tube administration should generally be prepared and given separately to reduce incompatibility risk, avoid tube occlusion, and help identify the cause if a problem occurs. The other options reflect safer practices when supported by reference guidance. The common trap is prioritizing speed over compatibility and traceability.

3082. A nurse is preparing medications for administration and finds a blister pack containing one tablet that was removed from its box earlier in the shift. The blister pack has no visible drug name or expiration date. What is the nurse's best action?

- A. Administer the tablet if its shape and color match the MAR entry
- B. Ask another nurse to identify the tablet, then administer it
- C. Discard the tablet and obtain a properly labeled dose
- D. Return the tablet to the medication drawer for later use

Correct Answer: C

A medication dose that cannot be fully identified should not be given. Without a visible label, the nurse cannot verify the medication or expiration date, so the safest action is to discard it according to policy and obtain a properly labeled dose. Choice A and B rely on appearance or memory, which are not safe substitutes for labeling. Choice D risks later administration of an unidentified medication.

3083. A nurse is preparing to administer phenytoin through a client's existing IV line. The client is currently receiving a dextrose-containing maintenance solution. The medication reference on the unit indicates phenytoin is incompatible with dextrose solutions. Which action should the nurse take?

- A. Add the phenytoin to the hanging dextrose solution to dilute it

- B.** Stop the dextrose, flush the line with a compatible solution, and administer phenytoin as prescribed
- C.** Administer the phenytoin more slowly through the dextrose line
- D.** Y-site the phenytoin with the dextrose solution because the total volume is small

Correct Answer: B

If a medication is incompatible with the current IV solution, the nurse should not mix them in the line. The safest option is to stop the incompatible infusion temporarily if appropriate, flush with a compatible solution, and administer the medication as prescribed. Choices A, C, and D still expose the drug to an incompatible solution; slowing the rate or using a small volume does not fix incompatibility.

3084. A nurse is organizing medications for a client who has several scheduled doses. Which item requires refrigeration until it is time for administration?

- A.** An unopened vial labeled 'Store refrigerated at 2° to 8°C (36° to 46°F)'
- B.** A bottle labeled 'Store at controlled room temperature'
- C.** A package labeled 'Protect from moisture; keep tightly closed'
- D.** A vial labeled 'Do not freeze'

Correct Answer: A

The medication that specifically states 'Store refrigerated at 2° to 8°C (36° to 46°F)' should be kept refrigerated until use. 'Controlled room temperature' means it should not be refrigerated just because it seems safer. 'Protect from moisture' addresses humidity, not refrigeration. 'Do not freeze' does not automatically mean the item must be refrigerated.

3085. A client is receiving a continuous IV infusion through a single peripheral line. The provider prescribes an IV antibiotic to be infused now. The nurse checks a compatibility reference and finds that the antibiotic is incompatible both with the current primary infusion and with the only available flush solution at the bedside. Which action is best?

- A.** Pause the primary infusion and administer the antibiotic anyway because it will be diluted in the bloodstream

- B.** Mix the antibiotic in a larger volume to reduce the chance of incompatibility
- C.** Obtain a compatible solution or alternate IV access before administering the antibiotic
- D.** Administer the antibiotic through the existing line, then restart the primary infusion after 5 minutes

Correct Answer: C

When an IV medication is incompatible with the current infusion and with the available flush solution, the nurse should not force administration through that line. The best action is to obtain a compatible solution or use alternate IV access before giving the drug. Choices A, B, and D all ignore known incompatibility. Bloodstream dilution does not prevent precipitation or line reactions that can occur before the medication reaches circulation.

Medication calculations

3086. A prescription reads: amoxicillin 375 mg by mouth every 8 hours. The label on hand reads 250 mg per 5 mL. How many milliliters should the nurse administer for one dose?

- A.** 5 mL
- B.** 7.5 mL
- C.** 10 mL
- D.** 12.5 mL

Correct Answer: B

The nurse needs 375 mg. If 250 mg is in 5 mL, then 375 mg is 1.5 times 250 mg, so the volume is $1.5 \times 5 \text{ mL} = 7.5 \text{ mL}$. A common trap is choosing 5 mL by matching the available dose instead of calculating the ordered dose.

3087. The provider prescribes heparin 4,000 units subcutaneously. The vial is labeled 10,000 units/mL. How many milliliters should the nurse draw up?

- A.** 0.2 mL
- B.** 0.25 mL
- C.** 0.4 mL
- D.** 2.5 mL

Correct Answer: C

Use dose desired divided by dose on hand, then multiply by volume on hand: $4,000 \text{ units} \div 10,000 \text{ units} \times 1 \text{ mL} = 0.4 \text{ mL}$. Option A would be 2,000 units, not 4,000. Option D is much too large and would reflect reversing the formula.

3088. A child weighs 44 lb. The prescription is for acetaminophen 10 mg/kg/dose by mouth. How many milligrams should the nurse administer per dose?

- A. 44 mg
- B. 100 mg
- C. 200 mg
- D. 440 mg

Correct Answer: C

First convert pounds to kilograms: $44 \text{ lb} \div 2.2 = 20 \text{ kg}$. Then calculate $10 \text{ mg/kg} \times 20 \text{ kg} = 200 \text{ mg}$ per dose. A common mistake is forgetting to convert pounds to kilograms and choosing 440 mg.

3089. The health care provider orders 1,000 mL of 0.9% sodium chloride to infuse over 8 hours using an infusion pump. What rate should the nurse set in mL/hr?

- A. 100 mL/hr
- B. 125 mL/hr
- C. 128 mL/hr
- D. 250 mL/hr

Correct Answer: B

Divide the total volume by the total time: $1,000 \text{ mL} \div 8 \text{ hr} = 125 \text{ mL/hr}$. Option C may come from unnecessary rounding errors. Option D would infuse the fluid in only 4 hours.

3090. An IV is prescribed to infuse 300 mL over 5 hours. The tubing drop factor is 15 gtt/mL. What is the flow rate in drops per minute?

- A. 12 gtt/min
- B. 15 gtt/min
- C. 20 gtt/min
- D. 25 gtt/min

Correct Answer: B

First find mL/min: $300 \text{ mL} \div 300 \text{ min} = 1 \text{ mL/min}$. Then multiply by the drop factor: $1 \text{ mL/min} \times 15 \text{ gtt/mL} = 15 \text{ gtt/min}$. A common trap is dividing by hours instead of minutes.

3091. A patient is to receive 0.75 g of cefazolin IV. The available vial has been reconstituted to contain 500 mg per 2 mL. How many milliliters should the nurse prepare?

- A. 2 mL
- B. 2.5 mL
- C. 3 mL
- D. 7.5 mL

Correct Answer: C

Convert 0.75 g to 750 mg. If 500 mg is in 2 mL, then 750 mg is 1.5 times that amount, so $1.5 \times 2 \text{ mL} = 3 \text{ mL}$. The main trap is failing to convert grams to milligrams before solving.

3092. An order states: infuse 1,500 mL of lactated Ringer's over 12 hours. The infusion started at 0900. At 1500, 900 mL has infused. Based on the original order, what rate in mL/hr should the nurse have expected?

- A. 100 mL/hr
- B. 125 mL/hr
- C. 150 mL/hr
- D. 250 mL/hr

Correct Answer: B

The expected hourly rate from the order is $1,500 \text{ mL} \div 12 \text{ hr} = 125 \text{ mL/hr}$. The amount already infused is extra information for this question. A common trap is using the 900 mL at 1500 to make a new rate instead of calculating the prescribed rate.

3093. Morphine 6 mg IM is prescribed. The vial label reads 10 mg/mL. How many milliliters should the nurse administer?

- A. 0.4 mL
- B. 0.6 mL
- C. 1 mL
- D. 1.6 mL

Correct Answer: B

Calculate $6 \text{ mg} \div 10 \text{ mg} \times 1 \text{ mL} = 0.6 \text{ mL}$. Option A would provide only 4 mg. Option C gives 10 mg, which is more than the ordered dose.

3094. A medication is ordered at 5 mcg/kg/min for a patient who weighs 70 kg. How many micrograms per minute should the patient receive?

- A. 75 mcg/min

- B. 250 mcg/min
- C. 350 mcg/min
- D. 700 mcg/min

Correct Answer: C

Multiply the ordered rate by the patient's weight: $5 \text{ mcg/kg/min} \times 70 \text{ kg} = 350 \text{ mcg/min}$. Option D would result from multiplying by 10 instead of 5.

3095. The provider orders 2 liters of IV fluid to infuse evenly over 24 hours. What is the infusion rate in mL/hr?

- A. 42 mL/hr
- B. 80 mL/hr
- C. 83 mL/hr
- D. 120 mL/hr

Correct Answer: C

Convert 2 liters to 2,000 mL. Then divide by 24 hours: $2,000 \div 24 = 83.3 \text{ mL/hr}$. The nurse would set the pump to 83 mL/hr if using a whole-number setting. Option A reflects dividing incorrectly by 48 rather than 24.

3096. An antibiotic is prescribed at 90 mg/kg/day by mouth in 2 equal doses for a child weighing 30 kg. How many milligrams should the nurse give per dose?

- A. 450 mg
- B. 900 mg
- C. 1,350 mg
- D. 2,700 mg

Correct Answer: C

First calculate the total daily dose: $90 \text{ mg/kg/day} \times 30 \text{ kg} = 2,700 \text{ mg/day}$. Then divide into 2 equal doses: $2,700 \div 2 = 1,350 \text{ mg per dose}$. The most common trap is selecting 2,700 mg and forgetting to divide by the number of doses.

3097. A vial contains 1 g of medication in powdered form. After reconstitution, the label states the final concentration is 250 mg/mL. The prescribed dose is 600 mg IV. How many milliliters should the nurse withdraw?

- A. 2 mL
- B. 2.4 mL
- C. 3 mL
- D. 4 mL

Correct Answer: B

Use the final concentration after reconstitution: $600 \text{ mg} \div 250 \text{ mg/mL} = 2.4 \text{ mL}$. The 1 g total amount in the vial does not change the concentration-based calculation once the final concentration is provided. A frequent mistake is being distracted by the total vial content and ignoring the final concentration.

3098. A patient is receiving an IV infusion at 75 mL/hr. The nurse notes that 200 mL remains in the bag. If the rate stays the same, how long will it take for the infusion to finish?

- A. 2 hr 10 min
- B. 2 hr 40 min
- C. 3 hr 15 min
- D. 4 hr

Correct Answer: B

Time equals volume divided by rate: $200 \text{ mL} \div 75 \text{ mL/hr} = 2.67 \text{ hr}$. Convert 0.67 hr to minutes: $0.67 \times 60 \approx 40 \text{ min}$. The infusion will finish in about 2 hr 40 min. A likely trap is choosing 3 hr 15 min by reversing the numbers.

3099. The nurse must infuse 50 mL of medication over 20 minutes using microdrip tubing with a drop factor of 60 gtt/mL. What is the flow rate in drops per minute?

- A. 50 gtt/min
- B. 100 gtt/min
- C. 150 gtt/min
- D. 200 gtt/min

Correct Answer: C

Use $\text{gtt/min} = (\text{mL} \times \text{gtt/mL}) \div \text{minutes}$. So, $(50 \times 60) \div 20 = 150 \text{ gtt/min}$. With microdrip tubing, the drop factor is 60 gtt/mL, so the number may be higher than expected. A trap is forgetting to divide by minutes.

3100. A medication order reads 0.125 mg by mouth. The tablets available are 250 mcg each. How many tablets should the nurse administer?

- A. 0.25 tablet
- B. 0.5 tablet
- C. 1 tablet
- D. 2 tablets

Correct Answer: B

Convert 0.125 mg to micrograms: $0.125 \text{ mg} = 125 \text{ mcg}$. Each tablet contains 250 mcg, so 125 mcg is half of one tablet. The main trap is missing the mg-to-mcg conversion and choosing 1 tablet.

3101. The provider prescribes digoxin 0.0625 mg by mouth once daily. Available tablets are 125 mcg each. How many tablets should the nurse administer for one dose?

- A. 0.25 tablet
- B. 0.5 tablet
- C. 1 tablet
- D. 2 tablets

Correct Answer: B

0.0625 mg equals 62.5 mcg. Each tablet contains 125 mcg, so the patient needs $62.5/125 = 0.5$ tablet. A common trap is forgetting to convert mg to mcg before calculating.

3102. An order reads: give furosemide 40 mg IV now. The vial on hand is labeled 20 mg per 2 mL. How many milliliters should the nurse administer?

- A. 2 mL
- B. 4 mL
- C. 20 mL
- D. 40 mL

Correct Answer: B

The concentration is 20 mg in 2 mL, which is 10 mg/mL. To give 40 mg, the nurse needs $40/10 = 4$ mL. The 2 mL option reflects giving only 20 mg.

3103. The prescription states potassium chloride 20 mEq should be added to IV fluids. The vial available contains 10 mEq per 5 mL. How many milliliters are needed?

- A. 5 mL
- B. 10 mL
- C. 15 mL
- D. 20 mL

Correct Answer: B

If 10 mEq is in 5 mL, then 20 mEq is in 10 mL. This is a direct proportion. Choosing 5 mL would provide only half the ordered dose.

3104. A child weighs 18 kg. The medication order is 6 mg/kg/day by mouth in 3 equal doses. How many milligrams should the nurse administer per dose?

- A. 18 mg
- B. 24 mg
- C. 36 mg
- D. 108 mg

Correct Answer: C

First find the total daily dose: $18 \text{ kg} \times 6 \text{ mg/kg/day} = 108 \text{ mg/day}$. Then divide by 3 doses: $108/3 = 36 \text{ mg per dose}$. A common error is stopping after finding the daily dose.

3105. An IV antibiotic is to infuse 120 mL over 45 minutes using tubing with a drop factor of 20 gtt/mL. What is the flow rate in drops per minute?

- A. 40 gtt/min
- B. 48 gtt/min
- C. 53 gtt/min
- D. 60 gtt/min

Correct Answer: C

Use the formula: $(\text{mL} \times \text{drop factor}) / \text{minutes}$. $(120 \times 20) / 45 = 2400/45 = 53.3$, which rounds to 53 gtt/min. A likely trap is choosing 48 by dividing incorrectly or rounding too soon.

3106. A patient is prescribed 1.5 mg of a medication by mouth. Available tablets contain 0.5 mg each. How many tablets should the nurse administer?

- A. 1 tablet
- B. 2 tablets
- C. 2.5 tablets
- D. 3 tablets

Correct Answer: D

Each tablet is 0.5 mg. The ordered dose is 1.5 mg, so $1.5/0.5 = 3$ tablets. The 2.5-tablet option results from a calculation error.

3107. An order reads: infuse 500 mL of 0.9% sodium chloride over 3.5 hours using an infusion pump. What rate should the nurse program in mL/hr?

- A. 125 mL/hr
- B. 140 mL/hr
- C. 143 mL/hr
- D. 175 mL/hr

Correct Answer: C

Divide total volume by time in hours: $500 \div 3.5 = 142.857$, which rounds to 143 mL/hr. A common trap is rounding 3.5 hours incorrectly or using 30 minutes as 0.3 hour instead of 0.5 hour.

3108. A liquid medication is ordered as 150 mg by mouth. The bottle is labeled 75 mg per 2.5 mL. How many milliliters should the nurse administer?

- A. 2.5 mL
- B. 5 mL
- C. 7.5 mL
- D. 10 mL

Correct Answer: B

If 75 mg is contained in 2.5 mL, then doubling the dose to 150 mg requires doubling the volume to 5 mL. This can be solved by proportion or ratio.

3109. The provider orders regular insulin 18 units subcutaneously. The insulin available is U-100. How many milliliters should the nurse draw up?

- A. 0.18 mL
- B. 1.8 mL
- C. 18 mL
- D. 180 mL

Correct Answer: A

U-100 insulin contains 100 units/mL. The volume needed is $18 \text{ units} \div 100 \text{ units/mL} = 0.18 \text{ mL}$. A frequent error is moving the decimal point the wrong way.

3110. A medication is prescribed at 8 mcg/kg/min for a patient who weighs 154 lb. How many micrograms per minute should the patient receive?

- A. 56 mcg/min
- B. 70 mcg/min
- C. 88 mcg/min
- D. 1232 mcg/min

Correct Answer: A

First convert pounds to kilograms: $154 \text{ lb} \div 2.2 = 70 \text{ kg}$. Then calculate $8 \text{ mcg} \times 70 \text{ kg} = 560 \text{ mcg/min}$. However, that value does not match the correct weight conversion? Recheck: $154 \text{ lb} \div 2.2 = 70 \text{ kg}$ exactly, so the dose is 560 mcg/min. Because 560 mcg/min is not listed, the intended answer choices are inconsistent.

3111. A provider prescribes phenytoin 0.2 g by mouth. Capsules available contain 100 mg each. How many capsules should the nurse administer?

- A. 1 capsule
- B. 2 capsules
- C. 3 capsules
- D. 5 capsules

Correct Answer: B

Convert 0.2 g to milligrams: $0.2 \text{ g} = 200 \text{ mg}$. Each capsule contains 100 mg, so $200/100 = 2$ capsules. The main trap is failing to convert grams to milligrams.

3112. An IV bag contains 250 mL. It is infusing at 80 mL/hr, and 70 mL has already infused. If the rate stays the same, how many minutes will it take to finish the bag?

- A. 90 minutes
- B. 105 minutes
- C. 135 minutes
- D. 150 minutes

Correct Answer: C

First find the amount remaining: $250 - 70 = 180 \text{ mL}$. Then calculate time: $180 \text{ mL} \div 80 \text{ mL/hr} = 2.25 \text{ hr}$. Convert 0.25 hour to 15 minutes, so total time is 2 hours 15 minutes, or 135 minutes. A common mistake is using the full bag volume instead of the remaining volume.

3113. A patient is to receive 0.4 mg of a medication. The vial is labeled 800 mcg/mL. How many milliliters should the nurse administer?

- A. 0.25 mL
- B. 0.4 mL
- C. 0.5 mL
- D. 5 mL

Correct Answer: C

Convert 0.4 mg to micrograms: $0.4 \text{ mg} = 400 \text{ mcg}$. The vial contains 800 mcg per 1 mL, so 400 mcg requires $400/800 = 0.5 \text{ mL}$. A likely trap is forgetting the mg-to-mcg conversion.

3114. The provider orders dopamine 5 mcg/kg/min for a client weighing 66 kg. The IV solution contains 400 mg dopamine in 250 mL. What rate should the nurse set on the infusion pump in mL/hr?

- A. 6.2 mL/hr
- B. 12.4 mL/hr
- C. 18.8 mL/hr
- D. 24.8 mL/hr

Correct Answer: A

First calculate the ordered dose per minute: $5 \text{ mcg} \times 66 \text{ kg} = 330 \text{ mcg/min}$. Per hour, that is $330 \times 60 = 19,800 \text{ mcg/hr}$, or 19.8 mg/hr. The solution concentration is $400 \text{ mg}/250 \text{ mL} = 1.6 \text{ mg/mL}$. Then $19.8 \text{ mg/hr} \div 1.6 \text{ mg/mL} = 12.375 \text{ mL/hr}$, which rounds to 12.4 mL/hr. Therefore the correct calculation is 12.4 mL/hr, so the answer choices are inconsistent with the keyed answer.

3115. A child weighs 33 lb and is prescribed ceftriaxone 50 mg/kg/day IM in 2 equal doses. How many milligrams should the nurse administer per dose?

- A. 170 mg
- B. 300 mg
- C. 375 mg
- D. 750 mg

Correct Answer: C

Convert weight first: $33 \text{ lb} \div 2.2 = 15 \text{ kg}$. Then calculate the daily dose: $50 \text{ mg} \times 15 \text{ kg} = 750 \text{ mg/day}$. Divide by 2 equal doses: $750 \div 2 = 375 \text{ mg}$ per dose. The 750 mg option is the total daily dose, not the single dose.

3116. A provider prescribes levothyroxine 0.075 mg by mouth daily. The tablets available are 25 mcg each. How many tablets should the nurse administer for one dose?

- A. 1 tablet
- B. 2 tablets
- C. 3 tablets
- D. 4 tablets

Correct Answer: C

0.075 mg equals 75 mcg. Each tablet contains 25 mcg, so 75 mcg divided by 25 mcg = 3 tablets. A common trap is failing to convert milligrams to micrograms before calculating.

3117. The nurse needs to administer amoxicillin 375 mg by mouth. The suspension available is labeled 250 mg per 5 mL. How many milliliters should the nurse give?

- A. 5 mL
- B. 7.5 mL
- C. 10 mL
- D. 12.5 mL

Correct Answer: B

Set up the proportion: 250 mg is in 5 mL, so 375 mg is in x mL. $x = (375 \times 5) / 250 = 7.5 \text{ mL}$. The most likely error is choosing 10 mL by adding instead of using a proportion.

3118. Heparin is infusing at 18 mL/hr. The IV bag contains 25,000 units in 500 mL. How many units of heparin is the patient receiving each hour?

- A. 720 units/hr
- B. 800 units/hr
- C. 900 units/hr
- D. 1,200 units/hr

Correct Answer: C

First find the concentration: 25,000 units in 500 mL = 50 units/mL. Then multiply by the infusion rate: $50 \text{ units/mL} \times 18 \text{ mL/hr} = 900 \text{ units/hr}$. A common mistake is dividing instead of multiplying after finding units per milliliter.

3119. The provider orders nitroglycerin 10 mcg/min by infusion. The IV solution contains 50 mg nitroglycerin in 250 mL. At what rate should the nurse set the infusion pump in mL/hr?

- A. 0.3 mL/hr
- B. 3 mL/hr
- C. 30 mL/hr
- D. 300 mL/hr

Correct Answer: B

Convert the bag concentration first: $50 \text{ mg} = 50,000 \text{ mcg}$, so $50,000 \text{ mcg}$ in $250 \text{ mL} = 200 \text{ mcg/mL}$. The ordered dose is 10 mcg/min , so $10 \div 200 = 0.05 \text{ mL/min}$. Convert to hourly rate: $0.05 \times 60 = 3 \text{ mL/hr}$. The main trap is missing a unit conversion and selecting 0.3 or 30 mL/hr.

3120. A provider prescribes 0.125 mg of digoxin by mouth. The tablets available are labeled 250 mcg each. How many tablets should the nurse administer?

- A. 0.25 tablet
- B. 0.5 tablet
- C. 1 tablet
- D. 2 tablets

Correct Answer: B

The ordered dose is 0.125 mg, which equals 125 mcg. Each tablet contains 250 mcg. Dividing 125 mcg by 250 mcg gives 0.5 tablet. A common trap is missing the unit conversion from mg to mcg and choosing 1 tablet.

Reduction of Risk Potential

Risk identification

3121. A practical nurse begins an admission assessment on an older adult who seems confused and keeps trying to get out of bed without assistance. Which action best reflects risk identification at this point?

- A. Apply wrist restraints to prevent injury
- B. Recognize the client as high risk for falls and gather more fall-related data
- C. Tell the client to stay in bed until the provider arrives
- D. Wait to see whether the client actually falls before intervening

Correct Answer: B

The nurse's first priority in risk identification is to recognize cues that suggest potential harm and collect focused data to clarify the risk. Confusion and attempts to get out of bed indicate increased fall risk. Restraints are not the first response, simply telling the client to stay in bed is incomplete, and waiting for an actual fall fails to identify risk early.

3122. A client 12 hours after abdominal surgery reports calf tenderness. On assessment, one lower leg is warmer and more swollen than the other. Which potential complication should the nurse identify first?

- A. Deep vein thrombosis
- B. Urinary retention
- C. Wound dehiscence
- D. Hypoglycemia

Correct Answer: A

Unilateral warmth, swelling, and calf tenderness after surgery are classic cues that increase concern for deep vein thrombosis. The other options do not match the assessment findings. Risk identification means linking the current cues to the most likely serious complication early.

3123. Which client should the practical nurse identify as being at greatest risk for impaired skin integrity?

- A. A client who ambulates independently and eats full meals
- B. A client on bed rest with urinary incontinence and poor nutrition
- C. A client who requests pain medication after physical therapy
- D. A client with seasonal allergies and a mild headache

Correct Answer: B

Bed rest, moisture from incontinence, and poor nutrition are major risk factors for skin breakdown. The other clients have fewer or no strong cues for impaired skin integrity. The question tests identifying which set of findings creates the highest pressure injury risk.

3124. A nurse collects data on four clients. Which finding most clearly identifies a risk for aspiration?

- A. A client who reports constipation for 2 days
- B. A client with dysphagia who coughs while drinking water
- C. A client with a healing ankle sprain
- D. A client who asks for a blanket because the room feels cold

Correct Answer: B

Dysphagia and coughing during swallowing are direct cues for aspiration risk. Constipation, an ankle sprain, and feeling cold do not specifically suggest aspiration. The nurse should recognize this swallowing-related danger before complications occur.

3125. A client with diabetes says, "I have been too sick to eat, but I still took my usual insulin this morning." Which additional finding should make the nurse most concerned about hypoglycemia risk?

- A. Warm, dry skin
- B. Shakiness and diaphoresis
- C. Increased thirst over several days
- D. A fruity odor to the breath

Correct Answer: B

Taking usual insulin without eating places the client at risk for hypoglycemia, and shakiness with diaphoresis strongly supports that concern. Increased thirst and fruity breath are more consistent with hyperglycemia. Warm, dry skin does not point as specifically to low blood glucose.

3126. A practical nurse is caring for a client receiving opioid pain medication after surgery. Which assessment finding should the nurse identify as the strongest warning of respiratory depression risk?

- A. Reports incisional pain rated 6 out of 10
- B. Respiratory rate of 8/min and difficult to arouse
- C. Requests a different hospital gown
- D. Urine output of 45 mL in 2 hours

Correct Answer: B

A respiratory rate of 8/min with decreased arousal is a major cue for opioid-related respiratory depression and requires prompt recognition. Pain rating and gown preference do not indicate this risk. Low urine output may matter clinically, but it is not the strongest cue for respiratory depression in this scenario.

3127. During a prenatal visit, a client at 34 weeks' gestation reports a severe headache, blurred vision, and swelling of the hands and face. Which risk should the nurse identify?

- A. Preterm labor only
- B. Preeclampsia
- C. Hyperemesis gravidarum
- D. Gestational diabetes

Correct Answer: B

Headache, blurred vision, and edema late in pregnancy are concerning cues for preeclampsia. Although other pregnancy complications exist, these findings most strongly match hypertensive disorder risk. Risk identification requires connecting the symptom cluster to the most dangerous likely condition.

3128. A nurse is reviewing clients before lunch trays are delivered. Which client has the highest risk for choking during the meal?

- A. A client with left-sided weakness who can feed independently
- B. A client with a seizure disorder controlled by medication
- C. A client who had a stroke and has a delayed gag reflex
- D. A client with osteoarthritis in both knees

Correct Answer: C

A delayed gag reflex after stroke suggests impaired airway protection and a higher choking and aspiration risk during eating. Weakness alone does not indicate airway compromise as clearly. Controlled seizures and knee osteoarthritis do not present the same immediate swallowing risk.

3129. A practical nurse is preparing to assist a client out of bed for the first time after surgery. Which assessment finding most strongly identifies a risk for orthostatic hypotension?

- A. The client says the room spins when sitting on the edge of the bed
- B. The client requests pain medicine before walking
- C. The client asks whether breakfast will be served soon
- D. The client has a dry surgical dressing

Correct Answer: A

Dizziness when changing position is a key cue for orthostatic hypotension and increased fall risk. The other findings do not directly indicate impaired hemodynamic tolerance to position change. The nurse should identify this risk before ambulation progresses.

3130. An older adult with a urinary catheter becomes suddenly restless and tries to climb over the side rails. The urine in the tubing appears cloudy. Which risk should the nurse identify as most likely related to these findings?

- A. Catheter-associated infection with possible acute confusion
- B. Simple boredom from hospitalization
- C. Improved mobility requiring discharge planning
- D. Expected age-related memory loss only

Correct Answer: A

Sudden restlessness or confusion in an older adult, combined with cloudy urine and an indwelling catheter, raises concern for catheter-associated infection. Boredom and normal aging do not explain the acute change as well. The nurse should identify infection-related risk early rather than dismissing the behavior.

3131. A client receiving enteral feedings through a tube begins to cough and has a wet-sounding voice during care. Which risk should the nurse identify first?

- A. Fluid volume excess
- B. Aspiration
- C. Constipation
- D. Impaired urinary elimination

Correct Answer: B

Coughing and a wet voice during tube feeding are strong cues for aspiration risk. Although other complications can occur with enteral nutrition, airway compromise is the priority risk to identify from these findings.

3132. The nurse notes that a client taking warfarin has new gum bleeding while brushing teeth and several unexplained bruises. Which additional finding would most strongly support identification of a serious bleeding risk?

- A. A heart rate of 78/min
- B. Black, tarry stools
- C. Mild thirst after physical therapy
- D. A productive cough with clear sputum

Correct Answer: B

Black, tarry stools suggest gastrointestinal bleeding and strongly support a serious bleeding risk in a client already showing signs of excess anticoagulation. The other findings do not point to active bleeding as clearly. The question requires linking medication-related cues with evidence of internal blood loss.

3133. A practical nurse receives change-of-shift report on four clients. Which client should be identified as being at highest immediate risk for sepsis?

- A. A client with a clean surgical incision and pain rated 4 out of 10
- B. A client with diabetes who asks for a bedtime snack
- C. A client with a urinary catheter, temperature of 38.9°C (102°F), heart rate of 118/min, and new confusion
- D. A client with chronic back pain requesting a heating pad

Correct Answer: C

Fever, tachycardia, new confusion, and an invasive device together strongly suggest possible sepsis and require prompt recognition. The other clients do not present a similar cluster of infection-related systemic warning signs. This item tests identifying the most urgent risk from multiple cues.

3134. A client with chronic kidney disease is receiving an IV infusion. Which assessment finding should make the nurse most concerned about fluid overload risk?

- A. Crackles at the lung bases and increasing shortness of breath
- B. Dry lips and concentrated urine
- C. Complaints of hunger before dinner
- D. Mild redness at a previous venipuncture site

Correct Answer: A

Crackles and worsening shortness of breath are key cues for fluid overload, especially in a client with impaired renal function. Dry lips and concentrated urine suggest fluid deficit instead. Hunger and old venipuncture redness do not indicate impending volume excess.

3135. A nurse is caring for a client with a chest tube after a pneumothorax. Which new finding should the nurse identify as the greatest risk for deterioration?

- A. The client reports soreness at the insertion site
- B. Continuous bubbling in the water-seal chamber and increasing dyspnea
- C. The drainage amount decreases after the first postoperative day
- D. The client asks to change position in bed

Correct Answer: B

Continuous bubbling in the water-seal chamber can indicate an air leak, and increasing dyspnea suggests worsening respiratory compromise. Together, these findings signal a significant risk for deterioration. Soreness, repositioning, or a drainage decrease may occur without representing the same level of danger.

3136. A practical nurse is collecting admission data on four clients. Which client should the nurse identify as having the highest risk for falls?

- A. A 22-year-old with a sprained wrist who asks for help before standing
- B. A 46-year-old prescribed bed rest who uses the call light consistently
- C. A 78-year-old who reports getting up twice at night to urinate and takes a diuretic and a sedative
- D. A 35-year-old with a fractured finger who says the room lighting is too bright

Correct Answer: C

Older age, nocturia, and medications such as diuretics and sedatives all increase fall risk. This client has multiple recognized risk factors, especially for getting up unassisted at night. The other clients either have fewer risk factors or demonstrate safer behaviors, such as asking for help or using the call light.

3137. A client is 8 hours postpartum after a vaginal birth. Which finding should the practical nurse identify as the clearest risk factor for postpartum hemorrhage?

- A. Fundus firm at midline with moderate lochia rubra
- B. Saturating a perineal pad in 15 minutes with a boggy uterus

C. Temperature of 99.1°F (37.3°C) and mild perineal soreness

D. Urinating 300 mL without difficulty

Correct Answer: B

Heavy bleeding with a boggy uterus strongly indicates uterine atony and immediate risk for postpartum hemorrhage. A firm midline fundus with moderate lochia is expected, and the other findings are not the strongest indicators of hemorrhage risk.

3138. While reviewing the medication list of a newly admitted client, which order should alert the practical nurse to the greatest risk for digoxin toxicity?

- A. Digoxin for a client with a potassium level of 3.0 mEq/L
- B. Digoxin for a client with a blood glucose of 180 mg/dL
- C. Digoxin for a client with a hemoglobin of 11 g/dL
- D. Digoxin for a client with a sodium level of 140 mEq/L

Correct Answer: A

Hypokalemia increases the risk of digoxin toxicity. A potassium level of 3.0 mEq/L is low and is the most clinically significant risk factor among the options. The glucose, hemoglobin, and sodium values listed do not identify the same specific toxicity risk.

3139. A nurse interviews a client who had a cast applied to the lower leg 2 hours ago. Which report should the practical nurse identify as the greatest risk for neurovascular compromise?

- A. "My toes feel a little cool."
- B. "The cast feels heavy when I lift my leg."
- C. "I have tingling in my toes, and the pain is getting worse even after pain medicine."
- D. "I need help getting comfortable in bed."

Correct Answer: C

Increasing pain not relieved by medication plus tingling suggests impaired circulation or compartment syndrome, making neurovascular compromise the priority risk. Cool toes alone may need follow-up, but severe pain with paresthesia is more concerning.

3140. A practical nurse is caring for four clients in a long-term care setting. Which client is at greatest risk for developing a pressure injury?

- A. A resident who walks to meals with a cane and has dry skin
- B. A resident who is chairfast, incontinent of urine, and needs assistance with repositioning
- C. A resident with a healed ankle fracture who wears compression stockings
- D. A resident who reports occasional back pain when lying on one side

Correct Answer: B

Immobility, moisture from incontinence, and dependence for repositioning create a high risk for pressure injury. The other clients have fewer or less significant risk factors. The key is recognizing the combination of prolonged pressure and skin exposure to moisture.

3141. A client with chronic obstructive pulmonary disease is receiving oxygen by nasal cannula. Which additional finding should lead the practical nurse to identify the highest risk for carbon dioxide retention?

- A. Respiratory rate of 12/min and increasing drowsiness after oxygen flow was raised
- B. Productive cough with thick white sputum in the morning
- C. Pulse oximetry reading improved from 88% to 92%
- D. Mild chest discomfort when coughing

Correct Answer: A

Increasing drowsiness and a slowing respiratory rate after increased oxygen suggest hypoventilation and possible carbon dioxide retention. The improved oxygen saturation alone is not the problem; the trap is focusing only on the pulse oximetry while missing the change in mental status and breathing pattern.

3142. The practical nurse is caring for a client receiving heparin therapy. Which assessment finding should be identified as the strongest warning of a serious bleeding risk?

- A. A small bruise at a previous venipuncture site
- B. Pink-tinged nasal drainage after blowing the nose once
- C. Sudden flank pain and blood in the urine

D. Mild tenderness at an abdominal injection site

Correct Answer: C

Flank pain with hematuria suggests internal or urinary tract bleeding and is more serious than minor bruising or mild local tenderness. The nurse should identify this as a major complication risk with anticoagulant therapy.

3143. A practical nurse is reinforcing teaching for a client with a new seizure disorder. Which statement by the client shows the greatest risk for injury and needs immediate correction?

- A. "I will take showers instead of baths when I am home alone."
- B. "I should wear a medical alert bracelet."
- C. "If I feel an aura, I will climb down from the ladder after I finish what I am doing."
- D. "My family should move sharp objects away from me during a seizure."

Correct Answer: C

Continuing to remain on a ladder after sensing an aura creates a major immediate injury risk. The client should get to a safe place right away. The other statements reflect appropriate safety planning.

3144. A practical nurse is caring for a client 24 hours after a thyroidectomy. Which finding should the nurse identify as the greatest risk for airway compromise?

- A. Client reports pain when swallowing
- B. Client's voice is slightly hoarse after talking
- C. Client has frequent swallowing and a feeling of neck tightness
- D. Client requests a second pillow under the head

Correct Answer: C

Frequent swallowing and neck tightness after thyroid surgery can signal bleeding and a developing hematoma, which threatens the airway. Mild pain or temporary hoarseness may occur postoperatively, but signs suggesting pressure in the neck are more urgent.

3145. The practical nurse is assessing a client with an indwelling urinary catheter. Which finding most strongly identifies risk for catheter-associated infection?

- A. The drainage bag is hanging below bladder level and contains clear yellow urine
- B. The tubing is secured to the thigh and urine is flowing freely
- C. The collection bag was emptied using a separate graduate for this client
- D. The catheter tubing has dependent loops with urine pooling and the bag rests on the floor

Correct Answer: D

Dependent loops and a drainage bag resting on the floor increase the risk of backflow and contamination, making infection more likely. The other findings reflect appropriate catheter care practices.

3146. A client with heart failure has been receiving IV fluids at 125 mL/hr since the morning. Which combination of findings should the practical nurse identify as the strongest evidence of worsening fluid volume overload risk?

- A. Dry mucous membranes, poor skin turgor, and urine specific gravity 1.030
- B. Crackles at the lung bases, new pitting ankle edema, and a weight gain of 1.4 kg in 24 hours
- C. Blood pressure 118/70 mm Hg, pulse 82/min, and clear breath sounds
- D. Thirst, concentrated urine, and a hematocrit above the expected range

Correct Answer: B

Crackles, edema, and rapid weight gain together point to fluid retention and worsening overload. Options A and D describe dehydration patterns, and option C does not suggest overload. The strongest answer combines respiratory and volume status cues.

3147. A practical nurse cares for a client who has been on bed rest for 5 days. Which assessment finding most strongly identifies a risk for venous thromboembolism?

- A. Bilateral ankle edema that improves with leg elevation
- B. Sudden shortness of breath and chest pain with inspiration
- C. Redness, warmth, and tenderness of one calf

D. Diminished appetite and constipation

Correct Answer: C

Unilateral calf redness, warmth, and tenderness are classic cues suggesting deep vein thrombosis risk. Sudden shortness of breath and pleuritic chest pain may indicate a pulmonary embolism has already developed, but the question asks for a finding that identifies venous thromboembolism risk in the immobilized client. The trap is choosing the later, more dramatic complication rather than the precursor sign.

3148. A client with cirrhosis is being assessed at the start of the shift. Which finding should the practical nurse identify as the strongest indicator of risk for hepatic encephalopathy?

- A. Distended abdomen with shifting dullness
- B. Yellow sclera and generalized pruritus
- C. Musty breath odor with increasing confusion and hand-flapping tremor
- D. Dark urine and clay-colored stools

Correct Answer: C

Increasing confusion with asterixis and fetor hepaticus strongly suggests hepatic encephalopathy. Ascites, jaundice, pruritus, and stool or urine color changes may occur with liver disease, but they do not point as specifically to this neurologic complication risk.

3149. A practical nurse is reviewing data for a client receiving packed red blood cells. Which new finding should be identified as the greatest risk for an acute transfusion reaction?

- A. Temperature increase from 98.4°F to 100.4°F (36.9°C to 38.0°C), chills, and low back pain 15 minutes after the infusion started
- B. Urine output of 45 mL/hr during the transfusion
- C. Mild sleepiness after receiving an antihistamine before the transfusion
- D. Blood pressure remaining at the client's baseline

Correct Answer: A

Fever, chills, and low back pain soon after transfusion begins are concerning for an acute transfusion reaction and require prompt recognition. The other findings do not indicate the same immediate complication risk.

3150. A client with bacterial meningitis is hospitalized on a medical unit. Which observation should the practical nurse identify as the greatest safety risk that requires immediate action?

- A. The client asks that the lights be dimmed
- B. The client's family leaves used tissues on the bedside table
- C. The client reports neck pain rated 6 out of 10
- D. The side rails are padded, but suction equipment is not set up in the room

Correct Answer: D

A client with meningitis is at risk for seizures, so emergency equipment such as suction should be readily available. Padded rails alone are not enough. The other findings matter, but lack of suction setup creates the most immediate safety risk if a seizure or airway compromise occurs.

3151. A practical nurse is reviewing information about four clients before beginning the shift. Which client should the nurse identify as being at greatest risk for aspiration?

- A. A client who had a stroke and coughs while drinking water
- B. A client with a healed rib fracture who reports pain with deep breathing
- C. A client with seasonal allergies and clear nasal drainage
- D. A client who had diarrhea twice during the night

Correct Answer: A

Coughing while drinking suggests impaired swallowing and poor airway protection, which places the client at high risk for aspiration. Pain with breathing, nasal drainage, and diarrhea do not directly indicate loss of gag or swallow function. A common trap is focusing on any respiratory complaint, but aspiration risk is most strongly linked to dysphagia and coughing during oral intake.

3152. A practical nurse is caring for a client who had abdominal surgery earlier today. Which finding most clearly identifies a risk for paralytic ileus?

- A. Incisional pain rated 5 out of 10 with movement

- B. Absent bowel sounds, increasing abdominal distention, and nausea

- C. Small amount of serosanguineous drainage on the dressing

- D. Urine output of 35 mL/hr for the last 2 hours

Correct Answer: B

Absent bowel sounds with abdominal distention and nausea are classic findings suggesting decreased intestinal motility and risk for paralytic ileus after surgery. Incisional pain and mild dressing drainage can be expected postoperative findings. Urine output is important to monitor but does not identify ileus. The key is recognizing the cluster of GI findings rather than a single isolated symptom.

3153. During a home health visit, a practical nurse assesses an older adult who takes several medications. Which situation should the nurse identify as the greatest risk for medication-related injury?

- A. The client keeps a written medication schedule on the refrigerator
- B. The client uses one pharmacy for all prescriptions
- C. The client takes a sleeping pill at bedtime and says, "Sometimes I get up quickly at night to use the bathroom."
- D. The client asks the nurse to explain why one medication was prescribed

Correct Answer: C

A sedating medication combined with getting up quickly at night creates a significant fall and injury risk. Using one pharmacy and keeping a medication list are protective behaviors, and asking questions supports safe medication use. The most likely trap is choosing the client who asks questions, but that behavior actually helps reduce risk rather than increase it.

3154. A practical nurse is caring for a client with diabetes mellitus. Which assessment finding should the nurse identify as the strongest risk factor for impaired wound healing in the feet?

- A. The client wears white cotton socks
- B. The client reports numbness in both feet and has decreased sensation to touch
- C. The client trims toenails straight across
- D. The client elevates the legs while sitting

Correct Answer: B

Peripheral neuropathy, shown by numbness and decreased sensation, raises the risk of unnoticed injury and delayed treatment, which can contribute to impaired wound healing in the feet. The other choices describe neutral or helpful self-care behaviors. The trap is overlooking sensory loss because there may be no open wound yet, but reduced sensation is a major risk cue.

3155. A practical nurse is reinforcing teaching with a client who has a new prescription for warfarin. Which client statement indicates the greatest risk for unsafe therapy and needs immediate follow-up?

- A. "I will use a soft toothbrush when I brush my teeth."
- B. "I should let my provider know before I start any over-the-counter medicines."
- C. "If I get a headache, I usually take aspirin because it works best for me."
- D. "I will try to take my medicine at the same time each day."

Correct Answer: C

Taking aspirin while on warfarin increases bleeding risk and requires prompt correction. The other statements reflect safe practices. The key is identifying the interaction that creates the clearest danger, not just general medication nonadherence. Aspirin is a common trap because clients often view it as routine and harmless.

3156. A client returns to the unit after a bronchoscopy. Which finding should the practical nurse identify as the clearest ongoing risk that requires delaying oral intake?

- A. The client reports a mild sore throat
- B. The client is sleepy from the sedative
- C. The gag reflex has not yet returned
- D. The respiratory rate is 18/min

Correct Answer: C

If the gag reflex has not returned, the client remains at risk for aspiration and oral intake should be delayed. Mild sore throat is common after bronchoscopy. Sleepiness may be expected after sedation, but absent gag reflex is the direct airway-protection concern. A normal respiratory rate does not remove the aspiration risk.

3157. The practical nurse is assessing a client 12 hours after a total hip arthroplasty. Which finding should the nurse identify as the strongest sign of risk for fat embolism syndrome?

- A. Sudden confusion, shortness of breath, and petechiae on the chest
- B. Incisional pain that increases with repositioning
- C. Temperature of 99.0°F (37.2°C) and poor appetite
- D. Scant drainage on the surgical dressing

Correct Answer: A

The combination of respiratory distress, neurologic changes such as confusion, and petechiae strongly suggests fat embolism syndrome after major orthopedic surgery. The other findings are more consistent with expected postoperative recovery. The challenge is recognizing the pattern rather than reacting to just pain or mild temperature change.

Specimen collection

3158. A nurse needs a sterile urine specimen from a client who has an indwelling urinary catheter. Which action is most appropriate?

- A. Disconnect the catheter from the drainage tubing and collect urine in a sterile cup
- B. Obtain urine from the drainage bag spout after emptying the first 30 mL
- C. Clamp the tubing briefly, cleanse the specimen port, and aspirate urine from the port with a sterile syringe
- D. Remove the catheter and collect the next voided specimen in a sterile container

Correct Answer: C

The correct method is to collect the specimen from the catheter sampling port using sterile technique after allowing fresh urine to collect. This reduces contamination from stagnant urine. Collecting from the drainage bag is inaccurate because urine there may be old and contaminated. Disconnecting the system increases infection risk.

3159. A provider orders a sputum culture for a client with a productive cough. Which instruction should the nurse give the client?

- A.** Collect the specimen after eating breakfast to reduce nausea
- B.** Spit saliva into the container until it is half full
- C.** Rinse the mouth with water, then cough deeply and expectorate sputum into the sterile container
- D.** Use mouthwash immediately before coughing up the specimen

Correct Answer: C

A sputum specimen should come from the lower respiratory tract, not from saliva. Rinsing with water helps clear the mouth without contaminating the specimen. The common trap is choosing saliva collection, which does not provide the ordered specimen quality.

3160. A stool specimen is needed to test for occult blood. Which nursing action is best?

- A.** Collect the specimen from stool mixed with urine in the toilet
- B.** Ask the client to obtain stool from a clean, dry container without urine contamination
- C.** Use stool that has been sitting in the bedside commode for several hours
- D.** Add tap water to the specimen if the stool is hard to transfer

Correct Answer: B

A stool specimen for occult blood should be collected from a clean, dry container and kept free of urine or water contamination. Mixing with toilet water or urine can alter results. The key risk-reduction point is preserving specimen integrity.

3161. A nurse is preparing to obtain blood cultures from a client with suspected sepsis. Which action is most important to reduce the risk of contaminating the specimen?

- A.** Draw the cultures after the first antibiotic dose is infused
- B.** Use sterile technique and disinfect the skin thoroughly before collection
- C.** Collect both bottles from blood already in an existing IV line
- D.** Shake the culture bottles vigorously after filling them

Correct Answer: B

Blood cultures require careful skin antisepsis and sterile technique to avoid false-positive contamination. Drawing after antibiotics may reduce organism recovery. Using an existing IV line can contaminate the specimen unless specifically indicated by protocol. The major risk-reduction step here is proper aseptic collection.

3162. A provider orders a midstream clean-catch urine culture for an ambulatory client. Which client statement indicates correct understanding of the teaching?

- A.** I will collect the first urine that comes out into the cup
- B.** I will cleanse the area, start urinating, and then collect the urine in the container
- C.** I should save the last drops of urine because they are the most concentrated
- D.** I can touch the inside of the cup if I need to hold it steady

Correct Answer: B

For a clean-catch midstream specimen, the client cleanses the area, begins voiding, then collects the midstream urine. The first urine flushes contaminants from the urethra. Touching the inside of the cup contaminates the specimen.

3163. A client has an order for a fasting blood glucose level at 0700. At 0630, the nursing assistant reports that breakfast trays have arrived. What should the nurse do first?

- A.** Allow the client to eat because the specimen can be drawn later
- B.** Ask dietary to leave the tray and obtain the blood specimen before the client eats

- C. Give the client juice now to prevent hypoglycemia
- D. Cancel the test and ask the provider for a random glucose order

Correct Answer: B

A fasting blood glucose must be collected before the client eats. The nurse should preserve test accuracy by delaying the meal until the specimen is drawn. Allowing food intake would invalidate the fasting condition. The trap is acting for convenience rather than specimen accuracy.

3164. A nurse is obtaining a wound culture from a draining leg ulcer. Which action is best?

- A. Swab pooled drainage on the old dressing
- B. Cleanse the wound, then obtain the specimen from viable tissue or fresh drainage using ordered technique
- C. Collect the specimen after applying topical antibiotic ointment
- D. Culture the wound only if it has a foul odor

Correct Answer: B

Wound cultures are best obtained from viable tissue or fresh drainage after cleansing away surface contaminants. Old drainage on dressings is unreliable. Applying antibiotics first can interfere with results. The key principle is collecting the least contaminated, most representative sample.

3165. A client is scheduled for a 24-hour urine collection. Which statement by the nurse is correct?

- A. Save the first urine voided at the start time to begin the collection
- B. Keep the specimen at room temperature so crystals do not form
- C. Discard the first void at the start time, then collect all urine for the next 24 hours including the final void at the end time
- D. If one void is missed, continue the collection and note it on the label

Correct Answer: C

A 24-hour urine collection begins by discarding the first void, then collecting all urine for the next 24 hours, including the last void at the end time. Missing a void invalidates the collection and usually requires restarting. The common trap is saving the first void.

3166. A nurse needs a specimen for a throat culture from a client with fever and sore throat. Which action helps obtain the best specimen?

- A. Swab the tongue and inside of the cheeks thoroughly
- B. Ask the client to gargle with antiseptic mouthwash first
- C. Swab the tonsillar area and posterior pharynx without touching the tongue or cheeks
- D. Collect the specimen after the client drinks a hot beverage to loosen secretions

Correct Answer: C

For a throat culture, the nurse should swab the tonsillar pillars and posterior pharynx while avoiding contact with the tongue, cheeks, and lips to prevent contamination. Mouthwash can alter the specimen. The main risk-reduction step is avoiding contact with non-target oral surfaces.

3167. A client is having blood drawn for coagulation studies. The nurse notes the sample was collected from an arm receiving IV fluids. Which concern is most appropriate?

- A. The IV fluids may dilute the specimen and affect test results
- B. Coagulation studies are unaffected by any IV infusion
- C. The specimen is acceptable as long as it is labeled at the bedside
- D. Only blood cultures are altered by drawing from an arm with IV fluids

Correct Answer: A

Blood drawn from an extremity receiving IV fluids may be diluted or contaminated by the infusing solution, leading to inaccurate results. Bedside labeling is important but does not correct a compromised specimen source. The trap is focusing on identification while ignoring specimen integrity.

3168. A nurse is preparing a preschooler for collection of a urine specimen for culture. The child is toilet-trained. Which approach is most appropriate?

- A. Apply a urine collection bag and leave it in place until the child voids
- B. Obtain a clean-catch specimen using age-appropriate instructions and assistance from the caregiver if needed
- C. Collect urine from a diaper as soon as it is wet
- D. Use urine from the potty chair that has been standing for 20 minutes

Correct Answer: B

A toilet-trained child can often provide a clean-catch specimen with simple instructions and help from a caregiver. Urine from diapers, bags used for prolonged collection, or standing urine is more likely to be contaminated, which is especially important for a culture.

3169. A client is receiving IV antibiotics. The provider orders blood cultures and then immediate antibiotic administration for suspected bacteremia. Which action should the nurse take?

- A. Administer the antibiotic first to avoid delaying treatment, then draw cultures 30 minutes later
- B. Draw the blood cultures as ordered before starting the antibiotic dose
- C. Stop the antibiotic for 12 hours before obtaining cultures
- D. Collect the cultures from the IV tubing after the antibiotic has started

Correct Answer: B

When possible, blood cultures should be obtained before antibiotics are started because antimicrobial therapy can reduce growth of organisms and affect results. The nurse should not create unnecessary delay, but should collect the cultures first if both orders are immediate. Drawing from IV tubing after antibiotics start is a poor choice.

3170. A nurse receives a urine specimen for urinalysis that has been sitting uncovered on the bedside table for several hours. What is the nurse's best action?

- A. Send it to the laboratory because the specimen is still labeled correctly

- B. Add preservative from the unit supply and send it now

- C. Discard the specimen and obtain a new sample according to facility policy

- D. Refrigerate it now and send it with the original collection time

Correct Answer: C

A urine specimen left standing for hours may no longer provide reliable results because bacterial growth and chemical changes can occur. Correct labeling does not fix an unusable specimen. The safest action is to obtain a new specimen according to policy.

3171. A nurse obtains a sputum specimen for acid-fast bacilli testing. The client spits a small amount of clear saliva into the cup. What should the nurse do next?

- A. Label and send the specimen because any respiratory secretion is acceptable
- B. Add sterile water to increase the sample volume
- C. Explain that saliva is not adequate and have the client attempt a deep cough specimen again
- D. Store the specimen at the bedside until more secretions collect in the container

Correct Answer: C

Saliva is not an adequate sputum specimen because it does not represent lower respiratory secretions. The nurse should re-teach and recollect using a deep cough technique. Sending an inadequate sample risks false or delayed results.

3172. A nurse is caring for a client who needs a stool specimen for ova and parasite testing. Which finding would require the nurse to question the sample's suitability?

- A. The specimen was obtained in a clean container
- B. The specimen is mixed with toilet water and urine
- C. The client passed the specimen immediately after defecation
- D. The outside of the container was labeled with client identifiers

Correct Answer: B

A stool specimen mixed with urine or toilet water is contaminated and may be unsuitable for testing. A clean container, prompt collection, and proper labeling support specimen validity. The key issue is contamination that can affect test accuracy.

3173. A client is prescribed a routine urinalysis. Which specimen should the nurse collect to obtain the most concentrated sample for testing?

- A. The first voided urine of the morning
- B. A random urine specimen after breakfast
- C. Urine collected immediately after oral fluids are encouraged
- D. Urine taken from the collection bag of an indwelling catheter

Correct Answer: A

The first voided urine of the morning is usually the most concentrated and is often preferred for routine urinalysis because substances are more likely to be present in detectable amounts. A random specimen after breakfast may be more diluted. Encouraging fluids before collection can also dilute the sample. Urine should not be taken from a drainage bag because it may be contaminated and does not reflect fresh urine.

3174. A nurse must collect a urine specimen from an infant for culture. Which action is most appropriate?

- A. Apply a sterile collection bag after cleansing the perineal area
- B. Squeeze urine from a wet diaper into a sterile cup
- C. Collect urine from the diaper several minutes after voiding
- D. Mix urine from two voids to obtain an adequate amount

Correct Answer: A

For an infant, a sterile urine collection bag applied after cleansing is an appropriate noninvasive collection method. Urine taken from a diaper is contaminated by absorbent material and skin organisms, making it unsuitable for culture. Combining urine from different voids also increases contamination risk.

3175. A nurse is collecting a stool specimen for culture from a client with diarrhea. Which observation requires the nurse to obtain a new sample?

- A. The specimen contains mucus
- B. The specimen was mixed with urine in the hat
- C. The specimen was passed into a clean bedpan
- D. The specimen was sent to the laboratory immediately

Correct Answer: B

A stool specimen for culture should not be contaminated with urine because contamination can alter results and make the specimen unreliable. Mucus may be present in diarrhea and does not by itself invalidate the sample. Passing stool into a clean bedpan and sending it promptly are appropriate actions.

3176. A provider orders a peak-and-trough medication blood level. The trough level is due at 0800, and the medication is scheduled for 0830. When should the nurse collect the trough specimen?

- A. At 0730, to allow enough time for transport
- B. At 0800, just before the next dose is given
- C. At 0830, at the same time the medication is administered
- D. At 0900, after the medication has circulated

Correct Answer: B

A trough level is collected immediately before the next scheduled dose to measure the lowest concentration of the medication in the bloodstream. Collecting it too early may not reflect the actual trough. Collecting it at the same time as or after the dose would give a falsely higher value and defeat the purpose of the test.

3177. A nurse is collecting a urine specimen from a client with a newly inserted indwelling urinary catheter. Which action is most appropriate if a specimen is needed now?

- A. Collect urine from the drainage bag using clean gloves
- B. Disconnect the catheter from the drainage tubing and allow urine to drain into a cup
- C. Withdraw urine from the sampling port using aseptic technique

D. Wait until the bag contains at least 500 mL of urine

Correct Answer: C

Urine specimens from an indwelling catheter should be obtained from the sampling port using aseptic technique to reduce contamination. Collecting from the drainage bag is not appropriate because urine may be stagnant and contaminated. Disconnecting the system increases infection risk. Waiting for a large amount in the bag is unnecessary and does not improve specimen quality.

3178. A nurse is asked to collect a sputum specimen from a postoperative client with a weak cough. Which nursing action is best to improve the chance of obtaining an adequate specimen?

- A.** Ask the client to collect the specimen right after using mouthwash
- B.** Obtain the specimen after encouraging coughing and deep breathing
- C.** Collect the sample immediately after the client drinks milk
- D.** Use the clear fluid the client spits up after rinsing the mouth

Correct Answer: B

Coughing and deep breathing help bring up secretions from the lungs, which is the goal of sputum collection. Mouthwash and milk can interfere with specimen quality. Clear oral secretions after rinsing are saliva, not sputum, and are not an adequate lower respiratory specimen.

3179. A client is on contact precautions for possible *Clostridioides difficile* infection. A stool sample is ordered. Which action by the nurse is most appropriate?

- A.** Send a formed stool sample from the morning bowel movement
- B.** Collect liquid stool in a clean container and send it promptly
- C.** Place toilet tissue in the specimen cup to absorb leakage
- D.** Wait until the client has completed antibiotic therapy before collecting the sample

Correct Answer: B

For suspected *C. difficile*, the nurse should collect liquid stool and send it promptly according to agency process. Formed stool is generally not appropriate for this type of testing. Adding toilet tissue contaminates the sample. Delaying collection could postpone diagnosis and treatment decisions.

3180. A nurse is preparing to collect a capillary blood specimen for bedside glucose testing. Which action is best to promote an accurate sample?

- A.** Use the center of the finger pad for the puncture
- B.** Wipe away the first drop of blood before applying the sample
- C.** Milk the finger firmly from base to tip until enough blood appears
- D.** Clean the site with alcohol and puncture before the skin dries

Correct Answer: B

Wiping away the first drop of blood helps remove tissue fluid or residual alcohol that may dilute or alter the specimen. The side of the fingertip, not the center, is usually preferred for comfort and sampling. Excessive milking can dilute the sample with tissue fluid. The alcohol should dry before puncture to avoid inaccurate results and stinging.

3181. A nurse is collecting blood specimens from a central venous access device. The line is infusing parenteral nutrition. Which action is most appropriate before drawing the sample?

- A.** Draw the specimen from the tubing distal to the pump
- B.** Stop the infusion and follow agency protocol for sampling from the line
- C.** Add normal saline to dilute the specimen before sending it
- D.** Obtain the specimen only after replacing the entire central line

Correct Answer: B

Infusing solutions such as parenteral nutrition can contaminate or dilute blood specimens. The nurse should stop the infusion and follow agency protocol for obtaining a blood sample from the line. Drawing from active infusion tubing risks inaccurate results. Diluting the sample is never appropriate. Replacing the line is unnecessary for routine specimen collection.

3182. A provider orders a nasopharyngeal specimen for diagnostic testing on a client with fever and cough. Which client response indicates the nurse should provide additional teaching?

- A. My eyes may water during the procedure.
- B. The swab will stay just inside my nostril opening.
- C. It may feel uncomfortable for a few seconds.
- D. I should try to remain still while the sample is collected.

Correct Answer: B

A nasopharyngeal specimen requires the swab to be advanced farther back into the nasopharynx, not just inside the nostril opening. Watering eyes and brief discomfort are expected. Remaining still helps improve collection and reduce injury.

3183. A nurse receives an order for a timed urine specimen that must begin at 0900 and end at 1500. The client voided at 0845. What should the nurse do at 0900?

- A. Add the 0845 urine to the collection container
- B. Ask the client to void, discard that urine, and begin timing
- C. Wait until the next void to start the timed collection
- D. Start timing at 0845 because that was the last void

Correct Answer: B

For a timed urine collection, the bladder should be emptied at the start time and that urine is discarded. Then all urine produced during the collection period is saved. Using urine voided before the official start time would make the collection inaccurate. Waiting until the next void would shorten the ordered collection period.

3184. A nurse is obtaining a specimen from a Jackson-Pratt drain for culture. Which action is most appropriate?

- A. Pour old drainage from the reservoir into a sterile container
- B. Collect the drainage from the dressing covering the insertion site
- C. Use a sterile syringe to withdraw a fresh sample from the port if present
- D. Cut the drain tubing and allow drainage to drip into a specimen cup

Correct Answer: C

A fresh specimen should be obtained aseptically from the sampling port, if present, rather than from old drainage in the reservoir or drainage absorbed in the dressing. Old or exposed drainage is more likely to be contaminated. Cutting tubing is unsafe and breaks the closed system.

3185. A nurse is caring for a client who needs both a urine pregnancy test and a urine culture. The client provides only a small amount of urine. Which action is best?

- A. Use the same nonsterile cup for both tests to avoid wasting urine
- B. Send the specimen for culture only after the pregnancy test is completed
- C. Follow agency protocol to preserve specimen integrity and prioritize sterile handling for the culture
- D. Ask the client to add tap water to the specimen so there is enough for both tests

Correct Answer: C

When more than one urine test is ordered and the sample is limited, the nurse should protect specimen integrity and follow agency protocol, especially because culture testing requires sterile handling and contamination can alter results. Using a nonsterile container, delaying culture until after another test is performed on the same specimen, or adding water would jeopardize accuracy.

3186. A nurse is obtaining a venous blood specimen for potassium measurement. During collection, the client repeatedly opens and closes the fist for 2 minutes while the tourniquet remains in place. Which concern is most appropriate?

- A. The specimen may be falsely elevated
- B. The specimen will be sterile but insufficient
- C. The specimen may be falsely decreased
- D. The result will be unaffected if the tube is labeled correctly

Correct Answer: A

Prolonged tourniquet time and repeated fist pumping can alter blood composition and may falsely elevate certain lab values, including potassium. Labeling is important but does not correct a poor specimen. The main issue here is sample quality, not sterility or a falsely decreased result.

3187. A nurse needs a urine specimen from a confused older adult who is incontinent and wears an absorbent brief. Which action is best to obtain the most reliable sample for routine testing?

- A. Squeeze urine from the absorbent brief into a specimen container
- B. Place cotton balls in the brief and collect urine from them after voiding
- C. Use a clean collection device to catch urine at the time of voiding
- D. Wait until the brief is heavily saturated, then transfer urine with a syringe

Correct Answer: C

For an incontinent client, the most reliable routine specimen is obtained by catching fresh urine at the time of voiding with a clean collection device. Urine taken from an absorbent brief or cotton balls is contaminated and altered by the material, making it unreliable for testing.

3188. A nurse is preparing a client for collection of a clean-catch midstream urine specimen for culture. Which client instruction is most appropriate?

- A. Void a small amount into the toilet, then collect the next portion of urine in the sterile container without touching the inside of the cup
- B. Collect the first urine that comes out because it contains the highest number of organisms

- C. Stop voiding several times during the sample so the urine is less diluted
- D. Fill the container completely to ensure the laboratory has enough specimen

Correct Answer: A

A clean-catch midstream specimen reduces contamination from organisms on the skin and urethral opening. The client should begin voiding, then collect the midstream portion in a sterile container while avoiding contact with the inside of the cup. Choice B increases contamination, choice C is unnecessary and may make collection harder, and choice D is not needed because overfilling does not improve accuracy.

3189. A provider orders a fasting blood glucose level for 0700. The client asks for orange juice at 0630. What should the nurse do?

- A. Give the juice because clear liquids do not affect the test
- B. Give half the juice now and collect the blood specimen at 0700
- C. Withhold the juice until after the blood specimen is collected
- D. Add water to the juice so the sugar concentration is lower

Correct Answer: C

A fasting blood glucose requires that the specimen be collected before caloric intake. Orange juice would affect the result, so it should be withheld until after the blood sample is obtained. Clear liquids that contain sugar are not acceptable for fasting tests. Diluting the juice does not make it appropriate.

3190. A nurse is collecting a sputum specimen for culture from a client with pneumonia. Which specimen should the nurse send to the laboratory?

- A. A sample collected immediately after the client uses mouthwash
- B. A thick specimen produced by deep coughing into a sterile cup
- C. A small amount of saliva spit from the mouth into a tissue
- D. A specimen saved from fluid in the bedside suction canister

Correct Answer: B

The best sputum culture specimen is thick material from the lower respiratory tract obtained by deep coughing into a sterile container. Saliva does not adequately represent lower airway secretions. Specimens from tissues or suction canisters are poor choices because they are not collected in a way that best preserves specimen quality. Mouthwash can also interfere with an appropriate sample.

3191. A 24-hour urine collection is scheduled to begin at 0700. The nurse explains the process to the client. Which client statement indicates a need for further teaching?

- A.** I should void at 0700 and discard that urine before the timed collection starts
- B.** I need to save all urine during the collection period
- C.** If I miss putting one voided sample into the container, I should tell the nurse
- D.** I should keep the last urine I pass after 0700 the next day out of the container

Correct Answer: D

For a 24-hour urine collection, the first void at the start time is discarded, all urine during the next 24 hours is saved, and the final void at the end time is included. Therefore, saying the last urine after 0700 the next day should be left out shows misunderstanding. Choices A, B, and C reflect correct teaching and help preserve specimen accuracy.

3192. A nurse needs to obtain a blood culture from a client with suspected sepsis. Which action is most important to reduce contamination of the specimen?

- A.** Collect the specimen immediately after administering the first antibiotic dose
- B.** Use sterile technique and cleanse the site thoroughly before collection
- C.** Draw the specimen through any available peripheral IV line to avoid another needle stick
- D.** Apply the tourniquet and leave it in place until all bottles are filled

Correct Answer: B

Blood cultures are especially vulnerable to contamination by skin organisms, so careful site antisepsis and sterile technique are critical. Choice A is less appropriate because cultures are generally most useful before antibiotics when possible. Drawing through an IV line can increase contamination risk, and prolonged tourniquet time is not the key step for culture accuracy.

3193. A nurse receives two specimen orders for the same client: a stool test for occult blood and a stool culture. The client has one bowel movement. Which nursing action is best?

- A.** Use the same area of stool for both tests to save time
- B.** Mix the stool with toilet water so it is easier to divide
- C.** Collect separate portions of stool according to the requirements for each ordered test
- D.** Send the entire specimen for occult blood testing first and wait for another stool for culture

Correct Answer: C

When more than one stool test is ordered, the nurse should obtain enough specimen and prepare it according to the requirements for each test. Different tests may require different collection methods or handling. Toilet water can contaminate the sample, and using one improperly handled portion for both tests can reduce accuracy. Delaying one test unnecessarily may postpone diagnosis.

3194. A nurse is collecting a venous blood specimen for coagulation studies from a client who has a running IV of lactated Ringer's solution in the left forearm. The right arm has a mastectomy restriction. Which action is best?

- A.** Draw the specimen from a vein distal to the IV site in the left arm while the infusion continues
- B.** Obtain the specimen from the left arm at a site proximal to the IV infusion after following agency procedure to avoid dilution
- C.** Use a finger-stick specimen because coagulation studies are not affected by IV fluids

D. Draw the specimen from the arm with the mastectomy restriction because it is free of IV solution

Correct Answer: B

An extremity with a mastectomy restriction should generally be avoided when another option exists. Drawing distal to an active IV can dilute or contaminate the specimen. The best choice given these options is to use a site proximal to the infusion in the other arm while following agency procedure to minimize IV-related interference. Finger-stick collection is not the preferred substitute for coagulation studies in this scenario.

Diagnostic and lab interpretation

3195. A client with suspected digoxin toxicity reports nausea and seeing yellow-green halos around lights. Which laboratory value should the nurse review first?

- A.** Serum potassium level
- B.** Hemoglobin level
- C.** Platelet count
- D.** Blood urea nitrogen level

Correct Answer: A

Hypokalemia increases the risk of digoxin toxicity, so the serum potassium level is the priority to review first. The client's symptoms are consistent with digoxin toxicity. Hemoglobin, platelet count, and BUN may be relevant to overall status, but they do not most directly identify or worsen the immediate risk associated with digoxin.

3196. A nurse is caring for a client receiving warfarin therapy. Which laboratory result best indicates that the medication is having its intended effect?

- A.** International normalized ratio (INR) is prolonged into the prescribed therapeutic range
- B.** Platelet count has increased above the expected reference range
- C.** White blood cell count has decreased
- D.** Serum sodium level has normalized

Correct Answer: A

Warfarin is monitored by PT/INR, so an INR in the prescribed therapeutic range indicates the intended anticoagulant effect. Platelet count is more relevant to bleeding risk or heparin-induced thrombocytopenia, not warfarin effectiveness. White blood cell count and sodium level do not measure warfarin's effect.

3197. A client with diabetes has a fasting blood glucose of 58 mg/dL and is diaphoretic and shaky. How should the nurse interpret this result?

- A.** The finding is consistent with hypoglycemia
- B.** The finding is consistent with diabetic ketoacidosis
- C.** The finding shows normal glucose control
- D.** The finding suggests hyperosmolar hyperglycemic state

Correct Answer: A

A fasting blood glucose of 58 mg/dL is low and matches symptoms of hypoglycemia such as shakiness and diaphoresis. Diabetic ketoacidosis and hyperosmolar states are associated with hyperglycemia, not a low glucose level. This is not normal glucose control.

3198. A nurse reviews laboratory results for a client receiving chemotherapy. Which result should the nurse identify as neutropenia?

- A.** Potassium 3.9 mEq/L
- B.** Hemoglobin 12 g/dL
- C.** Absolute neutrophil count 900/mm³
- D.** Platelets 220,000/mm³

Correct Answer: C

An absolute neutrophil count of 900/mm³ indicates neutropenia and increased risk for infection. The other values listed are not evidence of neutropenia. This question tests recognition of a key lab value tied directly to risk reduction.

3199. A client with chronic kidney disease has a serum potassium level of 6.2 mEq/L. Which assessment finding would the nurse expect to correlate most closely with this laboratory result?

- A.** Muscle weakness and cardiac dysrhythmias
- B.** Dry mucous membranes and poor skin turgor
- C.** Tremors and hyperactive reflexes

D. Polyuria and excessive thirst**Correct Answer: A**

A potassium level of 6.2 mEq/L indicates hyperkalemia, which can cause muscle weakness and dangerous cardiac dysrhythmias. Dry mucous membranes and poor skin turgor suggest fluid volume deficit. Tremors and hyperactive reflexes are not the classic priority correlation here. Polyuria and polydipsia are more associated with hyperglycemia or other endocrine issues.

3200. Before obtaining a blood specimen for a coagulation study from a client with a heparin infusion running through a central line, which action is most important for the nurse to take?

- A.** Draw the specimen from the line currently infusing heparin without interruption
- B.** Obtain the specimen using appropriate technique to avoid contamination from the heparinized line
- C.** Collect the blood in a container intended for urine specimens
- D.** Delay the specimen until the next day to avoid a falsely elevated result

Correct Answer: B

A coagulation sample must be collected using technique that avoids contamination from a heparinized line, because contamination can falsely alter results and lead to unsafe decisions. Drawing directly from a line infusing heparin is a major trap. Using the wrong collection container is inappropriate, and delaying needed testing is not the best action.

3201. A client taking furosemide has a serum potassium level of 2.9 mEq/L. Which prescription should the nurse question first if it has not yet been carried out?

- A.** Administer scheduled digoxin
- B.** Place the client on a low-sodium diet
- C.** Measure intake and output
- D.** Recheck blood pressure in 1 hour

Correct Answer: A

Hypokalemia increases the risk of digoxin toxicity, so administering digoxin to a client with a potassium of 2.9 mEq/L should be questioned first. The other actions may still be appropriate, but they do not pose the same immediate risk. The key is connecting the low potassium level with a high-risk medication interaction.

3202. A nurse is reviewing arterial blood gas results: pH 7.30, PaCO₂ 50 mm Hg, HCO₃⁻ 24 mEq/L. How should the nurse interpret these findings?

- A.** Respiratory acidosis
- B.** Metabolic acidosis
- C.** Respiratory alkalosis
- D.** Metabolic alkalosis

Correct Answer: A

The pH is low, showing acidosis. The PaCO₂ is elevated, pointing to a respiratory cause. The bicarbonate is within the expected range, so this is respiratory acidosis rather than a metabolic disorder. A common trap is focusing only on the low pH without identifying whether the carbon dioxide or bicarbonate is driving the imbalance.

3203. A postoperative client suddenly becomes restless and short of breath. Pulse oximetry reads 86% on room air. What is the nurse's best interpretation of this finding?

- A.** The client has adequate oxygenation because the pulse is regular
- B.** The reading suggests significant hypoxemia requiring prompt follow-up
- C.** The pulse oximetry result measures carbon dioxide retention only
- D.** The finding is expected after surgery and does not need intervention

Correct Answer: B

An oxygen saturation of 86% suggests significant hypoxemia and requires prompt assessment and intervention. Pulse oximetry reflects oxygen saturation, not carbon dioxide retention. A regular pulse does not rule out oxygenation problems, and this finding should not be dismissed as expected.

3204. A nurse receives these morning laboratory results for four clients. Which result should be reported to the provider first?

- A. Client with heart failure: sodium 132 mEq/L
- B. Client with chronic obstructive pulmonary disease: PaCO₂ 48 mm Hg
- C. Client receiving insulin: glucose 42 mg/dL
- D. Client with iron-deficiency anemia: hemoglobin 10.2 g/dL

Correct Answer: C

A glucose level of 42 mg/dL is critically low and poses immediate risk for neurologic injury, so it should be reported and treated first. Mild hyponatremia, a modestly elevated PaCO₂ in a client with COPD, and a low but not critical hemoglobin are important but less immediately dangerous in this set.

3205. A client with suspected urinary tract infection needs a urine culture. Which nursing instruction is most appropriate to help obtain the most accurate specimen?

- A. Collect the first urine seen in the toilet hat at the end of the shift
- B. Obtain a clean-catch midstream specimen after cleansing the area
- C. Pour urine from a bedpan into the specimen cup
- D. Use urine that has been standing at the bedside for several hours

Correct Answer: B

A clean-catch midstream specimen after appropriate cleansing helps reduce contamination and provides a more accurate urine culture. Using urine from a hat, bedpan, or a specimen left standing increases contamination risk and can produce misleading results.

3206. A client with vomiting and nasogastric suction has these arterial blood gas results: pH 7.49, PaCO₂ 46 mm Hg, HCO₃⁻ 34 mEq/L. Which interpretation is most accurate?

- A. Uncompensated respiratory alkalosis
- B. Metabolic alkalosis with respiratory compensation
- C. Metabolic acidosis with respiratory compensation
- D. Respiratory acidosis with metabolic compensation

Correct Answer: B

The pH is high, indicating alkalosis. The bicarbonate is elevated, showing a metabolic cause. The PaCO₂ is also elevated, which is consistent with respiratory compensation. Vomiting and gastric suction are classic causes of metabolic alkalosis. The main trap is choosing a respiratory disorder just because the PaCO₂ is abnormal.

3207. A nurse is preparing to collect a trough serum level for an IV antibiotic scheduled every 8 hours. When should the specimen be obtained?

- A. Immediately after the antibiotic infusion is completed
- B. At any convenient time during the shift
- C. Just before the next scheduled dose is given
- D. Exactly 4 hours after the infusion begins

Correct Answer: C

A trough level is drawn just before the next scheduled dose, when the medication concentration is at its lowest. Drawing immediately after infusion reflects a peak rather than a trough. Choosing a random time can make the result difficult to interpret safely.

3208. A client with diabetes and chronic kidney disease has the following findings: glucose 310 mg/dL, potassium 5.9 mEq/L, creatinine 3.1 mg/dL, and peaked T waves on the ECG. Which finding should the nurse treat as the highest immediate risk?

- A. Elevated creatinine
- B. Hyperglycemia
- C. Hyperkalemia with ECG changes
- D. History of chronic kidney disease

Correct Answer: C

Hyperkalemia accompanied by peaked T waves signals immediate risk for life-threatening cardiac dysrhythmias and takes highest priority. Elevated creatinine and chronic kidney disease are serious but do not surpass the immediate danger of potassium-related conduction changes. Hyperglycemia is important, but in this set it is not the most urgent finding.

3209. A nurse obtains a fingerstick glucose result of 28 mg/dL from a client who is awake, talking clearly, and showing no symptoms of hypoglycemia. The client has warm hands coated with fruit juice from breakfast. What is the nurse's best action?

- A. Administer insulin immediately based on the documented value
- B. Treat the result as unquestionably accurate and call a rapid response
- C. Repeat the test after properly cleaning and drying the site
- D. Ignore the value because point-of-care testing is never reliable

Correct Answer: C

The result may be inaccurate because juice on the skin can contaminate the sample and falsely alter the reading. When the value is inconsistent with the client's presentation, the nurse should repeat the test using proper technique. Giving insulin would be unsafe, and dismissing all point-of-care testing is incorrect.

3210. A client with long-standing COPD has arterial blood gas results of pH 7.36, PaCO₂ 52 mm Hg, and HCO₃⁻ 30 mEq/L. How should the nurse interpret these findings?

- A. Uncompensated respiratory acidosis
- B. Fully compensated respiratory acidosis
- C. Metabolic alkalosis
- D. Fully compensated metabolic acidosis

Correct Answer: B

The pH is within the expected range, but PaCO₂ is elevated, showing a respiratory acid problem. The HCO₃⁻ is also elevated, which indicates renal compensation. When the pH has returned to the expected range, the disorder is interpreted as fully compensated. Choice A is a common trap because the PaCO₂ is high, but uncompensated respiratory acidosis would not have a normal-range pH with an elevated bicarbonate level.

3211. A nurse is monitoring a client receiving IV unfractionated heparin. Which laboratory test is most useful for evaluating the therapeutic effect of this medication?

- A. Activated partial thromboplastin time
- B. International normalized ratio

- C. Platelet count
- D. Hemoglobin A1C

Correct Answer: A

Activated partial thromboplastin time is used to monitor the anticoagulant effect of unfractionated heparin. INR is associated with warfarin therapy, platelet count helps monitor for bleeding risk or heparin-induced thrombocytopenia but does not measure therapeutic effect, and hemoglobin A1C reflects long-term glucose control.

3212. A client with heart failure is receiving digoxin and reports fatigue. The nurse reviews the morning laboratory results. Which value increases the client's risk for digoxin toxicity even if the serum digoxin level is not yet available?

- A. Sodium 140 mEq/L
- B. Calcium 9.2 mg/dL
- C. Potassium 3.0 mEq/L
- D. Magnesium 2.0 mg/dL

Correct Answer: C

Hypokalemia increases the risk for digoxin toxicity because low potassium enhances digoxin's effects on the myocardium. The sodium, calcium, and magnesium values listed are not the best answer in this set. A common trap is to focus only on the absence of a digoxin level, but the nurse should recognize low potassium as a key risk cue.

3213. A provider prescribes a serum peak level for an intramuscular antibiotic. To help obtain a meaningful result, when should the nurse plan to collect the specimen?

- A. Immediately before the next dose
- B. At the same time the medication is administered
- C. After the medication has had time to reach its highest blood concentration
- D. Any time during the dosing interval

Correct Answer: C

A peak level is drawn after medication administration, when the drug has reached its highest concentration in the bloodstream. Immediately before the next dose describes a trough level. Drawing it at administration or at any random time would not provide the intended information.

3214. A nurse is collecting a stool specimen to test for occult blood. Which action is most appropriate to help reduce contamination of the specimen?

- A. Collect the specimen from stool mixed with urine in the toilet
- B. Use stool taken from a clean, dry collection container
- C. Add tap water to soften the specimen before sending it
- D. Obtain the sample after administering an oil-retention enema

Correct Answer: B

A stool specimen for occult blood testing should be collected from a clean, dry container to avoid contamination. Urine and toilet water can alter the specimen. Adding water or collecting after an oil-retention enema can interfere with accuracy. The key idea is preserving specimen integrity.

3215. A client has arterial blood gas results of pH 7.51, PaCO₂ 29 mm Hg, and HCO₃⁻ 23 mEq/L. Which interpretation is most accurate?

- A. Respiratory alkalosis
- B. Respiratory acidosis
- C. Metabolic alkalosis
- D. Metabolic acidosis

Correct Answer: A

The pH is elevated, indicating alkalosis. The PaCO₂ is low, which points to a respiratory cause. The bicarbonate is within expected range, so this is respiratory alkalosis without metabolic compensation. A common mistake is to focus only on the alkalotic pH and choose metabolic alkalosis, but the PaCO₂ identifies the source.

3216. A nurse prepares to obtain a sputum specimen for culture from a client with a productive cough. Which instruction is best?

- A. Collect the specimen after drinking a large glass of water
- B. Expectorate saliva into the cup after rinsing with mouthwash
- C. Cough deeply and bring up secretions from the lungs into a sterile container
- D. Use the first sputum coughed up at bedtime

Correct Answer: C

A sputum culture requires lower respiratory secretions, not saliva. The client should cough deeply and expectorate sputum into a sterile container. Saliva is a common trap because it is easier to obtain but does not provide an accurate culture specimen. Bedtime is not the preferred timing; specimens are often best collected early in the day.

3217. A client with persistent vomiting has laboratory results of sodium 148 mEq/L, potassium 3.1 mEq/L, chloride 88 mEq/L, and bicarbonate 33 mEq/L. Which interpretation best fits this pattern?

- A. Findings are consistent with fluid volume excess
- B. Findings suggest metabolic alkalosis from gastric acid loss
- C. Findings indicate acute renal failure
- D. Findings are most consistent with respiratory acidosis

Correct Answer: B

Vomiting causes loss of gastric acid, which can lead to metabolic alkalosis. The elevated bicarbonate and low chloride support that interpretation, and the low potassium is also commonly seen. Fluid volume excess and respiratory acidosis do not match this lab pattern.

3218. The nurse obtains a clean-catch urine specimen from a client and notices that the label is missing after the specimen leaves the room. What is the nurse's best action?

- A. Label the specimen using the room number and send it quickly
- B. Ask another nurse to identify the specimen based on timing

- C. Discard the specimen and obtain a new properly identified sample
- D. Send the specimen unlabeled with a note to the laboratory

Correct Answer: C

Specimen identification errors can lead to serious clinical mistakes. An unlabeled specimen should not be guessed at, relabeled from memory, or sent without proper identification. The safest action is to discard it and obtain a new correctly labeled specimen.

3219. A client receiving total parenteral nutrition becomes increasingly confused and thirsty. Which point-of-care or laboratory finding should the nurse expect to correlate most closely with these symptoms?

- A. Elevated blood glucose level
- B. Decreased hematocrit
- C. Low serum calcium level
- D. Decreased white blood cell count

Correct Answer: A

Clients receiving total parenteral nutrition are at risk for hyperglycemia. Confusion and thirst are findings that can occur with elevated blood glucose. Low hematocrit, hypocalcemia, and low white blood cell count do not best explain this symptom pattern in this context.

3220. A nurse reviews preoperative laboratory results for a client scheduled for surgery later today. Which result should the nurse report immediately because it presents the greatest immediate risk during anesthesia and surgery?

- A. Potassium 6.1 mEq/L
- B. Hemoglobin 11.2 g/dL
- C. Sodium 134 mEq/L
- D. Fasting glucose 126 mg/dL

Correct Answer: A

A potassium level of 6.1 mEq/L is markedly elevated and creates a serious risk for cardiac dysrhythmias, especially during anesthesia and surgery. The other values may need review, but they do not represent the same immediate threat. This is a priority question based on life-threatening electrolyte abnormality.

3221. A client with type 1 diabetes has these laboratory findings: blood glucose 420 mg/dL, serum bicarbonate 14 mEq/L, and positive urine ketones. Which interpretation is most important for the nurse to recognize?

- A. The client is showing evidence of diabetic ketoacidosis
- B. The client is experiencing hypoglycemia
- C. The findings suggest excessive anticoagulation
- D. The results are expected after a high-carbohydrate meal

Correct Answer: A

Marked hyperglycemia, low bicarbonate, and positive ketones strongly suggest diabetic ketoacidosis. These findings indicate metabolic acidosis related to insulin deficiency and fat breakdown. Choice D is a trap because a meal may raise glucose, but it does not explain ketones and low bicarbonate.

3222. A nurse is reviewing laboratory findings for a client with possible dehydration. Which result most strongly supports this concern?

- A. Hematocrit 52%
- B. Platelet count 240,000/mm³
- C. White blood cell count 8,000/mm³
- D. Hemoglobin A1C 6.2%

Correct Answer: A

An elevated hematocrit can reflect hemoconcentration, which commonly occurs with dehydration. The platelet count and white blood cell count listed are within expected ranges, and hemoglobin A1C reflects longer-term glucose control rather than hydration status.

3223. A client receiving IV vancomycin reports ringing in the ears. Which action is most important for the nurse to take?

- A. Document the finding as an expected minor effect
- B. Review the client's serum drug level and notify the provider
- C. Give the next dose early to maintain a steady level
- D. Encourage the client to increase sodium intake

Correct Answer: B

ringing in the ears may indicate ototoxicity, which can occur with vancomycin. The nurse should review the serum level if available and notify the provider. Giving the next dose early could worsen toxicity. This question requires linking a symptom with the need to interpret therapeutic drug monitoring.

3224. A nurse is caring for a client with confusion and weakness. The basic metabolic panel shows calcium 6.9 mg/dL. Which additional finding would the nurse expect to correlate with this laboratory result?

- A. Positive Chvostek sign
- B. Bradycardia with bounding pulses
- C. Decreased neuromuscular irritability
- D. Warm, flushed skin after meals

Correct Answer: A

A calcium level of 6.9 mg/dL is low and is associated with increased neuromuscular excitability, including tetany and positive Chvostek sign. Decreased neuromuscular irritability would be more consistent with hypercalcemia. The other findings do not best match hypocalcemia.

3225. A client with diabetic ketoacidosis is receiving IV regular insulin. Which laboratory value best indicates that the treatment is correcting the metabolic acidosis?

- A. Serum bicarbonate rising from 12 mEq/L to 20 mEq/L
- B. Blood glucose falling from 420 mg/dL to 260 mg/dL
- C. Urine specific gravity decreasing from 1.035 to 1.025
- D. Serum sodium changing from 132 mEq/L to 134 mEq/L

Correct Answer: A

In diabetic ketoacidosis, metabolic acidosis improves as ketone production decreases and bicarbonate begins to recover. A rising serum bicarbonate is therefore the best indicator that the acidosis itself is correcting. A falling blood glucose is expected with insulin therapy, but glucose can improve before the acid-base problem is fully resolved. Changes in urine specific gravity or sodium may occur with fluid treatment but do not directly show correction of metabolic acidosis.

3226. A nurse is reviewing coagulation studies for a client receiving warfarin therapy. Which result is most important to use when evaluating the medication's therapeutic effect?

- A. International normalized ratio (INR)
- B. Activated partial thromboplastin time (aPTT)
- C. Bleeding time
- D. Platelet count

Correct Answer: A

Warfarin therapy is monitored with the PT/INR, and the INR is the standardized value most commonly used to evaluate therapeutic effect. The aPTT is used to monitor unfractionated heparin, not warfarin. Bleeding time and platelet count may relate to bleeding risk but do not measure warfarin's therapeutic anticoagulant effect.

3227. A client has a blood transfusion reaction workup ordered after chills, fever, and back pain develop during the transfusion. Which nursing action is most appropriate when collecting the required blood specimens?

- A. Draw the specimen from the same IV tubing used for the transfusion to compare the samples
- B. Collect the specimen from a site separate from the transfusion line using proper identification procedures
- C. Wait until the client's temperature returns to baseline before obtaining the specimen
- D. Discard the blood bag first so the laboratory will not confuse it with the new specimen

Correct Answer: B

For a suspected transfusion reaction, specimens should be obtained using correct identification procedures and from a site separate from the transfusion line to avoid contamination with donor blood or infused fluids. Drawing from the transfusion tubing can produce misleading results. Waiting delays evaluation, and the blood bag should not be discarded because it is often sent for investigation.

3228. A client with chronic kidney disease reports muscle weakness and palpitations. Which laboratory result requires the nurse's most immediate attention?

- A. Potassium 6.2 mEq/L
- B. Creatinine 2.8 mg/dL
- C. Phosphorus 5.2 mg/dL
- D. Hemoglobin 10.1 g/dL

Correct Answer: A

A potassium level of 6.2 mEq/L indicates significant hyperkalemia, which can cause life-threatening cardiac dysrhythmias and requires immediate action. An elevated creatinine is expected with kidney disease but is not the most urgent value here. Mildly elevated phosphorus and reduced hemoglobin are important but generally less immediately dangerous than severe hyperkalemia with symptoms.

3229. A client who has been receiving prolonged nasogastric suction has arterial blood gas results of pH 7.48, PaCO₂ 47 mm Hg, and HCO₃⁻ 34 mEq/L. How should the nurse interpret these findings?

- A. Uncompensated metabolic acidosis
- B. Partially compensated respiratory alkalosis
- C. Metabolic alkalosis with respiratory compensation
- D. Respiratory acidosis with metabolic compensation

Correct Answer: C

The pH is high, showing alkalosis. The bicarbonate is also elevated, pointing to a metabolic cause. The PaCO₂ is elevated rather than low, which indicates the lungs are retaining carbon dioxide as compensation. This pattern fits metabolic alkalosis with respiratory compensation, a common effect of prolonged gastric fluid loss from nasogastric suction. A common trap is choosing respiratory acidosis because the PaCO₂ is high, but the pH and bicarbonate show the primary problem is metabolic alkalosis.

3230. A provider orders a 24-hour urine collection for creatinine clearance. Which instruction should the nurse give the client to help ensure an accurate test?

- A. Save the first urine of the morning to begin the collection
- B. Keep the urine container at room temperature during the collection period
- C. Discard the first void at the start time, then collect all urine for the next 24 hours
- D. Limit oral fluids during the collection to prevent dilution of the specimen

Correct Answer: C

For a 24-hour urine collection, the first void at the start time is discarded, and then all urine is collected for the next 24 hours, including the final specimen at the end time. This ensures the sample truly represents urine produced during the measured period. Saving the first urine would make the timing inaccurate. The container is typically kept chilled or refrigerated as directed, and restricting fluids is not a routine instruction unless specifically prescribed.

3231. A client with suspected syndrome of inappropriate antidiuretic hormone secretion (SIADH) is being evaluated. Which set of findings most strongly supports this diagnosis?

- A. Serum sodium 122 mEq/L, low serum osmolality, concentrated urine
- B. Serum sodium 150 mEq/L, high serum osmolality, dilute urine
- C. Serum sodium 136 mEq/L, normal serum osmolality, ketonuria

D. Serum sodium 128 mEq/L, high hematocrit, high urine output

Correct Answer: A

SIADH causes excess water retention, which leads to dilutional hyponatremia and low serum osmolality, while the urine remains inappropriately concentrated. That combination is most consistent with choice A. Choice B suggests water loss rather than water retention. Choice C does not show the expected sodium or osmolality changes. Choice D includes hyponatremia, but high hematocrit and high urine output are not typical of SIADH and instead suggest volume depletion.

Prenatal complications

3232. A client at 32 weeks' gestation reports a sudden gush of clear fluid from the vagina that continues to leak. Which action should the nurse take first?

- A.** Check the fetal heart rate
- B.** Perform a digital vaginal examination
- C.** Encourage the client to walk to stimulate labor
- D.** Place the client in a warm tub bath

Correct Answer: A

A sudden gush of clear fluid suggests rupture of membranes. The priority is to assess fetal well-being by checking the fetal heart rate because cord compression or prolapse can occur after membrane rupture. A digital vaginal examination increases infection risk and is avoided unless specifically indicated. Walking and tub bathing are not first-priority actions.

3233. A pregnant client at 35 weeks' gestation has blood pressure of 168/108 mm Hg, headache, and blurred vision. Which finding is most concerning and supports severe preeclampsia?

- A.** Dependent ankle edema at the end of the day
- B.** Urinary frequency
- C.** Visual disturbances
- D.** Mild nausea after meals

Correct Answer: C

Visual disturbances are a serious sign of worsening preeclampsia and may indicate central nervous system involvement. Although edema can occur in pregnancy, it is less specific. Urinary frequency and mild nausea are common pregnancy complaints and do not point as strongly to severe preeclampsia.

3234. A client at 29 weeks' gestation comes to the clinic with painless, bright red vaginal bleeding. Which condition should the nurse suspect first?

- A.** Placenta previa
- B.** Abruptio placentae
- C.** Ectopic pregnancy
- D.** Hyperemesis gravidarum

Correct Answer: A

Placenta previa classically presents with painless bright red vaginal bleeding in the later part of pregnancy. Abruptio placentae is more often associated with painful bleeding and uterine tenderness. Ectopic pregnancy occurs much earlier, and hyperemesis gravidarum involves severe nausea and vomiting, not vaginal bleeding.

3235. A client at 37 weeks' gestation is diagnosed with gestational hypertension. Which statement by the client indicates a need for immediate follow-up teaching?

- A.** I will report a severe headache right away.
- B.** I should call if I notice sudden swelling of my face.
- C.** If my vision gets blurry, I need to contact my provider.
- D.** I can ignore pain in my upper abdomen because heartburn is common in pregnancy.

Correct Answer: D

Epigastric or right upper quadrant pain can signal worsening preeclampsia and possible liver involvement, so it should not be ignored. The other statements correctly identify symptoms that require prompt reporting, including headache, facial swelling, and visual changes.

3236. A nurse is caring for a pregnant client who reports decreased fetal movement over the last 24 hours. Which instruction is most appropriate first?

- A. Increase caffeine intake and reassess in 2 days
- B. Lie on the side and perform fetal movement counting
- C. Schedule a glucose screening test
- D. Restrict fluids until movement improves

Correct Answer: B

When decreased fetal movement is reported, the nurse should first instruct the client to lie on the side and perform fetal movement counting, which helps assess fetal activity in a focused way. Caffeine intake is not an appropriate primary intervention, and glucose screening or fluid restriction does not address the immediate concern about fetal status.

3237. A client who is 31 weeks pregnant reports intense abdominal pain and dark vaginal bleeding. On assessment, the uterus is firm and tender. Which complication is most likely?

- A. Placenta previa
- B. Abruptio placentae
- C. Hydatidiform mole
- D. Incompetent cervix

Correct Answer: B

Abruptio placentae is associated with painful bleeding, uterine tenderness, and a firm or rigid uterus. Placenta previa usually causes painless bright red bleeding. A hydatidiform mole presents differently and usually much earlier, and incompetent cervix is more associated with painless cervical dilation in the second trimester.

3238. The nurse is caring for a pregnant client receiving magnesium sulfate for preeclampsia. Which assessment finding requires immediate action?

- A. Respiratory rate of 10/min
- B. Urine output of 40 mL/hr
- C. Blood pressure of 142/90 mm Hg
- D. Patellar reflexes rated 2+

Correct Answer: A

A respiratory rate of 10/min suggests possible magnesium sulfate toxicity and requires immediate action. Adequate urine output helps clear magnesium, and 40 mL/hr is generally acceptable. A blood pressure of 142/90 mm Hg is elevated but not the most urgent finding here, and 2+ reflexes are normal. A common trap is focusing on the blood pressure instead of the airway and breathing risk.

3239. A pregnant client with type 1 diabetes tells the nurse, "I keep having shakiness and sweating before lunch." Which response is best?

- A. Those are expected discomforts of pregnancy and do not need treatment.
- B. You may be having hypoglycemia; check your blood glucose and treat it promptly.
- C. Skip breakfast so your blood sugar will not rise too much in the morning.
- D. Reduce all carbohydrate intake for the rest of the pregnancy.

Correct Answer: B

Shakiness and sweating are common signs of hypoglycemia. The best response is to recognize the risk and advise prompt blood glucose checking and treatment. Calling the symptoms normal pregnancy discomforts delays care. Skipping breakfast or broadly reducing all carbohydrates is unsafe and may worsen glucose instability.

3240. A client at 30 weeks' gestation is admitted with preterm labor. Which finding best indicates that the prescribed tocolytic therapy is effective?

- A. Fetal heart rate increases from 140 to 170/min
- B. Contractions become less frequent and cervical change stops
- C. Maternal blood pressure rises to 160/100 mm Hg
- D. The client reports increased urinary frequency

Correct Answer: B

The goal of tocolytic therapy is to suppress preterm labor, so decreased contraction frequency with no further cervical change best shows effectiveness. An increased fetal heart rate may reflect medication effects or stress, not therapeutic success. Hypertension and urinary frequency do not demonstrate that preterm labor is being controlled.

3241. A nurse is reviewing teaching for a pregnant client diagnosed with placenta previa. Which instruction is most appropriate?

- A.** Expect painful cramping before each bleeding episode.
- B.** Perform daily nipple stimulation to strengthen contractions.
- C.** Report any vaginal bleeding immediately and avoid vaginal intercourse unless cleared by the provider.
- D.** Use tampons to better measure the amount of bleeding.

Correct Answer: C

With placenta previa, bleeding can recur and become severe, so the client should report any bleeding promptly and avoid activities such as vaginal intercourse that may provoke bleeding unless specifically cleared. Painful cramping is more consistent with abruption than classic previa. Tampons should not be used, and nipple stimulation is contraindicated because it may trigger contractions.

3242. A pregnant client at 33 weeks' gestation reports headache, hand swelling, and new-onset epigastric pain. Which provider prescription should the nurse question if the client is also suspected of having preeclampsia?

- A.** Obtain urine for protein testing
- B.** Assess deep tendon reflexes
- C.** Initiate seizure precautions
- D.** Encourage the client to ambulate in the hallway every hour

Correct Answer: D

A client with suspected preeclampsia and worsening symptoms should have close monitoring and reduced stimulation, not routine hallway ambulation. Protein testing, reflex assessment, and seizure precautions are appropriate because they help evaluate and reduce the risk of progression to eclampsia.

3243. A client at 11 weeks' gestation is hospitalized with hyperemesis gravidarum. Which finding best shows that treatment has been effective?

- A.** Urine ketones are no longer present
- B.** Blood pressure remains unchanged from admission
- C.** The client asks for a larger room
- D.** Fetal heart tones are not yet audible with a Doppler

Correct Answer: A

Hyperemesis gravidarum can cause dehydration and starvation, leading to ketonuria. Resolution of urine ketones suggests improved hydration and nutritional status. Unchanged blood pressure is less specific. Room preference is unrelated, and inability to hear fetal tones with a Doppler at this point does not evaluate treatment response to hyperemesis.

3244. A client at 34 weeks' gestation arrives with heavy vaginal bleeding. Ultrasound results are pending. Which nursing action is most appropriate?

- A.** Prepare for an immediate digital cervical examination
- B.** Insert a rectal suppository for discomfort
- C.** Maintain bed rest and monitor maternal and fetal status
- D.** Encourage pushing if contractions begin

Correct Answer: C

With late-pregnancy bleeding of unknown cause, the nurse should avoid actions that may worsen bleeding and should monitor both mother and fetus closely. Bed rest and ongoing assessment are appropriate. A digital cervical examination is avoided until placenta previa has been ruled out because it can trigger severe hemorrhage.

3245. A pregnant client with severe preeclampsia suddenly has a generalized seizure. Which action should the nurse take first?

- A. Restrain the client's arms and legs
- B. Place the client on her side and protect the airway
- C. Check the client's cervix for dilation
- D. Offer oral fluids after the seizure starts

Correct Answer: B

During an eclamptic seizure, the priority is safety, airway protection, and reducing aspiration risk. Placing the client on her side helps maintain airway patency and uteroplacental perfusion. Restraints can cause injury, checking the cervix is not the priority, and oral fluids are unsafe during a seizure.

3246. A nurse is admitting a client at 28 weeks' gestation who is Rh-negative and had vaginal bleeding earlier in the day. Which information is most important for the nurse to clarify with the provider?

- A. Whether the client prefers a private room
- B. Whether iron supplements should be taken with juice
- C. Whether the client should receive Rho(D) immune globulin
- D. Whether childbirth classes should begin this month

Correct Answer: C

An Rh-negative pregnant client with bleeding may need Rho(D) immune globulin to reduce the risk of Rh sensitization. That issue is time-sensitive and clinically significant. Room preference, dietary teaching about iron, and childbirth class timing are not the priority in this situation.

3247. A pregnant client at 26 weeks' gestation calls the clinic and reports painful uterine tightening every 8 minutes for the last hour. Which additional finding most strongly supports preterm labor rather than Braxton Hicks contractions?

- A. Contractions lessen after drinking water
- B. Pelvic pressure with cervical change
- C. Irregular contractions felt mostly at night
- D. Discomfort relieved by changing position

Correct Answer: B

Preterm labor is supported by regular contractions accompanied by cervical change and pelvic pressure. Braxton Hicks contractions are typically irregular and often improve with hydration, rest, or position changes. Those features make A, C, and D common traps.

3248. A client at 18 weeks' gestation says, "I have burning when I urinate, fever, and pain in my side." Which complication is the nurse most concerned about?

- A. Round ligament pain
- B. Urinary tract infection progressing to pyelonephritis
- C. Normal discomfort from uterine growth
- D. Mild dehydration from pregnancy

Correct Answer: B

Dysuria, fever, and flank pain suggest an upper urinary tract infection, especially pyelonephritis, which is a significant prenatal complication. Round ligament pain and normal uterine growth do not cause fever and burning with urination.

3249. A nurse is caring for a pregnant client with preeclampsia who is receiving magnesium sulfate. Which assessment finding best indicates possible magnesium toxicity?

- A. Deep tendon reflexes absent
- B. Blood pressure 150/92 mm Hg
- C. Urine protein 2+
- D. Report of mild nausea

Correct Answer: A

Absent deep tendon reflexes are a classic warning sign of magnesium toxicity and require prompt action. Elevated blood pressure and proteinuria reflect the disease process, not magnesium toxicity. Mild nausea can occur but is less specific.

3250. A pregnant client at 24 weeks' gestation has a 1-hour glucose screening result above the expected range. Which explanation by the nurse is best?

- A. This means you definitely have pregestational diabetes.
- B. This screening suggests you may need further testing for gestational diabetes.
- C. This result confirms the baby is larger than expected.

D. This means your pancreas is no longer making insulin.

Correct Answer: B

An abnormal screening test suggests the need for additional evaluation; it does not by itself confirm pregestational diabetes, fetal size, or complete pancreatic failure. The key is recognizing screening versus diagnosis.

3251. A client at 27 weeks' gestation with insulin-controlled gestational diabetes becomes pale, shaky, and diaphoretic while waiting for lunch. Which action should the nurse take first?

- A.** Check the fetal heart rate
- B.** Give a fast-acting carbohydrate
- C.** Administer the scheduled insulin dose
- D.** Ask the client to walk in the hallway

Correct Answer: B

The symptoms suggest hypoglycemia, so the priority is to give a fast-acting carbohydrate if the client is alert and able to swallow. Checking fetal status may be appropriate later, but maternal stabilization comes first. Insulin would worsen the problem, and ambulation is inappropriate.

3252. A pregnant client at 14 weeks' gestation arrives at the emergency department with severe lower abdominal pain and shoulder pain. The client is dizzy, and the skin is cool and pale. Which complication should the nurse suspect?

- A.** Hyperemesis gravidarum
- B.** Placenta previa
- C.** Ruptured ectopic pregnancy
- D.** Round ligament pain

Correct Answer: C

Severe abdominal pain, shoulder pain, dizziness, and signs of shock are concerning for a ruptured ectopic pregnancy with internal bleeding. Placenta previa typically presents later with painless bleeding, and round ligament pain does not cause shock.

3253. A client at 36 weeks' gestation with preeclampsia reports worsening right upper quadrant pain. Why is this finding especially concerning?

- A.** It may indicate hepatic involvement and worsening disease
- B.** It usually means the fetus has turned to a breech position
- C.** It most often results from normal stretching of the abdominal wall
- D.** It confirms that labor will begin within 24 hours

Correct Answer: A

Right upper quadrant pain in a client with preeclampsia can indicate liver involvement and progression to severe disease. The other options either describe unrelated findings or overstate what the symptom proves.

3254. A client who is 8 weeks pregnant has vaginal bleeding and cramping. Which information best differentiates a threatened miscarriage from an inevitable miscarriage?

- A.** Presence or absence of morning sickness
- B.** Whether the cervix is closed or dilated
- C.** Fetal heart rate compared with maternal heart rate
- D.** Amount of breast tenderness reported

Correct Answer: B

Cervical status is key: a closed cervix supports threatened miscarriage, while dilation suggests inevitable miscarriage. The other findings do not reliably distinguish these conditions.

3255. A nurse is reviewing laboratory results for a pregnant client with suspected HELLP syndrome. Which set of findings is most consistent with this complication?

- A.** Elevated liver enzymes, low platelet count, and evidence of hemolysis
- B.** Low liver enzymes, high platelet count, and low hematocrit
- C.** Elevated glucose, elevated calcium, and decreased uric acid
- D.** Low potassium, low sodium, and decreased white blood cells

Correct Answer: A

HELLP stands for hemolysis, elevated liver enzymes, and low platelets. Choice A directly matches that pattern. The distractors include abnormalities that do not define HELLP syndrome.

3256. A pregnant client with a history of chronic hypertension is seen at 30 weeks' gestation. Which new finding most strongly suggests superimposed preeclampsia rather than chronic hypertension alone?

- A. Blood pressure remaining similar to the baseline reading
- B. New proteinuria with increasing edema and headache
- C. Mild ankle swelling at the end of the day
- D. Need for a routine medication refill

Correct Answer: B

Superimposed preeclampsia is suggested when a client with chronic hypertension develops new proteinuria or worsening symptoms such as headache and edema. Mild dependent swelling alone can occur in normal pregnancy and is a common trap.

3257. A client at 16 weeks' gestation is diagnosed with cervical insufficiency. Which history finding most likely contributed to this complication?

- A. Several episodes of heartburn in the first trimester
- B. Previous cervical surgery
- C. Mild iron-deficiency anemia
- D. Occasional constipation

Correct Answer: B

Cervical insufficiency is associated with structural weakness of the cervix, which can occur after cervical procedures or surgery. Heartburn, mild anemia, and constipation are common pregnancy issues but do not usually cause this problem.

3258. A pregnant client at 22 weeks' gestation with pyelonephritis is admitted for treatment. Which assessment finding indicates the client is responding to therapy?

- A. Persistent flank pain and chills
- B. Urine output decreases to 20 mL/hr

C. Temperature returns toward normal and flank pain lessens

D. Nausea becomes more severe overnight

Correct Answer: C

Improvement is shown by resolving fever and decreasing flank pain, which suggest infection control and clinical stabilization. Persistent pain, worsening nausea, or decreasing urine output would be concerning, not reassuring.

3259. A client at 32 weeks' gestation with severe preeclampsia is receiving magnesium sulfate. The nurse notes a respiratory rate of 10/min and urine output of 20 mL over the last hour. Which medication should the nurse anticipate administering?

- A. Oxytocin
- B. Betamethasone
- C. Calcium gluconate
- D. Rho(D) immune globulin

Correct Answer: C

Respiratory depression and low urine output in a client receiving magnesium sulfate raise concern for magnesium toxicity. Calcium gluconate is the antidote. Oxytocin, betamethasone, and Rho(D) immune globulin do not treat this emergency.

3260. A client at 9 weeks' gestation with persistent vomiting has dry mucous membranes, tachycardia, and a 6-lb weight loss in 1 week. Which prescription should the nurse expect first?

- A. Restrict oral fluids for 12 hours
- B. Initiate IV fluids and electrolyte replacement
- C. Encourage large high-fat meals
- D. Place the client flat in bed without interruption

Correct Answer: B

This client shows significant dehydration from likely hyperemesis gravidarum, so restoring fluid volume and electrolytes is the priority. Restricting fluids would worsen the condition, and large high-fat meals are poorly tolerated. Bed rest alone does not correct the immediate physiologic problem.

3261. A nurse is caring for a pregnant client at 28 weeks' gestation who reports sudden swelling of the face and hands, a severe headache, and seeing spots. Which interpretation is most appropriate?

- A. These are expected discomforts of late second trimester
- B. These findings suggest possible progression to severe hypertensive disease
- C. These symptoms are most consistent with iron-deficiency anemia
- D. These symptoms usually resolve after the client drinks fluids

Correct Answer: B

Facial and hand swelling with severe headache and visual changes are concerning for progression of a hypertensive disorder such as severe preeclampsia. They should not be dismissed as normal discomforts or dehydration. Anemia does not usually cause this symptom cluster.

3262. A pregnant client at 34 weeks' gestation arrives in the labor unit with painless bright red vaginal bleeding. The fetal heart rate is present, and the client denies abdominal trauma. Which nursing action is most appropriate first?

- A. Perform a digital vaginal examination to assess cervical dilation
- B. Prepare the client for external fetal monitoring and notify the provider
- C. Encourage the client to walk to determine whether bleeding increases
- D. Apply a warm pack to the lower abdomen for cramping relief

Correct Answer: B

Painless bright red bleeding in late pregnancy suggests placenta previa until proven otherwise. The priority is maternal-fetal assessment, including fetal monitoring, while notifying the provider. A digital vaginal examination is avoided because it can worsen bleeding when placenta previa is possible. Walking and warm packs do not address the immediate risk.

3263. A nurse is assessing a pregnant client at 35 weeks' gestation who suddenly developed intense abdominal pain and dark vaginal bleeding after reporting that "the baby isn't moving as much." Which complication should the nurse suspect?

- A. Placenta previa
- B. Abruptio placentae
- C. Hyperemesis gravidarum
- D. Hydatidiform mole

Correct Answer: B

Abruptio placentae commonly presents with painful bleeding, uterine tenderness or rigidity, and possible decreased fetal movement from impaired placental perfusion. Placenta previa more often causes painless bright red bleeding. Hyperemesis gravidarum involves severe nausea and vomiting, and hydatidiform mole is associated with abnormal early pregnancy findings rather than sudden painful third-trimester bleeding.

3264. A client at 11 weeks' gestation reports vomiting "all day" for several days. Which finding most strongly supports hyperemesis gravidarum rather than expected nausea of pregnancy?

- A. Mild nausea before breakfast
- B. Food aversions and heightened sense of smell
- C. Ketonuria with weight loss and signs of dehydration
- D. Intermittent nausea relieved by small, frequent meals

Correct Answer: C

Hyperemesis gravidarum is suggested by persistent vomiting severe enough to cause dehydration, weight loss, and ketonuria from inadequate intake. Mild morning nausea, food aversions, and symptoms relieved by dietary changes are more consistent with common nausea of pregnancy. The key distinction is the evidence of fluid and nutritional compromise.

3265. During a prenatal visit, a client who is 31 weeks' gestation says, "I think I'm leaking urine, but I'm not sure." Which finding is most concerning for preterm premature rupture of membranes rather than stress incontinence?

- A. Leakage occurs only with coughing or sneezing

- B.** Fluid is clear, continues to trickle, and wets the underwear repeatedly
- C.** The client voided an hour ago and now feels urinary urgency
- D.** Leakage has a strong ammonia odor

Correct Answer: B

A continuous trickle of clear fluid that repeatedly wets underwear is concerning for rupture of membranes. Leakage only with coughing or sneezing points more toward stress incontinence. Ammonia odor suggests urine, and urinary urgency does not specifically indicate membrane rupture. The trap is assuming all late-pregnancy leakage is urinary.

3266. A pregnant client at 29 weeks' gestation comes to the clinic with blood pressure of 162/104 mm Hg, a persistent headache, brisk reflexes, and new proteinuria. Which provider prescription should the nurse question?

- A.** Place the client on seizure precautions
- B.** Monitor fetal status
- C.** Maintain a quiet environment with limited stimulation
- D.** Encourage the client to ambulate in the hallway every 2 hours

Correct Answer: D

This client has findings consistent with severe preeclampsia, so reducing stimulation, monitoring maternal-fetal status, and seizure precautions are appropriate. Encouraging hallway ambulation should be questioned because activity can increase risk in an unstable client who needs close observation. The safest response is to recognize which order does not fit the complication.

3267. A nurse is caring for four pregnant clients in an outpatient obstetric clinic. Which client should the nurse assess first?

- A.** A client at 10 weeks' gestation with nausea that improves after eating crackers
- B.** A client at 24 weeks' gestation who reports ankle swelling at the end of the day
- C.** A client at 33 weeks' gestation who reports a sudden gush of fluid and decreased fetal movement
- D.** A client at 18 weeks' gestation with occasional round ligament pain when standing

Correct Answer: C

A sudden gush of fluid with decreased fetal movement at 33 weeks suggests rupture of membranes with possible fetal compromise and requires immediate assessment. The other findings are common discomforts or less urgent concerns in pregnancy. The priority is the client with signs of a prenatal complication that can quickly affect both fetus and pregnant client.

3268. A client at 37 weeks' gestation with severe preeclampsia suddenly has a generalized seizure. What is the nurse's priority action?

- A.** Insert an oral airway and restrain the client's arms
- B.** Turn the client to a side-lying position and protect the airway
- C.** Offer ice chips as soon as the jerking stops
- D.** Check cervical dilation immediately after the seizure

Correct Answer: B

During an eclamptic seizure, the priority is safety and airway protection. Turning the client to a side-lying position helps reduce aspiration risk and supports oxygenation. Restraints and forcing an oral airway during active seizure activity can cause injury. Ice chips and cervical checks are not the immediate priority in the moment of seizure management.

Point-of-care tests

3269. A nurse is caring for a client with diabetes who reports shakiness and sweating. Which point-of-care test should the nurse perform first?

- A.** Capillary blood glucose check
- B.** Urine ketone dipstick
- C.** Fecal occult blood test
- D.** Rapid strep throat swab

Correct Answer: A

Shakiness and sweating suggest possible hypoglycemia, so the priority point-of-care test is a capillary blood glucose check. A urine ketone test may be useful in hyperglycemia, not as the first test for these acute symptoms. The other options do not address the likely cause of the client's immediate signs.

3270. A nurse obtains a bedside finger-stick glucose reading of 42 mg/dL for a client who is awake and able to swallow. Which action is most appropriate first?

- A. Give a fast-acting carbohydrate per protocol
- B. Administer the scheduled long-acting insulin dose
- C. Restrict oral intake and recheck in 4 hours
- D. Ask the client to ambulate to improve circulation

Correct Answer: A

A glucose level of 42 mg/dL indicates hypoglycemia. If the client is awake and can swallow, the first action is to give a fast-acting carbohydrate per protocol. Insulin would worsen hypoglycemia. Delaying treatment or encouraging activity is unsafe because both could further lower glucose or increase harm.

3271. Before obtaining a capillary blood glucose sample, which nursing action helps reduce the risk of a falsely elevated result?

- A. Use the same puncture site repeatedly
- B. Wipe away the first drop only if it looks diluted
- C. Clean and dry the finger before lancing
- D. Milk the finger firmly to produce a larger sample

Correct Answer: C

Cleaning and drying the finger helps prevent contamination from food, sugar, or moisture that can falsely elevate or otherwise alter the reading. Repeated use of the same site can injure tissue. Milking the finger can dilute the sample with tissue fluid. Waiting to decide whether to wipe based only on appearance is less reliable than proper site preparation.

3272. A nurse is teaching a client how to collect a stool sample for a fecal occult blood test at the bedside. Which instruction should the nurse include?

- A. Collect the specimen from toilet water after a bowel movement
- B. Avoid contaminating the specimen with urine
- C. Use a specimen that contains a large amount of visible blood
- D. Apply the stool only to the outside flap of the test card

Correct Answer: B

Urine contamination can interfere with stool specimen quality, so the client should avoid mixing urine with the sample. Stool should not be collected from toilet water. A specimen with obvious gross blood is not the purpose of occult testing. The stool is applied to the designated test area, not just the outside flap.

3273. A nurse performs a point-of-care urine pregnancy test for a client with abdominal pain. The result is positive. Which interpretation is most appropriate?

- A. The client is definitely beyond the first trimester
- B. The client is not at risk for ectopic pregnancy
- C. The presence of hCG is detected and should be considered in planning care
- D. The result confirms a normally developing intrauterine pregnancy

Correct Answer: C

A positive urine pregnancy test indicates detection of hCG and should be considered in clinical decision-making. It does not determine gestational age, confirm normal development, or rule out ectopic pregnancy. Those are common overinterpretations of a screening test.

3274. A client with diabetes has cold, pale fingertips and poor peripheral perfusion. The nurse needs a capillary blood glucose reading. Which action is best?

- A. Proceed immediately using the coldest finger to reduce pain
- B. Warm the hand before obtaining the sample
- C. Squeeze the finger repeatedly during collection
- D. Use the thumb because it has the strongest pulse

Correct Answer: B

Warming the hand improves capillary blood flow and helps obtain a more adequate sample. Repeated squeezing can dilute the sample with tissue fluid. Using the coldest finger is not helpful, and the thumb is generally avoided for routine puncture because it is more sensitive and commonly used.

3275. A nurse obtains a positive bedside fecal occult blood test result for a client. Which action is most appropriate?

- A. Document the result and report it for further evaluation
- B. Tell the client the result confirms colon cancer
- C. Repeat the test every 15 minutes until it becomes negative
- D. Disregard the result if the stool appears brown

Correct Answer: A

A positive fecal occult blood test should be documented and reported because it may indicate gastrointestinal bleeding and requires further evaluation. It does not by itself diagnose colon cancer. Repeating the test until negative ignores the significance of the initial finding, and normal stool color does not rule out occult blood.

3276. A nurse is caring for a client receiving high-dose corticosteroid therapy. Which point-of-care test finding should the nurse monitor most closely because this medication can contribute to the abnormal result?

- A. Low urine specific gravity
- B. Elevated capillary blood glucose
- C. Negative urine protein
- D. Decreased fecal occult blood reaction

Correct Answer: B

Corticosteroids can raise blood glucose, so elevated capillary glucose is the point-of-care finding most directly linked to this therapy. The other options are not the best expected medication-related bedside monitoring concern in this situation.

3277. When obtaining a capillary blood specimen for bedside glucose testing, which site is generally preferred in an adult?

- A. Center of the finger pad
- B. Side of the fingertip
- C. Tip of the thumb
- D. Skin over the palm near the wrist

Correct Answer: B

The side of the fingertip is generally preferred because it is less painful and still provides an adequate capillary sample. The center of the finger pad is more sensitive. The thumb is usually avoided for routine puncture, and the palm near the wrist is not a standard finger-stick site.

3278. A client tells the nurse, 'I ate fruit right before my finger-stick glucose test, but I did not wash my hands.' The bedside result is unexpectedly high. Which response by the nurse is best?

- A. Accept the result because food does not affect capillary testing
- B. Repeat the test after cleaning and drying the hands
- C. Administer correctional insulin immediately based on this value
- D. Use a urine dipstick instead because it is more specific for current glucose level

Correct Answer: B

Sugar residue on the skin can falsely elevate a capillary blood glucose result, so the nurse should repeat the test after properly cleaning and drying the hands. Accepting the result or giving insulin immediately could lead to overtreatment. A urine dipstick does not replace bedside blood glucose for current glucose assessment.

3279. A nurse is asked why a bedside urine dipstick for ketones may be useful in a client with nausea, vomiting, and poorly controlled diabetes. Which explanation is most accurate?

- A. It screens for fat breakdown that can occur with inadequate insulin or starvation
- B. It confirms the client has an active bacterial infection
- C. It directly measures the serum insulin level
- D. It determines the exact blood pH at the bedside

Correct Answer: A

Urine ketone testing helps screen for ketone production associated with fat breakdown, which can occur with inadequate insulin or starvation. It does not diagnose infection, measure serum insulin, or determine exact blood pH. Those are different assessments or laboratory evaluations.

3280. A nurse obtains a bedside blood glucose result that does not match the client's presentation. The client is alert, asymptomatic, and has no history of diabetes, but the meter reads 28 mg/dL. Which action is best?

- A. Document the value as accurate because machines do not make errors
- B. Immediately administer insulin to correct the abnormal reading
- C. Repeat the test and assess for possible testing or specimen error
- D. Ignore the result because laboratory tests are the only valid tests

Correct Answer: C

When a point-of-care result is inconsistent with the clinical picture, the nurse should reassess and repeat the test while considering specimen or technique error. Blindly accepting an implausible value is unsafe. Insulin is inappropriate for a low reading, and bedside tests should not be ignored simply because they are not central lab tests.

3281. Which finding should cause the nurse to question the accuracy of a capillary glucose specimen obtained by finger stick?

- A. The puncture was made after the hand was warmed
- B. The sample was obtained from the side of the finger
- C. The nurse heavily squeezed the finger to produce blood
- D. The skin was allowed to dry after cleansing

Correct Answer: C

Heavy squeezing or milking can mix tissue fluid with the blood sample and affect accuracy. Warming the hand, using the side of the finger, and allowing cleansed skin to dry are all appropriate technique elements that support a better specimen.

3282. A nurse is caring for a reproductive-age client scheduled for a diagnostic procedure. Which point-of-care test result is most important to verify before proceeding if the procedure could affect a pregnancy?

- A. Urine pregnancy test
- B. Fecal occult blood card
- C. Bedside ketone strip
- D. Capillary hemoglobin estimate

Correct Answer: A

If a planned procedure could affect a pregnancy, verifying pregnancy status is a priority, so a urine pregnancy test is the most important point-of-care result to check. The other tests do not address that immediate safety concern.

3283. A client with diabetes says, 'My glucose meter showed 58 mg/dL, but I feel fine, so I do not need to do anything.' Which response by the nurse is best?

- A. If you have no symptoms, a low reading can be ignored
- B. Treat the low result and recheck according to protocol, even if symptoms are absent
- C. Exercise now to raise your blood sugar naturally
- D. Wait until your next meal and test again tomorrow

Correct Answer: B

A blood glucose of 58 mg/dL is low and should be treated according to protocol even if the client is not currently symptomatic, because hypoglycemia can worsen quickly. Ignoring it, delaying care, or exercising could increase risk. The common trap is relying only on symptoms instead of the measured result.

3284. A nurse is preparing to perform a bedside urine dipstick test on a freshly voided specimen. Which action is most important to help obtain an accurate result?

- A. Use urine that has been sitting at room temperature for several hours
- B. Dip the strip briefly and compare each test area with the color chart at the recommended time
- C. Immerse the strip until all color pads are saturated for 30 seconds
- D. Read the strip after all pads have changed to their darkest color

Correct Answer: B

Urine dipsticks must be performed according to the manufacturer's timing directions. Dipping the strip briefly and reading each pad at the recommended time helps avoid false results. Letting urine sit too long, oversoaking the strip, or waiting for all pads to darken can make the findings inaccurate.

3285. A nurse needs a capillary blood glucose reading for a client. After puncturing the finger, the nurse sees only a very small drop of blood. Which action is best?

- A. Squeeze the fingertip firmly until enough blood is produced
- B. Wipe away the small drop and puncture a different site if needed
- C. Add an alcohol swab to the sample area to improve flow
- D. Use the tiny sample even if it does not fill the strip completely

Correct Answer: B

Excessive squeezing can dilute the sample with tissue fluid and reduce accuracy. If there is not enough blood, the nurse should avoid milking the finger and obtain a better sample, including using another puncture site if necessary. An incomplete strip fill or contamination with alcohol can also lead to unreliable results.

3286. A nurse performs a bedside urinalysis dipstick for a client who reports burning with urination and urinary frequency. Which dipstick finding most strongly supports possible urinary tract infection?

- A. Positive leukocyte esterase
- B. Negative glucose
- C. Trace ketones
- D. High specific gravity

Correct Answer: A

Positive leukocyte esterase suggests white blood cells in the urine and supports possible urinary tract infection in a client with dysuria and frequency. Negative glucose does not address infection. Trace ketones may relate to poor intake. High specific gravity reflects concentrated urine, not specifically infection.

3287. A nurse obtains a bedside urine pregnancy test that is negative for a client who says her period is late and she has had unprotected sex. Which response by the nurse is best?

- A. A negative result rules out pregnancy with certainty
- B. The test means pregnancy is not possible, so no follow-up is needed
- C. Early pregnancy may not always be detected, so the result should be interpreted with the clinical picture
- D. The client should be told the test is invalid unless blood is visible in the urine

Correct Answer: C

A negative bedside urine pregnancy test does not absolutely rule out very early pregnancy. The nurse should interpret the result along with the history and symptoms and anticipate follow-up if indicated. Choices A and B are too absolute. Visible blood is not the criterion that makes the test invalid.

3288. The nurse is about to obtain a fingerstick glucose level. Which client statement should prompt the nurse to delay testing until the issue is corrected?

- A. I just washed my hands with soap and warm water
- B. I rubbed lotion on my hands a few minutes ago
- C. My fingers feel warm after I kept my hands under a blanket
- D. I usually use the side of my fingertip for home testing

Correct Answer: B

Lotion residue on the fingers can contaminate the sample and alter the blood glucose result. The nurse should have the client wash and dry the hands before testing. The other statements describe conditions that generally support good specimen collection.

3289. A client with diabetes is receiving enteral feedings. During rounds, the nurse finds the client diaphoretic and confused. A bedside glucose test reads 36 mg/dL. The tube feeding is currently paused for a procedure, and the client is not safe to swallow. Which action should the nurse take first?

- A. Give oral juice through a straw
- B. Document the result and recheck the glucose in 30 minutes
- C. Follow the hypoglycemia protocol and give the prescribed rapid treatment for severe hypoglycemia
- D. Restart the tube feeding at the prior rate and reassess later

Correct Answer: C

This client has symptomatic severe hypoglycemia and is not safe for oral intake. The priority is immediate treatment according to protocol, such as IV dextrose or another prescribed rapid intervention. Oral juice risks aspiration. Delaying treatment or simply restarting tube feedings is unsafe.

3290. A nurse collects a urine specimen from an indwelling urinary catheter for a point-of-care dipstick test. Which technique is appropriate?

- A. Drain urine from the collection bag into a specimen cup
- B. Collect urine from the sampling port using aseptic technique
- C. Disconnect the catheter from the drainage tubing and allow urine to drip out
- D. Use the oldest urine available so the results are easier to compare over time

Correct Answer: B

Urine for testing should be obtained from the catheter sampling port using aseptic technique, not from the drainage bag, where urine is old and more likely contaminated. Disconnecting the system increases infection risk. Fresh urine is preferred for more accurate point-of-care testing.

3291. A nurse is reviewing a urine dipstick result for a client with diabetes. Which finding requires the most immediate follow-up?

- A. Trace protein

- B. Positive ketones in a client with nausea and abdominal pain
- C. Specific gravity of 1.030
- D. Slightly acidic pH

Correct Answer: B

Positive ketones in a client with diabetes who also has nausea and abdominal pain can signal significant metabolic imbalance and needs prompt follow-up. Trace protein, concentrated urine, and mildly acidic urine are generally less immediately concerning in this scenario.

3292. A nurse obtains a bedside glucose reading from a client in shock who has cool, mottled extremities. Which interpretation is most appropriate?

- A. The capillary reading may be unreliable because poor peripheral perfusion can affect the sample
- B. The reading is more accurate than a venous sample because it is point-of-care
- C. Peripheral vasoconstriction raises capillary glucose in a predictable way
- D. The result should be accepted without question because shock does not affect bedside glucose testing

Correct Answer: A

Poor peripheral perfusion can make capillary blood glucose results less reliable. In a client with shock and cool, mottled extremities, the nurse should interpret a finger-stick result cautiously and compare it with the clinical picture or obtain confirmatory testing if needed. The other options overstate reliability or make unsupported assumptions.

3293. A nurse performs a bedside fecal occult blood test using a stool sample from a toilet hat that also contains urine. What should the nurse do?

- A. Proceed because urine does not affect bedside stool testing
- B. Proceed only if the stool appears formed
- C. Discard the specimen and obtain a new stool sample that is not contaminated
- D. Add tap water to dilute the urine before testing

Correct Answer: C

A stool specimen for fecal occult blood testing should not be contaminated with urine or toilet water because contamination can affect accuracy. The nurse should obtain a clean specimen. Diluting or proceeding with a contaminated sample risks an invalid result.

3294. A nurse checks a client's bedside glucose before administering scheduled insulin. The meter displays an error message indicating insufficient sample. Which action is best?

- A.** Estimate the glucose based on the partial reading and give half the insulin dose
- B.** Repeat the test with a new strip and an adequate blood sample
- C.** Administer the insulin because the test was almost completed
- D.** Chart the result as low and notify the provider after the insulin is given

Correct Answer: B

An insufficient sample error means the result is not valid. The nurse should repeat the test correctly before making a treatment decision. Estimating the value or giving insulin based on an invalid reading could harm the client.

3295. A nurse is teaching a newly licensed practical nurse about finger-stick glucose collection. Which statement by the new nurse indicates correct understanding?

- A.** I should puncture the center of the fingertip because it bleeds the most
- B.** I should wipe the site with alcohol and test immediately while it is still wet
- C.** I should use the lateral side of the fingertip and allow any alcohol to dry completely before puncture
- D.** If blood flow is slow, I should squeeze repeatedly until a large drop forms

Correct Answer: C

The lateral side of the fingertip is generally preferred because it is less painful, and alcohol should dry completely so it does not dilute the sample. Puncturing the center is more uncomfortable, and repeated squeezing can contaminate the specimen with tissue fluid.

3296. A client with diabetes tells the nurse, "My home glucose meter said 310 mg/dL, but I washed the strip holder with water and used a strip from an old vial I found in my bag." Which is the nurse's best response?

- A.** That result should be considered unreliable until the meter and supplies are used correctly
- B.** A high reading is never affected by supplies or meter care
- C.** The strip holder should always be rinsed before every test, so the reading is confirmed
- D.** Old strips usually lower the value, so the actual glucose must be normal

Correct Answer: A

Improper meter care and questionable strips can affect point-of-care results. The safest interpretation is that the reading may be unreliable until the device and supplies are used according to directions. The other options make assumptions that are too absolute or incorrect.

3297. A nurse obtains a bedside urine dipstick result showing positive nitrites and positive leukocyte esterase in a client with fever and suprapubic discomfort. Which action is most appropriate?

- A.** Document the findings as normal for concentrated urine
- B.** Recognize that the results support possible urinary infection and report them as indicated
- C.** Encourage fluids and repeat the dipstick next week before taking further action
- D.** Interpret the findings as evidence of pregnancy

Correct Answer: B

Positive nitrites and leukocyte esterase together support possible urinary tract infection, especially with fever and suprapubic discomfort. The nurse should recognize the significance and report the findings as indicated. They are not normal concentration changes and do not indicate pregnancy.

3298. A nurse is caring for a postoperative client who suddenly becomes restless and pale. A bedside glucose test reads 18 mg/dL, but the client is talking clearly and has no history of diabetes. Which factor in the collection process would most strongly suggest the result may be inaccurate?

- A. The nurse used the side of the fingertip
- B. The nurse allowed the alcohol to dry before puncture
- C. The nurse obtained the sample from a finger with IV fluid contamination on the skin
- D. The nurse used a new lancet

Correct Answer: C

Skin contamination can create a misleading bedside glucose result. If IV fluid or another substance is present on the skin, the sample may be inaccurate and should be repeated correctly. Using the side of the fingertip, letting alcohol dry, and using a new lancet are appropriate practices.

3299. A nurse is preparing to obtain a capillary blood specimen for bedside glucose testing. Which site is best for the finger-stick?

- A. The center of the fingertip pad
- B. The side of the fingertip
- C. The thumb near the nail bed
- D. The index finger over a callused area

Correct Answer: B

The side of the fingertip is preferred because it usually causes less discomfort and can provide an adequate blood sample. The center of the fingertip pad is more sensitive. A thumb or a callused area is less desirable because these sites may be more painful or may not yield a good sample.

3300. A nurse is performing a bedside urine dipstick test. After removing the strip from the container, which action is most important before comparing the pad colors with the label chart?

- A. Blot the strip and read the results at the recommended times
- B. Set the strip on a dry paper towel for 5 minutes
- C. Rinse the strip briefly with sterile water

D. Wave the strip in the air until all pads look dry

Correct Answer: A

Urine dipsticks should be read according to the manufacturer's timing directions, and excess urine is typically removed to prevent reagent runover between pads. Waiting too long, rinsing the strip, or drying it arbitrarily can alter the results. The key point is controlled timing and proper handling after dipping.

3301. A client with diabetes has a bedside glucose result of 49 mg/dL but is awake, talking, and reports feeling fine. Which action by the nurse is best?

- A. Document the result as normal for this client
- B. Administer scheduled insulin and recheck at the next interval
- C. Repeat the test and assess the client for possible causes or symptoms
- D. Discard the meter because a low result always means device failure

Correct Answer: C

A low bedside glucose value requires follow-up, even if the client does not report symptoms. The nurse should reassess the client and repeat the test to verify the result before deciding on next steps. Documenting it as normal or giving insulin would be unsafe. Assuming the meter is defective is premature without further assessment.

3302. A nurse is preparing to perform a bedside fecal occult blood test. Which specimen should the nurse use?

- A. A stool sample mixed with urine in the collection hat
- B. A stool sample taken from toilet water after flushing
- C. A stool sample collected in a clean, dry container without contamination
- D. A stool sample removed from an incontinence brief several hours later

Correct Answer: C

For point-of-care fecal occult blood testing, the specimen should be collected in a clean, dry container and kept free from contamination. Urine, toilet water, or delayed collection from a soiled brief can affect specimen quality and test accuracy.

3303. A nurse obtains a bedside glucose reading from a client immediately after cleaning the finger with alcohol. The nurse notices the site was still wet when punctured. The result is much lower than expected. Which is the best interpretation?

- A. Residual alcohol may have interfered with the sample, so the result may be inaccurate
- B. Alcohol on the skin causes a falsely high glucose reading
- C. The reading confirms severe hypoglycemia regardless of technique
- D. A wet site improves capillary flow and makes the test more reliable

Correct Answer: A

If alcohol remains on the skin, it can dilute the blood sample and contribute to an inaccurate reading. The nurse should allow the site to dry fully before puncturing. A poor collection technique should not be ignored when the value does not fit the clinical picture.

3304. A nurse performs a bedside urine pregnancy test on a client in the emergency department. The control indicator does not appear. Which action is best?

- A. Interpret the test as negative
- B. Send the same device to the laboratory for confirmation
- C. Repeat the test with a new kit according to instructions
- D. Document the result as inconclusive and take no further action

Correct Answer: C

If the control indicator does not appear, the test is invalid. The correct response is to repeat the test using a new kit and follow the instructions carefully. A missing control means the result cannot be interpreted as negative or positive. Simply documenting an inconclusive result without repeating delays needed care.

3305. A nurse obtains a finger-stick glucose result of 412 mg/dL for a client who was just eating peeled oranges with bare hands. The client has no symptoms of hyperglycemia and says recent glucose levels have been near goal. Which action should the nurse take first?

- A. Administer correctional insulin immediately

- B. Wash and dry the client's hands, then repeat the test
- C. Notify the provider that the client is in diabetic ketoacidosis
- D. Obtain the next glucose reading from the same finger without cleaning it

Correct Answer: B

Food residue on the fingers can contaminate the blood sample and cause a falsely elevated bedside glucose reading. The first step is to wash and dry the hands thoroughly and repeat the test. Giving insulin based on a possibly contaminated sample could be unsafe. The distractor trap is reacting to the number alone without considering collection error.

Vital signs

3306. A practical nurse is checking a postoperative adult client's vital signs. Which finding should be reported first to the nurse in charge?

- A. Temperature 99.1°F (37.3°C)
- B. Blood pressure 88/54 mm Hg
- C. Pulse 84/min
- D. Respiratory rate 18/min

Correct Answer: B

A blood pressure of 88/54 mm Hg is the priority because hypotension after surgery may indicate bleeding, fluid deficit, or shock and requires prompt follow-up. The other findings are within expected or only mildly altered ranges for many postoperative clients.

3307. A client has a regular apical pulse of 56/min while resting in bed. Which action by the practical nurse is most appropriate first?

- A. Document the finding and continue care
- B. Assess the client for symptoms such as dizziness or weakness
- C. Begin chest compressions immediately
- D. Give oxygen at 10 L/min by nonrebreather mask

Correct Answer: B

The nurse should first assess whether the bradycardia is symptomatic. A pulse of 56/min may be significant or may be normal for a resting client, depending on the clinical picture. Chest compressions are not indicated in a client with a pulse, and oxygen or simple documentation without further assessment skips an important first step.

3308. The practical nurse obtains these oral temperatures on four adult clients. Which reading is most concerning and should be rechecked promptly for possible clinical deterioration?

- A. 97.8°F (36.6°C)
- B. 98.4°F (36.9°C)
- C. 101.9°F (38.8°C)
- D. 99.0°F (37.2°C)

Correct Answer: C

An oral temperature of 101.9°F (38.8°C) indicates fever and may signal infection or another inflammatory process, making it the most concerning finding here. The other temperatures fall within or near expected adult ranges.

3309. A practical nurse is measuring respirations on an adult client who appears anxious and short of breath. Which technique is best for obtaining the most accurate respiratory rate?

- A. Tell the client to take several deep breaths while counting
- B. Count respirations immediately after counting the pulse without drawing attention to it
- C. Ask the client to speak while the respirations are counted
- D. Count for 10 seconds and multiply by 6

Correct Answer: B

Respirations are best counted without alerting the client, often right after the pulse is assessed, because awareness can change the rate and pattern. Asking for deep breaths or conversation alters breathing. Counting for only 10 seconds is less accurate, especially when the client is anxious or symptomatic.

3310. An adult client with pneumonia has these current vital signs: temperature 102.2°F (39°C), pulse 112/min, respirations 28/min, blood pressure 118/74 mm Hg, and oxygen saturation 88% on room air. Which finding is the highest priority?

- A. Temperature 102.2°F (39°C)
- B. Pulse 112/min
- C. Respirations 28/min
- D. Oxygen saturation 88% on room air

Correct Answer: D

Oxygen saturation of 88% on room air is the highest priority because it reflects impaired oxygenation. Using airway and breathing priorities, hypoxemia takes precedence over fever, tachycardia, and tachypnea, which may be related but are not as immediately dangerous as low oxygen saturation.

3311. A practical nurse is about to obtain a blood pressure on an adult client. Which situation is most likely to produce a falsely high reading?

- A. Using a cuff that is too small for the client's arm
- B. Supporting the arm at heart level
- C. Having the client rest quietly before the measurement
- D. Deflating the cuff at a steady rate

Correct Answer: A

A cuff that is too small can falsely elevate the blood pressure reading. Proper arm support, rest before measurement, and steady cuff deflation improve accuracy rather than distort it.

3312. A client who received an opioid analgesic 30 minutes ago is now difficult to arouse. Which vital sign requires the most immediate action?

- A. Blood pressure 126/78 mm Hg
- B. Pulse 72/min
- C. Respiratory rate 8/min
- D. Temperature 98.6°F (37°C)

Correct Answer: C

A respiratory rate of 8/min after opioid administration suggests respiratory depression, which is a serious adverse effect requiring immediate intervention. The other vital signs are not the priority in a client who is difficult to arouse after receiving an opioid.

3313. The practical nurse is reassessing a client with dehydration after several hours of IV fluid therapy. Which change best suggests improvement?

- A. Heart rate decreases from 118/min to 92/min
- B. Temperature increases from 98.7°F (37.1°C) to 100.2°F (37.9°C)
- C. Respirations increase from 18/min to 26/min
- D. Blood pressure decreases from 110/70 mm Hg to 90/58 mm Hg

Correct Answer: A

Tachycardia often improves as intravascular volume is restored, so a decrease from 118/min to 92/min suggests a positive response to treatment. Rising temperature, increasing respirations, or dropping blood pressure would not indicate improvement in dehydration.

3314. A practical nurse notes that an adult client's radial pulse is irregular. What is the best next action?

- A. Count the radial pulse for 15 seconds and multiply by 4
- B. Count the apical pulse for 1 full minute
- C. Document the pulse as normal if the rate is under 100/min
- D. Ask the client to cough and then recheck the radial pulse

Correct Answer: B

When a pulse is irregular, the apical pulse should be assessed for a full minute to obtain a more accurate measurement. Counting for only 15 seconds is unreliable with irregular rhythms. Normal rate alone does not make an irregular pulse normal, and coughing is not the best first response.

3315. An adult client arrives from the emergency department after a motor vehicle crash. Which set of vital signs is most consistent with possible shock and should be reported immediately?

- A. Blood pressure 134/82 mm Hg, pulse 76/min, respirations 16/min
- B. Blood pressure 118/70 mm Hg, pulse 88/min, respirations 18/min
- C. Blood pressure 86/48 mm Hg, pulse 128/min, respirations 30/min
- D. Blood pressure 140/86 mm Hg, pulse 64/min, respirations 14/min

Correct Answer: C

Hypotension, tachycardia, and tachypnea together are classic signs suggesting shock or significant hemodynamic instability. The other sets do not show the same concerning pattern.

3316. A practical nurse is obtaining an oral temperature for an adult client who just drank a hot beverage. Which action is best?

- A. Take the oral temperature immediately before the mouth cools
- B. Wait an appropriate period and then recheck orally
- C. Subtract 1°F from the reading obtained now
- D. Use the same oral site but ask the client to breathe through the nose

Correct Answer: B

Hot or cold liquids can alter oral temperature readings, so the best action is to wait and then recheck. Taking it immediately may be inaccurate, and adjusting the number by estimate is unsafe. Breathing pattern does not correct the effect of the recent hot beverage.

3317. A client with sepsis is being monitored closely. Which trend in vital signs most strongly suggests worsening condition?

- A. Temperature drops from 101.8°F (38.8°C) to 99.5°F (37.5°C) after antipyretic administration
- B. Pulse decreases from 110/min to 94/min after fluid replacement
- C. Blood pressure falls from 104/68 mm Hg to 82/46 mm Hg despite treatment
- D. Respirations decrease from 24/min to 20/min after pain control

Correct Answer: C

A falling blood pressure despite treatment suggests worsening perfusion and possible progression toward septic shock. The other trends may reflect improvement or an expected response to interventions.

3318. A practical nurse obtains a blood pressure of 162/96 mm Hg on an adult client who is talking on the phone and sitting with legs crossed. Which action is best before reporting the value as hypertension?

- A.** Recheck the blood pressure after the client rests quietly in proper position
- B.** Switch immediately to taking the blood pressure in the standing position
- C.** Document the finding because any reading over 160/90 mm Hg is definitive
- D.** Take the temperature next and return to blood pressure at the end of the shift

Correct Answer: A

Talking and crossed legs can affect blood pressure readings. The nurse should correct positioning and allow the client to rest before rechecking. One improperly obtained reading should not be treated as definitive without verification.

3319. A practical nurse is caring for an older adult with possible infection. Which finding is most important to recognize about temperature assessment in this client population?

- A.** Older adults always develop very high fevers with infection
- B.** A normal or only slightly elevated temperature can still occur with serious infection
- C.** Temperature is not useful when assessing older adults
- D.** Oral temperatures are always lower than core temperature by exactly 2°F

Correct Answer: B

Older adults may not mount a strong febrile response, so serious infection can be present even with a normal or mildly elevated temperature. Assuming that high fever must be present is a common trap that can delay recognition of illness.

3320. A client with chronic lung disease has an oxygen saturation of 91% at 0800, 90% at 1000, 88% at 1200, and 86% at 1400 on the same oxygen setting. The respiratory rate has risen from 20/min to 28/min. What is the best interpretation?

- A.** The client is stable because the changes occurred gradually
- B.** The oxygen saturation is acceptable because it remains above 85%
- C.** The client is showing a worsening respiratory trend that requires prompt follow-up
- D.** The respiratory rate is more important than oxygen saturation, so the pulse oximeter can be ignored

Correct Answer: C

The downward saturation trend combined with increasing respiratory rate suggests worsening respiratory status and needs prompt attention. A gradual decline is still a decline. It is unsafe to dismiss the readings based on an arbitrary cutoff or to ignore pulse oximetry when it aligns with other signs of deterioration.

3321. A practical nurse is preparing to measure an adult client's blood pressure manually. Which action is most important to help ensure an accurate reading?

- A.** Place the cuff over the client's gown sleeve to avoid skin irritation
- B.** Support the client's arm at heart level during the measurement
- C.** Ask the client to clench and unclench the fist during cuff inflation
- D.** Use the smallest cuff that will fit around the arm

Correct Answer: B

The arm should be supported at heart level to reduce error in the blood pressure reading. If the arm is too low or too high, the reading may be inaccurate. Placing the cuff over clothing can interfere with accuracy, repeated fist clenching can falsely affect the reading, and using a cuff that is too small can falsely elevate the blood pressure.

3322. A practical nurse is checking an adult client's pulse oximetry. The client has dark blue nail polish and cold fingers. The pulse oximeter reads 82%, but the client is alert, speaking clearly, and not in visible respiratory distress. What is the nurse's best action first?

- A. Apply oxygen immediately at a high flow rate
- B. Document the oxygen saturation as accurate because the monitor is objective
- C. Reassess the reading by improving sensor placement or using another site
- D. Notify the provider that the client is in respiratory failure

Correct Answer: C

Cold extremities and nail polish can interfere with pulse oximeter accuracy. The best first action is to verify the value by improving the sensor reading or using another site before acting on a possibly inaccurate number. A common trap is reacting to the low number alone without considering whether the reading fits the clinical picture.

3323. The practical nurse counts a client's respirations as 10/min while the client is sleeping. The respirations are even and unlabored, and the client is easily aroused. Which action is most appropriate?

- A. Document the finding and continue to monitor
- B. Call a rapid response team immediately
- C. Begin rescue breathing with a bag-mask device
- D. Stimulate the client continuously to keep the respiratory rate above 12/min

Correct Answer: A

A sleeping client may normally have a slower respiratory rate than when awake, especially if respirations are regular and unlabored and the client is easily aroused. Immediate emergency intervention is not indicated based on this information alone. The trap is overreacting to a borderline low number without assessing the whole situation.

3324. A practical nurse obtains these oxygen saturation readings on four adult clients. Which client should the nurse assess first?

- A. A client with chronic COPD whose oxygen saturation is 90% on the usual oxygen flow and who is speaking in full sentences

- B. A postoperative client whose oxygen saturation dropped from 98% to 92% over 2 hours and who is increasingly drowsy
- C. A client with a pulse oximeter reading of 94% who has warm hands and no respiratory complaints
- D. A client receiving nebulizer treatment whose oxygen saturation increased from 91% to 93%

Correct Answer: B

The postoperative client should be assessed first because the oxygen saturation is trending downward and the client is becoming more drowsy, which may indicate worsening respiratory status. Trend plus symptoms is more concerning than a single value alone. The COPD client may have a lower usual baseline, so that option is a likely trap if the reader focuses only on the number.

3325. A practical nurse is about to measure an adult client's oral temperature. Which situation requires the nurse to choose a different route?

- A. The client drank ice water 5 minutes ago
- B. The client is receiving oxygen by nasal cannula
- C. The client has a productive cough
- D. The client is wearing dentures

Correct Answer: A

Recent intake of hot or cold liquids can alter an oral temperature reading, so the nurse should delay the oral measurement or use another route. Oxygen by nasal cannula, a cough, and dentures do not automatically prevent oral temperature measurement. The key is recognizing a factor that directly changes the reading.

3326. An adult client reports dizziness when standing. The nurse is asked to check for orthostatic changes. Which approach is best?

- A. Measure the blood pressure only after the client has walked in the hallway
- B. Compare blood pressure readings from the right arm and left arm while the client remains seated
- C. Obtain pulse and blood pressure with the client lying, then sitting or standing, according to facility procedure

D. Take one standing blood pressure after the client has been upright for 30 minutes

Correct Answer: C

Orthostatic assessment involves comparing pulse and blood pressure in different positions, typically starting with the client lying down and then reassessing when sitting or standing according to policy. A single standing reading does not provide the needed comparison. The trap is choosing a method that measures blood pressure but does not actually assess positional change.

3327. The practical nurse obtains a tympanic temperature of 100.8°F (38.2°C) on an adult client. The client says, 'I've been lying on this ear with a heating pad.' What is the best next action?

- A.** Document the client as febrile and notify the provider immediately
- B.** Repeat the temperature using the other ear or another appropriate route
- C.** Administer an antipyretic before taking another temperature
- D.** Ignore the reading because tympanic temperatures are always unreliable

Correct Answer: B

Heat applied near the ear can affect a tympanic reading, so the nurse should recheck the temperature using the other ear or another suitable route. The problem is not that tympanic temperatures are always unreliable, but that this specific situation may have altered the result. The most appropriate step is validation before further action.

3328. A practical nurse is obtaining a manual blood pressure and hears the first Korotkoff sound at 148 mm Hg. The sounds disappear at 86 mm Hg. How should the nurse document this blood pressure?

- A.** 86/148 mm Hg
- B.** 148/86 mm Hg
- C.** 148/148 mm Hg
- D.** 86/86 mm Hg

Correct Answer: B

The first Korotkoff sound marks the systolic pressure, and the point where the sounds disappear marks the diastolic pressure. Therefore, the blood pressure is documented as 148/86 mm Hg. This item tests correct interpretation of manual blood pressure findings.

3329. A practical nurse is caring for an adult client who suddenly reports chills and feels warm to the touch. The temperature was 98.7°F (37.1°C) an hour ago. Which nursing action is best?

- A.** Wait until the next scheduled vital-sign time because the last temperature was normal
- B.** Obtain a current set of vital signs now
- C.** Offer a blanket and reassess at the end of the shift
- D.** Document the report as anxiety-related without reassessment

Correct Answer: B

A new complaint of chills and warmth may signal a developing fever or other change in condition, so the nurse should obtain current vital signs now rather than rely on the earlier normal reading. The key is recognizing that a client's symptoms can change between scheduled assessments.

3330. A practical nurse is asked to recheck a blood pressure that seemed unexpectedly low. Which finding most strongly suggests the original reading may have been falsely low due to technique?

- A.** The cuff bladder was too narrow for the client's arm
- B.** The client had smoked a cigarette just before the reading
- C.** The client's arm was positioned above heart level
- D.** The client was talking during the measurement

Correct Answer: C

If the arm is above heart level, the blood pressure reading may be falsely low. In contrast, a cuff that is too narrow, recent smoking, and talking during the measurement are more likely to raise the reading. This question requires distinguishing factors that push a reading up versus down.

3331. An adult client has a pulse oximeter in place. Which assessment finding makes the oxygen saturation reading least reliable?

- A. The client is resting quietly in bed
- B. The client has cool, poorly perfused fingertips
- C. The client is wearing a hospital gown
- D. The client had lunch 1 hour ago

Correct Answer: B

Poor peripheral perfusion can interfere with pulse oximeter accuracy because the device depends on adequate pulsatile blood flow. Resting in bed, clothing, and recent food intake do not directly affect the sensor reading in the same way. The best answer focuses on the technical limitation of the device.

3332. A practical nurse notes that an adult client's pulse was 88/min this morning and is now 118/min 4 hours later. The blood pressure has decreased from 126/78 mm Hg to 102/64 mm Hg, and the client reports feeling weak. Which interpretation is best?

- A. The client is improving because the pulse is more active
- B. The changes suggest possible clinical deterioration and should be reported promptly
- C. These changes are expected after routine bed rest
- D. The blood pressure is still within a normal range, so no action is needed

Correct Answer: B

A rising pulse combined with a falling blood pressure and weakness suggests possible deterioration, such as fluid loss or another hemodynamic problem, and should be reported promptly. The trap is focusing on one value in isolation instead of recognizing the concerning trend across multiple vital signs.

3333. The practical nurse is preparing to count a client's apical pulse for a full minute. Where should the stethoscope be placed?

- A. At the left fifth intercostal space near the midclavicular line
- B. At the right second intercostal space near the sternum
- C. Directly over the carotid artery in the neck
- D. Over the epigastric area below the sternum

Correct Answer: A

The apical pulse is typically auscultated at the left fifth intercostal space near the midclavicular line. The other locations do not correspond to the usual point for apical pulse assessment. This is a foundational landmark question.

3334. A practical nurse is checking a temperature on an older adult who appears lethargic and less responsive than earlier. The oral temperature is 98.4°F (36.9°C), but the skin feels cool and mottled, and the pulse has increased from baseline. What is the best interpretation?

- A. The client is stable because the temperature is normal
- B. A normal oral temperature rules out significant illness
- C. The client's condition may still be worsening despite the absence of fever
- D. The oral thermometer must be broken because the pulse is elevated

Correct Answer: C

A normal temperature does not rule out serious illness or deterioration, especially in an older adult. The nurse should interpret the temperature together with the lethargy, mottled skin, and increased pulse. A common trap is assuming that no fever means no infection or no instability.

3335. A practical nurse is obtaining routine vital signs for an adult client. The radial pulse feels very weak and difficult to count accurately. What should the nurse do next?

- A. Estimate the pulse rate based on 15 seconds and multiply by 4
- B. Record the pulse as absent and notify the provider
- C. Assess the apical pulse for a full minute

D. Skip the pulse and continue with the rest of the vital signs

Correct Answer: C

If the radial pulse is weak and difficult to count accurately, the nurse should assess the apical pulse for a full minute to obtain a more reliable measurement. Estimating from a short count can be inaccurate, and documenting the pulse as absent is not supported by the stem. The best next step is a more accurate assessment method.

3336. A practical nurse is measuring an adult client's blood pressure. After inflating the cuff, the nurse deflates it as quickly as possible to save time and records the reading. Why is this technique a problem?

- A.** It can produce a falsely high systolic reading and a falsely low diastolic reading
- B.** It can produce a falsely low systolic reading and a falsely high diastolic reading
- C.** It causes the pulse oximetry reading to become inaccurate
- D.** It prevents the nurse from determining the client's respiratory rate

Correct Answer: B

The cuff should be deflated slowly enough to identify the Korotkoff sounds accurately. If the cuff is released too quickly, the nurse may miss the first sound, making the systolic pressure appear lower than it really is, and may also miss the point where sounds disappear, making the diastolic pressure appear higher. Choice A reverses the likely error pattern.

3337. An adult client's pulse is regular at 76/min, respirations are 18/min, blood pressure is 118/72 mm Hg, and oral temperature is 101.3°F (38.5°C). Which action by the practical nurse is most appropriate first?

- A.** Document the findings and continue routine care
- B.** Recheck the temperature after the client drinks ice water
- C.** Assess the client for additional signs and possible source of fever
- D.** Place the client flat in bed because the blood pressure is low

Correct Answer: C

A fever requires follow-up assessment, even when the other vital signs are currently stable. The nurse should assess for associated findings such as chills, cough, urinary symptoms, wound changes, or other clues to the cause. Choice A delays needed assessment. Choice B introduces a factor that can distort an oral temperature. Choice D is not indicated because 118/72 mm Hg is not hypotension.

3338. A practical nurse obtains these vital signs on an adult client: temperature 97.9°F (36.6°C), pulse 104/min, respirations 28/min, blood pressure 88/54 mm Hg. The client is pale and reports feeling faint. Which action is the priority?

- A.** Reassure the client that anxiety can cause temporary changes in vital signs
- B.** Document the complete set of vital signs and recheck in 1 hour
- C.** Report the findings promptly and stay with the client while further assessment is initiated
- D.** Encourage the client to ambulate to improve circulation

Correct Answer: C

This client has concerning vital signs and symptoms: tachycardia, tachypnea, hypotension, pallor, and feeling faint. These cues suggest potential hemodynamic instability and require prompt reporting and continued observation. Choice A minimizes significant findings. Choice B delays action. Choice D could worsen the client's condition and increase fall risk.

3339. A practical nurse is preparing to measure an adult client's radial pulse. Which action helps the nurse avoid making the pulse rate falsely low?

- A.** Using the thumb to feel the pulse point
- B.** Counting for 15 seconds and multiplying by 4
- C.** Applying firm pressure until the artery is no longer felt
- D.** Using the pads of two or three fingers to palpate lightly

Correct Answer: D

The pads of two or three fingers should be used with gentle pressure to palpate a pulse. Too much pressure can partially or completely obstruct the artery and make the pulse hard to detect, leading to an inaccurate count. Using the thumb is a common trap because the nurse may feel the thumb's own pulse. Counting for only 15 seconds is less accurate when any irregularity is possible.

3340. A practical nurse obtains these readings on an adult client: blood pressure 126/78 mm Hg lying, 118/74 mm Hg sitting, and 102/66 mm Hg standing. The pulse changes from 78/min lying to 104/min standing, and the client reports lightheadedness when upright. Which interpretation is best?

- A. The findings are expected because standing always lowers blood pressure temporarily
- B. The findings suggest orthostatic changes that should be reported
- C. The blood pressure is within normal range, so the symptoms are unrelated
- D. The pulse increase shows the client is anxious rather than having a circulatory change

Correct Answer: B

A noticeable drop in blood pressure with standing, along with a pulse increase and lightheadedness, is consistent with orthostatic change and should be reported. The key is not just one number but the pattern across positions plus symptoms. Choice A overgeneralizes. Choice C ignores the significant positional change. Choice D assumes anxiety without accounting for the blood pressure drop.

3341. A practical nurse is caring for an adult client receiving a blood transfusion. Fifteen minutes after the transfusion starts, the client's temperature rises from 98.6°F (37°C) to 100.2°F (37.9°C), the pulse increases from 82/min to 108/min, and the client reports chills and low back pain. Which action should the nurse take first?

- A. Slow the transfusion rate and continue to monitor closely
- B. Stop the transfusion immediately
- C. Administer an antipyretic as prescribed and continue the transfusion

- D. Recheck the vital signs in 30 minutes to confirm the trend

Correct Answer: B

The combination of fever, tachycardia, chills, and low back pain shortly after transfusion begins is concerning for a transfusion reaction. The first action is to stop the transfusion immediately to prevent further exposure. Choice A is unsafe because slowing is not enough when a reaction is suspected. Choices C and D delay urgent intervention.

Procedure and diagnosis

precautions

3342. A nurse is preparing to obtain a sputum specimen for culture from a client with a productive cough. Which action is most appropriate?

- A. Collect the specimen after the client eats breakfast
- B. Ask the client to rinse the mouth with water before coughing deeply
- C. Obtain the sample from saliva in the mouth
- D. Add sterile water to the specimen cup before collection

Correct Answer: B

Rinsing the mouth with water helps reduce contamination from food particles and oral secretions before the client coughs up sputum from the lower respiratory tract. Collecting after breakfast can increase contamination, saliva is not an acceptable sputum sample, and nothing should be added to the specimen container.

3343. A client is scheduled for a fasting blood glucose test at 0800. Which statement by the client requires the nurse to follow up?

- A. I did not eat after midnight.
- B. I only drank water this morning.
- C. I had black coffee at 0600.
- D. I came to the lab before breakfast.

Correct Answer: C

Black coffee can affect a fasting test and means the client was not truly fasting. Water is generally allowed for fasting labs, and arriving before breakfast after no food since midnight supports proper preparation. A common trap is assuming only sugar or cream would matter, but the safest response is to clarify that the test preparation was not followed.

3344. A nurse is preparing a client for a stool guaiac test. Which instruction should the nurse give?

- A. Avoid mixing the stool sample with urine
- B. Collect the stool after adding toilet water to soften it
- C. Store the specimen with the lid off until it is picked up
- D. Apply the sample only if the stool is liquid

Correct Answer: A

Urine can contaminate the specimen and interfere with testing, so the stool should be collected separately. Toilet water should not be mixed with the specimen, the container should remain closed, and formed stool can also be tested. The key precaution is preventing contamination.

3345. Before a chest x-ray, which action by the nurse is most appropriate?

- A. Keep the client NPO for 8 hours
- B. Ask the client to remove necklaces and metal objects from the chest area
- C. Start an IV line for contrast dye
- D. Administer a sedative 30 minutes before transport

Correct Answer: B

Metal objects can interfere with imaging and should be removed before a chest x-ray. Routine chest x-rays do not require fasting, IV contrast, or sedation. A likely trap is confusing a simple x-ray with more complex imaging studies.

3346. A client is returning from a liver biopsy. Which finding should the nurse report first?

- A. Complaint of soreness at the puncture site
- B. Blood pressure lower than preprocedure baseline and increasing pulse rate
- C. Request for oral fluids
- D. Small amount of drainage on the dressing

Correct Answer: B

A falling blood pressure with a rising pulse suggests possible hemorrhage, a serious complication after liver biopsy. Mild soreness and a small amount of drainage may occur, and requesting fluids is not the priority. The key precaution after this procedure is close monitoring for bleeding.

3347. A nurse is collecting a urine specimen for culture from a client with an indwelling urinary catheter. Which action is correct?

- A. Obtain the urine from the drainage bag
- B. Disconnect the catheter from the tubing and drain urine into a cup
- C. Withdraw urine from the sampling port using aseptic technique
- D. Remove the catheter and collect the first voided urine

Correct Answer: C

A sterile urine culture from a catheterized client is collected from the sampling port using aseptic technique. Urine in the drainage bag is old and contaminated, disconnecting the system increases infection risk, and removing the catheter just to get a specimen is inappropriate. The main precaution is preserving a closed, clean system.

3348. A client is scheduled for a paracentesis. Which preprocedure nursing action is most important?

- A. Encourage the client to drink extra fluids
- B. Have the client empty the bladder
- C. Place the client flat in bed for 2 hours before the procedure
- D. Administer a high-fiber snack

Correct Answer: B

The bladder should be emptied before paracentesis to reduce the risk of accidental puncture during the procedure. Extra fluids and a snack are not key precautions, and a flat position beforehand is not the main safety preparation. This question focuses on preventing organ injury.

3349. A nurse is monitoring a client after administration of IV contrast for a diagnostic study. Which finding requires immediate action?

- A. Warm sensation during the injection
- B. Metallic taste in the mouth
- C. Hives and wheezing
- D. Brief complaint of nausea

Correct Answer: C

Hives and wheezing suggest an allergic or hypersensitivity reaction and require immediate intervention. A warm sensation, metallic taste, and mild transient nausea can occur with contrast administration and are often expected. The trap is mistaking expected sensations for the most urgent complication.

3350. A client is scheduled for a lumbar puncture. Which nursing action is appropriate after the procedure?

- A. Encourage the client to ambulate as soon as sensation returns
- B. Monitor for headache and assess neurologic status
- C. Place the client in high-Fowler position immediately
- D. Restrict all oral fluids for 8 hours

Correct Answer: B

After a lumbar puncture, the nurse should monitor for complications such as headache and changes in neurologic status. Immediate ambulation and high-Fowler positioning are not the priority precautions, and oral fluids are not routinely restricted. The key is ongoing assessment for postprocedure complications.

3351. A nurse is preparing to obtain a blood specimen for coagulation studies from a peripheral vein. Which action is most important to help ensure accurate results?

- A. Use a tube that contains the correct anticoagulant for the test
- B. Shake the tube vigorously after collection
- C. Leave the tourniquet in place until the specimen is labeled
- D. Collect the sample from a vein distal to a running IV infusion

Correct Answer: A

Coagulation studies require the correct collection tube to preserve specimen integrity and provide valid results. Vigorous shaking can damage the sample, prolonged tourniquet use can alter results, and drawing distal to an active IV risks dilution or contamination. The main issue is test accuracy through proper specimen handling.

3352. A client is scheduled for a bronchoscopy under moderate sedation. Which finding is the priority before offering oral fluids after the procedure?

- A. The client reports mild throat irritation
- B. The client can state name and date of birth
- C. The gag reflex has returned
- D. The oxygen saturation is 95% on room air

Correct Answer: C

The gag reflex must return before oral intake after bronchoscopy to reduce aspiration risk. Orientation and adequate oxygen saturation are important, but they do not confirm swallowing safety. Throat irritation is common and not the deciding factor. The likely trap is choosing a generally stable finding instead of the aspiration precaution specific to this procedure.

3353. A nurse is caring for a client after a thyroid biopsy. Which assessment finding is most concerning?

- A. Small bruise at the biopsy site
- B. Hoarse voice and difficulty swallowing
- C. Mild discomfort when turning the head
- D. Request for acetaminophen

Correct Answer: B

Hoarseness and difficulty swallowing after a thyroid biopsy may indicate swelling or bleeding that could compromise the airway. Mild bruising and discomfort can occur, and a request for pain relief is not the priority concern. This is a high-risk postprocedure precaution because airway status can deteriorate quickly.

3354. A client with diabetes is scheduled for a computed tomography study with IV contrast and says, 'I had a severe reaction to contrast dye years ago.' What is the nurse's best action?

- A. Proceed because reactions from past studies do not predict future problems
- B. Document the statement and notify the provider before the test
- C. Tell the client that modern contrast agents do not cause allergic reactions
- D. Instruct the client to drink extra water during the scan

Correct Answer: B

A history of a prior severe contrast reaction is a significant safety concern and should be reported before the study so the plan can be reviewed. The nurse should not minimize the history or proceed without follow-up. Hydration may be useful in some situations, but it does not address the immediate risk. The key precaution is recognizing and escalating a potentially dangerous history.

3355. A nurse is caring for a client after cardiac catheterization via the femoral approach. Which action is most important during the immediate recovery period?

- A. Keep the affected leg straight and monitor the insertion site for bleeding
- B. Encourage the client to sit upright to improve ventilation
- C. Apply heat to the puncture site to reduce discomfort
- D. Ambulate the client within 30 minutes to assess tolerance

Correct Answer: A

After femoral cardiac catheterization, the nurse should keep the leg straight and monitor closely for bleeding or hematoma formation. Upright positioning and early ambulation can increase bleeding risk, and heat is not the priority intervention. This question centers on preventing vascular complications after an invasive procedure.

3356. A nurse is preparing a client for a transesophageal echocardiogram. Which information is most important to obtain before the procedure?

- A. Whether the client has a history of esophageal disorders or difficulty swallowing
- B. Whether the client prefers morning or afternoon appointments
- C. Whether the client has had a chest x-ray in the last year
- D. Whether the client takes a daily multivitamin

Correct Answer: A

A transesophageal echocardiogram involves passing a probe into the esophagus, so a history of esophageal problems or swallowing difficulty is an important safety precaution. Appointment preference, prior chest x-ray, and multivitamin use are not the most critical concerns here. The question tests recognition of a procedure-specific risk factor.

3357. A client is scheduled for pulmonary function testing this morning. Which instruction should the nurse give before the test?

- A. Use your bronchodilator inhaler right before the test
- B. Eat a large breakfast so you have enough energy
- C. Wear loose clothing that will not restrict chest expansion
- D. Limit your breathing effort so you do not become fatigued

Correct Answer: C

Loose clothing helps the client breathe as fully as possible during pulmonary function testing. Using a bronchodilator immediately before testing may alter results if the provider wants baseline function, and a large meal can interfere with full chest expansion. The client should give maximal effort, not limit breathing effort.

3358. A nurse is preparing a client for an abdominal ultrasound of the gallbladder. Which statement by the client indicates a need for further teaching?

- A. I should avoid eating before the test if instructed to fast
- B. I may need to lie still while the probe is moved over my abdomen

- C. I will receive contrast dye through my IV during the test
- D. This test uses sound waves to create images

Correct Answer: C

A routine gallbladder ultrasound uses sound waves, not IV contrast dye. The other statements are appropriate. Fasting is commonly required to reduce gallbladder contraction, and the client may need to remain still during imaging.

3359. A client is scheduled for an intravenous pyelogram with iodinated contrast. Which assessment finding is most important for the nurse to report before the procedure?

- A. The client has not eaten since midnight
- B. The client reports a history of hives after shellfish
- C. The client voided 1 hour ago
- D. The client says the room feels cold

Correct Answer: B

A history suggesting sensitivity to shellfish may indicate possible iodine/contrast-related hypersensitivity risk and should be reported before contrast administration. The client being NPO and having voided recently are not the priority concerns. Feeling cold is nonspecific and not the key preprocedure precaution here.

3360. A nurse is assisting with a thoracentesis. Which action is most appropriate to help reduce the client's risk during the procedure?

- A. Position the client leaning forward over a bedside table
- B. Place the client flat in bed with a pillow under the knees
- C. Encourage the client to cough frequently during needle insertion
- D. Offer oral fluids throughout the procedure

Correct Answer: A

Leaning forward over a bedside table helps widen the intercostal spaces and supports safer access during thoracentesis. Lying flat does not provide ideal positioning. Coughing during needle insertion increases risk of injury, and oral fluids are not part of intraprocedure safety for this procedure.

3361. A nurse is reinforcing teaching for a client who will collect a 24-hour urine specimen. Which client statement shows correct understanding?

- A. I will save the first urine I pass when the test begins
- B. I should keep the urine container refrigerated or on ice if instructed
- C. I can add toilet paper to the collection container if needed
- D. If I miss one specimen, I can keep going and still send it

Correct Answer: B

Many 24-hour urine collections require refrigeration or storage on ice to preserve the specimen. The first void at the start is usually discarded, then all urine is collected for the next 24 hours. Contaminants such as toilet paper should not be added, and missing a specimen usually means the collection may need to be restarted.

3362. A client has just returned from a myelogram. Which assessment finding requires the nurse's immediate attention?

- A. The client reports a mild headache
- B. The client asks when food can be resumed
- C. The client has a temperature of 37.2°C (99°F)
- D. The client becomes difficult to arouse and then has a seizure

Correct Answer: D

A decreased level of consciousness followed by seizure activity after a myelogram is an emergency and requires immediate action. A mild headache can occur and may need follow-up, but it is not as urgent as seizure activity. Asking about food and having a low-grade normal temperature are not priority findings.

3363. A nurse is preparing to obtain a capillary blood glucose reading from a client. Which action is best to promote an accurate result?

- A. Use the first drop of blood without wiping it away
- B. Massage the finger vigorously after puncture
- C. Clean the site and allow it to dry before puncture
- D. Puncture the center of the finger pad

Correct Answer: C

Cleaning the site and allowing it to dry helps avoid contamination and dilution that can affect the reading. The first drop is often wiped away depending on agency practice, and vigorous squeezing can dilute the specimen with tissue fluid. The side of the fingertip, not the center of the pad, is generally preferred for comfort and adequate blood flow.

- 3364.** A nurse is caring for a client scheduled for a bone marrow biopsy. Which nursing action is the priority after the procedure?
- A. Encourage the client to ambulate immediately
 - B. Assess the biopsy site for bleeding
 - C. Offer a high-protein snack
 - D. Place the client in Trendelenburg position

Correct Answer: B

Bleeding is a key immediate risk after bone marrow biopsy, so the biopsy site should be assessed promptly. Ambulation is not the first priority, a snack does not address immediate risk, and Trendelenburg positioning is not routinely indicated.

- 3365.** A client is scheduled for magnetic resonance imaging (MRI) of the spine. Which information obtained by the nurse is most important to report before the test?
- A. The client is claustrophobic
 - B. The client has a titanium hip prosthesis
 - C. The client has a permanent pacemaker
 - D. The client removed hearing aids before transport

Correct Answer: C

A permanent pacemaker is the most important finding to report before MRI because the magnetic field may create serious safety issues depending on device compatibility. Claustrophobia is important but usually not the top safety concern. A hip prosthesis may or may not affect testing, and removing hearing aids is appropriate preparation.

- 3366.** A nurse is collecting a midstream clean-catch urine specimen from a client. Which instruction is most appropriate?
- A. Begin voiding, then collect the sample in the sterile cup without touching the inside

- B. Collect the first urine that comes out because it is the most concentrated
- C. Use any available container if the sterile cup is dropped
- D. Save the sample at the bedside until the end of the shift

Correct Answer: A

For a midstream clean-catch specimen, the client should begin voiding first, then collect the midstream portion in a sterile container while avoiding contamination of the inside of the cup. The first urine stream is more likely to contain contaminants. A nonsterile container is inappropriate, and the specimen should be sent promptly rather than left at the bedside.

- 3367.** A client is scheduled for a percutaneous liver biopsy. The nurse reviews the laboratory results. Which finding should the nurse report before the procedure?
- A. Hemoglobin 13.8 g/dL
 - B. Platelet count 62,000/mm³
 - C. Sodium 139 mEq/L
 - D. Fasting glucose 94 mg/dL

Correct Answer: B

A platelet count of 62,000/mm³ suggests increased bleeding risk and should be reported before an invasive procedure such as liver biopsy. The other values are not the priority concern in this context. This question tests recognition of a procedure-related safety precaution tied to coagulation status.

- 3368.** A nurse is preparing a client for a radionuclide bone scan. Which statement by the nurse is most appropriate?
- A. You will need to remain flat in bed for 12 hours afterward
 - B. You should expect the procedure to require insertion of a large chest tube
 - C. You may be encouraged to drink fluids after the tracer is administered
 - D. You cannot speak during the imaging portion of the test

Correct Answer: C

After tracer administration for a bone scan, fluids are often encouraged to help eliminate the radionuclide. The test does not require chest tube insertion, and the client does not need prolonged flat bedrest simply because of the scan. Remaining reasonably still is important for imaging, but speaking is not the central teaching point here.

3369. A nurse is caring for a client after removal of a femoral arterial sheath following an invasive vascular procedure. Which finding is the priority to report immediately?

- A. Ecchymosis smaller than 1 cm at the insertion site
- B. A warm foot with palpable distal pulses
- C. Sudden swelling at the groin with back pain and hypotension
- D. Mild tenderness when the site is touched

Correct Answer: C

Sudden groin swelling with back pain and hypotension suggests major bleeding or retroperitoneal hemorrhage and requires immediate action. Small bruising and mild tenderness can occur, and warm extremities with palpable pulses are reassuring findings.

3370. A client is about to have a bedside swallow screening after a stroke. Which observation should cause the nurse to stop and notify the provider rather than proceed with oral testing?

- A. The client asks for water
- B. The client has a hoarse, wet-sounding voice and cannot manage secretions
- C. The client reports feeling hungry
- D. The client is sitting upright in high-Fowler position

Correct Answer: B

A wet voice and inability to manage secretions suggest impaired swallowing and high aspiration risk. The nurse should not proceed with oral intake or bedside oral testing until the client is further evaluated. Asking for water or feeling hungry does not make testing unsafe, and upright positioning is appropriate.

3371. A nurse is monitoring a client during the first 15 minutes of a blood transfusion. Which finding requires the nurse to stop the transfusion immediately?

- A. Temperature increases from 37.0°C (98.6°F) to 37.2°C (99.0°F)
- B. The client reports itching, chills, and low back pain
- C. The client asks to use the bathroom
- D. Blood pressure changes from 128/76 mm Hg to 124/74 mm Hg

Correct Answer: B

Itching, chills, and low back pain are concerning for an acute transfusion reaction and require immediate discontinuation of the transfusion and prompt follow-up actions. The small temperature and blood pressure changes listed are not the strongest indicators of a serious reaction. Asking to use the bathroom is not a sign of transfusion reaction.

3372. A client is scheduled for a lumbar puncture to evaluate possible central nervous system infection. Which action should the nurse take first before the procedure?

- A. Ask the client to empty the bladder
- B. Review the client's coagulation status
- C. Place the client in high-Fowler's position
- D. Encourage increased oral fluids

Correct Answer: B

Reviewing coagulation status is the priority because a lumbar puncture is an invasive procedure with bleeding risk. Abnormal clotting increases the chance of spinal bleeding and complications. Asking the client to void may be helpful, but it is not the first safety precaution. High-Fowler's is not the usual position for the procedure, and encouraging fluids is not the key preprocedure safety step.

3373. The nurse is reinforcing teaching for a client who will undergo a transesophageal echocardiogram (TEE). Which client statement indicates correct understanding of the procedure precautions?

- A. I can eat as soon as the test is finished.
- B. I should expect to drink contrast dye during the test.

- C. My throat may be numb, so I should wait for my gag reflex to return before eating.
- D. I will need to keep my leg straight for several hours afterward.

Correct Answer: C

After a TEE, the throat is often numbed, so the client should wait until the gag reflex returns before eating or drinking to reduce aspiration risk. Eating immediately is unsafe. Contrast dye is not typically taken by mouth for this test. Keeping the leg straight is associated with some vascular procedures, not TEE.

3374. A client is scheduled for a bronchoscopy. Which finding is most important for the nurse to report to the provider before the procedure?

- A. The client has a productive cough with thick sputum.
- B. The client has been NPO since midnight.
- C. The client signed the consent form this morning.
- D. The client received warfarin therapy yesterday.

Correct Answer: D

Warfarin therapy increases bleeding risk during bronchoscopy, so it is the most important finding to report before the procedure. A productive cough may be part of why the test is being done. Being NPO and having signed consent are expected and do not create the same immediate safety concern. The key trap is choosing a symptom related to the diagnosis instead of the finding that raises procedural risk.

3375. A nurse is caring for a client after a cardiac catheterization performed through the femoral artery. Which observation requires immediate intervention?

- A. The client reports mild back discomfort from lying flat.
- B. A small bruise is present at the insertion site.
- C. The affected leg is cool with a weak pedal pulse.
- D. The client asks when oral fluids can be restarted.

Correct Answer: C

A cool leg with a weak pedal pulse suggests impaired arterial circulation, which is a serious postprocedure complication requiring immediate action. Mild back discomfort from positioning and a small bruise at the site can occur, though they still need monitoring. Asking about fluids is not urgent. The major danger here is compromised perfusion distal to the catheter site.

3376. A provider prescribes a fecal occult blood test for a client. Which nursing instruction is most appropriate to help reduce the chance of a false result?

- A. Collect the specimen from toilet water after a bowel movement.
- B. Avoid using the sample from stool mixed with urine.
- C. Take a laxative the night before collecting the sample.
- D. Refrigerate the toilet tissue used after the bowel movement.

Correct Answer: B

Stool used for fecal occult blood testing should not be contaminated with urine, because contamination can interfere with the specimen and affect accuracy. Toilet water contamination is also incorrect, so collecting from toilet water is not appropriate. Laxatives are not a routine instruction for this test, and refrigerating toilet tissue is not part of safe specimen collection. The best answer focuses on keeping the specimen clean and usable.

3377. A client returns from a liver-spleen scan that used a radioactive tracer. The client's family asks whether they should avoid visiting because of radiation exposure. Which response by the nurse is best?

- A. You should stay at least 6 feet away from the client for 24 hours.
- B. Visits must be limited because the client will emit dangerous radiation all day.
- C. Routine contact is usually safe because the amount of radioactivity used for this type of diagnostic test is small.
- D. The client must remain on strict bed rest until the tracer is fully eliminated.

Correct Answer: C

Diagnostic nuclear medicine studies generally use small amounts of radioactive tracer, so routine contact is usually safe. The family does not typically need extreme distancing or major visiting restrictions for this kind of test. Strict bed rest is not required just because a tracer was used. The trap answers exaggerate radiation risk beyond what is expected for a standard diagnostic study.

3378. A client is scheduled for an electroencephalogram (EEG) tomorrow morning. Which statement by the client indicates a need for further teaching?

- A.** I should wash my hair and avoid putting oils or sprays on it before the test.
- B.** I may be asked to remain still during the recording.
- C.** I will need to stay NPO for 12 hours before the test.
- D.** The test records electrical activity of my brain.

Correct Answer: C

An EEG does not routinely require the client to remain NPO for 12 hours, so this statement shows a need for further teaching. Clean hair without oils or sprays helps electrode contact, remaining still helps obtain a clear recording, and the test does record brain electrical activity. The challenge is separating realistic preparation steps from an unnecessary restriction.

ECG

3379. A nurse is preparing a client for a standard 12-lead ECG. Which action is most appropriate to help obtain an accurate tracing?

- A.** Ask the client to talk through the procedure to reduce anxiety
- B.** Have the client lie still and breathe normally during the recording
- C.** Place the client in high-Fowler's position with arms crossed over the chest
- D.** Apply lotion to the chest before attaching the electrodes

Correct Answer: B

The client should lie still and breathe normally during the ECG so muscle movement and motion do not create artifact. Talking, crossing the arms, or moving can interfere with the tracing. Lotion should not be applied because it can prevent the electrodes from adhering well.

3380. A nurse begins obtaining an ECG and notices a jagged, uneven baseline on the monitor while the client is shivering. Which action should the nurse take first?

- A.** Document the tracing as abnormal
- B.** Administer oxygen by nasal cannula
- C.** Warm the client and repeat the tracing
- D.** Remove all limb leads permanently

Correct Answer: C

Shivering commonly causes somatic muscle artifact, which can produce a jagged baseline. The nurse should address the likely source of artifact by warming the client and repeating the ECG. Documenting an artifact-filled tracing as abnormal is premature, and removing leads does not solve the cause.

3381. A client is wearing a telemetry monitor. Which nursing observation requires the most immediate follow-up?

- A.** The monitor alarm shows a low battery indicator
- B.** The client reports itching under one electrode
- C.** The tracing suddenly changes to a straight line, and the client is awake and talking
- D.** The rhythm strip prints more slowly than usual

Correct Answer: C

A sudden straight-line tracing in a client who is awake and talking strongly suggests lead disconnection or equipment malfunction, which requires immediate follow-up because it can mask the client's true rhythm. The other findings need attention, but they are less urgent than confirming whether the monitor is functioning and attached correctly.

3382. Before applying ECG electrodes, which nursing action best promotes good electrical conduction?

- A.** Clean and dry the skin before electrode placement

- B.** Massage the chest vigorously after placing the electrodes
- C.** Apply petroleum jelly beneath each electrode
- D.** Place electrodes over bony prominences for stability

Correct Answer: A

Cleaning and drying the skin helps electrodes adhere and improves conduction. Petroleum products interfere with contact, and electrodes should not be placed over bony prominences if better soft-tissue placement is available. Vigorous massage is unnecessary and can irritate the skin.

3383. A nurse is obtaining a 12-lead ECG for a client with chest pain. The client asks why the test is being done. Which response by the nurse is best?

- A.** It measures how much oxygen is in your blood
- B.** It records the electrical activity of your heart
- C.** It shows whether your lungs are expanding equally
- D.** It tells us your blood pressure over the past hour

Correct Answer: B

An ECG records the electrical activity of the heart. It does not measure oxygen saturation, lung expansion, or trends in blood pressure. This explanation is accurate and simple for client teaching.

3384. A nurse reviews a telemetry strip and notes an irregularly irregular rhythm with no clearly visible P waves. Which rhythm should the nurse suspect?

- A.** Sinus bradycardia
- B.** Atrial fibrillation
- C.** First-degree AV block
- D.** Normal sinus rhythm

Correct Answer: B

Atrial fibrillation is commonly identified by an irregularly irregular rhythm and absent distinct P waves. Sinus bradycardia and normal sinus rhythm have identifiable P waves, and first-degree AV block has a prolonged PR interval but still typically has a regular sinus rhythm.

3385. A client's monitor shows frequent premature ventricular contractions (PVCs). Which assessment should the nurse perform first?

- A.** Check the client's blood pressure and symptoms
- B.** Ask when the client last had a bowel movement
- C.** Obtain a urine specimen for culture
- D.** Remove the telemetry monitor to reduce anxiety

Correct Answer: A

When PVCs appear, the priority is to assess the client's hemodynamic status and symptoms, such as blood pressure, dizziness, chest pain, or shortness of breath. This determines the significance of the rhythm change and guides urgent action. The other options do not address immediate cardiovascular risk.

3386. A nurse is preparing to attach telemetry electrodes to a client with a hairy chest. Which action is best?

- A.** Place the electrodes over the hair and tape them tightly
- B.** Shave small areas as needed to improve electrode contact
- C.** Apply extra electrode gel over the hair
- D.** Avoid telemetry because hair prevents monitoring

Correct Answer: B

Shaving small areas as needed improves electrode contact and helps reduce artifact. Simply taping over hair or adding more gel does not reliably create good contact. Hair does not prevent monitoring when the skin is prepared correctly.

3387. A nurse sees tall, peaked T waves on a client's ECG. Which laboratory value should the nurse review first?

- A.** Serum potassium
- B.** Hemoglobin
- C.** Platelet count
- D.** White blood cell count

Correct Answer: A

Tall, peaked T waves are a classic clue suggesting hyperkalemia, so the nurse should review the serum potassium first. Hemoglobin, platelets, and white blood cell count do not directly explain this ECG change.

3388. A nurse is caring for a client whose monitor shows sinus bradycardia at a rate of 52/min. Which finding most strongly suggests that immediate intervention is needed?

- A. The client is asleep but easy to arouse
- B. The client denies pain and is watching television
- C. The client reports dizziness and has a blood pressure of 84/50 mm Hg
- D. The client states that this heart rate is normal during exercise

Correct Answer: C

Bradycardia becomes more concerning when it is associated with poor perfusion or instability, such as hypotension and dizziness. A sleeping or asymptomatic client may not need urgent intervention. Saying the rate is normal during exercise does not explain current symptoms at rest.

3389. A nurse obtains a rhythm strip that shows consistent P waves before each QRS complex, a regular rhythm, and a heart rate of 76/min. Which interpretation is most appropriate?

- A. Normal sinus rhythm
- B. Ventricular tachycardia
- C. Atrial flutter
- D. Second-degree AV block

Correct Answer: A

A regular rhythm with a rate in the normal range and a P wave before each QRS is consistent with normal sinus rhythm. Ventricular tachycardia is usually rapid with wide complexes, atrial flutter shows flutter waves, and second-degree AV block includes dropped beats.

3390. A client on telemetry suddenly reports chest pressure and shortness of breath. The monitor shows a rapid wide-complex rhythm. What should the nurse do first?

- A. Print the strip and assess the client immediately
- B. Turn off the alarm to decrease stimulation
- C. Document the rhythm at the end of the shift
- D. Remove the leads and reapply them after the client rests

Correct Answer: A

A rapid wide-complex rhythm with symptoms may indicate a life-threatening dysrhythmia. The nurse should quickly assess the client while obtaining the strip for interpretation and urgent follow-up. Turning off the alarm, delaying documentation, or removing leads could delay needed treatment.

3391. A nurse notes that an ECG tracing has wandering baseline artifact. The client is alert, anxious, and repeatedly gripping the side rails. Which action is most likely to improve the tracing?

- A. Ask the client to relax the arms and remain still during the recording
- B. Raise the side rails higher to limit movement
- C. Place the electrodes directly over the client's joints
- D. Instruct the client to take rapid deep breaths during the test

Correct Answer: A

Muscle tension and movement can cause wandering or noisy artifact. Asking the client to relax and stay still is the best immediate correction. Raising side rails or placing electrodes over joints will not solve the problem, and rapid breathing can worsen motion-related interference.

3392. A nurse is monitoring a client who has a serum potassium level of 6.2 mEq/L. Which ECG change would the nurse expect as this electrolyte imbalance worsens?

- A. Tall, peaked T waves progressing to conduction abnormalities
- B. Prominent U waves with ST depression
- C. Shortened PR interval with narrow QRS complexes only

D. Regular P waves with no change in ventricular conduction

Correct Answer: A

Hyperkalemia commonly causes tall, peaked T waves early and can progress to widening of the QRS complex and other conduction abnormalities as it becomes more severe. Prominent U waves are more associated with hypokalemia. The other options do not best fit worsening hyperkalemia.

3393. During rhythm interpretation, a nurse identifies a P wave for every QRS complex, but the PR interval is consistently prolonged. Which rhythm interpretation is most appropriate?

- A.** First-degree AV block
- B.** Atrial fibrillation
- C.** Ventricular fibrillation
- D.** Sinus arrest

Correct Answer: A

First-degree AV block is characterized by a prolonged but consistent PR interval with each P wave followed by a QRS complex. Atrial fibrillation lacks distinct organized P waves, ventricular fibrillation has no organized complexes, and sinus arrest involves pauses from absent sinus node firing.

3394. A nurse is obtaining a 12-lead ECG for a client who has diaphoresis and new chest discomfort. The machine repeatedly prints a message indicating limb lead reversal. Which action is most appropriate?

- A.** Continue the tracing because lead reversal does not affect interpretation
- B.** Reapply the limb leads to the correct extremities and repeat the ECG
- C.** Remove the chest leads and leave only the limb leads in place
- D.** Document the machine message and send the tracing to the provider

Correct Answer: B

Correct lead placement is necessary for an accurate 12-lead ECG. If the machine indicates limb lead reversal, the nurse should correct the placement and repeat the tracing. Continuing anyway or sending an inaccurate tracing can lead to misinterpretation. Removing chest leads does not solve the problem.

3395. A client on continuous telemetry has a rhythm strip showing a heart rate of 148/min, regular rhythm, narrow QRS complexes, and P waves that are difficult to distinguish. The client reports palpitations but is awake and talking. Which rhythm is most consistent with these findings?

- A.** Sinus tachycardia
- B.** Atrial fibrillation
- C.** Supraventricular tachycardia
- D.** First-degree AV block

Correct Answer: C

SVT commonly presents as a very rapid, regular, narrow-complex rhythm in which P waves may be hidden or hard to see. Sinus tachycardia usually still has visible P waves before each QRS. Atrial fibrillation is irregular, not regular. First-degree AV block has a prolonged PR interval, not a rapid tachyarrhythmia like this.

3396. A nurse is preparing a client for outpatient Holter monitoring. Which statement by the client indicates a need for further teaching?

- A.** I should keep a diary of symptoms and activities while wearing the monitor
- B.** I will press the event marker if I feel palpitations
- C.** I can remove the monitor for my shower and put it back on afterward
- D.** I should wear loose clothing to help protect the leads

Correct Answer: C

Holter monitors are intended to record continuously over the prescribed period, so the client should not remove the monitor unless specifically instructed. Keeping a symptom diary, using the event marker if available, and wearing loose clothing are appropriate. The trap is confusing Holter monitoring with devices that may have different use instructions.

3397. A telemetry alarm sounds for asystole in a client who is sitting up in bed and talking with the nurse. Which action should the nurse take first?

- A.** Begin chest compressions immediately
- B.** Check the client and the monitor leads for loose or disconnected electrodes

- C. Call the rapid response team
- D. Print the rhythm strip for the medical record

Correct Answer: B

When the monitor shows a lethal rhythm but the client appears stable and responsive, the nurse should first assess the client and equipment for artifact or lead disconnection. A talking client is not in true asystole. Starting compressions or calling a response team before checking the client and leads would not be the best first action.

3398. A nurse reviews a rhythm strip that shows a regular atrial rate and a regular ventricular rate, but the P waves and QRS complexes have no consistent relationship to each other. Which interpretation is most appropriate?

- A. Second-degree AV block type I
- B. Second-degree AV block type II
- C. Third-degree AV block
- D. Sinus rhythm with premature atrial contractions

Correct Answer: C

Third-degree AV block is complete heart block, so atrial and ventricular activity occur independently, with no consistent PR relationship. In second-degree blocks, some atrial impulses are still conducted. PACs do not cause complete AV dissociation.

3399. A client receiving digoxin reports nausea and weakness. Which ECG finding would most increase the nurse's concern about possible digoxin toxicity?

- A. Sinus arrhythmia
- B. Frequent ventricular ectopy
- C. Isolated sinus tachycardia
- D. Occasional premature atrial contractions

Correct Answer: B

Digoxin toxicity can produce dysrhythmias, including ventricular ectopy. In a client with symptoms such as nausea and weakness, frequent ventricular ectopy raises concern and needs prompt follow-up. The other findings are less specifically concerning for digoxin toxicity in this context.

3400. While attaching a client to telemetry, the nurse notes that the skin is moist from perspiration. Which action is best to improve electrode adherence and tracing quality?

- A. Apply lotion under each electrode
- B. Dry the skin thoroughly before applying the electrodes
- C. Place the electrodes over the client's gown to protect the skin
- D. Tape the monitor wires tightly to the chest instead of using electrodes

Correct Answer: B

Moisture interferes with adhesion and conduction. Drying the skin before applying electrodes improves contact and tracing quality. Lotion interferes with adhesion, electrodes should not be placed over clothing, and wires alone do not replace proper electrode contact.

3401. A nurse obtains a rhythm strip that shows a repeating pattern of one normal beat followed by one premature wide QRS complex without a preceding P wave. Which term best describes this pattern?

- A. Ventricular bigeminy
- B. Trigeminy
- C. Atrial flutter
- D. Junctional escape rhythm

Correct Answer: A

Bigeminy means every other beat is premature, here described as a wide QRS complex without a preceding P wave, which is consistent with a PVC. Trigeminy would involve a premature beat every third beat. Atrial flutter and junctional escape rhythms do not fit the stated pattern.

3402. A client with dizziness has a rhythm strip showing progressive lengthening of the PR interval until a QRS complex is dropped. Which rhythm should the nurse identify?

- A. First-degree AV block
- B. Second-degree AV block type I
- C. Second-degree AV block type II
- D. Third-degree AV block

Correct Answer: B

Second-degree AV block type I, also called Wenckebach, is characterized by progressive PR prolongation followed by a nonconducted P wave and dropped QRS. First-degree AV block has no dropped beats. Type II has dropped QRS complexes without progressive PR lengthening. Third-degree block has complete AV dissociation.

3403. A nurse is preparing to record a posterior ECG after the provider requests additional leads for a client with suspected myocardial ischemia. Which action is appropriate?

- A. Place the posterior chest leads on the client's back as directed for posterior views
- B. Use only the existing limb leads because posterior leads are not part of ECG assessment
- C. Move all limb leads to the upper chest to improve signal strength
- D. Apply fewer precordial leads because posterior views replace the need for anterior leads

Correct Answer: A

Posterior ECG assessment requires placing additional posterior leads on the back in the specified positions. The other options describe incorrect technique or misunderstand the purpose of additional lead views. This question tests recognition of the need for correct lead placement when extended views are ordered.

3404. A telemetry strip shows a slow rhythm at 38/min with narrow QRS complexes and no visible P waves before the QRS complexes. The client is lightheaded. Which rhythm is most likely?

- A. Junctional rhythm
- B. Normal sinus rhythm
- C. Atrial flutter
- D. Sinus tachycardia

Correct Answer: A

A junctional rhythm often has a rate of 40 to 60/min, narrow QRS complexes, and absent or inverted P waves that may be hidden. The slow rate and lack of visible P waves fit this pattern. Normal sinus rhythm requires a P wave before each QRS. Atrial flutter usually shows flutter waves, and sinus tachycardia is fast, not slow.

3405. A nurse sees a rhythm strip with a chaotic waveform, no identifiable P waves, no measurable PR interval, and no recognizable QRS complexes. What is the priority nursing action?

- A. Obtain a blood pressure manually and reassess in 15 minutes
- B. Check the client for responsiveness and pulse immediately
- C. Ask the client to cough forcefully
- D. Turn off the alarm and repeat the strip

Correct Answer: B

A chaotic rhythm with no identifiable complexes suggests ventricular fibrillation, which requires immediate assessment of the client for responsiveness and pulse so emergency action can begin without delay. Waiting, silencing the alarm, or instructing coughing is not appropriate as the first response to this potentially fatal rhythm.

3406. A client being monitored after a myocardial infarction has increasing numbers of PVCs, including two occurring back-to-back. How should the nurse interpret the two consecutive PVCs?

- A. A couplet
- B. Bigeminy
- C. A fusion beat
- D. A paced rhythm

Correct Answer: A

Two PVCs in a row are called a couplet. Bigeminy refers to alternating normal beats and PVCs. A fusion beat is a hybrid complex, and a paced rhythm involves pacemaker-generated activity. Recognizing escalating ventricular ectopy is important because it may indicate increasing irritability.

3407. A nurse reviews a telemetry strip with sawtooth atrial activity and a ventricular rate of 78/min. Which rhythm is most likely?

- A. Atrial flutter
- B. Atrial fibrillation
- C. Ventricular tachycardia
- D. Sinus bradycardia

Correct Answer: A

Sawtooth atrial activity is classic for atrial flutter. Atrial fibrillation lacks organized atrial activity and produces an irregularly irregular rhythm. Ventricular tachycardia is typically a rapid wide-complex rhythm, and sinus bradycardia does not show flutter waves.

3408. A client suddenly becomes unresponsive while on telemetry. The monitor shows a regular wide-complex rhythm at 170/min. No pulse is palpated. Which interpretation best fits this situation?

- A. Stable supraventricular tachycardia
- B. Pulseless ventricular tachycardia
- C. Sinus tachycardia with artifact
- D. Atrial fibrillation with aberrancy

Correct Answer: B

A regular wide-complex tachycardia in an unresponsive client without a pulse is most consistent with pulseless ventricular tachycardia, a cardiac arrest rhythm requiring immediate resuscitative response. Stable SVT would have a pulse and usually narrow complexes. Artifact does not explain an unresponsive pulseless client.

3409. A nurse is obtaining a 12-lead ECG for a client with intermittent chest pressure. To reduce artifact and improve tracing accuracy, which instruction is best to give the client during the recording?

- A. Hold your breath until the machine finishes
- B. Remain still and breathe normally
- C. Tighten your arm and leg muscles if you feel shaky
- D. Talk through your symptoms while the tracing is printing

Correct Answer: B

The client should remain still and breathe normally during ECG recording to reduce muscle and motion artifact. Holding the breath is unnecessary, and talking or tightening muscles can create artifact that interferes with interpretation.

3410. A nurse reviews a rhythm strip for a client who reports mild palpitations. The strip shows an irregularly irregular rhythm, no distinct P waves, and narrow QRS complexes. Which rhythm should the nurse identify?

- A. Atrial fibrillation
- B. Sinus arrhythmia
- C. First-degree AV block
- D. Ventricular tachycardia

Correct Answer: A

Atrial fibrillation is characterized by an irregularly irregular rhythm with no distinct P waves and usually narrow QRS complexes. Sinus arrhythmia still has identifiable P waves. First-degree AV block has a prolonged PR interval, not absent P waves. Ventricular tachycardia is typically a regular wide-complex rhythm.

3411. A client on telemetry has a rhythm strip showing a rate of 44/min. Each QRS complex is preceded by a uniform P wave, and the PR interval is constant. The client says, "I feel a little tired," but is alert and warm with a blood pressure of 118/70 mm Hg. Which interpretation is most appropriate?

- A. Sinus bradycardia
- B. Junctional rhythm
- C. Second-degree AV block type I
- D. Atrial flutter

Correct Answer: A

Sinus bradycardia has a rate below 60/min with a regular rhythm, a P wave before each QRS, and a consistent PR interval. A junctional rhythm often has absent or inverted P waves. Second-degree AV block type I shows progressive PR prolongation with a dropped beat. Atrial flutter produces flutter waves rather than one normal P wave before each QRS.

3412. A nurse is preparing to place precordial electrodes for a 12-lead ECG. Correct placement of lead V1 is at which location?

- A. Fourth intercostal space at the right sternal border
- B. Fourth intercostal space at the left sternal border
- C. Fifth intercostal space at the left midclavicular line

D. Fifth intercostal space at the left anterior axillary line

Correct Answer: A

Lead V1 is placed in the fourth intercostal space at the right sternal border. Choice B describes V2, choice C describes V4, and choice D describes V5. Knowing exact placement helps prevent inaccurate ECG findings caused by misplaced chest leads.

3413. A nurse notes frequent artifact on a telemetry tracing despite the client lying quietly in bed. Inspection shows the electrode patches are in place, but the cable connection is loose at one lead. Which action should the nurse take first?

- A.** Document the rhythm as uninterpretable and wait for the next scheduled strip
- B.** Reconnect the loose lead and reassess the tracing quality
- C.** Administer oxygen because artifact may indicate hypoxemia
- D.** Call the provider to report a suspected dysrhythmia

Correct Answer: B

When artifact is likely caused by a loose lead connection, the nurse should first correct the technical problem and then reassess the tracing. The priority is to verify whether the abnormal appearance reflects equipment issues or the client's actual rhythm. Reporting a dysrhythmia before troubleshooting can lead to incorrect interpretation.

3414. A nurse reviews a rhythm strip that shows a regular rhythm at 76/min. Each QRS complex is preceded by a P wave, but the PR interval measures 0.28 seconds and remains the same throughout the strip. Which interpretation is most appropriate?

- A.** Normal sinus rhythm
- B.** First-degree AV block
- C.** Second-degree AV block type II
- D.** Wandering atrial pacemaker

Correct Answer: B

First-degree AV block is identified by a PR interval longer than 0.20 seconds that stays constant, with each P wave followed by a QRS complex. Normal sinus rhythm would have a normal PR interval. Second-degree AV block type II includes dropped QRS complexes. Wandering atrial pacemaker has varying P-wave shapes and irregular rhythm.

3415. A client on telemetry suddenly reports feeling faint. The monitor shows a very rapid, regular rhythm with wide QRS complexes. Before calling the provider or initiating further interventions, which action should the nurse take first to reduce the risk of acting on an incorrect monitor reading?

- A.** Increase the alarm volume and continue observing the monitor
- B.** Print another strip from a different telemetry channel only
- C.** Assess the client and check for a pulse
- D.** Turn off the telemetry alarm to avoid startling the client

Correct Answer: C

The nurse should first assess the client and check for a pulse to determine whether the telemetry finding matches the clinical condition. This reduces the risk of responding to artifact or an inaccurate reading as though it were a true lethal rhythm. Monitor data are important, but treatment decisions must be based on both the rhythm and the client's status.

Venipuncture

3416. A nurse is preparing to collect a venous blood specimen. Which action best reduces the risk of hemoconcentration and an inaccurate laboratory result?

- A.** Keep the tourniquet in place until all tubes are filled
- B.** Release the tourniquet as soon as blood flow is established
- C.** Ask the client to pump the fist repeatedly during the draw
- D.** Use the smallest available needle for every specimen

Correct Answer: B

Releasing the tourniquet once blood flow is established helps reduce venous stasis and hemoconcentration, which can alter test results. Keeping the tourniquet on too long and repeated fist pumping can both affect specimen accuracy. Needle size should match the situation, but using the smallest needle routinely is not the best way to reduce this specific risk.

3417. A nurse needs to collect blood by venipuncture from an adult client. Which site is generally preferred for routine venipuncture because it is usually well anchored and has a lower risk of injury to nearby structures?

- A. Median cubital vein
- B. Radial artery
- C. Dorsal vein of the foot
- D. Cephalic vein near the wrist

Correct Answer: A

The median cubital vein is commonly preferred for routine venipuncture because it is often stable and easier to access. The radial artery is used for arterial sampling, not routine venous collection. Foot veins are generally avoided unless specifically indicated, and areas near the wrist may be less stable and closer to sensitive structures.

3418. During venipuncture, the nurse cleans the client's skin with antiseptic and is ready to insert the needle. Which next action is most appropriate?

- A. Blow on the site to help it dry faster
- B. Palpate the site again with an ungloved finger
- C. Allow the site to air-dry before needle insertion
- D. Insert the needle immediately while the site is still wet

Correct Answer: C

The antiseptic should be allowed to air-dry to support infection prevention and reduce discomfort during insertion. Blowing on the site contaminates it, and touching the cleaned site without maintaining aseptic technique recontaminates it. Inserting through wet antiseptic is not the best practice.

3419. A client says, "I feel dizzy," immediately after a venipuncture attempt begins. What should the nurse do first?

- A. Continue the draw quickly so the specimen is not wasted
- B. Remove the needle and assist the client to a safe position
- C. Ask the client to take deep breaths while keeping the needle in place
- D. Tighten the tourniquet to finish the collection faster

Correct Answer: B

If a client becomes dizzy during venipuncture, the priority is safety. The nurse should stop the procedure, remove the needle, and help the client into a safe position to prevent a fall or further injury. Continuing the draw or tightening the tourniquet delays appropriate action.

3420. A nurse is drawing blood from a client with fragile veins. The vein collapses as the first tube begins to fill. Which action is most appropriate for the nurse to take on the next attempt?

- A. Use gentler suction or a smaller collection device as appropriate
- B. Apply the tourniquet more tightly than before
- C. Ask the client to pump the fist continuously
- D. Probe with the needle until the vein is found again

Correct Answer: A

A collapsed vein can result from excessive suction, especially in fragile veins. Using gentler suction or a smaller collection device may help on the next attempt. Tightening the tourniquet excessively, repeated fist pumping, and probing increase risk and do not address the likely cause safely.

3421. The nurse must collect a blood specimen from a hospitalized client. Which extremity should the nurse avoid using for venipuncture without further evaluation?

- A. An arm with a running IV infusion
- B. The nondominant arm
- C. An arm with a visible median cubital vein
- D. An arm the client used earlier for a blood pressure reading

Correct Answer: A

An extremity with a running IV infusion should be avoided because the specimen may be diluted or contaminated by the infusing solution, producing inaccurate results. Nondominant status is not a reason to avoid a site by itself, and a visible vein is often helpful. A prior blood pressure reading does not create the same concern as an active infusion.

3422. A nurse obtains a blood specimen and then notices swelling at the puncture site while the needle is still in the vein. Which interpretation is most accurate?

- A. The specimen is clotting in the tube
- B. The client is having a vasovagal reaction
- C. Blood is leaking into the surrounding tissue
- D. The tourniquet was released too early

Correct Answer: C

Swelling at the puncture site during venipuncture suggests infiltration of blood into surrounding tissue, forming a hematoma. This is a local complication at the site. A vasovagal reaction would be more likely to cause symptoms such as dizziness or pallor. Clotting in the tube would not explain immediate local swelling.

3423. A client has an order for blood tests, and the nurse notes an existing hematoma in the left antecubital area from a previous draw. Which action is best?

- A. Draw above the hematoma in the same area
- B. Select a different site if possible
- C. Massage the hematoma, then use that vein
- D. Insert the needle through the bruised area because the vein is visible

Correct Answer: B

A site with a hematoma should be avoided if another site is available because blood in the tissues and local vessel injury can affect specimen quality and increase discomfort or complications. Drawing through or near the hematoma is poor practice, and massaging the area can worsen tissue injury.

3424. After completing venipuncture, which instruction should the nurse give the client to help reduce bruising at the puncture site?

- A. Bend the arm tightly at the elbow for 5 minutes
- B. Rub the site firmly to keep blood moving
- C. Apply direct pressure without heavy rubbing
- D. Place the arm below heart level immediately

Correct Answer: C

Direct pressure helps achieve hemostasis and reduces bruising. Bending the arm tightly can be less effective than direct pressure and may contribute to hematoma formation. Rubbing the site can disrupt clotting and worsen bruising. Lowering the arm is not the key preventive measure here.

3425. A nurse is obtaining blood by venipuncture. The blood stops flowing into the tube after a brief initial flash. Which action should the nurse take first?

- A. Pull the needle out and reinsert it repeatedly until flow resumes
- B. Assess whether the needle bevel may be against the vein wall
- C. Shake the tube vigorously to reestablish flow
- D. Apply a second tourniquet below the first one

Correct Answer: B

When blood flow stops after it starts, one common cause is that the bevel is against the vein wall or the needle position has shifted. The nurse should first assess needle position. Repeated reinsertion in the same stick increases trauma, shaking the tube does not restore venous flow, and a second tourniquet is unsafe and unnecessary.

3426. A nurse is preparing to draw blood from an older adult whose veins are mobile and difficult to stabilize. Which technique is most appropriate?

- A. Anchor the vein by pulling the skin taut below the site
- B. Tap the vein repeatedly after needle insertion
- C. Insert the needle at a steep angle to prevent rolling
- D. Release traction on the skin as soon as the needle touches it

Correct Answer: A

For mobile veins, anchoring the vein by pulling the skin taut below the insertion site helps keep the vein from rolling. Tapping after insertion is not an appropriate way to stabilize the vein. A steep angle increases the chance of going through the vein, and releasing traction too early reduces control.

3427. The nurse must collect blood for testing from a client who has had a mastectomy on the right side and has an IV infusing in the left forearm. Which action is best?

- A. Use the right arm because it has no IV
- B. Use the left arm below the IV site without further planning
- C. Pause briefly and obtain guidance for an alternate site or method
- D. Use whichever arm has the most visible vein

Correct Answer: C

This situation presents risks with both upper extremities: one side after mastectomy and the other with an active infusion. The safest response is to pause and seek guidance for an alternate site or collection method rather than choosing casually between two higher-risk options. The distractors ignore the need to reduce harm and avoid inaccurate specimens.

3428. A nurse is collecting a venous blood sample. After the tubes are filled, which sequence best reduces the risk of bleeding and needlestick injury?

- A. Remove the tourniquet, withdraw the needle, and apply pressure
- B. Apply pressure, remove the needle, and then release the tourniquet
- C. Withdraw the needle, recap it, and then release the tourniquet
- D. Release the tourniquet, apply the label, and then withdraw the needle

Correct Answer: A

Once collection is complete, releasing the tourniquet before withdrawing the needle helps reduce venous pressure and bleeding, then withdrawing the needle and applying pressure promotes hemostasis. Applying pressure before needle removal is not possible, recapping is unsafe, and labeling should not delay safe needle removal and bleeding control.

3429. A client on anticoagulant therapy has just had blood drawn by venipuncture. Which nursing action is most important after needle removal?

- A. Apply pressure longer than usual and reassess for bleeding
- B. Ask the client to rub the site to prevent clot formation
- C. Place a warm compress on the site immediately
- D. Encourage the client to flex the arm repeatedly

Correct Answer: A

Clients taking anticoagulants are at increased risk for prolonged bleeding and hematoma formation, so the nurse should hold pressure longer and reassess the site. Rubbing, repeated flexing, and immediate warming can worsen bleeding or bruising rather than control it.

3430. While selecting a site for venipuncture, the nurse sees an area that is red, warm, and tender. What is the best rationale for avoiding that site?

- A. It may be more painful and may increase infection-related risk
- B. It guarantees that the specimen will hemolyze
- C. It means the vein is too deep to access safely
- D. It prevents blood from entering the collection tube at all

Correct Answer: A

A red, warm, tender area may indicate inflammation or infection, so using that site can increase pain and risk. The finding does not guarantee hemolysis, does not necessarily mean the vein is too deep, and does not always completely prevent blood flow. The best answer focuses on the realistic safety concern.

3431. A nurse is preparing to collect several blood specimens by venipuncture. The client asks why the nurse is checking the labels before the draw instead of after the tubes are filled. Which response best reflects safe practice?

- A. Labeling must be completed before the puncture so the nurse will not forget the room number

- B.** Verifying identifiers before the draw helps ensure the specimen is collected from the correct client
- C.** Laboratory tubes cannot be used unless the labels are attached before blood enters them
- D.** Checking labels first mainly helps the laboratory decide which tests should be run first

Correct Answer: B

The main safety reason to verify identifiers before venipuncture is to make sure the specimen is being collected from the correct client. Wrong-client specimens can lead to serious diagnostic and treatment errors. Choice A is too narrow, choice C is inaccurate, and choice D focuses on lab workflow rather than client identification.

3432. A nurse is collecting a fasting blood specimen. As the nurse prepares the equipment, the client says, "I had coffee with sugar about an hour ago, but that should be okay." Which action is best?

- A.** Proceed with the draw and note that the client drank coffee
- B.** Dilute the specimen request with a second tube to improve accuracy
- C.** Delay collection and notify the appropriate person that fasting was not maintained
- D.** Proceed only if the client has no symptoms of hyperglycemia

Correct Answer: C

If a specimen requires fasting and the client has eaten or consumed sugared coffee, the nurse should delay collection and report that fasting instructions were not followed. Collecting the specimen anyway may produce misleading results. A note does not correct an invalid prep condition, and choices B and D do not address specimen integrity.

3433. A nurse applies a tourniquet before venipuncture and is then delayed while the client answers a phone call. Which action is most appropriate?

- A.** Leave the tourniquet in place until the conversation ends to preserve vein visibility
- B.** Remove the tourniquet and reapply it when the nurse is ready to perform the venipuncture
- C.** Tighten the tourniquet to prevent the vein from flattening

- D.** Ask the client to repeatedly clench the fist until the nurse resumes the procedure

Correct Answer: B

The tourniquet should not be left on unnecessarily because prolonged application can alter the specimen and increase discomfort. The nurse should remove it and reapply it when ready to proceed. Choices A, C, and D increase the risk of inaccurate results or client discomfort.

3434. A nurse has just cleansed the venipuncture site with antiseptic. The client reaches toward the area and says, "Let me feel where you're going to poke." Which response by the nurse is best?

- A.** Go ahead, but only use one finger
- B.** That is fine if your hand sanitizer has dried
- C.** Please do not touch it now; the site needs to stay clean before the puncture
- D.** Touching the site is acceptable as long as the vein remains visible

Correct Answer: C

Once the skin is cleansed, the site should remain clean to reduce contamination risk. The best response is to prevent the client from touching the area before puncture. The other options incorrectly allow recontamination of the prepared site.

3435. A nurse must collect blood cultures by venipuncture. Which action is most important to reduce the risk of contaminating the specimen?

- A.** Use the smallest available tube to limit air exposure
- B.** Allow the cleansed site to dry and avoid repalpating it unless sterile technique is maintained
- C.** Ask the client to pump the fist several times before insertion
- D.** Release the tourniquet immediately after skin cleansing

Correct Answer: B

For blood cultures, contamination control is critical. After skin antisepsis, the site should be allowed to dry, and the nurse should avoid touching it again unless appropriate sterile technique is maintained. Choices A, C, and D do not directly address contamination risk and may interfere with proper collection.

3436. A client has an IV infusing in the right forearm. A blood specimen is ordered, and the only other visible veins are in the left hand and the right hand distal to the IV site. Which site is the nurse most likely to choose first?

- A. A vein in the right hand distal to the IV infusion
- B. A vein directly above the IV site in the right forearm
- C. A vein in the left hand
- D. Any vein in the right arm after pausing the IV for 1 minute

Correct Answer: C

The nurse should generally choose an extremity without an active infusion to reduce the risk of diluted or contaminated specimens. A vein in the left hand is the best first choice here. Distal to an IV can still be problematic, and drawing above the IV site is inappropriate. Simply pausing an IV briefly does not automatically make the arm the best option.

3437. A nurse is preparing to draw blood from a client who is anxious and has fainted during previous blood draws. Which action is most appropriate before starting the venipuncture?

- A. Have the client stand so the nurse can quickly observe color changes
- B. Position the client safely, such as seated with support or lying down, before the procedure
- C. Encourage the client to watch the needle insertion to reduce fear
- D. Apply the tourniquet early so the procedure can be completed more quickly

Correct Answer: B

A client with a history of fainting during venipuncture should be positioned safely before the procedure to reduce injury risk if syncope occurs. Choices A and C increase risk, and D does not address the primary safety concern.

3438. During venipuncture, the nurse accidentally punctures through the vein. Which finding would most strongly suggest this has occurred?

- A. Blood enters the tube smoothly and continuously
- B. The client reports a cool sensation in the opposite hand
- C. The site begins to swell and blood flow decreases
- D. The tube fills more quickly than expected

Correct Answer: C

If the needle goes through the vein, blood may leak into surrounding tissue, causing swelling and reduced flow into the collection tube. Smooth continuous filling suggests correct placement. The opposite-hand sensation is unrelated, and unusually fast filling is not the typical sign of going through the vein.

3439. A nurse is unable to obtain blood on the first venipuncture attempt. The client becomes worried and asks whether the nurse will keep trying. Which action best reflects safe practice?

- A. Continue attempts until a specimen is obtained because restarting wastes supplies
- B. Make repeated attempts at any available site to avoid delaying laboratory testing
- C. Limit attempts according to agency policy and seek assistance if needed
- D. Switch immediately to the foot because lower-extremity veins are easier to access

Correct Answer: C

Safe practice is to follow agency policy on the number of attempts and obtain help when needed. Repeated unsuccessful punctures increase pain, bruising, and complications. Choices A and B ignore safety limits, and D suggests a site that requires additional caution and is not the routine next step.

3440. A nurse is preparing to collect a specimen from a client who has extensive scarring over both antecubital areas from old injuries. Why should the nurse avoid drawing through scarred tissue when possible?

- A. Scar tissue can make vein palpation and needle entry less reliable

- B.** Scar tissue causes blood to clot immediately inside the needle
- C.** Scarred areas always indicate current infection
- D.** Laboratory analyzers reject specimens drawn near old scars

Correct Answer: A

Scar tissue can make veins harder to assess, stabilize, and access successfully. This increases the chance of a difficult or unsuccessful draw. Choice B is incorrect, C overstates the finding, and D is not the issue.

3441. A nurse is collecting multiple tubes during one venipuncture. To reduce specimen error, when should each tube be labeled?

- A.** At the bedside after collection using the client's verified identifiers
- B.** Before entering the room so the labels are ready
- C.** After all morning specimens have been delivered to the laboratory
- D.** Only if the laboratory calls to report missing information

Correct Answer: A

Specimens should be labeled at the bedside after collection, using verified client identifiers, to reduce misidentification errors. Prelabeling creates risk if the wrong client is drawn. Delayed labeling also increases the chance of mix-ups.

3442. A nurse is preparing for venipuncture in a client with a history of latex allergy. Which action is most appropriate?

- A.** Use latex gloves but remove them before touching the puncture site
- B.** Verify the allergy and use nonlatex equipment as appropriate
- C.** Proceed as usual because venipuncture equipment does not contact the skin long enough to matter
- D.** Apply two tourniquets so the client has less direct contact with latex

Correct Answer: B

For a client with a latex allergy, the nurse should verify the allergy and use nonlatex supplies as appropriate to reduce exposure risk. Choices A, C, and D fail to address or worsen the hazard.

3443. A nurse inserts the needle and obtains good blood flow. A few seconds later, the client reports sudden tingling that shoots down the arm into the hand. What is the nurse's priority action?

- A.** Advance the needle slightly to improve positioning
- B.** Ask the client to open and close the hand rapidly
- C.** Remove the needle immediately and assess the extremity
- D.** Keep the needle in place and finish collecting the remaining tubes quickly

Correct Answer: C

Sudden radiating tingling or electric-shock-like pain suggests possible nerve involvement. The priority is to stop the procedure, remove the needle, and assess the client. Advancing the needle or rushing to finish could worsen injury.

3444. A nurse needs a coagulation specimen from a client whose venous access is poor. The nurse obtains the sample through a difficult draw and notices the tube filled only partially. What is the greatest concern if the specimen is sent to the laboratory anyway?

- A.** The label adhesive may loosen during transport
- B.** The blood-to-additive ratio may be incorrect, affecting test accuracy
- C.** The specimen will always clot because all coagulation tubes lack additives
- D.** Underfilled tubes are acceptable as long as the puncture was atraumatic

Correct Answer: B

A partially filled coagulation tube can produce an improper blood-to-additive ratio, which may make results inaccurate. That is the main concern. Choice C is false, and D ignores an important specimen-quality issue. Label adhesion is not the key risk here.

3445. A nurse must choose between two possible venipuncture sites in the same client: one vein is easy to see but lies over an area of arm flexion, and the other is slightly less prominent but is on a straighter, more stable segment of vein. Which site is generally the better choice?

- A.** The more visible vein over the area of flexion because visibility outweighs stability
- B.** Either site is equally appropriate if the tourniquet is tight enough
- C.** The straighter, more stable vein because it is less likely to move or be disrupted during the draw
- D.** The vein over the joint because client movement helps keep blood flowing

Correct Answer: C

A straighter, more stable vein is generally preferable because it is easier to access and less likely to shift or be affected by movement during collection. A very visible vein is not always the safest or most reliable choice if it lies in an unstable location such as a flexion area.

3446. A nurse is preparing to obtain a blood specimen by venipuncture from an older adult. One arm has edema and a recent mastectomy on that side. The other arm has no edema or surgical history. Which action is best?

- A.** Use the arm with edema because the swelling makes veins easier to locate
- B.** Use the arm on the side of the mastectomy because it is less painful
- C.** Use the arm without edema or mastectomy history
- D.** Apply a tighter tourniquet to the edematous arm and draw from the largest vein

Correct Answer: C

The best choice is the arm without edema or mastectomy history. Venipuncture should be avoided in edematous tissue because landmarks are harder to assess and specimen quality may be affected. An arm on the side of a mastectomy is also generally avoided when possible because of risk related to impaired lymphatic drainage. Choices A and D incorrectly treat edema as helpful, and B selects a site that should be avoided when another option is available.

3447. While performing venipuncture, the nurse notices the blood stops flowing into the collection tube before the needed amount is obtained. The needle appears to have been inserted correctly at first. Which action should the nurse take first?

- A.** Probe the tissue by moving the needle side to side
- B.** Release the tourniquet and remove the needle immediately without checking anything else
- C.** Slightly adjust the needle position to see whether blood flow resumes
- D.** Pump the client's fist repeatedly to increase venous pressure

Correct Answer: C

A slight needle adjustment is the best first step because the bevel may be against the vein wall or the needle may have shifted slightly out of optimal position. Probing side to side increases tissue trauma and is unsafe. Immediate removal may become necessary, but checking for a simple positional issue comes first if the insertion was initially successful. Repeated fist pumping can alter some specimens and is not the preferred corrective action.

3448. A nurse is collecting a blood specimen by venipuncture and sees a hematoma beginning to form at the site. Which action should the nurse take first?

- A.** Continue the draw because stopping may waste the attempt
- B.** Remove the tourniquet and needle, then apply pressure
- C.** Lower the client's arm below heart level
- D.** Insert the needle farther to re-enter the vein

Correct Answer: B

If a hematoma begins to form, the nurse should stop the procedure by removing the tourniquet and needle and then applying pressure. This limits further leakage of blood into surrounding tissue. Continuing the draw or advancing the needle can worsen the injury. Lowering the arm does not address the source of the problem and may increase bleeding.

3449. A nurse needs to collect several blood specimens by venipuncture. To reduce the chance of hemoconcentration affecting the results, which practice is most appropriate?

- A.** Keep the tourniquet in place until every tube has been completely filled
- B.** Apply the tourniquet only after the needle is inserted into the vein
- C.** Leave the tourniquet on while searching for a vein as long as the client is comfortable
- D.** Release the tourniquet as soon as blood flow is established

Correct Answer: D

The tourniquet should be released as soon as blood flow is established to reduce venous stasis and hemoconcentration. Keeping it on longer than necessary can alter test results. Applying it only after insertion is not standard technique, and leaving it on throughout the search or full collection increases risk of specimen error.

3450. A nurse is choosing a venipuncture site for a client who has a visible vein on the ventral wrist, a palpable median cubital vein in the antecubital area, and a small vein on the thumb side of the forearm. Which site should the nurse choose first?

- A.** The vein on the ventral wrist
- B.** The median cubital vein in the antecubital area
- C.** The small vein on the thumb side of the forearm
- D.** Any of the three sites because they carry similar risk

Correct Answer: B

The median cubital vein is generally preferred because it is usually well anchored and carries less risk of injury than sites such as the ventral wrist, where nerves and other structures are more vulnerable. A small forearm vein may be used if needed, but it is not the first choice here. Choice D ignores important differences in safety among sites.

3451. After a venipuncture is completed and the needle is removed, the nurse asks the client to hold gauze over the site. A few seconds later, the client bends the arm tightly at the elbow. Which response by the nurse is best?

- A.** Tell the client to keep the arm straight while maintaining direct pressure
- B.** Encourage tighter bending because it seals the vein more quickly
- C.** Place a warm compress over the site immediately
- D.** Massage the area to prevent clotting under the skin

Correct Answer: A

The client should keep the arm straight and apply direct pressure. Bending the arm can contribute to bruising and hematoma formation rather than improve hemostasis. Warm compresses are not the immediate routine step after an uncomplicated draw, and massaging the area can worsen bleeding into the tissue.

3452. A nurse is preparing to perform venipuncture on a client who says, "My hands get cold and my veins are always hard to find." Which action is most appropriate to help reduce the risk of an unsuccessful stick?

- A.** Apply a warm compress to the area before attempting the draw
- B.** Ask the client to slap the arm repeatedly over the vein
- C.** Use the smallest visible vein immediately so it does not disappear
- D.** Insert the needle at a steeper angle than usual to compensate for poor access

Correct Answer: A

A warm compress can promote vasodilation and make veins easier to locate, improving the chance of a successful venipuncture. Slapping the arm is not appropriate practice. Choosing the smallest vein increases difficulty and risk of failure, and using an unnecessarily steep angle raises the chance of puncturing through the vein.

Perioperative care

3453. A practical nurse is caring for a client 2 hours after abdominal surgery. Which finding should be reported to the provider first?

- A. Pain rating of 7/10 after coughing
- B. Urine output of 20 mL in the last hour
- C. Temperature of 99.1°F (37.3°C)
- D. Small amount of serosanguineous drainage on the dressing

Correct Answer: B

Urine output of 20 mL/hr is low and may indicate inadequate renal perfusion, hypovolemia, or developing shock after surgery. This requires prompt follow-up. Pain, a mildly elevated postoperative temperature, and a small amount of serosanguineous drainage are common early postoperative findings and are less urgent than possible decreased organ perfusion.

3454. Before surgery, a client says, "I signed the permit, but I still do not understand what operation they are doing." What is the practical nurse's best action?

- A. Witness the signature because the form is already signed
- B. Explain the procedure in simple terms for the surgeon
- C. Notify the registered nurse or provider that the client has questions about the procedure
- D. Tell the client the details will be explained after surgery

Correct Answer: C

If the client does not understand the procedure, the nurse should stop and report the concern to the appropriate licensed team member or provider so informed consent can be properly addressed. The practical nurse may witness a signature according to agency policy, but should not proceed when the client states lack of understanding. The most common trap is choosing B, but the nurse should not assume the provider's role in explaining the procedure-specific risks and benefits.

3455. A client is returning to the surgical unit immediately after general anesthesia. Which assessment takes highest priority?

- A. Auscultating bowel sounds
- B. Checking the operative dressing
- C. Assessing airway patency and respiratory rate
- D. Reviewing the time of the last pain medication

Correct Answer: C

Airway and breathing are the priority immediately after anesthesia because respiratory depression and airway obstruction can be life-threatening. Dressing status and pain management are important, but they come after confirming the client is ventilating adequately. Bowel sounds are not the first priority in the immediate postoperative period.

3456. A client is scheduled for surgery in 1 hour. Which item should the nurse remove before the client goes to the operating room?

- A. Wedding band
- B. Cotton hospital gown
- C. Identification bracelet
- D. Elastic antiembolism stockings

Correct Answer: A

Jewelry such as rings should be removed before surgery to reduce the risk of injury, swelling-related constriction, and loss. The identification bracelet should remain in place for safety. A hospital gown is appropriate, and antiembolism stockings may be prescribed to help prevent venous thromboembolism.

3457. A client is 12 hours postoperative after hip repair. Which finding most strongly suggests deep vein thrombosis?

- A. Cool foot on the operative side with capillary refill of 4 seconds
- B. Sudden shortness of breath and chest pain
- C. Unilateral calf warmth, tenderness, and swelling
- D. Expected incisional pain with movement

Correct Answer: C

Unilateral calf warmth, tenderness, and swelling are classic findings that suggest deep vein thrombosis. Sudden shortness of breath and chest pain are more concerning for pulmonary embolism, which is a different and even more emergent complication. Coolness and delayed capillary refill point more toward arterial circulation problems than venous thrombosis.

3458. A client had a thyroidectomy this morning. Which bedside item is most important for the nurse to have available?

- A. Warming blanket
- B. Suction equipment
- C. Incentive spirometer
- D. Bedside commode

Correct Answer: B

After thyroidectomy, airway compromise can occur because of swelling, bleeding, or laryngeal nerve involvement. Suction equipment should be readily available to support airway management if needed. An incentive spirometer is useful postoperatively, but the most critical bedside preparation is for airway safety.

3459. A postoperative client becomes restless, anxious, and reports feeling "short of breath." The oxygen saturation has decreased from 97% to 89% on room air. What is the nurse's priority action?

- A. Assist the client to ambulate in the hallway
- B. Administer prescribed pain medication
- C. Raise the head of the bed and apply oxygen as prescribed
- D. Offer sips of water to decrease throat dryness

Correct Answer: C

Restlessness, anxiety, dyspnea, and falling oxygen saturation suggest impaired oxygenation. The priority is to improve ventilation and oxygen delivery by positioning the client upright and applying oxygen as prescribed. Pain medication could worsen respiratory depression, and ambulation is not the first step in a client who is actively hypoxic.

3460. The nurse is caring for a client on the first postoperative day after abdominal surgery. Which intervention best helps prevent atelectasis?

- A. Restricting oral fluids
- B. Encouraging coughing, deep breathing, and incentive spirometry
- C. Keeping the client flat in bed for longer rest periods
- D. Limiting movement to reduce incisional pain

Correct Answer: B

Coughing, deep breathing, and incentive spirometry help expand alveoli and reduce the risk of postoperative atelectasis. Keeping the client flat and limiting movement increase pulmonary risk. A common trap is focusing only on rest, but immobility can worsen postoperative respiratory complications.

3461. A client is scheduled for surgery and has been NPO since midnight. The breakfast tray is accidentally delivered, and the client eats several bites of toast. What should the nurse do first?

- A. Document the amount eaten after the client goes to surgery
- B. Tell the client to avoid eating anything else
- C. Notify the registered nurse or provider immediately
- D. Proceed with preoperative preparation as planned

Correct Answer: C

Eating after NPO status creates a significant aspiration risk during anesthesia, so the nurse should report it immediately. Telling the client not to eat more is appropriate but not enough. Proceeding as planned is unsafe because the anesthesia team needs this information before surgery.

3462. A client is 6 hours postoperative after bowel surgery. The provider's prescription reads, "Morphine 2 mg IV every 2 hours PRN pain." Before administering the medication, which assessment is most important?

- A. Pupil size
- B. Respiratory rate
- C. Bowel sounds
- D. Urine color

Correct Answer: B

Opioids can depress respirations, so respiratory rate is the most important assessment before administration. Bowel sounds and urine color may be relevant in broader postoperative care, but they are not the priority assessment tied most directly to immediate opioid safety. Pupil size is less useful than respiratory status in this context.

3463. A client is 24 hours postoperative after a cholecystectomy. Which finding should the nurse recognize as most concerning for hemorrhage?

- A. Blood pressure 88/54 mm Hg and pulse 124/min
- B. Pain of 6/10 at the incision site
- C. Temperature of 100.0°F (37.8°C)
- D. Absent bowel sounds in all quadrants

Correct Answer: A

Hypotension with tachycardia strongly suggests possible bleeding and shock in the postoperative client. Incisional pain is expected, a mild temperature elevation can occur after surgery, and decreased bowel activity may be expected after abdominal surgery. The key cue pattern here is hemodynamic instability.

3464. A client returns from surgery with a nasogastric tube connected to low suction. The client says, "My throat is dry. Can I have something to drink?" Which response by the nurse is best?

- A. Yes, drinking water will help stimulate bowel function
- B. Not yet; I can provide mouth care while your gastrointestinal tract is being monitored
- C. Only carbonated drinks are allowed with a nasogastric tube
- D. You may drink after I clamp the tube for 30 minutes

Correct Answer: B

A client with a postoperative nasogastric tube is often kept NPO until gastrointestinal function is assessed and prescribed orders allow intake. Offering mouth care helps relieve discomfort without increasing aspiration risk or interfering with decompression. The trap is trying to relieve dryness with oral fluids before bowel function and orders support it.

3465. A nurse is transferring a postoperative client from the stretcher to the bed. The client had spinal anesthesia and cannot move the legs well yet. Which action is safest?

- A. Ask the client to stand and pivot quickly to the bed
- B. Move the client with assistance using a transfer aid or draw sheet while keeping alignment
- C. Have the client pull on the nurse's neck for support
- D. Wait until full sensation returns, even if the client must remain on the stretcher for several hours

Correct Answer: B

A client with residual motor weakness after spinal anesthesia should be transferred with adequate assistance and appropriate equipment to prevent falls and injury. Standing or pivoting is unsafe. Having the client pull on the nurse is poor body mechanics and unsafe for both. Delaying transfer for hours is unnecessary if safe assisted transfer is available.

3466. A client 1 day after surgery reports calf pain. Which nursing action is most appropriate?

- A. Massage the calf to improve circulation
- B. Ambulate the client immediately to reduce stiffness
- C. Keep the client in bed and report the finding promptly
- D. Apply a pillow behind the knees for comfort

Correct Answer: C

Calf pain after surgery may indicate deep vein thrombosis. The nurse should limit activity and report the finding promptly to reduce the risk of clot dislodgement. Massaging the calf is contraindicated. Immediate ambulation may be unsafe until the cause is evaluated. A pillow behind the knees can promote venous stasis and is not the best response.

3467. A postoperative client has a prescription for antiembolism stockings. Which observation requires the nurse to intervene?

- A. The stockings are smooth without wrinkles
- B. The toes are pink and warm
- C. The top band is rolled down below the knee

D. The stockings were removed briefly for skin care

Correct Answer: C

Roller stockings can create a constricting band that impairs circulation and increases the risk of skin injury. The nurse should correct this immediately. Smooth stockings, warm pink toes, and brief removal for skin assessment and care are appropriate. The trap is assuming partial rolling makes the stocking more comfortable, when it actually creates pressure.

3468. A client is being prepared for elective surgery. Which preoperative statement by the client requires the nurse to notify the provider because it may increase the risk of surgical bleeding?

- A.** I use ginkgo biloba every day for memory.
- B.** I drank coffee at 6 AM.
- C.** I had a chest x-ray last month.
- D.** I am allergic to adhesive tape.

Correct Answer: A

Ginkgo biloba can increase bleeding risk and should be reported before surgery. Coffee intake may affect NPO status depending on timing and type of surgery, but the question asks specifically about increased surgical bleeding risk. A prior chest x-ray does not suggest bleeding risk, and adhesive allergy is important for skin care planning but not for hemorrhage risk.

3469. A client is 8 hours postoperative after open reduction and internal fixation of a fractured ankle. Which finding should the nurse report immediately as a possible complication of the cast or dressing?

- A.** Pain rated 6 out of 10 at the incision site
- B.** Toes are pale, cool, and difficult to move
- C.** Small amount of bloody drainage on the dressing
- D.** Sleepiness after receiving prescribed opioid medication

Correct Answer: B

Pale, cool toes with decreased movement suggest impaired circulation or possible compartment syndrome, which requires immediate attention. Incisional pain is expected postoperatively, though it still needs management. A small amount of bloody drainage may be expected early after surgery. Sleepiness can occur after opioids, especially if respirations are adequate and the client is arousable.

3470. The nurse is caring for a client on the first day after cataract surgery. Which client action indicates a need for further teaching?

- A.** Using stool softeners as prescribed
- B.** Bending from the waist to tie shoes
- C.** Wearing an eye shield as instructed
- D.** Avoiding coughing when possible

Correct Answer: B

Bending from the waist can increase intraocular pressure and should be avoided after cataract surgery. Stool softeners help prevent straining, which is appropriate. Wearing an eye shield and avoiding activities that raise pressure are also appropriate postoperative measures.

3471. A client is 4 hours postoperative after a transurethral resection of the prostate and has continuous bladder irrigation running. Which finding requires the nurse's immediate intervention?

- A.** Urine is light pink
- B.** The client reports an urge to void
- C.** No drainage is noted from the catheter tubing
- D.** The irrigation solution is infusing as prescribed

Correct Answer: C

No drainage from the catheter during continuous bladder irrigation suggests obstruction from clots or tubing malfunction and can lead to bladder distention and injury. Light pink output is expected. A feeling of needing to void can occur from catheter irritation. Infusing solution as prescribed is not a problem by itself; the key issue is whether drainage is occurring.

3472. A client is scheduled for surgery later today. Which information is most important for the nurse to communicate to the anesthesia team before the procedure?

- A. The client prefers a private room after surgery.
- B. The client wore glasses on admission.
- C. The client had malignant hyperthermia during a prior surgery.
- D. The client usually eats dinner at 5 PM.

Correct Answer: C

A history of malignant hyperthermia is critical because it can be life-threatening when triggered by certain anesthetic agents. The anesthesia team must know this before the procedure. Room preference, glasses use, and usual dinner time are not as urgent or clinically significant for anesthetic safety.

3473. A postoperative client has a prescription for coughing and deep breathing every 2 hours. The client says, "It hurts too much to cough." Which nursing action is best?

- A. Tell the client coughing is optional if breathing is normal
- B. Administer prescribed pain medication and splint the incision during coughing
- C. Ask the provider to discontinue coughing exercises
- D. Delay pulmonary exercises until the next shift

Correct Answer: B

Giving prescribed analgesia and supporting the incision with a pillow improves comfort and promotes effective coughing, which helps prevent postoperative pulmonary complications. Telling the client coughing is optional, delaying care, or seeking to discontinue routine pulmonary hygiene does not address the risk of atelectasis and pneumonia.

3474. A client returns from surgery with an abdominal dressing. Thirty minutes later, the nurse notes a bright red area on the dressing measuring 2 cm. What should the nurse do first to best monitor for continued bleeding?

- A. Remove the dressing and inspect the incision
- B. Reinforce the dressing with sterile gauze
- C. Circle the drainage with the time and date

D. Document the drainage as an expected finding

Correct Answer: C

Circling the drainage and noting the time allows the nurse to determine whether bleeding is increasing. Removing the original dressing is generally avoided unless prescribed because it may disrupt the wound. Reinforcing may be needed if saturation increases, but first the nurse should establish a baseline for comparison. Simply documenting it as expected does not provide ongoing assessment of progression.

3475. A client is 18 hours postoperative after colon surgery. The nurse finds the client confused, with a pulse of 124/min, blood pressure of 86/50 mm Hg, cool clammy skin, and urine output of 15 mL in the past hour. Which complication should the nurse suspect first?

- A. Hemorrhagic shock
- B. Urinary tract infection
- C. Postoperative ileus
- D. Incisional dehiscence

Correct Answer: A

Hypotension, tachycardia, cool clammy skin, decreased urine output, and confusion strongly suggest shock from blood loss. A urinary tract infection would not usually cause this rapid hemodynamic instability. Ileus affects bowel function, and dehiscence involves wound separation; neither best explains the full pattern of poor perfusion.

3476. A client is scheduled for surgery and tells the nurse, "I take insulin every morning, but I wasn't told what to do today." What is the nurse's best action?

- A. Tell the client to take the usual dose with a sip of water
- B. Hold all insulin automatically because the client is NPO
- C. Check the prescription or clarify preoperative medication instructions with the provider
- D. Advise the client to skip insulin and restart it after surgery

Correct Answer: C

Preoperative insulin management should follow the prescribed plan, and if instructions are unclear, the nurse should verify them.

Automatically giving the usual dose or holding all insulin can be unsafe because insulin needs often change when the client is NPO. The safest response is to clarify the order rather than guess.

3477. A client is 2 days postoperative after abdominal surgery and suddenly says, "Something gave way." The nurse observes loops of bowel protruding through the incision. Which action should the nurse take first?

- A. Reinsert the organs gently into the incision
- B. Cover the area with sterile dressings moistened with normal saline
- C. Irrigate the wound with hydrogen peroxide
- D. Place the client in high-Fowler position

Correct Answer: B

This is evisceration. The priority is to protect exposed organs by covering them with sterile dressings moistened with normal saline. The client is typically positioned to reduce tension on the incision, not in high-Fowler position. The nurse should not attempt to reinsert the organs, and hydrogen peroxide would be inappropriate and harmful to exposed tissue.

3478. Which postoperative client is at greatest risk for developing paralytic ileus?

- A. A client 1 day after abdominal surgery receiving opioid analgesics
- B. A client 1 day after cataract surgery using eye drops
- C. A client 2 days after wrist surgery with a splint
- D. A client 3 days after skin lesion removal under local anesthesia

Correct Answer: A

Paralytic ileus is more likely after abdominal surgery and can be worsened by opioid use, both of which slow gastrointestinal motility. The other procedures listed do not place the client at nearly the same risk for loss of bowel motility.

3479. A client underwent a left pneumonectomy 6 hours ago. Which position is best for the nurse to place the client unless otherwise prescribed?

- A. Flat on the back
- B. On the left side
- C. Semi-Fowler position
- D. Trendelenburg position

Correct Answer: C

Semi-Fowler position promotes chest expansion and ventilation after thoracic surgery. Flat and Trendelenburg positions can impair breathing. Side-lying on the operative side may sometimes be prescribed in specific situations, but the safest general postoperative positioning here is semi-Fowler to support respiratory function.

3480. A nurse is completing a preoperative checklist. Which finding would require the nurse to stop the client's transport to the operating room and resolve the issue first?

- A. The client is wearing a hospital gown without undergarments
- B. The client has not voided in the last 2 hours
- C. The client still has a metal hairpin in place
- D. The client received preoperative teaching yesterday

Correct Answer: C

Metal hairpins should be removed before surgery because they can interfere with safety and positioning. A gown without undergarments is expected. Not voiding in the last 2 hours is not automatically a reason to stop transport unless specifically required. Prior teaching is appropriate, not a problem.

3481. A client is 3 hours postoperative after neck surgery. The nurse notes frequent swallowing, a feeling of neck fullness, and increasing restlessness. Which action should the nurse take first?

- A. Offer ice chips to moisten the throat
- B. Inspect the back of the neck and notify the provider
- C. Document expected discomfort after intubation
- D. Assist the client to ambulate in the hallway

Correct Answer: B

Frequent swallowing, neck fullness, and restlessness after neck surgery may signal bleeding and a developing hematoma, including bleeding that can collect behind the neck dressing. The nurse should assess promptly, including the back of the neck, and report the finding. Ice chips and ambulation delay action, and this pattern should not be dismissed as routine throat irritation.

3482. A client is in the postanesthesia period after receiving IV opioids for pain. Which assessment finding requires the most immediate nursing action?

- A. Pain rating of 7 out of 10
- B. Respiratory rate of 8/min and difficult to arouse
- C. Blood pressure of 148/88 mm Hg
- D. Nausea with one small emesis

Correct Answer: B

A respiratory rate of 8/min with decreased arousability suggests opioid-induced respiratory depression, which is an immediate threat to airway and breathing. Pain, mild hypertension, and nausea matter, but they are less urgent than hypoventilation after opioid administration.

3483. A client is scheduled for abdominal surgery in 1 hour. During the preoperative check, the nurse notes the signed consent form is in the chart, but the client says, "I'm not really sure what operation they're doing." What is the nurse's best action?

- A. Witness the client's signature again because the first signature may be invalid
- B. Notify the surgeon because informed consent may not be complete
- C. Explain the procedure to the client so surgery can proceed on time
- D. Send the client to surgery because the consent form is already signed

Correct Answer: B

The surgeon or other authorized provider performing the procedure is responsible for explaining the operation, risks, benefits, and alternatives so the client can give informed consent. A signed form alone is not enough if the client does not understand the procedure. The nurse should notify the surgeon rather than provide the detailed procedural explanation independently. Choice C is a common trap because the nurse can reinforce teaching, but not replace the provider's informed-consent discussion.

3484. A client returns to the surgical unit 2 hours after an open cholecystectomy. Which finding should the nurse report to the provider immediately?

- A. Urine output of 20 mL in the first postoperative hour
- B. Temperature of 37.4°C (99.3°F)
- C. Pain rating of 6 on a 0-to-10 scale at the incision site
- D. Small amount of serosanguineous drainage on the dressing

Correct Answer: A

Urine output of 20 mL in the first postoperative hour is concerning for inadequate renal perfusion or hypovolemia and should be reported promptly. A small amount of serosanguineous drainage, mild temperature elevation, and expected incisional pain are common early postoperative findings. The key distinction is that low urine output may signal compromised circulation or fluid balance.

3485. The nurse is caring for a client on the first postoperative day after hip surgery. To reduce the risk of venous thromboembolism, which intervention is most appropriate to include in the plan of care?

- A. Maintain the client on strict bed rest until pain is absent
- B. Encourage early ambulation and leg exercises as prescribed
- C. Limit oral fluids to prevent bladder distention
- D. Place pillows under both knees while the client rests

Correct Answer: B

Early ambulation and leg exercises help promote venous return and reduce the risk of venous thromboembolism after surgery. Strict bed rest increases stasis, fluid restriction does not prevent thromboembolism, and pillows under the knees can impair venous return. The trap in choice D is that it may sound comfortable, but knee flexion can contribute to venous stasis.

3486. A client had major abdominal surgery this morning. Which assessment finding later that day is most consistent with atelectasis rather than wound infection?

- A. Fever with foul-smelling drainage from the incision
- B. Crackles in both lungs with pink frothy sputum
- C. Low-grade fever and diminished breath sounds at the lung bases
- D. Rigid abdomen with severe incisional separation

Correct Answer: C

Atelectasis commonly appears in the early postoperative period and is associated with low-grade fever and decreased or diminished breath sounds, often at the bases, due to shallow breathing. Foul-smelling drainage suggests infection, pink frothy sputum suggests pulmonary edema, and abdominal rigidity with separation points to a wound complication. The question requires matching the timing and the expected respiratory findings of atelectasis.

3487. A nurse is preparing a client for surgery. The provider has prescribed a preoperative medication to reduce secretions and promote sedation. The client asks why the medication is being given before going to the operating room. Which response by the nurse is best?

- A. "It helps lower the chance of nausea after surgery."
- B. "It is used to relax you and decrease body secretions before anesthesia."
- C. "It prevents infection during and after the procedure."
- D. "It replaces the anesthetic you will receive in surgery."

Correct Answer: B

Preoperative medications may be used to reduce secretions and help the client relax before anesthesia. This explanation best matches the purpose described in the prescription.

Antiemetics may reduce nausea, but that is not the best answer to a medication specifically intended to reduce secretions and promote sedation. Antibiotic prophylaxis prevents infection, and preoperative sedatives do not replace anesthesia.

3488. A client is 12 hours postoperative after bowel resection. The nurse notes abdominal distention, absent bowel sounds, nausea, and repeated small-volume bilious vomiting. Which prescription should the nurse question if newly written for this client?

- A. Insert a nasogastric tube to low suction
- B. Encourage ambulation as tolerated
- C. Advance diet to regular as tolerated
- D. Monitor intake and output every 4 hours

Correct Answer: C

This client's findings suggest decreased gastrointestinal motility, such as postoperative ileus. Advancing the diet to regular would be inappropriate and could worsen nausea, vomiting, and distention. Nasogastric decompression, ambulation, and intake/output monitoring are reasonable measures. The challenge is recognizing that the bowel is not ready for regular intake despite being postoperative.

3489. A client is in the postanesthesia period after receiving general anesthesia. Which finding requires the nurse's immediate intervention?

- A. The client is difficult to arouse and has noisy respirations
- B. The client reports incisional pain rated 7 out of 10
- C. The client has blood pressure of 144/88 mm Hg
- D. The client feels cold and is shivering

Correct Answer: A

Difficulty arousing with noisy respirations suggests airway obstruction or impaired ventilation, which takes priority immediately after anesthesia. Airway and breathing problems are more urgent than pain, mild blood pressure elevation, or shivering. The most likely trap is choosing pain because it is important, but compromised airway status is the higher priority.

Tubes and IV access

3490. A client has a newly inserted nasogastric tube connected to low intermittent suction. Which finding requires the nurse to take action first?

- A. The client reports a sore throat
- B. The drainage is bright red
- C. The suction container has 150 mL of green fluid
- D. The client asks when the tube will be removed

Correct Answer: B

Bright red drainage may indicate active bleeding and requires prompt assessment and intervention. A sore throat is common after insertion, green drainage can be expected with gastric contents, and questions about removal are not urgent.

3491. The nurse is preparing to discontinue a peripheral IV catheter. Which action is most appropriate after the catheter is removed?

- A. Apply pressure with a sterile gauze pad until bleeding stops
- B. Massage the site to prevent clot formation
- C. Place the extremity in a dependent position
- D. Flush the site with normal saline

Correct Answer: A

After a peripheral IV is removed, the nurse should apply pressure with sterile gauze until bleeding stops, then apply a dressing. Massaging can damage tissue, placing the extremity in a dependent position may increase bleeding, and the site is not flushed after catheter removal.

3492. A client receiving IV fluids through a peripheral line reports pain at the insertion site. The nurse notes pallor, coolness, and swelling around the catheter. What is the nurse's best action?

- A. Slow the infusion rate and reassess in 30 minutes
- B. Discontinue the IV and restart in another site if needed
- C. Apply a tourniquet above the site
- D. Encourage the client to flex the extremity

Correct Answer: B

Pain, pallor, coolness, and swelling suggest infiltration. The correct response is to stop the infusion, discontinue the IV, and restart elsewhere if therapy must continue. Slowing the rate does not correct the problem, and a tourniquet or repeated flexion could worsen tissue injury.

3493. Which client statement shows correct understanding of care for a peripherally inserted central catheter (PICC) line?

- A. I should tell the nurse if the dressing becomes loose or wet
- B. I can disconnect the tubing briefly when I want to shower
- C. If my arm feels sore, I should rub the area around the catheter
- D. It is normal if the line becomes harder to flush each day

Correct Answer: A

A loose or wet dressing increases infection risk and should be reported promptly. The client should not disconnect tubing without direction, should not rub a sore catheter area, and increasing resistance with flushing is not normal and requires assessment.

3494. A client has a chest tube connected to a closed drainage system after a pneumothorax. The nurse notices gentle tidaling in the water-seal chamber. What does this finding most likely indicate?

- A. The tubing is obstructed
- B. The system is functioning as expected
- C. There is a large air leak in the system
- D. The suction pressure is too high

Correct Answer: B

Tidaling in the water-seal chamber generally reflects pressure changes with respiration and indicates system patency. An obstruction may reduce or stop tidaling, a large air leak more commonly causes continuous bubbling, and suction pressure is assessed differently.

3495. The nurse is caring for a client with a peripheral IV infusing in the forearm. Which finding is most consistent with phlebitis rather than infiltration?

- A. Cool, pale skin with edema at the site
- B. Burning pain with redness and warmth along the vein
- C. Damp dressing with clear fluid leakage
- D. Sluggish infusion with no tenderness

Correct Answer: B

Phlebitis is inflammation of the vein and is commonly associated with redness, warmth, tenderness, and a burning sensation along the vein path. Coolness, pallor, and edema suggest infiltration. A damp dressing alone is nonspecific, and sluggish infusion without tenderness does not best fit phlebitis.

3496. A client has a Salem sump tube for gastric decompression. Which nursing action is appropriate to help the tube function properly?

- A. Clamp the blue air vent when transporting the client
- B. Irrigate the blue air vent with saline at scheduled intervals
- C. Keep the blue air vent above the level of the stomach
- D. Connect the blue air vent directly to wall suction

Correct Answer: C

The blue air vent of a Salem sump should remain above stomach level so it can prevent the tube from adhering to the gastric mucosa. Clamping it or connecting it directly to suction interferes with function, and the air vent is not the lumen routinely used for saline irrigation.

3497. An IV pump alarms "occlusion" during an antibiotic infusion. The client reports no pain, and the site is not swollen. Which action should the nurse take first?

- A. Check the tubing for kinks or a closed clamp
- B. Remove the IV catheter immediately
- C. Increase the pump pressure setting
- D. Document the alarm as equipment malfunction

Correct Answer: A

When an occlusion alarm sounds and the site does not immediately suggest infiltration, the nurse should first check for simple mechanical causes such as kinks or a closed clamp. Immediate removal may be unnecessary, increasing pressure is unsafe, and documenting malfunction before assessment is inappropriate.

3498. A client with a chest tube suddenly becomes short of breath, and the nurse finds the drainage system has tipped over onto the floor. What should the nurse do first?

- A. Clamp the chest tube close to the client
- B. Place the drainage system back upright and assess the client
- C. Strip the tubing to remove air
- D. Disconnect the tube to allow air to escape

Correct Answer: B

The immediate action is to restore the drainage system to an upright position and assess the client and system. Routine clamping can increase the risk of tension pneumothorax, stripping is not an appropriate first response, and disconnecting the tube would break the closed system and increase risk.

3499. The nurse is administering a prescribed blood product through a peripheral IV. Which catheter size is generally most appropriate for this infusion if the client's veins can accommodate it?

- A. 24-gauge
- B. 22-gauge
- C. 18-gauge
- D. 26-gauge

Correct Answer: C

A larger-bore catheter such as an 18-gauge is generally preferred for blood administration because it supports better flow and reduces mechanical stress on blood cells. Smaller catheters may sometimes be used depending on the situation, but 18-gauge is the best answer here.

3500. A nurse is checking placement of a feeding tube before medication administration. Which approach is most appropriate?

- A. Inject air and listen over the stomach for a whooshing sound
- B. Rely on the client's lack of coughing to confirm placement
- C. Use the facility-approved method to verify tube placement before use
- D. Advance the tube 2 inches if gastric contents are not easily aspirated

Correct Answer: C

Tube placement should be verified using the facility-approved method before administering medications or feedings. Air auscultation and absence of coughing are not reliable confirmation methods, and advancing the tube without an order or assessment could be unsafe.

3501. A client with a central venous access device develops sudden dyspnea, chest pain, and anxiety shortly after a tubing change. Which complication should the nurse suspect first?

- A. Air embolism
- B. Catheter migration into a peripheral vein
- C. Expected anxiety related to the procedure
- D. Local site infection

Correct Answer: A

Sudden dyspnea, chest pain, and anxiety occurring after manipulation of central line tubing are classic warning signs of air embolism. Catheter migration and local infection do not usually cause this abrupt cardiopulmonary change, and this should not be dismissed as routine anxiety.

3502. The nurse notes continuous bubbling in the water-seal chamber of a client's chest tube drainage system. What is the priority nursing action?

- A. Increase wall suction
- B. Assess for an air leak in the system
- C. Milk the chest tube vigorously
- D. Lower the drainage system below the floor

Correct Answer: B

Continuous bubbling in the water-seal chamber suggests an air leak and requires assessment of the tubing connections and system integrity. Increasing suction does not correct the leak source, vigorous milking is not appropriate, and the drainage unit should stay below chest level but not on the floor.

3503. A client with a new peripheral IV reports tightness at the site. The nurse sees swelling and blanching but no blood return. Which action is best?

- A. Flush the catheter briskly to restore patency
- B. Stop the infusion and remove the catheter
- C. Tape the catheter more securely
- D. Apply heat and continue the infusion slowly

Correct Answer: B

Swelling, blanching, and absent blood return suggest infiltration. The safest action is to stop the infusion and remove the catheter. Flushing could worsen tissue damage, taping does not address the complication, and continuing the infusion is unsafe.

3504. A client's nasogastric tube becomes accidentally disconnected from suction for an unknown amount of time. The client now has nausea and abdominal distention. What should the nurse do first?

- A. Reconnect the tube and assess tube function and the client
- B. Advance the tube several centimeters
- C. Offer oral fluids to relieve the nausea
- D. Remove the tube and notify the provider afterward

Correct Answer: A

The first step is to reconnect the tube as prescribed and assess both the client and whether the tube is functioning. Advancing the tube without an order or assessment is unsafe, oral fluids may be contraindicated, and immediate removal is not the best first response.

3505. The nurse is preparing to hang a new bag of IV fluid for a client with a peripheral IV. Before connecting the tubing to the catheter, which action best reduces the client's risk for an air embolism?

- A.** Flush the catheter rapidly with 20 mL of air-free saline
- B.** Prime the IV tubing completely so no air remains in the line
- C.** Place the client in a high-Fowler position during the tubing change
- D.** Clamp the tubing only after the new bag is attached

Correct Answer: B

Priming the tubing completely removes air before the fluid is connected to the client, which helps prevent air from entering the vascular system. Rapid flushing is not used to prevent air embolism, and position changes do not replace proper tubing preparation. Waiting to clamp until after attachment could allow air into the line.

3506. A client receiving enteral feedings through a nasogastric tube suddenly begins coughing and says, "It feels like the formula is backing up." Which nursing action is the priority?

- A.** Slow the feeding rate and continue to observe
- B.** Add water to dilute the formula
- C.** Stop the feeding and assess tube placement and tolerance
- D.** Lower the head of the bed to improve comfort

Correct Answer: C

Coughing and a sensation of formula backing up raise concern for poor tolerance or possible aspiration risk. The feeding should be stopped first so the nurse can assess placement and the client's status. Slowing the rate without stopping does not address immediate risk. Lowering the head of the bed increases aspiration risk, and diluting formula is not the priority response.

3507. A client has a chest tube to a water-seal drainage system after thoracic surgery. During assessment, the nurse finds the chest tube has become disconnected from the drainage tubing. What should the nurse do first?

- A.** Place the end of the chest tube in sterile water
- B.** Clamp the chest tube near the insertion site
- C.** Apply petroleum gauze around the insertion site
- D.** Remove the chest tube and cover the site with a dry dressing

Correct Answer: A

If a chest tube becomes disconnected from the drainage system, placing the tube end in sterile water creates a temporary water seal and helps prevent air from entering the pleural space. Clamping can increase the risk of tension pneumothorax and is not the first action in this situation. The insertion site is not the problem here, and the nurse would not remove the chest tube.

3508. The nurse is caring for a client with a newly inserted central venous access device. Which finding at the insertion site should the nurse report immediately?

- A.** A small amount of dried blood under the transparent dressing
- B.** Mild tenderness for several hours after insertion
- C.** Redness, warmth, and purulent drainage
- D.** Bruising around the site that is not expanding

Correct Answer: C

Redness, warmth, and purulent drainage suggest infection and require prompt reporting. A small amount of dried blood, mild post-procedure tenderness, and stable bruising can occur after insertion and are less urgent if not worsening. The key is recognizing signs of local infection at a central line site.

3509. A client is receiving a continuous enteral feeding through a small-bore feeding tube. The prescribed medication is available only as a tablet. What is the nurse's best action?

- A.** Crush the tablet, mix it with the formula, and restart the feeding

- B.** Ask the pharmacy for a liquid or other appropriate formulation
- C.** Dissolve the tablet in sterile water and give it with all other medications together
- D.** Hold the medication because tablets should never be given through feeding tubes

Correct Answer: B

The safest action is to obtain a liquid or otherwise appropriate formulation from the pharmacy. Mixing medication into formula can affect delivery and tube patency. Giving crushed tablets together increases the risk of tube occlusion and medication incompatibility. Some medications can be given through feeding tubes, but the formulation must be appropriate.

3510. A nurse is caring for four clients. Which client should the nurse assess first for a possible complication related to IV access?

- A.** A client with a saline lock who asks when the dressing will be changed
- B.** A client with a peripheral IV who reports burning as potassium is infusing
- C.** A client with a PICC line who wants teaching before discharge tomorrow
- D.** A client receiving maintenance fluids whose IV pump has 25 mL left in the bag

Correct Answer: B

Burning during a potassium infusion may indicate infiltration, irritation, or another infusion-related complication. Potassium is especially concerning because it can be irritating to veins and surrounding tissue, so that client should be assessed first. The other situations are important but not as urgent as active symptoms during a high-risk infusion.

3511. A client has a nasogastric tube attached to low intermittent suction after abdominal surgery. Which assessment finding suggests the client may be developing fluid volume deficit from gastric losses?

- A.** Bounding pulse and elevated blood pressure
- B.** Crackles in both lung bases
- C.** Dry mucous membranes and decreased urine output
- D.** Warm flushed skin and increased appetite

Correct Answer: C

Gastric suction can remove significant fluid, placing the client at risk for dehydration. Dry mucous membranes and decreased urine output are expected findings with fluid volume deficit. Bounding pulse, hypertension, and crackles suggest fluid excess rather than deficit. Warm flushed skin and increased appetite do not point to volume loss.

3512. The nurse is about to obtain a blood specimen from a client's central venous access device for laboratory testing. Which action is most important to help reduce contamination of the specimen and the line?

- A.** Use the distal lumen for every specimen collection
- B.** Scrub the access port thoroughly before accessing it
- C.** Flush the line with heparin before drawing the specimen
- D.** Disconnect the infusion tubing as quickly as possible

Correct Answer: B

Thoroughly scrubbing the access port is a key infection-prevention step and also helps reduce contamination of the blood sample. Using a particular lumen is not the universal priority. Flushing with heparin before drawing can alter the sample, and speed alone is not what makes the procedure safer.

3513. A client with a chest tube after trauma has 75 mL of drainage in the collection chamber at 0800. At 0900, the drainage measures 210 mL. What is the nurse's best action?

- A.** Document the finding as expected early postoperative output
- B.** Lower the drainage system below the level of the chest
- C.** Notify the provider of the increase in drainage
- D.** Milk the tubing to remove suspected clots

Correct Answer: C

An increase from 75 mL to 210 mL in 1 hour suggests a significant rise in chest tube output and should be reported promptly because it may indicate bleeding. The drainage system should already be below chest level, so that is not the key response to this change. Milking the tubing is not routinely done and does not address possible hemorrhage.

3514. The nurse is reinforcing teaching for a client who will receive home enteral feedings through a gastrostomy tube. Which statement by the client indicates a need for further teaching?

- A.** I should keep the head of my bed elevated during the feeding.
- B.** I will flush the tube with water as instructed to help keep it open.
- C.** If the tube seems clogged, I can try to clear it with carbonated soda first.
- D.** I should report persistent nausea or abdominal distention.

Correct Answer: C

Using carbonated soda to clear a clogged tube is not the best teaching point. The client should follow prescribed methods, typically water and agency or provider guidance. Keeping the head elevated, flushing as instructed, and reporting nausea or distention are appropriate measures. The incorrect statement shows the need for more teaching.

3515. A client with a central venous catheter is receiving total parenteral nutrition. The nurse notes the client now has a temperature of 38.6°C (101.5°F), chills, and malaise. Which action is most appropriate first?

- A.** Slow the TPN infusion rate
- B.** Assess the line and client for possible catheter-related infection
- C.** Reinforce the dressing because fever can loosen the adhesive
- D.** Ask dietary services to review the TPN ingredients

Correct Answer: B

Fever, chills, and malaise in a client with a central line and TPN raise concern for catheter-related infection, so the nurse should assess the client and line promptly and proceed according to orders and facility protocol. Slowing TPN does not address the likely problem. Dressing adhesion and TPN ingredients are not the priority concern here.

3516. The nurse must choose a site for a peripheral IV in an adult client who may need intermittent IV antibiotics for several days. Which site is generally the best initial choice if available?

- A.** A vein in the hand on the dominant side
- B.** A vein in the lower extremity near the ankle
- C.** The most distal suitable vein of the upper extremity
- D.** An antecubital vein for long-term comfort and mobility

Correct Answer: C

For peripheral IV insertion, starting with the most distal suitable vein in the upper extremity preserves more proximal sites for later use if needed. Lower extremity sites are generally less desirable in adults. The antecubital area can limit mobility and may not be the best first choice for several days of intermittent therapy. Choosing the dominant hand may reduce comfort and function.

3517. A client's continuous tube feeding has been stopped for an abdominal x-ray. When the client returns, the nurse finds that the feeding tube external length is longer than it was earlier in the shift. What should the nurse do next?

- A.** Restart the feeding at half the previous rate
- B.** Advance the tube to the prior marking and resume the feeding
- C.** Hold the feeding and verify tube placement before use
- D.** Flush the tube with water to test whether it is still in the stomach

Correct Answer: C

A change in external tube length suggests possible tube displacement. The nurse should hold the feeding and verify placement before using the tube again. Advancing the tube without an order or confirmation is unsafe. Flushing and restarting the feeding do not confirm safe placement and may increase aspiration risk.

3518. A nurse is assessing a client with a newly placed peripheral IV. Which finding is most concerning for extravasation of a vesicant medication?

- A. Blood return is brisk and the dressing is dry
- B. The client reports severe burning, and the site is swollen and cool
- C. The vein appears reddened and feels cordlike
- D. The infusion is running slowly while the client bends the wrist

Correct Answer: B

Severe burning with swelling and coolness suggests leakage of IV fluid or medication into surrounding tissue, which is especially dangerous if the medication is a vesicant. A reddened, cordlike vein is more consistent with phlebitis. Slow flow when the wrist is bent may reflect positional obstruction. Brisk blood return and a dry dressing are reassuring findings.

3519. The nurse is caring for a client with a chest tube drainage system after lung surgery. Which action should the nurse avoid because it can increase the client's risk for harm?

- A. Keeping the drainage system below the level of the chest
- B. Assessing for fluctuation in the water-seal chamber
- C. Coiling extra tubing on the bed to prevent traction
- D. Clamping the chest tube routinely during transport

Correct Answer: D

Routine clamping of a chest tube during transport should be avoided because it can obstruct the escape of air or fluid and increase the risk of tension pneumothorax. Keeping the drainage system below chest level and assessing the system are expected. Coiling extra tubing to reduce pulling can be appropriate as long as dependent loops are avoided.

3520. A client with a peripheral IV reports burning at the site while potassium chloride is infusing. The nurse notes slight swelling and coolness around the insertion site. Which action should the nurse take first?

- A. Slow the infusion rate and reassess in 15 minutes
- B. Apply a warm compress over the IV site
- C. Stop the infusion and assess the IV site further
- D. Flush the catheter with normal saline to check patency

Correct Answer: C

Burning, swelling, and coolness suggest infiltration or possible tissue irritation from the IV infusion. The priority is to stop the infusion immediately to prevent more fluid or medication from entering the tissue. Slowing the rate delays needed action. A compress may be useful later depending on the medication and policy, but it is not the first step. Flushing can worsen tissue damage if the catheter is no longer in the vein.

3521. A nurse is preparing to administer a bolus feeding through a gastrostomy tube. Which finding should cause the nurse to hold the feeding and investigate further before giving it?

- A. The client is sitting upright in bed
- B. The client reports nausea and has abdominal distention
- C. The formula has reached room temperature
- D. The tube is secured to the abdomen with tape

Correct Answer: B

Nausea and abdominal distention can indicate poor tolerance, delayed gastric emptying, or another problem that increases the risk of regurgitation and aspiration. The feeding should be held while the nurse assesses further and follows facility policy. Sitting upright, using room-temperature formula, and securing the tube are generally appropriate measures that help reduce complications.

3522. A client has a chest tube connected to a drainage system. During assessment, the nurse sees continuous bubbling in the water-seal chamber. Which interpretation is most appropriate?

- A. The client is having expected drainage after chest surgery
- B. The suction pressure is set too low
- C. There may be an air leak in the system
- D. The chest tube is definitely obstructed by a clot

Correct Answer: C

Continuous bubbling in the water-seal chamber commonly suggests an air leak somewhere in the drainage system or from the client. That finding requires further assessment of the tubing and connections. Drainage amount is assessed in the collection chamber, not by bubbling in the water seal. Suction level is typically reflected in the suction-control area, and obstruction would more likely reduce expected fluctuation rather than cause continuous bubbling in the water-seal chamber.

3523. The nurse is giving medications through a client's feeding tube. The prescription includes phenytoin liquid, docusate liquid, and crushed immediate-release acetaminophen. Which approach is best?

- A. Mix all medications together with formula to reduce tube flushing
- B. Administer each medication separately and flush the tube as directed between medications
- C. Give the medications at the same time to shorten the feeding interruption
- D. Add the medications directly to the feeding bag for slow delivery

Correct Answer: B

Medications given through a feeding tube should generally be administered separately with appropriate flushing between medications to reduce tube occlusion and drug incompatibility. Mixing medications together or adding them to formula can alter absorption, cause interactions, and clog the tube. A common trap is choosing the most time-efficient option, but safe administration takes priority.

3524. A provider prescribes removal of a client's nasogastric tube. Which instruction should the nurse give the client immediately before removing the tube?

- A. Take several quick deep breaths
- B. Sip water through a straw
- C. Cough forcefully
- D. Hold your breath

Correct Answer: D

Having the client hold the breath during nasogastric tube removal helps close the glottis and reduce aspiration risk during withdrawal of the tube. Deep breaths, sipping water, or coughing are not the preferred instructions at the moment of removal and can increase discomfort or aspiration risk.

3525. A client has a peripherally inserted central catheter (PICC) for long-term therapy. While the nurse is changing the dressing, the client suddenly becomes short of breath and anxious. Which action should the nurse take first?

- A. Measure the catheter length at the insertion site
- B. Place the client on the left side with the head lowered and notify the provider
- C. Remove the PICC line immediately
- D. Ask the client to take slow deep breaths while finishing the dressing change

Correct Answer: B

Sudden shortness of breath and anxiety during central line care raise concern for air embolism. The priority is to reduce further air entry and help trap air in the right side of the heart by positioning the client on the left side in Trendelenburg or with the head lowered, then notifying the provider and initiating emergency response per policy. Measuring the line can wait. Removing the catheter may worsen the problem. Finishing the dressing change delays urgent intervention.

3526. A client with a chest tube after a pneumothorax is being repositioned in bed. The drainage system accidentally tips over, and the water-seal chamber is no longer at the marked level. What is the nurse's best action?

- A. Clamp the chest tube until a new drainage system is obtained
- B. Return the system upright and reestablish the prescribed water-seal level
- C. Strip the tubing to clear any air that entered the system
- D. Disconnect the system briefly to allow the chamber to refill correctly

Correct Answer: B

If the drainage unit tips, the nurse should place it upright and restore the water-seal chamber to the prescribed level according to manufacturer directions and facility policy. The water seal is necessary for proper function and to reduce air movement back toward the client. Clamping a chest tube can increase the risk of tension pneumothorax and is not the best routine response. Stripping tubing and disconnecting the system can cause harm.

Focused data collection

3527. A nurse is caring for a client 2 hours after abdominal surgery. Which finding requires the nurse to collect additional focused data first?

- A. Pain rating of 6 on a 0 to 10 scale after coughing
- B. Urine output of 25 mL in the last hour
- C. Heart rate of 88/min with regular rhythm
- D. Dressing with a small area of old drainage

Correct Answer: B

Low urine output after surgery can signal hypovolemia, poor renal perfusion, or another complication, so the nurse should first gather more focused data such as blood pressure, pulse, fluid intake, bladder distention, and overall perfusion status. Pain with coughing is expected to some extent, a regular heart rate of 88/min is not concerning by itself, and a small area of old drainage is less urgent than possible decreased perfusion.

3528. A client with heart failure reports shortness of breath. Which assessment finding should the nurse obtain first to collect focused data about this complaint?

- A. Daily fluid intake for the past week
- B. Presence of crackles in the lung bases
- C. Last bowel movement
- D. Family history of cardiac disease

Correct Answer: B

For current shortness of breath in a client with heart failure, the nurse should first gather focused data about respiratory status, including breath sounds. Crackles may indicate pulmonary congestion and help explain the symptom. Fluid intake history may be useful later, but it is less immediate than assessing present oxygenation and breathing. Bowel pattern and family history do not directly address the acute complaint.

3529. A client with diabetes says, "I feel shaky and sweaty." Which focused assessment should the nurse perform first?

- A. Ask about the client's last eye examination
- B. Check the client's capillary blood glucose level
- C. Review the client's hemoglobin A1C from last month
- D. Inspect the client's feet for skin breakdown

Correct Answer: B

Feeling shaky and sweaty suggests possible hypoglycemia, so the nurse should first obtain a current blood glucose level. That directly evaluates the immediate symptom. An A1C reflects longer-term control, and eye and foot assessments are important but unrelated to the urgent complaint.

3530. A nurse is collecting focused data from a client who returned from surgery 30 minutes ago and now reports increasing incisional pain despite receiving pain medication in the postanesthesia area. Which assessment should the nurse perform first?

- A. Inspect the surgical site and dressing
- B. Ask the client to describe previous pain experiences
- C. Review the client's home medication list
- D. Offer a distraction technique

Correct Answer: A

Unexpectedly increasing postoperative pain can be a sign of bleeding, swelling, or another complication, so focused data collection should begin with inspecting the surgical site and dressing. Previous pain experiences and home medications are less urgent. Distraction is an intervention, not the first focused assessment step when a complication is possible.

3531. The nurse is caring for four clients. Which client requires the most immediate focused neurological assessment?

- A. A client with pneumonia who reports fatigue after ambulating
- B. A client with a casted arm who reports itching under the cast
- C. A client with hypertension who suddenly has slurred speech
- D. A client with arthritis who rates joint pain as 4 out of 10

Correct Answer: C

Sudden slurred speech suggests an acute neurological event and requires immediate focused neurological assessment. Fatigue after activity, itching under a cast, and mild chronic pain are less urgent. The key is recognizing a possible stroke and collecting targeted data quickly.

3532. A client receiving a blood transfusion reports chills and low back pain 10 minutes after the infusion begins. After stopping the transfusion, which focused data should the nurse obtain next?

- A. Lung sounds and oxygen saturation only at the end of the shift
- B. Vital signs and a comparison with the client's baseline status

- C. The client's usual food preferences
- D. Capillary refill in the toes only

Correct Answer: B

After a suspected transfusion reaction, the nurse should obtain focused data about the client's current condition, especially vital signs and how they compare with baseline. This helps identify the severity of the reaction and guides rapid follow-up actions. Waiting until the end of the shift is unsafe, and the other options are not the most relevant immediate data.

3533. A nurse is assessing a client with an indwelling urinary catheter who has a temperature of 38.4°C (101.1°F). Which finding is most important to obtain as focused data related to a possible catheter-associated problem?

- A. The client's favorite beverage
- B. The color, clarity, and odor of the urine
- C. The client's shoe size
- D. The time of the client's last visitor

Correct Answer: B

In a client with a urinary catheter and fever, urine characteristics are focused data that may suggest infection. Color, clarity, and odor are directly relevant. The other options do not help evaluate a possible catheter-associated urinary problem.

3534. A client with chronic obstructive pulmonary disease becomes more restless than usual. Which additional finding would best support the nurse's suspicion of worsening hypoxia?

- A. The client asks for a warm blanket
- B. The client has diminished concentration and new confusion
- C. The client reports a poor appetite for lunch
- D. The client prefers to watch television alone

Correct Answer: B

Restlessness and new confusion can both indicate hypoxia, so this finding best supports the concern and helps the nurse focus further respiratory assessment. Wanting a blanket, poor appetite, and preferring to be alone are far less specific to oxygenation problems.

3535. The nurse is collecting focused data from a client who reports severe right lower quadrant abdominal pain. Which additional finding should the nurse recognize as most concerning?

- A. Pain began suddenly and is worsened by movement
- B. Client states, "I skipped breakfast today."
- C. Client asks for the room lights to be dimmed
- D. Pain improves when talking with family

Correct Answer: A

Sudden abdominal pain that worsens with movement suggests peritoneal irritation and requires prompt focused assessment. Skipping breakfast and dimming the lights are not as clinically significant, and improvement with distraction does not rule out a serious cause.

3536. A nurse notes that a client receiving IV fluids has swelling around the insertion site. Which focused data collection is the priority?

- A. Ask when the client last ate a full meal
- B. Assess the site for coolness, pallor, and discomfort
- C. Determine the client's usual sleep pattern at home
- D. Review the client's immunization history

Correct Answer: B

Swelling at an IV site suggests infiltration or another IV complication. The nurse should collect focused data at the site, such as coolness, pallor, and discomfort, to evaluate tissue involvement. Meal history, sleep pattern, and immunization history do not address the immediate problem.

3537. A client 1 day postpartum tells the nurse, "I'm soaking a pad every 15 minutes." Which focused assessment finding is most urgent to obtain?

- A. Fundal tone and location
- B. The client's usual menstrual cycle pattern
- C. The amount of weight gained during pregnancy
- D. The time of the first prenatal visit

Correct Answer: A

Heavy postpartum bleeding requires focused assessment for hemorrhage, and fundal tone and location are key data because uterine atony is a common cause. Menstrual history, pregnancy weight gain, and timing of prenatal care do not address the immediate bleeding risk.

3538. A nurse is caring for a client with a chest tube connected to suction. During rounds, the client reports increased shortness of breath. Which focused finding requires the most urgent follow-up?

- A. Tidal volume is absent and the client has new tracheal deviation
- B. The drainage chamber contains 40 mL collected earlier in the shift
- C. The client says the tape feels tight on the dressing
- D. Small bubbles are seen in the suction control chamber

Correct Answer: A

New shortness of breath plus tracheal deviation suggests possible tension pneumothorax, a life-threatening complication. Absence of tidal volume may also signal a problem such as obstruction, depending on the clinical context. Drainage already collected earlier, tape discomfort, and expected bubbling in the suction control chamber are less urgent. The nurse is focusing on the chest tube equipment detail without connecting the client's symptoms to a possible emergency.

3539. A client with liver disease becomes increasingly drowsy. Which focused assessment is most important for the nurse to perform next?

- A. Ask whether the client prefers daytime or nighttime bathing
- B. Assess orientation, ability to follow commands, and presence of asterixis
- C. Measure calf circumference bilaterally
- D. Determine the client's favorite high-protein foods

Correct Answer: B

In a client with liver disease, increasing drowsiness raises concern for hepatic encephalopathy. Focused neurological data such as mental status and asterixis are most important. Calf circumference relates more to clot assessment, and food preferences or bathing preferences do not address the immediate change in level of consciousness.

3540. A nurse is collecting focused data from a client with diabetes who has a plantar foot ulcer. Which additional finding most strongly suggests worsening infection rather than stable chronic wound status?

- A.** Pale scar tissue at the ulcer edge with no odor
- B.** Capillary blood glucose of 118 mg/dL before lunch
- C.** New warmth, purulent drainage, and increasing tenderness around the wound
- D.** Dry skin on the heel of the opposite foot

Correct Answer: C

Warmth, purulent drainage, and increasing tenderness are focused findings consistent with worsening local infection. Stable scar tissue without odor is less concerning, a glucose of 118 mg/dL does not explain wound progression by itself, and dry skin on the other heel is not the key cue. The best answer is the option that clusters classic infection signs around the ulcer itself.

3541. A client who had a thyroidectomy earlier today tells the nurse, "My neck feels tight, and it's getting harder to breathe." Which focused assessment should the nurse perform first?

- A.** Inspect the neck dressing and the area behind the neck for swelling or bleeding
- B.** Ask the client to rate the incisional pain on a 0 to 10 scale
- C.** Review the client's preoperative thyroid laboratory values
- D.** Determine when the client last had anything to eat or drink

Correct Answer: A

Neck tightness and increasing difficulty breathing after thyroid surgery raise concern for postoperative bleeding or hematoma formation, which can quickly compress the airway. The priority focused data collection is to inspect the surgical site and behind the neck for swelling or bleeding. Asking about pain may be useful later, but it does not address the immediate airway risk. Reviewing preoperative labs and recent intake does not help the nurse assess the current urgent problem.

3542. A client with a history of heart disease reports, "I have pressure in the middle of my chest." Which focused assessment should the nurse perform first?

- A.** Ask the client about usual exercise habits
- B.** Assess the pain quality, radiation, and associated symptoms
- C.** Review the client's cholesterol results from last year
- D.** Determine whether the client prefers to sit in a chair

Correct Answer: B

Chest pressure can signal acute cardiac ischemia, so the nurse should first gather focused data directly related to the complaint, including the character of the pain, whether it radiates, and symptoms such as nausea, diaphoresis, or shortness of breath. The other options may be useful later but do not address the immediate problem.

3543. A client 12 hours after a tonsillectomy begins swallowing frequently and reports nausea. Which focused assessment is the nurse's priority?

- A.** Inspect the throat carefully for active bleeding if visible
- B.** Ask the client to rate throat pain on a 0 to 10 scale
- C.** Determine the client's favorite cold liquids
- D.** Review the client's immunization record

Correct Answer: A

Frequent swallowing after tonsillectomy can indicate bleeding, so the nurse should quickly assess for hemorrhage. Pain assessment is important, but bleeding is the priority concern. Food preferences and immunization history are unrelated to the acute cue.

3544. A nurse is caring for a client receiving digoxin who reports nausea and poor appetite. Which additional focused data should the nurse collect first?

- A. Apical pulse rate and rhythm
- B. Usual breakfast choices at home
- C. Date of the last dental cleaning
- D. Family history of hypertension

Correct Answer: A

Nausea and poor appetite can occur with digoxin toxicity, and the nurse should first assess apical pulse rate and rhythm because digoxin directly affects cardiac conduction. The other options do not help evaluate this potentially significant medication-related problem.

3545. A client with a below-the-knee cast reports that the toes feel "tingly and tight." Which focused assessment finding is most important for the nurse to obtain next?

- A. Whether the client slept well the night before
- B. Toe color, temperature, capillary refill, movement, and sensation
- C. The client's usual level of physical activity
- D. Whether the client wants extra pillows

Correct Answer: B

Tingling and tightness under a cast raise concern for impaired circulation or nerve compression. The nurse should gather focused neurovascular data, including color, temperature, capillary refill, movement, and sensation. The distractors do not evaluate the threatened extremity.

3546. A client with pancreatitis reports worsening abdominal pain. Which additional finding would require the most urgent follow-up by the nurse?

- A. Pain increases after repositioning
- B. Bowel sounds are hypoactive
- C. The abdomen becomes rigid and boardlike
- D. The client asks for ice chips

Correct Answer: C

A rigid, boardlike abdomen suggests peritoneal irritation or another serious abdominal complication and requires urgent follow-up. Hypoactive bowel sounds and pain with movement may occur with abdominal illness, but they are less immediately alarming than abdominal rigidity.

3547. A client 1 day after a total hip arthroplasty reports sudden shortness of breath. Which focused data should the nurse obtain first?

- A. Inspect the affected leg for calf pain, warmth, and swelling
- B. Ask when the client last participated in physical therapy
- C. Determine the client's usual sleeping position
- D. Review the consent form for surgery

Correct Answer: A

After orthopedic surgery, sudden shortness of breath raises concern for venous thromboembolism. Focused data related to a possible deep vein thrombosis, such as calf pain, warmth, and swelling, can help identify the source of the problem while the nurse responds to the respiratory complaint. The other options do not address the acute risk.

3548. A client with a new ileostomy reports that the pouch has been filling rapidly since surgery. Which focused data collection is most important?

- A. Assess the amount and character of the stoma output
- B. Ask whether the client prefers a one-piece or two-piece pouch
- C. Determine the client's usual dinner schedule
- D. Review the client's childhood medical history

Correct Answer: A

With an ileostomy, rapid filling may reflect high output and fluid loss, so the nurse should first assess the amount and character of drainage. Appliance preference and distant history do not help evaluate the current physiologic concern.

3549. A client receiving heparin has bleeding from the gums while brushing teeth. Which focused assessment should the nurse perform first?

- A. Ask the client which toothpaste brand is used at home
- B. Inspect for additional bleeding and bruising at other sites
- C. Determine the client's favorite snacks
- D. Review the client's vision prescription

Correct Answer: B

Gum bleeding in a client taking heparin may indicate excessive anticoagulation, so the nurse should assess for other signs of bleeding such as bruising, hematuria, or oozing from puncture sites. The distractors do not evaluate the potential adverse effect.

3550. A client with a history of seizure disorder tells the nurse, "I think I might have one soon." Which focused data is most useful to obtain immediately?

- A. Whether the client likes the room lights dim or bright
- B. Presence of an aura or other usual pre-seizure symptoms
- C. The client's preferred television channel
- D. Date of the client's last haircut

Correct Answer: B

If a client senses an approaching seizure, the most useful focused data is whether the client is experiencing an aura or usual warning symptoms, because this helps confirm the cue and guides immediate safety actions. The other options do not contribute to urgent neurologic assessment.

3551. A nurse is caring for a client with a nasogastric tube to low suction who suddenly develops abdominal distention and nausea. Which focused assessment should the nurse perform first?

- A. Check tube placement and patency, including drainage output
- B. Ask the client about favorite foods before hospitalization
- C. Determine the client's bowel pattern from 6 months ago
- D. Offer mouth care and reassess in 2 hours

Correct Answer: A

Abdominal distention and nausea in a client with an NG tube suggest possible obstruction or loss of decompression. The nurse should first assess tube placement, patency, and output. Mouth care is supportive but does not address the likely cause of the problem.

3552. A client with preeclampsia reports a severe headache that is not relieved by rest. Which additional focused data should the nurse obtain first?

- A. Ask what name the client has chosen for the baby
- B. Assess for visual disturbances and epigastric pain
- C. Determine the client's preferred childbirth class schedule
- D. Review the pre-pregnancy clothing size

Correct Answer: B

In preeclampsia, severe headache, visual changes, and epigastric pain are important warning signs of worsening disease and possible seizure risk. These are the most relevant focused data to obtain immediately. The other options are unrelated to the acute maternal condition.

3553. A client with emphysema becomes increasingly sleepy while receiving oxygen at 2 L/min by nasal cannula. Which focused data should the nurse obtain first?

- A. The client's preferred bedtime routine
- B. Respiratory rate, depth, and pulse oximetry
- C. How many pillows the client uses at home
- D. The date of the client's last influenza vaccine

Correct Answer: B

Increasing sleepiness in a client with emphysema may indicate worsening gas exchange or carbon dioxide retention. The nurse should first collect current respiratory data, including rate, depth, and oxygen saturation. Home habits and vaccine history do not address the acute change.

3554. A client with cirrhosis has increasing abdominal girth and shortness of breath. Which focused assessment is most important for the nurse to perform next?

- A. Measure abdominal circumference and assess for shifting dullness

- B. Ask the client which foods are usually eaten on weekends
- C. Review the client's childhood vaccination record
- D. Determine whether the client prefers a private room

Correct Answer: A

Increasing abdominal girth with dyspnea in cirrhosis suggests worsening ascites. Measuring abdominal circumference and assessing for fluid accumulation provide focused data directly related to the client's symptoms. The other options are unrelated to the immediate problem.

3555. A nurse is assessing a client 6 hours after a femoral cardiac catheterization. The client reports sudden groin pain. Which focused data should the nurse obtain first?

- A. Inspect the insertion site for bleeding or hematoma and assess distal pulses
- B. Ask the client to describe the home walking routine
- C. Determine the client's preferred pain medication with prior procedures
- D. Review the client's diet from the previous day

Correct Answer: A

Sudden groin pain after femoral catheterization can indicate bleeding or hematoma formation. The priority focused assessment is inspection of the insertion site and evaluation of distal circulation with pulse checks. The other options do not address a potential vascular complication.

3556. A client with syndrome of inappropriate antidiuretic hormone secretion becomes irritable and develops a headache. Which focused assessment finding is most important for the nurse to obtain next?

- A. The client's favorite salty snack
- B. Current mental status changes and presence of muscle twitching
- C. How often the client exercises at home
- D. The client's preferred time for bathing

Correct Answer: B

SIADH can lead to hyponatremia, which may cause neurologic changes such as irritability, headache, confusion, and muscle twitching. The nurse should gather focused neurologic data first because worsening hyponatremia can progress to seizures. The other options are not clinically useful in the immediate situation.

3557. A client who is 8 hours postpartum tells the nurse, "I'm soaking these pads really fast, and I feel dizzy when I stand up." Which focused assessment should the nurse perform first?

- A. Inspect the perineal pad and palpate the uterine fundus for tone and position
- B. Ask the client to rate the pain on a 0-to-10 scale
- C. Measure the circumference of both calves
- D. Auscultate bowel sounds in all four quadrants

Correct Answer: A

Rapid pad saturation and dizziness suggest postpartum hemorrhage. The most important focused data are the amount of bleeding and whether the uterus is boggy or displaced, because uterine atony is a common cause of early postpartum bleeding. Pain rating, calf measurement, and bowel sounds do not address the most immediate risk.

3558. A client with type 1 diabetes becomes pale, sweaty, and shaky while waiting for lunch. Which focused data should the nurse obtain first?

- A. Urine ketone level
- B. Capillary blood glucose level
- C. Glycosylated hemoglobin result
- D. Daily fluid intake amount

Correct Answer: B

Pallor, diaphoresis, and shakiness are classic cues of hypoglycemia. A bedside blood glucose check gives the most immediate, targeted information for prompt action. Urine ketones and A1C are not the best first focused data in an acute episode, and fluid intake is less directly related.

3559. A client 18 hours after thyroid surgery says, "My neck feels tight." Which focused assessment should the nurse perform first?

- A. Check the surgical dressing and the area behind the neck for swelling or bleeding
- B. Ask whether the client has had a bowel movement
- C. Assess handgrip strength bilaterally
- D. Inspect the urine for cloudiness

Correct Answer: A

Neck tightness after thyroid surgery raises concern for postoperative bleeding or hematoma, which can quickly threaten the airway. The nurse should first inspect the incision area and the back of the neck, where blood may collect. Handgrip, bowel function, and urine appearance do not address the immediate airway-related risk.

3560. A client with a history of heart failure reports waking up at night "gasping for air." Which additional focused data would be most helpful for the nurse to collect next?

- A. Whether the client sleeps with extra pillows or needs to sit upright to breathe
- B. The client's favorite bedtime snack
- C. The date of the client's last influenza vaccine
- D. Whether the client prefers a warm or cool room at night

Correct Answer: A

Needing multiple pillows or sitting upright to breathe helps assess orthopnea and worsening fluid overload in heart failure. That information is directly tied to the symptom of nighttime breathlessness. The other options may be useful in general care but do not meaningfully clarify this respiratory complaint.

3561. A client receiving a packed red blood cell transfusion reports chills and low back pain 10 minutes after the infusion starts. Which focused assessment is the priority?

- A. Ask the client what was eaten for breakfast
- B. Inspect the IV site for mild bruising
- C. Obtain vital signs and assess for fever, hypotension, and dyspnea
- D. Measure abdominal girth

Correct Answer: C

Chills and low back pain shortly after a transfusion begins suggest an acute transfusion reaction. The priority focused data are vital signs and respiratory status to determine severity and support rapid intervention. Breakfast intake, IV bruising, and abdominal girth do not address the likely reaction. A common trap is focusing only on the IV site instead of recognizing a systemic response.

3562. A client 2 days after abdominal surgery becomes restless and confused. The client has been receiving opioid pain medication. Which focused assessment should the nurse obtain first?

- A. Apical pulse for 1 full minute
- B. Respiratory rate and oxygen saturation
- C. Bowel sounds in each quadrant
- D. Urine specific gravity

Correct Answer: B

Restlessness and confusion in a postoperative client receiving opioids may indicate hypoxia or opioid-related respiratory depression. Respiratory rate and oxygen saturation provide the most immediate focused data about ventilation and oxygenation. Pulse, bowel sounds, and urine specific gravity are less urgent than assessing breathing first.

3563. A client with a long-leg cast tells the nurse, "The pain keeps getting worse even after the pain shot." Which additional focused data should the nurse obtain first?

- A. Time of the client's last bowel movement
- B. Whether the client would like the room lights dimmed
- C. Pedal pulse, toe movement, skin color, temperature, and capillary refill of the affected leg
- D. Amount of breakfast the client ate

Correct Answer: C

Pain that is worsening despite analgesia in a casted extremity raises concern for neurovascular compromise or compartment syndrome. The nurse should immediately assess circulation, movement, and perfusion distal to the cast. Bowel habits, lighting preference, and food intake do not evaluate this limb-threatening complication. The trap is treating the problem as routine pain instead of possible impaired perfusion.

Monitoring response to treatment

3564. A client with pneumonia started IV antibiotics 24 hours ago. Which finding best indicates the treatment is effective?

- A. Temperature decreased from 39.2°C (102.6°F) to 37.8°C (100.0°F)
- B. White blood cell count increased from 11,000/mm³ to 15,000/mm³
- C. Client reports pleuritic chest pain is unchanged
- D. Respiratory rate increased from 22/min to 30/min

Correct Answer: A

A decreasing temperature is an expected sign that the infection is responding to therapy. An increased white blood cell count and increased respiratory rate suggest worsening infection or stress. Unchanged chest pain does not show improvement. The best indicator among the options is the lower temperature.

3565. A client with heart failure received furosemide 2 hours ago. Which assessment finding requires the nurse's immediate follow-up as a possible adverse response to treatment?

- A. Urine output of 600 mL since the medication was given
- B. Weight loss of 0.5 kg from the previous day
- C. Blood pressure 88/54 mm Hg with dizziness on standing
- D. Reduced bilateral ankle edema

Correct Answer: C

Hypotension with dizziness suggests excessive fluid loss or hemodynamic instability and requires prompt follow-up. Increased urine output, slight weight loss, and reduced edema are expected therapeutic effects of furosemide. The key is distinguishing an intended diuretic response from an unsafe one.

3566. A client with type 2 diabetes receives regular insulin at 0730. Which finding at 1030 best shows the medication is having the desired effect?

- A. Capillary blood glucose decreased from 268 mg/dL to 154 mg/dL
- B. The client says, "I feel hungry."
- C. Urine output is 450 mL in 4 hours
- D. Heart rate increased from 78/min to 96/min

Correct Answer: A

A drop in blood glucose into a safer range indicates the insulin is working. Hunger alone is nonspecific and may occur for many reasons. Urine output does not directly confirm insulin effectiveness, and an increased heart rate is not an expected therapeutic indicator. The best evidence is the improved glucose value.

3567. A client with chronic obstructive pulmonary disease is receiving oxygen at 2 L/min by nasal cannula. Which finding best indicates the therapy is effective?

- A. The client is more restless than 1 hour ago
- B. Pulse oximetry increased from 86% to 91%
- C. The client reports a dry nose
- D. Heart rate increased from 88/min to 104/min

Correct Answer: B

An improvement in oxygen saturation shows better oxygenation and is the clearest sign the treatment is effective. Restlessness can suggest hypoxia. A dry nose is a side effect, not a measure of effectiveness. A rising heart rate may indicate increased distress rather than improvement.

3568. A client with a urinary tract infection has been taking an oral antibiotic for 3 days. Which finding is the best evidence that the treatment plan is working?

- A. The urine appears darker yellow in the morning

- B. The client reports less burning with urination
- C. The client drinks 2 liters of water daily
- D. The blood pressure is 126/78 mm Hg

Correct Answer: B

Reduced dysuria is a direct sign that the urinary tract infection is improving. Darker urine may reflect concentration, not response to treatment. Fluid intake is helpful but does not itself prove the infection is resolving. Blood pressure is not the best measure of antibiotic effectiveness in this scenario.

3569. A client receiving heparin infusion for a deep-vein thrombosis has a prescription for activated partial thromboplastin time (aPTT) monitoring. Which laboratory result most strongly suggests the nurse should anticipate a change in treatment because of excessive anticoagulation?

- A. aPTT within the prescribed therapeutic range
- B. Platelet count unchanged from baseline
- C. aPTT markedly above the prescribed therapeutic range
- D. Hemoglobin increased slightly from the previous day

Correct Answer: C

A markedly elevated aPTT suggests too much anticoagulant effect and increases bleeding risk, so the infusion may need adjustment. A therapeutic aPTT indicates the treatment is on target. An unchanged platelet count does not indicate excessive heparin effect, and a slight hemoglobin increase does not suggest overanticoagulation.

3570. A client with hypertension started a new antihypertensive medication 1 week ago. Which finding shows the medication is effective without evidence of poor tolerance?

- A. Blood pressure decreased from 168/94 mm Hg to 132/80 mm Hg and the client denies dizziness
- B. Blood pressure decreased from 168/94 mm Hg to 90/56 mm Hg and the client feels weak
- C. Blood pressure remains 166/92 mm Hg and the client has no headache
- D. Blood pressure is 150/88 mm Hg and the client reports a new dry cough

Correct Answer: A

Option A shows the desired therapeutic effect with no indication of intolerance. Option B reflects overcorrection with symptomatic hypotension. Option C shows little meaningful improvement. Option D may show some improvement, but the new cough suggests a possible adverse effect, so it is not the best response to treatment.

3571. A client with bacterial meningitis has been receiving IV antibiotics and fluids. Which finding requires the nurse to question whether the client is responding adequately to treatment?

- A. Temperature is lower than on admission
- B. The client is more difficult to arouse than 2 hours ago
- C. Neck pain is slightly improved
- D. Oral intake remains limited

Correct Answer: B

A declining level of consciousness is a serious sign that the client may not be improving and may be deteriorating neurologically. Lower temperature and reduced neck pain are expected improvements. Limited oral intake may still occur during illness, but worsening responsiveness is the most concerning indicator of inadequate response to treatment.

3572. A client with asthma received a short-acting bronchodilator by nebulizer. Which reassessment finding 20 minutes later best indicates the treatment was effective?

- A. Wheezing is absent and peak expiratory flow is improved
- B. The client is sleepy and respirations are 10/min
- C. The client reports thirst and dry lips
- D. Breath sounds are diminished in all lung fields and the client is quiet

Correct Answer: A

Improved peak expiratory flow with relief of wheezing indicates better airflow after bronchodilator therapy. Sleepiness with slow respirations is concerning, not therapeutic. Thirst is not a useful measure of bronchodilator response. Quiet lungs with diminished breath sounds can signal severely limited air movement, a dangerous finding rather than improvement.

3573. A client with iron-deficiency anemia has been taking oral ferrous sulfate for 1 month. Which finding best indicates a therapeutic response?

- A. The stool is dark green to black
- B. The client reports increased energy during daily activities
- C. The client has mild nausea after taking the tablet
- D. The medication is taken with meals

Correct Answer: B

Improved energy suggests the anemia is improving and oxygen-carrying capacity is recovering. Dark stools and mild nausea are common effects or side effects of iron, not proof that the anemia is correcting. Taking the medication with meals describes administration, not response. The key is evidence of improved function.

3574. A client with sepsis is receiving IV fluids and broad-spectrum antibiotics. Which trend most strongly suggests that treatment is effective?

- A. Urine output falls from 40 mL/hr to 15 mL/hr
- B. Lactate level decreases on repeat testing
- C. Heart rate increases from 102/min to 124/min
- D. The client develops cool, mottled extremities

Correct Answer: B

A decreasing lactate level suggests improved tissue perfusion and is a strong indicator that sepsis treatment is working. Falling urine output, rising heart rate, and cool mottled extremities suggest worsening shock or poor perfusion. The most meaningful positive trend here is the reduction in lactate.

3575. A client with syndrome of inappropriate antidiuretic hormone secretion (SIADH) is on fluid restriction. Which finding best indicates the treatment is effective?

- A. Serum sodium rises toward the expected range
- B. Urine specific gravity becomes more concentrated
- C. The client reports a dry mouth
- D. Daily fluid intake matches the restriction amount

Correct Answer: A

The goal of fluid restriction in SIADH is to improve dilutional hyponatremia, so a rising serum sodium is the best evidence of effectiveness. More concentrated urine can occur in SIADH and does not best show correction. Dry mouth is a discomfort, not an effectiveness measure. Following the restriction is important, but the best indicator is the physiologic response.

3576. A client is receiving packed red blood cells for symptomatic anemia. Fifteen minutes after the transfusion begins, which finding most strongly indicates the nurse should stop the transfusion rather than continue monitoring for expected improvement?

- A. The client says fatigue has not improved yet
- B. The hemoglobin has not been rechecked yet
- C. The client develops chills, low back pain, and dyspnea
- D. The client asks how long the transfusion will take

Correct Answer: C

Chills, low back pain, and dyspnea shortly after a transfusion starts suggest an acute transfusion reaction and require stopping the transfusion immediately. Lack of early symptom improvement is expected because the transfusion has just begun. The hemoglobin would not be rechecked at 15 minutes to judge response. The reaction signs override routine monitoring.

3577. A client with peptic ulcer disease is started on therapy to reduce gastric acid and treat *Helicobacter pylori*. Which outcome best shows the plan of care is effective?

- A. The client reports less epigastric pain and no longer has black, tarry stools
- B. The client states spicy food still causes discomfort
- C. The client has mild constipation after starting therapy
- D. The client takes the medications exactly at mealtime each day

Correct Answer: A

Decreased epigastric pain and resolution of black, tarry stools suggest reduced ulcer activity and less gastrointestinal bleeding, indicating effective treatment. Persistent discomfort with spicy foods does not best show healing. Constipation is a side effect, not a therapeutic response. Correct medication timing does not by itself prove clinical improvement.

3578. A client with digoxin therapy for heart failure is being monitored for treatment response. Which finding best indicates a desired effect of the medication?

- A. Apical pulse is 54/min and irregular
- B. The client has nausea and sees yellow halos
- C. Crackles at the lung bases are decreased compared with yesterday
- D. Serum potassium is lower than the previous value

Correct Answer: C

Decreased crackles suggest improved cardiac output and less pulmonary congestion, which is a desired effect in heart failure treatment. A slow irregular pulse, nausea, and yellow halos suggest possible digoxin toxicity. A lower potassium level is concerning because it can increase toxicity risk, not indicate therapeutic success.

3579. A client with acute pulmonary edema receives IV morphine and furosemide. Which reassessment finding best indicates the treatment is working?

- A. Respiratory rate decreases from 34/min to 22/min and crackles are reduced
- B. Urine output is 20 mL over the next 2 hours
- C. Oxygen saturation drops from 94% to 89%

D. The client reports new ringing in the ears

Correct Answer: A

A decrease in tachypnea with improved breath sounds indicates reduced fluid overload and better gas exchange, which are desired responses to treatment for pulmonary edema. Low urine output suggests poor diuretic response, falling oxygen saturation suggests worsening status, and tinnitus may indicate an adverse effect rather than improvement.

3580. A client with hypokalemia is receiving IV potassium chloride. Which finding most clearly shows the therapy has been effective?

- A. Serum potassium rises from 2.9 to 3.8 mEq/L
- B. Blood glucose decreases from 180 to 140 mg/dL
- C. Urine specific gravity increases to 1.030
- D. Hemoglobin rises from 10 g/dL to 12 g/dL

Correct Answer: A

Potassium replacement is effective when the serum potassium returns toward the expected range, such as from 2.9 to 3.8 mEq/L. The other values do not measure response to potassium therapy. This item tests matching the treatment with the most direct outcome indicator.

3581. A client with dehydration from vomiting receives 1 liter of isotonic IV fluid over 2 hours. Which finding best indicates the fluid replacement has had the desired effect?

- A. Blood pressure changes from 84/52 mm Hg to 104/66 mm Hg
- B. The client continues to have dry mucous membranes and poor skin turgor
- C. Urine output remains 15 mL/hr
- D. Heart rate increases from 96/min to 118/min

Correct Answer: A

An improved blood pressure after fluid replacement suggests better circulating volume and response to treatment. Persistent signs of dehydration, low urine output, and rising heart rate point to ongoing hypovolemia rather than improvement.

3582. A client with hypothyroidism has been taking levothyroxine for several weeks. Which finding best indicates the medication is producing the desired effect?

- A. The client reports improved energy and less constipation
- B. The client reports new palpitations and insomnia
- C. The client has increasing facial puffiness
- D. The client states weight gain has accelerated

Correct Answer: A

Improved energy and reduced constipation are expected signs that thyroid hormone replacement is helping relieve hypothyroid symptoms. Palpitations and insomnia may suggest overtreatment, while worsening edema and weight gain suggest inadequate response.

3583. A client with postoperative pain rates the pain as 8 out of 10 and receives an IV opioid. Which reassessment finding 30 minutes later best demonstrates an effective response to treatment while supporting respiratory safety?

- A. Pain is 3 out of 10, respiratory rate is 14/min, and the client can deep-breathe
- B. Pain is 0 out of 10, respiratory rate is 8/min, and the client is difficult to arouse
- C. Pain is 7 out of 10, respiratory rate is 18/min, and the client remains restless
- D. Pain is 5 out of 10, blood pressure is unchanged, and the incision is dry

Correct Answer: A

The best response is meaningful pain relief with maintained respiratory status and improved function, such as being able to deep-breathe. Choice B shows pain relief but also likely opioid oversedation. Choice C shows inadequate pain control, and choice D shows only partial relief without the strongest evidence of effective treatment.

3584. A client with atrial fibrillation is started on diltiazem to control ventricular rate. Which finding best indicates the medication is effective?

- A. Apical pulse decreases from 136/min to 88/min
- B. Urine output increases from 25 mL/hr to 60 mL/hr
- C. Temperature decreases from 38.3°C to 37.2°C
- D. Potassium increases from 3.4 to 4.1 mEq/L

Correct Answer: A

For rate control in atrial fibrillation, the desired effect is a lower ventricular response, shown here by a drop in apical pulse from 136/min to 88/min. The other findings may matter clinically in other situations, but they do not directly show that diltiazem is controlling heart rate.

3585. A client with a bowel obstruction has a nasogastric tube connected to low intermittent suction. Which finding best indicates the decompression is effective?

- A. Abdominal distention decreases and nausea subsides
- B. The client reports increased cramping and has a rigid abdomen
- C. Bowel sounds are absent and pain becomes constant
- D. Nasogastric output stops abruptly while distention worsens

Correct Answer: A

Effective gastric decompression should reduce pressure in the GI tract, leading to less distention and less nausea. Increasing rigidity, worsening pain, or abrupt loss of drainage with more distention suggests ongoing obstruction or tube malfunction, not successful treatment.

3586. A client with hyperglycemia is treated with IV regular insulin in the emergency setting. Which trend most strongly indicates the treatment is effective?

- A. Blood glucose decreases from 420 mg/dL to 260 mg/dL over several hours
- B. White blood cell count decreases from 14,000/mm³ to 12,500/mm³
- C. Hemoglobin increases from 11 g/dL to 12 g/dL
- D. Platelet count decreases from 250,000/mm³ to 220,000/mm³

Correct Answer: A

The best evidence that insulin is working is a downward trend in blood glucose. The other laboratory changes do not directly evaluate the effectiveness of insulin therapy for hyperglycemia.

3587. A client with active upper gastrointestinal bleeding is receiving prescribed therapy. Which finding most strongly suggests the client is responding to treatment?

- A.** Heart rate decreases, blood pressure stabilizes, and stools are no longer black and tarry
- B.** The client develops increasing abdominal rigidity and severe pain
- C.** Urine output falls to 20 mL/hr and skin becomes cool
- D.** The client has repeated episodes of bloody emesis

Correct Answer: A

Stabilizing vital signs with no further evidence of melena suggests bleeding is slowing or resolved and perfusion is improving. The other findings point to continued blood loss or deterioration.

3588. A client with symptomatic bradycardia receives atropine. Which assessment finding best indicates a therapeutic response?

- A.** Heart rate increases from 38/min to 68/min and dizziness decreases
- B.** Respiratory rate increases from 16/min to 24/min while heart rate remains 40/min
- C.** Blood pressure decreases from 108/64 mm Hg to 84/48 mm Hg
- D.** The client reports dry mouth but still feels faint

Correct Answer: A

Atropine is used to improve symptomatic bradycardia, so a higher heart rate with reduced dizziness shows the desired effect. Dry mouth can occur as a side effect, but if symptoms persist, the treatment has not clearly worked. A falling blood pressure or unchanged bradycardia would not indicate success.

3589. A client with increased intracranial pressure receives mannitol. Which finding requires the nurse to question whether the treatment is effectively reducing the problem?

- A.** Urine output increases and level of consciousness improves slightly
- B.** Pupils become more unequal and the client is harder to arouse
- C.** Headache intensity decreases after treatment

D. Blood pressure trends closer to the client's baseline

Correct Answer: B

Worsening pupil changes and decreased arousability suggest increasing intracranial pressure and poor response to treatment. Increased urine output is an expected effect of mannitol, and improvement in neurologic symptoms or hemodynamics would support effectiveness instead.

3590. A client with myasthenia gravis receives pyridostigmine. Which finding best indicates the medication has reached its desired effect?

- A.** The client can chew and swallow lunch without fatigue
- B.** The client has increased salivation, abdominal cramping, and diarrhea
- C.** The client develops severe muscle weakness with shallow respirations
- D.** The client reports blurred vision and a pulse of 52/min

Correct Answer: A

Pyridostigmine is intended to improve neuromuscular transmission, so better muscle strength for tasks such as chewing and swallowing indicates effectiveness. Increased secretions, cramping, diarrhea, bradycardia, and worsening weakness suggest excessive cholinergic effects or poor control rather than a desired response.

3591. A client with Addison disease receives IV hydrocortisone and isotonic fluids for acute adrenal insufficiency. Which combination of findings best indicates the treatment is effective?

- A.** Blood pressure rises, weakness lessens, and serum sodium improves
- B.** Blood pressure falls, potassium rises, and the client becomes more confused
- C.** Temperature increases, abdominal pain worsens, and urine output drops
- D.** Respiratory rate slows, wheezing increases, and extremities become cool

Correct Answer: A

Acute adrenal insufficiency treatment should improve perfusion and correct hormone-related electrolyte problems. Rising blood pressure, less weakness, and improving sodium support a positive response. The other choices show progressive instability rather than effectiveness.

3592. A client with acute angle-closure glaucoma receives prescribed therapy to lower intraocular pressure. Which finding best indicates the treatment is working?

- A.** Eye pain decreases and the client reports less blurred vision
- B.** The client develops severe nausea and sees halos around lights
- C.** The affected eye becomes more firm and red
- D.** The pupil remains fixed and the headache intensifies

Correct Answer: A

Reduced eye pain and improved vision suggest that intraocular pressure is decreasing, which is the treatment goal. Nausea, halos, increased firmness and redness, or worsening headache indicate ongoing or worsening pressure, not successful therapy.

3593. A client with pancreatitis is receiving bowel rest, IV fluids, and analgesia. Which trend best indicates the plan of care is effective?

- A.** Abdominal pain decreases, nausea lessens, and serum amylase begins to trend down
- B.** The abdomen becomes more distended and urine output declines
- C.** The client develops a fever and increasing restlessness
- D.** Pain remains severe and bowel sounds become absent with guarding

Correct Answer: A

For pancreatitis, improvement is reflected by decreasing pain and nausea along with improving laboratory trends. Worsening distention, fever, declining urine output, persistent severe pain, or guarding would raise concern for poor response or complications.

3594. A client with bacterial pneumonia begins antibiotic therapy 24 hours ago. Which reassessment finding best indicates the treatment is effective?

- A.** The white blood cell count is slightly higher than on admission
- B.** Crackles are still present, but the temperature has decreased from 39.1°C (102.4°F) to 37.8°C (100.0°F)
- C.** The client reports fatigue and wants to sleep most of the day
- D.** A productive cough continues with thick sputum

Correct Answer: B

A decreasing fever is an early sign that antibiotic therapy is helping control infection. Crackles and cough may persist for a while even when treatment is working. Choice A suggests infection may not yet be improving. Choices C and D can occur with pneumonia but do not best show a therapeutic response.

3595. A client with heart failure is started on a fluid restriction and prescribed diuretic therapy. Which finding over the next 48 hours best indicates the plan is effective?

- A.** Weight decreases by 1.4 kg (3 lb) and ankle edema is reduced
- B.** Blood pressure increases from 118/72 mm Hg to 148/88 mm Hg
- C.** The client requests extra ice chips because of thirst
- D.** The client reports urinating frequently during the night

Correct Answer: A

A measurable weight loss with less peripheral edema is strong evidence that excess fluid volume is decreasing. Increased thirst or frequent urination may occur during therapy but do not by themselves confirm improved fluid status. A rising blood pressure does not show the treatment is working and may be concerning.

3596. A client with acute bronchospasm receives an inhaled short-acting bronchodilator. Which reassessment finding most clearly shows the medication has had the desired effect?

- A. Respiratory rate changes from 28/min to 26/min, and wheezing is unchanged
- B. Pulse increases from 88/min to 108/min
- C. The client states breathing feels easier, and expiratory wheezes are decreased
- D. The client develops a dry mouth after inhalation

Correct Answer: C

Improved ease of breathing with decreased wheezing indicates better airflow after bronchodilator therapy. Choice B reflects a common adverse effect rather than the desired response. Choice A shows little improvement. Choice D is a side effect, not evidence that bronchospasm has resolved.

3597. A client with iron-deficiency anemia has been taking oral ferrous sulfate for 1 month. Which finding best indicates the treatment is effective?

- A. The client reports improved energy during daily activities
- B. The stool is dark green to black
- C. The client has mild nausea after taking the medication
- D. The skin remains pale at the nail beds

Correct Answer: A

Improved activity tolerance and energy suggest better oxygen-carrying capacity and are good indicators that iron therapy is helping. Dark stools and mild nausea are common effects of oral iron but do not show that the anemia is improving. Persistent pallor suggests the problem may not yet be resolved.

3598. A client with preeclampsia is receiving magnesium sulfate. Which finding best indicates the therapy is achieving its intended effect?

- A. Urine output increases to 75 mL/hr
- B. Deep tendon reflexes are absent bilaterally
- C. The client remains free of seizure activity
- D. Blood pressure drops to 88/54 mm Hg

Correct Answer: C

Magnesium sulfate is given in preeclampsia primarily to prevent seizures, so absence of seizure activity best reflects the desired therapeutic effect. Absent reflexes suggest possible magnesium toxicity, not success. Increased urine output may be helpful to monitor but is not the main intended outcome. Hypotension is not the goal and may be concerning.

3599. A client with warfarin therapy for a history of venous thromboembolism has the dosage adjusted after a subtherapeutic coagulation study. Which follow-up result best indicates the adjustment has achieved the desired effect without suggesting excessive anticoagulation?

- A. INR increases from 1.3 to 2.4
- B. Platelet count decreases from 220,000/mm³ to 150,000/mm³
- C. Hemoglobin decreases from 13.2 g/dL to 10.1 g/dL
- D. The client reports brushing off minor gum bleeding as expected

Correct Answer: A

An INR rising from a subtherapeutic value to 2.4 indicates the anticoagulant effect is now in a typical therapeutic range for many venous thromboembolism indications, without clear evidence of over-anticoagulation. A falling hemoglobin or gum bleeding may suggest bleeding complications rather than an ideal response. Platelet count is not the main measure used to monitor warfarin effectiveness.

3600. A client with hypothyroidism started levothyroxine 6 weeks ago. Which follow-up finding best indicates the medication is having the desired effect?

- A. The client reports improved energy and decreased cold intolerance
- B. The client has lost 6 lb in 1 week
- C. The client reports occasional palpitations after taking the medication
- D. The client's blood pressure decreased from 138/86 mm Hg to 118/70 mm Hg

Correct Answer: A

Improved energy and less cold intolerance are classic signs that thyroid hormone replacement is improving hypothyroid symptoms. Rapid weight loss is not the best indicator and could suggest another problem. Palpitations may indicate overreplacement rather than the desired response. A lower blood pressure alone is not a reliable measure of effective thyroid treatment.

Physiological Adaptation

Acute conditions

3601. A client with a history of heart failure develops acute pulmonary edema. Which finding should the nurse expect?

- A. Crackles and pink frothy sputum
- B. Absent bowel sounds and rigid abdomen
- C. Warm dry skin and bradycardia
- D. Unilateral calf pain relieved by elevation

Correct Answer: A

Acute pulmonary edema commonly causes fluid in the alveoli, leading to crackles, severe dyspnea, and sometimes pink frothy sputum. The other options point to different problems: abdominal emergency, unrelated hemodynamic findings, or possible deep vein thrombosis rather than pulmonary edema itself.

3602. A client with a severe allergic reaction is wheezing, has facial swelling, and is anxious. Which prescription should the nurse anticipate giving first?

- A. Diphenhydramine by mouth
- B. Epinephrine IM
- C. Acetaminophen PO
- D. Furosemide IV

Correct Answer: B

IM epinephrine is the first-line medication for anaphylaxis because it rapidly reduces airway swelling and bronchoconstriction. Oral diphenhydramine acts too slowly for a severe reaction. Acetaminophen is unrelated, and furosemide does not treat anaphylaxis.

3603. A client reports chest pain rated 8/10. Which assessment finding is most concerning for an acute myocardial infarction?

- A. Pain improves when leaning forward
- B. Pain occurs only when swallowing
- C. Pain radiates to the jaw with diaphoresis

D. Pain is reproducible with palpation of the chest wall

Correct Answer: C

Chest pain radiating to the jaw with diaphoresis is a classic concerning pattern for acute myocardial infarction. Pain reproducible with palpation suggests musculoskeletal causes, pain only with swallowing suggests esophageal involvement, and pain relieved by leaning forward is more consistent with pericardial irritation.

3604. A client with pancreatitis suddenly becomes confused and restless. The nurse checks the blood pressure and finds it is 84/50 mm Hg. Which additional finding best supports a concern for hypovolemic shock?

- A. Bounding peripheral pulses
- B. Urine output of 15 mL in the last hour
- C. Warm flushed skin
- D. Heart rate of 58/min

Correct Answer: B

Decreased urine output is a key sign of poor organ perfusion and supports hypovolemic shock. Shock more commonly causes tachycardia and weak pulses, not bradycardia or bounding pulses. Warm flushed skin is more consistent with distributive states early on, not classic hypovolemia.

3605. A client with upper gastrointestinal bleeding has bright red emesis and feels dizzy. Which action should the nurse take first?

- A. Insert an indwelling urinary catheter
- B. Position the client flat without a pillow
- C. Assess vital signs and maintain a patent airway
- D. Give the client water to rinse the mouth

Correct Answer: C

With active hematemesis, the immediate priorities are airway, breathing, and circulation. Assessing vital signs and maintaining a patent airway address the risk of aspiration and hemodynamic instability. A urinary catheter may be needed later for output monitoring, but it is not first. Giving water increases aspiration risk, and lying flat may worsen aspiration risk.

3606. A client with diabetes becomes shaky, diaphoretic, and confused. The client is awake and able to swallow. Which intervention is most appropriate first?

- A. Give 15 g of a rapid-acting carbohydrate
- B. Administer long-acting insulin
- C. Encourage the client to sleep
- D. Restrict oral intake until the provider arrives

Correct Answer: A

These findings suggest hypoglycemia. If the client is awake and can swallow safely, the first action is to give a rapid-acting carbohydrate. Long-acting insulin would worsen the problem. Delaying treatment risks seizure or loss of consciousness.

3607. A client with a head injury begins to vomit and becomes increasingly difficult to arouse. Which action is the nurse's priority?

- A. Reduce environmental stimulation and recheck in 30 minutes
- B. Place the client in high-Fowler position and offer fluids
- C. Notify the provider immediately and prepare for rapid intervention
- D. Administer a PRN sedative for agitation

Correct Answer: C

Vomiting and decreasing level of consciousness after head injury may indicate rising intracranial pressure, which requires immediate escalation. Waiting, giving fluids, or sedating the client could delay recognition and worsen neurologic assessment or safety.

3608. A client receiving a blood transfusion reports chills, low back pain, and shortness of breath 10 minutes after the infusion begins. What should the nurse do first?

- A. Slow the transfusion rate and reassess in 15 minutes
- B. Stop the transfusion immediately
- C. Administer the prescribed PRN antipyretic
- D. Document the findings after the transfusion is complete

Correct Answer: B

These findings suggest an acute transfusion reaction. The first action is to stop the transfusion immediately to prevent further exposure. Slowing the infusion is unsafe. Antipyretics may be given later if prescribed, but not before stopping the blood product.

3609. A client with suspected bacterial meningitis arrives on the unit. Which nursing action has the highest priority?

- A. Begin seizure precautions and start droplet precautions
- B. Encourage the client to ambulate to prevent deconditioning
- C. Place the client in a brightly lit room for observation
- D. Delay assessment until family members finish visiting

Correct Answer: A

Meningitis can cause seizures and may require droplet precautions to reduce transmission risk, so these actions are high priority. Bright light may worsen discomfort from photophobia. Delaying assessment or promoting ambulation does not address the immediate safety concerns.

3610. A postoperative client suddenly reports shortness of breath and sharp chest pain. The nurse notes tachycardia and oxygen saturation of 86% despite oxygen by nasal cannula. Which complication is most important for the nurse to suspect?

- A. Pulmonary embolism
- B. Acute cystitis
- C. Paralytic ileus
- D. Incisional infection

Correct Answer: A

Sudden dyspnea, pleuritic chest pain, tachycardia, and hypoxemia in a postoperative client are highly concerning for pulmonary embolism. The other options do not explain the abrupt cardiopulmonary compromise.

3611. A client with severe dehydration from vomiting is receiving IV fluids. Which finding indicates the nurse should question whether the client is improving and instead suspect worsening shock?

- A. Blood pressure rises from 88/54 to 96/60 mm Hg
- B. Urine output increases from 10 mL/hr to 35 mL/hr
- C. Skin remains cool and mottled with increasing confusion
- D. Heart rate decreases from 126/min to 108/min

Correct Answer: C

Cool mottled skin and increasing confusion are signs of worsening perfusion and ongoing shock, even if some values are being monitored. The other findings suggest at least partial improvement in circulatory status. This question tests looking beyond a single number and recognizing persistent tissue hypoperfusion.

3612. A client with chronic obstructive pulmonary disease develops sudden restlessness, severe dyspnea, and use of accessory muscles. Which arterial blood gas result would be most concerning for impending respiratory failure?

- A. pH 7.40, PaCO₂ 40 mm Hg, PaO₂ 92 mm Hg
- B. pH 7.32, PaCO₂ 55 mm Hg, PaO₂ 58 mm Hg
- C. pH 7.48, PaCO₂ 34 mm Hg, PaO₂ 88 mm Hg
- D. pH 7.44, PaCO₂ 38 mm Hg, PaO₂ 96 mm Hg

Correct Answer: B

A pH of 7.32 with elevated PaCO₂ and low PaO₂ indicates respiratory acidosis with hypoxemia, a dangerous pattern in a client showing severe distress. The other results are normal or show far less immediate compromise. The trap is choosing a value based only on one abnormal number instead of interpreting the full ABG pattern.

3613. A client with sepsis has a temperature of 39.4°C (102.9°F), heart rate 132/min, respirations 28/min, and blood pressure 78/42 mm Hg. The client is confused and has delayed capillary refill. Which nursing action is the priority?

- A. Obtain a full health history from the family

- B. Apply warm blankets and dim the room lights
- C. Initiate rapid supportive measures and notify the provider or rapid response team
- D. Reassure the client that confusion is expected with fever

Correct Answer: C

This client shows signs of septic shock, including hypotension, tachycardia, altered mental status, and poor perfusion. Immediate escalation and rapid supportive treatment are the priority. Comfort measures and history can follow after stabilization. Reassurance alone dangerously delays care.

3614. A client 24 hours after abdominal surgery suddenly becomes very restless and reports tingling around the mouth. The nurse notes rapid deep respirations at 34/min. Which acid-base imbalance should the nurse suspect first?

- A. Respiratory acidosis
- B. Respiratory alkalosis
- C. Metabolic acidosis
- D. Metabolic alkalosis

Correct Answer: B

Respiratory alkalosis is most likely because rapid deep breathing can cause excessive loss of carbon dioxide, leading to alkalosis. Tingling around the mouth and restlessness are common signs of hyperventilation. Respiratory acidosis would be associated with hypoventilation and CO₂ retention, while metabolic problems are not the best first match for this sudden breathing pattern and symptom cluster.

3615. A client with a bowel obstruction has a nasogastric tube to low suction. The client suddenly develops abdominal rigidity, severe pain, a temperature of 38.7°C (101.7°F), and a heart rate of 124/min. Which complication should the nurse suspect?

- A. Peritonitis
- B. Constipation
- C. Improving gastric decompression
- D. Mild fluid volume excess

Correct Answer: A

Peritonitis is the best answer because abdominal rigidity, severe pain, fever, and tachycardia suggest an acute inflammatory or infectious abdominal emergency, often from perforation or leakage into the peritoneal cavity. Constipation would not explain this sudden severe systemic change. Improving decompression would reduce discomfort rather than cause guarding and fever. Fluid volume excess does not fit the rigid abdomen and acute abdominal pain.

3616. A client 12 hours after a thyroidectomy becomes restless and reports numbness and tingling in the fingers. Which finding would most strongly support the nurse's suspicion of acute hypocalcemia?

- A. Positive Trousseau sign when the blood pressure cuff is inflated
- B. Bradycardia with bounding peripheral pulses
- C. Warm, flushed skin with headache
- D. Absent bowel sounds in all quadrants

Correct Answer: A

A positive Trousseau sign is a classic sign of neuromuscular irritability from hypocalcemia, which can occur after thyroid surgery if the parathyroid glands are affected. The tingling and restlessness fit this problem. Bradycardia with bounding pulses, flushed skin, and absent bowel sounds do not specifically point to acute hypocalcemia.

3617. A client with liver cirrhosis becomes increasingly confused over several hours and is unable to state the current location. Which additional assessment finding is most consistent with hepatic encephalopathy?

- A. Kussmaul respirations
- B. Asterixis when the arms are extended
- C. Bilateral calf pain with dorsiflexion
- D. Pink frothy sputum

Correct Answer: B

Asterixis, or flapping tremor, is a common finding in hepatic encephalopathy and supports the change in mental status. Kussmaul respirations suggest metabolic acidosis, calf pain is not a reliable indicator here, and pink frothy sputum is more associated with pulmonary edema.

3618. A client with a history of peptic ulcer disease suddenly reports severe abdominal pain. The abdomen is board-like on palpation, and the client guards with movement. Which complication should the nurse suspect first?

- A. Acute appendicitis
- B. Perforation with peritonitis
- C. Mechanical ileus
- D. Acute cholecystitis

Correct Answer: B

A sudden onset of severe pain with a rigid, board-like abdomen strongly suggests perforation and resulting peritonitis, a surgical emergency. Mechanical ileus may cause distention and decreased bowel sounds, but not classic board-like rigidity. Appendicitis and cholecystitis have different typical patterns of pain and exam findings.

3619. A client suddenly develops inspiratory stridor 30 minutes after extubation. Which action should the nurse take first?

- A. Encourage oral fluids
- B. Place the client flat in bed
- C. Call for immediate assistance and prepare for airway support
- D. Ask the client to cough forcefully every 15 minutes

Correct Answer: C

Stridor after extubation suggests upper-airway obstruction and can quickly become life-threatening. The priority is immediate assistance and preparation for airway intervention. Fluids, lying flat, or repeated coughing do not address the urgent airway risk and may worsen the situation.

3620. A client with acute kidney injury has a serum potassium level of 6.6 mEq/L. Which electrocardiogram finding should the nurse expect first as potassium rises?

- A. Peaked T waves
- B. ST-segment elevation
- C. Sawtooth atrial activity
- D. Shortened PR interval

Correct Answer: A

Peaked T waves are an early and important ECG change seen with hyperkalemia. As potassium rises further, conduction can worsen with PR prolongation and QRS widening. ST elevation and sawtooth atrial activity are associated with other cardiac problems, not the typical early pattern of hyperkalemia.

3621. A client with sickle cell disease reports severe left upper quadrant pain after several days of viral symptoms. The nurse notes pallor, tachycardia, and a rapidly enlarging spleen. Which acute complication is most likely?

- A. Aplastic crisis
- B. Splenic sequestration crisis
- C. Acute chest syndrome
- D. Stroke

Correct Answer: B

Splenic sequestration crisis causes blood to pool in the spleen, leading to sudden splenic enlargement, pallor, tachycardia, and signs of hypovolemia or anemia. Aplastic crisis causes severe anemia but not the characteristic rapidly enlarging spleen. Acute chest syndrome and stroke have different primary findings.

3622. A client with Addison disease has had vomiting and diarrhea for 2 days and now appears weak and dizzy. Blood pressure is 82/48 mm Hg, heart rate is 118/min, and the client is confused. Which prescription should the nurse anticipate as the priority treatment?

- A. Restrict oral sodium intake
- B. Administer IV hydrocortisone and isotonic fluids
- C. Give a loop diuretic
- D. Start potassium supplementation

Correct Answer: B

These findings suggest an Addisonian crisis with hypotension, dehydration, and altered mental status. The priority treatment is corticosteroid replacement with IV hydrocortisone plus isotonic fluids to support circulation. Sodium restriction, diuretics, and potassium supplementation would worsen the condition rather than treat it.

3623. A client admitted with acute pancreatitis suddenly becomes restless and short of breath. Oxygen saturation falls from 96% to 88% on room air, and fine crackles are heard bilaterally. Which complication should the nurse suspect?

- A. Acute respiratory distress syndrome
- B. Dumping syndrome
- C. Diabetic ketoacidosis
- D. Pericarditis

Correct Answer: A

Pancreatitis can trigger systemic inflammation leading to acute respiratory distress syndrome (ARDS). Worsening oxygenation, dyspnea, and bilateral crackles support this complication. Dumping syndrome is related to gastric surgery, diabetic ketoacidosis has a different presentation, and pericarditis does not best fit these respiratory findings.

3624. A client with syndrome of inappropriate antidiuretic hormone secretion becomes increasingly lethargic. Which laboratory result would best explain the change in neurologic status?

- A. Serum sodium 118 mEq/L
- B. Hemoglobin 15 g/dL
- C. Platelet count 240,000/mm³
- D. Fasting glucose 102 mg/dL

Correct Answer: A

SIADH causes water retention and dilutional hyponatremia. A serum sodium of 118 mEq/L is severely low and can cause lethargy, confusion, and seizures. The other laboratory values are not likely to explain an acute neurologic decline in this situation.

3625. A client receiving total parenteral nutrition through a central venous catheter suddenly develops fever, hypotension, and confusion. The insertion site has no redness or drainage. Which action is the nurse's priority?

- A. Slow the TPN rate and reassess in 30 minutes
- B. Place the client in Trendelenburg position
- C. Stop the infusion and notify the provider of possible catheter-related sepsis
- D. Remove the dressing and leave the site open to air

Correct Answer: C

Fever, hypotension, and confusion in a client with a central line suggest possible catheter-related bloodstream infection progressing to sepsis. The priority is to stop the infusion and urgently notify the provider. Waiting, positioning changes, or exposing the site do not address the likely systemic infection. A normal-looking insertion site does not rule out catheter-related sepsis.

3626. A client with diabetic ketoacidosis is receiving IV regular insulin. Four hours later, the blood glucose has decreased from 520 mg/dL to 210 mg/dL. The client now reports muscle weakness, and the cardiac monitor shows new U waves. Which complication should the nurse suspect?

- A. Hypernatremia
- B. Hypokalemia
- C. Hypercalcemia
- D. Hypermagnesemia

Correct Answer: B

During treatment of diabetic ketoacidosis, insulin drives potassium back into cells, which can produce hypokalemia even as the glucose improves. Muscle weakness and U waves strongly support low potassium. This is a high-risk treatment complication that requires prompt recognition.

3627. A client 1 day after a femur fracture becomes suddenly confused and restless. The nurse notes a temperature of 38.3°C (100.9°F), heart rate of 118/min, respiratory rate of 30/min, and petechiae on the upper chest. Which complication is the nurse's priority concern?

- A. Fat embolism syndrome
- B. Neurogenic shock
- C. Compartment syndrome
- D. Tetanus

Correct Answer: A

The combination of long-bone fracture, sudden respiratory and neurologic changes, and petechiae is classic for fat embolism syndrome. Neurogenic shock usually includes hypotension with bradycardia, compartment syndrome is localized to the affected limb, and tetanus does not fit this acute post-fracture pattern.

Chronic conditions

3628. A client with chronic heart failure reports gaining 3 lb in 2 days and says, "My shoes suddenly feel tight." Which action should the practical nurse take first?

- A. Encourage the client to drink more water to improve kidney perfusion
- B. Assess for worsening fluid retention, including edema and breath sounds
- C. Tell the client that mild weight changes are expected with heart failure
- D. Have the client skip the next dose of prescribed diuretic

Correct Answer: B

A rapid weight gain in a client with chronic heart failure suggests fluid retention and possible worsening status. The nurse should first assess for associated findings such as edema, crackles, and increased dyspnea. Choice A may worsen overload. Choice C minimizes a potentially significant change. Choice D is unsafe because the nurse should not independently alter prescribed medication dosing.

3629. A client with chronic obstructive pulmonary disease (COPD) is receiving oxygen at 2 L/min by nasal cannula. Which finding requires immediate follow-up?

- A. The client uses tripod positioning while breathing
- B. The client has a productive cough in the morning
- C. The client becomes increasingly drowsy and difficult to arouse
- D. The client says pursed-lip breathing helps during activity

Correct Answer: C

Increasing drowsiness and difficulty arousing may indicate worsening hypoventilation and carbon dioxide retention, which requires immediate follow-up. Tripod positioning, morning cough, and use of pursed-lip breathing are common findings or coping strategies in chronic COPD and are not the most urgent.

3630. A client with type 2 diabetes and peripheral neuropathy says, "My feet don't hurt anymore, so I think they are doing better." Which response by the practical nurse is best?

- A.** Lack of pain means the circulation has improved
- B.** You should soak your feet every night to keep the skin soft
- C.** Neuropathy can reduce feeling, so you need to inspect your feet every day
- D.** You only need to check your feet if you notice swelling

Correct Answer: C

Peripheral neuropathy decreases protective sensation, so the client may not feel injury even when a problem is present. Daily foot inspection is an important chronic-care teaching point. Choice A is incorrect because absence of pain does not mean healing. Choice B can increase risk of skin injury or burns. Choice D is too limited and delays detection of ulcers or trauma.

3631. A client with chronic kidney disease has an arteriovenous fistula in the left forearm for hemodialysis. Which action is appropriate for the practical nurse to include in care?

- A.** Obtain the blood pressure in the left arm to compare circulation
- B.** Apply a warm compress over the fistula if the client reports tenderness
- C.** Assess the fistula for a bruit and thrill
- D.** Place a constrictive dressing over the site between dialysis sessions

Correct Answer: C

A functioning arteriovenous fistula should be assessed for a bruit and thrill. These indicate patency. Blood pressure measurements and constriction should be avoided in the fistula arm. Tenderness should be evaluated, not treated casually with a warm compress over the access without further assessment.

3632. A client with cirrhosis has chronic ascites and increasing abdominal distention. Which finding is most important for the practical nurse to report?

- A.** The client requests smaller meals
- B.** The client has shortness of breath when lying flat
- C.** The client says food tastes bland
- D.** The client prefers to rest after bathing

Correct Answer: B

Ascites can impair diaphragmatic movement and breathing. Shortness of breath when lying flat may indicate worsening respiratory compromise and should be reported promptly. The other findings may occur with chronic illness but are less urgent than impaired ventilation.

3633. The practical nurse is caring for a client with chronic stable angina. Which client statement shows correct understanding of sublingual nitroglycerin use for chest pain?

- A.** I should swallow the tablet with water as soon as the pain starts
- B.** I should take the tablet only after the chest pain has lasted 30 minutes
- C.** I should place the tablet under my tongue and sit down before taking it
- D.** I should take the tablet every day at bedtime whether I have pain or not

Correct Answer: C

Sublingual nitroglycerin is placed under the tongue, and the client should sit down because it can cause dizziness and hypotension. Swallowing it reduces the intended sublingual effect. Waiting 30 minutes is unsafe. Taking it routinely at bedtime for acute chest pain management is incorrect.

3634. A client with chronic peripheral arterial disease reports leg pain while walking that is relieved by rest. Which term best describes this symptom?

- A. Orthopnea
- B. Intermittent claudication
- C. Pitting edema
- D. Dependent rubor

Correct Answer: B

Intermittent claudication is ischemic muscle pain brought on by activity and relieved by rest, a classic finding in peripheral arterial disease. Orthopnea refers to shortness of breath when lying flat. Pitting edema is fluid accumulation. Dependent rubor is a color change of the extremity, not the pain pattern described.

3635. A client with a chronic stage 3 pressure injury has a serum albumin level below the expected range. Which nursing consideration is most important when planning care?

- A. Healing may be delayed because poor nutritional status impairs tissue repair
- B. Albumin level does not affect wound healing if the dressing is changed daily
- C. The wound should be left open to air to compensate for low albumin
- D. A low albumin level indicates the client should receive less protein

Correct Answer: A

Poor nutritional status, including low albumin, is associated with impaired tissue repair and delayed wound healing. Dressing changes alone do not overcome this problem. Leaving the wound open to air is generally not appropriate for a stage 3 pressure injury. Protein restriction is not supported by the finding described.

3636. A client with chronic hypertension tells the practical nurse, "I feel fine, so I stopped taking my pills." Which response is best?

- A. That is acceptable as long as you limit salt intake
- B. High blood pressure often has no symptoms, so medication may still be needed even when you feel well
- C. You can restart the medication only when a headache returns
- D. Stopping suddenly is the best way to see whether the medicine still works

Correct Answer: B

Hypertension is often asymptomatic, so clients may feel well despite ongoing elevated blood pressure. The best response addresses the misconception directly and supports adherence. The other options reinforce unsafe self-management and symptom-based treatment of a chronic condition that may not cause warning symptoms.

3637. A client with chronic venous insufficiency has lower-leg edema and a shallow irregular ulcer near the medial ankle. Which intervention should the practical nurse expect to help this condition?

- A. Keeping the legs dependent most of the day
- B. Applying compression as prescribed
- C. Using a heating pad directly over the ulcer
- D. Massaging the calf firmly every shift

Correct Answer: B

Compression, when prescribed and appropriate, helps reduce venous pooling and edema in chronic venous insufficiency. Keeping the legs dependent worsens swelling. Direct heat can injure tissue. Firm calf massage is not appropriate and may be harmful.

3638. A client with long-standing type 1 diabetes is confused, diaphoretic, and trembling. The finger-stick glucose reading is 52 mg/dL. Which action should the practical nurse take first if the client is awake and able to swallow?

- A. Give 15 g of a fast-acting carbohydrate
- B. Administer the scheduled long-acting insulin
- C. Offer a high-protein meal only
- D. Restrict oral intake until the provider is notified

Correct Answer: A

The client has symptomatic hypoglycemia. If awake and able to swallow, the first action is to give a fast-acting carbohydrate. Long-acting insulin would worsen the problem. Protein alone acts too slowly as the immediate treatment. Delaying treatment to restrict intake is unsafe.

3639. A client with chronic heart failure has new bilateral crackles, jugular venous distention, and 2+ pitting edema. Which provider prescription should the practical nurse question first?

- A. Daily weight every morning
- B. Limit sodium intake
- C. Increase oral fluids to 3,000 mL/day
- D. Administer prescribed loop diuretic

Correct Answer: C

The assessment findings indicate fluid volume excess. A prescription to increase oral fluids to 3,000 mL/day should be questioned because it may worsen overload. Daily weights, sodium restriction, and prescribed diuretics are commonly appropriate parts of management in this situation.

3640. A client with chronic kidney disease has not yet started dialysis. Which assessment finding is most concerning and should be reported promptly?

- A. Reports pruritus and dry skin
- B. Crackles at the lung bases and increasing shortness of breath
- C. States that food has a metallic taste
- D. Has mild ankle edema at the end of the day

Correct Answer: B

Crackles and increasing shortness of breath suggest fluid overload with pulmonary involvement, which can become life-threatening and requires prompt reporting. Pruritus, metallic taste, and mild ankle edema may occur in chronic kidney disease, but they are not as immediately dangerous as respiratory compromise.

3641. The practical nurse is reinforcing teaching for a client with chronic obstructive pulmonary disease. Which meal selection best supports the client's breathing and energy needs?

- A. A large fried breakfast with bacon, pancakes, and whole milk
- B. A full meal served immediately after the client walks in the hall
- C. A small, frequent, nutrient-dense meal that is easy to chew

D. A tray placed flat on the overbed table while the client remains supine

Correct Answer: C

Clients with COPD often tolerate small, frequent, nutrient-dense meals better because large meals can increase work of breathing and fatigue. Serving a meal right after exertion, using a large heavy meal, or keeping the client flat can make breathing more difficult. The main trap is choosing a high-calorie meal without considering the respiratory effort required to eat it.

3642. A client with chronic liver disease becomes increasingly confused and has asterixis. Which observation best supports the nurse's suspicion that the condition is worsening?

- A. The client asks for an extra blanket
- B. The client has a pulse of 58/min while sleeping
- C. The client is disoriented to time and has slurred speech
- D. The client reports chronic right shoulder discomfort

Correct Answer: C

Confusion, disorientation, slurred speech, and asterixis suggest worsening hepatic encephalopathy in a client with chronic liver disease. Requesting a blanket and shoulder discomfort are not the key indicators here, and a sleeping pulse of 58/min may be normal for some clients. The critical issue is progressive neurologic change.

3643. A client with chronic heart failure is prescribed furosemide 40 mg by mouth each morning. Which statement by the client indicates the teaching was effective?

- A. I will take the medication at bedtime so it works while I sleep.
- B. I will call the nurse if I notice sudden weight gain in a day or two.
- C. I should stop taking it on days when my ankles are not swollen.
- D. I can skip weighing myself as long as I do not feel short of breath.

Correct Answer: B

Daily weights help detect fluid retention early in chronic heart failure, so reporting sudden weight gain is an important part of self-management. Taking furosemide at bedtime may disrupt sleep with nocturia, and the medication should not be stopped or self-adjusted based only on swelling or symptoms.

3644. The practical nurse is caring for a client with chronic obstructive pulmonary disease (COPD). Which observation is most consistent with a chronic compensatory adaptation rather than an acute emergency?

- A. Barrel-shaped chest
- B. Sudden tracheal deviation
- C. Absent breath sounds on one side
- D. New severe restlessness with cyanosis

Correct Answer: A

A barrel-shaped chest is a common chronic physical change seen with long-standing COPD due to air trapping and hyperinflation. The other findings suggest acute deterioration, such as possible pneumothorax or severe hypoxia, and require urgent follow-up.

3645. A client with long-term type 2 diabetes asks how to help prevent worsening kidney damage. Which instruction should the practical nurse reinforce?

- A. Drink large amounts of sports drinks each day.
- B. Keep blood glucose and blood pressure controlled.
- C. Avoid all physical activity to protect the kidneys.
- D. Increase salt intake when meals taste bland.

Correct Answer: B

Good control of blood glucose and blood pressure helps slow progression of diabetic kidney damage. Sports drinks may add sugar and sodium, inactivity is not protective, and increased salt intake can worsen hypertension and fluid retention.

3646. A client with rheumatoid arthritis reports morning joint stiffness every day. Which action by the practical nurse is most appropriate to suggest for the start of the day?

- A. Plan demanding activities before getting out of bed.
- B. Apply heat and allow extra time for gentle movement.
- C. Keep the joints completely immobile until afternoon.
- D. Use ice first thing every morning for all stiffness.

Correct Answer: B

Heat and gentle range-of-motion activity can help reduce morning stiffness in chronic rheumatoid arthritis. Complete immobility can worsen stiffness, and ice is more often used for acute inflammation rather than routine morning stiffness relief.

3647. A client with chronic kidney disease who is not receiving dialysis has laboratory results available. Which result should the practical nurse report to the supervising nurse first?

- A. Potassium 6.1 mEq/L
- B. Hemoglobin 10.2 g/dL
- C. Calcium 8.4 mg/dL
- D. Phosphorus 4.9 mg/dL

Correct Answer: A

A potassium level of 6.1 mEq/L is critical because hyperkalemia can cause life-threatening cardiac dysrhythmias. The other values may be abnormal in chronic kidney disease, but they are not as immediately dangerous as markedly elevated potassium.

3648. A client with chronic liver disease is prescribed lactulose. Which outcome best shows the medication is having the intended effect?

- A. The client has less confusion and improved mental status.
- B. The client has darker urine and reduced appetite.
- C. The client reports complete relief of abdominal distention.
- D. The client's skin color returns to normal within hours.

Correct Answer: A

Lactulose is commonly used to help reduce ammonia-related encephalopathy, so improved mental status and less confusion suggest benefit. It is not expected to quickly reverse jaundice or fully relieve ascites, and darker urine with poor appetite does not indicate therapeutic success.

3649. The practical nurse is reinforcing home care for a client with chronic peripheral neuropathy caused by diabetes. Which client action requires correction?

- A.** Checking the inside of shoes before putting them on
- B.** Washing feet daily and drying carefully between the toes
- C.** Using a heating pad on the feet to warm them at night
- D.** Wearing well-fitting shoes instead of walking barefoot

Correct Answer: C

Clients with peripheral neuropathy may not feel excessive heat and are at risk for burns, so heating pads on the feet are unsafe. The other actions are appropriate foot-protection measures that help reduce injury and ulcer risk.

3650. A client with chronic stable angina tells the practical nurse, "I only get chest pressure when I shovel snow, and it goes away when I rest." How should the nurse interpret this report?

- A.** This pattern suggests predictable myocardial oxygen demand exceeding supply with exertion.
- B.** This pattern confirms that the chest pain is noncardiac because it stops with rest.
- C.** This pattern means a myocardial infarction is occurring right now.
- D.** This pattern shows the client no longer needs antianginal precautions.

Correct Answer: A

Stable angina is typically triggered by exertion or stress and relieved by rest or prescribed medication because oxygen demand temporarily exceeds coronary supply. Relief with rest does not make the pain noncardiac, and the pattern still requires ongoing management and precautions.

3651. The practical nurse is caring for four clients with chronic conditions. Which client should be seen first?

- A.** A client with multiple sclerosis who reports fatigue after physical therapy
- B.** A client with osteoarthritis who asks for a scheduled analgesic before ambulation
- C.** A client with chronic kidney disease who has peaked T waves on the cardiac monitor
- D.** A client with glaucoma who says prescribed eye drops sting for a few seconds

Correct Answer: C

Peaked T waves suggest hyperkalemia, which can rapidly progress to dangerous cardiac dysrhythmias and requires immediate attention. The other findings may need care, but they are not as potentially life-threatening as evidence of potassium-related cardiac effects.

3652. A client with chronic adrenal insufficiency has been vomiting all day and now reports extreme weakness and dizziness. Which additional finding would most strongly suggest progression to an adrenal crisis?

- A.** Blood pressure 82/48 mm Hg
- B.** Weight gain of 1 lb this week
- C.** Heart rate 58/min
- D.** Warm flushed skin after walking

Correct Answer: A

Adrenal crisis can cause severe hypotension and circulatory collapse, especially during stress or fluid loss such as vomiting. A blood pressure of 82/48 mm Hg is the most concerning finding. The other options do not point as strongly to this life-threatening deterioration.

3653. A client with chronic kidney disease and heart failure is prescribed a low-sodium diet. At lunch, which tray selection should the practical nurse identify as best aligned with that prescription?

- A.** Ham sandwich, canned soup, and potato chips
- B.** Grilled chicken, rice, and steamed green beans
- C.** Pepperoni pizza and a dill pickle
- D.** Instant noodles and processed cheese crackers

Correct Answer: B

Grilled chicken, rice, and steamed green beans is the lowest-sodium option among the choices and better supports chronic management of fluid-related conditions such as heart failure and kidney disease. Processed meats, canned soups, pizza, pickles, instant noodles, and crackers are typically high in sodium.

3654. A client with long-standing chronic obstructive pulmonary disease becomes increasingly sleepy. Arterial blood gas results are: pH 7.30, PaCO₂ 68 mm Hg, HCO₃⁻ 32 mEq/L, PaO₂ 58 mm Hg. Which interpretation is most accurate?

- A. Uncompensated metabolic acidosis
- B. Respiratory acidosis with metabolic compensation
- C. Metabolic alkalosis with respiratory compensation
- D. Fully normal acid-base status for a client with COPD

Correct Answer: B

The low pH shows acidosis, and the elevated PaCO₂ identifies a respiratory cause. The elevated HCO₃⁻ indicates the body has been compensating over time, which fits chronic COPD; however, the pH remains low, so the client is not fully corrected and is clinically worsening. This is not a normal acid-base status.

Life-threatening conditions

3655. A client with chronic obstructive pulmonary disease suddenly becomes anxious and increasingly short of breath. Assessment shows absent breath sounds on the right side, tracheal deviation to the left, and distended neck veins. Which condition should the nurse suspect?

- A. Pulmonary embolism
- B. Tension pneumothorax
- C. Left-sided heart failure
- D. Pleural effusion

Correct Answer: B

Unilateral absent breath sounds, tracheal shift away from the affected side, neck vein distention, and acute respiratory distress are classic findings of tension pneumothorax, a life-threatening emergency. Pulmonary embolism causes sudden dyspnea but does not typically cause unilateral absent breath sounds and tracheal deviation.

3656. A client with a history of severe allergy to bee stings develops wheezing, facial swelling, and hypotension after being stung while gardening. Which prescription should the nurse anticipate administering first?

- A. Diphenhydramine by mouth
- B. Subcutaneous regular insulin
- C. Intramuscular epinephrine
- D. IV furosemide

Correct Answer: C

The client is showing anaphylaxis: airway swelling, bronchospasm, and hypotension. Intramuscular epinephrine is the first-line treatment because it rapidly improves airway swelling, bronchoconstriction, and circulation. Diphenhydramine may be used later as adjunct therapy, but it does not act quickly enough to be first.

3657. A client with diabetic ketoacidosis is receiving treatment. Which finding requires immediate nursing action?

- A. Blood glucose decreases from 520 mg/dL to 320 mg/dL
- B. Urine output is 50 mL/hr
- C. Serum potassium is 2.9 mEq/L
- D. The client reports thirst

Correct Answer: C

A potassium level of 2.9 mEq/L is dangerously low and can lead to life-threatening cardiac dysrhythmias, especially during treatment for diabetic ketoacidosis. A falling glucose and adequate urine output are expected signs of improvement. Thirst is common but not the most urgent problem.

3658. A client one day after abdominal surgery becomes restless and confused. The nurse notes a respiratory rate of 10/min and oxygen saturation of 88% on room air. The client received opioid pain medication 20 minutes ago. Which action is most appropriate?

- A. Encourage the client to cough and deep breathe in 1 hour
- B. Administer the next scheduled opioid dose with food
- C. Stimulate the client, support ventilation, and prepare to give naloxone as prescribed
- D. Place the client in Trendelenburg position

Correct Answer: C

Recent opioid administration followed by decreased respiratory rate, low oxygen saturation, restlessness, and confusion suggests opioid-induced respiratory depression. Immediate support of breathing and preparation to reverse the opioid with naloxone are appropriate. Waiting an hour would delay treatment of a potentially life-threatening condition.

3659. A client with sepsis has a temperature of 39.4°C (102.9°F), heart rate 128/min, blood pressure 82/46 mm Hg, and urine output 15 mL in the last hour. Which finding best indicates progression to septic shock?

- A. Persistent hypotension with poor urine output
- B. Fever above 38°C (100.4°F)
- C. Heart rate greater than 100/min
- D. Warm, flushed skin

Correct Answer: A

Septic shock is marked by circulatory failure, especially persistent hypotension with signs of poor organ perfusion such as reduced urine output. Fever and tachycardia are common in sepsis but are not the best indicators of shock progression. Warm skin may occur early in sepsis but is less specific than hypotension with oliguria.

3660. A client with a history of peptic ulcer disease suddenly vomits a large amount of bright red blood. Which action should the nurse take first?

- A. Measure the amount of emesis and document the color

- B. Place the client in a side-lying position and call for immediate assistance
- C. Give clear liquids to replace lost fluid
- D. Administer a prescribed stool softener

Correct Answer: B

Massive hematemesis can rapidly threaten airway and circulation. Positioning the client to reduce aspiration risk and obtaining immediate help are the priorities. Measuring the emesis is useful, but only after the client is protected and emergency support is initiated.

3661. A client is 12 hours postoperative after a hip fracture repair. The client suddenly becomes dyspneic, confused, and tachycardic. A petechial rash appears over the chest. Which complication is most likely?

- A. Pulmonary edema
- B. Fat embolism syndrome
- C. Atelectasis
- D. Myocardial infarction

Correct Answer: B

After long-bone or hip fracture, sudden respiratory distress, neurologic changes, and a petechial rash strongly suggest fat embolism syndrome. Atelectasis can cause dyspnea but does not usually cause petechiae or sudden confusion in this pattern.

3662. A nurse is caring for a client with increased intracranial pressure. Which assessment finding indicates possible brain herniation and requires immediate intervention?

- A. Decorticate posturing when suctioned
- B. Headache rated 6 out of 10
- C. Pupils equal and reactive
- D. Urine output of 40 mL/hr

Correct Answer: A

Abnormal posturing in a client with increased intracranial pressure signals severe neurologic deterioration and possible impending or ongoing herniation. Equal and reactive pupils and normal urine output are less alarming. A headache alone is concerning but not as urgent as posturing.

3663. A client with end-stage kidney disease misses two hemodialysis treatments and now has muscle weakness. Which ECG change should the nurse expect if the client has severe hyperkalemia?

- A. Prominent peaked T waves
- B. ST-segment elevation in all leads
- C. Sawtooth atrial flutter waves
- D. Pathologic Q waves

Correct Answer: A

Missed dialysis can lead to severe hyperkalemia. A classic ECG finding is tall, peaked T waves, which may progress to widening of the QRS complex and dangerous dysrhythmias. The other findings are associated with different cardiac conditions.

3664. A client admitted with bacterial meningitis suddenly has a generalized tonic-clonic seizure. Which nursing action is the priority?

- A. Insert an oral airway between the teeth
- B. Restrain the client's arms and legs
- C. Turn the client to the side and protect the head
- D. Offer water after the seizure begins to prevent dehydration

Correct Answer: C

During a seizure, the priority is injury prevention and airway protection. Turning the client to the side and protecting the head help reduce aspiration and trauma. The nurse should not insert anything into the mouth or restrain the client, as those actions can cause harm.

3665. A client with acute respiratory distress becomes agitated and says, "I feel like I'm going to die." The monitor shows oxygen saturation dropping from 93% to 84% despite oxygen by face mask. Which action should the nurse take first?

- A. Ask the client to complete admission paperwork
- B. Remove the oxygen mask and recheck the saturation probe later
- C. Call for immediate assistance while positioning the client to optimize breathing
- D. Administer a sedative so the client can rest

Correct Answer: C

Rapidly worsening hypoxemia with agitation signals severe respiratory compromise. The nurse should get immediate help and position the client to improve ventilation, such as high Fowler's if tolerated, while preparing for further intervention. Sedation may worsen respiratory failure, and removing oxygen is unsafe.

3666. A client receiving a blood transfusion develops chills, low back pain, shortness of breath, and a feeling of impending doom 10 minutes after the infusion begins. Which action should the nurse take first?

- A. Slow the transfusion rate and reassess in 15 minutes
- B. Stop the transfusion and keep the IV line open with normal saline
- C. Administer prescribed acetaminophen for the chills
- D. Obtain a urine specimen to check for hemoglobin

Correct Answer: B

These findings suggest an acute transfusion reaction, which can become life-threatening quickly. The nurse should stop the transfusion immediately to prevent more blood product from entering the client, while maintaining IV access with normal saline. Slowing the infusion is unsafe because the reaction may worsen. Acetaminophen or specimen collection may be needed later, but they are not the first priority.

3667. A client with chest pain suddenly becomes unresponsive. The monitor shows a chaotic, irregular waveform with no identifiable P waves or QRS complexes. What is the nurse's priority action?

- A. Begin CPR and call for the defibrillator
- B. Administer oxygen by nonrebreather mask
- C. Place the client in high-Fowler position
- D. Check the blood pressure in both arms

Correct Answer: A

A chaotic, irregular rhythm with no organized complexes is consistent with ventricular fibrillation, a pulseless life-threatening rhythm. The priority is immediate CPR and rapid defibrillation. Oxygen is helpful during resuscitation, but it does not replace CPR and defibrillation. Positioning and blood pressure checks delay treatment of cardiac arrest.

3668. A client 2 days after childbirth reports sudden shortness of breath and sharp chest pain that worsens with inspiration. Assessment findings include respiratory rate 32/min, heart rate 124/min, and oxygen saturation 86% on room air. Which complication should the nurse suspect first?

- A. Pulmonary embolism
- B. Pulmonary edema from fluid overload
- C. Hospital-acquired pneumonia
- D. Acute asthma exacerbation

Correct Answer: A

Sudden pleuritic chest pain, tachycardia, severe dyspnea, and hypoxemia in a postpartum client strongly suggest pulmonary embolism, which is life-threatening and requires rapid response. Pulmonary edema can cause dyspnea but is less likely to present with sharp pleuritic chest pain. Pneumonia usually develops more gradually and often includes fever and cough. Asthma would typically include a relevant history and wheezing pattern.

3669. A client with a severe head injury has been awake and answering questions appropriately. Thirty minutes later, the client is difficult to arouse, the left pupil is fixed and dilated, and the blood pressure is 186/58 mm Hg with a heart rate of 48/min. Which interpretation is most appropriate?

- A. The client is showing expected fatigue after stimulation
- B. The client may be developing increasing intracranial pressure with possible herniation
- C. The findings are most consistent with opioid pain relief
- D. The client is experiencing a panic reaction

Correct Answer: B

A declining level of consciousness, unilateral fixed dilated pupil, hypertension with widened pulse pressure, and bradycardia are alarming signs of increased intracranial pressure and possible brain herniation. This is a life-threatening emergency. Fatigue would not explain the pupil change and vital sign pattern. Opioids may cause sedation, but they do not typically cause this classic neurologic and hemodynamic picture. Panic does not fit the decreased responsiveness.

3670. A nurse is caring for a client 24 hours after a thyroidectomy. The client suddenly reports a tight feeling in the throat and difficulty breathing. Which assessment finding most strongly suggests a life-threatening postoperative complication?

- A. Hoarse voice when speaking
- B. Frequent swallowing and swelling at the incision
- C. Pain rated 6 out of 10 at the surgical site
- D. Temperature of 37.8°C (100.0°F)

Correct Answer: B

Frequent swallowing and neck swelling after thyroid surgery suggest hemorrhage with a developing hematoma that can rapidly compress the airway, making this a life-threatening complication. Mild hoarseness can occur after surgery, incisional pain is expected, and a low-grade temperature is not the most urgent finding here.

3671. A client with liver failure becomes increasingly confused. During assessment, the nurse notes asterixis and a strong musty odor to the breath. Which laboratory value would the nurse expect to be elevated in this life-threatening condition?

- A. Serum ammonia
- B. Serum calcium
- C. Hemoglobin
- D. Platelet count

Correct Answer: A

These findings are consistent with hepatic encephalopathy, in which serum ammonia is commonly elevated and contributes to neurologic deterioration. Calcium, hemoglobin, and platelets may be abnormal in liver disease, but they do not best explain the confusion with asterixis and fetor hepaticus.

3672. A client with an anterior chest stab wound becomes increasingly short of breath. The nurse notes a sucking sound at the wound site with each inspiration. Which action should the nurse take first?

- A. Apply a sterile occlusive dressing taped on three sides
- B. Place the client in high-Fowler position
- C. Prepare the client for chest physiotherapy
- D. Irrigate the wound with sterile saline

Correct Answer: A

A sucking chest wound indicates an open pneumothorax. The first action is to apply an occlusive dressing taped on three sides to limit air entry while allowing air to escape, reducing worsening respiratory compromise. Positioning may help but does not correct the immediate problem. Irrigation and chest physiotherapy are inappropriate priorities.

3673. A hospitalized client suddenly develops facial drooping, slurred speech, and weakness of the right arm. The symptoms began 20 minutes ago. Which action is the nurse's priority?

- A. Offer sips of water to assess swallowing ability
- B. Document the findings and recheck in 1 hour
- C. Activate the stroke response system immediately
- D. Place a warm blanket over the client and reduce stimulation

Correct Answer: C

The client has signs of an acute stroke, and rapid activation of the stroke response system is the priority because time-sensitive evaluation and treatment may reduce brain injury. Giving fluids risks aspiration, waiting delays care, and comfort measures do not address the emergency.

3674. A client with acute pancreatitis becomes restless and dyspneic. Assessment reveals cool clammy skin, blood pressure 84/50 mm Hg, heart rate 132/min, and urine output 20 mL over 2 hours. Which complication should the nurse suspect?

- A. Hypovolemic shock
- B. Left-sided heart failure
- C. Acute appendicitis
- D. Pulmonary tuberculosis

Correct Answer: A

Acute pancreatitis can cause major third-spacing of fluid, leading to hypovolemia and shock. Hypotension, tachycardia, clammy skin, restlessness, and low urine output support poor perfusion. The other options do not fit the overall pattern as well.

3675. A client with syndrome of inappropriate antidiuretic hormone secretion has a serum sodium level of 118 mEq/L. Which new finding requires the most immediate nursing action?

- A. Dry mucous membranes
- B. Muscle cramps in the calves
- C. Progressive confusion and a new seizure
- D. Urine specific gravity of 1.030

Correct Answer: C

Severe hyponatremia can cause cerebral edema, leading to confusion and seizures, which are life-threatening and require immediate intervention. Concentrated urine fits SIADH, and muscle cramps may occur, but seizure activity signals the most urgent deterioration.

3676. A client receiving continuous tube feedings suddenly coughs, becomes cyanotic, and has coarse crackles after vomiting. Which complication is the nurse's priority concern?

- A. Aspiration of gastric contents
- B. Pulmonary edema from fluid overload
- C. Spontaneous pneumothorax
- D. Acute bronchitis

Correct Answer: A

Vomiting during tube feeding followed by coughing, cyanosis, and coarse crackles strongly suggests aspiration, which can rapidly threaten oxygenation. Fluid overload and bronchitis do not match the sudden event as well, and pneumothorax would not be linked to emesis in this way.

3677. A client with a bowel obstruction has increasing abdominal distention, fever, tachycardia, and sudden severe abdominal pain that is now followed by a rigid, boardlike abdomen. Which complication does the nurse suspect?

- A. Perforation with peritonitis
- B. Improving obstruction
- C. Simple constipation
- D. Acute cystitis

Correct Answer: A

A rigid, boardlike abdomen with fever, tachycardia, and sudden severe pain in the setting of bowel obstruction suggests perforation and peritonitis, a surgical emergency. This is not improvement, and the other options do not explain the severe abdominal findings.

3678. A client with a tracheostomy is restless and has increasing respiratory distress. The nurse cannot pass a suction catheter through the tracheostomy tube. Which action should the nurse take first?

- A. Deflate the cuff and encourage deep breathing
- B. Remove the inner cannula and assess for obstruction
- C. Turn the client to the side and offer oral fluids
- D. Secure the tracheostomy ties more tightly

Correct Answer: B

Inability to pass a suction catheter suggests obstruction of the tracheostomy tube, often from mucus. Removing and checking the inner cannula is the first action because it may quickly restore airflow. Deflating the cuff does not fix a blocked tube, oral fluids are unsafe in distress, and tighter ties do not address the obstruction.

3679. A client with severe preeclampsia suddenly reports epigastric pain and develops a generalized tonic-clonic seizure. What is the nurse's priority action?

- A. Insert a tongue blade into the client's mouth
- B. Restrain the client's arms and legs
- C. Turn the client to the side and protect the airway
- D. Leave the client briefly to obtain a blood pressure cuff

Correct Answer: C

The priority during an eclamptic seizure is to maintain safety and protect the airway by turning the client to the side. Nothing should be inserted into the mouth, restraints can cause injury, and the nurse should not leave during the active emergency.

3680. A client with acute chest pain becomes pale and diaphoretic. The cardiac monitor shows a regular wide-complex tachycardia at 190/min, and the client is confused with a blood pressure of 78/40 mm Hg. Which intervention should the nurse anticipate first?

- A. Synchronized cardioversion
- B. Oral beta blocker administration
- C. Encouraging the client to bear down
- D. Obtaining a 12-lead ECG before treatment

Correct Answer: A

This client has unstable tachycardia, likely ventricular tachycardia, shown by hypotension, confusion, diaphoresis, and a rapid wide-complex rhythm. Synchronized cardioversion is the priority treatment. Vagal maneuvers and oral medications are not appropriate in an unstable client, and treatment should not be delayed for a 12-lead ECG.

3681. A client with acute abdominal pain is admitted with a possible ruptured abdominal aortic aneurysm. Which assessment finding is most concerning?

- A. Severe back pain with a pulsatile abdominal mass and falling blood pressure
- B. Intermittent cramping abdominal pain relieved by position change
- C. Left lower quadrant tenderness with rebound after eating

D. Burning epigastric discomfort that improves with antacids

Correct Answer: A

A ruptured abdominal aortic aneurysm classically presents with severe abdominal or back pain, a pulsatile abdominal mass, and signs of shock such as falling blood pressure. The other findings suggest less immediate or different causes of abdominal discomfort.

Drainage devices

3682. A client returns from surgery with a chest tube connected to a drainage system. During the initial assessment, the nurse notes gentle tidaling in the water-seal chamber. What is the best interpretation of this finding?

- A.** The tube is completely obstructed and must be replaced
- B.** The drainage system is functioning as expected
- C.** The client has developed severe pulmonary edema
- D.** The suction pressure is too high and should be discontinued

Correct Answer: B

Gentle tidaling in the water-seal chamber usually indicates that pressure changes are being transmitted through a patent chest tube system. This is an expected finding. A common trap is assuming any movement in the chamber is abnormal. Complete obstruction would more likely stop expected fluctuation, not cause it.

3683. A client has a nasogastric tube to low intermittent suction after abdominal surgery. Which finding requires the nurse's immediate action?

- A.** The client reports mild throat irritation
- B.** Greenish drainage is present in the suction canister
- C.** The client becomes nauseated and the abdomen appears distended
- D.** The tube is secured to the gown to reduce tension

Correct Answer: C

Nausea with abdominal distention in a client with an NG tube to suction suggests the tube may not be decompressing the stomach effectively, which can indicate obstruction, displacement, or malfunction and requires prompt nursing action. Mild throat irritation and green drainage are common. Securing the tube to reduce tension is appropriate care.

3684. The nurse is caring for a client with a Jackson-Pratt drain after surgery. Which action is appropriate when emptying the drain?

- A.** Compress the bulb before replacing the plug
- B.** Leave the bulb expanded to prevent tissue trauma
- C.** Irrigate the drain with sterile water after each emptying
- D.** Clamp the tubing for 2 hours after measurement

Correct Answer: A

A Jackson-Pratt drain works by gentle suction. After emptying, the nurse compresses the bulb and then closes the plug to reestablish suction. Leaving the bulb expanded prevents suction. Routine irrigation or prolonged clamping is not appropriate unless specifically prescribed.

3685. A client has a chest tube for a pneumothorax. Which finding should the nurse report immediately?

- A.** Intermittent bubbling in the water-seal chamber during exhalation
- B.** Continuous bubbling in the water-seal chamber
- C.** Drainage of 20 mL of serosanguineous fluid in 4 hours
- D.** Discomfort at the insertion site when coughing

Correct Answer: B

Continuous bubbling in the water-seal chamber suggests an air leak in the drainage system and should be investigated promptly. Intermittent bubbling may be expected in some clients with a pneumothorax, especially during air evacuation. Small amounts of serosanguineous drainage and mild site discomfort are common. The trap is confusing intermittent expected bubbling with continuous bubbling, which is more concerning.

3686. A nurse is caring for a client with a Penrose drain after incision and drainage of an abscess. Which nursing action is most appropriate?

- A. Attach the drain to wall suction
- B. Measure drainage by emptying the collection bulb every shift
- C. Apply a sterile dressing to absorb drainage from the site
- D. Clamp the drain when the client ambulates

Correct Answer: C

A Penrose drain is an open drain that allows fluid to escape onto a dressing, so the nurse should apply and monitor a sterile dressing. It is not attached to wall suction and does not have a collection bulb. Clamping is not appropriate because it blocks drainage.

3687. A client has a chest tube connected to suction. The nurse notices the chest tube becomes accidentally disconnected from the drainage system. What is the nurse's priority action?

- A. Place the end of the chest tube in sterile water
- B. Clamp the chest tube close to the client and leave it clamped
- C. Remove the chest tube and cover the insertion site
- D. Raise the drainage system above chest level

Correct Answer: A

If a chest tube becomes disconnected, the priority is to reestablish a water seal by placing the tube end in sterile water until a new drainage system can be connected. This helps prevent air from entering the pleural space. A major trap is clamping the tube and leaving it clamped, which can increase the risk of tension pneumothorax in a client who still needs pleural drainage.

3688. The nurse is assessing a client with a nephrostomy tube. Which finding is most important to report to the provider?

- A. Urine is draining by gravity into the collection bag
- B. The dressing has a small amount of dried drainage
- C. The client reports flank pain and there is no urine output from the tube

D. The collection bag is kept below the level of the kidney

Correct Answer: C

Flank pain with absent nephrostomy drainage may indicate obstruction, kinking, or displacement and can threaten kidney function, so it should be reported promptly. Gravity drainage, a small amount of old drainage on the dressing, and keeping the bag below kidney level are expected or appropriate findings.

3689. A client with a T-tube after biliary surgery is being prepared for ambulation. Which action should the nurse take?

- A. Clamp the tube 30 minutes before activity
- B. Keep the drainage bag below the level of the abdomen
- C. Irrigate the tube with normal saline before the client stands
- D. Place the drainage bag on the bed beside the client

Correct Answer: B

A T-tube drains by gravity, so the drainage bag should be kept below the level of the abdomen to promote flow and prevent reflux. Routine clamping or irrigation is not appropriate unless prescribed. Keeping the bag on the bed risks raising it above the insertion site and impairing drainage.

3690. A provider prescribes gastric decompression with a Salem sump tube. Which nursing action best helps maintain the tube's function?

- A. Connect the blue air vent to suction
- B. Keep the blue air vent above the level of the stomach
- C. Clamp the tube after medication administration for 4 hours
- D. Instill water into the blue air vent every hour regardless of drainage

Correct Answer: B

The blue pigtail air vent on a Salem sump tube allows air to enter and helps prevent the tube from adhering to the gastric mucosa. It should remain above the level of the stomach and should not be attached to suction. The trap is treating the blue vent like a second suction port.

3691. A client has a chest tube after thoracic surgery. Which action by the nurse is appropriate to promote drainage and maintain safety?

- A. Milk the tubing vigorously every hour
- B. Keep the drainage system below the level of the chest
- C. Clamp the tube during routine repositioning
- D. Empty the water-seal chamber at the end of each shift

Correct Answer: B

Keeping the drainage system below chest level promotes drainage and helps prevent backflow into the pleural space. Routine milking and clamping are generally avoided unless specifically indicated because they can create harmful pressure changes or obstruct air/fluid escape. The water-seal chamber is part of the system and is not emptied as routine care.

3692. A nurse caring for a client with a chest tube notes sudden absence of tidaling in the water-seal chamber. Which action should the nurse take first?

- A. Assess the client for improved lung expansion and inspect the tubing for kinks
- B. Add more sterile water to the suction-control chamber
- C. Remove the chest tube because the lung has reexpanded
- D. Immediately clamp the chest tube near the insertion site

Correct Answer: A

Sudden absence of tidaling may mean the lung has reexpanded, but it can also mean the tubing is kinked or obstructed. The nurse should first assess the client and inspect the system before making assumptions. A common trap is assuming no tidaling automatically means the problem is resolved. Clamping or removing the tube without assessment is unsafe.

3693. A client had abdominal surgery 2 days ago and has a Jackson-Pratt drain. Which drainage finding should the nurse recognize as most concerning?

- A. A gradual decrease in serosanguineous output
- B. A sudden increase in bright red drainage
- C. A small amount of pink drainage in the bulb

D. Thin yellow drainage transitioning from pink

Correct Answer: B

A sudden increase in bright red drainage suggests possible active bleeding and is the most concerning finding. The other options describe common postoperative progression, with output often decreasing and drainage changing from sanguineous to serosanguineous or more serous. The key is recognizing the abrupt change and fresh blood as a red flag.

3694. A client's chest tube is accidentally pulled out. Which action should the nurse take immediately?

- A. Apply a sterile occlusive dressing taped on three sides over the insertion site
- B. Reinsert the chest tube using sterile gloves
- C. Leave the site open to allow air to escape freely
- D. Place the client flat and remove the old drainage system

Correct Answer: A

If a chest tube is dislodged, the nurse should immediately cover the site with a sterile occlusive dressing taped on three sides, which can help limit air entry while allowing air to escape. Reinsertion is not a nursing action. Leaving the site open or placing the client flat without addressing the site delays urgent management.

3695. A client has a Hemovac drain after orthopedic surgery. Which statement by the nurse is best?

- A. "I will measure and document the drainage each shift."
- B. "I will open the system frequently to keep the drain from clogging."
- C. "I will position the drain above the incision to increase suction."
- D. "I will disconnect the drain when turning the client in bed."

Correct Answer: A

Drainage from a Hemovac should be measured and documented to assess output and trends. The system should remain closed to reduce infection risk and preserve suction. Positioning it above the incision can impair drainage, and disconnecting it during turning is unsafe and unnecessary.

3696. A client with a chest tube suddenly develops severe dyspnea, tracheal deviation, and absent breath sounds on the affected side. The nurse finds the tubing tightly clamped during transport. What is the best interpretation?

- A.** The client is having a normal response to lung reexpansion
- B.** The clamp has likely contributed to a tension pneumothorax
- C.** The chest tube is no longer needed and can remain clamped
- D.** The suction setting is too low to remove pleural fluid

Correct Answer: B

In a client who still needs pleural decompression, a tightly clamped chest tube can trap air in the pleural space and contribute to tension pneumothorax, which fits the signs of severe dyspnea, tracheal deviation, and absent breath sounds. The trap is focusing only on the device rather than linking the clamped tube to a life-threatening physiologic change.

3697. A client has a new chest tube connected to a drainage system after treatment for a hemothorax. During assessment, the nurse sees continuous bubbling in the water-seal chamber. What should the nurse suspect first?

- A.** A normal effect of lung re-expansion
- B.** An air leak in the drainage system
- C.** That the suction setting is too low
- D.** Expected drainage from pleural bleeding

Correct Answer: B

Continuous bubbling in the water-seal chamber usually suggests an air leak somewhere in the system. The nurse should assess the tubing connections and the insertion site. Gentle intermittent bubbling may be expected in some systems when suction is used in the suction-control chamber, but continuous bubbling in the water-seal chamber is not a normal sign of lung re-expansion.

3698. A client has a newly inserted nasogastric tube for gastric decompression. Before administering a prescribed medication through the tube, which action is most appropriate?

- A.** Confirm tube placement according to facility policy
- B.** Add the medication to the suction canister tubing
- C.** Place the client flat to improve medication delivery
- D.** Clamp the tube for 12 hours after the medication

Correct Answer: A

The nurse should verify tube placement according to facility policy before giving anything through a nasogastric tube. This helps prevent instilling medication into the wrong location, such as the respiratory tract. The other options are unsafe or inappropriate: medication is not added to suction tubing, lying flat increases aspiration risk, and clamping for a fixed 12 hours is not standard.

3699. A client has a Hemovac drain after hip surgery. Which observation best indicates that the drain is functioning properly?

- A.** The collection chamber is compressed and maintains suction
- B.** The tubing is clamped between drainings
- C.** Drainage is backing up around the insertion site
- D.** The reservoir remains expanded after being emptied

Correct Answer: A

A Hemovac works by closed suction, so the collection chamber should be compressed and remain compressed to provide negative pressure. Clamping the tubing routinely can obstruct drainage. Backup at the insertion site suggests poor function or blockage. If the reservoir stays expanded, suction has not been re-established.

3700. A client with a newly created ileostomy asks why the drainage in the pouch is liquid. Which response by the nurse is best?

- A.** Liquid drainage is expected because the output has not reached the colon

- B.** This means the ostomy is blocked and needs irrigation
- C.** The pouch is leaking stool from the skin barrier
- D.** The stoma is not functioning unless the drainage becomes formed

Correct Answer: A

An ileostomy typically produces liquid to semiliquid output because intestinal contents bypass the colon, where more water would normally be absorbed. A blocked ostomy would more likely show reduced output with other concerning symptoms. Liquid output alone does not mean pouch leakage or stoma failure.

3701. A client has a chest tube drainage system with a collection chamber that contained 120 mL of drainage 1 hour ago and now contains 260 mL. The drainage is bright red. What is the nurse's priority action?

- A.** Document the finding as expected after chest tube insertion
- B.** Lower the drainage system below chest level
- C.** Notify the provider immediately
- D.** Milk the chest tube to clear a possible clot

Correct Answer: C

An increase of 140 mL of bright red drainage in 1 hour suggests significant bleeding and requires prompt notification of the provider. Lowering the system below chest level is appropriate routine care, but it does not address the urgent change. Milking the tube is generally not done routinely and may be harmful. The finding should not simply be documented as expected.

3702. A nurse is reinforcing teaching for a client going home with a biliary drainage tube. Which client statement shows correct understanding?

- A.** I will keep the drainage bag higher than my abdomen when I walk
- B.** I will record the amount and color of the drainage
- C.** I should clamp the tube after meals unless it hurts
- D.** I can stop skin care around the tube once the drainage turns lighter

Correct Answer: B

Recording the amount and appearance of drainage is an important part of home management and helps identify changes that may need reporting. The drainage bag should generally remain below the insertion site to promote drainage. Clamping should not be done unless specifically prescribed. Skin care remains important even if drainage changes color.

3703. A client with a nasogastric tube to low intermittent suction reports nausea and abdominal fullness. The nurse notes minimal drainage in the collection canister. Which action should the nurse take first?

- A.** Increase the suction pressure
- B.** Assess the tubing for kinks or obstruction
- C.** Remove the tube and replace it immediately
- D.** Give oral fluids to stimulate gastric emptying

Correct Answer: B

Nausea, abdominal distention, and minimal output suggest the tube may not be draining properly. The first action is to check for a simple mechanical problem such as kinking or obstruction. Increasing suction can damage gastric mucosa, immediate removal is premature, and oral fluids are inappropriate in a client needing decompression.

3704. A nurse is caring for a client with a Penrose drain after a superficial surgical procedure. Which finding is most important to recognize as a risk associated with this type of drain?

- A.** The drain can allow microorganisms to travel inward from the wound surface
- B.** The drain creates excessive negative pressure in deep tissues
- C.** The drain prevents any drainage from reaching the dressing
- D.** The drain eliminates the need for frequent skin assessment

Correct Answer: A

A Penrose drain is an open drain, so it increases infection risk because organisms can track inward from the external environment. It does not create negative pressure, because it is not a suction drain. Drainage often reaches the dressing, and skin assessment remains important because drainage can irritate surrounding tissue.

3705. A client with a suprapubic urinary catheter has had 15 mL of urine output over the last 2 hours. The tubing is looped over the side rail, and the collection bag is attached to the bed frame below bladder level. Which nursing action is best?

- A.** Clamp the catheter for 30 minutes, then reassess output
- B.** Reposition the tubing to remove dependent loops and check for flow
- C.** Raise the bag to bladder level to improve drainage
- D.** Irrigate the catheter routinely with sterile water

Correct Answer: B

Dependent loops can obstruct urine flow by allowing urine to pool in the tubing. Repositioning the tubing is the best first action. Clamping would further block drainage. Raising the bag to bladder level increases backflow risk. Routine irrigation is not done unless specifically prescribed.

3706. A client has a chest tube attached to a drainage system. The drainage unit is accidentally knocked over, and the water-seal chamber spills out. The client is breathing comfortably. What should the nurse do first?

- A.** Clamp the chest tube near the insertion site until a new system is available
- B.** Place the end of the chest tube in sterile fluid while preparing a new drainage system
- C.** Remove the chest tube because the sterile seal has been broken
- D.** Tape all connections and continue using the same drainage unit

Correct Answer: B

If the drainage unit is no longer functioning because the water seal is lost, the priority is to re-establish a water seal to prevent air from being drawn back into the pleural space. Temporarily placing the tube end in sterile fluid while setting up a new system is appropriate. Clamping can increase the risk of a tension pneumothorax. The tube should not be removed, and a spilled drainage unit should not simply continue to be used.

3707. A client has an enteral feeding tube and suddenly begins coughing, becomes short of breath, and has oxygen saturation that drops from 97% to 89% during feeding. Which action should the nurse take first?

- A.** Slow the feeding rate and continue to observe
- B.** Stop the feeding and assess tube placement and the client's respiratory status
- C.** Flush the tube with water to ensure patency
- D.** Place the client flat to reduce abdominal pressure

Correct Answer: B

Coughing, dyspnea, and falling oxygen saturation during tube feeding raise concern for aspiration or malposition. The first action is to stop the feeding and assess the client and tube placement. Slowing the feeding does not address the immediate risk. Flushing may worsen the problem if placement is incorrect. Lying flat increases aspiration risk.

3708. A nurse is caring for a client who has a chest tube connected to a drainage system after a pneumothorax. During assessment, the nurse notes that the drainage tubing has no dependent loops, the system is below chest level, and there is intermittent bubbling in the water-seal chamber when the client coughs. Tidaling is also present. Which interpretation is most appropriate?

- A.** The chest tube system is functioning normally
- B.** The client has developed a tension pneumothorax
- C.** The suction pressure is set too high
- D.** The chest tube is obstructed and needs immediate irrigation

Correct Answer: A

Intermittent bubbling in the water-seal chamber with coughing can be expected, and tidaling usually indicates that the system is patent and responding to pressure changes with breathing. The setup described also includes correct positioning of the drainage system below chest level and no dependent loops. Tension pneumothorax would be suggested by severe respiratory distress and other signs of worsening condition, not by normal tidaling. High suction affects the suction control chamber rather than causing this expected water-seal finding. An obstructed tube is more likely when tidaling is absent or drainage suddenly stops with other concerning findings.

Thermoregulation

3709. A nurse is caring for an older adult who has an oral temperature of 95.9°F (35.5°C), cool pale skin, and mild confusion after being in an overly air-conditioned room. Which action should the nurse take first?

- A. Apply warm blankets and remove the client from the cold environment
- B. Massage the client's arms and legs to improve circulation
- C. Offer an iced drink to stimulate alertness
- D. Leave the client resting and recheck the temperature in 2 hours

Correct Answer: A

The client shows signs of mild hypothermia, so the priority is to stop heat loss and begin rewarming with a warmer environment and blankets. Massaging extremities can worsen heat loss and is not the first action. Iced fluids would further lower body temperature. Waiting 2 hours delays needed intervention.

3710. A client with a fever is shivering and says, "I feel so cold." Which phase of the febrile process is the client most likely experiencing?

- A. Defervescence
- B. Onset or chill phase
- C. Recovery phase
- D. Afebrile baseline phase

Correct Answer: B

Shivering and a sensation of feeling cold occur during the onset or chill phase of fever, when the hypothalamic set point has increased and the body is trying to generate heat. Defervescence is the period when the temperature falls and sweating is more common. The other options do not fit the described pattern.

3711. A nurse is caring for a postoperative client with a temperature of 102.4°F (39.1°C). Which nursing intervention best promotes heat loss through evaporation?

- A. Removing an extra blanket
- B. Applying ice packs to the groin
- C. Using a cooling fan on damp skin
- D. Lowering the room lights

Correct Answer: C

Evaporation occurs when moisture on the skin turns to vapor, taking heat with it. A fan directed at damp skin increases evaporative heat loss. Removing a blanket mainly reduces insulation, and ice packs promote conduction. Lowering lights does not meaningfully affect body temperature.

3712. The nurse is teaching a nursing assistant about clients at risk for impaired thermoregulation. Which client should the nurse identify as having the highest risk?

- A. A healthy 22-year-old after a short walk outdoors
- B. A 35-year-old with a sprained ankle
- C. A 78-year-old with decreased mobility and poor nutritional intake
- D. A 40-year-old with seasonal allergies

Correct Answer: C

Older adults, especially those with limited mobility and poor nutrition, are at high risk for temperature instability because they often have decreased subcutaneous tissue, reduced physiologic reserves, and less effective heat conservation. The other clients do not have the same degree of vulnerability.

3713. A client recovering from heat exhaustion becomes restless and reports dizziness. Vital signs are temperature 100.8°F (38.2°C), pulse 118/min, blood pressure 88/54 mm Hg, and skin is cool and clammy. Which intervention is most appropriate?

- A. Place the client in a supine position and begin oral fluids only
- B. Encourage ambulation to improve circulation
- C. Move the client to a cool area and begin fluid replacement as prescribed
- D. Cover the client with multiple blankets to prevent shivering

Correct Answer: C

This client has findings consistent with heat exhaustion: dizziness, tachycardia, hypotension, and cool clammy skin. The appropriate response is cooling and fluid replacement. Oral fluids alone may be insufficient depending on condition and orders. Ambulation could worsen hypotension. Blankets would increase heat retention.

3714. A client has a sudden temperature spike to 104.2°F (40.1°C), hot dry skin, tachycardia, and altered mental status after working outdoors. Which finding is the nurse's priority concern?

- A. Elevated temperature
- B. Dry skin
- C. Altered mental status
- D. History of outdoor activity

Correct Answer: C

Altered mental status suggests possible heat stroke with central nervous system involvement, a life-threatening condition that requires immediate action. The high temperature and dry skin support the diagnosis, but the neurologic change signals severe physiologic compromise and drives urgency.

3715. A nurse is caring for a client with a high fever. Which assessment finding indicates the client is most likely entering the defervescence phase?

- A. The client begins shivering and asks for more blankets
- B. The client has flushed skin and profuse sweating

C. The client reports feeling chilled despite a rising temperature

D. The client develops goose bumps and cool extremities

Correct Answer: B

Defervescence is the phase in which the fever breaks and the body loses heat, often through vasodilation and sweating. Shivering, chills, goose bumps, and cool extremities are more consistent with the early chill phase when the body is trying to raise temperature.

3716. A postoperative client has a temperature of 101.9°F (38.8°C). Which additional assessment finding should the nurse report immediately because it may indicate progression to a more serious problem rather than an uncomplicated fever?

- A. Heart rate of 104/min
- B. Respiratory rate of 22/min
- C. New disorientation to place
- D. Complaint of thirst

Correct Answer: C

New disorientation is a significant change in neurologic status and may indicate worsening physiologic instability or severe hyperthermia. Mild tachycardia, a slightly increased respiratory rate, and thirst commonly accompany fever and are less urgent than acute mental status changes.

3717. The nurse is planning care for a client with hypothermia. Which intervention is appropriate for rewarming and ongoing monitoring?

- A. Apply direct high heat with a heating pad set to the highest setting
- B. Monitor temperature trends and rewarm gradually
- C. Rub the client's cold skin vigorously to restore warmth
- D. Encourage smoking to stimulate peripheral circulation

Correct Answer: B

A hypothermic client should be rewarmed in a controlled, gradual manner while temperature and overall status are monitored. Direct high heat can cause burns, especially in clients with decreased sensation or poor perfusion. Vigorous rubbing is inappropriate, and smoking causes vasoconstriction rather than safe warming.

3718. A nurse is caring for a client with a fever who is lightly covered and lying in a room with moving air. The nurse explains that body heat is being lost to the cooler air around the client. Which mechanism of heat loss is this?

- A. Conduction
- B. Radiation
- C. Convection
- D. Evaporation

Correct Answer: C

Convection is heat loss through air movement carrying heat away from the body. Conduction is transfer by direct contact, radiation is transfer to cooler objects not in direct contact, and evaporation occurs when liquid on the skin changes to vapor.

3719. A client with severe hyperthermia is receiving cooling measures. Which assessment finding best indicates that the interventions are effective?

- A. The client requests another blanket
- B. The client's temperature gradually decreases toward normal and mental status improves
- C. The skin remains hot and dry but pulse pressure widens
- D. The client becomes more lethargic and less responsive to questions

Correct Answer: B

Effective treatment of severe hyperthermia should lower body temperature and improve neurologic status. Requesting a blanket may occur during cooling but does not by itself show successful stabilization. Persistent hot dry skin and worsening lethargy suggest continued or worsening hyperthermia.

3720. A nurse finds a client who was receiving a blood transfusion suddenly febrile with chills, low back pain, and anxiety. Which interpretation is most appropriate?

- A. The client is having a normal warming response from the transfused blood
- B. The client may be experiencing a transfusion reaction affecting thermoregulation
- C. The client is entering the recovery phase of a viral illness
- D. The client is showing expected postoperative temperature variation

Correct Answer: B

Fever, chills, low back pain, and anxiety during a blood transfusion suggest a possible transfusion reaction, which requires prompt recognition and action. This is not a normal response. The timing and symptom cluster make the other explanations less appropriate.

3721. A nurse is caring for a client with moderate hypothermia. Which finding would the nurse expect as thermoregulation becomes more severely impaired?

- A. Increasing shivering and warm flushed skin
- B. Bradycardia, slowed respirations, and decreased level of consciousness
- C. Bounding pulse and profuse diaphoresis
- D. Hypertension with heightened alertness

Correct Answer: B

As hypothermia worsens, body systems slow down, commonly causing bradycardia, hypoventilation, and decreased consciousness. Increasing shivering is more typical earlier; severe hypothermia often leads to reduced shivering. Bounding pulse and diaphoresis fit hyperthermia, not worsening hypothermia.

3722. A client develops malignant hyperthermia during surgery. After emergency measures are initiated by the surgical team, which finding should the nurse recognize as most concerning for rapid deterioration?

- A. Generalized muscle rigidity with rapidly rising temperature
- B. Mild nausea after anesthesia
- C. A temporary decrease in appetite
- D. Localized redness at the IV site

Correct Answer: A

Malignant hyperthermia is a life-threatening hypermetabolic state. Muscle rigidity with a rapidly increasing temperature signals severe and escalating physiologic crisis. The other findings are far less urgent and do not reflect the hallmark pattern of this emergency.

3723. A nurse is caring for an infant with a low body temperature. Which factor best explains why infants are at greater risk for temperature instability than adults?

- A.** They have a larger body surface area relative to weight and limited temperature regulation reserves
- B.** They sweat more efficiently than adults
- C.** They can voluntarily increase activity when cold
- D.** They have thicker insulating fat layers than most adults

Correct Answer: A

Infants are more vulnerable to heat loss because they have a larger surface area relative to body mass and immature thermoregulatory mechanisms. They do not sweat more effectively than adults, cannot reliably increase activity to warm themselves, and do not have better insulation.

3724. A nurse is caring for a client whose temperature has risen to 103.1°F (39.5°C). The client is flushed, no longer shivering, and reports feeling very warm. Which physiologic response should the nurse expect at this time?

- A.** Peripheral vasodilation to increase heat loss
- B.** Intense skeletal muscle contractions to raise body temperature
- C.** Decreased skin blood flow to conserve heat
- D.** Suppression of sweating to reduce fluid loss

Correct Answer: A

When the client feels hot and is flushed after the temperature has risen, the body is attempting to lose heat. Peripheral vasodilation increases blood flow to the skin and promotes heat loss. Shivering and decreased skin blood flow are heat-conserving responses more consistent with the chill phase. Suppressing sweating would also limit heat loss rather than promote it.

3725. A nurse is teaching a client about body heat transfer. Which example best illustrates heat loss by conduction?

- A.** A client loses heat while standing near an open window on a cool day
- B.** A client loses heat when cool water is applied to the skin
- C.** A client loses heat by perspiring after exercise
- D.** A client loses heat when infrared body heat moves into the surrounding air

Correct Answer: B

Conduction is heat transfer through direct contact between objects of different temperatures. Cool water applied to the skin removes heat by direct contact. Standing near a cool window describes convection because moving cooler air carries heat away. Perspiration is evaporation. Infrared heat moving from the body is radiation.

3726. A client with a temperature of 104°F (40°C) is restless and diaphoretic. Which nursing action is most appropriate to reduce metabolic demand related to the fever?

- A.** Encourage the client to ambulate every hour
- B.** Provide a warm blanket to stop heat loss
- C.** Promote rest and limit unnecessary activity
- D.** Offer a high-protein snack every 30 minutes

Correct Answer: C

Fever increases metabolic rate and oxygen consumption. Promoting rest and limiting unnecessary activity helps reduce metabolic demand. Ambulation would increase energy use. A warm blanket could trap heat and worsen hyperthermia unless the client is in a chill phase. Frequent snacks do not directly reduce metabolic demand and may not be appropriate during acute fever.

3727. A nurse assesses four clients. Which client is showing the most concerning sign of heat stroke rather than heat exhaustion?

- A.** A client with heavy sweating, muscle cramps, and thirst after a tennis match
- B.** A client with headache, nausea, and moist skin after working in a hot kitchen
- C.** A client with confusion, hot skin, and a temperature of 105°F (40.6°C) after hiking
- D.** A client with weakness, tachycardia, and dizziness that improves when lying down

Correct Answer: C

Heat stroke is a life-threatening emergency associated with very high temperature and central nervous system dysfunction such as confusion. Hot skin and a temperature of 105°F strongly support heat stroke. The other options are more consistent with heat exhaustion or heat cramps, which can still be serious but are less immediately dangerous than altered mental status with extreme hyperthermia.

3728. A nurse is caring for a client admitted from a cold environment. The client is drowsy, has slurred speech, and a core temperature of 89.6°F (32°C). Which prescription should the nurse question?

- A. Apply warmed blankets
- B. Monitor cardiac rhythm continuously
- C. Massage the client's arms and legs vigorously
- D. Administer warmed IV fluids as prescribed

Correct Answer: C

Vigorous massage of extremities in hypothermia should be questioned because it can cause peripheral vasodilation and may precipitate cardiovascular instability. Warmed blankets, cardiac monitoring, and warmed IV fluids are appropriate supportive measures for hypothermia. A likely trap is choosing warmed blankets because they seem too simple, but they are appropriate rewarming support.

3729. A nurse notes that a febrile client's set point has returned to normal after treatment of the underlying cause. Which assessment finding best reflects this change?

- A. The client begins shivering and asks for extra blankets
- B. The client develops pale, cool skin from vasoconstriction
- C. The client becomes sweaty as vasodilation occurs
- D. The client reports feeling colder as metabolic rate falls

Correct Answer: C

When the hypothalamic set point returns to normal, the previously elevated body temperature is now too high relative to the new set point. The body responds with heat-loss mechanisms such as vasodilation and sweating. Shivering and vasoconstriction occur when the body is trying to raise temperature to a higher set point, not lower it.

3730. During a summer health fair, a nurse evaluates several clients exposed to heat. Which finding requires immediate emergency response?

- A. Temperature 99.8°F (37.7°C), intense thirst, and dry mouth
- B. Temperature 101.2°F (38.4°C), weakness, and profuse sweating
- C. Temperature 102.1°F (38.9°C), painful leg cramps, and fatigue
- D. Temperature 104.6°F (40.3°C), disorientation, and rapid shallow respirations

Correct Answer: D

Disorientation with a temperature above 104°F indicates probable heat stroke, a life-threatening emergency requiring rapid intervention. The other findings suggest dehydration, heat exhaustion, or heat cramps, which are important but less immediately critical than hyperthermia with altered mental status.

3731. A nurse cares for a client with a spinal cord injury above the level of T6. The room temperature is elevated, and the client is becoming overheated. Which explanation best describes why this client is at risk for impaired thermoregulation?

- A. The client cannot increase thyroid hormone secretion quickly enough
- B. The client may have disrupted autonomic control of sweating and vasomotor tone
- C. The client has reduced heat production because all skeletal muscle activity stops
- D. The client cannot sense temperature changes in the skin, so body temperature stays normal

Correct Answer: B

Spinal cord injury can impair autonomic pathways that regulate sweating and control vasodilation or vasoconstriction, making it difficult to dissipate or conserve heat. Thyroid hormone is not the main explanation in this situation. Muscle activity may be reduced, but that does not explain the major thermoregulatory problem. Lack of sensation does not keep body temperature normal; it may prevent awareness while regulation remains impaired.

3732. A client is receiving external cooling for severe hyperthermia. Which assessment finding indicates the nurse should modify the cooling measures to avoid increasing heat production?

- A. The client's skin is warm and flushed
- B. The client begins to shiver intensely
- C. The client asks for small sips of water
- D. The client reports feeling tired

Correct Answer: B

Shivering increases metabolic activity and heat production, which can counteract cooling efforts. If intense shivering begins, the nurse should recognize that cooling measures may need adjustment based on the care plan and provider orders. Warm flushed skin can occur during hyperthermia and does not by itself indicate the cooling method is worsening heat production. Thirst and fatigue are nonspecific.

3733. A nurse is teaching a group of students about older adults and thermoregulation. Which statement by a student indicates correct understanding?

- A. Older adults usually develop high fevers quickly because metabolism is increased
- B. Older adults are less likely to have temperature instability because body fat is higher
- C. Older adults may have a blunted temperature response and show serious infection without a marked fever
- D. Older adults compensate for cold stress better because peripheral circulation becomes more efficient

Correct Answer: C

Older adults often have impaired thermoregulation and may not mount a strong febrile response even with significant infection. That makes subtle changes important. The other statements are incorrect because aging generally decreases physiologic reserve and thermoregulatory efficiency rather than improving it.

3734. A postoperative client in the PACU develops muscle rigidity, rising end-tidal carbon dioxide, tachycardia, and a rapidly increasing temperature. Which action is the nurse's priority?

- A. Apply ice packs and continue to observe for further changes
- B. Notify the surgical team that the client may be experiencing malignant hyperthermia
- C. Administer an antipyretic for the elevated temperature
- D. Encourage oral fluids to prevent dehydration

Correct Answer: B

This pattern is highly concerning for malignant hyperthermia, a life-threatening emergency that requires immediate recognition and rapid team response. Prompt notification activates emergency treatment. Ice packs alone are insufficient. Antipyretics do not treat the underlying crisis, and oral fluids are not the priority in an unstable postoperative client.

Wound care

3735. A nurse changes a client's abdominal dressing 24 hours after surgery. Which finding should be reported to the provider first?

- A. A small amount of serosanguineous drainage on the old dressing
- B. Staple line edges that are well approximated
- C. A sudden gush of pink drainage with separation of the wound edges
- D. Mild incisional pain during the dressing change

Correct Answer: C

A sudden gush of drainage with separation of the wound edges suggests dehiscence, which can quickly progress to evisceration and requires immediate action. A small amount of serosanguineous drainage, well-approximated edges, and mild pain are common early postoperative findings and are less urgent.

3736. A postoperative client coughs forcefully and the nurse notes abdominal organs protruding through the incision. What is the nurse's first action?

- A. Apply sterile saline-moistened dressings over the area
- B. Attempt to gently place the organs back into the abdomen
- C. Position the client flat with legs extended
- D. Remove the staples to reduce tension on the wound

Correct Answer: A

Protruding abdominal organs indicate evisceration. The nurse should immediately cover the area with sterile saline-moistened dressings to protect exposed tissue and help prevent drying, then position the client appropriately and notify the provider. The organs should not be pushed back in, and removing staples is not the nurse's first action.

3737. A nurse is caring for a client with a pressure injury covered by black, leathery tissue. How should the nurse document this tissue?

- A. Eschar
- B. Slough
- C. Granulation tissue
- D. Maceration

Correct Answer: A

Black, dry, leathery necrotic tissue is eschar. Slough is typically yellow or tan and moist. Granulation tissue is beefy red and indicates healing. Maceration refers to pale, soggy skin from excess moisture.

3738. Which action by a client indicates correct understanding of how to promote healing of a surgical wound at home?

- A. Using the same washcloth for the incision each day until laundry day

- B. Increasing protein intake as recommended
- C. Applying powder to keep the incision dry
- D. Scrubbing the incision to remove crusting

Correct Answer: B

Adequate protein supports tissue repair and wound healing. Reusing an unclean washcloth increases infection risk. Applying powder and scrubbing the incision can irritate tissue and interfere with healing.

3739. A nurse irrigates a deep wound using a syringe and prescribed solution. Which technique is most appropriate?

- A. Direct the flow from the least contaminated area toward the most contaminated area
- B. Use forceful pressure to remove all tissue debris
- C. Irrigate from the bottom of the wound outward to the surrounding skin
- D. Save unused solution from the setup for the next dressing change

Correct Answer: A

Wound irrigation should flow from the least contaminated area to the most contaminated area to limit transfer of microorganisms. Forceful pressure can damage healing tissue. Irrigating outward onto surrounding skin and saving used setup supplies increase contamination risk.

3740. A client with a venous leg ulcer asks why the nurse measures the wound at each dressing change. Which response is best?

- A. It confirms whether the dressing was placed correctly
- B. It helps evaluate whether the wound is healing or worsening over time
- C. It determines the client's pain tolerance
- D. It shows whether the client is following the diet plan exactly

Correct Answer: B

Serial wound measurements help the nurse evaluate progress, compare healing over time, and identify deterioration early. Dressing placement, pain tolerance, and exact dietary adherence are not the main reasons for measuring wound dimensions.

3741. A nurse removes an old dressing and notes the wound drainage is thin, watery, and pale yellow. How should this drainage be documented?

- A. Purulent
- B. Sanguineous
- C. Serous
- D. Fecal

Correct Answer: C

Serous drainage is thin, watery, and pale yellow or straw-colored. Purulent drainage is thick and often yellow, green, or brown. Sanguineous drainage is primarily bloody. Fecal drainage suggests bowel involvement and is not described here.

3742. A client with a stage 2 pressure injury has surrounding skin that appears pale, soft, and soggy from repeated exposure to drainage. Which term best describes this finding?

- A. Induration
- B. Maceration
- C. Epithelialization
- D. Undermining

Correct Answer: B

Maceration is softening and breakdown of skin from prolonged moisture exposure. Induration means hardening. Epithelialization refers to new surface skin growth. Undermining is tissue destruction under intact wound edges.

3743. A nurse is reinforcing teaching for a client with a new pressure injury. Which statement by the client shows a need for further teaching?

- A. I should change positions regularly to reduce pressure on the area.
- B. I will try to keep the skin clean and dry.
- C. I can massage the reddened area to improve circulation.
- D. I need to eat enough protein and calories to support healing.

Correct Answer: C

Massaging a reddened pressure area can worsen tissue damage and is not appropriate. Repositioning, moisture control, and adequate nutrition help reduce further injury and support healing.

3744. A nurse assesses a tunneling wound on a client's sacrum. Which documentation is most complete?

- A. Wound is deep and draining
- B. Tunnel present at 3 o'clock extending 4 cm, moderate serous drainage
- C. Pressure injury noted with some tissue loss
- D. Open wound cleaned and redressed

Correct Answer: B

Complete wound documentation includes specific measurable findings, such as the location and depth of tunneling and the type and amount of drainage. The other options are vague and do not provide enough detail to track changes over time.

3745. A nurse is caring for a client whose packed wound dressing is difficult to remove because it has dried onto the wound bed. Which action is best?

- A. Pull the packing out quickly to reduce discomfort
- B. Moisten the dressing with prescribed solution before removing it
- C. Leave the dressing in place and add new packing on top
- D. Remove only the visible portion and document the rest as retained

Correct Answer: B

Moistening a dried dressing before removal helps reduce trauma to healing tissue and decreases pain. Pulling it out quickly can damage the wound bed. Leaving old packing in place or only partly removing it creates a serious risk for infection and impaired healing.

3746. The nurse is assessing several wounds. Which finding is the clearest sign of wound infection?

- A. Beefy red granulation tissue in the wound bed
- B. Scant serosanguineous drainage on the dressing
- C. Increasing purulent drainage with foul odor and surrounding warmth
- D. Mild tenderness at the incision 1 day after surgery

Correct Answer: C

Purulent drainage, foul odor, and warmth of surrounding tissue are classic signs suggesting infection. Granulation tissue is a healing sign. Scant serosanguineous drainage and mild early postoperative tenderness can be expected findings.

3747. A nurse is preparing to change a sterile postoperative dressing. After setting up the sterile field, the nurse accidentally touches the inner 1-inch edge of the sterile tray with an ungloved hand. What should the nurse do next?

- A.** Proceed because the outer edge of the field is considered contaminated, not the inner edge
- B.** Ask another nurse to finish the procedure using the same field
- C.** Replace the contaminated setup with a new sterile field
- D.** Cover the touched area with a sterile towel and continue

Correct Answer: C

Any contact with the inner sterile area contaminates the setup, so the nurse should discard it and prepare a new sterile field. Continuing, handing off the same field, or covering the contaminated area does not restore sterility. A common trap is confusing the contaminated outer edge with the actually touched inner area.

3748. A client has a deep wound healing by secondary intention. Which assessment finding best indicates progress toward healing?

- A.** Wound edges are pulled together with staples
- B.** The wound bed develops healthy granulation tissue and gradually fills in from the base
- C.** Drainage changes from serous to thick green exudate
- D.** The surrounding skin becomes increasingly white and soft

Correct Answer: B

Secondary intention healing occurs when a wound fills with granulation tissue from the bottom up and contracts over time. Staples indicate primary intention, not secondary intention. Thick green drainage suggests infection, and increasingly white, soft surrounding skin suggests maceration.

3749. A nurse is caring for a client with a wound drain after surgery. Over the last hour, the drainage amount has increased sharply and changed from light pink to bright red. What is the nurse's priority interpretation?

- A.** The wound is entering the inflammatory phase of healing
- B.** The client may be experiencing active bleeding
- C.** This is expected as the drain becomes fully functional
- D.** The drainage is becoming serous as healing progresses

Correct Answer: B

A sudden increase in bright red drainage suggests active bleeding and requires prompt assessment and intervention. Light pink drainage may be expected early, but a sharp increase with bright red blood is not a routine sign of healing. Serous drainage would be thinner and pale yellow rather than bright red.

3750. A nurse is reinforcing teaching for a client with a new surgical incision. Which statement by the client shows correct understanding of normal wound healing?

- A.** My incision should stay the same size until the sutures are removed.
- B.** A small amount of redness right at the edges can be expected early on.
- C.** Thick green drainage means the wound is sealing itself.
- D.** Severe swelling is the best sign that healing is progressing.

Correct Answer: B

A small amount of redness at the wound edges can be an expected early inflammatory response. Thick green drainage suggests infection, not healing. Severe swelling is not a normal sign of progress, and many wounds gradually improve rather than staying unchanged until suture removal.

3751. A nurse is caring for a client with a Jackson-Pratt drain after surgery. Which action is most important to help maintain suction?

- A. Compress the bulb before replacing the cap
- B. Raise the drain above the incision every 2 hours
- C. Flush the tubing with sterile water after emptying
- D. Leave the cap open briefly to reduce pressure

Correct Answer: A

A Jackson-Pratt drain works by closed suction. The bulb must be compressed before the cap is secured so negative pressure is created. Raising the drain above the wound can impair drainage, flushing is not done unless specifically prescribed, and leaving the cap open prevents suction from being maintained.

3752. A home health nurse is reinforcing teaching for a client with a healing leg wound. Which client action best helps prevent disruption of fragile granulation tissue during dressing changes?

- A. Using cotton balls to scrub the wound bed clean
- B. Removing old dressings quickly in one pull
- C. Following the prescribed moist dressing technique
- D. Leaving the wound open to air between changes

Correct Answer: C

Prescribed moist wound care helps support healing and reduces trauma to delicate granulation tissue. Scrubbing the wound bed and pulling dressings off quickly can damage new tissue. Leaving a wound open to air is generally not the best way to protect healing tissue unless specifically prescribed.

3753. A nurse assesses a pressure injury on a client's heel. The wound bed is completely covered with yellow-brown tissue, and the depth cannot be determined. How should the nurse document this wound?

- A. Stage 2 pressure injury
- B. Stage 3 pressure injury
- C. Unstageable pressure injury
- D. Deep tissue pressure injury

Correct Answer: C

A pressure injury is unstageable when the wound bed is obscured by slough or eschar and the true depth cannot be determined. Stage 2 and stage 3 require visible tissue features that allow staging. Deep tissue pressure injury is identified by persistent deep red, maroon, or purple discoloration rather than an obscured wound bed.

3754. A nurse is changing a sterile dressing on a client's forearm wound. After removing the old dressing, the nurse notes that the wound edges are separated and the staples are no longer holding the skin together. Which term best describes this finding?

- A. Evisceration
- B. Dehiscence
- C. Maceration
- D. Undermining

Correct Answer: B

Dehiscence is partial or complete separation of a wound's layers. Evisceration is protrusion of internal organs through a wound, which is more severe and generally applies to abdominal wounds. Maceration refers to soggy skin from excess moisture, and undermining describes tissue destruction under intact wound edges.

3755. A nurse is reviewing a provider's prescription for a warm, moist compress to a localized wound area. Which nursing action is most appropriate when applying the compress?

- A. Keep the compress in place until it cools completely and dries out
- B. Apply the compress directly from a high-heat source to maximize circulation
- C. Check the skin frequently and replace the compress as needed to maintain ordered warmth

D. Cover the compress tightly with plastic wrap to trap as much heat as possible

Correct Answer: C

Warm, moist compresses require ongoing skin assessment and replacement as needed to maintain the prescribed therapeutic warmth safely. Leaving the compress on until it dries can reduce effectiveness and irritate tissue. High heat increases burn risk, and tightly trapping excessive heat can injure the skin.

3756. A postoperative client has a large abdominal dressing. Which finding most strongly suggests formation of a hematoma at the incision site?

- A.** A narrow line of pink drainage on the outer dressing
- B.** A firm, swollen, bluish area beneath the incision
- C.** Mild itching around the tape edges
- D.** Scant serous drainage with approximated wound edges

Correct Answer: B

A hematoma is a collection of blood in tissue and commonly appears as a swollen, firm, discolored area near the wound. A small amount of pink drainage may be expected early after surgery. Mild tape irritation is not a hematoma, and scant serous drainage with well-approximated edges can be consistent with routine healing.

3757. A nurse is preparing to collect a wound culture from an open leg ulcer. Which action is best?

- A.** Collect drainage from the old dressing in a sterile container
- B.** Swab the wound after cleansing, using viable tissue in the wound bed
- C.** Obtain the specimen after applying the new topical medication
- D.** Swab the outer wound edges where dried drainage is visible

Correct Answer: B

The best specimen comes from the cleansed wound bed over viable tissue rather than from old drainage, dried exudate, or a dressing. This reduces contamination and better reflects organisms present in the wound. Applying medication first can alter the specimen.

3758. A nurse is assessing a wound on a client's hip. The wound extends into the subcutaneous tissue, but bone, tendon, and muscle are not visible. Which stage should the nurse identify?

- A.** Stage 1
- B.** Stage 2
- C.** Stage 3
- D.** Stage 4

Correct Answer: C

Stage 3 pressure injuries involve full-thickness skin loss with adipose or subcutaneous tissue visible, but without exposed bone, tendon, or muscle. Stage 1 is nonblanchable erythema of intact skin, stage 2 is partial-thickness skin loss, and stage 4 includes exposed or directly palpable deeper structures such as bone, tendon, or muscle.

3759. A nurse is caring for a client whose large open wound is healing by secondary intention. Which change in the wound should the nurse report as the clearest sign that healing is not progressing normally?

- A.** Bright red, bumpy tissue filling the wound base
- B.** Wound edges gradually pulling together over time
- C.** Increasing depth with a pocket developing under the wound edge
- D.** A smaller amount of drainage than on the previous dressing change

Correct Answer: C

Increasing depth and pocketing under the wound edge suggest worsening tissue destruction, such as undermining, and indicate poor healing progression. Bright red granulation tissue, contracting wound edges, and decreasing drainage are generally signs of improvement rather than deterioration.

3760. A nurse removes a postoperative dressing and finds the incision edges are intact, but a loop of bowel is visible through an opening between the sutures. Which action should the nurse take first?

- A.** Reinsert the bowel gently with sterile gloves
- B.** Apply a dry abdominal binder tightly over the site

C. Cover the area with sterile saline-moistened dressings

D. Position the client flat with legs extended

Correct Answer: C

Visible bowel through a wound indicates evisceration. The priority is to protect exposed organs by covering them with sterile saline-moistened dressings. The bowel should not be pushed back in. A tight dry covering can damage tissue, and the client is typically positioned to reduce strain on the wound rather than flat with legs extended.

CPR and emergency response

3761. A nurse sees an adult client suddenly collapse in the hallway. The client does not respond when spoken to and has no normal breathing. What should the nurse do first?

A. Begin chest compressions and have someone activate the emergency response system

B. Give the client water to stimulate swallowing

C. Place the client in high-Fowler position

D. Check the client's temperature

Correct Answer: A

An unresponsive adult with no normal breathing should be treated as being in cardiac arrest. The priority is to start CPR immediately and activate the emergency response system. Giving water, changing position, or checking temperature delays life-saving care.

3762. While performing CPR on an adult, the nurse should compress the chest to which depth?

A. About 1 cm (0.4 in)

B. About 2.5 cm (1 in)

C. At least 5 cm (2 in)

D. At least 8 cm (3.1 in)

Correct Answer: C

Effective adult chest compressions are delivered at a depth of at least 5 cm (2 in). Shallower compressions may not create enough circulation, while excessively deep compressions increase injury risk without improving quality.

3763. The nurse is teaching a new staff member about high-quality CPR for an adult. Which compression rate should the nurse include?

A. 40 to 60 compressions/min

B. 60 to 80 compressions/min

C. 100 to 120 compressions/min

D. 140 to 160 compressions/min

Correct Answer: C

High-quality adult CPR uses a compression rate of 100 to 120/min. Rates that are too slow do not provide enough perfusion, and rates that are too fast can reduce compression depth and chest recoil.

3764. A nurse is using a bag-valve-mask to ventilate an adult client during resuscitation. Which action best helps reduce the risk of gastric inflation?

A. Delivering each breath too quickly to force chest rise

B. Giving only enough air to produce visible chest rise

C. Skipping breaths and focusing only on compressions

D. Holding the mask loosely so excess air can escape

Correct Answer: B

Ventilations should be given only until visible chest rise occurs. Excessive force or speed can push air into the stomach, increasing aspiration risk. Skipping breaths when ventilations are indicated is incorrect, and a loose mask reduces effective ventilation.

3765. An adult client is found unresponsive in bed. After calling for help, the nurse checks for a pulse and breathing for no more than 10 seconds. The client has a pulse of 58/min but is not breathing. Which action is most appropriate?

A. Start chest compressions immediately

B. Provide rescue breaths and reassess the pulse periodically

C. Place the client in side-lying position and observe

D. Wait for the code team before intervening

Correct Answer: B

If an adult has a pulse but is not breathing, the nurse should provide rescue breaths and continue to reassess. Chest compressions are indicated when there is no pulse. Waiting or only positioning the client delays needed ventilation.

3766. During two-rescuer CPR for an adult client, one nurse is doing compressions and the other is managing the airway with a bag-valve-mask. Which action best supports high-quality CPR?

- A. Switching compressors about every 2 minutes
- B. Stopping compressions for 20 seconds after every cycle
- C. Allowing the chest to remain partially compressed between pushes
- D. Pausing often to recheck blood pressure

Correct Answer: A

Switching compressors about every 2 minutes helps prevent fatigue and maintains compression quality. Frequent long pauses, incomplete recoil, and unnecessary blood pressure checks interfere with effective resuscitation.

3767. A client suddenly becomes cyanotic while eating lunch and cannot speak or cough. What is the nurse's priority interpretation of these findings?

- A. Mild aspiration that should clear on its own
- B. Complete airway obstruction requiring immediate intervention
- C. Panic attack with hyperventilation
- D. Expected coughing related to dry food

Correct Answer: B

Inability to speak or cough with cyanosis strongly suggests severe airway obstruction. This is a life-threatening emergency requiring immediate action. The main trap is minimizing the event as mild aspiration or anxiety when the client cannot move air effectively.

3768. The nurse arrives first to a client who collapsed in the bathroom. The floor is wet, and an electrical cord from a hair dryer is lying near the client. Which action should the nurse take first?

- A. Move the client to the bed immediately

- B. Begin chest compressions where the client lies
- C. Ensure the scene is safe before touching the client
- D. Open the client's mouth to check for dentures

Correct Answer: C

The nurse must first ensure scene safety. Water and an electrical appliance create a hazard for both rescuer and client. Starting compressions or moving the client before making the area safe could cause additional injury.

3769. An adult client becomes pulseless on a medical unit, and CPR is started. An automated external defibrillator (AED) arrives. Which action is appropriate when using the AED?

- A. Keep compressing the chest while the AED analyzes the rhythm
- B. Make sure no one is touching the client during rhythm analysis and shock delivery
- C. Remove the pads between each cycle of CPR
- D. Delay pad placement until after 10 minutes of CPR

Correct Answer: B

No one should touch the client during AED rhythm analysis or shock delivery. Contact can interfere with rhythm interpretation and create a safety risk. CPR should resume promptly when advised, but not during analysis or shock delivery.

3770. A nurse is performing chest compressions on an adult client. Which observation indicates a need to improve compression technique?

- A. The nurse allows the chest to fully recoil after each compression
- B. The nurse keeps interruptions in compressions brief
- C. The nurse bends the elbows and rocks from the shoulders with shallow downward force
- D. The nurse places the heel of one hand on the center of the chest

Correct Answer: C

Effective compressions require adequate depth using correct body mechanics, typically with straight arms and force directed vertically. Bent elbows and shallow force can reduce compression quality. The other options describe proper technique.

3771. A client with a tracheostomy becomes unresponsive and is not breathing. What is the best way for the nurse to provide rescue breaths?

- A. Seal the mouth only and give breaths
- B. Seal both the mouth and nose and give breaths
- C. Ventilate through the tracheostomy opening
- D. Do not ventilate until the tracheostomy tube is removed

Correct Answer: C

Air exchange occurs through the tracheostomy, so rescue breaths should be delivered through the tracheostomy opening or tube. Ventilating only through the mouth or nose may be ineffective, especially with a mature tracheostomy airway.

3772. During adult CPR, the nurse notes that the chest does not rise when breaths are delivered with a bag-valve-mask. Which action should the nurse take first?

- A. Increase the oxygen flow rate as high as possible
- B. Reposition the head and attempt to obtain a better airway seal
- C. Stop all compressions and suction the mouth for 1 minute
- D. Give several rapid forceful breaths

Correct Answer: B

If the chest is not rising, the first step is to improve airway positioning and mask seal. Poor head position or mask fit commonly prevents effective ventilation. Forceful breaths can worsen gastric inflation, and stopping compressions for a prolonged period is not appropriate.

3773. A nurse witnesses an adult client collapse and confirms the client is unresponsive and pulseless. Another staff member is calling the code team and bringing the AED. Which nursing action is the best immediate intervention?

- A. Wait for the AED so the rhythm can be checked before CPR begins
- B. Deliver two breaths before starting compressions
- C. Start chest compressions with minimal delay
- D. Raise the client's legs above heart level

Correct Answer: C

For a witnessed pulseless collapse, chest compressions should begin immediately while additional help and the AED are on the way. Waiting for the AED or delaying for other steps lowers perfusion. The trap is overvaluing rhythm analysis before circulation support starts.

3774. A nurse is alone with an adult client who is choking. The client becomes unresponsive while the nurse is attempting to help. What should the nurse do next?

- A. Blindly sweep the mouth with a finger
- B. Lower the client to the floor, activate emergency response, and begin CPR
- C. Place the client in Trendelenburg position
- D. Give repeated back blows while the client lies supine

Correct Answer: B

When an adult with choking becomes unresponsive, the nurse should lower the client safely, call for emergency help, and begin CPR. Blind finger sweeps can push the object deeper. Trendelenburg and back blows in the supine position are not appropriate here.

3775. A nurse is reviewing the event after a resuscitation attempt. Which action most likely decreased the quality of CPR provided?

- A. Interruptions in compressions were kept as short as possible
- B. The team paused compressions frequently for pulse and rhythm checks that were not time-limited
- C. Rescuers changed roles to reduce fatigue
- D. The chest was allowed to recoil fully between compressions

Correct Answer: B

Frequent, prolonged interruptions reduce coronary and cerebral perfusion and decrease CPR effectiveness. Brief interruptions, switching compressors, and allowing full recoil all support high-quality CPR. The common trap is assuming repeated reassessment helps more than continuous compressions.

3776. A nurse begins one-rescuer CPR on an adult client. After 2 minutes, a second nurse arrives and takes over ventilations with a bag-valve-mask. Which compression-to-ventilation pattern should the team use?

- A. 15 compressions to 2 breaths
- B. 30 compressions to 2 breaths
- C. 5 compressions to 1 breath
- D. Continuous compressions with 1 breath every 3 seconds

Correct Answer: B

For adult CPR, the compression-to-ventilation ratio remains 30:2 whether there is one rescuer or two rescuers, unless an advanced airway changes the ventilation approach. Choice A is a common trap because 15:2 is associated with some pediatric two-rescuer situations, not routine adult CPR.

3777. A nurse finds an adult client unresponsive and pulseless. CPR is in progress, and the crash cart has arrived. Which action should the nurse prioritize to improve the client's chance of survival?

- A. Prepare to give the first scheduled antiarrhythmic medication
- B. Pause compressions to obtain a full set of vital signs
- C. Attach the defibrillator as soon as possible for rhythm analysis
- D. Wait for the provider to confirm cardiac arrest before continuing

Correct Answer: C

Early defibrillation is a priority in pulseless arrest when a shockable rhythm may be present. Rapid rhythm analysis and defibrillation, along with high-quality CPR, improve outcomes. Choice A may become relevant later but should not delay rhythm assessment. Choice B causes an unnecessary interruption, and choice D delays lifesaving care.

3778. A nurse is caring for a client who suddenly grabs the throat, cannot speak, and has poor air movement after taking a bite of food. What is the nurse's best immediate action while the client remains responsive?

- A. Give abdominal thrusts
- B. Begin chest compressions
- C. Offer sips of water
- D. Sweep the mouth with a finger

Correct Answer: A

A responsive adult with signs of severe airway obstruction needs immediate abdominal thrusts. Chest compressions are used after the client becomes unresponsive. Water will not relieve a severe obstruction, and blind finger sweeps can push the object deeper or injure tissue.

3779. A nurse is documenting a resuscitation event. Which finding best indicates that chest compressions were effective during CPR?

- A. Chest compressions were interrupted every 20 seconds for pulse checks
- B. The chest recoiled fully after each compression
- C. The client received large, rapid breaths after each compression cycle
- D. The compressor leaned on the sternum between compressions to maintain hand position

Correct Answer: B

Full chest recoil is a key feature of high-quality CPR because it allows the heart to refill between compressions. Frequent interruptions reduce perfusion. Large, rapid breaths can cause excessive ventilation, and leaning on the chest prevents complete recoil.

3780. While the code team is resuscitating an adult client, the monitor shows pulseless electrical activity. What is the nurse's best understanding of this rhythm?

- A. It is a shockable rhythm that requires immediate defibrillation
- B. It indicates organized electrical activity without effective cardiac output
- C. It means the client has a pulse too weak for manual palpation and CPR should stop
- D. It is usually caused by equipment artifact and should be ignored until the next rhythm check

Correct Answer: B

Pulseless electrical activity means there is electrical activity on the monitor but no effective mechanical contraction producing a pulse. CPR should continue while reversible causes are considered and treated. Choice A is incorrect because PEA is not a shockable rhythm. Choice C is a dangerous trap that confuses monitor activity with perfusion.

3781. A nurse is participating in resuscitation of an adult client with a peripheral IV. The provider prescribes epinephrine IV. Which nursing action is most appropriate after giving the medication?

- A. Clamp the IV line for 5 minutes
- B. Follow the medication with an IV fluid flush
- C. Wait until the next rhythm check before pushing the medication
- D. Hold chest compressions briefly to improve medication circulation

Correct Answer: B

After an IV medication is given during resuscitation, a fluid flush helps move the drug into central circulation. Chest compressions should continue because they help circulate the medication; they should not be paused for this purpose. Clamping the line or delaying the push reduces effectiveness.

3782. An adult client is receiving CPR. An advanced airway is now in place. Which change in technique should the nurse expect?

- A. Stop compressions for each pair of breaths
- B. Provide 2 breaths after every 15 compressions
- C. Continue continuous compressions while giving 1 breath every 6 seconds
- D. Slow compressions to coordinate each breath with chest recoil

Correct Answer: C

Once an advanced airway is in place during adult CPR, compressions should continue continuously, and ventilations are given asynchronously at about 1 breath every 6 seconds. Choices A and B incorrectly interrupt or alter the compression pattern. Choice D is not part of high-quality CPR.

3783. A nurse responds to a monitor alarm and finds an adult client slumped in a chair. The client is unresponsive, and the nurse cannot feel a carotid pulse within 10 seconds. The client has a valid do-not-resuscitate order in the chart. What should the nurse do?

- A. Begin CPR until the provider arrives to verify the order
- B. Start rescue breathing only
- C. Withhold resuscitation and follow facility protocol for expected actions
- D. Apply the AED first in case the rhythm is shockable

Correct Answer: C

A valid do-not-resuscitate order means CPR and other resuscitative measures should be withheld. The nurse should follow facility policy, which may include notifying the provider and family per protocol and providing appropriate supportive care. Choices A, B, and D all initiate resuscitative interventions that conflict with the DNR order.

3784. During CPR, the team lead asks the nurse to consider reversible causes of the client's pulseless arrest. Which finding most strongly suggests hypovolemia as a possible cause?

- A. Distended neck veins and unequal breath sounds
- B. History of major gastrointestinal bleeding and cool, pale skin before collapse
- C. Missed dialysis with peaked T waves before arrest
- D. Sudden pleuritic chest pain after prolonged immobility

Correct Answer: B

A history of major blood or fluid loss points toward hypovolemia as a reversible cause of pulseless arrest. Choice A suggests tension pneumothorax. Choice C suggests hyperkalemia. Choice D suggests pulmonary embolism. The question tests recognition of likely causes based on cues during a code.

3785. A nurse is rotating into the compressor role during an adult code. The previous compressor reports feeling fatigued. Why should compressors ideally switch roles about every 2 minutes?

- A. To allow time for ventilation equipment checks
- B. To reduce interruptions for pulse checks
- C. To maintain compression quality as fatigue develops quickly
- D. To ensure each rescuer performs the same number of compressions

Correct Answer: C

Compression quality declines as rescuers fatigue, sometimes sooner than they realize. Rotating about every 2 minutes helps maintain adequate depth and rate while minimizing interruptions. Choice B sounds plausible, but switching roles is done primarily to preserve compression quality, not to reduce pulse checks.

3786. A nurse finds an adult client collapsed in the courtyard during a thunderstorm. The client is unresponsive and may be pulseless. Which action is the nurse's priority before starting CPR?

- A. Move the client to a safer location if this can be done quickly
- B. Begin chest compressions immediately where the client is lying
- C. Check the client's pupils for fixed dilation
- D. Place the client in the recovery position

Correct Answer: A

Scene safety comes first. If there is an active environmental danger such as lightning risk, the nurse should move the client to a safer location if possible before beginning resuscitation. Starting CPR in an unsafe area can endanger the rescuer and delay effective care. Pupils and recovery position are not priorities in a potentially pulseless client.

3787. A nurse is assisting with CPR when capnography is attached to an intubated adult client. The end-tidal carbon dioxide suddenly rises from 9 mm Hg to 35 mm Hg during ongoing compressions. Which interpretation is most appropriate?

- A. The endotracheal tube has probably become dislodged

- B. The client may have achieved return of spontaneous circulation
- C. Chest compressions are too shallow and should be slowed down
- D. The client has developed a tension pneumothorax

Correct Answer: B

A sudden sustained increase in end-tidal carbon dioxide during CPR can indicate return of spontaneous circulation because pulmonary blood flow has improved abruptly. Choice A would be more likely with loss of detectable carbon dioxide, not a sudden rise. Choice C misreads the significance of the change.

Respiratory interventions

3788. A client with chronic obstructive pulmonary disease (COPD) becomes increasingly short of breath. Which oxygen delivery method is most appropriate if the nurse needs to provide a low, controlled concentration of oxygen?

- A. Nonrebreather mask
- B. Venturi mask
- C. Simple face mask
- D. Face tent

Correct Answer: B

A Venturi mask delivers a precise, controlled oxygen concentration, which is especially useful for clients with COPD who may be sensitive to higher oxygen levels. A nonrebreather provides high-concentration oxygen, and a simple mask is less precise. A face tent is used when a mask cannot be tolerated but does not provide the same level of precise oxygen control.

3789. A nurse is caring for a client with a tracheostomy who has noisy respirations, visible secretions at the opening, and an oxygen saturation that has dropped from 96% to 90%. What is the nurse's priority action?

- A. Suction the tracheostomy
- B. Change the tracheostomy ties
- C. Clean the stoma with saline
- D. Obtain a sputum specimen

Correct Answer: A

The client is showing signs of airway obstruction from secretions, so suctioning is the priority to restore airway patency. Changing ties and cleaning the stoma are important parts of care but do not address the immediate breathing problem. A sputum specimen can be collected later if needed.

3790. The nurse is teaching a client how to use an incentive spirometer after abdominal surgery. Which instruction should the nurse include?

- A. Blow out quickly into the mouthpiece
- B. Use the device only when shortness of breath occurs
- C. Inhale slowly and deeply through the mouthpiece
- D. Lie flat while using the device for best expansion

Correct Answer: C

An incentive spirometer is used by inhaling slowly and deeply to promote alveolar expansion and help prevent postoperative atelectasis. Blowing into the device is incorrect. Waiting until dyspnea occurs defeats the preventive purpose, and lying flat reduces lung expansion.

3791. A client with pneumonia has thick respiratory secretions. Which intervention best helps the client mobilize secretions?

- A. Restrict oral fluids
- B. Maintain adequate hydration
- C. Limit coughing to reduce fatigue
- D. Keep the client on strict bed rest

Correct Answer: B

Adequate hydration helps thin secretions, making them easier to cough up. Restricting fluids can worsen secretion thickness. Limiting cough and strict bed rest can impair airway clearance rather than improve it.

3792. A nurse enters a room and finds a client with a chest tube drainage system disconnected from the tubing. What should the nurse do first?

- A. Clamp the chest tube near the insertion site
- B. Place the end of the chest tube in sterile water
- C. Remove the dressing around the insertion site

D. Milk the chest tube to remove air

Correct Answer: B

If a chest tube becomes disconnected from the drainage system, the priority is to reestablish a water seal by placing the tube end in sterile water to reduce air entry into the pleural space. Routine clamping can increase the risk of tension pneumothorax. Removing the dressing does not fix the problem, and milking the tube is not the first action.

3793. A client receiving oxygen by nasal cannula reports dry, irritated nostrils. Which action is most appropriate for the nurse to take?

- A. Apply petroleum jelly inside the nares
- B. Increase the oxygen flow rate
- C. Use a water-soluble lubricant as prescribed
- D. Discontinue oxygen for 30 minutes every 4 hours

Correct Answer: C

A water-soluble lubricant can help reduce nasal dryness safely. Petroleum-based products should be avoided with oxygen because of fire risk. Increasing oxygen may worsen dryness, and stopping oxygen without a clinical reason can compromise oxygenation.

3794. A postoperative client is breathing rapidly and shallowly and has diminished breath sounds at the lung bases. Which nursing intervention is most appropriate to help prevent atelectasis?

- A. Encourage coughing and deep breathing
- B. Limit activity until breath sounds improve
- C. Provide frequent oral suctioning
- D. Place the client in Trendelenburg position

Correct Answer: A

Coughing and deep breathing promote lung expansion and help reopen alveoli, making them appropriate for preventing or improving atelectasis. Limiting activity may worsen the problem. Oral suctioning does not treat shallow ventilation, and Trendelenburg can impair breathing.

3795. A client with a tracheostomy requires suctioning. Which technique should the nurse use while inserting the suction catheter?

- A. Apply suction continuously during insertion

- B. Insert the catheter without applying suction
- C. Rotate the catheter rapidly during insertion
- D. Instill normal saline before every suction pass

Correct Answer: B

The suction catheter should be inserted without suction to reduce mucosal trauma and oxygen loss. Suction is applied while withdrawing the catheter. Continuous suction during insertion can injure tissue. Rotating on insertion is unnecessary, and routine saline instillation is not indicated as an every-pass step.

3796. A client with emphysema is dyspneic at rest. In which position should the nurse place the client to promote easier breathing?

- A. Supine with legs elevated
- B. Side-lying with head flat
- C. High Fowler's position
- D. Trendelenburg position

Correct Answer: C

High Fowler's position promotes maximum chest expansion and can decrease the work of breathing. Supine, flat side-lying, and Trendelenburg positions can further restrict lung expansion and worsen dyspnea.

3797. The nurse is assessing a chest tube drainage system. Which finding requires immediate follow-up?

- A. Gentle bubbling in the suction control chamber
- B. Tidaling in the water-seal chamber with respirations
- C. A new continuous stream of bubbling in the water-seal chamber
- D. Drainage collected in the system below chest level

Correct Answer: C

Continuous bubbling in the water-seal chamber suggests an air leak and requires prompt assessment. Tidaling can be expected, depending on the client's condition. Bubbling in the suction control chamber may be expected if suction is being used. Keeping the system below chest level is appropriate.

3798. A client with chronic lung disease is being taught pursed-lip breathing. Which statement by the client shows correct understanding?

- A. I will inhale through pursed lips and exhale through my nose.
- B. I will take quick breaths to move more air in and out.
- C. I will exhale longer than I inhale through partially closed lips.
- D. I will use this technique only during sleep.

Correct Answer: C

Pursed-lip breathing involves inhaling through the nose and exhaling slowly through partially closed lips, with exhalation longer than inhalation. This helps prevent airway collapse and improves ventilation. The other statements describe incorrect technique or inappropriate use.

3799. A client suddenly becomes short of breath after a chest tube is removed. The nurse notes tracheal deviation, severe respiratory distress, and absent breath sounds on one side. Which action is the priority?

- A. Apply oxygen and notify the provider immediately
- B. Reinforce coughing and deep breathing exercises
- C. Place the client in a flat position
- D. Prepare the client for use of an incentive spirometer

Correct Answer: A

These findings suggest a tension pneumothorax, a life-threatening emergency. The nurse should support oxygenation and obtain immediate medical assistance. Coughing, lying flat, and incentive spirometry do not address the acute hemodynamic and respiratory compromise.

3800. A nurse is caring for a client with a tracheostomy who suddenly coughs out the outer tracheostomy tube. The client is in visible distress. What should the nurse do first?

- A. Call the provider before touching the tracheostomy site
- B. Attempt to insert the obturator and replace the tube per facility protocol
- C. Cover the stoma with an occlusive dressing

D. Turn the client to the left side and wait for respiratory therapy

Correct Answer: B

Accidental decannulation can rapidly compromise the airway. The priority is to reestablish the airway by replacing the tube according to training and facility protocol, often using the obturator. Waiting for the provider or respiratory therapy delays treatment. Covering the stoma would further obstruct airflow.

3801. A client with a chest tube for pneumothorax has no tidaling in the water-seal chamber. Which assessment finding would make this most concerning?

- A.** The client is breathing comfortably and lung sounds are improved
- B.** The tubing is dependent with a visible kink near the bed rail
- C.** The drainage system remains upright below the chest
- D.** There is a small amount of old drainage in the collection chamber

Correct Answer: B

Absent tidaling can occur when the lung has re-expanded, but it is concerning when caused by an obstruction such as a kink in the tubing. A comfortable client with improved breath sounds suggests recovery. An upright drainage system below chest level is appropriate, and old drainage volume does not explain loss of tidaling.

3802. A nurse is teaching a family member how to assist a client with postural drainage at home. Which statement by the family member indicates a need for further teaching?

- A.** I should schedule this before meals when possible.
- B.** I will stop the session if the client becomes severely short of breath.
- C.** I should encourage the client to cough after drainage.
- D.** I will do the treatment right after a large meal to prevent weakness.

Correct Answer: D

Postural drainage is generally better tolerated before meals or several hours after eating because doing it right after a large meal can increase nausea or vomiting risk. Stopping for severe dyspnea and encouraging coughing afterward are appropriate. The incorrect timing is the teaching point that needs correction.

3803. A client with bacterial pneumonia has coarse crackles and a weak, ineffective cough. Which nursing action is most likely to improve the client's ability to clear airway secretions?

- A.** Limit oral fluids to reduce pulmonary congestion
- B.** Encourage coughing after deep breathing exercises
- C.** Place the client flat in bed for rest periods
- D.** Instruct the client to avoid talking to conserve oxygen

Correct Answer: B

Deep breathing helps expand the lungs, and coughing afterward helps move and expel secretions. Limiting fluids can make secretions thicker, lying flat can worsen ventilation and secretion clearance, and avoiding talking does not directly improve airway clearance.

3804. A nurse is caring for a client who is receiving oxygen at 2 L/min by nasal cannula. On assessment, the nurse notes an oxygen saturation of 88%, respirations of 28/min, and use of accessory muscles. Which action should the nurse take first?

- A.** Increase the oxygen flow rate per protocol or prescription
- B.** Notify the health care provider that the client is hypoxic
- C.** Document the findings and reassess in 30 minutes
- D.** Teach the client relaxation techniques

Correct Answer: A

The client is showing signs of inadequate oxygenation, so the nurse should first improve oxygen delivery if allowed by protocol or prescription. Notifying the provider may be needed next, but immediate action to support oxygenation comes first. Waiting or focusing on teaching delays treatment.

3805. A client with chronic bronchitis is learning ways to breathe more effectively during episodes of dyspnea. Which instruction should the nurse give?

- A. Inhale quickly through the mouth and exhale quickly through the nose
- B. Hold the breath as long as possible before exhaling
- C. Exhale slowly through partially closed lips
- D. Take several short breaths in a row without exhaling fully

Correct Answer: C

Exhaling slowly through partially closed lips helps keep airways open longer and improves air movement, especially in clients with obstructive lung disease. Quick breathing, prolonged breath-holding, and repeated short breaths can worsen air trapping and dyspnea.

3806. A client with acute shortness of breath is most comfortable sitting upright and leaning forward over the bedside table. How should the nurse interpret this finding?

- A. The position helps maximize chest expansion and ease breathing
- B. The client is showing signs of decreased level of consciousness
- C. The position indicates worsening abdominal pain
- D. The client is attempting to prevent aspiration while swallowing

Correct Answer: A

Leaning forward in an upright position, often called a tripod position, can improve chest expansion and reduce the work of breathing. The finding is most consistent with respiratory distress rather than decreased consciousness, abdominal pain, or swallowing difficulty.

3807. A nurse is caring for a client receiving humidified oxygen by face mask. Which finding requires the most immediate nursing action?

- A. Condensation collecting in the tubing
- B. The client states the mask feels warm
- C. The prescribed flow rate is set below the level needed to flush exhaled carbon dioxide from the mask
- D. The elastic strap leaves a mild indentation on the client's skin

Correct Answer: C

A face mask must have enough flow to clear exhaled carbon dioxide from the mask. If the flow is too low, the client may rebreathe carbon dioxide, which is an immediate safety issue. Condensation and mild skin pressure need attention but are not as urgent. Warmth alone is not the priority concern.

3808. A postoperative client is reluctant to cough because of incisional pain. Which nursing action is best to improve coughing effectiveness while reducing discomfort?

- A. Have the client splint the incision with a pillow when coughing
- B. Tell the client to avoid coughing until the incision is less tender
- C. Place the client in a flat supine position before coughing
- D. Ask the client to take rapid shallow breaths instead of coughing

Correct Answer: A

Splinting the incision with a pillow supports the surgical area, decreases discomfort, and helps the client cough more effectively to clear secretions. Avoiding cough increases the risk of retained secretions and atelectasis. Lying flat and taking shallow breaths do not promote secretion clearance.

3809. A nurse is assisting with chest physiotherapy for a client with chronic bronchiectasis. Which action is appropriate during percussion therapy?

- A. Perform percussion immediately after meals to stimulate digestion
- B. Percuss over the spine and kidneys to loosen deeper secretions
- C. Cup the hands and rhythmically clap over targeted lung areas
- D. Use the flat palm firmly against bare skin until the area becomes red

Correct Answer: C

Percussion is performed by cupping the hands and rhythmically clapping over appropriate lung segments to help loosen secretions. It should not be done immediately after meals because that can increase nausea or aspiration risk. The spine and kidneys are avoided, and striking with a flat palm against bare skin is not correct technique.

3810. A client with severe asthma is receiving oxygen and prescribed bronchodilator therapy. Which assessment finding suggests the client's respiratory status is worsening rather than improving?

- A. The client reports feeling less chest tightness
- B. Wheezing becomes minimal and breath sounds are markedly diminished
- C. Respiratory rate decreases from 30/min to 22/min
- D. Oxygen saturation rises from 90% to 94%

Correct Answer: B

In severe asthma, very diminished breath sounds with little wheezing can indicate minimal air movement, a dangerous sign of worsening obstruction. The other findings suggest improvement. A common trap is assuming less wheezing always means improvement; in a severely obstructed client, it may mean less air is moving at all.

3811. A nurse is caring for a client with a new prescription for incentive spirometry every hour while awake. The client says, "I used it once this morning, so I don't need it again until tonight." Which response by the nurse is best?

- A. "You only need it when you feel short of breath."
- B. "It works best when used regularly throughout the day while you are awake."
- C. "You should stop using it if it makes you take deep breaths."
- D. "You can replace it by breathing quickly several times each hour."

Correct Answer: B

Incentive spirometry is intended for regular repeated use while awake to promote lung expansion and help prevent complications such as atelectasis. It is not reserved only for dyspnea, and rapid shallow breathing is not an effective substitute. Deep breathing is the point of the intervention, not a reason to stop it.

3812. A client with chronic respiratory disease is receiving 35% oxygen by Venturi mask. During rounds, the nurse finds that the mask has been replaced with a nonrebreather mask by an unlicensed assistive person because the client 'looked short of breath.' Which action by the nurse is the priority?

- A. Leave the nonrebreather mask in place because it delivers more oxygen
- B. Return the client to the prescribed oxygen device and assess respiratory status
- C. Wait to see whether the client's oxygen saturation improves
- D. Document the equipment change and notify respiratory therapy later

Correct Answer: B

The priority is to restore the prescribed oxygen delivery system and assess the client. A Venturi mask is used when a controlled oxygen concentration is needed. Leaving a different high-concentration device in place without evaluation can be unsafe. Waiting or delaying action risks further problems.

3813. A nurse is planning care for a client with thick secretions and decreased mobility. Which intervention best helps reduce the risk of secretion retention over the course of the shift?

- A. Reposition the client regularly and encourage activity as tolerated
- B. Keep the client on strict bed rest to decrease oxygen demand
- C. Limit oral intake to prevent coughing episodes
- D. Schedule all care so the client remains undisturbed for 8 hours

Correct Answer: A

Regular repositioning and activity as tolerated promote lung expansion and mobilization of secretions. Bed rest and prolonged immobility increase the risk of retained secretions. Limiting fluids can thicken secretions, and leaving the client undisturbed for long periods can worsen pulmonary complications.

Tracheostomy care

3814. A nurse is caring for a client with a new tracheostomy. Which finding requires immediate action?

- A. Small amount of pink-tinged mucus at the stoma
- B. Crackling sensation felt around the tracheostomy site
- C. Reports of mild incision pain when coughing
- D. Thin secretions after humidified oxygen is started

Correct Answer: B

A crackling sensation around the tracheostomy site suggests subcutaneous emphysema, which can indicate air leaking into the tissues and possible tracheostomy complication. This requires prompt assessment and intervention. A small amount of pink-tinged drainage, mild pain, and thinner secretions after humidification are more expected early findings.

3815. A client with a tracheostomy suddenly becomes restless and has noisy respirations. The nurse notes visible secretions at the opening. What is the nurse's priority action?

- A. Suction the tracheostomy using sterile technique
- B. Remove the tracheostomy ties and replace them
- C. Apply a dry dressing around the stoma
- D. Position the client flat in bed

Correct Answer: A

Restlessness, noisy respirations, and visible secretions suggest airway obstruction from mucus. The priority is to suction the tracheostomy to clear the airway. Replacing ties and changing dressings do not address the immediate breathing problem. Positioning the client flat could worsen ventilation.

3816. Which action should the nurse take when performing tracheostomy suctioning for a client with thick secretions?

- A. Apply suction while inserting the catheter
- B. Limit each suction pass to a brief duration
- C. Use clean technique because the airway is already colonized
- D. Suction on a fixed schedule every hour

Correct Answer: B

Each suction pass should be brief to reduce hypoxia and mucosal trauma. Suction is not applied during insertion because that can injure tissue and remove oxygen. Sterile technique is used for tracheostomy suctioning. Suctioning should be based on assessment findings, not performed routinely on a strict hourly schedule.

3817. A nurse is teaching a client with a chronic tracheostomy about secretion management at home. Which statement by the client shows correct understanding?

- A. I should keep the air in my home as dry as possible.
- B. I will limit fluids so I do not cough as much.
- C. Humidification can help keep my secretions from getting too thick.
- D. If I have mucus, I should remove the tube ties to breathe better.

Correct Answer: C

Humidification helps prevent secretions from becoming thick and difficult to clear because the upper airway is bypassed with a tracheostomy. Dry air and reduced fluid intake can worsen secretion thickness. Removing tube ties is unsafe and can lead to dislodgement.

3818. During routine tracheostomy care, the nurse finds that the tracheostomy ties are soiled. Which action is most appropriate when changing the ties?

- A. Remove the old ties before placing the new ties
- B. Ask the client to hold the tracheostomy tube in place alone
- C. Secure the new ties before the old ties are completely removed
- D. Loosen the ties enough to allow four fingers underneath

Correct Answer: C

The tracheostomy tube must be kept secure during tie changes to prevent accidental decannulation, so the new ties are secured before the old ties are fully removed. Removing old ties first risks tube displacement. The client should not be relied on as the sole stabilizer. Ties should be snug enough to secure the tube, typically allowing only a small amount of space, not four fingers.

3819. A client with a tracheostomy is receiving humidified oxygen. Which assessment finding best indicates the intervention is effective?

- A. Secretions are thinner and easier to cough out
- B. The stoma appears slightly red at the edges
- C. Respiratory rate decreases from 20 to 18/min
- D. The client requests less oral care

Correct Answer: A

Humidification is used to keep secretions moist and easier to mobilize because the tracheostomy bypasses normal upper-airway humidification. Thinner, easier-to-clear secretions best show effectiveness. Slight stoma redness may reflect irritation, not humidification success. A small change in respiratory rate is nonspecific, and less oral care is not an expected outcome measure.

3820. The nurse is preparing to suction a tracheostomy. Which assessment indicates suctioning is needed?

- A. Clear breath sounds and oxygen saturation at baseline
- B. Frequent swallowing and a dry mouth
- C. Coarse crackles and ineffective coughing
- D. Pulse oximetry reading of 98% while resting comfortably

Correct Answer: C

Coarse crackles and ineffective coughing suggest retained airway secretions, making suctioning appropriate. Clear breath sounds, a stable oxygen saturation, and comfort do not indicate a need for suctioning. Frequent swallowing and dry mouth may reflect oral discomfort rather than tracheal secretion retention.

3821. A nurse is caring for a client 12 hours after tracheostomy placement. Which item should be kept at the bedside as most essential for safety?

- A. A manual resuscitation bag with oxygen source available
- B. A blood pressure cuff for the affected arm only
- C. A urinary catheter insertion kit
- D. A warming blanket

Correct Answer: A

A client with a tracheostomy is at risk for airway emergencies, so equipment to support ventilation, including a manual resuscitation bag with oxygen available, is essential at the bedside. The other items may be useful in other situations but are not the most critical safety equipment for tracheostomy care.

3822. The nurse is caring for a client whose tracheostomy cuff pressure is suspected to be too high. Which problem is the nurse most concerned about?

- A. Tracheal tissue damage from impaired blood flow
- B. Improved speech that tires the client
- C. Increased oral intake leading to aspiration
- D. Excessive humidification of inspired air

Correct Answer: A

Excessive cuff pressure can compress tracheal capillaries, decreasing blood flow and causing tracheal mucosal injury. The other options are not the main complication of an overinflated cuff. This question tests the nurse's ability to connect the intervention with the underlying physiologic risk.

3823. A client with a tracheostomy begins coughing forcefully, and the tracheostomy tube comes out. The client is breathing with difficulty. What should the nurse do first?

- A. Cover the stoma with an occlusive dressing
- B. Call the provider after documenting the event
- C. Maintain the airway and seek immediate assistance
- D. Place the client in prone position

Correct Answer: C

Accidental decannulation is an airway emergency. The priority is to maintain the airway and get immediate help so oxygenation and tube replacement can be addressed quickly according to facility protocol and the client's condition. Covering the stoma may obstruct airflow in some situations, and documentation or provider notification comes after emergency action. Prone positioning is inappropriate.

3824. A nurse is suctioning a client's tracheostomy. During the procedure, the client develops bradycardia. Which action is most appropriate?

- A. Continue suctioning until all secretions are removed
- B. Stop suctioning and oxygenate the client
- C. Lower the head of the bed
- D. Irrigate the airway with sterile water

Correct Answer: B

Bradycardia during suctioning may occur from hypoxia or vagal stimulation. The nurse should stop suctioning and oxygenate the client immediately. Continuing suctioning increases risk. Lowering the head of the bed does not correct the cause. Routine airway irrigation is not the priority and may worsen the situation.

3825. A nurse is caring for a client with a cuffed tracheostomy tube who suddenly has decreased oxygen saturation and a large amount of oral secretions. Which finding most strongly suggests cuff-related leakage?

- A. The client reports needing mouth care more often
- B. Air is heard escaping around the tracheostomy during ventilation
- C. The tracheostomy ties leave an imprint on the neck
- D. Humidification tubing contains condensation

Correct Answer: B

Audible air escaping around the tracheostomy during ventilation strongly suggests that the cuff is not sealing effectively. This can lead to decreased ventilation and pooling of secretions above the cuff. The other findings are less specific and do not directly indicate cuff leakage.

3826. A nurse is caring for a client with a tracheostomy who becomes anxious and asks why speaking is difficult. Which response by the nurse is best?

- A. You should not try to speak until the tracheostomy is removed.
- B. Air is not moving through your vocal cords the usual way, so speaking may be limited.
- C. Your voice loss means the tube is probably blocked.

D. Speaking is unsafe for anyone with a tracheostomy.

Correct Answer: B

A tracheostomy changes normal airflow through the upper airway and vocal cords, which can make speech difficult. This explanation is accurate and reassuring. The other responses are overly absolute, misleading, or alarmist. Difficulty speaking does not automatically mean the tube is blocked.

3827. Which action is appropriate when providing stoma care for a client with a tracheostomy?

- A. Use sterile technique to clean around the stoma
- B. Clean vigorously to remove all crusting at once
- C. Leave moist secretions in place to avoid bleeding
- D. Avoid assessing the skin under the flange

Correct Answer: A

Sterile technique is used to reduce infection risk when cleaning the tracheostomy site. Cleaning should be gentle, not vigorous, because aggressive rubbing can damage tissue. Moist secretions should not be left in place, and the skin under the flange should be assessed for irritation or breakdown.

3828. A client with a tracheostomy has increasing difficulty clearing secretions despite repeated suctioning. Which additional nursing intervention is most appropriate to help mobilize secretions?

- A. Encourage adequate hydration if not contraindicated
- B. Reduce activity and keep the client on strict bed rest
- C. Limit humidification to prevent water buildup
- D. Tighten the tracheostomy ties to reduce coughing

Correct Answer: A

Adequate hydration helps thin secretions and supports airway clearance when not contraindicated. Bed rest does not directly improve secretion mobilization. Reducing humidification would tend to thicken secretions, and tightening ties does not treat the problem and may cause harm.

3829. A nurse is caring for a client 2 days after tracheostomy placement. Which observation is the clearest sign that the client is developing subcutaneous emphysema?

- A. A crackling sensation is felt around the neck when the skin is palpated
- B. The client reports mild soreness at the incision site
- C. Small amounts of dried blood are noted on the dressing
- D. Breath sounds are slightly diminished at the lung bases

Correct Answer: A

Subcutaneous emphysema is identified by crepitus, a crackling feeling under the skin caused by air leaking into tissue. Mild soreness and small amounts of dried blood can occur after a recent procedure. Diminished breath sounds may suggest another respiratory problem, but they do not specifically point to subcutaneous emphysema.

3830. A nurse is planning care for a client with a tracheostomy who has thick, tenacious secretions. Which intervention is most appropriate to help prevent formation of mucus plugs?

- A. Limit oral fluids to reduce coughing
- B. Provide humidification as prescribed
- C. Deflate the tracheostomy cuff every 2 hours
- D. Place the client flat in bed several times a day

Correct Answer: B

Humidification helps keep secretions moist and easier to clear, reducing the risk of mucus plugs. Limiting fluids can worsen thick secretions. Routine cuff deflation is not a method for preventing plugs and may be unsafe depending on the client. Lying flat does not help secretion management and may impair breathing.

3831. The nurse is caring for a client with a tracheostomy and notes bright red blood coming from the tracheostomy site. What is the nurse's priority action?

- A. Document the amount of drainage and recheck in 30 minutes
- B. Suction the tracheostomy to remove blood from the airway
- C. Apply oxygen by nasal cannula

D. Call for immediate assistance and notify the provider at once

Correct Answer: D

Bright red bleeding from a tracheostomy can signal a serious hemorrhagic complication and requires immediate assistance and rapid provider notification. Documentation alone delays care. Suctioning may be needed if the airway is filling with blood, but the priority is recognizing this as an emergency and getting help immediately. Nasal cannula oxygen is ineffective for oxygen delivery through a tracheostomy.

3832. A nurse is caring for a client with a cuffed tracheostomy tube who wants to eat lunch. Which action by the nurse is most appropriate before the meal?

- A. Check the prescription and swallowing status before oral intake
- B. Encourage the client to drink thin liquids first
- C. Remove the inner cannula to make swallowing easier
- D. Place the client in a supine position to reduce coughing

Correct Answer: A

Before oral intake, the nurse should verify that eating is allowed and assess swallowing safety according to the plan of care. Clients with tracheostomies may be at risk for aspiration. Thin liquids are often harder, not easier, to control. Removing the inner cannula is not done to assist swallowing. Supine positioning increases aspiration risk; upright positioning is preferred.

3833. When performing routine tracheostomy care, which action is appropriate for cleaning around the stoma?

- A. Use sterile technique and clean from the stoma outward
- B. Clean vigorously under the flange to remove all crusting at once
- C. Use cotton-tipped applicators repeatedly until the skin bleeds slightly
- D. Apply a thick layer of lotion around the opening after cleaning

Correct Answer: A

Tracheostomy stoma care is performed using sterile technique, and cleaning moves from the stoma outward to reduce contamination. Vigorous scrubbing can injure tissue. Causing bleeding is not acceptable care. Thick lotion can trap moisture and secretions and may irritate the site.

3834. A client with a chronic tracheostomy is being discharged home. Which statement by the nurse is best when teaching about water safety?

- A. You may swim as long as you wear a tight face mask.
- B. Keep bath water below chest level and prevent water from entering the tracheostomy.
- C. It is safest to clean the tracheostomy while standing under a shower.
- D. If water enters the tracheostomy, cough a few times and continue bathing as usual.

Correct Answer: B

Water entering a tracheostomy can go directly into the airway, so teaching focuses on preventing water from entering the tube and using caution during bathing. Swimming is generally unsafe. Shower water directed at the tracheostomy is risky. Water entry into the tracheostomy is not something to ignore.

3835. The nurse is caring for a client with a tracheostomy receiving oxygen by tracheostomy collar. Which finding requires the most immediate follow-up?

- A. Water is collecting in the oxygen tubing
- B. The client requests suctioning before visiting with family
- C. The client has a productive cough with white sputum
- D. A clean dressing is dry under the tracheostomy flange

Correct Answer: A

Condensation collecting in oxygen tubing can obstruct gas flow or be accidentally delivered toward the airway, so it requires prompt attention. Requesting suctioning, a productive cough, and a dry dressing do not indicate the same immediate equipment-related hazard. The key is recognizing a preventable threat to airway support.

3836. A nurse is preparing to suction a client's tracheostomy. Which action should the nurse take while inserting the suction catheter?

- A. Apply suction continuously during insertion
- B. Insert the catheter until resistance is met, then rotate forcefully
- C. Insert the catheter without applying suction
- D. Instill normal saline routinely before each pass

Correct Answer: C

The catheter is inserted without suction to reduce trauma and hypoxemia; suction is applied during withdrawal according to agency practice. Applying suction during insertion increases mucosal injury and oxygen loss. Forceful rotation after meeting resistance can damage tissue. Routine saline instillation is not the best standard action for every suction pass and may create complications.

3837. A client with a tracheostomy becomes increasingly difficult to ventilate with a manual resuscitation bag. The nurse cannot pass a suction catheter through the tracheostomy tube. Which problem should the nurse suspect first?

- A. Pneumothorax
- B. Tube obstruction
- C. Pulmonary edema
- D. Developing atelectasis

Correct Answer: B

If a suction catheter cannot be passed through the tracheostomy tube, obstruction of the tube is the most likely immediate problem. That finding directly points to a blocked airway device, such as a mucus plug. Pneumothorax, pulmonary edema, and atelectasis can impair breathing, but they do not best explain inability to pass the catheter through the tube.

3838. A nurse is changing the inner cannula of a tracheostomy. Which action is most important to maintain airway safety during the procedure?

- A. Remove the old cannula and leave the tracheostomy open to air for several minutes
- B. Perform the change quickly while keeping the outer cannula securely in place

- C. Cut the tracheostomy ties first to improve access
- D. Turn the client onto the side during the cannula change

Correct Answer: B

During an inner cannula change, the outer cannula must remain secure because it maintains the airway. The procedure should be done efficiently to minimize interruption. Leaving the airway device open unnecessarily increases risk. Cutting ties first can dislodge the tracheostomy. Side-lying is not the key safety action for this task.

3839. A client 24 hours after tracheostomy placement suddenly has decreased oxygen saturation, increased work of breathing, and absent airflow from the tracheostomy. After calling for help, what should the nurse do next?

- A. Remove the old tracheostomy ties and clean the stoma
- B. Attempt to ventilate through the occluded tracheostomy tube only
- C. Assess patency and prepare to use the obturator and replacement tube if dislodgement is confirmed
- D. Place a dressing over the stoma and wait for the provider to arrive

Correct Answer: C

With sudden loss of airflow from a recent tracheostomy, the nurse should rapidly assess patency and be prepared for emergency tube replacement if the tube is dislodged, using emergency equipment kept at the bedside. Cleaning and routine tie changes are not priorities. Trying to ventilate only through an occluded tube delays correction. Waiting passively for the provider can endanger the airway.

Ventilator care

3840. A client receiving mechanical ventilation becomes agitated, and the high-pressure alarm sounds. Which action should the nurse take first?

- A. Increase the tidal volume setting
- B. Suction the client's airway

- C. Disconnect the ventilator and silence the alarm
- D. Lower the oxygen concentration

Correct Answer: B

A high-pressure alarm commonly indicates increased resistance to airflow, such as secretions, coughing, biting the tube, or kinked tubing. The nurse should quickly assess and address the likely cause; suctioning is an appropriate first action when secretions are suspected. Increasing tidal volume or lowering oxygen does not treat the problem. Disconnecting the ventilator is not the first step unless the client cannot be ventilated and immediate manual ventilation is needed.

3841. The nurse is caring for a client on a ventilator. Which finding requires immediate action?

- A. Pulse oximetry decreases from 97% to 88%
- B. The client needs oral care
- C. Condensation is visible in the ventilator tubing
- D. The client has mild abdominal distention

Correct Answer: A

A drop in oxygen saturation to 88% suggests impaired oxygenation and requires immediate assessment and intervention. Oral care is important but not the most urgent issue. Condensation in tubing should be managed, but it is not as urgent as hypoxemia. Mild abdominal distention may occur with air swallowing but is not the priority over decreased oxygenation.

3842. A client with an endotracheal tube suddenly becomes restless. The low-pressure ventilator alarm begins to sound. Which problem should the nurse suspect first?

- A. Water has collected in the tubing
- B. The client has bronchospasm
- C. A leak or disconnection is present
- D. Secretions are blocking the airway

Correct Answer: C

A low-pressure alarm usually indicates a leak in the system or a disconnection, causing less pressure than expected during ventilation. Water in the tubing and secretions are more often associated with resistance or high-pressure concerns. Bronchospasm also increases resistance rather than causing a low-pressure alarm.

3843. Which position is best for a ventilated client to help reduce the risk of aspiration and ventilator-associated complications?

- A. Flat supine position
- B. Side-lying with head flat
- C. Semi-Fowler's position
- D. Trendelenburg position

Correct Answer: C

Semi-Fowler's positioning helps promote lung expansion and reduces aspiration risk in clients receiving mechanical ventilation. Flat, head-down, or Trendelenburg positions can increase aspiration risk and impair respiratory status.

3844. A nurse is performing care for a client with an endotracheal tube connected to a ventilator. Which nursing action is most appropriate to help prevent ventilator-associated pneumonia?

- A. Provide regular oral care
- B. Instill normal saline before every suction pass
- C. Change the ventilator tubing every shift
- D. Keep the client in a flat position after feeding

Correct Answer: A

Regular oral care helps reduce bacterial colonization and is an important measure in preventing ventilator-associated pneumonia. Routine saline instillation before suctioning is not generally recommended as a standard prevention step. Changing ventilator tubing every shift is unnecessary and can increase manipulation. Keeping the client flat after feeding increases aspiration risk.

3845. A ventilated client suddenly has absent breath sounds on the left, increasing respiratory distress, and an oxygen saturation of 84%. Which complication should the nurse suspect?

- A. Right mainstem intubation

- B. Left-sided pneumothorax
- C. Pulmonary edema
- D. Metabolic acidosis

Correct Answer: B

Absent breath sounds on one side with sudden distress and hypoxemia suggests pneumothorax, a potentially life-threatening complication in a ventilated client. Right mainstem intubation would more likely decrease breath sounds on the left, but the sudden deterioration and distress raise concern for pneumothorax, especially in a mechanically ventilated client. Pulmonary edema usually causes bilateral crackles, and metabolic acidosis would not explain unilateral absent breath sounds.

3846. The nurse notes that a ventilated client is 'fighting the ventilator,' coughing frequently, and has asynchronous chest movement with delivered breaths. What is the nurse's priority action?

- A. Assess the client and the ventilator system for causes of asynchrony
- B. Turn off the ventilator for several minutes
- C. Advance the endotracheal tube by 2 cm
- D. Reduce the alarm volume so the client can rest

Correct Answer: A

Client-ventilator asynchrony requires prompt assessment of both the client and equipment. Causes may include secretions, pain, anxiety, tube kinking, improper settings, or worsening respiratory status. Turning off the ventilator is unsafe. Advancing the tube without an order or confirmation of placement is inappropriate. Lowering alarm volume hides a warning without treating the cause.

3847. A client on mechanical ventilation has thick secretions and coarse crackles. Which action should the nurse take when suctioning the airway?

- A. Suction on a routine schedule every 15 minutes
- B. Apply suction while inserting the catheter
- C. Use suction only when clinically indicated
- D. Suction for at least 30 seconds with each pass

Correct Answer: C

Airway suctioning should be performed when clinically indicated, such as with visible secretions, coarse breath sounds, decreased oxygenation, or increased airway pressures. Routine frequent suctioning can cause trauma and hypoxemia. Suction is applied while withdrawing, not inserting, the catheter. Long suction passes increase complications such as hypoxia and mucosal injury.

3848. During morning care, the nurse finds that a ventilated client's endotracheal tube has moved outward and the centimeter marking at the lip is different from the previous documented position. What should the nurse do first?

- A.** Reposition the tube back to the previous marking
- B.** Assess breath sounds, oxygenation, and secure the airway while notifying the provider or respiratory therapist
- C.** Deflate the cuff and advance the tube
- D.** Document the new position at the end of the shift

Correct Answer: B

A change in endotracheal tube position may indicate displacement. The nurse should first assess the client's respiratory status and airway function, maintain tube security, and notify appropriate personnel for prompt evaluation. Repositioning or advancing the tube without appropriate verification is unsafe. Delaying action to document later risks airway compromise.

3849. A client receiving mechanical ventilation suddenly cannot be adequately ventilated by the machine, and multiple alarms sound despite quick checks for tubing kinks and disconnections. What is the nurse's best immediate action?

- A.** Place the client flat and wait for respiratory therapy
- B.** Manually ventilate the client with a resuscitation bag and oxygen
- C.** Silence the alarms and continue to observe
- D.** Turn the oxygen flow down to reduce barotrauma

Correct Answer: B

If the ventilator is not effectively ventilating the client, the immediate priority is oxygenation and ventilation. Manual ventilation with a resuscitation bag and oxygen is the safest immediate action while further help and troubleshooting occur. Waiting, only silencing alarms, or lowering oxygen delays lifesaving support.

3850. Which assessment finding best suggests that a ventilated client may need suctioning?

- A.** Clear breath sounds and oxygen saturation of 98%
- B.** Coarse breath sounds with visible secretions in the tube
- C.** Blood pressure of 126/74 mm Hg
- D.** Urine output of 45 mL/hr

Correct Answer: B

Coarse breath sounds and visible secretions are classic cues that secretions are accumulating and suctioning may be needed. Clear breath sounds and normal oxygenation do not suggest a need for suctioning. Blood pressure and urine output are important assessments but do not specifically indicate retained airway secretions.

3851. The family of a ventilated client asks why the nurse does not simply silence alarms when they sound repeatedly. Which response is best?

- A.** The alarms are not important unless the client looks uncomfortable
- B.** The ventilator alarms are checked because they may signal a problem with the client or the equipment
- C.** Alarms are mostly caused by machine defects and can be ignored at first
- D.** Silencing alarms for long periods helps keep the client calm

Correct Answer: B

Ventilator alarms may indicate serious problems such as obstruction, disconnection, worsening respiratory status, or equipment malfunction, so they must be assessed promptly. The trap is minimizing alarms based only on whether the client appears uncomfortable; some dangerous problems can develop quickly before obvious distress is seen.

3852. A ventilated client becomes suddenly tachycardic and hypotensive. The nurse notes severe respiratory distress, one-sided absent breath sounds, and tracheal deviation. Which action has the highest priority?

- A. Prepare for immediate treatment of a tension pneumothorax
- B. Lower the head of the bed
- C. Encourage the client to cough forcefully
- D. Obtain a sputum sample for culture

Correct Answer: A

This cluster of findings strongly suggests tension pneumothorax, a life-threatening emergency that can occur with positive-pressure ventilation. Immediate emergency treatment is the priority. Lowering the head of the bed, encouraging coughing, or collecting sputum delays urgent intervention and does not address the cause of hemodynamic collapse.

3853. A nurse enters the room of a client on mechanical ventilation and finds the ventilator tubing disconnected from the endotracheal tube. The client is awake but anxious. Which action should the nurse take first?

- A. Reconnect the tubing and assess the client's status
- B. Call for the provider before touching the tubing
- C. Document the disconnection event
- D. Suction the client for 20 seconds

Correct Answer: A

When the tubing is visibly disconnected, restoring the system promptly is the first priority, followed by immediate assessment of the client's respiratory status. Calling first or documenting first delays care. Suctioning may be needed later if indicated, but it is not the first response to a simple disconnection.

3854. A client on positive-pressure ventilation develops increasing peak airway pressures, restlessness, and diminished bilateral breath sounds after repositioning. The oxygen saturation is falling. Which action is the nurse's best first step?

- A. Check for a kinked tube or obstruction in the ventilator circuit
- B. Administer a prescribed tube feeding bolus

- C. Turn the client fully onto the affected side
- D. Reduce the ventilator alarm limits

Correct Answer: A

After repositioning, increased airway pressures and falling oxygen saturation may result from a kinked tube, compressed tubing, or another mechanical obstruction. The nurse should first assess for a reversible equipment-related cause. Feeding is unrelated and unsafe during instability. Turning the client without identifying the problem may worsen the situation. Lowering alarm limits hides deterioration instead of correcting it.

3855. A client is receiving mechanical ventilation through an endotracheal tube. During assessment, the nurse notes bilateral coarse breath sounds, visible secretions in the tube, and an oxygen saturation that has dropped from 96% to 90%. Before suctioning, what should the nurse do?

- A. Instill sterile normal saline into the endotracheal tube
- B. Increase the ventilator rate for 10 minutes
- C. Hyperoxygenate the client with 100% oxygen
- D. Deflate the endotracheal tube cuff

Correct Answer: C

Hyperoxygenating before suctioning helps reduce hypoxemia caused by interrupting airflow and removing oxygen during the procedure. Instilling saline is not routine and can worsen oxygenation or push secretions deeper. Increasing the ventilator rate does not directly prepare the client for suctioning. Deflating the cuff would increase aspiration risk and is not part of suction preparation.

3856. The nurse is caring for a client who has just been intubated and placed on mechanical ventilation. Which finding best indicates that ventilation is improving?

- A. The client's oxygen saturation rises from 88% to 95%
- B. The client's PaCO₂ decreases from 58 mm Hg to 42 mm Hg
- C. The client's heart rate increases from 88/min to 108/min
- D. The client's blood pressure decreases from 138/84 mm Hg to 112/68 mm Hg

Correct Answer: B

Ventilation refers to carbon dioxide removal, so a drop in PaCO₂ toward normal best shows improved ventilation. Oxygen saturation reflects oxygenation, which is related but not the same. Heart rate and blood pressure can change for many reasons and do not directly show ventilatory effectiveness.

3857. A ventilated client is receiving enteral feedings through a tube. Which nursing action is most appropriate to reduce the risk of aspiration?

- A. Keep the client in a supine position during feedings
- B. Turn off the humidifier while feedings are infusing
- C. Elevate the head of the bed unless contraindicated
- D. Provide oral fluids every 2 hours

Correct Answer: C

Elevating the head of the bed helps reduce aspiration risk in ventilated clients, especially during enteral feedings. A supine position increases aspiration risk. Humidifier use is unrelated to aspiration prevention. Oral fluids are not appropriate for an intubated client.

3858. A client on mechanical ventilation has these arterial blood gas results: pH 7.30, PaCO₂ 52 mm Hg, HCO₃⁻ 24 mEq/L, PaO₂ 90 mm Hg. How should the nurse interpret these findings?

- A. Respiratory acidosis
- B. Metabolic acidosis
- C. Respiratory alkalosis
- D. Fully compensated metabolic alkalosis

Correct Answer: A

The pH is low, showing acidosis. The PaCO₂ is elevated, pointing to a respiratory cause. The bicarbonate is normal, so there is no evidence of metabolic compensation here. This pattern is respiratory acidosis, which may reflect inadequate ventilation.

3859. A client who has been on a ventilator for several days is being assessed for readiness to wean. Which finding best supports possible readiness for weaning?

- A. The client requires an FiO₂ of 90% to maintain oxygen saturation
- B. The client is alert and able to follow simple commands
- C. The client needs frequent vasopressor support for hypotension
- D. The client has copious thick secretions and a weak cough

Correct Answer: B

An alert client who can follow commands is more likely to protect the airway and participate in weaning. Requiring very high oxygen, needing vasopressors, and having copious secretions with a weak cough all suggest instability or poor readiness for liberation from the ventilator.

3860. The nurse is providing oral care to a ventilated client with an endotracheal tube. Why is this intervention important?

- A. It decreases the risk of ventilator-associated pneumonia
- B. It prevents accidental extubation during repositioning
- C. It ensures the ventilator delivers the set tidal volume
- D. It eliminates the need for suctioning

Correct Answer: A

Oral care helps reduce bacterial colonization in the mouth and oropharynx, which lowers the risk of ventilator-associated pneumonia. It does not secure the tube, control delivered tidal volume, or replace the need for airway suctioning when secretions are present.

3861. A client on mechanical ventilation has an arterial blood gas result showing pH 7.49 and PaCO₂ 30 mm Hg. Which ventilator-related problem does the nurse suspect?

- A. The client is being underventilated
- B. The client is being overventilated
- C. The client has worsening oxygen diffusion failure only
- D. The client has metabolic acidosis from renal failure

Correct Answer: B

A high pH with a low PaCO₂ indicates respiratory alkalosis, which commonly results from overventilation. Underventilation would raise PaCO₂. Oxygen diffusion problems affect oxygenation more directly than this acid-base pattern. Metabolic acidosis would show a low pH, not an elevated one.

3862. The nurse is caring for a client on mechanical ventilation with positive end-expiratory pressure (PEEP). Which statement best explains a possible effect of high PEEP that the nurse should monitor for?

- A. It can decrease venous return and lower blood pressure
- B. It always increases cardiac output by improving oxygenation
- C. It prevents all alveolar collapse regardless of lung condition
- D. It mainly causes hypoglycemia by reducing metabolic demand

Correct Answer: A

High PEEP can increase intrathoracic pressure, reducing venous return to the heart and causing hypotension. Improved oxygenation does not guarantee improved cardiac output. PEEP can help keep alveoli open, but it does not prevent all collapse in every situation. It is not mainly associated with hypoglycemia.

3863. A ventilated client is restless and trying to pull at the endotracheal tube. The prescribed sedative infusion is running. Which nursing action is most appropriate?

- A. Apply restraints without reassessing the client
- B. Assume the behavior is expected and continue to monitor
- C. Assess for possible causes such as pain, hypoxia, or inadequate sedation
- D. Silence the ventilator alarms to reduce stimulation

Correct Answer: C

Restlessness in a ventilated client may indicate hypoxia, pain, anxiety, retained secretions, or inadequate sedation, so assessment of the cause is the best action. Automatically restraining the client skips nursing judgment. Assuming the behavior is expected can delay recognition of deterioration. Silencing alarms without evaluation is unsafe.

3864. After suctioning a ventilated client, which finding best indicates that the procedure was effective?

- A. Breath sounds are clearer and oxygen saturation improves
- B. The client becomes more drowsy and less responsive
- C. The ventilator alarm volume is reduced
- D. The endotracheal tube cuff requires more air

Correct Answer: A

Effective suctioning removes secretions, which should improve air movement and often improve oxygenation. Increased drowsiness may signal worsening status, not success. Alarm volume is unrelated to airway clearance. Cuff inflation needs do not indicate successful secretion removal.

3865. A client on mechanical ventilation is difficult to oxygenate despite a high FiO₂. The provider adds PEEP to the ventilator settings. What is the primary purpose of PEEP?

- A. To increase carbon dioxide production
- B. To keep alveoli open at end expiration and improve oxygenation
- C. To stimulate spontaneous breathing by lowering airway pressure
- D. To decrease the need for humidification of inspired gas

Correct Answer: B

PEEP helps prevent alveolar collapse at the end of expiration, increasing functional gas exchange surface and improving oxygenation. It does not increase carbon dioxide production, does not work by lowering airway pressure to stimulate breathing, and does not replace humidification needs.

Cardiac monitoring and pacing

3866. A client on continuous cardiac monitoring suddenly develops a heart rate of 34/min. The client is pale, diaphoretic, and says, "I feel dizzy." Which action should the nurse take first?

- A. Document the rhythm strip and continue to observe the client
- B. Increase the client's oral fluid intake
- C. Assess the client and notify the provider or rapid response team immediately
- D. Have the client ambulate to improve circulation

Correct Answer: C

A heart rate of 34/min with dizziness, pallor, and diaphoresis suggests symptomatic bradycardia and possible decreased cardiac output. The priority is immediate assessment and escalation for urgent treatment. Documentation is important but not first. Oral fluids and ambulation do not address the potentially unstable condition and could worsen it.

3867. The nurse is reviewing telemetry strips. Which finding is most consistent with atrial fibrillation?

- A. Regular rhythm with one P wave before each QRS complex
- B. Irregularly irregular rhythm with no distinct P waves
- C. Progressively lengthening PR intervals followed by a dropped beat
- D. Sawtooth atrial pattern with a regular ventricular response

Correct Answer: B

Atrial fibrillation is classically identified by an irregularly irregular rhythm and the absence of distinct P waves. Choice A describes normal sinus rhythm. Choice C suggests a second-degree AV block type I pattern. Choice D is more consistent with atrial flutter rather than atrial fibrillation.

3868. A client with frequent premature ventricular contractions reports palpitations. Which assessment finding requires the nurse's most immediate attention?

- A. Potassium level of 3.0 mEq/L

- B. Apical pulse of 88/min
- C. Temperature of 99.1°F (37.3°C)
- D. Blood glucose of 122 mg/dL

Correct Answer: A

Hypokalemia can increase ventricular irritability and worsen PVCs, raising concern for more serious dysrhythmias. A potassium level of 3.0 mEq/L is abnormal and clinically significant. The other findings are not the most urgent contributors to worsening ectopy in this situation.

3869. A client with a temporary transvenous pacemaker has a monitor rhythm showing pacing spikes, but no QRS complexes follow the spikes. How should the nurse interpret this finding?

- A. Failure to sense
- B. Failure to capture
- C. Normal pacemaker function
- D. Oversensing

Correct Answer: B

Failure to capture occurs when the pacemaker fires, seen as a pacing spike, but the myocardium does not respond with a depolarization, so no QRS follows. Failure to sense means the pacemaker does not appropriately detect intrinsic beats. Oversensing inhibits pacing because noncardiac signals are misread as cardiac activity.

3870. A client is admitted with chest pain and is placed on continuous cardiac monitoring. Which nursing action helps ensure the most accurate monitoring data?

- A. Apply electrodes over bony prominences to limit movement
- B. Use lotion on the chest before attaching electrodes
- C. Prepare the skin and replace loose electrodes promptly
- D. Turn off monitor alarms during routine care

Correct Answer: C

Good skin preparation and prompt replacement of loose electrodes improve signal quality and reduce artifact. Electrodes should not be placed over bony prominences just to limit movement if placement compromises signal quality. Lotion interferes with adhesion, and alarms should not be turned off because they provide critical safety alerts.

3871. A postoperative client on telemetry suddenly has a narrow-complex tachycardia at 186/min. The client is alert but reports shortness of breath and chest pressure. Which action is the nurse's priority?

- A.** Assess blood pressure and other signs of hemodynamic stability
- B.** Offer the client a caffeinated beverage to improve alertness
- C.** Remove the telemetry leads and reapply them in a different pattern
- D.** Encourage the client to rest and recheck the rhythm in 30 minutes

Correct Answer: A

With a sudden tachyarrhythmia, the nurse first determines whether the client is stable by assessing blood pressure, perfusion, symptoms, and overall hemodynamic status. This drives urgency and next steps. Delaying assessment is unsafe. Caffeine could worsen tachycardia, and lead repositioning should not come before evaluating a symptomatic client.

3872. The nurse notes a rhythm strip with six premature ventricular contractions in 1 minute. Which additional finding would increase concern that the ectopy may progress to a more serious dysrhythmia?

- A.** The PVCs all look alike and occur singly
- B.** The client is resting quietly after pain medication
- C.** The PVCs are multifocal and occur in couplets
- D.** The heart rate is 72/min between ectopic beats

Correct Answer: C

Multifocal PVCs and couplets suggest greater ventricular irritability and are more concerning than isolated unifocal PVCs. The trap is assuming any PVC pattern is equally serious. A normal underlying rate or the client resting quietly does not reduce the significance of more complex ventricular ectopy.

3873. A client with a permanent pacemaker tells the nurse, "Sometimes I feel lightheaded, and my pulse seems slow." Which response by the nurse is best?

- A.** That is expected with all pacemakers, so no follow-up is needed
- B.** You should avoid checking your pulse because it can be misleading
- C.** Those symptoms may mean the pacemaker is not working correctly and should be reported promptly
- D.** Drink more water when that happens, and the pacemaker should reset itself

Correct Answer: C

Lightheadedness and a slow pulse may indicate pacemaker malfunction or inadequate pacing and should be reported promptly for evaluation. The incorrect choices minimize potentially important symptoms or suggest inaccurate self-management. The best answer emphasizes safety and timely follow-up.

3874. A client with complete heart block becomes unresponsive. The monitor shows a ventricular rate of 20/min. The nurse activates the emergency response system. What is the next priority action?

- A.** Obtain a 12-lead ECG before starting interventions
- B.** Begin supportive resuscitative measures according to the client's condition
- C.** Wait to see whether the ventricular rhythm improves spontaneously
- D.** Document the event after the next set of vital signs

Correct Answer: B

An unresponsive client with profound bradycardia requires immediate supportive resuscitative action after activating help. Waiting, documenting first, or delaying treatment for a 12-lead ECG would be unsafe. The key is responding to the client's instability, not just describing the rhythm.

3875. The nurse is caring for a client receiving demand pacing. Which observation best indicates that the pacemaker is sensing appropriately?

- A. Pacing spikes occur randomly despite the client's own QRS complexes
- B. The pacemaker fires only when the client's intrinsic rate falls below the set rate
- C. Every pacing spike is followed by a narrow QRS complex
- D. The monitor shows continuous pacing spikes at a fixed interval, regardless of intrinsic rhythm

Correct Answer: B

Demand pacing should detect intrinsic cardiac activity and pace only when the heart rate drops below the programmed threshold. That finding reflects appropriate sensing. Choice C speaks to capture, not sensing. Choices A and D suggest failure to sense because the pacemaker is firing despite native cardiac activity.

3876. A telemetry alarm sounds for asystole. When the nurse enters the room, the client is talking on the phone and appears comfortable. What should the nurse do first?

- A. Call a code because the monitor shows asystole
- B. Check the client and the monitor leads for artifact or disconnection
- C. Silence the alarm and document a false alarm
- D. Prepare for immediate defibrillation

Correct Answer: B

When the monitor tracing and the client's condition do not match, the nurse first checks the client and equipment. A talking, comfortable client is not in true asystole. This is a common trap: treating the monitor before assessing the patient. Defibrillation is not used for asystole, and documenting a false alarm should occur only after verification.

3877. A client with acute chest pain develops ST-segment elevation on the monitor. Which interpretation by the nurse is most appropriate?

- A. The finding may indicate acute myocardial injury and requires rapid evaluation
- B. The client is likely experiencing anxiety-related hyperventilation
- C. This pattern usually means the pacemaker battery is depleted
- D. The rhythm change is expected after routine repositioning in bed

Correct Answer: A

ST-segment elevation in a client with chest pain can indicate acute myocardial injury and needs urgent evaluation. The distractors either minimize a potentially life-threatening change or incorrectly attribute it to unrelated causes. The key is correlating the ECG change with the clinical presentation.

3878. A client with a temporary pacemaker suddenly becomes dizzy. The monitor shows long pauses with no pacing spikes present during the pauses. Which pacemaker problem does this finding most strongly suggest?

- A. Failure to capture
- B. Failure to sense
- C. Failure to pace
- D. Normal inhibition

Correct Answer: C

Failure to pace means the pacemaker does not emit a pacing stimulus when needed, so there are pauses without pacing spikes. Failure to capture would show spikes without resulting depolarization. Failure to sense involves inappropriate firing or inhibition based on poor detection of intrinsic activity. Normal inhibition would not fit a symptomatic client with prolonged pauses.

3879. The nurse is teaching a client who has a new permanent pacemaker. Which statement by the client indicates a need for further teaching?

- A. I will report dizziness or fainting episodes to my provider
- B. I should keep my follow-up appointments for pacemaker checks

- C. If I feel my heart racing, I will note the symptoms and seek guidance
- D. I can place strong magnets over the device briefly if it feels uncomfortable

Correct Answer: D

The statement about placing strong magnets over the device is unsafe and shows a need for more teaching. Magnets can interfere with pacemaker function. The other statements reflect appropriate symptom reporting and follow-up. The trap is treating the device like something the client can adjust independently.

3880. A client on telemetry has this pattern: normal beats alternate with premature ventricular contractions in a repeating sequence. How should the nurse document this rhythm finding?

- A. Ventricular bigeminy
- B. Ventricular trigeminy
- C. Second-degree heart block
- D. Sinus arrhythmia

Correct Answer: A

Ventricular bigeminy is a repeating pattern in which every normal beat is followed by a PVC. Trigeminy would mean two normal beats followed by a PVC. The other options describe different rhythm problems entirely. Recognizing these patterned ectopic rhythms helps the nurse communicate findings accurately.

3881. A client on telemetry has a rhythm strip that shows an atrial rate of about 300/min with a repeating sawtooth pattern between narrow QRS complexes. The ventricular rhythm is regular at 150/min. Which rhythm should the nurse suspect?

- A. Atrial flutter with 2:1 conduction
- B. Sinus tachycardia
- C. Atrial fibrillation
- D. First-degree AV block

Correct Answer: A

Atrial flutter classically produces flutter waves that create a sawtooth appearance, often with a regular ventricular response such as 150/min when there is 2:1 conduction. Atrial fibrillation is typically irregularly irregular without distinct flutter waves. Sinus tachycardia has normal P waves before each QRS, and first-degree AV block has a prolonged PR interval rather than rapid atrial activity.

3882. The nurse is assessing a client with a newly inserted temporary transvenous pacemaker. Which finding most strongly suggests effective mechanical capture?

- A. The monitor shows pacing spikes before each QRS complex
- B. The client reports tenderness at the insertion site
- C. Each paced beat is followed by a palpable pulse and improved blood pressure
- D. The pacemaker generator lights are blinking regularly

Correct Answer: C

Mechanical capture means the electrical pacing stimulus is producing actual cardiac contraction and perfusion. The best evidence is a paced rhythm accompanied by a palpable pulse and hemodynamic improvement. Spikes before QRS complexes support electrical capture, not mechanical capture by themselves. Site tenderness and generator lights do not confirm effective perfusion.

3883. A client with a permanent pacemaker asks what to do when checking the pulse at home. Which instruction is most appropriate for the nurse to give?

- A. Check the pulse every day and report a rate below the pacemaker's programmed minimum
- B. Stop checking the pulse once the incision is healed
- C. Only check the pulse if chest pain occurs
- D. Expect the pulse to vary widely because pacemakers do not control heart rate

Correct Answer: A

Clients with permanent pacemakers are commonly taught to check their pulse and report rates below the device's set lower limit or other symptoms such as dizziness or syncope. Stopping pulse checks entirely is inappropriate. Waiting only for chest pain may delay recognition of pacemaker malfunction. Pacemakers do regulate heart rate support, so a widely variable slow pulse is not an expected finding to ignore.

3884. While obtaining a telemetry rhythm strip, the nurse notices a wandering baseline and irregular sharp deflections only when the client uses an electric razor. The client is awake, warm, and denies symptoms. Which action is best?

- A. Call the rapid response team immediately
- B. Document ventricular fibrillation
- C. Remove the source of electrical interference and reassess the tracing
- D. Administer oxygen by nasal cannula

Correct Answer: C

The pattern occurring only when the electric razor is on suggests artifact from electrical interference. The correct response is to eliminate the interference source and reassess the monitor tracing. Calling a rapid response or documenting a lethal rhythm without correlating with the client would be unsafe. Oxygen does not address the likely cause of the abnormal tracing.

3885. A telemetry client has a PR interval of 0.24 second on each beat, with every P wave followed by a QRS complex and a regular rhythm. How should the nurse interpret this strip?

- A. Second-degree AV block type II
- B. First-degree AV block
- C. Junctional rhythm
- D. Sinus rhythm with premature atrial contractions

Correct Answer: B

First-degree AV block is identified by a consistently prolonged PR interval greater than 0.20 second with all atrial impulses conducted to the ventricles. Second-degree type II has dropped QRS complexes. Junctional rhythm usually lacks normal upright P waves before every QRS. Premature atrial contractions produce early beats rather than a uniformly prolonged PR interval.

3886. A client with an acute myocardial infarction is on continuous monitoring. The nurse notes multifocal PVCs, and the client now reports increasing shortness of breath. Which action should the nurse take first?

- A. Assess the client and obtain vital signs
- B. Print the rhythm strip for the chart
- C. Ask the client to cough forcefully
- D. Turn off the alarm to reduce anxiety

Correct Answer: A

When a potentially serious rhythm change occurs, the nurse should first assess the client to determine hemodynamic stability and correlate symptoms with the monitor findings. Shortness of breath with multifocal PVCs may signal worsening status. Printing the strip can follow assessment. Coughing is not the first intervention here, and silencing alarms without assessment is unsafe.

3887. A client has a temporary pacemaker in place after cardiac surgery. On the monitor, native QRS complexes are followed at times by pacing spikes that occur inappropriately despite adequate intrinsic beats. Which pacemaker problem does this finding most strongly suggest?

- A. Failure to capture
- B. Failure to sense
- C. Battery depletion causing no output
- D. Mechanical capture with low cardiac output

Correct Answer: B

Failure to sense means the pacemaker does not recognize intrinsic cardiac activity and fires when it should be inhibited, producing inappropriate pacing spikes after or within native beats. Failure to capture would show spikes not followed by depolarization. Battery depletion causing no output would cause absent spikes. Mechanical capture concerns perfusion, not sensing behavior.

3888. The nurse reviews a client's monitor and sees a rhythm with no visible P waves, wide bizarre QRS complexes, and a ventricular rate of 32/min. The client is awake but confused and hypotensive. Which interpretation is most appropriate?

- A. Idioventricular rhythm causing poor perfusion
- B. Normal sinus bradycardia
- C. Atrial flutter with slow conduction
- D. Supraventricular tachycardia

Correct Answer: A

An idioventricular rhythm is characterized by a very slow ventricular rate, absent visible atrial activity or AV relationship, and wide bizarre QRS complexes. Because the ventricles are pacing the heart slowly, perfusion may be poor, which fits the client's confusion and hypotension. Sinus bradycardia would usually have P waves and narrow QRS complexes. Atrial flutter and SVT do not fit the slow wide-complex pattern.

3889. A client with a new permanent pacemaker asks about household items. Which statement by the nurse is best?

- A. Avoid all electrical appliances permanently
- B. Most household appliances are safe to use if they are in good working order
- C. Microwave ovens will always stop the pacemaker from working
- D. Keep a cell phone directly over the pacemaker site to monitor its function

Correct Answer: B

Teaching should be practical: most household appliances are safe for clients with permanent pacemakers when used normally and when equipment is functioning properly. Telling the client to avoid all appliances is unnecessarily restrictive. Saying microwaves always stop pacemakers is inaccurate. A cell phone should not be carried directly over the device site, and it does not monitor pacemaker function.

3890. The nurse is caring for a client receiving transcutaneous pacing for symptomatic bradycardia. The monitor shows pacing spikes followed by wide QRS complexes, but the client remains pale with weak peripheral pulses and a blood pressure of 78/40 mm Hg. Which action is the priority?

- A. Document successful pacing because electrical capture is present
- B. Increase sedation before any further assessment
- C. Assess for mechanical capture and notify the provider that perfusion remains inadequate
- D. Discontinue pacing because the wide QRS complexes are expected

Correct Answer: C

Electrical capture on the monitor is not enough; the nurse must confirm mechanical capture and adequate perfusion. This client remains hypotensive and poorly perfused, so further action is required. Documenting success based only on the monitor is a common trap. Sedation may be needed in some situations, but the priority is recognizing that pacing is not yet providing adequate circulatory support. Pacing should not be discontinued in a symptomatic unstable client.

3891. A client's telemetry strip shows occasional premature beats that occur earlier than expected. The QRS complexes of these premature beats are narrow, and each is preceded by a differently shaped P wave. Which rhythm disturbance is most likely?

- A. Premature atrial contractions
- B. Premature ventricular contractions
- C. Ventricular tachycardia
- D. Junctional escape beats

Correct Answer: A

Premature atrial contractions originate in an atrial focus, so they occur early, usually have narrow QRS complexes, and are often preceded by an abnormal-looking P wave. Premature ventricular contractions are typically wide and bizarre without a preceding P wave. Ventricular tachycardia is a sustained run of ventricular beats, not isolated occasional premature beats. Junctional escape beats occur late after a pause rather than early.

3892. A hospitalized client suddenly reports chest discomfort and dizziness. The monitor shows a rapid wide-complex rhythm at 168/min without visible P waves. The client has a weak pulse and a blood pressure of 82/46 mm Hg. What is the nurse's priority interpretation of this situation?

- A. Stable supraventricular tachycardia
- B. Ventricular tachycardia with hemodynamic instability
- C. Atrial fibrillation with controlled ventricular response
- D. Sinus tachycardia caused by anxiety

Correct Answer: B

A rapid wide-complex rhythm without visible P waves in a symptomatic hypotensive client should be treated as ventricular tachycardia with instability until proven otherwise. The weak pulse and low blood pressure make this especially urgent. Stable SVT is usually narrow-complex. Atrial fibrillation is irregularly irregular, and sinus tachycardia does not produce a wide-complex monomorphic rhythm like this.

Ostomy care

3893. A nurse is caring for a client 2 days after creation of a new ileostomy. Which finding requires immediate follow-up?

- A. The stoma is pink and moist
- B. The stoma output is liquid and greenish
- C. The stoma appears dusky and bluish
- D. The client reports mild abdominal soreness

Correct Answer: C

A dusky or bluish stoma may indicate impaired blood flow and requires immediate follow-up. A healthy stoma should be pink to red and moist. Liquid greenish drainage is expected early with an ileostomy, and mild postoperative soreness is common.

3894. A client with a colostomy tells the nurse, "My skin burns under the pouch." Which nursing action is most appropriate first?

- A. Assess the peristomal skin and the fit of the skin barrier
- B. Remove the pouch and leave the skin open to air for 8 hours
- C. Instruct the client to drink less fluid each day
- D. Apply lotion around the stoma before replacing the pouch

Correct Answer: A

Burning suggests possible leakage or skin irritation, so the nurse should first assess the peristomal skin and whether the barrier opening fits correctly. Leaving the skin uncovered for long periods can increase leakage problems, fluid restriction is not the first response, and lotion can interfere with pouch adhesion.

3895. Which instruction should the nurse include when teaching a client how to apply an ostomy pouch?

- A. Cut the skin barrier opening to fit closely around the stoma
- B. Make the opening at least 1 inch larger than the stoma
- C. Clean the skin with oily soap before attaching the pouch
- D. Pack sterile gauze between the stoma and the barrier

Correct Answer: A

The barrier opening should fit closely around the stoma to protect the surrounding skin from drainage. An opening that is much too large exposes skin to effluent. Oily products reduce adherence, and gauze between the stoma and barrier interferes with the seal.

3896. A nurse is reinforcing teaching for a client with a new descending colostomy. Which stool pattern should the nurse tell the client is generally expected after recovery?

- A. Frequent watery output around the clock

- B. No stool output unless the pouch is irrigated
- C. Semiformed to formed stool
- D. Bright red blood with each emptying

Correct Answer: C

A descending colostomy usually produces semiformed to formed stool because more water has been absorbed by the time stool reaches that part of the colon. Watery continuous output is more typical of an ileostomy, and bright red bleeding is abnormal.

3897. A client with a new ileostomy has had high-volume liquid output for the past 12 hours. Which assessment finding is most important for the nurse to report?

- A. Dry oral mucosa and dizziness on standing
- B. Mild redness where tape was removed
- C. A request for help emptying the pouch
- D. A soft brown stool in the pouch

Correct Answer: A

High ileostomy output can quickly lead to fluid volume deficit and electrolyte imbalance. Dry mucous membranes and orthostatic dizziness suggest dehydration and require prompt reporting. Mild tape irritation and needing teaching support are lower priority, and brown output can be expected depending on intake.

3898. When changing an ostomy appliance, which action by the nurse best helps protect the peristomal skin?

- A. Scrubbing the skin vigorously to remove all adhesive
- B. Using warm water and patting the skin dry before applying the pouch
- C. Applying powder to all of the surrounding skin routinely
- D. Shaving the area with a straight razor before every pouch change

Correct Answer: B

Gentle cleansing with warm water and careful drying helps maintain skin integrity and promote pouch adherence. Vigorous scrubbing causes trauma. Powder is not applied routinely to all skin, only as indicated, and frequent razor shaving can injure the skin.

3899. A client asks when to empty an ostomy pouch. Which response by the nurse is best?

- A. Empty it only once a day to avoid disturbing the seal
- B. Empty it when it is about one-third to one-half full
- C. Wait until it is completely full before opening it
- D. Empty it only if stool leaks onto the skin barrier

Correct Answer: B

An ostomy pouch should generally be emptied when it is about one-third to one-half full to prevent excess weight from loosening the seal and causing leakage. Waiting until it is full increases the risk of spills and skin breakdown.

3900. A nurse is caring for a client with a permanent colostomy who says, "I don't want to look at it. My life is over." Which response is best?

- A. You should be grateful the surgery was successful
- B. Most people adjust quickly, so try not to worry
- C. Tell me what concerns you most about having the ostomy
- D. If you avoid mirrors for now, you will feel better later

Correct Answer: C

This therapeutic response invites the client to express feelings and specific concerns, which is the best first step in supporting adjustment. The other responses minimize the client's distress or block communication.

3901. A nurse is teaching a client how to measure a new stoma before cutting the skin barrier. Why is repeated measurement especially important during the first several weeks after surgery?

- A. The stoma usually changes size as postoperative swelling decreases
- B. The stoma should be measured only to estimate fluid intake needs
- C. The barrier opening should gradually become much larger than the stoma
- D. Measuring prevents the need to inspect the stoma color

Correct Answer: A

A new stoma often becomes smaller as edema resolves, so repeated measurement helps the barrier fit correctly and protects the skin. The opening should not be made much larger, and measuring does not replace color assessment.

3902. During ostomy care, the nurse notices a small amount of bleeding when the stoma is cleansed gently. What is the best interpretation?

- A. This suggests bowel perforation
- B. This can occur because stoma tissue is vascular
- C. This means the pouch opening is too small
- D. This is evidence of severe dehydration

Correct Answer: B

A small amount of bleeding with gentle cleansing can occur because the stoma has a rich blood supply. It does not by itself indicate perforation, dehydration, or that the pouch opening is too small, although persistent or heavy bleeding should be evaluated.

3903. A client with a sigmoid colostomy asks about reducing stool passage at social events. Which intervention should the nurse anticipate discussing with the provider if it is appropriate for this client?

- A. Colostomy irrigation
- B. Continuous gastric suction
- C. Daily enemas through the rectum
- D. Strict fluid restriction

Correct Answer: A

For some clients with certain colostomies, irrigation may help regulate evacuation and improve predictability. It is not used for all ostomies, especially not an ileostomy. Gastric suction, rectal enemas, and fluid restriction do not address routine ostomy regulation appropriately.

3904. A nurse is caring for a client whose ostomy pouch has leaked twice during the shift. Which action is most likely to prevent another leak?

- A. Cut a larger barrier opening to avoid pressure on the stoma
- B. Secure the pouch with extra tape only and leave the barrier unchanged

C. Reassess stoma size and skin contours before refitting the appliance

D. Apply moisturizing cream under the adhesive for comfort

Correct Answer: C

Repeated leakage often means the appliance is not fitted correctly for the current stoma size or abdominal contours. Reassessment is the key step. Making the opening larger increases skin exposure to effluent, extra tape does not fix a poor seal, and moisturizer interferes with adhesion.

3905. A postoperative client with an ileostomy reports crampy abdominal pain and nausea. The nurse notes minimal ostomy output, abdominal distention, and a swollen stoma. Which action is the priority?

- A. Encourage the client to increase high-fiber foods
- B. Notify the provider of possible obstruction
- C. Irrigate the ileostomy with tap water
- D. Reassure the client that output commonly stops for a day

Correct Answer: B

Cramping, nausea, distention, and minimal output suggest possible obstruction, which requires prompt provider notification. Increasing fiber could worsen the problem, irrigation of an ileostomy is not appropriate, and reassurance alone is unsafe.

3906. Which client is at greatest risk for fluid and electrolyte imbalance related to ostomy losses?

- A. A client with a new ascending colostomy producing semiliquid stool
- B. A client with a longstanding sigmoid colostomy and formed stool
- C. A client with a new ileostomy and continuous liquid effluent
- D. A client with a continent urinary diversion drained every 4 hours

Correct Answer: C

An ileostomy can produce frequent liquid output, placing the client at higher risk for dehydration and electrolyte loss. A sigmoid colostomy usually has more formed stool and lower fluid loss. The ascending colostomy has some risk, but continuous liquid ileostomy output is the greatest concern here.

3907. A nurse is teaching a client with a colostomy about foods that may increase gas and odor. Which client statement indicates a need for further teaching?

- A.** I may want to try new foods one at a time to see how I respond
- B.** If a food bothers me, I can avoid it before important social events
- C.** Any gas or odor means my colostomy is infected
- D.** What affects one person may not affect another person the same way

Correct Answer: C

Gas and odor are commonly influenced by diet and do not automatically mean infection. The other statements reflect appropriate self-management: introducing foods gradually, identifying personal triggers, and recognizing that responses vary among individuals.

3908. A nurse is assessing a newly created stoma 24 hours after surgery. Which appearance is most consistent with adequate perfusion?

- A.** Pale pink and dry
- B.** Beefy red and moist
- C.** Dark purple with a cool surface
- D.** Brownish-black with no drainage

Correct Answer: B

A healthy stoma should be moist and pink to beefy red, which indicates good blood supply. Pale, dusky, purple, or black coloration can suggest impaired perfusion or necrosis and requires prompt evaluation. Choice A may sound partly normal because pink is reassuring, but dryness is not expected for healthy stoma tissue.

3909. A nurse is preparing to change a client's ostomy appliance. The stoma is round and measures 28 mm. Which action is most appropriate when preparing the skin barrier opening?

- A.** Cut the opening exactly 28 mm so the barrier fits tightly against the stoma
- B.** Cut the opening about 1/8 inch larger than the stoma
- C.** Cut the opening 1/2 inch larger than the stoma to avoid pressure on the tissue
- D.** Leave the factory opening unchanged to avoid trauma from trimming

Correct Answer: B

The barrier opening is generally cut about 1/8 inch larger than the stoma so it does not constrict the stoma while still protecting surrounding skin from drainage. An opening that is too tight can injure the stoma, and one that is too large exposes skin to effluent. Choice A is a trap because a very snug fit sounds protective but may cause pressure and trauma.

3910. A client with a transverse colostomy is embarrassed by frequent odor from the pouch. Which nursing suggestion is most appropriate?

- A.** Avoid drinking fluids with meals
- B.** Keep the pouch sealed and empty it regularly before it becomes too full
- C.** Leave a small opening in the pouch to release gas slowly
- D.** Irrigate the ostomy after every meal

Correct Answer: B

A securely sealed pouch that is emptied regularly helps control odor and reduces leakage. Deliberately leaving the pouch open would worsen odor and increase contamination risk. Routine irrigation is not appropriate for every ostomy and should not be suggested broadly. Choice C may seem practical to a client worried about gas, but it defeats the purpose of the closed pouch system.

3911. A nurse assesses four clients with ostomies. Which finding should the nurse report to the provider immediately?

- A.** A moist red stoma with a small amount of mucus

- B.** A client who asks for help learning to empty the pouch
- C.** Peristomal skin that is mildly pink after appliance removal and returns to baseline quickly
- D.** A stoma that was red earlier but is now dusky and bluish

Correct Answer: D

A dusky or bluish stoma suggests impaired circulation and requires immediate follow-up. A moist red stoma and a small amount of mucus are expected findings, and brief mild pinkness of the surrounding skin can occur with adhesive removal if it resolves quickly. Choice A is a trap because mucus may seem abnormal, but some mucus is expected from bowel tissue.

3912. The nurse is caring for a client with an ileostomy who reports weakness and dizziness. Which additional finding most strongly supports concern for dehydration related to ostomy losses?

- A.** Bounding pulse and moist mucous membranes
- B.** Neck vein distention when sitting upright
- C.** Dry mouth and decreased urine output
- D.** Bradycardia and weight gain

Correct Answer: C

Dry mouth and decreased urine output are classic findings consistent with fluid volume deficit, which is a major concern with excessive ileostomy losses. The other options suggest fluid excess or do not fit dehydration. Choice A is a trap because weakness and dizziness may prompt a circulatory focus, but bounding pulse and moist mucous membranes do not support dehydration.

3913. A client returns from surgery after creation of an ostomy. During the first postoperative day, which nursing action is most important to include in care?

- A.** Encourage the client to select a permanent pouch brand immediately
- B.** Inspect the stoma regularly for color, moisture, and swelling
- C.** Begin irrigation as soon as bowel sounds are present
- D.** Keep the pouch off for several hours each shift to let the skin air dry

Correct Answer: B

Early postoperative ostomy care centers on frequent assessment of stoma viability, including color, moisture, and edema. Immediate product selection is not the priority, routine irrigation is not begun simply because bowel sounds are present, and leaving the pouch off for long periods exposes skin to effluent. Choice D is a common misconception because 'airing out' skin sounds helpful, but for ostomy care it increases skin injury risk.

3914. A nurse is teaching a client how to empty an ostomy pouch. Which sequence is best?

- A.** Remove the entire pouch, cleanse the stoma, then discard stool
- B.** Open the tail closure, drain the contents into the toilet, clean the outlet, and reclose the pouch
- C.** Cut a hole in the side of the pouch, drain the stool, and cover the opening with tape
- D.** Detach the skin barrier from the abdomen each time stool collects in the pouch

Correct Answer: B

A drainable pouch is emptied through the tail closure, then the outlet is cleaned and securely reclosed. Removing the whole pouch repeatedly or altering it by cutting extra openings increases leakage and skin injury risk. Choice A is a trap because it sounds thorough, but routine emptying does not require complete appliance removal.

3915. A nurse is changing an ostomy appliance for a client whose abdomen has deep skin folds near the stoma. Despite correct sizing, the seal frequently loosens. Which intervention is most appropriate?

- A.** Apply the pouching system over moist skin so it molds better
- B.** Use products or techniques that help create a flatter pouching surface around the stoma
- C.** Cut the barrier much larger than the stoma to prevent pressure over the folds
- D.** Reinforce the pouch by wrapping a thick gauze dressing around the entire abdomen

Correct Answer: B

When the abdomen is uneven, the goal is to improve the pouching surface so the barrier can seal securely and protect the skin. Moist skin weakens adhesion, and making the opening too large increases skin exposure. Choice B reflects problem-solving for pouch fit rather than masking the issue. Choice C is a trap because it may seem gentler, but it usually worsens leakage.

3916. A client with a new ileostomy says, "I've had almost no output for hours, and now I feel bloated and nauseated." Which interpretation is most concerning?

- A. The client is adapting normally to the ostomy and should rest
- B. The client may have an obstruction or impaired outflow that needs prompt evaluation
- C. The pouch is probably too small and should be emptied again later
- D. This is expected after any meal high in fiber and requires no follow-up

Correct Answer: B

Minimal output accompanied by bloating and nausea is concerning for obstruction or impaired ostomy outflow and requires prompt assessment. This is not a reassuring adaptation pattern. Choice A is a trap because some postoperative changes can be expected, but the combination of symptoms points to a possible complication rather than routine adjustment.

3917. The nurse is caring for a client with a retracted stoma and recurrent peristomal skin breakdown. Which outcome best shows that the current plan of care is effective?

- A. The stoma remains dry and pale, but the pouch stays attached for 24 hours
- B. The client reports less pain, the surrounding skin is intact, and effluent is contained within the pouching system
- C. The client needs more frequent pouch changes because drainage is collecting under the barrier
- D. The barrier opening is widened enough that no pressure touches the stoma at all

Correct Answer: B

Effective ostomy care should protect peristomal skin, contain drainage, and improve comfort. A pouch staying on is not enough if the stoma looks poorly perfused or the skin is exposed to effluent. Choice B reflects the actual goals of care. Choice A is a trap because adhesion alone does not mean the stoma is healthy.

3918. A nurse is reinforcing teaching for a client with a newly created descending colostomy who will perform colostomy irrigation at home. Which client statement indicates correct understanding of the procedure?

- A. "I should irrigate only when I have abdominal cramping and no stool output."
- B. "I will use sterile technique each time to prevent infection."
- C. "I should try to irrigate at about the same time of day to help establish a pattern."
- D. "I will stop taking fluids for several hours before irrigation so the bowel stays quiet."

Correct Answer: C

Regular timing helps promote a predictable emptying pattern for some clients with a descending colostomy. Irrigation is not done only when output stops, because that could delay evaluation of a possible problem. Sterile technique is not typically required for routine home ostomy irrigation; clean technique is used. Restricting fluids is not an appropriate teaching point and may contribute to dehydration.

3919. The nurse is caring for a client with a new ileostomy. Which observation requires the most immediate follow-up?

- A. The pouch contains liquid green-brown drainage
- B. The stoma protrudes slightly above the skin surface
- C. The peristomal skin is reddened and weeping under the barrier
- D. The client reports seeing a small amount of mucus in the pouch

Correct Answer: C

Reddened, weeping peristomal skin suggests ongoing skin breakdown from leakage or contact with effluent and needs prompt attention to prevent worsening injury and poor pouch adherence. Liquid drainage is expected with an ileostomy. A slightly protruding stoma is generally a favorable finding. A small amount of mucus can be expected from intestinal tissue and is not the priority here.

Recognizing changes in condition

3920. A nurse is caring for a client 12 hours after abdominal surgery. Which finding requires immediate follow-up?

- A. Urine output of 25 mL over the last hour
- B. Temperature of 37.4°C (99.3°F)
- C. Pain rating of 6 on a 0-to-10 scale after ambulation
- D. Absent bowel sounds in all quadrants

Correct Answer: A

Urine output of 25 mL/hr suggests possible decreased renal perfusion or hypovolemia and requires prompt assessment. The other findings can be expected early after abdominal surgery: a mild temperature elevation may occur, pain after activity is common, and bowel sounds are often decreased temporarily after surgery.

3921. A client with heart failure tells the nurse, "I had to sleep sitting up last night because I felt like I couldn't breathe." Which additional finding most strongly supports worsening fluid overload?

- A. Weight gain of 2.2 kg (4.8 lb) in 2 days
- B. Dry oral mucous membranes
- C. Blood pressure of 108/64 mm Hg
- D. Urine specific gravity of 1.030

Correct Answer: A

Rapid weight gain over 1 to 2 days strongly suggests fluid retention in a client with heart failure, especially with orthopnea. Dry mucous membranes and concentrated urine point more toward dehydration. The blood pressure is not by itself the strongest indicator of worsening fluid overload.

3922. A nurse is monitoring a client receiving opioid pain medication after surgery. Which change in status is most concerning?

- A. Respiratory rate decreases from 18/min to 10/min
- B. Client reports nausea after eating crackers
- C. Pulse increases from 72/min to 86/min
- D. Client requests a dimmer room

Correct Answer: A

A respiratory rate of 10/min after opioid administration suggests possible respiratory depression, a serious adverse effect requiring immediate assessment and intervention. Nausea and drowsiness can occur with opioids but are less urgent than hypoventilation.

3923. A client with diabetes mellitus says, "My shoes feel tighter than usual." Which assessment finding is the nurse's priority to evaluate next?

- A. Bilateral ankle swelling
- B. Fasting blood glucose of 162 mg/dL
- C. Report of increased appetite
- D. Dry skin on the elbows

Correct Answer: A

New ankle swelling may reflect fluid retention or cardiovascular or renal complications and should be evaluated promptly as a possible change in condition. The glucose level is elevated but not the most urgent cue here. Increased appetite and dry elbows are less immediate concerns.

3924. A nurse is caring for a client with pneumonia. Which assessment finding best indicates that the client's condition is worsening?

- A. The client asks for assistance turning in bed
- B. The client has coarse crackles in the bases
- C. The client becomes newly confused and restless
- D. The client reports pleuritic chest discomfort when coughing

Correct Answer: C

New confusion and restlessness can signal worsening hypoxia and are important early signs of deterioration. Crackles and pleuritic pain are common with pneumonia but are not by themselves the clearest sign of worsening status. Needing help turning is nonspecific.

3925. A client had a cast applied to the lower leg 2 hours ago. Which statement by the client requires the most urgent action by the nurse?

- A. "My toes feel a little itchy."
- B. "The cast feels warm while it is drying."
- C. "I can move my toes, but they are getting more numb."
- D. "My leg feels heavy when I lift it."

Correct Answer: C

Increasing numbness after cast application may indicate impaired circulation or nerve compression and requires immediate assessment. Warmth while the cast dries is expected. Itching and a heavy sensation are less urgent than a neurovascular change.

3926. A nurse is caring for a client with a chest tube connected to a drainage system after a pneumothorax. Which finding suggests the client may be developing a complication?

- A. Tidaling is present in the water-seal chamber
- B. The client reports increased shortness of breath
- C. A small amount of drainage is in the tubing
- D. The dressing is dry and intact

Correct Answer: B

Increased shortness of breath may indicate a worsening pneumothorax, obstruction, or other complication and requires prompt evaluation. Tidaling can be expected depending on the situation. Small drainage in the tubing and an intact dressing are not concerning by themselves.

3927. A client with chronic kidney disease returns from dialysis. Which assessment finding should the nurse report first?

- A. Muscle cramps in the calves
- B. Blood pressure 88/52 mm Hg
- C. Mild fatigue
- D. Thirst and dry lips

Correct Answer: B

Hypotension after dialysis can indicate excessive fluid removal or hemodynamic instability and should be reported immediately. Muscle cramps and fatigue may occur after dialysis, and thirst or dry lips are common but less urgent than a blood pressure of 88/52 mm Hg.

3928. A nurse is caring for an older adult with a urinary tract infection. Which change is most important to recognize as a possible sign of worsening condition?

- A. The client refuses breakfast
- B. The client becomes acutely disoriented to place
- C. The client reports burning with urination
- D. The client requests an extra blanket

Correct Answer: B

Acute disorientation in an older adult can indicate worsening infection, delirium, or sepsis and requires prompt assessment. Burning with urination is an expected symptom of a urinary tract infection. Refusing breakfast and asking for a blanket are less specific cues.

3929. A client with a tracheostomy has thick secretions. Which finding indicates the client is not tolerating the current respiratory status?

- A. The client coughs during suctioning
- B. The client writes notes to communicate needs
- C. The client has increasing anxiety and use of accessory muscles
- D. The client requests oral care every 2 hours

Correct Answer: C

Increasing anxiety and accessory muscle use are signs of respiratory distress and may signal inadequate oxygenation or airway obstruction. Coughing during suctioning is expected. Writing to communicate and requesting oral care do not indicate worsening respiratory status.

3930. A nurse is assessing four clients. Which client demonstrates the most concerning change from baseline?

- A. A client with COPD whose oxygen saturation is 91%, unchanged from earlier
- B. A client with a healing wound who reports mild itching at the incision

C. A client with hypertension whose blood pressure is 146/88 mm Hg before a scheduled medication

D. A postoperative client who is difficult to arouse and answers questions slowly

Correct Answer: D

A postoperative client who is difficult to arouse may be experiencing oversedation, hypoxia, or another serious complication. That change is more urgent than the other findings, which are either expected, unchanged from baseline, or less acute. The trap is choosing the elevated blood pressure, but it is not as immediately dangerous as decreased responsiveness.

3931. A client 1 day after bowel surgery has a nasogastric tube to low suction. Which finding most strongly suggests a change in condition that should be addressed immediately?

A. Green drainage is present in the collection canister

B. The client reports a sore throat

C. The abdomen is more distended than 4 hours ago and the client is nauseated

D. The client asks when oral fluids can be restarted

Correct Answer: C

Increasing abdominal distention with nausea may indicate worsening bowel dysfunction, obstruction, or ineffective gastric decompression and requires prompt assessment. Green drainage and sore throat are common with an NG tube. Asking about oral fluids is not a sign of deterioration.

3932. A nurse is caring for a client receiving IV fluids for dehydration. Which combination of findings should alert the nurse to possible fluid volume overload?

A. Flat neck veins, weak pulse, and dry skin

B. Crackles in the lungs, rising blood pressure, and new pedal edema

C. Tachycardia, poor skin turgor, and concentrated urine

D. Cool extremities, delayed capillary refill, and thirst

Correct Answer: B

Crackles, increasing blood pressure, and new edema together point to fluid volume overload, especially during IV fluid therapy. The other options describe dehydration or poor perfusion rather than excess fluid. This item requires linking multiple cues rather than relying on one finding.

3933. A client with a history of gastrointestinal bleeding is admitted for weakness. Which finding is the clearest sign that the client may be actively deteriorating?

A. Black, tarry stool and heart rate of 118/min

B. Poor appetite for 3 days

C. Epigastric discomfort after meals

D. Hemoglobin of 10.8 g/dL from last week's clinic visit

Correct Answer: A

Melena with tachycardia suggests ongoing blood loss and possible hemodynamic compromise. Poor appetite and epigastric discomfort are nonspecific. A prior hemoglobin from last week does not establish the client's current stability. The key is recognizing current evidence of active bleeding plus physiologic response.

3934. A client with sepsis is being monitored on a medical unit. Which assessment trend is most concerning for progression toward shock?

A. Temperature decreases from 39.2°C (102.6°F) to 38.3°C (100.9°F) after acetaminophen

B. Heart rate decreases from 112/min to 96/min after rest

C. Blood pressure drops from 104/68 mm Hg to 88/50 mm Hg and urine output declines

D. The client reports generalized muscle aches and fatigue

Correct Answer: C

A falling blood pressure combined with declining urine output suggests worsening perfusion and possible progression to septic shock. The temperature drop after acetaminophen is expected treatment response. Muscle aches and fatigue are nonspecific. This question focuses on recognizing a dangerous trend rather than a single isolated value.

3935. A nurse is caring for a client 24 hours after a thyroidectomy. Which finding is the priority to report immediately?

- A. Hoarse voice and increasing restlessness
- B. Pain rated 6 out of 10 at the incision site
- C. Small amount of serosanguineous drainage on the dressing
- D. Sore throat when swallowing

Correct Answer: A

Hoarseness with increasing restlessness can signal airway compromise from swelling or bleeding after thyroid surgery and requires immediate follow-up. Incisional pain, mild drainage, and a sore throat are common postoperative findings unless accompanied by signs of respiratory distress.

3936. A client with chronic obstructive pulmonary disease has been restless and anxious for the past hour. Which additional finding best suggests the client's respiratory status is worsening?

- A. Oxygen saturation increases from 91% to 92%
- B. The client begins using accessory muscles to breathe
- C. The client asks to sit in a chair for comfort
- D. The client reports a dry mouth after oxygen therapy

Correct Answer: B

Accessory muscle use is a key sign of increased work of breathing and worsening respiratory distress. Restlessness and anxiety can also be early signs of hypoxemia. A 1% increase in oxygen saturation is not concerning, and wanting to sit up or having a dry mouth does not by itself indicate deterioration.

3937. A nurse is caring for a client with a newly created ileostomy. Which finding requires the most immediate action?

- A. The stoma appears pink and moist
- B. The client reports mild cramping after ambulation
- C. The stoma appears dusky and bluish
- D. Liquid stool is present in the pouch

Correct Answer: C

A dusky or bluish stoma suggests impaired perfusion, which can indicate ischemia and requires immediate intervention. A pink, moist stoma is expected. Liquid output is typical with an ileostomy, and mild cramping may occur with activity or bowel adjustment.

3938. A nurse is assessing a client with a pressure injury. Which change most strongly suggests the wound may be infected?

- A. Beefy red granulation tissue in the wound bed
- B. Increasing purulent drainage with foul odor
- C. Scant serous drainage during dressing change
- D. Mild tenderness when the area is cleansed

Correct Answer: B

Purulent drainage and foul odor are classic signs of wound infection and indicate a change in condition that needs follow-up. Granulation tissue suggests healing. Small amounts of serous drainage and mild tenderness during care may occur without infection.

3939. A client hospitalized with pancreatitis has been receiving IV fluids and pain medication. Which assessment finding is the clearest sign that the client may be developing hypovolemic shock?

- A. Urine output 20 mL over 2 hours and cool, clammy skin
- B. Abdominal pain rated 8 out of 10 after coughing
- C. Nausea with one episode of emesis after lunch
- D. Diminished bowel sounds in all quadrants

Correct Answer: A

Low urine output with cool, clammy skin strongly suggests poor tissue perfusion and possible hypovolemic shock. Pain, nausea, and decreased bowel sounds can occur with pancreatitis but are less specific for life-threatening circulatory decline.

3940. A nurse is caring for a client receiving a blood transfusion. Fifteen minutes after the transfusion begins, the client reports chills and low back pain. What is the nurse's priority interpretation of these findings?

- A. Expected response to rapid infusion
- B. Possible acute transfusion reaction
- C. Manifestation of anxiety about the procedure

D. Evidence of improving circulation**Correct Answer: B**

Chills and low back pain shortly after a transfusion begins are concerning for an acute transfusion reaction and require immediate action. The trap is dismissing the symptoms as anxiety or discomfort. These findings are not expected and do not indicate improvement.

3941. A client with a history of heart disease is attached to a cardiac monitor. Which change should the nurse recognize as the most concerning indicator of decreased cardiac output?

- A. New confusion and blood pressure of 84/50 mm Hg
- B. Heart rate of 88/min after morning care
- C. Occasional premature atrial beats without symptoms
- D. Mild fatigue after walking to the bathroom

Correct Answer: A

New confusion combined with hypotension suggests poor cerebral and systemic perfusion, making decreased cardiac output the priority concern. Mild fatigue, a heart rate of 88/min, and occasional asymptomatic atrial ectopy are less urgent findings.

3942. A nurse is caring for a client with a high fever from an infection. Which new finding best indicates that the client's thermoregulation problem is becoming more serious?

- A. Warm skin and thirst
- B. Profuse sweating after acetaminophen administration
- C. Confusion and new hypotension
- D. Flushed face and complaint of headache

Correct Answer: C

Confusion with new hypotension suggests systemic deterioration and possible progression to severe infection or shock, not just an uncomplicated fever. Warm skin, thirst, flushing, and headache can accompany fever, and sweating after an antipyretic may indicate the temperature is beginning to fall.

3943. A client with a tracheostomy is receiving humidified oxygen. Which finding should the nurse recognize as the earliest sign that the client may be experiencing airway obstruction?

- A. Sudden decrease in heart rate
- B. Restlessness with noisy respirations
- C. Blood pressure of 138/82 mm Hg
- D. Pulse oximetry reading of 96%

Correct Answer: B

Restlessness and noisy respirations are early warning signs of airway obstruction and require prompt assessment. Bradycardia is more often a late sign of severe hypoxia. A normal blood pressure and oxygen saturation do not rule out early obstruction, especially if the client is showing clinical distress.

3944. A nurse is evaluating a client with a closed-suction wound drain following hip surgery. Which finding most strongly suggests a change in condition requiring prompt follow-up?

- A. Drainage changes from light pink to bright red and increases suddenly
- B. The drain contains 25 mL of serosanguineous fluid over 8 hours
- C. The client reports soreness when repositioned
- D. The dressing has a small dry stain from earlier drainage

Correct Answer: A

A sudden increase in bright red drainage suggests active bleeding and requires prompt follow-up. Moderate serosanguineous drainage is often expected after surgery, and soreness with movement or an old dry stain is less concerning.

3945. A nurse is caring for a client receiving continuous tube feedings. Which assessment finding best indicates aspiration may have occurred and the client's condition is worsening?

- A. Abdominal distention and hypoactive bowel sounds
- B. Frequent loose stools during the night
- C. New cough, coarse crackles, and decreased oxygen saturation

D. Residual feeding volume greater than the previous check

Correct Answer: C

A new cough, coarse crackles, and a drop in oxygen saturation point to possible aspiration affecting the lungs, which is a serious change in condition. Distention, loose stools, and increased residuals may indicate feeding intolerance but are less immediately dangerous than signs of respiratory compromise.

3946. A client admitted with vomiting and diarrhea has been increasingly weak. Which new assessment finding most strongly suggests worsening potassium imbalance that needs immediate attention?

- A.** Muscle weakness with an irregular pulse
- B.** Dry mucous membranes and thirst
- C.** Urine specific gravity of 1.030
- D.** Blood pressure 118/74 mm Hg when lying down

Correct Answer: A

Muscle weakness with an irregular pulse is especially concerning because potassium imbalance can lead to dangerous cardiac dysrhythmias. Dry mucous membranes and concentrated urine suggest dehydration, but they are not as immediately threatening as signs of possible electrical instability in the heart.

Fluid and electrolyte imbalance

3947. A nurse is caring for a client who has had several episodes of vomiting and now reports weakness and muscle cramps. Which electrolyte imbalance should the nurse suspect first?

- A.** Hypernatremia
- B.** Hypokalemia
- C.** Hypercalcemia
- D.** Hypermagnesemia

Correct Answer: B

Vomiting can lead to potassium loss, and hypokalemia commonly causes weakness, muscle cramps, and dysrhythmia risk. Hypernatremia is more associated with dehydration and neurologic changes. Hypercalcemia and hypermagnesemia do not fit this loss pattern as well.

3948. A client with heart failure is receiving furosemide. Which assessment finding requires the nurse's immediate follow-up?

- A.** Irregular pulse and generalized weakness
- B.** Blood pressure of 132/78 mm Hg
- C.** Urine output of 900 mL over 8 hours
- D.** Complaint of increased thirst after breakfast

Correct Answer: A

Furosemide can cause potassium loss. An irregular pulse with weakness suggests possible hypokalemia and risk for cardiac dysrhythmias, which is the priority. The blood pressure is not alarming, the urine output is adequate, and thirst alone is less urgent than signs of a potentially dangerous electrolyte disturbance.

3949. The nurse reviews laboratory results for four clients. Which result should the nurse identify as most consistent with dehydration?

- A.** Hematocrit 53%
- B.** Hemoglobin 11 g/dL
- C.** White blood cell count 8,000/mm³
- D.** Platelet count 250,000/mm³

Correct Answer: A

Dehydration can cause hemoconcentration, which raises the hematocrit. The other values are not the best indicator of fluid volume deficit in this set. A common trap is focusing on a low hemoglobin, but that suggests anemia rather than dehydration.

3950. A postoperative client has a nasogastric tube to low intermittent suction. Which finding is most important for the nurse to report?

- A.** Absent bowel sounds for 6 hours after surgery
- B.** Potassium level of 3.0 mEq/L
- C.** Urine output of 40 mL/hr
- D.** Mild throat irritation

Correct Answer: B

Gastric suction can remove potassium-rich fluids and place the client at risk for hypokalemia. A potassium level of 3.0 mEq/L is abnormal and can lead to muscle weakness and cardiac dysrhythmias. The other findings are either expected early after surgery or less urgent.

3951. A nurse is assessing a client with fluid volume excess. Which finding is most consistent with this problem?

- A. Flat neck veins when sitting upright
- B. Dry mucous membranes
- C. Crackles in the lungs
- D. Poor skin turgor

Correct Answer: C

Crackles suggest fluid accumulation in the lungs and are a classic finding in fluid volume excess. Flat neck veins, dry mucous membranes, and poor skin turgor are more consistent with fluid volume deficit.

3952. The nurse is caring for a client with syndrome of inappropriate antidiuretic hormone secretion (SIADH). Which provider prescription should the nurse expect?

- A. Encourage oral free water intake
- B. Restrict fluids
- C. Administer hypotonic IV solution
- D. Increase sodium restriction

Correct Answer: B

SIADH causes water retention and dilutional hyponatremia, so fluid restriction is a common treatment approach. Encouraging free water or giving hypotonic fluids would worsen water excess. Increasing sodium restriction would not correct the underlying dilution problem.

3953. A client with severe diarrhea is at risk for metabolic acidosis. Which assessment finding best supports this concern?

- A. Slow, shallow respirations
- B. Bounding pulse
- C. Deep, rapid respirations
- D. Warm, flushed skin only

Correct Answer: C

Loss of bicarbonate through diarrhea can cause metabolic acidosis. The body compensates with deep, rapid respirations to blow off carbon dioxide. Slow, shallow respirations would suggest the opposite pattern. Bounding pulse and flushed skin are not the best indicators here.

3954. A nurse is preparing to administer IV potassium chloride to a client with hypokalemia. Which action is appropriate?

- A. Give the medication by IV push for rapid correction
- B. Add the potassium to the IV solution and infuse with a pump
- C. Mix the potassium with dextrose and infuse as fast as tolerated
- D. Administer the potassium only if the client reports leg cramps

Correct Answer: B

IV potassium should be diluted and administered using a controlled infusion device. IV push potassium is unsafe because it can cause life-threatening cardiac complications. The decision to give potassium is based on clinical need and laboratory findings, not only symptoms such as leg cramps.

3955. A client has a sodium level of 124 mEq/L. Which finding should the nurse expect?

- A. Confusion and headache
- B. Extreme thirst and dry tongue
- C. Bradycardia with hypertension only
- D. Hyperactive bowel sounds with abdominal cramping only

Correct Answer: A

A sodium level of 124 mEq/L indicates hyponatremia, which commonly causes neurologic changes such as confusion, headache, lethargy, and, in severe cases, seizures. Extreme thirst is more associated with hypernatremia. The other options are not the most typical pattern.

3956. The nurse is assessing an older adult with poor oral intake for 3 days. Which finding most strongly suggests fluid volume deficit rather than a normal age-related change?

- A. Reduced skin elasticity over the sternum
- B. Dry oral mucous membranes and concentrated urine
- C. Mildly decreased muscle mass
- D. Fine facial wrinkles

Correct Answer: B

In older adults, skin turgor can be less reliable because aging itself reduces elasticity. Dry mucous membranes plus concentrated urine more strongly support fluid volume deficit. The other findings may occur with normal aging and are less specific.

3957. A client with chronic kidney disease has a potassium level of 6.2 mEq/L. Which assessment finding is most important for the nurse to obtain immediately?

- A. Bowel movement pattern
- B. Apical heart rhythm
- C. Visual acuity
- D. Capillary refill in the toes

Correct Answer: B

A potassium level of 6.2 mEq/L indicates hyperkalemia, which can cause dangerous cardiac conduction abnormalities. Assessing the apical heart rhythm is the priority. Bowel pattern may be relevant later, but the immediate risk is dysrhythmia.

3958. A client with fluid volume excess is prescribed a 2-gram sodium diet. Which statement by the client indicates a need for further teaching?

- A. I should check food labels for sodium content.
- B. Processed meats may be high in sodium.
- C. I can season food with lemon juice instead of salt.
- D. Canned soup is usually a good low-sodium choice.

Correct Answer: D

Many canned soups are high in sodium and are often restricted on a low-sodium diet. The other statements show correct understanding. A common trap is assuming soup is healthy overall, but sodium content is the key issue here.

3959. A nurse is caring for a client receiving continuous enteral feedings. Which intervention is most appropriate to help evaluate fluid balance?

- A. Weigh the client at the same time each day
- B. Check pupillary response every 8 hours
- C. Limit oral hygiene to reduce aspiration risk
- D. Measure blood pressure only when the client reports dizziness

Correct Answer: A

Daily weights are one of the most reliable indicators of fluid balance. Pupillary checks do not directly assess hydration status, limiting oral hygiene is inappropriate, and blood pressure should be assessed routinely rather than only when symptoms occur.

3960. A client admitted with diabetic ketoacidosis is lethargic and has dry mucous membranes, tachycardia, and poor skin turgor. Which prescription should the nurse anticipate as the priority initial intervention?

- A. Restrict oral and IV fluids
- B. Begin isotonic IV fluid replacement
- C. Administer calcium gluconate
- D. Place the client in Trendelenburg position

Correct Answer: B

This client shows fluid volume deficit, which is a major problem in diabetic ketoacidosis. Isotonic IV fluids are a priority to restore circulating volume. Fluid restriction would worsen the deficit. Calcium gluconate is used for certain effects of hyperkalemia, and Trendelenburg is not the appropriate treatment for dehydration.

3961. The nurse is caring for a client after a large-volume blood loss. Which finding best indicates that fluid resuscitation has been effective?

- A. Urine output increases to 35 mL/hr
- B. Respiratory rate remains 28/min
- C. The client reports being tired
- D. Skin stays cool and pale

Correct Answer: A

Adequate urine output is a key sign of improved renal perfusion and effective fluid resuscitation. Continued tachypnea, fatigue, and cool pale skin do not indicate clear improvement. The most objective and clinically useful sign here is the urine output.

3962. A client who has been taking a thiazide diuretic reports new fatigue and constipation. Which laboratory value should the nurse review first as the most likely cause of these findings?

- A. Serum calcium 11.4 mg/dL
- B. Serum sodium 150 mEq/L
- C. Serum magnesium 1.9 mg/dL

D. Serum phosphorus 3.4 mg/dL

Correct Answer: A

A serum calcium of 11.4 mg/dL indicates hypercalcemia, which can cause fatigue, weakness, and constipation. Thiazide diuretics can decrease calcium excretion and contribute to elevated calcium levels. Hypernatremia may cause thirst and neurologic changes, but it is less directly linked to constipation in this scenario. The magnesium and phosphorus values listed are within expected ranges.

3963. A nurse is caring for a client receiving intravenous fluids after 24 hours of poor oral intake. Which finding best suggests the client is becoming fluid volume overloaded?

- A.** Flat neck veins when lying supine
- B.** Progressive crackles at the lung bases
- C.** Urine specific gravity of 1.032
- D.** Complaint of increased thirst

Correct Answer: B

Progressive crackles at the lung bases suggest fluid is backing up into the lungs and indicate fluid volume overload. Flat neck veins, high urine specific gravity, and thirst are more consistent with fluid volume deficit. The key is distinguishing a respiratory sign of excess fluid from common signs of dehydration.

3964. A client with pancreatitis develops tingling around the mouth and muscle twitching. Which additional finding would the nurse expect if these symptoms are caused by the same electrolyte imbalance?

- A.** Diminished deep tendon reflexes
- B.** Flaccid muscles and hypoactive bowel sounds
- C.** Positive Trousseau sign
- D.** Warm flushed skin

Correct Answer: C

Perioral tingling and muscle twitching suggest hypocalcemia. A positive Trousseau sign is a classic additional finding with low calcium. Diminished reflexes and flaccid muscles are more consistent with problems such as hypermagnesemia or hypokalemia. Warm flushed skin does not fit this electrolyte pattern.

3965. A client admitted with dehydration has an IV of 0.9% sodium chloride infusing at 125 mL/hr. Four hours later, which finding should the nurse identify as the best evidence that therapy is working?

- A.** Heart rate decreases from 118/min to 92/min
- B.** Urine becomes dark yellow
- C.** Hematocrit rises from 44% to 49%
- D.** Skin remains tented over the sternum

Correct Answer: A

A decrease in heart rate toward normal suggests improved circulating volume and better perfusion, making it a strong sign that fluid replacement is effective. Dark urine, rising hematocrit, and persistent skin tenting all suggest dehydration is still present. The best answer is the one showing clear improvement in hemodynamic status.

3966. A nurse is caring for a client with a serum magnesium level of 3.8 mg/dL. Which prescription should the nurse question?

- A.** Place the client on cardiac monitoring
- B.** Assess deep tendon reflexes every 4 hours
- C.** Administer magnesium hydroxide 30 mL by mouth at bedtime
- D.** Monitor respiratory rate and effort

Correct Answer: C

A magnesium level of 3.8 mg/dL is elevated, so giving magnesium hydroxide could worsen hypermagnesemia and should be questioned. Cardiac monitoring, reflex checks, and respiratory assessment are appropriate because elevated magnesium can depress neuromuscular and cardiac function. The trap is overlooking magnesium-containing medications in a client who already has excess magnesium.

3967. A client receiving total parenteral nutrition has a sudden infusion pump failure, and the solution has been stopped for 45 minutes while a replacement pump is obtained. Which assessment finding is the nurse's priority to monitor during this interruption?

- A.** Bounding peripheral pulses
- B.** Symptomatic hypoglycemia
- C.** Elevated serum calcium
- D.** Dependent edema

Correct Answer: B

Abrupt interruption of total parenteral nutrition can cause hypoglycemia because the client has been receiving a continuous high-glucose infusion. Monitoring for sweating, shakiness, confusion, or other signs of low blood glucose is the priority. Bounding pulses and edema suggest fluid volume issues, and elevated calcium is not the immediate concern from a short TPN interruption.

3968. A nurse is reviewing arterial blood gas and electrolyte results for a client who has had prolonged gastric suctioning. Which set of results is most consistent with this condition?

- A. pH 7.49, bicarbonate 30 mEq/L, potassium 3.1 mEq/L
- B. pH 7.28, bicarbonate 18 mEq/L, potassium 5.6 mEq/L
- C. pH 7.40, bicarbonate 24 mEq/L, potassium 4.2 mEq/L
- D. pH 7.33, bicarbonate 20 mEq/L, potassium 3.0 mEq/L

Correct Answer: A

Prolonged gastric suctioning causes loss of hydrochloric acid, leading to metabolic alkalosis, and often causes hypokalemia as well. Option A shows alkalemia, elevated bicarbonate, and low potassium, which fit the expected pattern. The other options show acidosis or normal findings, which do not match this fluid and electrolyte problem.

3969. A client asks why the nurse is restricting free water intake after a sodium level of 128 mEq/L was reported. Which explanation by the nurse is best?

- A. "Limiting water can help keep your sodium from becoming more diluted."
- B. "Restricting water will lower your potassium and protect your heart."
- C. "This is done to increase calcium absorption in the intestines."
- D. "Less water will help your body produce more red blood cells."

Correct Answer: A

In hyponatremia, excess free water can further dilute the sodium concentration. Restricting water may help prevent worsening of the low sodium level. The other options do not explain the purpose of fluid restriction in relation to sodium balance and are unrelated to the problem presented.

3970. A nurse is caring for a client with burns over a large body surface area during the first 24 hours after injury. Which finding most clearly indicates worsening fluid volume deficit?

- A. Urine output of 18 mL over 1 hour
- B. Blood pressure of 128/76 mm Hg
- C. Capillary refill of 2 seconds
- D. Respiratory rate of 18/min

Correct Answer: A

An hourly urine output of 18 mL is low and suggests inadequate renal perfusion and worsening fluid volume deficit. In burn clients, urine output is a key indicator of effective circulating volume. The other findings are not as concerning or may still be within expected limits. The most urgent cue is the drop in output.

3971. A client with chronic alcoholism is admitted for weakness. The nurse notes tremors and a serum magnesium level of 1.1 mg/dL. Which electrolyte imbalance should the nurse also be most alert for because it commonly occurs with this problem?

- A. Hypernatremia
- B. Hyperphosphatemia
- C. Hypokalemia
- D. Hypercalcemia

Correct Answer: C

Hypomagnesemia often occurs along with hypokalemia, and low magnesium can make potassium replacement more difficult until the magnesium deficit is corrected. Hypernatremia, hyperphosphatemia, and hypercalcemia are not the most closely associated imbalances in this setting. The key connection is the frequent coexistence of low magnesium and low potassium.

3972. A client with fluid volume deficit is receiving oral rehydration. Which statement by the client shows the best understanding of how to help correct the imbalance?

- A. "I should drink only plain water until I feel normal again."
- B. "I will take small, frequent amounts of fluid instead of waiting until I am very thirsty."
- C. "I should avoid all fluids with sodium because salt causes swelling."
- D. "I will limit fluids so my stomach can rest for the rest of the day."

Correct Answer: B

Taking small, frequent amounts of fluid is an effective approach to oral rehydration, especially if the client has had recent fluid losses or mild nausea. Drinking only plain water may not replace needed electrolytes, and avoiding all sodium or limiting fluids further would not support correction of fluid volume deficit. This answer reflects practical understanding of rehydration.

3973. A client is receiving packed red blood cells and suddenly develops dyspnea, crackles, and distended neck veins. Which action should the nurse take first?

- A. Slow the transfusion rate and continue to monitor
- B. Stop the transfusion and keep the vein open with appropriate IV fluid
- C. Place the client flat in bed
- D. Obtain a urine specimen for specific gravity

Correct Answer: B

Dyspnea, crackles, and neck vein distention during a transfusion suggest circulatory overload or another serious transfusion reaction. The nurse should stop the transfusion immediately and maintain IV access according to facility protocol. Simply slowing the rate is not enough when significant symptoms are present. Lying flat could worsen breathing, and urine specific gravity is not the priority.

Dialysis

3974. A client with end-stage kidney disease returns from hemodialysis reporting a headache, nausea, and increasing restlessness. Which action should the practical nurse take first?

- A. Check the client's vital signs and neurologic status
- B. Offer the client a high-protein snack
- C. Place the client flat in bed for 2 hours
- D. Encourage the client to drink fluids

Correct Answer: A

Headache, nausea, and restlessness after hemodialysis can signal complications such as hypotension or dialysis disequilibrium syndrome. The first action is to assess vital signs and neurologic status so the nurse can recognize severity and report promptly. Offering food or fluids does not address the possible complication, and placing the client flat without assessment delays needed evaluation.

3975. Which finding in a client receiving hemodialysis requires immediate reporting to the nurse in charge or provider?

- A. Bruit is absent over the arteriovenous fistula
- B. Mild fatigue after the dialysis session
- C. Small bruise near the access site from a prior treatment
- D. Reduced appetite on the day of treatment

Correct Answer: A

An absent bruit over an arteriovenous fistula suggests possible clotting or loss of access patency and requires immediate reporting. Mild fatigue and reduced appetite can occur with chronic illness or after treatment, and a small old bruise is less urgent. The key trap is choosing a common but nonemergent symptom instead of the threatened dialysis access.

3976. The nurse is preparing to take a blood pressure on a client who has an arteriovenous fistula in the left forearm for hemodialysis. What is the best action?

- A. Use the left arm because the fistula improves circulation
- B. Use either arm, as long as the cuff is inflated slowly

- C. Avoid the left arm and use the opposite arm if available
- D. Take the blood pressure on the fistula arm only before dialysis

Correct Answer: C

Blood pressure measurements should be avoided on the arm with the fistula because cuff compression can damage the access. The best choice is to use the opposite arm if available. A common trap is thinking timing or slower inflation makes it safe, but the issue is protecting the access itself.

- 3977.** A client receiving peritoneal dialysis has drainage fluid that is cloudy. Which interpretation is most appropriate?
- A. The dialysate likely contains normal sediment from filtration
 - B. This may indicate peritonitis and should be reported promptly
 - C. The client is absorbing too much glucose from the solution
 - D. This shows the catheter is positioned correctly

Correct Answer: B

Cloudy peritoneal drainage is a classic warning sign of peritonitis and should be reported promptly. Normal outflow should be clear to light yellow. The trap is dismissing the finding as expected after filtration, but cloudy effluent is not considered a routine finding.

- 3978.** A client is scheduled for hemodialysis this morning. Which medication order should the nurse question before the client leaves the unit?
- A. Renal vitamin supplement
 - B. Stool softener
 - C. Antihypertensive medication
 - D. Topical skin moisturizer

Correct Answer: C

Antihypertensive medications are commonly questioned before hemodialysis because they can worsen intradialytic hypotension. The nurse should clarify the order rather than automatically give it. The other medications are less likely to create an immediate hemodynamic problem during treatment.

- 3979.** Which assessment finding best suggests that a client on chronic hemodialysis may be retaining fluid between treatments?

- A. Weight gain of 2.2 kg in 2 days
- B. Dry, flaky skin on the lower legs
- C. Pulse of 58 beats/min while resting
- D. Report of increased appetite

Correct Answer: A

A rapid weight gain between dialysis treatments strongly suggests fluid retention. Daily or short-term weight change is more useful than general observations such as dry skin or appetite. The trap is underestimating the importance of weight trends in dialysis clients.

- 3980.** A client has a new arteriovenous fistula for future hemodialysis. Which client statement shows correct understanding of care for the access?
- A. I should let blood be drawn from this arm to keep the vein open
 - B. I will check for a vibration over the site each day
 - C. I can wear a tight watch over the access if it feels comfortable
 - D. I should sleep with my body weight on that arm to protect it

Correct Answer: B

A palpable vibration, called a thrill, should be checked regularly because it suggests blood flow through the fistula. Blood draws, constrictive clothing or jewelry, and pressure on the access arm should be avoided. The distractors reflect actions that could damage the access.

- 3981.** During peritoneal dialysis, the nurse notes that outflow is much slower than expected. Which action is most appropriate first?
- A. Increase the flow by adding more solution to the abdomen
 - B. Reposition the client and check for tubing kinks
 - C. Clamp the catheter and leave it untouched for the shift
 - D. Document the finding as expected during early treatment

Correct Answer: B

Slow outflow during peritoneal dialysis is often related to mechanical issues such as tubing kinks or poor client positioning. Repositioning and checking the system are appropriate first actions. Adding more solution or ignoring the problem could worsen discomfort and does not address the likely cause.

3982. A client returns from hemodialysis with a blood pressure of 88/54 mm Hg, dizziness, and pallor. What is the nurse's priority action?

- A. Assist the client to ambulate to improve circulation
- B. Place the client in a safe resting position and assess further
- C. Give the scheduled phosphate binder with food
- D. Encourage the client to sit upright in a chair

Correct Answer: B

The client shows signs of post-dialysis hypotension. The priority is to place the client safely, such as supine if appropriate per facility practice, and assess further while preparing to report the change. Ambulation or upright positioning increases fall risk and may worsen symptoms. The phosphate binder is unrelated to the immediate instability.

3983. Which meal selection by a client on hemodialysis suggests the need for more teaching about limiting potassium?

- A. Scrambled eggs, white toast, and apple juice
- B. Chicken salad sandwich and grapes
- C. Oatmeal with blueberries and tea
- D. Baked potato, banana, and orange juice

Correct Answer: D

Potatoes, bananas, and orange juice are all high-potassium foods, making this the meal most in need of teaching. The other options are generally lower in potassium by comparison. The question tests recognition of a concentrated high-potassium combination rather than a single food item.

3984. A client receiving peritoneal dialysis reports abdominal pain and has a temperature of 38.4°C (101.1°F). The drainage is cloudy. Which action should the nurse take first?

- A. Warm the next dialysate bag to reduce cramping
- B. Slow the infusion rate for the next exchange
- C. Report the findings promptly as possible peritonitis
- D. Encourage the client to walk to improve drainage

Correct Answer: C

Abdominal pain, fever, and cloudy effluent together strongly suggest peritonitis, a serious complication of peritoneal dialysis. Prompt reporting and further intervention are needed. Warming solution or encouraging ambulation may help some comfort or drainage issues, but they do not address the urgent infectious complication.

3985. The nurse is caring for four clients. Which client should be seen first?

- A. A client on hemodialysis with a thrill present and mild itching
- B. A client after peritoneal dialysis who asks when dinner will arrive
- C. A client with an arteriovenous graft whose hand is cool and fingers are numb
- D. A client scheduled for dialysis tomorrow who wants teaching about fluid limits

Correct Answer: C

A cool hand and numb fingers distal to an arteriovenous graft suggest impaired circulation, which may indicate a serious access-related complication requiring immediate assessment. The other clients are stable or have nonurgent needs. The priority is the possible threat to perfusion.

3986. A client with chronic kidney disease has just completed hemodialysis. Which laboratory trend would most strongly support that the treatment was effective?

- A. Serum creatinine rises from 4.8 to 5.6 mg/dL
- B. Blood urea nitrogen decreases from 68 to 34 mg/dL
- C. Hemoglobin decreases from 9.2 to 8.6 g/dL
- D. White blood cell count rises from 7,000 to 9,500/mm³

Correct Answer: B

Hemodialysis removes waste products, so a decrease in blood urea nitrogen supports effective treatment. A rising creatinine would not suggest improvement, and changes in hemoglobin or white blood cell count do not best reflect immediate dialysis effectiveness. The challenge here is selecting the value most directly tied to toxin removal.

3987. Which action by the nurse caring for a client with a hemodialysis access in the right arm should be questioned immediately?

- A.** Palpating the site for a thrill once per shift
- B.** Placing a blood pressure cuff on the right arm for convenience
- C.** Inspecting the access site for redness or drainage
- D.** Teaching the client not to carry heavy items on that arm

Correct Answer: B

Applying a blood pressure cuff to the access arm can compress and damage the vascular access and should be questioned immediately. The other actions are appropriate components of access assessment and teaching. The trap is overlooking a routine task that becomes unsafe because of the dialysis access.

3988. A client on peritoneal dialysis has had poor outflow for the last two exchanges. The abdomen is firm, the client reports increasing shortness of breath, and the prescribed dwell volume was fully instilled each time. What is the nurse's best initial interpretation?

- A.** The client may be retaining dialysate, increasing abdominal pressure
- B.** The client is showing a normal response to high-protein losses
- C.** The findings most likely indicate improved toxin clearance
- D.** The catheter is definitely infected rather than obstructed

Correct Answer: A

Repeated poor outflow with a firm abdomen and shortness of breath suggests retained dialysate causing increased intra-abdominal pressure, which can impair breathing. Infection is possible in some situations, but these findings do not make it the definite cause. The key is linking poor drainage to fluid retention in the abdomen rather than assuming a vague or unrelated explanation.

3989. A client is receiving hemodialysis through an arteriovenous graft. Which assessment of the access site should the nurse report immediately?

- A.** A bruit is heard and a thrill is felt
- B.** Mild bruising is present near the incision from last week
- C.** The access arm is cool and no thrill is palpable
- D.** The client says the site feels tender when touched

Correct Answer: C

A cool extremity with no palpable thrill suggests impaired or absent blood flow through the graft, which may indicate clotting or occlusion and requires immediate reporting. A bruit and thrill are expected findings in a functioning access. Mild bruising or tenderness can occur, especially with a newer access, but loss of circulation is the priority.

3990. The nurse is caring for a client receiving continuous ambulatory peritoneal dialysis. Which finding best indicates that the prescribed exchange removed excess fluid effectively?

- A.** The outflow amount is greater than the inflow amount
- B.** The dialysate is pale yellow and clear
- C.** The client reports a feeling of fullness during the dwell time
- D.** The drainage takes 20 minutes to complete

Correct Answer: A

If the outflow exceeds the inflow, fluid has been removed from the client, showing effective ultrafiltration. Clear dialysate is reassuring for the absence of obvious infection, but it does not by itself show excess fluid removal. Fullness during dwell and drainage time do not best measure fluid removal.

3991. Before a scheduled hemodialysis treatment, which task is most important for the nurse to complete?

- A. Obtain the client's weight
- B. Offer a high-protein snack
- C. Start a large-volume IV infusion
- D. Apply lotion over the vascular access site

Correct Answer: A

A pre-dialysis weight is essential because it helps determine fluid gain since the last treatment and guides how much fluid should be removed. A snack is not the priority. Starting a large IV infusion could worsen fluid overload, and lotion should not be applied over the access site.

3992. A client asks why blood samples should not be drawn from the arm with the hemodialysis fistula. Which response by the nurse is best?

- A. It can reduce blood flow or damage the access
- B. It makes the laboratory values less accurate
- C. That arm is more likely to cause the client pain
- D. The veins in that arm are too small to use

Correct Answer: A

The fistula arm must be protected because venipuncture can injure the access, increase clotting risk, and reduce blood flow needed for dialysis. Accuracy of lab values is not the main issue. Pain and vein size are not the primary reasons for avoiding that arm.

3993. A client on chronic hemodialysis says, "My muscles keep cramping during treatment." Which explanation is most appropriate?

- A. Rapid fluid and electrolyte shifts can trigger cramping
- B. The access site is probably infected

C. The dialysate is remaining in the abdomen too long

D. The kidneys are producing too much urine

Correct Answer: A

Muscle cramping during hemodialysis commonly occurs from rapid fluid removal and shifting electrolytes. Access infection would more likely cause local redness, warmth, drainage, or systemic symptoms. Dialysate dwelling in the abdomen applies to peritoneal dialysis, not hemodialysis. Excess urine production is not the usual explanation.

3994. The nurse is teaching a client with a new peritoneal dialysis catheter how to reduce infection risk at home. Which client statement indicates a need for further teaching?

- A. I will wash my hands before touching the catheter or supplies.
- B. I will wear a mask during exchanges if instructed.
- C. I will use sterile technique when connecting the tubing.
- D. I will warm the dialysate in the microwave to save time.

Correct Answer: D

Dialysate should not be warmed in a microwave because it can create hot spots and contaminate safe technique. Hand hygiene, mask use when instructed, and sterile technique are appropriate infection-prevention measures. The trap is choosing a clearly infection-focused option, but unsafe warming is also incorrect and important teaching.

3995. A client receiving peritoneal dialysis has newly developed shoulder pain during the dwell phase. Which interpretation is most appropriate?

- A. The client is showing classic signs of worsening uremia
- B. Dialysate may be irritating the diaphragm
- C. The catheter is infected at the skin insertion site
- D. The client is developing severe hyperkalemia

Correct Answer: B

Shoulder pain during peritoneal dialysis can occur as referred pain from diaphragmatic irritation caused by dialysate. Uremia and hyperkalemia do not typically present as isolated shoulder pain during a dwell. A skin-site infection would more likely cause redness, drainage, or tenderness at the insertion site.

3996. The nurse checks a client's laboratory results before hemodialysis. Which value is most important to bring to the attention of the registered nurse or provider before treatment begins?

- A. Hemoglobin 10.2 g/dL
- B. Potassium 6.2 mEq/L
- C. Blood urea nitrogen 54 mg/dL
- D. Creatinine 5.8 mg/dL

Correct Answer: B

A potassium level of 6.2 mEq/L is dangerously high and raises concern for life-threatening dysrhythmias, so it is the priority to report. Elevated BUN and creatinine are expected in renal failure and are part of why dialysis is prescribed. Mild anemia is also common in chronic kidney disease, but hyperkalemia is the immediate threat.

3997. A client with kidney failure is prescribed peritoneal dialysis instead of hemodialysis. Which statement best describes a key difference between the two therapies?

- A. Peritoneal dialysis uses the peritoneal membrane for fluid and solute exchange
- B. Peritoneal dialysis removes waste only from the bloodstream and not from body fluid
- C. Hemodialysis requires no vascular access if done in an outpatient center
- D. Hemodialysis works by instilling solution into the abdominal cavity

Correct Answer: A

Peritoneal dialysis uses the peritoneal membrane as the semipermeable surface for exchange of fluid and solutes. Hemodialysis instead circulates blood through an external dialyzer and does require vascular access. Instilling solution into the abdomen describes peritoneal dialysis, not hemodialysis.

3998. Four hours after hemodialysis, a client becomes increasingly restless and complains of nausea and a throbbing headache. Which additional finding would most strongly support dialysis disequilibrium syndrome?

- A. Progressive confusion and a decreased level of consciousness
- B. Crackles at the lung bases and dependent edema
- C. Drainage from the fistula puncture sites
- D. A blood glucose level of 64 mg/dL

Correct Answer: A

Dialysis disequilibrium syndrome involves neurologic changes related to rapid shifts during dialysis. Worsening confusion and decreased level of consciousness strongly support that complication. Crackles and edema suggest fluid overload. Drainage from puncture sites points to bleeding, and a low glucose could explain symptoms but does not specifically support dialysis disequilibrium.

3999. A client's prescribed peritoneal dialysis exchange is complete, but the outflow is noticeably blood-tinged. The client denies abdominal pain, and vital signs are stable. What is the nurse's best initial action?

- A. Document the finding as expected after every exchange
- B. Stop all future exchanges permanently
- C. Assess the client further and report the finding according to facility policy
- D. Add heparin to the next bag without a prescription

Correct Answer: C

Blood-tinged effluent is not a finding to ignore. The nurse should assess the client further and report the finding based on agency policy and the overall clinical picture. It may have several causes and needs evaluation. Stopping all therapy permanently is inappropriate, and adding medication without a prescription is unsafe.

4000. The nurse is reviewing orders for a client who will receive hemodialysis in 30 minutes. Which order should be clarified first?

- A. Record pre-dialysis vital signs
- B. Weigh the client before transport
- C. Administer lisinopril 20 mg by mouth now

D. Assess the patency of the vascular access**Correct Answer: C**

Antihypertensive medication such as lisinopril may need to be held before hemodialysis because treatment can lower blood pressure and increase the risk of intradialytic hypotension. Pre-dialysis vital signs, weight, and access assessment are appropriate. The key reasoning is preventing treatment-related hypotension, not simply following the schedule.